



**Cochrane
Library**

Cochrane Database of Systematic Reviews

Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults (Review)

Mayo-Wilson E, Montgomery P

Mayo-Wilson E, Montgomery P.

Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults.

Cochrane Database of Systematic Reviews 2013, Issue 9. Art. No.: CD005330.

DOI: [10.1002/14651858.CD005330.pub4](https://doi.org/10.1002/14651858.CD005330.pub4).

www.cochranelibrary.com

Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults (Review)

Copyright © 2013 The Cochrane Collaboration. Published by John Wiley & Sons, Ltd.

WILEY

TABLE OF CONTENTS

ABSTRACT	1
PLAIN LANGUAGE SUMMARY	2
SUMMARY OF FINDINGS	3
BACKGROUND	6
OBJECTIVES	6
METHODS	7
RESULTS	9
Figure 1.	10
Figure 2.	12
Figure 3.	14
Figure 4.	17
Figure 5.	19
Figure 6.	21
DISCUSSION	23
AUTHORS' CONCLUSIONS	24
ACKNOWLEDGEMENTS	25
REFERENCES	26
CHARACTERISTICS OF STUDIES	45
DATA AND ANALYSES	310
Analysis 1.1. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 1 Anxiety (any measure), self-rated at post-treatment.	317
Analysis 1.2. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 2 Anxiety (any measure), self-rated at post-treatment (type of control).	319
Analysis 1.3. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 3 Anxiety (any measure), self-rated at post-treatment (by disorder).	321
Analysis 1.4. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 4 Anxiety (any measure), self-rated at post-treatment (by media).	323
Analysis 1.5. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 5 Anxiety (any measure), self-rated at post-treatment (by therapy).	325
Analysis 1.6. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 6 Anxiety (any measure), self-rated at post-treatment (by setting).	328
Analysis 1.7. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias).	330
Analysis 1.8. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 8 Specific symptoms, self-rated at post-treatment.	332
Analysis 1.9. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 9 Specific symptoms, self-rated at post-treatment (type of control).	333
Analysis 1.10. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 10 Specific symptoms, self-rated at post-treatment (by disorder).	335
Analysis 1.11. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 11 Specific symptoms, self-rated at post-treatment (by media).	337
Analysis 1.12. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 12 Specific symptoms, self-rated at post-treatment (by therapy).	339
Analysis 1.13. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 13 Specific symptoms, self-rated at post-treatment (by setting).	340
Analysis 1.14. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias).	342
Analysis 1.15. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 15 Specific symptoms, self-rated at post-treatment (studies also reporting clinician rated).	344
Analysis 1.16. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 16 Specific symptoms, clinician-rated at post-treatment.	345
Analysis 1.17. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 17 Specific symptoms, clinician-rated at post-treatment.	346

Analysis 1.18. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 18 General symptoms, self-rated at post-treatment.	347
Analysis 1.19. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 19 Behavioural Test (level), clinician-rated at post-treatment.	348
Analysis 1.20. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 20 Behavioural Test (symptoms), self-rated at post-treatment.	348
Analysis 1.21. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 21 Response (specific), self-rated at post-treatment.	349
Analysis 1.22. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 22 Response (specific), self-rated at post-treatment (type of control).	350
Analysis 1.23. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 23 Response (specific), self-rated at post-treatment (by disorder).	351
Analysis 1.24. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 24 Response (specific), self-rated at post-treatment (by media).	352
Analysis 1.25. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 25 Response (specific), self-rated at post-treatment (by therapy).	353
Analysis 1.26. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 26 Response (specific), self-rated at post-treatment (by setting).	355
Analysis 1.27. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 27 Response (specific), clinician-rated at post-treatment.	356
Analysis 1.28. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 28 Response (general), self-rated at post-treatment.	356
Analysis 1.29. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 29 Recovery, clinician-rated at post-treatment.	356
Analysis 1.30. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 30 Depression, self-rated at post-treatment.	357
Analysis 1.31. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 31 Depression, clinician-rated at post-treatment.	358
Analysis 1.32. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 32 Depression improvement, self-rated at post-treatment.	359
Analysis 1.33. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 33 Disability, self-rated at post-treatment.	359
Analysis 1.34. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 34 Disability, clinician-rated at post-treatment.	360
Analysis 1.35. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 35 Quality of Life, self-rated at post-treatment.	360
Analysis 1.36. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 36 Dropout at post-treatment.	361
Analysis 1.37. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 37 Anxiety (any measure), self-rated at follow-up.	362
Analysis 1.38. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 38 Specific symptoms, self-rated at follow-up.	363
Analysis 1.39. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 39 Specific symptoms, clinician-rated at follow-up.	363
Analysis 1.40. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 40 General symptoms, self-rated at follow-up.	364
Analysis 1.41. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 41 Behavioural Test (level), clinician-rated at follow-up.	364
Analysis 1.42. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 42 Behavioural Test (symptoms), self-rated at follow-up.	365
Analysis 1.43. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 43 Response (specific), self-rated at follow-up.	365
Analysis 1.44. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 44 Response (specific), clinician-rated at follow-up.	365
Analysis 1.45. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 45 Response (general), self-rated at follow-up.	365

Analysis 1.46. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 46 Recovery, clinician-rated at follow-up.	366
Analysis 1.47. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 47 Depression, self-rated at follow-up.	366
Analysis 1.48. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 48 Depression, clinician-rated at follow-up.	366
Analysis 1.49. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 49 Disability, self-rated at follow-up.	367
Analysis 1.50. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 50 Disability, clinician-rated at follow-up.	367
Analysis 1.51. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 51 Quality of Life, self-rated at follow-up.	368
Analysis 1.52. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 52 Anxiety (any measure), self-rated at >6m.	368
Analysis 1.53. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 53 Specific symptoms, self-rated at >6m.	368
Analysis 1.54. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 54 General symptoms, self-rated at >6m.	369
Analysis 1.55. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 55 Response (specific), self-rated at >6m.	369
Analysis 1.56. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 56 Depression, self-rated at >6m.	369
Analysis 1.57. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 57 Disability, self-rated at >6m.	370
Analysis 1.58. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 58 Quality of Life, self-rated at >6m.	370
Analysis 2.1. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 1 Anxiety (any measure), self-rated at post-treatment.	377
Analysis 2.2. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 2 Anxiety (any measure), self-rated at post-treatment (group versus individual).	378
Analysis 2.3. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 3 Anxiety (any measure), self-rated at post-treatment (by disorder).	379
Analysis 2.4. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 4 Anxiety (any measure), self-rated at post-treatment (by media).	380
Analysis 2.5. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 5 Anxiety (any measure), self-rated at post-treatment (by therapy).	381
Analysis 2.6. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 6 Anxiety (any measure), self-rated at post-treatment (by setting).	382
Analysis 2.7. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias).	383
Analysis 2.8. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 8 Specific symptoms, self-rated at post-treatment.	384
Analysis 2.9. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 9 Specific symptoms, self-rated at post-treatment (group versus individual).	385
Analysis 2.10. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 10 Specific symptoms, self-rated at post-treatment (by disorder).	386
Analysis 2.11. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 11 Specific symptoms, self-rated at post-treatment (by media).	387
Analysis 2.12. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 12 Specific symptoms, self-rated at post-treatment (by therapy).	388
Analysis 2.13. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 13 Specific symptoms, self-rated at post-treatment (by setting).	389
Analysis 2.14. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias).	390
Analysis 2.15. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 15 Specific symptoms, clinician-rated at post-treatment.	391

Analysis 2.16. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 16 General symptoms, self-rated at post-treatment.	391
Analysis 2.17. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 17 Behavioural Test (level), clinician-rated at post-treatment.	392
Analysis 2.18. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 18 Behavioural Test (symptoms), self-rated at post-treatment.	392
Analysis 2.19. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 19 Response (specific), self-rated at post-treatment.	392
Analysis 2.20. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 20 Response (specific), self-rated at post-treatment (group versus individual).	393
Analysis 2.21. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 21 Response (specific), self-rated at post-treatment (by disorder).	394
Analysis 2.22. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 22 Response (specific), self-rated at post-treatment (by media).	394
Analysis 2.23. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 23 Response (specific), self-rated at post-treatment (by therapy).	395
Analysis 2.24. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 24 Response (specific), self-rated at post-treatment (by setting).	396
Analysis 2.25. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 25 Response (specific), clinician-rated at post-treatment.	397
Analysis 2.26. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 26 Response (general), self-rated at post-treatment.	397
Analysis 2.27. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 27 Recovery, clinician-rated at post-treatment.	398
Analysis 2.28. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 28 Depression, self-rated at post-treatment.	398
Analysis 2.29. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 29 Depression, clinician-rated at post-treatment.	398
Analysis 2.30. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 30 Depression improvement, self-rated at post-treatment.	399
Analysis 2.31. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 31 Disability, self-rated at post-treatment.	399
Analysis 2.32. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 32 Disability, clinician-rated at post-treatment.	399
Analysis 2.33. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 33 Quality of Life, self-rated at post-treatment.	400
Analysis 2.34. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 34 Dropout at post-treatment.	400
Analysis 2.35. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 35 Anxiety (any measure), self-rated at follow-up.	401
Analysis 2.36. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 36 Specific symptoms, self-rated at follow-up.	401
Analysis 2.37. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 37 Specific symptoms, clinician-rated at follow-up.	402
Analysis 2.38. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 38 General symptoms, self-rated at follow-up.	402
Analysis 2.39. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 39 Behavioural Test (level), clinician-rated at follow-up.	403
Analysis 2.40. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 40 Behavioural Test (symptoms), self-rated at follow-up.	403
Analysis 2.41. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 41 Response (specific), self-rated at follow-up.	403
Analysis 2.42. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 42 Response (specific), clinician-rated at follow-up.	403
Analysis 2.43. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 43 Recovery, clinician-rated at follow-up.	404

Analysis 2.44. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 44 Depression, self-rated at follow-up.	404
Analysis 2.45. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 45 Depression, clinician-rated at follow-up.	405
Analysis 2.46. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 46 Depression improvement, self-rated at follow-up.	405
Analysis 2.47. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 47 Disability, self-rated at follow-up.	405
Analysis 2.48. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 48 Disability, clinician-rated at follow-up.	405
Analysis 2.49. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 49 Quality of Life, self-rated at follow-up.	406
Analysis 2.50. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 50 Anxiety (any measure), self-rated at >6m.	406
Analysis 2.51. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 51 Specific symptoms, self-rated at >6m.	406
Analysis 2.52. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 52 Specific symptoms, clinician-rated at >6m.	407
Analysis 2.53. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 53 General symptoms, self-rated at >6m.	407
Analysis 2.54. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 54 Behavioural Test (level), clinician-rated at >6m.	407
Analysis 2.55. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 55 Behavioural Test (symptoms), self-rated at >6m.	408
Analysis 2.56. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 56 Response (specific), self-rated at >6m.	408
Analysis 2.57. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 57 Response (specific), clinician-rated at >6m.	408
Analysis 2.58. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 58 Response (general), self-rated at >6m.	409
Analysis 2.59. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 59 Recovery, clinician-rated at >6m.	409
Analysis 2.60. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 60 Depression, self-rated at >6m.	409
Analysis 2.61. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 61 Depression improvement, self-rated at >6m.	409
Analysis 2.62. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 62 Disability, self-rated at >6m.	410
Analysis 2.63. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 63 Quality of Life, self-rated at >6m.	410
APPENDICES	410
CONTRIBUTIONS OF AUTHORS	412
DECLARATIONS OF INTEREST	412
SOURCES OF SUPPORT	412
DIFFERENCES BETWEEN PROTOCOL AND REVIEW	412
INDEX TERMS	412

[Intervention Review]

Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults

Evan Mayo-Wilson¹, Paul Montgomery²¹Research Department of Clinical, Educational and Health Psychology, University College London, London, UK. ²Centre for Evidence-Based Intervention, Social Policy & Intervention, University of Oxford, Oxford, UK**Contact:** Evan Mayo-Wilson, Research Department of Clinical, Educational and Health Psychology, University College London, 1-19 Torrington Place, London, WC1E 7HB, UK. evan.mayowilson@gmail.com, e.mayo-wilson@ucl.ac.uk.**Editorial group:** Cochrane Common Mental Disorders Group.**Publication status and date:** New, published in Issue 9, 2013.**Citation:** Mayo-Wilson E, Montgomery P. Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults. *Cochrane Database of Systematic Reviews* 2013, Issue 9. Art. No.: CD005330. DOI: [10.1002/14651858.CD005330.pub4](https://doi.org/10.1002/14651858.CD005330.pub4).

Copyright © 2013 The Cochrane Collaboration. Published by John Wiley & Sons, Ltd.

ABSTRACT

Background

Anxiety disorders are the most common mental health problems. They are chronic and unremitting. Effective treatments are available, but access to services is limited. Media-delivered behavioural and cognitive behavioural interventions (self-help) aim to deliver treatment with less input from professionals compared with traditional therapies.

Objectives

To assess the effects of media-delivered behavioural and cognitive behavioural therapies for anxiety disorders in adults.

Search methods

Published and unpublished studies were considered without restriction by language or date. The Cochrane Depression, Anxiety and Neurosis Review Group's Specialized Register (CCDANCTR) was searched all years to 1 January 2013. The CCDANCTR includes relevant randomised controlled trials from the following bibliographic databases: *The Cochrane Library* (all years), EMBASE (1974 to date), MEDLINE (1950 to date) and PsycINFO (1967 to date). Complementary searches were carried out on Ovid MEDLINE (1950 to 23 February 2013) and PsycINFO (1987 to February, Week 2, 2013), together with International trial registries (the trials portal of the World Health Organization (ICTRP) and ClinicalTrials.gov). Reference lists from previous meta-analyses and reports of randomised controlled trials were checked, and authors were contacted for unpublished data.

Selection criteria

Randomised controlled trials of media-delivered behavioural or cognitive behavioural therapy in adults with anxiety disorders (other than post-traumatic stress disorder) compared with no intervention (including attention/relaxation controls) or compared with face-to-face therapy.

Data collection and analysis

Both review authors independently screened titles and abstracts. Study characteristics and outcomes were extracted in duplicate. Outcomes were combined using random-effects models, and tests for heterogeneity and for small study bias were conducted. We examined subgroup differences by type of disorder, type of intervention provided, type of media, and recruitment methods used.

Main results

One hundred and one studies with 8403 participants were included; 92 studies were included in the quantitative synthesis. These trials compared several types of media-delivered interventions (with varying levels of support) with no treatment and with face-to-face

interventions. Inconsistency and risk of bias reduced our confidence in the overall results. For the primary outcome of symptoms of anxiety, moderate-quality evidence showed medium effects compared with no intervention (standardised mean difference (SMD) 0.67, 95% confidence interval (CI) 0.55 to 0.80; 72 studies, 4537 participants), and low-quality evidence of small effects favoured face-to-face therapy (SMD -0.23, 95% CI -0.36 to -0.09; 24 studies, 1360 participants). The intervention was associated with greater response than was seen with no treatment (risk ratio (RR) 2.34, 95% CI 1.81 to 3.03; 21 studies, 1547 participants) and was not significantly inferior to face-to-face therapy in these studies (RR 0.78, 95% CI 0.56 to 1.09; 10 studies, 575 participants), but the latter comparison included versions of therapies that were not as comprehensive as those provided in routine clinical practice. Evidence suggested benefit for secondary outcome measures (depression, mental-health related disability, quality of life and dropout), but this evidence was of low to moderate quality. Evidence regarding harm was lacking.

Authors' conclusions

Self-help may be useful for people who are not able or are not willing to use other services for people with anxiety disorders; for people who can access it, face-to-face cognitive behavioural therapy is probably clinically superior. Economic analyses were beyond the scope of this review.

Important heterogeneity was noted across trials. Recent interventions for specific problems that incorporate clinician support may be more effective than transdiagnostic interventions (i.e. interventions for multiple disorders) provided with no guidance, but these issues are confounded in the available trials.

Although many small trials have been conducted, the generalisability of their findings is limited. Most interventions tested are not available to consumers. Self-help has been recommended as the first step in the treatment of some anxiety disorders, but the short-term and long-term effectiveness of media-delivered interventions has not been established. Large, pragmatic trials are needed to evaluate and to maximise the benefits of self-help interventions.

PLAIN LANGUAGE SUMMARY

Self-help for anxiety disorders

Anxiety disorders are common. They interfere with normal living, and they tend not to go away without treatment. Effective treatments are available, including cognitive behavioural therapy. These are known to work when delivered in person, but many people cannot access face-to-face treatment. This review examined 101 clinical trials of self-help and statistically analysed 92 of them. In these trials, 8403 people received self-help or were assigned to a control condition.

Overall, self-help with some support from a professional appears to be more effective than no treatment. Only half of the people who used self-help were better at the end of treatment, but self-help may still be considered effective because people with anxiety do not tend to get better without treatment. Self-help may be less effective than normal face-to-face therapy. Some results were difficult to interpret because the effects of treatments varied and the risk of overestimating the results was serious because of limitations in the study methods. We conclude that self-help is probably better than no treatment, but many people with an anxiety disorder would get better results from treatment provided by a skilled psychologist. Furthermore, most of the self-help materials used in these studies are intended for research and are not available to the public, so the results reported here may not apply to commercially available products.

SUMMARY OF FINDINGS

Summary of findings for the main comparison.

Media-delivered interventions compared with no intervention for anxiety disorders

Patient or population: adults with anxiety

Settings: recruited from referrals and advertising

Intervention: media-delivered cognitive behavioural therapy or behavioural therapy

Comparison: wait-list, no intervention, treatment as usual

Outcomes	Illustrative comparative risks* (95% CI)		Relative effect (95% CI)	No. of participants (studies)	Quality of the evidence (GRADE)	Comments
	Assumed risk	Corresponding risk				
	No intervention	Media-delivered intervention				
Anxiety (any measure), self-rated at post-treatment		Across studies, the typical scores in intervention conditions were, on average, 0.67 standard deviations better (with a 95% CI ranging from 0.55 to 0.78 standard deviations) than scores in the control conditions		4537 (72 studies)	⊕⊕⊕⊖ moderate ¹	This is a medium effect
Response (specific), self-rated at post-treatment	180 per 1000	421 per 1000	RR 2.34 (1.81 to 3.03)	1547 (21 studies)	⊕⊕⊖⊖ low ^{1,2}	The probability of clinical improvement doubled, but most people did not improve
Depression, self-rated at post-treatment		Across studies, the typical scores in intervention conditions were, on average, 0.47 standard deviations better (with a 95% CI ranging from 0.36 to 0.58 standard deviations) than scores in the control conditions		3682 (50 studies)	⊕⊕⊕⊖ moderate ¹	This is a medium effect
Quality of life, self-rated at post-treatment		Across studies, the typical scores in intervention conditions were, on average, 0.36 standard deviations better (with a 95% CI ranging from 0.22 to 0.49 standard deviations) than scores in the control conditions		1344 (18 studies)	⊕⊕⊖⊖ low ^{1,2}	This is a medium effect

*The basis for the **assumed risk** (e.g. the median control group risk across studies) is provided in footnotes. The **corresponding risk** (and its 95% confidence interval) is based on the assumed risk in the comparison group and the **relative effect** of the intervention (and its 95% CI).

CI: Confidence interval; **RR:** Risk ratio.

GRADE Working Group grades of evidence:

High quality: Further research is very unlikely to change our confidence in the estimate of effect.

Moderate quality: Further research is likely to have an important impact on our confidence in the estimate of effect and may change the estimate.

Low quality: Further research is very likely to have an important impact on our confidence in the estimate of effect and is likely to change the estimate.

Very low quality: We are very uncertain about the estimate.

¹Downgraded for inconsistency.

²Downgraded for risk of bias.

Summary of findings 2.

Media-delivered interventions compared with face-to-face intervention for anxiety disorders

Patient or population: adults with anxiety

Settings: recruited from referrals and advertising

Intervention: media-delivered cognitive behavioural therapy or behavioural therapy

Comparison: face-to-face interventions

Outcomes	Illustrative comparative risks* (95% CI)		Relative effect (95% CI)	No. of participants (studies)	Quality of the evidence (GRADE)	Comments
	Assumed risk	Corresponding risk				
	Face-to-face intervention	Media-delivered intervention				
Anxiety (any measure), self-rated at post-treatment		SMD -0.23 (-0.36 to -0.09) Across studies, the typical scores in intervention conditions were, on average, 0.23 standard deviations worse (with a 95% CI ranging from 0.09 to 0.36 standard deviations) than scores in the control conditions		1360 (24)	⊕⊕⊕⊖ low 2,3	This is a small effect in favour of face-to-face intervention
Response (specific), self-rated at post-treatment	537 per 1000	419 per 1000	RR 0.78 (0.56 to 1.09)	575 (10)	⊕⊕⊕⊖ very low 1,2,3	The difference was not statistically significant

Depression, self-rated at post-treatment	Across studies, the typical scores in intervention conditions were, on average, 0.01 standard deviations worse (with a 95% CI ranging from -0.21 to 0.22 standard deviations) than scores in the control conditions	906 (13)	⊕⊕⊕⊕ very low 1,2,3	The difference was not statistically significant
Quality of life, self-rated at post-treatment	Across studies, the typical scores in intervention conditions were, on average, 0.08 standard deviations better (with a 95% CI ranging from -0.23 to 0.38 standard deviations) than scores in the control conditions	282 (4)	⊕⊕⊕⊕ very low 1,2,3	The difference was not statistically significant

*The basis for the **assumed risk** (e.g. the median control group risk across studies) is provided in footnotes. The **corresponding risk** (and its 95% confidence interval) is based on the assumed risk in the comparison group and the **relative effect** of the intervention (and its 95% CI).

CI: Confidence interval; **RR:** Risk ratio.

GRADE Working Group grades of evidence:

High quality: Further research is very unlikely to change our confidence in the estimate of effect.

Moderate quality: Further research is likely to have an important impact on our confidence in the estimate of effect and may change the estimate.

Low quality: Further research is very likely to have an important impact on our confidence in the estimate of effect and is likely to change the estimate.

Very low quality: We are very uncertain about the estimate.

¹Downgraded for inconsistency.

²Downgraded for risk of bias.

³Downgraded for indirectness.

BACKGROUND

Description of the condition

Anxiety disorders include generalised anxiety disorder (GAD), obsessive compulsive disorder (OCD), panic disorder (with or without agoraphobia), social phobia (social anxiety disorder) and specific phobias (APA 1994). They are characterised by excessive fears and worries that are difficult to control. Avoidance of feared situations or stimuli (including thoughts) is a cardinal behavioural feature of most anxiety disorders.

Anxiety disorders are the most common class of mental disorders. They are seen across cultures (Demyttenaere 2004) and will be experienced by about 18% of adults every year and by about 32% of people during their lifetimes (Kessler 2005a; Kessler 2005b). Half of all anxiety disorders begin by the age of 11 years (Kessler 2005a; Kessler 2005b), and most are chronic and unremitting (Bruce 2005). Anxiety reduces quality of life (Cramer 2005; Rapaport 2005; Saarni 2007) and contributes to disability (Bloom 2011), causing severe occupational impairment and lost productivity (Greenberg 1999; Ormel 1994). Despite their severity and prevalence, few people with anxiety disorders ever obtain professional treatment, and fewer obtain specialty care (Wittchen 2002).

Description of the intervention

Cognitive behavioural therapy (CBT) and behavioural therapy (BT) are well-established and effective treatments in the short and medium term (Mitte 2005a; Mitte 2005b; Butler 2006). They are the most researched therapies for anxiety, and they are recommended by the National Institute for Health and Care Excellence (NICE) as front-line treatments for both adults and young people (Kendall 2011; NCCMH 2006; Pilling 2013). Despite its effectiveness, face-to-face therapy is expensive and difficult to access.

To increase access to care and to reduce burden on therapists, computer programmes, books and other types of media have been developed to deliver CBT and BT. *Self-help* has been defined as "(a) a therapeutic intervention administered through text-audiotape, videotape or computer text, or through group meetings or individual exercises such as 'therapeutic writing', and (b) designed to be conducted predominantly independently of professional contact" (Bower 2001). *Guided self-help* usually involves minimal contact that is primarily "of supportive or facilitative nature, and is meant to support the patient in working through the standardised psychological treatment" (Cuijpers 2010); it is commonly prescribed in primary and secondary care (Clark 2009). An overwhelming majority of behavioural and cognitive therapists use self-help materials, most often in addition to traditional therapy (Keeley 2002).

How the intervention might work

Behavioural therapy has proved effective in the treatment of people with anxiety. Exposure therapy puts participants in direct and prolonged contact with feared situations (Marks 1971). Exposure in vivo (Emmelkamp 1975) aims to evoke fearful responses and to help participants develop coping strategies that will generalise to everyday circumstances (Foa 1986). Some reviews suggest that progressive muscle relaxation (Jacobson 1938) and controlled breathing are efficacious (Conrad 2007). Little evidence of long-term benefit has been found when relaxation is used alone; muscle

relaxation is sometimes used as an attentional control in the evaluation of treatments for anxiety. Applied relaxation teaches users to use relaxation techniques as coping mechanisms during exposure therapy, and it may provide some other benefits (Öst 1987; Öst 1988).

Cognitive behavioural therapy often includes (1) behavioural experiments, which aim to test one's fears rather than habituate to a stimulus (Bennett-Levy 2004), and (2) cognitive restructuring, which teaches people to recognise negative thoughts as they occur and to challenge unrealistic thoughts (Wells 1997). Treatment for anxiety may include other specific techniques, including exposure with ritual prevention for OCD (Steketee 1982), attention training for social anxiety (Rapee 2009) and meta-cognitive training for GAD (Wells 2006). CBT may also include many non-specific components that are common to effective forms of psychotherapy (e.g. goal setting, psychoeducation).

Why it is important to do this review

Media-delivered interventions are intended to be inexpensive and easily accessible. Many literature reviews (Bessell 2002; Bower 2001; Glasgow 1978; Kaltenthaler 2002; Newman 2003; Richards 2003b; Riordan 1989; Schrank 1981; Stevens 1982; Mataix-Cols 2006; Przeworski 2006; Pull 2006; van Boeijen 2005b) and meta-analyses (Andrews 2010; Bower 2001; Cuijpers 2009; Cuijpers 2010; den Boer 2004; Gould 1993b; Griffiths 2006; Griffiths 2010b; Haug 2012; Hirai 2006; Kaltenthaler 2008; Lewis 2012; Marrs 1995; Menchola 2007; Norton 2007; Reger 2009; Scogin 1990; Spek 2007) have suggested that media-delivered CBT may be effective in treating psychological problems, including anxiety, alone or in conjunction with other interventions.

Differences in inclusion criteria and methodological inconsistencies contribute to different conclusions across these reviews. An analysis of interventions for GAD found a large overall effect ($g = 0.80$, 95% CI 0.29 to 1.30) but significant heterogeneity (Mitte 2005a), similar to a recent review (Haug 2012), which reported a large effect across disorders ($g = 0.78$, 95% CI 0.67 to 0.90). However, another review (Bower 2001) reports a much smaller effect of self-help manuals ($d = 0.41$; 95% CI 0.09 to 0.72), similar to an earlier review of bibliotherapy for many conditions (Marrs 1995). Several reviews reference the *Cochrane Handbook for Systematic Reviews of Interventions*, but one summed items on the risk of bias tool and failed to follow Cochrane methods for assessing each item (Andrews 2010). Despite the title, one recent review was not systematic (Wade 2010). A recent meta-analysis (Lewis 2012) missed many studies that should have been included and suggested that media-delivered interventions have large effects. The most recent analysis used single coding and included no assessment of bias (Haug 2012). Research in this area is advancing quickly, with many studies reported each year, and a rigorous and up-to-date review is needed to estimate the effects of media-delivered interventions provided with or without professional support.

OBJECTIVES

To assess the effects of media-delivered behavioural and cognitive behavioural therapies for anxiety disorders in adults.

METHODS

Criteria for considering studies for this review

Types of studies

Randomised controlled trials and cluster-randomised trials with a parallel-group design, in which intervention and control groups were enrolled concurrently.

Types of participants

Adults (older than 18 years) with an anxiety disorder (as measured through diagnostic interview or questionnaire) other than post-traumatic stress disorder (PTSD) or acute stress disorder were included.

We included studies of health anxiety because it shares the cardinal features of anxiety disorders, and it is included in the *Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition (DSM-V)*, as “illness anxiety disorder”.

We excluded PTSD and acute stress disorder, which differ from other anxiety disorders. They may involve physical injury, difficulty separating past from present events and feelings of guilt. Reactions to severe stress are treated separately in the *Tenth Revision of the International Classification of Diseases (ICD-10)* and in *DSM-V*.

Studies of people experiencing worries or stress as the result of upcoming events (e.g. medical procedures) were excluded unless participants demonstrated symptoms of an underlying anxiety disorder (e.g. phobic avoidance of dental examinations). Other fears were excluded (e.g. “test anxiety”, “fear of public speaking”) unless participants met criteria for an anxiety disorder (e.g. specific phobia, social anxiety).

Studies including participants with other problems (e.g. depression) were eligible if participants with anxiety (including those with co-morbid problems) could be separated from other participants (i.e. those whose problems do not include anxiety), or if most of the population had an anxiety disorder. In the absence of diagnostic interviews, studies were included when average scores on validated measures of anxiety were indicative of caseness.

No restrictions by setting were applied.

Types of interventions

Experimental intervention

Interventions must have delivered CBT or BT using printed materials, audio recordings, video recordings, or computers (including Internet), or via some combination of media. It must have been possible to use them as stand-alone interventions (i.e. without a therapist), but we included studies with researcher or therapist contact and support. Computer programmes that could be used without the aid of a therapist were included whilst virtual reality systems, which use technology to facilitate therapist-delivered interventions, were excluded. Interventions that encouraged users to enlist the help of a spouse or a friend were included.

We included studies of media-delivered interventions alone and studies of media-delivered interventions as adjuncts to treatment as usual or another intervention.

Comparator

We compared media-delivered interventions with no intervention and with face-to-face behavioural or cognitive behavioural interventions. Only intervention arms in which the difference was a media-delivered intervention were compared. Thus, ‘10 hours of therapy’ versus ‘10 hours of therapy plus booklet-delivered CBT’ was eligible; ‘10 hours of therapy’ versus ‘5 hours of therapy plus booklet-delivered CBT’ was not eligible. If data had been available, we also would have compared media-delivered interventions with medication, psychoeducation and other forms of media-delivered interventions.

If it had been possible to analyse the effects of media-delivered interventions with and without adjunct medication, we would have treated these separately; most trials included participants with and without concurrent medication, so it was not possible to separate “standalone” and “adjunct” treatments. Instead, we report the number of participants in each trial who are taking medication.

Types of outcome measures

Clinician- and self-rated measures were analysed separately because service users and clinicians may have different views about a person’s improvement, and because these outcomes may be at risk of bias for different reasons.

Primary outcomes

1. Symptoms of anxiety:

a) Continuous symptom measures (e.g. Agoraphobic Cognitions Questionnaire (Chambless 1984), Beck Anxiety Inventory (Beck 1988), Liebowitz Social Anxiety Scale (Liebowitz 1987));

b) Response (e.g. Clinical Global Impression (Zaider 2003));

c) Recovery (e.g. no longer met criteria for diagnosis).

Secondary outcomes

2. Behavioural test (level of hierarchy achieved).

3. Behavioural test (subjective units of distress).

4. Depression.

5. Mental-health related disability.

6. Quality of life.

7. Dropout.

Timing of outcome assessment

Outcome measures were grouped by length of follow-up. We analysed outcomes at post-treatment, at follow-up less than six months after randomisation and at follow-up longer than six months after randomisation. For outcomes measured at multiple time points in a single period, we included the longest time point after randomisation.

Search methods for identification of studies

Published and unpublished studies were considered without language restriction, although all searches were conducted in English and all contacts with authors were initiated in English. No date restrictions were applied.

Electronic searches

Searches were conducted using the following databases ([Appendix 1](#)).

- The Cochrane Depression, Anxiety and Neurosis Review Group's Specialized Register (CCDANCTR-Studies and CCDANCTR-References) (all years to 1 January 2013). This search was conducted at the editorial base by the Group's Trials Search Co-ordinator (TSC).
- The Cochrane Central Register of Controlled Trials (CENTRAL) (all years to 2 April 2012) restricted to records submitted from the CCDANCTR-References Register (SR-DEPRESSN). Original search was conducted by the author team.
- Ovid MEDLINE (all years to 23 February 2013).
- Ovid PsycINFO (all years to February, Week 2, 2013).

International trial registries were also searched via the World Health Organization's trials portal ([ICTRP](#)) and [ClinicalTrials.gov](#) to identify unpublished or ongoing studies (all years to 1 January 2013).

Searching other resources

References from previous meta-analyses and from articles retrieved during the search were examined for additional studies, and authors of included studies were contacted for unpublished data.

Data collection and analysis

Selection of studies

Two review authors independently screened titles and abstracts to determine which were eligible for inclusion. We were not blind to study authors, institutions, journal of publication or results. Disagreements regarding eligibility were resolved by seeking additional information and through discussion.

Data extraction and management

Data regarding methodology and outcomes were extracted independently in Excel by EM-W and a second rater.

We included the following information from each study:

- Year;
- Location (country, urban/rural);
- Method of recruitment;
- Diagnosis (*DSM* category, method of assessment);
- Inclusion criteria;
- Risk of bias (see later).

Participants:

- Socio-demographics (e.g. age, sex);
- Co-morbidities.

For each intervention and comparison group of interest:

- Type of intervention;
- Type of media;
- Duration;
- Contact with participants.

For each outcome of interest:

- Time points (collected and reported);
- Loss to follow-up.

Main comparisons

1. Media-delivered intervention versus no treatment.
2. Media-delivered intervention versus face-to-face intervention.
3. Media-delivered intervention versus psychoeducation.
4. Media-delivered intervention versus medication.

Assessment of risk of bias in included studies

EM-W and a second rater (JJ, RC or AH) coded each included study using the Cochrane Collaboration's tool for assessing risk of bias ([Higgins 2011](#)). We judged whether each study was at low, high or unclear risk of bias in relation to sequence generation; allocation concealment; blinding of personnel; blinding of outcome assessors; incomplete outcome data; selective outcome reporting; and other sources of bias. Disagreements were resolved through discussion and by seeking further information.

Measures of treatment effect

Studies often report outcomes using multiple definitions and outcome measures. We gave preference to data that involved the least manipulation by authors or inference by review authors, that is, we extracted raw values at endpoint (e.g. means, standard deviations) rather than calculated effect sizes (e.g. Cohen's *d*).

Unit of analysis issues

Cluster-randomised trials

For each cluster-randomised trial, we would have determined whether or not its data incorporated sufficient controls for clustering (e.g. robust standard errors, hierarchical linear models). If the data did not have proper controls, then we would have attempted to obtain an appropriate estimate of the data's intra-cluster correlation coefficient (ICC). If we could not find an estimate in the report of the trial, then we would have requested an estimate from the trial report authors. If the authors could not provide an estimate, then we would have obtained one from a similar study and conducted a sensitivity analysis to determine whether the results were robust when different values were imputed. We would have used the ICC estimate to control the trial's data for clustering ([Higgins 2011](#)).

Cross-over trials

For cross-over trials, we extracted and analysed data from the first period only.

Studies with multiple treatment groups

For factorial studies, we included all comparisons. In other studies with multiple eligible intervention groups (e.g. interventions with or without a specific component), all eligible intervention groups were combined to estimate an average treatment effect compared with the control.

Dealing with missing data

Missing data, and methods for imputing such data, may affect the magnitude and direction of effects. For all analyses, we attempted

to include all randomly assigned study participants. When analyses were reported for completers and controlled for the effects of missing cases (e.g. using mixed effects), we extracted the latter. If data were missing for some cases, or if reasons for dropout were not reported, we contacted study authors to request missing data and further information about dropouts. We conducted a sensitivity analysis that excluded studies at high risk of bias (see [Sensitivity analysis](#)).

Assessment of heterogeneity

Differences among included studies are discussed in terms of their participants, interventions, outcomes and methods. For each meta-analysis, we also visually inspected forest plots to see whether the confidence intervals of individual studies had poor overlap (a rough indication of statistical heterogeneity); conducted a χ^2 test; and calculated the I^2 statistic ([Higgins 2011](#)). We considered meta-analyses to have heterogeneity when the P value for χ^2 was less than 0.10 and I^2 was greater than 25%.

Assessment of reporting biases

For each meta-analysis that included 10 or more studies, we drew a funnel plot and looked for asymmetry to assess the possibility of small study or reporting bias. We also compared effects using random-effects and fixed-effect models (see [Sensitivity analysis](#)), and we identified unpublished studies through correspondence and trial registration databases.

Data synthesis

In studies of transdiagnostic interventions (i.e. interventions for multiple anxiety disorders) that reported outcomes separately for participants with specific diagnoses (e.g. panic disorder, social anxiety), symptoms of anxiety were extracted for each diagnostic group where possible.

We used Review Manager (RevMan) Version 5.1 ([Review Manager 2011](#)) to conduct all meta-analyses. Risk ratios (RRs) and 95% confidence intervals (CIs) were calculated for dichotomous outcomes and were combined with the use of Mantel-Haenszel methods. Standardised mean differences (SMDs) and 95% CIs were calculated for continuous measures and were combined by using inverse variance methods. Random-effects models were used because studies included different interventions and populations. When studies reported more than one measure of a particular outcome (e.g. symptoms of social anxiety measured using two scales), we averaged the results in Comprehensive Meta-Analysis Version 2 software ([Borenstein 2005](#)) before entering them into RevMan (e.g. [Imdad 2010](#)); the specific measures included in

each analysis for each study are listed in the [Characteristics of included studies](#). For all analyses, the area to the right of the 'line of no effect' (SMD > 0, RR > 1) indicates a favourable outcome for media-delivered interventions.

Subgroup analysis and investigation of heterogeneity

For the primary outcome, we conducted the following subgroup analyses to explore possible differences across the disorders and interventions included in this review. Because many factors co-varied (e.g. several research groups contributed multiple studies in which the intervention and the population were very similar), these were considered hypothesis generating rather than hypothesis testing.

1. Types of controls.
 - a. Wait-list, attention controls, treatment as usual, adjunct to a treatment versus other treatment alone.
 - b. Group psychotherapy, individual psychotherapy.
2. Types of disorders: GAD, health anxiety, mixed populations, OCD, panic disorder, social anxiety, specific phobias.
3. Types of interventions: CBT, exposure/desensitisation, relaxation.
4. Types of media: book, computer/Internet, other.
5. Types of recruitment: advertising only, referral only, other.

In future updates, we plan to investigate the level of therapist contact as a subgroup analysis.

Sensitivity analysis

We conducted the following sensitivity analyses to determine whether findings were robust to methodological decisions made throughout the review process.

1. We repeated the analyses using fixed-effect models.
2. To investigate the influence of bias, we repeated the primary meta-analysis by excluding studies at high risk of bias as the result of incomplete outcome data, which we considered the most important source of bias in the available data.

RESULTS

Description of studies

Results of the search

The searches retrieved 1456 (CCDANCTR) and 1323 (Medline and PsycINFO) citations ([Figure 1](#)). After duplicates were removed electronically, two review authors screened 2801 abstracts.

Figure 1. Study flow diagram.

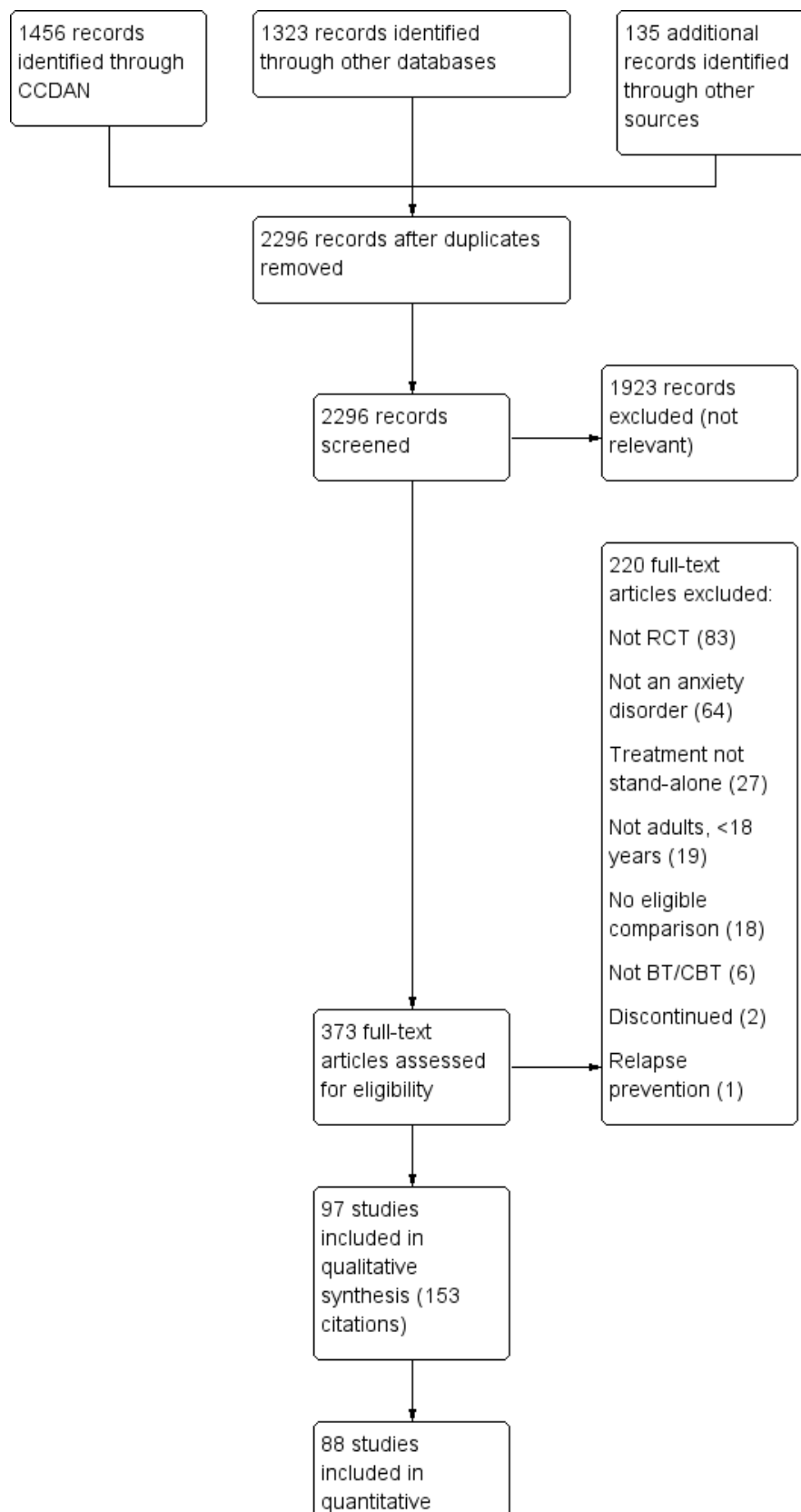


Figure 1. (Continued)

included in
quantitative
synthesis
(meta-analysis)

[77 versus
no-intervention,
29 versus
face-to-face
intervention]

Included studies

One hundred and one studies reported in 159 papers were included. Eighteen studies included multiple eligible comparisons, which we treated separately, and 21 studies included multiple eligible intervention groups, which we combined for analysis.

Included studies randomly assigned 8403 participants (mean 83, median 60); the largest included 274 participants with anxiety, depression or mixed anxiety and depression ([Proudfoot 2004a](#)). Across studies, approximately 4277 participants received media-delivered intervention, 2850 received no intervention, 916 received face-to-face intervention and 383 were assigned to a group not included in this review.

Ten included studies were not included in the quantitative synthesis because they reported no outcome data that could be analysed ([Barrera 1977](#); [Botella 2009](#); [Holdsworth 1996](#); [Koszycki 2011](#); [Salaberria 1995](#); [Tyrer 1988](#)) or compared only two eligible interventions ([Fraser 2001](#); [Matthews 2011](#); [Titov 2010b](#); [Tortella-Feliu 2011](#)). Thus, characteristics are described for 101 studies, and 91 are included in a meta-analysis.

Design

All studies randomly assigned individuals ([Characteristics of included studies](#)). One study randomly assigned general practices, then randomly assigned participants within practices ([van Boeijen 2005a](#)).

Setting

Studies were reported between 1973 and 2012; more were reported in 2011 than in any previous year. Most were conducted in four countries: UK (26), Australia (21), USA (17) and Sweden (20).

Participants

Studies included participants who were predominantly female (67% mean of means), white (94%) and in their 30s (37 years), who had more than a high school education (13 years). Most had long-standing disorders (14 years), and many participants were taking an antidepressant or some other psychiatric medication (35%).

Interventions targeted panic (19 panic disorder, 3 panic disorder with agoraphobia, 3 panic attacks), social anxiety (19 unspecified, 2 specific subtype, 2 general subtype), specific phobias (11 spider, 2 snake, 1 flying, 1 heights), GAD (10), OCD (5), health anxiety (2), several diagnoses (11) or unspecified symptoms of anxiety (14). Some studies included participants with symptoms of anxiety or depression (9); these were included because a minority of

participants had depression only, and further details are included in the [Characteristics of included studies](#) ([Fletcher 2005](#); [Grime 2004](#); [Holdsworth 1996](#); [Mauder 2009](#); [Mead 2005](#); [Proudfoot 2004a](#); [Seekles 2011](#); [Tyrer 1988](#); [Zetterqvist 2003](#)). Most studies assessed participants through a clinical interview (61 face-to-face interviews, 28 telephone interviews) or relied on a previous diagnosis (2), although some used questionnaires (10).

Participants were recruited through advertising (56), referral (24), mixed advertising and referral (15), college classes ([Lewis 1978](#); [Muller 2011](#); [Shoenberger 2008](#)), prison ([Mauder 2009](#)) or workplace ([Grime 2004](#)); the recruitment strategy was unclear in one study ([Tolin 2011](#)). Exclusion rates varied with method of recruitment and ranged from 0% to 99% (mean 48%). Studies reported that participants were excluded for suicidality (66), depression (54), drug or alcohol misuse (53) or physical health problems (33). Studies also explicitly excluded participants who had received psychotherapy previously (20) or were receiving it currently (64). Most studies (66) restricted the amount or type of medication that participants could be taking or asked participants to remain on a stable dose throughout the study.

Interventions

Of the included studies, 81 compared a media-delivered intervention with no treatment (including wait-list and attention controls). Additionally, 29 compared a media-delivered intervention with a face-to-face intervention. Four compared a media-delivered intervention only with another form of media-delivered intervention; studies including more than one type of media-delivered intervention and those comparing multiple forms of media-delivered intervention only were idiosyncratic in design and objectives, thus a systematic synthesis was not possible. No studies that included a medication group reported outcomes that could be synthesised ([Koszycki 2011](#); [Tyrer 1988](#)). Some studies provided information about anxiety or relaxation instructions to control for non-specific intervention factors ([Al-Kubaisy 1992](#); [Carlbring 2003](#); [Furmark 2009b](#); [Gilroy 2000](#); [Greist 2002](#); [Marks 2004](#); [Schneider 2005](#)); we treated these as attention controls rather than as active comparisons because the comparisons were described with the use of terms like “relaxation placebo” ([Gilroy 2000](#)) and “self-relaxation as a placebo” ([Marks 2004](#)).

In most studies, interventions were developed by the authors (89). Exposure and behavioural experiments were the most common components (84). Most studies provided a version of cognitive behavioural therapy (70); others tested exposure alone (19), desensitisation alone (2), muscle relaxation (6), applied relaxation (3) or “self-examination therapy”, which the author

described as a type of CBT but which appears similar to problem solving (Bowman 1997). Interventions were designed to take up to 12 weeks to complete (mean 57 days to post-treatment assessment), but some consisted of a single session (Heading 2001; Matthews 2011), and others did not specify how long the interventions should take to complete, with some lasting up to six months before assessment (Carlborg 2003; Wright 1997a).

Almost as many studies examined the use of books and booklets (39) as studied Internet-delivered interventions (43). Most Internet-delivered interventions did not exceed the capabilities of printed materials, that is, they provided static content on a Website or in a PDF. Some studies tested computer-delivered interventions (11), mixed media (8) or a computerised telephone system (Greist 2002), and some of these were interactive. Interventions in 13 studies (including 10 of the computerised interventions) had to be used in a general practice office, a clinic, or a workplace.

Therapist contact

In most studies, participants had some contact with providers during treatment, and this is an important source of heterogeneity across studies (82). Contact occurred face-to-face (35), by e-mail (22), by e-mail and phone (11), by e-mail and short message service (SMS) (1), by Internet forum only (1) or by telephone (12). When providers had contact with participants, this ranged from 10 minutes to about 6 hours. When studies with no contact and studies in which some contact was common to both groups were excluded (Kenardy 2003; Salaberría 1995), about eight (mean of means) contacts during treatment (six median of means) lasted about 103 (standard deviation (SD) 92) minutes (median 81). A few studies included automated reminders (e-mail or SMS), which we recorded separately.

Excluded studies

We immediately excluded studies that were not randomised controlled trials, treatments for post-traumatic stress, treatments

for stress associated with medical procedures and other interventions that were not relevant to this review. In other studies, participants did not have anxiety disorders (64); these included studies of hoarding, perfectionism, public speaking, specific fears, stress, test anxiety and trichotillomania (currently listed as an impulse control disorder). Additionally, we excluded studies in which participants were younger than 18 years of age (21), interventions were not stand-alone (27) or no eligible comparison was available (18). In others, the intervention was not BT or CBT, including a book about snakes (2) and interventions that were educational only (2), attentional retraining alone (2) or designed for group leaders (1; Characteristics of excluded studies). We also excluded a relapse prevention study (Wright 1997) that followed another study in this review (Febbraro 1997a). Ninety studies were identified through correspondence and the checking of reference lists and were excluded.

Two studies appeared likely to be eligible but were discontinued early because of poor recruitment; neither reported any results (Hetherington 2004; Jones 2002b).

Ongoing studies

We identified 11 studies that appear to be ongoing (Characteristics of ongoing studies).

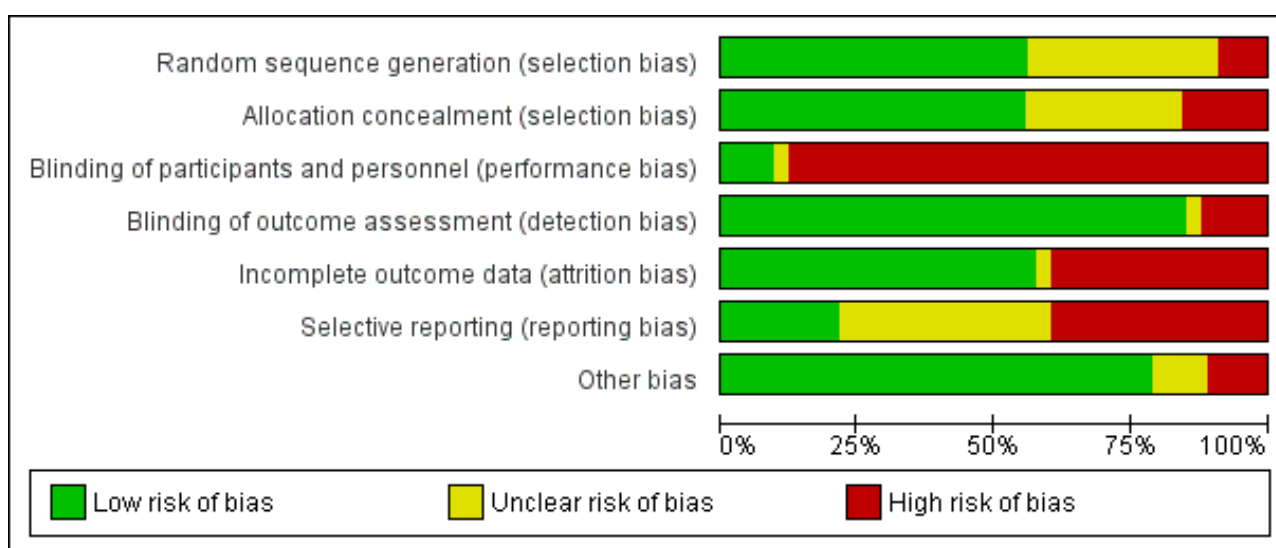
Studies awaiting classification

We identified 15 studies that appear to be completed but not yet reported (Characteristics of studies awaiting classification).

Risk of bias in included studies

Because of the nature of the intervention, it was impossible to blind participants, so all studies were at high risk of bias. Other sources of bias are described later (see also Figure 2).

Figure 2. Risk of bias graph: review authors' judgements about each risk of bias item presented as percentages across all included studies.



Allocation

Sequence generation

All included studies were described as randomised controlled trials: 57 specified the method of sequence generation and were at low risk of bias, but the method of randomisation was not described in 34. Ten studies had high risk of bias because randomisation appeared to be inadequate or undermined.

Allocation concealment

Allocation concealment was adequate in most studies reporting the method of randomisation, and 54 studies were at low risk of bias; risk of bias was unclear in 31 and high in 16.

Blinding

Providers

In nine studies, participants had no contact with researchers or clinicians, or providers were blind. Providers were not blind in 89 studies, which were at high risk of bias, and provider blinding was unclear in three studies.

Assessors

Most studies either did not report assessor-rated outcomes or used blind assessors, thus 85 were at low risk of bias. Blinding of assessors was at high risk of bias in 13 studies and unclear in three. Of the studies that used assessor ratings (46), most (30) reported that assessors were blind.

Incomplete outcome data

For response and recovery (dichotomous) data, we assumed that dropouts in both groups did not improve.

For continuous outcomes, 60 studies reported no dropout or analysed outcomes in a manner that mitigated the effect of missing cases and measures. Methods of analysis were at high risk of bias in 38 studies. It was unclear how three studies handled missing data.

Selective reporting

Most of the studies in the review included multiple outcome measures. Only 20 studies appeared to be free of selective outcome reporting, and 40 clearly omitted measured outcomes and were at high risk of bias. Risk of selective outcome reporting was unclear in 41 studies that reported all outcomes mentioned in the text but did not reference a protocol.

Other potential sources of bias

In addition to the sources of bias already discussed, 11 studies were at risk of bias for other reasons, which are described in the [Characteristics of included studies](#).

Effects of interventions

See: [Summary of findings for the main comparison](#); [Summary of findings 2](#)

Comparison 1: Media-delivered interventions versus no intervention

Primary Outcomes

1.1 Symptoms of anxiety

1.1.a Continuous symptom measures

Any measure of anxiety (mean effect of all measures of anxiety in each study)

At post-treatment, 72 studies, including 4537 participants, reported any self-rated measure of anxiety ([Figure 3](#)). A medium effect was noted (SMD 0.67, 95% CI 0.55 to 0.78), although heterogeneity was substantial ($\text{Chi}^2 = 227.97$, degrees of freedom (df) = 75, $P < 0.00001$, $I^2 = 67\%$). Only 13 studies, including 684 participants, reported controlled follow-up within six months; a slightly smaller overall effect was reported (SMD 0.49, 95% CI 0.24 to 0.75) with substantial heterogeneity ($\text{Chi}^2 = 32.96$, df = 14, $P = 0.003$, $I^2 = 58\%$). Long-term follow-up (> 6 months) was reported in five studies, including 287 participants; the overall effect was not significant (SMD 0.18, 95% CI -0.15 to 0.52), though heterogeneity approached significance ($\text{Chi}^2 = 6.60$, df = 4, $P = 0.16$, $I^2 = 39\%$).

Figure 3. 1 Self-help compared with no treatment. 1.3 Anxiety (any measure), self-rated at post-treatment (by disorder).

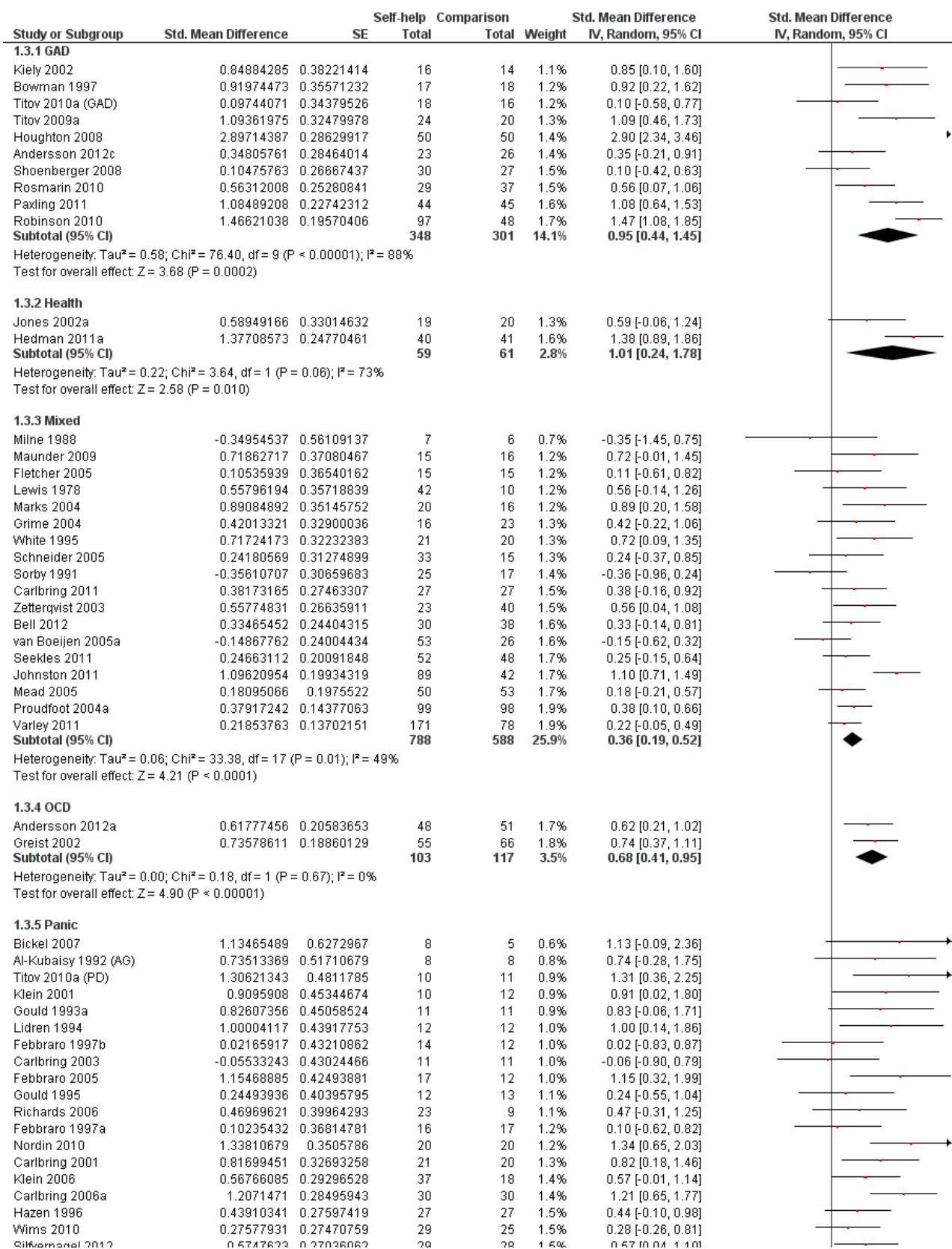


Figure 3. (Continued)

Hazen 1996	0.43910341	0.27597419	27	27	1.5%	0.44 [-0.10, 0.98]
Wims 2010	0.27577931	0.27470759	29	25	1.5%	0.28 [-0.26, 0.81]
Silfvermager 2012	0.5747623	0.27036062	29	28	1.5%	0.57 [0.04, 1.10]
Ruwaard 2010	0.51358888	0.26523588	27	31	1.5%	0.51 [-0.01, 1.03]
Kenardy 2003	0.3336855	0.21070172	49	44	1.7%	0.33 [-0.08, 0.75]
Subtotal (95% CI)			421	376	24.4%	0.62 [0.45, 0.79]

Heterogeneity: $\tau^2 = 0.03$; $\chi^2 = 25.89$, $df = 20$ ($P = 0.17$); $I^2 = 23\%$
Test for overall effect: $Z = 7.09$ ($P < 0.00001$)

1.3.6 Social

Al-Kubaisy 1992 (SAD)	0.20281117	0.53149451	7	8	0.8%	0.20 [-0.84, 1.24]
Chung 2008	0.57329182	0.4668355	24	14	0.9%	0.57 [-0.34, 1.49]
Abramowitz 2009	0.66109477	0.45457393	11	10	0.9%	0.66 [-0.23, 1.55]
Titov 2010a (SAD)	0.45271679	0.42272676	12	11	1.0%	0.45 [-0.38, 1.28]
Bowler 2012	0.83389387	0.31634046	21	21	1.3%	0.83 [0.21, 1.45]
Berger 2009	0.76032998	0.29265662	31	21	1.4%	0.76 [0.19, 1.33]
Carlbring 2007	1.073247	0.28404078	29	28	1.4%	1.07 [0.52, 1.63]
Botella 2010	1.02777241	0.24539616	62	29	1.6%	1.03 [0.55, 1.51]
Titov 2008b	1.19386936	0.24134292	41	40	1.6%	1.19 [0.72, 1.67]
Titov 2008c	0.71471816	0.2182533	61	35	1.7%	0.71 [0.29, 1.14]
Furmark 2009b	0.33294456	0.21596143	86	29	1.7%	0.33 [-0.09, 0.76]
Titov 2008a	0.95156029	0.21218673	50	49	1.7%	0.95 [0.54, 1.37]
Furmark 2009a	0.66359874	0.19844528	80	40	1.7%	0.66 [0.27, 1.05]
Rapee 2007	0.38354427	0.19449526	56	52	1.7%	0.38 [0.00, 0.76]
Andersson 2012b	0.73742538	0.14798603	94	100	1.9%	0.74 [0.45, 1.03]
Subtotal (95% CI)			665	487	21.3%	0.73 [0.59, 0.87]

Heterogeneity: $\tau^2 = 0.01$; $\chi^2 = 16.06$, $df = 14$ ($P = 0.31$); $I^2 = 13\%$
Test for overall effect: $Z = 10.49$ ($P < 0.00001$)

1.3.7 Specific

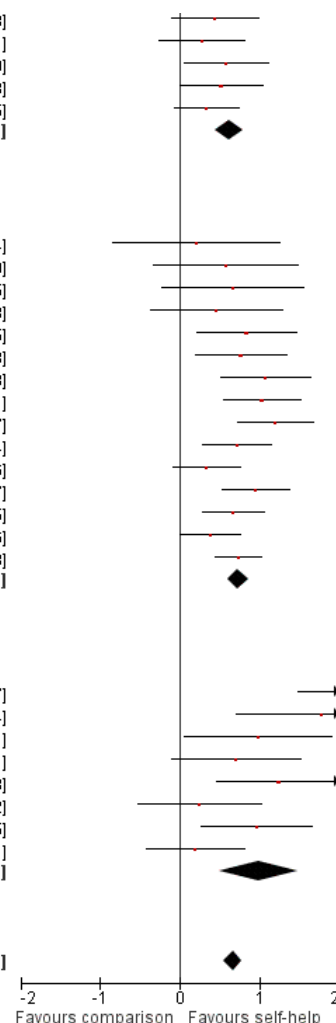
Hassan 1992	2.76960971	0.66187314	10	8	0.6%	2.77 [1.47, 4.07]
Al-Kubaisy 1992 (SP)	1.76927035	0.54470589	10	9	0.7%	1.77 [0.70, 2.84]
Rosen 1976	0.98329932	0.47098824	17	7	0.9%	0.98 [0.06, 1.91]
Granado 2007	0.70115148	0.41241809	13	12	1.0%	0.70 [-0.11, 1.51]
Gilroy 2000	1.23922982	0.40281171	15	15	1.1%	1.24 [0.45, 2.03]
Heading 2001	0.24847332	0.39609255	13	13	1.1%	0.25 [-0.53, 1.02]
Muller 2011	0.96288551	0.35211954	18	18	1.2%	0.96 [0.27, 1.65]
Smith 1997	0.19053525	0.3172841	30	15	1.3%	0.19 [-0.43, 0.81]
Subtotal (95% CI)			126	97	7.9%	0.99 [0.50, 1.48]

Heterogeneity: $\tau^2 = 0.30$; $\chi^2 = 19.13$, $df = 7$ ($P = 0.008$); $I^2 = 63\%$
Test for overall effect: $Z = 3.97$ ($P < 0.0001$)

Total (95% CI)

			2510	2027	100.0%	0.67 [0.55, 0.78]
--	--	--	-------------	-------------	---------------	--------------------------

Heterogeneity: $\tau^2 = 0.16$; $\chi^2 = 227.97$, $df = 75$ ($P < 0.00001$); $I^2 = 67\%$
Test for overall effect: $Z = 11.42$ ($P < 0.00001$)
Test for subgroup differences: $\chi^2 = 17.04$, $df = 6$ ($P = 0.009$), $I^2 = 64.8\%$



Specific and general symptoms of anxiety

Effects described previously (any measure of anxiety) include the average of all measures of anxiety for each study, which are listed in the [Characteristics of included studies](#). Most studies in the main analysis measured symptoms of a particular disorder (e.g. Leibowitz Social Anxiety Scale), but to ensure that the results were not influenced by the inclusion of broader measures (e.g. Beck Anxiety Inventory), we also analysed specific and general measures separately.

At post-treatment, 55 studies including 3192 participants reported a self-rated measure of a specific anxiety disorder. A medium to large effect was noted (SMD 0.68, 95% CI 0.57 to 0.80), though heterogeneity was moderate ($\chi^2 = 128.49$, $df = 58$, $P < 0.00001$, $I^2 = 55\%$). Only 10 studies including 439 participants reported controlled follow-up within six months, and a slightly smaller overall effect was found (SMD 0.61, 95% CI 0.27 to 0.94) with substantial heterogeneity ($\chi^2 = 31.13$, $df = 11$, $P = 0.0004$, $I^2 = 65\%$). Long-term follow-up was reported in two studies including 194 participants; the overall effect was not significant (SMD 0.20, 95% CI -0.22 to 0.62), although heterogeneity approached

significance ($\chi^2 = 5.90$, $df = 4$, $P = 0.21$, $I^2 = 32\%$). The pattern of results in subgroup analyses was unchanged.

Additionally, 13 studies including 629 participants reported a clinician-rated measure of a specific anxiety disorder. There was a large effect (SMD 1.04, 95% CI 0.50 to 1.58), though heterogeneity was considerable ($\chi^2 = 124.76$, $df = 14$, $P < 0.00001$, $I^2 = 89\%$). Only five studies including 98 participants reported controlled follow-up within six months, and a larger overall effect was described (SMD 1.12, 95% CI 0.70 to 1.53). No studies reported long-term follow-up. At post-treatment, studies reporting a clinician-rated measure were consistent with all self-rated measures (SMD 0.70, 95% CI 0.46 to 0.95); larger effects on clinician-rated measures may be explained by bias.

At post-treatment, 48 studies including 3101 participants reported a self-rated measure of general symptoms. There was a medium effect (SMD 0.58, 95% CI 0.43 to 0.73), though heterogeneity was considerable ($\chi^2 = 176.38$, $df = 47$, $P < 0.00001$, $I^2 = 73\%$). Only nine studies including 569 participants reported controlled follow-up within six months, and a small effect was described (SMD 0.22, 95% CI 0.06 to 0.39) with no important heterogeneity ($\chi^2 = 7.46$, $df = 8$, $P = 0.49$, $I^2 = 0\%$). Long-term follow-up was reported in

two studies including 194 participants; the overall effect was not significant (SMD -0.07, 95% CI -0.63 to 0.49), but the studies were inconsistent ($\text{Chi}^2 = 3.19$, $\text{df} = 1$, $P = 0.07$, $I^2 = 69\%$).

1.1.b Response

At post-treatment, 21 studies including 1547 participants reported response to treatment according to study criteria (Figure 4). Overall, intervention participants were twice as likely to respond (RR 2.34, 95% CI 1.81 to 3.03), although heterogeneity was moderate ($\text{Chi}^2 = 40.30$, $\text{df} = 20$, $P = 0.005$, $I^2 = 50\%$). Only two studies including

128 participants reported controlled follow-up within six months, and the effect was maintained (RR 2.12, 95% CI 1.18 to 3.79) with no heterogeneity ($\text{Chi}^2 = 0.35$, $\text{df} = 1$, $P = 0.55$, $I^2 = 0\%$). No studies reported long-term follow-up. Several studies reported exceptional effect sizes, but only 18% of all participants in the comparison groups were classified as responders at the end of treatment; in absolute terms, media-delivered interventions were associated with a 31% (95% CI 0.22 to 0.40) increase in response (number needed to treat for an additional beneficial outcome (NNTB = 3)).

Figure 4. 1 Self-help compared with no treatment. 1.26 Response at post-treatment (by disorder).

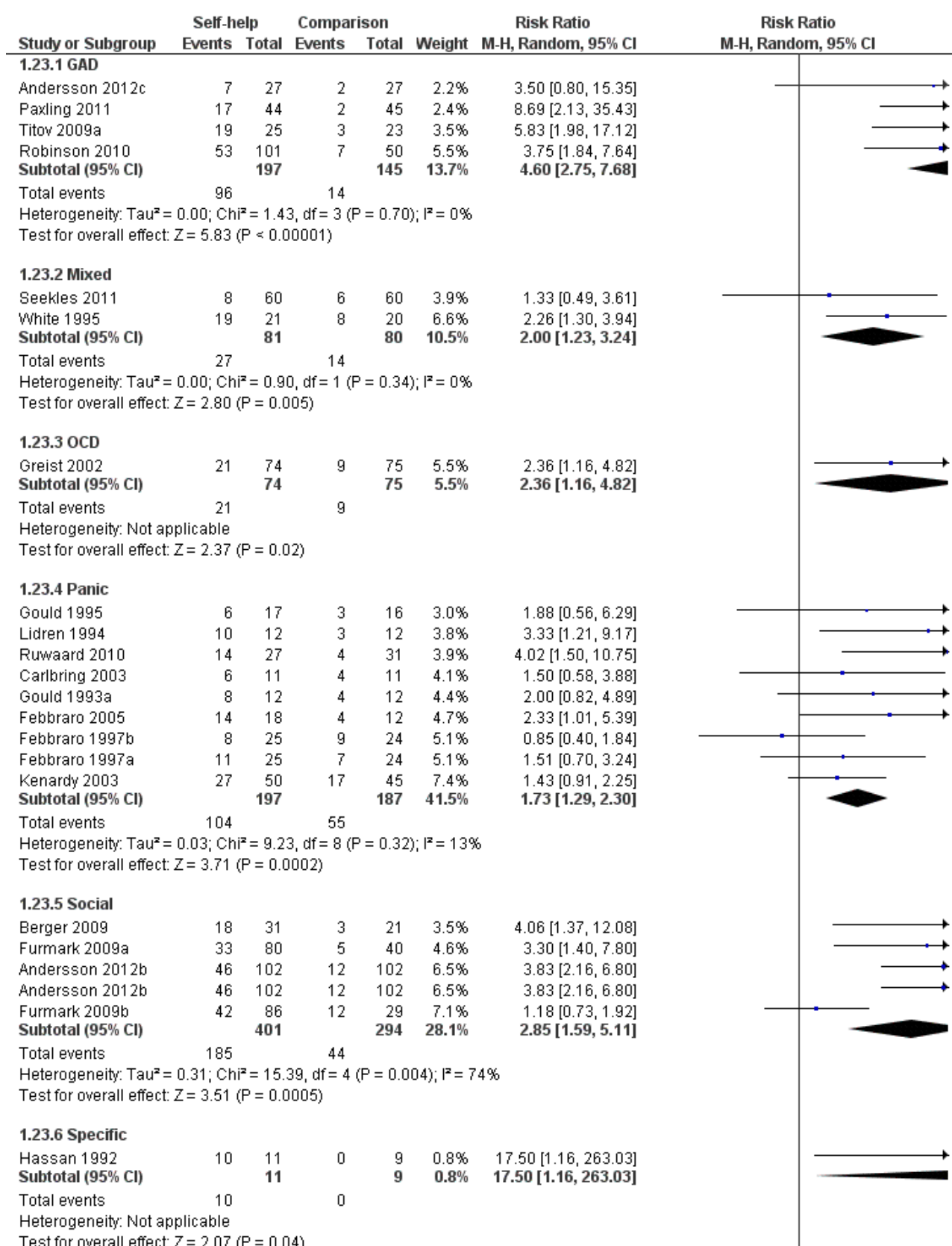
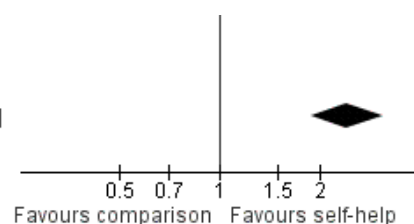


Figure 4. (Continued)

Heterogeneity: Not applicable
Test for overall effect: $Z = 2.07$ ($P = 0.04$)

Total (95% CI)	961	790	100.0%	2.42 [1.88, 3.12]
Total events	443	136		
Heterogeneity: $\tau^2 = 0.17$; $\chi^2 = 44.04$, $df = 21$ ($P = 0.002$); $I^2 = 52\%$				
Test for overall effect: $Z = 6.85$ ($P < 0.00001$)				
Test for subgroup differences: $\chi^2 = 13.75$, $df = 5$ ($P = 0.02$), $I^2 = 63.6\%$				



1.1.c Recovery

Only nine studies with 605 participants reported the number of participants no longer meeting criteria for diagnosis at the end of treatment. Overall, media-delivered interventions were associated with a fivefold increase in recovery (RR 5.36, 95% CI 1.62 to 17.68), but heterogeneity was considerable ($\chi^2 = 74.23$, $df = 8$, $P < 0.00001$, $I^2 = 89\%$). Only one study reported more than 20% recovery in the comparison group, which could be explained by participants seeking treatment outside the study (Baillie 2002); in the other studies, only 6% of participants in the comparison groups recovered, and media-delivered interventions were associated with a 45% (95% CI 0.26 to 0.65) increase in recovery (NNTB = 2).

Secondary outcomes

1.2 Behavioural test (level achieved)

Some studies of animal or situational phobias also reported behavioural measures. At post-treatment, seven studies including 250 participants reported the level completed in a behavioural assessment; a large overall effect was described (SMD 1.11, 95% CI 0.36 to 1.87), but heterogeneity was considerable ($\chi^2 = 37.20$, $df = 6$, $P < 0.00001$, $I^2 = 84\%$). When one study (Hassan 1992) that reported an implausible effect was excluded (SMD 8.17), a medium to large effect was still described (SMD 0.76, 95% CI 0.32 to 1.21). At follow-up, one study including 30 participants reported a large difference (SMD 1.23, 95% CI 0.47 to 1.99). No studies reported long-term follow-up.

1.3 Behavioural test (symptoms)

At post-treatment, five studies including 191 participants reported symptom ratings, and the overall effect was not significant (SMD 0.59, 95% CI -0.33 to 1.51), but heterogeneity was considerable ($\chi^2 = 30.48$, $df = 4$, $P < 0.00001$, $I^2 = 87\%$). At follow-up, one study including 30 participants reported no significant difference (SMD 0.60, 95% CI -0.11 to 1.31). No studies reported long-term follow-up.

1.4 Depression

At post-treatment, 50 studies including 3682 participants reported a self-rated measure of depression. A medium effect was described (SMD 0.47, 95% CI 0.36 to 0.58), although heterogeneity was substantial ($\chi^2 = 120.40$, $df = 50$, $P < 0.00001$, $I^2 = 58\%$). Only seven studies including 526 participants reported controlled follow-up within six months, and the combined effect was small (SMD 0.28, 95% CI 0.05 to 0.52), but heterogeneity was substantial ($\chi^2 = 9.89$, $df = 6$, $P = 0.13$, $I^2 = 39\%$). When the largest study in the review, which targeted both anxiety and depression (Proudfoot 2004a), was excluded, the effect at follow-up was not significant (SMD 0.20, 95% CI -0.04 to 0.43), and the results were not significantly heterogeneous ($\chi^2 = 6.00$, $df = 5$, $P = 0.31$, $I^2 = 17\%$). Long-term follow-up was reported in two studies including 194 participants;

the overall effect was not significant (SMD -0.25, 95% CI -0.58 to 0.08), and heterogeneity was not significant ($\chi^2 = 1.11$, $df = 1$, $P = 0.29$, $I^2 = 10\%$).

1.5 Mental-health related disability

At post-treatment, 20 studies including 1629 participants reported a self-rated measure of disability. A medium effect was found (SMD 0.48, 95% CI 0.33 to 0.63), although heterogeneity was substantial ($\chi^2 = 37.21$, $df = 19$, $P = 0.007$, $I^2 = 49\%$). Only seven studies including 513 participants reported controlled follow-up within six months, and no significant effect was found (SMD 0.21, 95% CI -0.15 to 0.57), but heterogeneity was substantial ($\chi^2 = 22.09$, $df = 6$, $P = 0.001$, $I^2 = 73\%$). Long-term follow-up was reported in three studies including 147 participants; the overall effect was not significant (SMD -0.14, 95% CI -0.33 to 0.60), and heterogeneity was not significant ($\chi^2 = 3.50$, $df = 2$, $P = 0.17$, $I^2 = 43\%$).

1.6 Quality of life

At post-treatment, 18 studies including 1344 participants reported a self-rated measure of quality of life. A medium effect was found (SMD 0.36, 95% CI 0.22 to 0.49), although heterogeneity was moderate ($\chi^2 = 24.58$, $df = 17$, $P = 0.10$, $I^2 = 31\%$). Only three studies including 211 participants reported controlled follow-up within six months, and the combined effect was not significant (SMD 0.08, 95% CI -0.19 to 0.35) with no heterogeneity ($\chi^2 = 0.95$, $df = 2$, $P = 0.62$, $I^2 = 0\%$). One study reported no effect at long-term follow-up (SMD -0.06, 95% CI -0.48 to 0.36).

1.7 Dropout

Of those receiving media-delivered interventions and no-intervention, 855 (20%) and 463 (16%) did not complete the study. A small difference favoured no treatment (RR 0.96, 95% CI 0.94 to 0.99), and heterogeneity was moderate ($\chi^2 = 129.33$, $df = 77$, $P = 0.0002$, $I^2 = 40\%$).

Comparison 2: Media-delivered interventions versus face-to-face interventions

Primary Outcomes

2.1 Symptoms of anxiety

2.1.a Continuous symptom measures

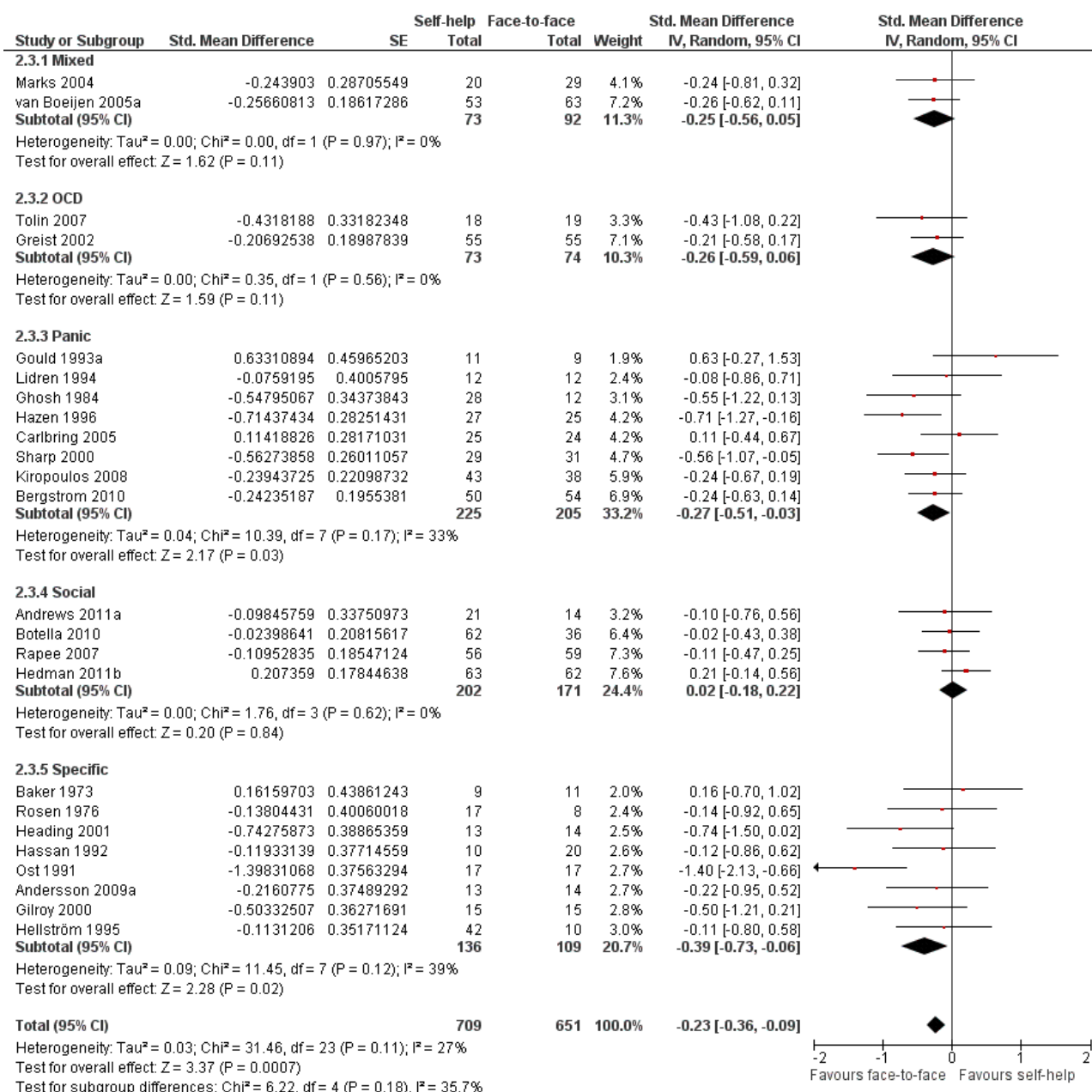
Any measure of anxiety (mean effect of all measures of anxiety in each study)

At post-treatment, 24 studies, including 1360 participants, reported any self-rated measure of anxiety (Figure 5). A small effect favoured face-to-face intervention (SMD -0.23, 95% CI -0.36 to -0.09), and moderate heterogeneity approached significance ($\chi^2 = 31.46$, $df = 23$, $P = 0.11$, $I^2 = 27\%$). Only 11 studies including 677 participants reported controlled follow-up within six months, and a slightly

smaller effect favoured face-to-face intervention (SMD -0.14, 95% CI -0.30 to -0.01) with no heterogeneity ($\text{Chi}^2 = 6.50$, $\text{df} = 10$, $P = 0.77$, $I^2 = 0\%$). Long-term follow-up (> 6 months) was reported in eight studies including 423 participants; the overall difference was not significant (SMD -0.16, 95% CI -0.50 to 0.18), although heterogeneity was substantial ($\text{Chi}^2 = 18.09$, $\text{df} = 7$, $P = 0.01$, $I^2 = 61\%$) and was

attributable to one study that reported a large effect favouring face-to-face intervention (Ost 1991); in that study, values for participants in the media-delivered intervention group were consistent with those of other studies, and the face-to-face intervention appeared to be very effective.

Figure 5. 2 Self-help compared with face-to-face therapy. 2.3 Anxiety (any measure), self-rated at post-treatment (by disorder).



Specific and general symptoms of anxiety

At post-treatment, 23 studies including 1255 participants reported a self-rated measure of a specific anxiety disorder. A small effect favoured face-to-face intervention (SMD -0.23, 95% CI -0.37 to -0.10), and moderate heterogeneity approached significance ($\text{Chi}^2 = 30.57$, $\text{df} = 22$, $P = 0.11$, $I^2 = 28\%$). Only 10 studies including 573

participants reported controlled follow-up within six months, and the overall difference was not significant (SMD -0.15, 95% CI -0.31 to -0.02) with no heterogeneity ($\text{Chi}^2 = 8.61$, $\text{df} = 9$, $P = 0.47$, $I^2 = 0\%$). Long-term follow-up (> 6 months) was reported in eight studies including 423 participants; the overall difference was not significant (SMD -0.20, 95% CI -0.54 to 0.14), although heterogeneity was substantial ($\text{Chi}^2 = 18.08$, $\text{df} = 7$, $P = 0.01$, $I^2 = 61\%$) and was

attributable to one study that reported a large effect favouring face-to-face intervention ([Ost 1991](#)). The pattern of results in subgroup analyses was unchanged.

Additionally, six studies including 418 participants reported a clinician-rated measure of a specific anxiety disorder. The difference was not significant (SMD -0.13, 95% CI -0.33 to 0.06), and no heterogeneity was noted ($\text{Chi}^2 = 3.97$, $\text{df} = 5$, $P = 0.55$, $I^2 = 0\%$). Only three studies including 185 participants reported controlled follow-up within six months, and the effect was similar to the effect at post-treatment (SMD -0.19, 95% CI -0.69 to 0.30). Only one study including 98 participants reported long-term follow-up, and the effect was similar to the effect at post-treatment (SMD -0.13, 95% CI -0.53 to 0.28).

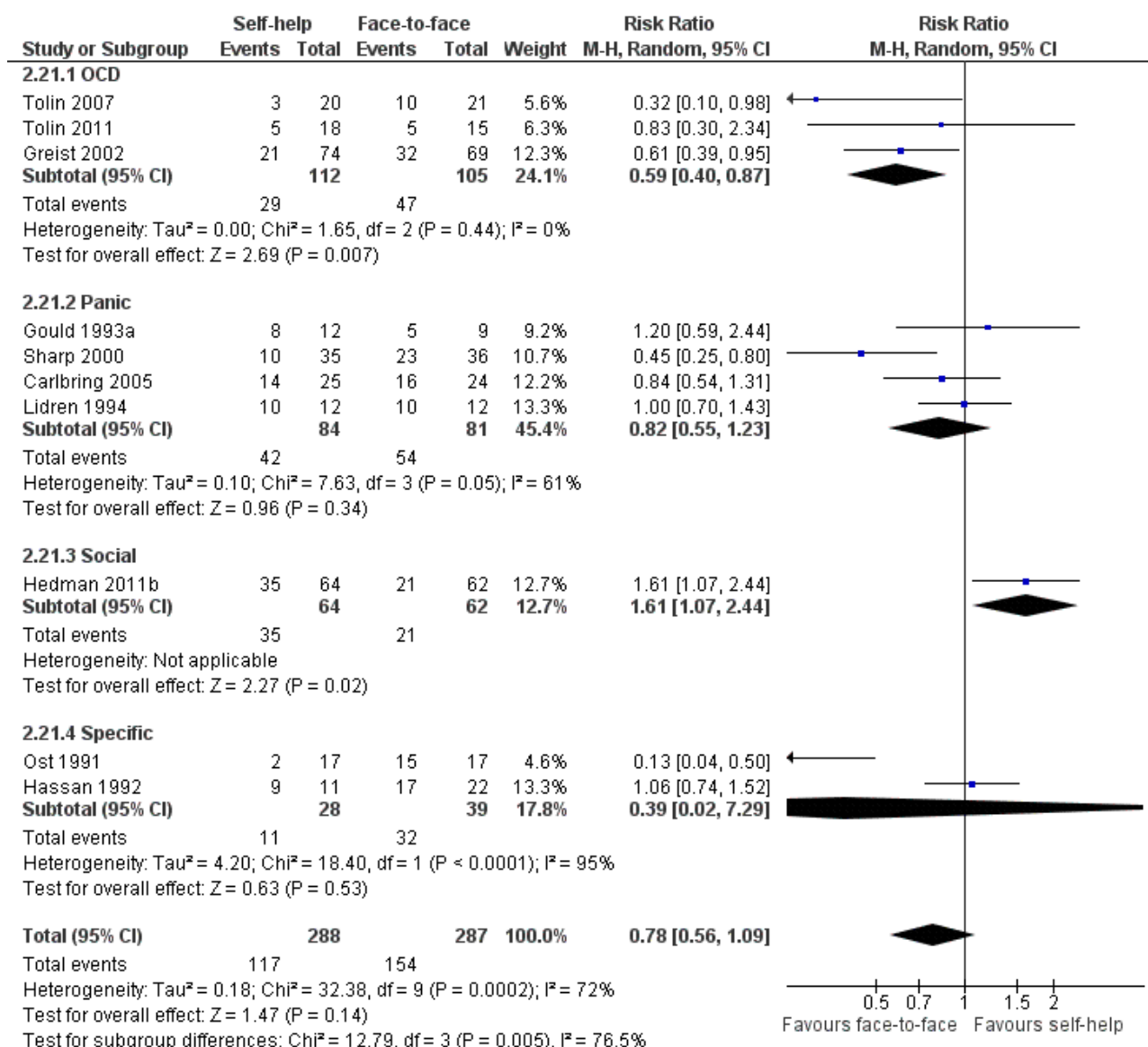
At post-treatment, 11 studies including 627 participants reported a self-rated measure of general symptoms. No significant difference between groups was noted (SMD -0.08, 95% CI -0.36 to 0.20), although heterogeneity was substantial ($\text{Chi}^2 = 30.19$, $\text{df} = 10$, $P = 0.0008$, $I^2 = 67\%$). Only five studies including 308 participants reported controlled follow-up within 6 months, and no significant

difference (SMD 0.10, 95% CI -0.30 to 0.09) and no important heterogeneity were described ($\text{Chi}^2 = 1.24$, $\text{df} = 4$, $P = 0.87$, $I^2 = 0\%$). Long-term follow-up was reported in four studies including 241 participants; no significant difference was noted (SMD 0.04, 95% CI -0.22 to 0.30), and no heterogeneity was reported ($\text{Chi}^2 = 1.34$, $\text{df} = 3$, $P = 0.72$, $I^2 = 0\%$).

2.1.b Response

At post-treatment, 10 studies including 575 participants reported response to treatment ([Figure 6](#)). Overall effects were not significant (RR 0.78, 95% CI 0.56 to 1.09), but heterogeneity was substantial ($\text{Chi}^2 = 32.38$, $\text{df} = 9$, $P = 0.0002$, $I^2 = 72\%$). The absolute difference also was not significant (RD -0.15, 95% CI -0.35 to 0.04), but the absolute rate of response was only 47% overall. Only four studies including 224 participants reported controlled follow-up within six months, and although no significant difference was found (RR 0.99, 95% CI 0.73 to 1.35), heterogeneity was substantial ($\text{Chi}^2 = 7.97$, $\text{df} = 3$, $P = 0.05$, $I^2 = 62\%$). Three studies including 116 participants reported long-term follow-up and no significant difference (RR 0.58, 95% CI 0.20 to 1.64), but heterogeneity was considerable ($\text{Chi}^2 = 8.68$, $\text{df} = 2$, $P = 0.01$, $I^2 = 77\%$).

Figure 6. Forest plot of comparison: 2 Media-delivered intervention compared with face-to-face therapy, outcome. 2.21 Response (specific), self-rated at post-treatment (by disorder).



2.1.c Recovery

Only six studies including 587 participants reported the number of participants no longer meeting criteria for diagnosis at the end of treatment. No significant difference in recovery was found (RR 1.05, 95% CI 0.88 to 1.24), and no heterogeneity was noted ($\chi^2 = 2.65$, $df = 5$, $P = 0.75$, $I^2 = 0\%$). Only three studies including 354 participants reported controlled follow-up within six months; no significant difference was found (RR 1.04, 95% CI 0.70 to 1.56), but heterogeneity was substantial ($\chi^2 = 4.02$, $df = 2$, $P = 0.13$, $I^2 = 50\%$). Only two studies including 147 participants reported long-term follow-up; no significant difference was found (RR 0.90, 95% CI 0.64 to 1.25), but the studies were inconsistent ($\chi^2 = 3.60$, $df = 1$, $P = 0.06$, $I^2 = 72\%$).

Secondary outcomes

2.2 Behavioural test (level achieved)

Six studies included behavioural tests. At post-treatment, five studies including 211 participants reported the level completed; a small to medium effect was found (SMD -0.44, 95% CI -0.76 to -0.13), and no important heterogeneity was noted ($\chi^2 = 4.75$, $df = 4$, $P = 0.31$, $I^2 = 16\%$). Two studies including 60 participants reported the level completed at follow-up; no significant difference was noted (SMD -0.02, 95% CI -0.74 to 0.71), but the studies were inconsistent ($\chi^2 = 2.02$, $df = 1$, $P = 0.16$, $I^2 = 50\%$). Two studies including 122 participants reported the level completed at long-term follow-up; no significant difference was found (SMD -0.1, 95% CI -0.97 to 0.76), but the studies were not consistent ($\chi^2 = 3.83$, $df = 1$, $P = 0.05$, $I^2 = 74\%$).

2.3 Behavioural test (symptoms)

At post-treatment, five studies including 209 participants reported self-rated symptoms, the overall difference was not significant (SMD -0.57, 95% CI -1.35 to 0.21) and considerable heterogeneity was noted ($\text{Chi}^2 = 26.16$, $\text{df} = 4$, $P < 0.0001$, $I^2 = 85\%$). One study including 30 participants reported symptoms at follow-up, and no significant difference was noted (SMD -0.09, 95% CI -0.79 to 0.60). Three studies including 162 participants reported symptoms at long-term follow-up, and no significant difference was found (SMD -0.46, 95% CI -1.19 to 0.27), but heterogeneity was considerable ($\text{Chi}^2 = 8.93$, $\text{df} = 2$, $P = 0.01$, $I^2 = 78\%$).

2.4 Depression

At post-treatment, 13 studies including 906 participants reported a self-rated measure of depression. No overall difference was found (SMD -0.01, 95% CI -0.22 to 0.21), although heterogeneity was substantial ($\text{Chi}^2 = 28.27$, $\text{df} = 12$, $P = 0.005$, $I^2 = 58\%$). Only five studies including 403 participants reported controlled follow-up within six months, and the combined effect was not significant (SMD 0.09, 95% CI -0.19 to 0.36), but heterogeneity was moderate ($\text{Chi}^2 = 7.11$, $\text{df} = 4$, $P = 0.13$, $I^2 = 44\%$). Long-term follow-up was reported in five studies including 339 participants; the overall effect was not significant (SMD 0.08, 95% CI -0.14 to 0.30), and no important heterogeneity was noted ($\text{Chi}^2 = 4.08$, $\text{df} = 4$, $P = 0.40$, $I^2 = 2\%$).

2.5 Mental-health related disability

At post-treatment, 10 studies including 670 participants reported a self-rated measure of disability. No significant difference was found (SMD -0.07, 95% CI -0.33 to 0.20), although heterogeneity was substantial ($\text{Chi}^2 = 25.58$, $\text{df} = 9$, $P = 0.002$, $I^2 = 65\%$). Only five studies including 337 participants reported controlled follow-up within six months, and the combined effect was not significant (SMD -0.10, 95% CI -0.39 to 0.19), but heterogeneity was moderate ($\text{Chi}^2 = 6.80$, $\text{df} = 4$, $P = 0.15$, $I^2 = 41\%$). Long-term follow-up was reported in three studies including 242 participants; the overall effect was not significant (SMD -0.21, 95% CI -0.47 to 0.04), and no heterogeneity was noted ($\text{Chi}^2 = 0.11$, $\text{df} = 2$, $P = 0.95$, $I^2 = 0\%$).

2.6 Quality of life

At post-treatment, four studies including 282 participants reported a self-rated measure of disability. No significant difference was found (SMD 0.08, 95% CI -0.23 to 0.38), although heterogeneity was moderate ($\text{Chi}^2 = 4.82$, $\text{df} = 3$, $P = 0.19$, $I^2 = 38\%$). Only two studies including 143 participants reported controlled follow-up within six months, and the combined effect favoured media-delivered intervention (SMD 0.40, 95% CI -0.07 to 0.73) with no heterogeneity noted ($\text{Chi}^2 = 0.64$, $\text{df} = 1$, $P = 0.43$, $I^2 = 0\%$). One study reported no difference at long-term follow-up (SMD -0.15, 95% CI -0.41 to 0.70).

2.7 Dropout

Of those receiving media-delivered interventions and face-to-face interventions, 855 (20%) and 166 (18%) did not complete the studies. This difference was not significant (RR 1.08, 95% CI 0.90 to 1.30), and no heterogeneity was noted ($\text{Chi}^2 = 25.77$, $\text{df} = 26$, $P = 0.48$, $I^2 = 0\%$).

Comparison 3: Media-delivered interventions versus psychoeducation

No outcomes could be analysed.

Comparison 4: Media-delivered interventions versus medication

No outcomes could be analysed.

Subgroup analyses

Comparison 1: Media-delivered interventions versus no intervention

We examined the impact of methodological differences.

Types of controls

The specific control condition appeared related to relative effects ($\text{Chi}^2 = 21.84$, $\text{df} = 3$, $P = 0.0001$, $I^2 = 86\%$). Specifically, interventions had similar effects compared with wait-list and attention controls ($\text{Chi}^2 = 3.99$, $\text{df} = 2$, $P = 0.14$, $I^2 = 50\%$), but the overall effect was small when participants were recruited from clinical settings and interventions were compared with treatment as usual (SMD 0.21, 95% CI 0.02 to 0.40). Without studies of treatment as usual, the overall effect was not importantly different from the primary result (SMD 0.72, 95% CI 0.61 to 0.84), and heterogeneity remained substantial ($\text{Chi}^2 = 183.67$, $\text{df} = 68$, $P < 0.00001$, $I^2 = 63\%$).

Types of disorders

Effects were significantly different across types of disorders ($\text{Chi}^2 = 17.04$, $\text{df} = 6$, $P = 0.009$, $I^2 = 65\%$). Studies of mixed disorders reported the smallest combined effect (SMD 0.36, 95% CI 0.19 to 0.52). After these were excluded, subgroups were no longer heterogeneous ($\text{Chi}^2 = 3.96$, $\text{df} = 5$, $P = 0.56$, $I^2 = 0\%$); the overall effect remained medium to large (SMD 0.77, 95% CI 0.64 to 0.91), and heterogeneity remained substantial ($\text{Chi}^2 = 157.63$, $\text{df} = 57$, $P < 0.00001$, $I^2 = 64\%$).

Types of interventions

Effects were not significantly different across types of interventions ($\text{Chi}^2 = 1.88$, $\text{df} = 3$, $P = 0.60$, $I^2 = 0\%$), although the effect for relaxation was not significant. A medium effect for media-delivered CBT was found (SMD 0.62, 95% CI 0.51 to 0.73), along with substantial heterogeneity ($\text{Chi}^2 = 127.24$, $\text{df} = 55$, $P < 0.00001$, $I^2 = 57\%$).

Types of media

Effects were significantly different across types of media ($\text{Chi}^2 = 7.70$, $\text{df} = 3$, $P = 0.05$, $I^2 = 61\%$) and were largest for Internet-delivered interventions (SMD 0.79, 95% CI 0.62 to 0.96), although heterogeneity within this group was substantial ($\text{Chi}^2 = 132.37$, $\text{df} = 35$, $P < 0.00001$, $I^2 = 74\%$).

Types of recruitment

Studies that advertised through mass media (including the Internet) reported larger effects than studies recruiting only through clinical referrals ($\text{Chi}^2 = 14.98$, $\text{df} = 2$, $P = 0.0006$, $I^2 = 87\%$), although heterogeneity remained substantial among studies that used advertising ($\text{Chi}^2 = 168.47$, $\text{df} = 54$, $P < 0.00001$, $I^2 = 68\%$).

Comparison 2: Media-delivered interventions versus face-to-face interventions

Types of disorders

Effects were not significantly different across types of disorders ($\text{Chi}^2 = 6.22$, $\text{df} = 4$, $P = 0.18$, $I^2 = 35.7\%$), although studies of specific phobias reported the largest difference between media-delivered and face-to-face interventions (SMD -0.39, 95% CI -0.73 to -0.06),

and studies of social anxiety reported the smallest (SMD 0.02, 95% CI -0.18 to 0.22).

Types of interventions

Disorders varied with types of therapy, and the difference between media-delivered exposure and face-to-face exposure might be larger than the difference between different deliveries of CBT ($\chi^2 = 2.36$, $df = 1$, $P = 0.12$, $I^2 = 57.6\%$), but no between-group heterogeneity was noted after one study was removed from the exposure group (Ost 1991). Effects did not vary between individual and group CBT.

Types of media

Effects were not significantly different across types of media ($\chi^2 = 5.07$, $df = 3$, $P = 0.17$, $I^2 = 40.9\%$).

Types of recruitment

Consistent with the first comparison, studies recruiting through advertisements found smaller differences than studies recruiting only through clinical referrals ($\chi^2 = 2.94$, $df = 1$, $P = 0.09$, $I^2 = 66.0\%$), that is, recruiting participants through advertising was associated with more favourable outcomes for media-delivered therapy.

DISCUSSION

Summary of main results

This review suggests that media-delivered interventions may be superior to no intervention for people with anxiety; there were positive effects on symptoms of anxiety and depression, response and recovery from illness, disability, and quality of life. Face-to-face interventions may be superior to media-delivered interventions; there were differences favouring face-to-face treatment for symptoms of anxiety, but no significant differences in response and recovery from illness, disability and quality of life. Statistical heterogeneity suggests that effects probably vary across participants and interventions. Effects may be maintained after cessation of treatment, but few studies included follow-up after six months.

Overall, interventions for specific disorders may be superior to interventions for any anxiety disorder. However, one recent transdiagnostic study reported effects that were consistent with the effects of interventions for specific disorders (Titov 2010a). The largest effects were reported in studies of Internet-based CBT, which may be a superior medium for delivering interventions; however, many of these studies offered only documents for download and printing (PDFs) or access to non-interactive Websites. More recent trials follow current models for treatment, so changes in intervention components and content may correlate with changes in disorders studied and technologies used, thus confounding these effects.

Overall completeness and applicability of evidence

In general, studies were conducted in high-income, English-speaking countries among white, female, middle-aged participants; these results may not generalise to other settings or participants. Trials had high exclusion rates, for example, one study excluded 100 of 333 applications for high scores on a measure of depression, or for risk of suicide (Titov 2008a). Despite the large numbers of studies and participants in this review, the combined results of efficacy studies cannot demonstrate effectiveness in the

general population, and few controlled comparisons suggest that benefits may be maintained after cessation of treatment.

For people seeking treatment, the magnitude of these effects should be interpreted with caution. For example, standardised effects appear similar to effective pharmacotherapies (Kapczinski 2003; Schneier 2001), but there was virtually no response among wait-list and no-treatment controls. It is also true that wait-list participants who receive many face-to-face psychotherapies do not respond, but within-group changes in these studies were not comparable with evidence-based face-to-face interventions in which most participants respond or recover (e.g. Clark 2006).

Differences between media-delivered interventions and face-to-face interventions were small, but some studies (particularly the older ones) provided face-to-face interventions designed to control for specific effects of media-delivered interventions rather than face-to-face interventions as they would be provided in usual services. Established interventions may be more effective than these diminished controls. For example, one study demonstrated that a single session of therapist-guided exposure was greatly superior to media-delivered intervention for people with specific phobias (Ost 1991).

On the other hand, these interventions increase the number of people receiving help. Anxiety disorders are very common, most people with anxiety never receive treatment and these disorders are naturally chronic and unremitting. For people who would not receive another intervention, NNTBs of 2 and 3 may be valid estimates of treatment efficacy.

Among trials that included some contact with participants, the amount of therapist input varied greatly. Interventions in these studies may not be representative of the interventions that users can access outside of clinical trials or outside of particular services that are designed to offer guided self-help.

Quality of the evidence

This review includes large numbers of studies and participants. Most trials reported the primary outcome, self-rated symptoms of anxiety, and the evidence was of moderate quality compared with no treatment (Summary of findings for the main comparison) but of low quality compared with face-to-face intervention (Summary of findings 2).

Overall, methods of randomisation and allocation concealment appear unlikely to influence overall effects. However, participants and providers were not blind, and assessors were blind in only 64% of studies using clinician ratings. Compared with self-ratings, clinician ratings suggest a greater effect compared with no treatment and a smaller difference compared with face-to-face therapy. Studies reporting self-ratings and clinician ratings have reported effects on self-ratings that were consistent with the average effect for all studies. Additionally, the post-treatment effect for recovery (SMD 0.75, 95% CI 0.61 to 0.90) was slightly larger than the effect for symptoms of anxiety. Few studies referenced a protocol and reported all outcomes, thus the results of this review may be overestimated because of selective outcome reporting, which could explain the relatively larger effects on clinician-rated outcomes and recovery. Compared with no intervention and compared with face-to-face intervention, 10 of 82 studies and 5 of 29 studies did not report a measure of anxiety that could

be included in the main analysis. Incomplete outcome data may also lead to overestimation of continuous outcomes; however, for dichotomous outcomes, we assumed that dropouts did not respond, thus the response and recovery outcomes are unlikely to be overestimated due to missing data. Anxiety disorders tend to be chronic and naturally unremitting, so the assumption that dropouts did not improve may represent the true outcome for most missing cases. Subgroup analyses should be interpreted with caution, as some variables may correlate with risk of bias, for example, average effects of exposure alone may be overestimated compared with the effects of CBT.

Safety data were inadequately reported. Whilst a few studies mentioned adverse events in intervention or control groups, a systematic synthesis was not possible. It is unclear whether adverse events were not reported because they do not occur, or if they were not reported because they were not monitored; most trials excluded participants at greatest risk. Similarly, measures of participant satisfaction, the need for alternative interventions and costs were reported so selectively and incompletely that a formal synthesis would be inappropriate.

Statistical heterogeneity was substantial in most analyses (i.e. $I^2 > 50\%$). We attempted to explain this heterogeneity through prespecified analyses, but inconsistency remained within subgroups. Many of the individual studies were not significantly different from zero, but most reported positive effects compared with no intervention.

The quality of most outcomes was low according to GRADE criteria (GRADE 2004), although the quality of response and recovery data was moderate. Outcomes were downgraded for risk of selective outcome reporting. Continuous data were downgraded due to risk of bias, especially for incomplete outcome data.

Potential biases in the review process

This review included a comprehensive search for published and unpublished data. We obtained some unpublished data for 25 studies: Seven completely unpublished studies were included, we obtained an additional report of seven published papers (e.g. a thesis) and authors provided unpublished outcomes for nine studies reported only in published papers. To examine the effects of small study bias, the main analysis was repeated for each comparison using a fixed-effect model. The magnitude and the precision of the effects were essentially unchanged compared with no treatment (SMD 0.64, 95% CI 0.58 to 0.70) and compared with face-to-face therapy (SMD, -0.21, 95% CI -0.32 to -0.10). Funnel plots appeared symmetrical; however, these tests for small study bias were unlikely to identify bias in a review with a large number of small studies, each of which received similar weight in the analysis. The [Characteristics of studies awaiting classification](#) include several trials that may be complete, and the [Characteristics of excluded studies](#) include two trials that were discontinued.

Several trials included participants with anxiety or depression. Two did not report symptoms of anxiety (Holdsworth 1996; Tyrer 1988), and one excluded participants with depression from the analysis because so few people with depression were recruited (Maunder 2009). Six others contributed to the primary analysis (Fletcher 2005; Grime 2004; Mead 2005; Proudfoot 2004a; Seekles 2011; Zetterqvist 2003). Excluding these studies did not importantly change the overall effect (SMD 0.70, 95% CI 0.58 to 0.82; $\chi^2 = 210.97$, $df = 69$, P

< 0.00001 , $I^2 = 67\%$); furthermore, these studies were not included in an analysis restricted to symptoms of a specific anxiety disorder, which was also consistent with the primary analysis.

Agreements and disagreements with other studies or reviews

Many recent reviews have focused on Internet- and computer-delivered interventions, and this review has a wider scope. After accounting for differences in inclusion criteria, this review still includes more trials than previous syntheses of self-help for anxiety. As a result, few trials appear in both this review and most other meta-analyses, but overlap is difficult to characterise for other reasons. For example, one review aimed to include studies with less than an hour of contact (Lewis 2012); by contacting the authors, we discovered that many included studies did not meet this criterion. In addition to studies of PTSD, one review (Haug 2012) included a study that was not randomised (the wait-list taken from another trial in Tillfors 2008), one study that the author described as quasi-randomised (Luccock 2008) and one study that combined self-help with another intervention (i.e. Andersson 2006 compared self-help with in vivo exposure vs waiting list). One meta-analysis refers to studies that report outcome data but do not appear in the quantitative summary (Spek 2007). Another meta-analysis omitted many studies that we included, and it included studies that met neither our inclusion criteria nor its stated criteria (Hirai 2006).

Effects on symptoms of anxiety were somewhat smaller than those reported in recent reviews (Andrews 2010; Cuijpers 2010; Griffiths 2010b; Haug 2012; Lewis 2012; Reger 2009), and we report effects for secondary outcomes that have not been widely analysed. Furthermore, this review includes more information about the characteristics of included studies than was provided in previous reviews. This review also includes a comprehensive assessment of risk of bias, which identifies some important limitations in the results that have not been fully considered elsewhere.

AUTHORS' CONCLUSIONS

Implications for practice

We found evidence of efficacy compared with no intervention, and media-delivered interventions could be useful for people who are not seeking other services, but only a small number of people respond and recover when using media-delivered interventions alone. For people seeking services, face-to-face interventions appear to be superior to media-delivered interventions, leading to higher rates of recovery in practice.

This review identifies a lack of evidence about safety. No reports suggest that media-delivered interventions are unsafe, although trials have been limited to people who do not have severe depression, substance misuse or other risk factors. Confidence in the results of this review are also mitigated by risk of bias and differences among the included studies.

Many of the included studies provided therapist input averaging just over an hour. Contact during the intervention for research and treatment purposes could be related to compliance and efficacy.

Studies used a variety of delivery formats, and users may prefer different media. Although effects for Internet-delivered

interventions were larger than effects for other media, books with similar content could be equally effective.

This evidence is consistent with recent guidance indicating that people seeking treatment for social anxiety disorder in England and Wales should be offered individual cognitive therapy first; those who decline individual therapy should be offered supported self-help (Pilling 2013). Current recommendations for the treatment of generalised anxiety disorder suggest a stepped-care approach (Kendall 2011), which is not consistent with clinical results in this review, but health economic modelling based on these clinical results might inform an update of these guidelines.

Implications for research

Many studies have demonstrated the efficacy of particular interventions in particular populations; however, some of these interventions are not available outside a few academic groups. Most studies were conducted by the intervention developers. Further research is needed to examine publicly available interventions (e.g. self-help books) and interventions that could be widely rolled-out. Some of the included studies have also been examined in uncontrolled longitudinal studies (Hilvert-Bruce 2012; Newton 2012; Sunderland 2012), but large effectiveness trials are needed to demonstrate that such interventions work for large populations.

The benefits that can be achieved using media-delivered interventions may be limited; however, improved treatment manuals, increased interactivity and other innovations might lead to better outcomes than those observed here.

In many trials, study dropout rate was high, and some participants completing assessments did not complete treatment. Large studies could investigate mediators and moderators of compliance by monitoring how participants use Websites and which core components they receive, for example, a case series preceding an ongoing trial includes details about the interventions offered and actual uptake by participants (Stott 2013). Many trials in this review share limitations that have been documented in reports of complex interventions (Grant 2013), and future research would be more useful if similar data about implementation and uptake of interventions were reported consistently across trials (Mayo-Wilson 2007).

Most people do not respond to media-delivered interventions, but it is unclear why response and recovery rates are so low. Identifying

the differences between responders and non-responders could help improve treatment effectiveness and could help clinicians determine how to allocate people with anxiety to different services in a stepped-care framework.

Few studies have compared media-delivered CBT with CBT as it is normally delivered. Future studies of face-to-face therapies might use media-delivered CBT as a control condition (rather than no treatment). In addition, future studies might examine media-delivered interventions to prime people before they begin face-to-face therapy, to support face-to-face therapy or to help maintain gains from face-to-face therapy to examine the effects of media-delivered interventions at various stages in a stepped-care pathway.

Finally, this review includes a large number of studies and multiple co-variables, in addition to the main outcomes. Although the subgroup analyses already described are informative, a multivariate meta-regression might be a useful way to explore the importance of potential mediators and moderators, including contact with professionals.

ACKNOWLEDGEMENTS

Many authors of included and excluded studies answered questions and provided information about methods, study characteristics and outcomes; we are very grateful for their help in conducting this review.

We gratefully acknowledge support from the Centre for Evidence-Based Intervention (Department of Social Policy and Intervention; University of Oxford). EM-W also thanks the Centre for Outcomes Research and Effectiveness (Research Department of Clinical, Educational & Health Psychology; University College London) and the National Collaborating Centre for Mental Health (NCCMH). We thank Rachel Churchill, the Cochrane Methods Groups and anonymous reviewers for their helpful suggestions.

CRG Funding Acknowledgement:

The National Institute for Health Research (NIHR) is the largest single funder of the Cochrane Depression, Anxiety and Neurosis Group.

Disclaimer:

The views and opinions expressed therein are those of the authors and do not necessarily reflect those of the NIHR, the NHS or the Department of Health.

REFERENCES

References to studies included in this review

Abramowitz 2009 {published and unpublished data}

- * Abramowitz JS, Moore EL, Braddock AE, Harrington DL. Self-help cognitive-behavioral therapy with minimal therapist contact for social phobia: a controlled trial. *Journal of Behavioral Therapy and Experimental Psychiatry* 2009;**40**(1):98-105.

Al-Kubaisy 1992 {published and unpublished data}

- * Al-Kubaisy T. Clinician-Accompanied Exposure and Self-Exposure in Phobia Reduction: A Controlled Study. London: Institute of Psychiatry, 1990.

Al-Kubaisy T, Marks IM, Logsdail S, Marks MP, Lovell K, Sungar M, et al. Role of exposure homework in phobia reduction: a controlled study. *Behavior Therapy* 1992;**23**(4):599-621.

Park JM, Mataix-Cols D, Marks IM, Ngamthipwatthana T, Marks M, Araya R, et al. Two-year follow-up after a randomised controlled trial of self- and clinician-accompanied exposure for phobia/panic disorders. *The British Journal of Psychiatry* 2001;**178**:543-8.

Al-Kubaisy 1992 (AG) {published and unpublished data}

- * Al-Kubaisy T. Clinician-Accompanied Exposure and Self-Exposure in Phobia Reduction: A Controlled Study. London: Institute of Psychiatry, 1990.

Al-Kubaisy T, Marks IM, Logsdail S, Marks MP, Lovell K, Sungar M, et al. Role of exposure homework in phobia reduction: a controlled study. *Behavior Therapy* 1992;**23**(4):599-621.

Park JM, Mataix-Cols D, Marks IM, Ngamthipwatthana T, Marks M, Araya R, et al. Two-year follow-up after a randomised controlled trial of self- and clinician-accompanied exposure for phobia/panic disorders. *The British Journal of Psychiatry* 2001;**178**:543-8.

Al-Kubaisy 1992 (SAD) {published and unpublished data}

- * Al-Kubaisy T. Clinician-Accompanied Exposure and Self-Exposure in Phobia Reduction: A Controlled Study. London: Institute of Psychiatry, 1990.

Al-Kubaisy T, Marks IM, Logsdail S, Marks MP, Lovell K, Sungar M, et al. Role of exposure homework in phobia reduction: a controlled study. *Behavior Therapy* 1992;**23**(4):599-621.

Park JM, Mataix-Cols D, Marks IM, Ngamthipwatthana T, Marks M, Araya R, et al. Two-year follow-up after a randomised controlled trial of self- and clinician-accompanied exposure for phobia/panic disorders. *The British Journal of Psychiatry* 2001;**178**:543-8.

Al-Kubaisy 1992 (SP) {published and unpublished data}

- * Al-Kubaisy T. Clinician-Accompanied Exposure and Self-Exposure in Phobia Reduction: A Controlled Study. London: Institute of Psychiatry, 1990.

Al-Kubaisy T, Marks IM, Logsdail S, Marks MP, Lovell K, Sungar M, et al. Role of exposure homework in phobia reduction: a controlled study. *Behavior Therapy* 1992;**23**(4):599-621.

Park JM, Mataix-Cols D, Marks IM, Ngamthipwatthana T, Marks M, Araya R, et al. Two-year follow-up after a randomised controlled trial of self- and clinician-accompanied exposure for phobia/panic disorders. *The British Journal of Psychiatry* 2001;**178**:543-8.

Andersson 2009a {published data only (unpublished sought but not used)}

- * Andersson G, Waara J, Jonsson U, Malmaeus F, Carlbring P, Ost LG. Internet-based self-help versus one-session exposure in the treatment of spider phobia: a randomized controlled trial. *Cognitive Behaviour Therapy* 2009;**38**(2):114-20.

Andersson 2012a {published data only}

- * Andersson E, Enander J, Andrén P, Hedman E, Ljótsson B, Hursti T, et al. Internet-based cognitive behaviour therapy for obsessive-compulsive disorder: a randomized controlled trial. *Psychological Medicine* 2012;**21**:1-11.

Andersson 2012b {published data only}

- * Andersson G, Carlbring P, Furmark T, SOFIE Research Group. Therapist experience and knowledge acquisition in Internet-delivered CBT for social anxiety disorder: a randomized controlled trial. *PLoS ONE* 2012;**7**(5):e37411.

Andersson 2012c {published data only}

- * Andersson G, Paxling B, Roch-Norlund P, Östman G, Norgren A, Almlöv J, et al. Internet-based psychodynamic versus cognitive behavioral guided self-help for generalized anxiety disorder: a randomized controlled trial. *Psychotherapy and Psychosomatics* 2012;**81**(6):344-55.

Andrews 2011a {published and unpublished data}

- * Andrews GD, Davies M, Titov N. Effectiveness randomized controlled trial of face to face versus Internet cognitive behaviour therapy for social phobia. *Australian and New Zealand Journal of Psychiatry* 2011;**45**(4):337-40.

Baillie 2002 {published and unpublished data}

- * Baillie A. The Effectiveness of Stepped Cognitive-Behavioural Intervention for Panic Attacks. North Ryde, Australia: Psychology Department, Macquarie University, 2002.

Baillie AJ, Rapee RM. Predicting who benefits from psychoeducation and self help for panic attacks. *Behaviour Research and Therapy* 2004;**42**(5):513-27.

Baker 1973 {published and unpublished data}

- * Baker B L, Cohen DC, Saunders JT. Self-directed desensitization for acrophobia. *Behaviour Research and Therapy* 1973;**11**(1):79-89.

Barrera 1977 {published data only (unpublished sought but not used)}

- * Barrera M, Rosen G. Detrimental effects of a self-reward contracting program on subjects' involvement in self-administered desensitization. *Journal of Consulting and Clinical Psychology* 1977;**45**(6):1180-1.

Bell 2012 {published data only}

Bell C. Computerised cognitive behaviour therapy (CBT) for the treatment of anxiety and effects on neuropsychological functioning and hormone levels (ACTRN12606000349549), 2008. http://www.anzctr.org.au/trial_view.aspx?id=1534 (accessed 1 May 2013).

- * Bell CJ, Colhoun HC, Carter FA, Frampton CM. Effectiveness of computerised cognitive behaviour therapy for anxiety disorders in secondary care. *Australian and New Zealand Journal of Psychiatry* 2012;**46**(7):630-40.

Carter FA, Bell CJ, Colhoun HC. Suitability and acceptability of computerised cognitive behaviour therapy for anxiety disorders in secondary care. *Australian and New Zealand Journal of Psychiatry* 2012;**47**(2):142-52.

Berger 2009 {published and unpublished data}

Andersson G, Carlbring P. Commentary on Berger, Hohl, and Caspar's (2009) Internet-based treatment for social phobia: a randomized controlled trial. *Journal of Clinical Psychology* 2009 Oct;**65**(10):1036-8.

Berger T, Hohl E, Caspar F. Internet-based treatment for social phobia: a 6-month follow-up. *Zeitschrift für Klinische Psychologie und Psychotherapie* 2010;**39**(4):217-21.

- * Berger T, Hohl E, Caspar F. Internet-based treatment for social phobia: a randomized controlled trial. *Journal of Clinical Psychology* 2009;**65**(10):1021-35.

Bergstrom 2010 {published data only (unpublished sought but not used)}

- * Bergstrom J, Andersson G, Ljotsson B, Ruck C, Andreevitch S, Karlsson A, et al. Internet-versus group-administered cognitive behaviour therapy for panic disorder in a psychiatric setting: a randomised trial. *BMC Psychiatry* 2010;**10**:54.

Bickel 2007 {published and unpublished data}

- * Woolaway BK. An empirical test of CALM for PD: a computer-administered learning module for panic disorder. An empirical test of CALM for PD: a computer-administered learning module for panic disorder. PhD Dissertation. Columbus, Ohio: The Ohio State University, 2007.

Botella 2009 {published data only (unpublished sought but not used)}

- * Botella C, Gallego M, Garcia-Palacios A, Banos R, Quero S, Alcaniz M. The acceptability of an internet-based self-help treatment for fear of public speaking. *British Journal of Guidance & Counselling* 2009;**37**(3):297-311.

Botella 2010 {published data only (unpublished sought but not used)}

- * Botella C, Gallego M, Garcia-Palacios A, Guillen V, Banos R, Quero S, et al. An Internet-based self-help treatment for fear of public speaking: a controlled trial. *Cyberpsychology, Behavior, and Social Networking* 2010;**13**(4):407-21.

Bowler 2012 {published data only}

- * Bowler JO, Mackintosh B, Dunn BD, Mathews A, Dalgleish T, Hoppitt L. A comparison of cognitive bias modification for interpretation and computerized cognitive behavior therapy: effects on anxiety, depression, attentional control, and interpretive bias. *Journal of Consulting and Clinical Psychology* 2012;**80**(6):1021-33.

Bowman 1997 {published and unpublished data}

- * Bowman D, Scogin F, Floyd M, Patton E, Gist L. Efficacy of self-examination therapy in the treatment of generalized anxiety disorder. *Journal of Counseling Psychology* 1997;**44**(3):267-73.

Floyd M, McKendree-Smith N, Bailey E, Stump J, Scogin F, Bowman D. Two-year follow-up of self-examination therapy for generalized anxiety disorder. *Journal of Anxiety Disorders* 2002;**16**(4):369-75.

Carlbring 2001 {published and unpublished data}

Carlbring P. Panic! Its Prevalence, Diagnosis and Treatment via the Internet. Uppsala, Sweden: Acta Universitatis Upsaliensis, 2004.

- * Carlbring P, Westling B, Ljungstrand P, Ekselius L, Andersson G. Treatment of panic disorder via the Internet: a randomized trial of a self-help program. *Behavior Therapy* 2001;**32**(4):751-64.

Carlbring 2003 {published and unpublished data}

- * Carlbring P, Ekselius L, Andersson G. Treatment of panic disorder via the Internet: a randomized trial of CBT vs. applied relaxation. *Journal of Behavioral Therapy and Experimental Psychiatry* 2003;**34**(2):129-40.

Carlbring 2005 {published and unpublished data}

Andersson G, Carlbring P, Grimlund A. Predicting treatment outcome in internet versus face to face treatment of panic disorder. *Computers in Human Behavior* 2008; Vol. 24, issue 5:1790-801.

Carlbring P. Panic! Its Prevalence, Diagnosis and Treatment via the Internet. Uppsala, Sweden: Acta Universitatis Upsaliensis, 2004.

- * Carlbring P, Nilsson-Ihrfelt E, Waara J, Kollenstama C, Buhrman M, Kaldova V, et al. Treatment of panic disorder: live therapy vs. self-help via the Internet. *Behaviour Research and Therapy* 2005;**43**(10):1321-33.

Carlbring 2006a {published and unpublished data}

Almlov JC. Therapist effects in guided internet-delivered CBT for anxiety disorders. *Behavioural and Cognitive Psychotherapy* 2011; Vol. 39, issue 3:311-22.

Carlbring P. Panic! Its prevalence, diagnosis and treatment via the internet. Panic! Its Prevalence, Diagnosis and Treatment via the Internet. Uppsala, Sweden: Acta Universitatis Upsaliensis, 2004.

* Carlbring P, Bohman S, Brunt S, Buhrman M, Westling BE, Ekselius L, et al. Remote treatment of panic disorder: a randomized trial of internet-based cognitive behavior therapy supplemented with telephone calls. *American Journal of Psychiatry* 2006;**163**(12):2119-25.

Carlbring PM. All at once or one at a time? A randomized controlled trial comparing two ways to deliver bibliotherapy for panic disorder. *Cognitive Behaviour Therapy* 2011; Vol. 40, issue 3:228-35.

Carlbring 2007 {published and unpublished data}

Almlov JC. Therapist effects in guided internet-delivered CBT for anxiety disorders. *Behavioural and Cognitive Psychotherapy* 2011; Vol. 39, issue 3:311-22.

* Carlbring P, Gunnarsdottir M, Hedensjö L, Andersson G, Ekselius L, Furmark T. Treatment of social phobia: randomised trial of internet-delivered cognitive-behavioural therapy with telephone support. *British Journal of Psychiatry* 2007;**190**:123-8.

Carlbring P, Nordgren LB, Furmark T, Andersson G. Long-term outcome of internet-delivered cognitive-behavioural therapy for social phobia: a 30-month follow-up. *Behaviour Research and Therapy* 2009;**47**(10):848-50.

Carlbring 2011 {published data only}

* Carlbring P, Maurin L, Torngren C, Linna E, Eriksson T, Sparthar E, et al. Individually-tailored, Internet-based treatment for anxiety disorders: a randomized controlled trial. *Behaviour Research and Therapy* 2011;**49**(1):18-24.

Chung 2008 {published data only}

* Chung YS, Kwon JH. The efficacy of bibliotherapy for social phobia. *Brief Treatment and Crisis Intervention* 2008;**8**(4):390-401.

Donnan 1990 {published data only (unpublished sought but not used)}

* Donnan P, Hutchinson A, Paxton R, Grant B, Firth M. Self-help materials for anxiety: a randomized controlled trial in general practice. *British Journal of General Practice* 1990;**40**(341):498-501.

Febbraro 1997a {published and unpublished data}

Febbraro G, Clum G, Roodman A, Wright J. The limits of bibliotherapy: a study of self-administered interventions in individuals with panic attacks. *Behavior Therapy* 1999;**30**:209-22.

* Febbraro GA. An investigation of the differential effectiveness of bibliotherapy and self-regulatory treatments in individuals with panic attacks. Dissertation Abstracts International: Section B: The Sciences and Engineering 1997.

Roodman AA. Assessment and feedback. Unpublished Master's thesis, 1995.

Febbraro 1997b {published and unpublished data}

Febbraro G, Clum G, Roodman A, Wright J. The limits of bibliotherapy: a study of self-administered interventions in individuals with panic attacks. *Behavior Therapy* 1999;**30**:209-22.

* Febbraro GA. An investigation of the differential effectiveness of bibliotherapy and self-regulatory treatments in individuals with panic attacks. Dissertation Abstracts International: Section B: The Sciences and Engineering 1997.

Roodman AA. Assessment and feedback. Unpublished Master's thesis, 1995.

Febbraro 2005 {published and unpublished data}

* Febbraro G. An investigation into the effectiveness of bibliotherapy and minimal contact interventions in the treatment of panic attacks. *Journal of Clinical Psychology* 2005;**61**(6):763-79.

Fletcher 2005 {published and unpublished data}

* Fletcher J, Lovell K, Bower P, Campbell M, Dickens C. Process and outcome of a non-guided self-help manual for anxiety and depression in primary care: a pilot study. *Behavioural and Cognitive Psychotherapy* 2005;**33**:319-31.

Fraser 2001 {published data only (unpublished sought but not used)}

* Fraser JK. Three versus six sessions of computer-aided vicarious exposure treatment for spider phobia. *Behaviour Change* 2001; Vol. 18, issue 4:213-23.

Furmark 2009a {published data only (unpublished sought but not used)}

* Furmark T, Carlbring P, Hedman E, Sonnenstein A, Clevberger P, Bohman B, et al. Guided and unguided self-help for social anxiety disorder: randomised controlled trial. *British Journal of Psychiatry* 2009;**195**(5):440-7.

Hedman E, Furmark T, Carlbring P, Ljotsson B, Ruck C, Lindefors N, et al. A 5-year follow-up of internet-based cognitive behavior therapy for social anxiety disorder [NCT01145690]. *Journal of Medical Internet Research* 2011; Vol. 13, issue 2:e39.

Furmark 2009b {published data only (unpublished sought but not used)}

* Furmark T, Carlbring P, Hedman E, Sonnenstein A, Clevberger P, Bohman B, et al. Guided and unguided self-help for social anxiety disorder: randomised controlled trial. *British Journal of Psychiatry* 2009;**195**(5):440-7.

Hedman E, Furmark T, Carlbring P, Ljotsson B, Ruck C, Lindefors N, et al. A 5-year follow-up of internet-based cognitive behavior therapy for social anxiety disorder. *Journal of Medical Internet Research* 2011; Vol. 13, issue 2:e39.

Ghosh 1984 {published and unpublished data}

Ghosh A, Marks I. Self-treatment of agoraphobia by exposure. *Behavior Therapy* 1987;**18**(1):3-16.

* Ghosh A, Marks I, Carr A. Controlled study of self-exposure treatment for phobias: preliminary communication. *Journal of the Royal Society of Medicine* 1984;**77**(6):483-7.

Ghosh A, Marks I, Carr A. Therapist contact and outcome of self-exposure treatment for phobias: a controlled study. *British Journal of Psychiatry* 1988;**152**:234-8.

Gilroy 2000 {published and unpublished data}

* Gilroy LJ, Kirkby KC, Daniels BA, Menzies RG, Montgomery IM. Controlled comparison of computer-aided vicarious exposure versus live exposure in the treatment of spider phobia. *Behavior Therapy* 2000;**31**(4):733-44.

Gilroy LJ, Kirkby KC, Daniels BA, Menzies RG, Montgomery IM. Long-term follow-up of computer-aided vicarious exposure versus live graded exposure in the treatment of spider phobia. *Behavior Therapy* 2003;**34**(1):65-76.

Gould 1993a {published and unpublished data}

* Gould RA, Clum GA, Shapiro D. The use of bibliotherapy in the treatment of panic: a preliminary investigation. *Behavior Therapy* 1993;**24**(2):241-52.

Gould 1995 {published and unpublished data}

* Gould RA, Clum GA. Self-help plus minimal therapist contact in the treatment of panic disorder: a replication and extension. *The American Journal of Psychiatry* 1995;**26**(3):533-46.

Granado 2007 {published data only}

* Granado LC, Ranvaud R, Peláez JR. A spiderless arachnophobia therapy: comparison between placebo and treatment groups and six-month follow-up study. *Neural Plasticity* 2007;**2007**:10241.

Greist 2002 {published data only (unpublished sought but not used)}

* Greist JH, Marks IM, Baer L, Kobak KA, Wenzel KW, Hirsch MJ, et al. Behavior therapy for obsessive-compulsive disorder guided by a computer or by a clinician compared with relaxation as a control. *Journal of Clinical Psychiatry* 2002;**63**(2):138-45.

Mataix-Cols DMI. Obsessive-compulsive symptom dimensions as predictors of compliance with and response to behaviour therapy: results from a controlled trial. *Psychotherapy and Psychosomatics* 2002;**71**(5):255-62.

McCrone P, Marks IM, Greist JH, Baer L, Kobak KA, Wenzel KW, et al. Cost-effectiveness of computer-aided behaviour therapy for obsessive-compulsive disorder. *Psychotherapy & Psychosomatics* 2007;**76**(4):249-50.

McCrone P, Knapp M, Proudfoot J, Ryden C, Cavanagh K, Shapiro DA, et al. Erratum. Cost-effectiveness of computerised cognitive-behavioural therapy for anxiety and depression in primary care: randomised controlled trial. *British Journal of Psychiatry* 2004;**185**:440.

Grime 2004 {published and unpublished data}

* Grime PR. An open, randomised study, to compare the effects of a computerised cognitive behavioural therapy programme

(‘beating the blues’) plus conventional care vs. conventional care alone on absence from work due to anxiety, depression or stress. Faculty of Occupational Medicine of the Royal College of Physicians, 2001.

Grime PR. Computerized cognitive behavioural therapy at work: a randomized controlled trial in employees with recent stress-related absenteeism. *Occupational Medicine* 2004; Vol. 54, issue 5:353-9.

Hassan 1992 {published and unpublished data}

* Hassan AAM. A comparison of computer-based symbolic modelling and conventional methods in the treatment of spider phobia. University of Leeds, England, 1992.

Hazen 1996 {published and unpublished data}

* Hazen A, Walker J, Eldridge G. Anxiety sensitivity and treatment outcome in panic disorder. *Anxiety* 1996;**2**(1):34-9.

Walker JG, Eldridge A, Hazen RJ, O’Riordan J, Frankel S. Evaluation of a self-help book and workbook for treatment of panic disorder used independently in a self-help group or a professional group. University of Manitoba, 1996.

Heading 2001 {published and unpublished data}

* Heading K, Kirkby KC, Martin F, Daniels BA, Gilroy LJ, Menzies RG. Controlled comparison of single-session treatments for spider phobia: live graded exposure alone versus computer-aided vicarious exposure. *Journal of Behavior Therapy and Experimental Psychiatry* 2001;**18**:103-13.

Hedman 2011a {published data only}

Hedman E, Andersson E, Lindefors N, Andersson G, Ruck C, Ljótsson B. Costeffectiveness and longterm effectiveness of Internetbased cognitive behaviour therapy for severe health anxiety. *Psychological Medicine* 2013;**43**(2):363-74.

* Hedman E, Andersson G, Andersson E, Ljótsson B, Ruck C, Asmundson GJ, et al. Internet-based cognitive-behavioural therapy for severe health anxiety: randomised controlled trial. *British Journal of Psychiatry* 2011;**198**:230-6.

Hedman EEMA, Hedman E. Internet-based cognitive-behavioural therapy for severe health anxiety: randomised controlled trial. Author’s reply. *British Journal of Psychiatry* 2011; Vol. 199, issue 3:251.

Udo II. Internet-based CBT for severe health anxiety. *British Journal of Psychiatry* 2011; Vol. 199, issue 3:250-1.

Hedman 2011b {published data only}

* Hedman E, Andersson G, Ljótsson B, Andersson E, Rück C, Mörtberg E, et al. Internet-based cognitive behavior therapy vs. cognitive behavioral group therapy for social anxiety disorder: a randomized controlled non-inferiority trial. *PLoS One* 2011;**6**(3):e18001.

Hedman E, Andersson E, Ljótsson B, Andersson G, Andersson E, Schalling M, et al. Clinical and genetic outcome determinants of Internet- and group-based cognitive behavior therapy for social anxiety disorder. *Acta Psychiatrica Scandinavica* 2012;**126**(2):126-36.

- Hedman EA. Cost-effectiveness of Internet-based cognitive behavior therapy vs. cognitive behavioral group therapy for social anxiety disorder: results from a randomized controlled trial. *Behaviour Research and Therapy* 2011; Vol. 49, issue 11:729-36.
- Hellström 1995** {published data only (unpublished sought but not used)}
- * Hellström K, Öst L. One-session therapist directed exposure vs two forms of manual directed self-exposure in the treatment of spider phobia. *Behaviour Research and Therapy* 1995;**33**(8):959-65.
- Holdsworth 1996** {published and unpublished data}
- * Holdsworth N, Paxton R, Seidel S, Thomson D, Shrubbs S. Parallel evaluations of new guidance materials for anxiety and depression in primary care. *Journal of Mental Health* 1996;**5**(2):195-207.
- Houghton 2008** {published and unpublished data}
- * Houghton VT. A quantitative study of the effectiveness of mindfulness-based stress reduction treatment, using an internet-delivered self-help program, for women with generalized anxiety disorder. Dissertation Abstracts International: Section B: The Sciences and Engineering.69(5-B) 2008.
- Johnston 2011** {published data only}
- * Johnston L, Titov N, Andrews G, Spence J, Dear BF. A RCT of a transdiagnostic internet-delivered treatment for three anxiety disorders: examination of support roles and disorder-specific outcomes. *PLoS ONE* 2011;**6**(11):e28079.
- Jones 2002a** {published data only (unpublished sought but not used)}
- * Jones FA. The role of bibliotherapy in health anxiety: an experimental study. *British Journal of Community Nursing* 2002;**7**(10):498-504.
- Kenardy 2003** {published and unpublished data}
- * Kenardy JA, Dow MGT, Johnston DW, Newman MG, Thomson A, Barr Taylor C. A comparison of delivery methods of cognitive-behavioral therapy for panic disorder: an international multicenter trial. *Journal of Consulting and Clinical Psychology* 2003;**71**(6):1068-75.
- Kenardy JR. Cognitive behaviour therapy for panic disorder: long-term follow up. *Cognitive Behaviour Therapy* 2005;**34**(2):75-8.
- Kiely 2002** {published and unpublished data}
- Kiely BG. Bibliotherapy for GAD: mediating and predictor variables influencing outcome. Unpublished manuscript, 2002.
- * Kiely BG. Bibliotherapy for generalised anxiety disorder: a controlled trial: comparison and exploration of factors related to outcome. Unpublished manuscript, 2002.
- Kiropoulos 2008** {published and unpublished data}
- * Kiropoulos LA, Klein B, Austin DW, Gilson K, Pier C, Mitchell J, et al. Is internet-based CBT for panic disorder and agoraphobia as effective as face-to-face CBT?. *Journal of Anxiety Disorders* 2008;**22**:1273-84.
- Klein 2001** {published and unpublished data}
- * Klein B, Richards JC. A brief Internet-based treatment for panic disorder. *Behavioural and Cognitive Psychotherapy* 2001;**29**(1):113-7.
- Klein 2006** {published data only (unpublished sought but not used)}
- * Klein B, Richards J. Efficacy of Internet therapy for panic disorder. *Journal of Behavior Therapy and Experimental Psychiatry* 2006;**37**(3):213-38.
- Koszycski 2011** {published data only}
- Bradwejn JK. Efficacy of self-administered cognitive behavior therapy and sertraline in panic disorder. Proceedings of the 156th Annual Meeting of the American Psychiatric Association; May 17-22, 2003; San Francisco, California.
- * Koszycski D, Taljaard M, Segal Z, Bradwejn J. A randomized trial of sertraline, self-administered cognitive behavior therapy, and their combination for panic disorder. *Psychological Medicine* 2011;**41**(2):373-83.
- Lewis 1978** {published data only (unpublished sought but not used)}
- * Lewis C. Self-administered relaxation training for anxiety management and the contribution of money deposits and locus of control. University of Oregon, 1977.
- Lewis CE, Biglan A, Steinbock E. Self-administered relaxation training and money deposits in the treatment of recurrent anxiety. *Journal of Consulting and Clinical Psychology* 1978;**46**(5):1274-83.
- Lidren 1994** {published and unpublished data}
- Lidren D, Watkins P, Gould R, Clum G. A comparison of bibliotherapy and group therapy in the treatment of panic disorder. *Journal of Consulting and Clinical Psychology* 1994;**62**(4):865-9.
- * Lidren DM. The differential efficacy of self-help and group therapy in the treatment of panic disorder and agoraphobia: a treatment comparison study. Washington State University, 1993.
- Marks 2004** {published and unpublished data}
- * Marks I, Kenwright M, McDonough M, Whittaker M, Mataix-Cols D. Saving clinicians' time by delegating routine aspects of therapy to a computer: a randomized controlled trial in phobia/panic disorder. *Psychological Medicine* 2004;**34**(1):9-17.
- McCrone P, Marks IM, Mataix-Cols D, Kenwright M, McDonough M. Computer-aided self-exposure therapy for phobia/panic disorder: a pilot economic evaluation. *Cognitive Behaviour Therapy* 2009;**38**(2):91-9.
- Matthews 2011** {published data only}
- * Matthews AJ. Online exposure for spider phobia: continuous versus intermittent exposure. *Behaviour Change* 2011; Vol. 28, issue 3.

Mauder 2009 {published data only (unpublished sought but not used)}

* Mauder L, Cameron L, Moss M, Muir D, Evans N, Paxton R, et al. Effectiveness of self-help materials for anxiety adapted for use in prison: a pilot study. *Journal of Mental Health* 2009;**18**(3):262-71.

Mauder L, Cameron L, Robson S, Quayle G. Adapting Self-Help Materials for the Prison Setting. Northumberland: Tyne and Wear Mental Health Trust, 2004.

McNamee 1989 {published data only (unpublished sought but not used)}

* McNamee G, O'Sullivan G, Lelliott P, Marks I. Telephone-guided treatment for housebound agoraphobics with panic disorder: exposure vs. relaxation. *Behavior Therapy* 1989;**20**(4):491-7.

Mead 2005 {published and unpublished data}

* Mead N, MacDonald W, Bower P, Lovell K, Richards D, Roberts C, et al. The clinical effectiveness of guided self-help versus waiting-list control in the management of anxiety and depression: a randomized controlled trial. *Psychological Medicine* 2005;**35**(11):1633-43.

Young C, Brosan L, Shelfer L, Beazley P. The clinical effectiveness of guided self-help versus waiting-list control in the management of anxiety and depression: a randomised controlled trial: Comment. *Psychological Medicine* 2006;**36**(5):725-6.

Milne 1988 {published and unpublished data}

* Milne D, Covitz F. A comparative evaluation of anxiety management materials in general practice. *Health Education Journal* 1988;**47**(2/3):67-9.

Moritz 2010 {published data only (unpublished sought but not used)}

* Moritz S, Jelinek L, Hauschildt M, Naber D. How to treat the untreated: effectiveness of a self-help metacognitive training program (myMCT) for obsessive-compulsive disorder. *Dialogues in Clinical Neuroscience* 2010;**12**(2):209-20.

Muller 2011 {published data only (unpublished sought but not used)}

* Muller BHK. One-session computer-based exposure treatment for spider-fearful individuals: efficacy of a minimal self-help intervention in a randomised controlled trial. *Journal of Behavior Therapy and Experimental Psychiatry* 2011;**42**(2):179-84.

Nordin 2010 {published data only (unpublished sought but not used)}

* Nordin S, Carlbring P, Cuijpers P, Andersson G. Expanding the limits of bibliotherapy for panic disorder: randomized trial of self-help without support but with a clear deadline. *Behavior Therapy* 2010;**41**(3):267-76.

Ost 1991 {published and unpublished data}

* Öst L, Salkovskis P, Hellström K. One-session therapist-directed exposure vs. self-exposure in the treatment of spider phobia. *Behavior Therapy* 1991;**22**(3):407-22.

Paxling 2011 {published data only}

Almlov JC. Therapist effects in guided internet-delivered CBT for anxiety disorders. *Behavioural and Cognitive Psychotherapy* 2011; Vol. 39, issue 3:311-22.

* Paxling B, Almlov J, Dahlin M, Carlbring P, Breitholtz E, Eriksson T, et al. Guided Internet-delivered cognitive behavior therapy for generalized anxiety disorder: a randomized controlled trial. *Cognitive Behaviour Therapy* 2011; Vol. 40, issue 3:159-73.

Proudfoot 2004a {published and unpublished data}

McCrone P, Knapp M, Proudfoot J, Ryden C, Cavanagh K, Shapiro DA, et al. Cost-effectiveness of computerised cognitive behavioural therapy for anxiety and depression in primary care. *British Journal of Psychiatry* 2004;**185**:55-62.

Proudfoot J, Goldberg D, Mann A, Everitt B, Marks I, Gray JA. Computerized, interactive, multimedia cognitive-behavioural program for anxiety and depression in general practice. *Psychological Medicine* 2003;**33**(2):217-27.

* Proudfoot J, Ryden C, Everitt B, Shapiro DA, Goldberg D, Mann A, et al. Clinical efficacy of computerised cognitive-behavioural therapy for anxiety and depression in primary care: randomised controlled trial. *British Journal of Psychiatry* 2004;**185**:46-54.

Rapee 2007 {published and unpublished data}

* Rapee RM, Abbott MJ, Baillie AJ, Gaston JE. Treatment of social phobia through pure self-help and therapist-augmented self-help. *British Journal of Psychiatry* 2007;**191**:246-52.

Richards 2003a {published data only (unpublished sought but not used)}

* Richards A, Barkham M, Cahill J, Richards D, Williams C, Heywood P. PHASE: a randomised, controlled trial of supervised self-help cognitive behavioural therapy in primary care. *British Journal of General Practice* 2003;**53**(495):764-70.

Richards 2006 {published data only (unpublished sought but not used)}

* Richards JC, Klein B, Austin DW. Internet cognitive behavioural therapy for panic disorder: does the inclusion of stress management information improve end-state functioning?. *Clinical Psychologist* 2006;**10**(1):2-15.

Robinson 2010 {published data only (unpublished sought but not used)}

* Robinson E, Titov N, Andrews G, McIntyre K, Schwencke G, Solley K. Internet treatment for generalized anxiety disorder: a randomized controlled trial comparing clinician vs. technician assistance. *PLoS ONE* 2010;**5**(6):e10942.

Rosen 1976 {published data only (unpublished sought but not used)}

Rosen GM, Glasgow RE, Barrera M. A two-year follow-up on systematic desensitization with data pertaining to the external validity of laboratory fear assessment. *Journal of Consulting & Clinical Psychology* 1977;**45**(6):1188-9.

* Rosen GM, Glasgow RE, Barrera NM. A controlled study to assess the clinical efficacy of totally self-administered systematic desensitization. *Journal of Consulting and Clinical Psychology* 1976;**44**(2):208-17.

Rosmarin 2010 {published and unpublished data}

* Rosmarin DH. A randomized controlled evaluation of a spiritually integrated cognitive behavioral intervention for sub-clinical anxiety among Jews, delivered via the internet. Bowling Green State University, 2010.

Rosmarin DH, Pargament KI, Pirutinsky S, Mahoney A. A randomized controlled evaluation of a spiritually integrated treatment for subclinical anxiety in the Jewish community, delivered via the internet. *Journal of Anxiety Disorders* 2010;**24**(7):799-808.

Ruwaard 2010 {published data only}

* Ruwaard J, Broeksteeg J, Schrieken B, Emmelkamp P, Lange A. Web-based therapist-assisted cognitive behavioral treatment of panic symptoms: a randomized controlled trial with a three-year follow-up. *Journal of Anxiety Disorders* 2010;**24**:387-96.

Salaberria 1995 {published and unpublished data}

* Salaberria IK, Echeburua OE. Psychological treatment in social phobia: an experimental study. [Spanish] Tratamiento psicologico de la fobia social: un estudio experimental. Analisis y Modificacion de Conducta 1995; Vol. .21, issue 76:151-79.

Salaberria K, Echeburua E. Long-term outcome of cognitive therapy's contribution to the treatment of generalized social phobia. *Behavior Modification* 1998;**22**(3):262-84.

Schneider 2005 {published data only (unpublished sought but not used)}

* Schneider AJ, Mataix-Cols D, Marks IM, Bachofen M. Internet-guided self-help with or without exposure therapy for phobic and panic disorders. *Psychotherapy and Psychosomatics* 2005;**74**(3):154-64.

Seekles 2011 {published data only}

* Seekles W, van Straten A, Beekman A, van Marwijk H, Cuijpers P. Effectiveness of guided self-help for depression and anxiety disorders in primary care: a pragmatic randomized controlled trial. *Psychiatry Research* 2011;**187**(1-2):113-20.

Seekles W, van Straten A, Beekman A, van Marwijk H, Cuijpers P. Stepped care for depression and anxiety. From primary care to specialized mental health care: a randomised controlled trial testing the effectiveness of a stepped care program among primary care patients with mood or anxiety disorders. *BMC Health Services Research* 2009;**9**:90.

Sharp 2000 {published and unpublished data}

Power KG, Sharp DM, Swanson V, Simpson RJ. Therapist contact in cognitive behaviour therapy for panic disorder and agoraphobia in primary care. *Clinical Psychology & Psychotherapy* 2000;**7**(1):37-46.

* Sharp DM, Power KG, Swanson V. Reducing therapist contact in cognitive behaviour therapy for panic disorder and agoraphobia in primary care: global measures of outcome in a randomised controlled trial. *British Journal of General Practice* 2000;**50**(461):963-8.

Shoenberger 2008 {published and unpublished data}

* Shoenberger D. Preliminary investigation of the effectiveness of a self-help intervention for chronic worry. University of Nevada, 2008.

Silfvernagel 2012 {published data only}

* Silfvernagel K, Carlbring P, Kabo J, Edström S, Eriksson J, Månson L, et al. Individually tailored internet-based treatment of young adults and adults with panic symptoms: a randomized controlled trial. *Journal of Medical Internet Research* 2012; Vol. 14, issue 3:e65.

Smith 1997 {published data only (unpublished sought but not used)}

* Smith KL, Kirkby KC, Montgomery IM, Daniels BA. Computer-delivered modeling of exposure for spider phobia: relevant versus irrelevant exposure. *Journal of Anxiety Disorders* 1997;**11**(5):489-97.

Sorby 1991 {published and unpublished data}

* Sorby NG, Reavley W, Huber JW. Self help programme for anxiety in general practice: controlled trial of an anxiety management booklet. *British Journal of General Practice* 1991;**41**(351):417-20.

Tarrier 1986 {published and unpublished data}

* Tarrier N, Main CJ. Applied relaxation training for generalised anxiety and panic attacks: the efficacy of a learnt coping strategy on subjective reports. *British Journal of Psychiatry* 1986;**149**:330-6.

Titov 2008a {published and unpublished data}

Titov N, Andrews G, Johnston L, Schwencke G, Choi I. Shyness programme: longer term benefits, cost-effectiveness, and acceptability. *Australian & New Zealand Journal of Psychiatry* 2009;**43**(1):36-44.

* Titov N, Andrews G, Schwencke G, Drobny J, Einstein D. Shyness 1: distance treatment of social phobia over the Internet. *Australian and New Zealand Journal of Psychiatry* 2008;**42**(7):585-94.

Titov N, Gibson M, Andrews G, McEvoy P. Internet treatment for social phobia reduces comorbidity. *Australian & New Zealand Journal of Psychiatry* 2009 Aug;**43**(8):754-9.

Titov 2008b {published and unpublished data}

Titov N, Andrews G, Johnston L, Schwencke G, Choi I. Shyness programme: longer term benefits, cost-effectiveness, and

acceptability. *Australian & New Zealand Journal of Psychiatry* 2009;**43**(1):36-44.

* Titov N, Andrews G, Schwencke G. Shyness 2: treating social phobia online: replication and extension. *Australian and New Zealand Journal of Psychiatry* 2008;**42**(7):595-605.

Titov N, Gibson M, Andrews G, McEvoy P. Internet treatment for social phobia reduces comorbidity. *Australian & New Zealand Journal of Psychiatry* 2009 Aug;**43**(8):754-9.

Titov 2008c {published and unpublished data}

* Titov N, Andrews G, Choi I, Schwencke G, Mahoney A. Shyness 3: Randomized controlled trial of guided versus unguided Internet-based CBT for social phobia. *Australian and New Zealand Journal of Psychiatry* 2008;**42**(12):1030-40.

Titov N, Gibson M, Andrews G, McEvoy P. Internet treatment for social phobia reduces comorbidity. *Australian & New Zealand Journal of Psychiatry* 2009 Aug;**43**(8):754-9.

Titov 2009a {published and unpublished data}

Lorian CNT. Changes in risk-taking over the course of an internet-delivered cognitive behavioral therapy treatment for generalized anxiety disorder. *Journal of Anxiety Disorders* 2012; Vol. 26, issue 1:140-9.

* Titov N, Andrews G, Robinson E, Schwencke G, Johnston L, Solley K, et al. Clinician-assisted Internet-based treatment is effective for generalized anxiety disorder: randomized controlled trial. *Australian and New Zealand Journal of Psychiatry* 2009;**43**(10):905-12.

Titov 2010a {published data only}

* Titov N, Andrews G, Johnston L, Robinson E, Spence J. Transdiagnostic Internet treatment for anxiety disorders: a randomized controlled trial. *Behaviour Research and Therapy* 2010 Sep;**48**(9):890-9.

Titov 2010a (GAD) {published data only}

Titov N, Andrews G, Johnston L, Robinson E, Spence J. Transdiagnostic Internet treatment for anxiety disorders: a randomized controlled trial. *Behaviour Research and Therapy* 2010;**48**(9):890-9.

Titov 2010a (PD) {published data only}

Titov N, Andrews G, Johnston L, Robinson E, Spence J. Transdiagnostic Internet treatment for anxiety disorders: a randomized controlled trial. *Behaviour Research and Therapy* 2010;**48**(9):890-9.

Titov 2010a (SAD) {published data only}

Titov N, Andrews G, Johnston L, Robinson E, Spence J. Transdiagnostic Internet treatment for anxiety disorders: a randomized controlled trial. *Behaviour Research and Therapy* 2010;**48**(9):890-9.

Titov 2010b {published data only}

* Titov N, Andrews G, Schwencke G, Robinson E, Peters L, Spence J. Randomized controlled trial of internet cognitive behavioural treatment for social phobia with and without

motivational enhancement strategies. *Australian and New Zealand Journal of Psychiatry* 2010;**44**(10):938-45.

Tolin 2007 {published and unpublished data}

* Tolin D F, Hannan S, Maltby N, Diefenbach GJ, Worhunsky P, Brady RE. A randomized controlled trial of self-directed versus therapist-directed cognitive-behavioral therapy for obsessive-compulsive disorder patients with prior medication trials. *Behavior Therapy* 2007;**38**(2):179-91.

Tolin 2011 {published data only}

* Tolin DF, Gretchen JD, Gilliam CM. Stepped care versus standard cognitive-behavioral therapy for obsessive-compulsive disorder: a preliminary study of efficacy and costs. *Depression and Anxiety* 2011;**28**(4):314-23.

Tortella-Feliu 2011 {published data only}

* Tortella-Feliu M, Botella C, Llabres J, Breton-Lopez JM, del Amo AR, Banos RM, et al. Virtual reality versus computer-aided exposure treatments for fear of flying. *Behavior Modification* 2011;**35**(1):3-30.

Tyrer 1988 {published data only (unpublished sought but not used)}

Tyrer P, Seivewright H, Johnson T. The Nottingham Study of Neurotic Disorder: predictors of 12-year outcome of dysthymic, panic and generalized anxiety disorder. *Psychological Medicine* 2004 Nov;**34**(8):1385-94.

Tyrer P, Seivewright N, Ferguson B, Murphy S, Darling C, Brothwell J, et al. The Nottingham Study of Neurotic Disorder: relationship between personality status and symptoms. *Psychological Medicine* 1990;**20**(2):423-31.

Tyrer P, Seivewright N, Ferguson B, Murphy S, Johnson AL. The Nottingham study of neurotic disorder: effect of personality status on response to drug treatment, cognitive therapy and self-help over two years. *British Journal of Psychiatry* 1993;**162**:219-26.

* Tyrer P, Seivewright N, Murphy S, Ferguson B, Kingdon D, Barczak P, et al. The Nottingham study of neurotic disorder: comparison of drug and psychological treatments. *Lancet* 1988 Jul 30;**2**(8605):235-40.

van Boeijen 2005a {published and unpublished data}

* van Boeijen CA, van Oppen P, van Balkom AJ, Visser S, Kempe PT, Blankenstein N, et al. Treatment of anxiety disorders in primary care practice: a randomised controlled trial. *British Journal of General Practice* 2005;**55**(519):763-9.

Varley 2011 {published and unpublished data}

* Varley R, Webb, TL, Sheeran P. Making self-help more helpful: a randomized controlled trial of the impact of augmenting self-help materials with implementation intentions on promoting the effective self-management of anxiety symptoms. *Journal of Consulting and Clinical Psychology* 2010;**79**(1):123-8.

Walker 2001 {published and unpublished data}

* Walker JR, Cox BJ, Frankel S, Torgrud LJ. Evaluating two cognitive-behavioral self-help approaches for generalized social

phobia. Proceedings of the World Congress of Cognitive and Behavioral Therapies; July 22-25, 2001; Vancouver, BC.

White 1995 {published and unpublished data}

White J. "Stresspac": three-year follow-up of a controlled trial of self-help package for the anxiety disorders. *Behavioural and Cognitive Psychotherapy* 1998;**26**(2):133-41.

* White J. Stresspac: a controlled trial of a self-help package for the anxiety disorders. *Behavioural and Cognitive Psychotherapy* 1995;**23**(2):89-107.

Wims 2010 {published data only}

* Wims E, Titov N, Andrews G, Choi I. Clinician-assisted Internet-based treatment is effective for panic: a randomized controlled trial. *Australian & New Zealand Journal of Psychiatry* 2010;**44**(7):599-607.

Zetterqvist 2003 {published and unpublished data}

* Zetterqvist K, Maanmies J, Strom L, Andersson G. Randomized controlled trial of internet-based stress management. *Cognitive Behaviour Therapy* 2003;**32**(3):151-60.

References to studies excluded from this review

Allen 1973 {published data only}

* Allen GJ. Treatment of test anxiety by group-administered and self-administered relaxation and study counseling. *Behavior Therapy* 1973;**4**(3):349-60.

Altmaier 1981 {published data only}

* Altmaier EM, Woodward M. Group vicarious desensitization of test anxiety. *Journal of Counseling Psychology* 1981;**28**(5):467-9.

Andersson 2006 {published data only}

* Andersson G, Carlbring P, Holmström A, Sparthian E, Furmark T, Nilsson-Ihrfelt E, et al. Internet-based self-help with therapist feedback and in vivo group exposure for social phobia: a randomized controlled trial. *Journal of Consulting and Clinical Psychology* 2006 Aug;**74**(4):677-86.

Arpin-Cribbie 2008 {published data only}

* Arpin-Cribbie CA. Perfectionism related cognitions and psychological distress: a randomized trial evaluating the relative effectiveness of a web-based cognitive behavioral intervention protocol. York University, Canada, 2007.

Ayres 1993 {published data only}

* Ayres J, Ayres FE, Baker AL, Colby N, De Blasy C, Dimke D, et al. Two empirical tests of a videotape designed to reduce public speaking anxiety. *Journal of Applied Communication Research* 1993;**21**(2):132-47.

Bennett 1985 {published data only}

* Bennett G, Millard M. Compliance with relaxation training: the effect of providing information. *Behavioural Psychotherapy* 1985;**13**(2):110-9.

Berger 2011 {published data only}

* Berger T, Caspar F, Richardson R, Kneubühler B, Sutter D, Andersson G. Internet-based treatment of social phobia: a

randomized controlled trial comparing unguided with two types of guided self-help. *Behaviour Research and Therapy* 2011;**29**:158-69.

Billings 2008 {published data only}

* Billings D, Cook R, Hendrickson A, Dove D. A web-based approach to managing stress and mood disorders in the workforce. *Journal of Occupational and Environmental Medicine* 2008;**50**(8):960-8.

Boettcher 2012 {published data only}

Boettcher J, Berger T, Renneberg B. Internet-based attention training for social anxiety: a randomized controlled trial. *Cognitive Therapy Research* 2012;**36**:522-36.

Bornas 2001 {published data only}

* Bornas X, Tortella-Feliu M, Llabres J, Fullana MA. Computer-assisted exposure treatment for flight phobia: a controlled study. *Psychotherapy Research* 2001;**11**(3):259-73.

Botella 1999 {published data only}

* Botella C, Garcia-Palacios A. The possibility of reducing therapist contact and total length of therapy in the treatment of panic disorder. *Behavioural and Cognitive Psychotherapy* 1999;**27**(3):231-47.

Carpenter 1994 {published data only}

* Carpenter DJ, Gatchel RJ, Hasegawa T. Effectiveness of a videotaped behavioral intervention for dental anxiety: the role of gender and the need for information. *Behavioral Medicine* 1994;**20**(3):123-32.

Chiauzzi 2008 {published data only}

* Chiauzzi E, Brevard J, Thurn C, Decembrele S, Lord S. MyStudentBody-Stress: an online stress management intervention for college students. *Journal of Health Communication* 2008;**13**(6):555-72.

Coldwell 1998 {published data only}

* Coldwell SE, Getz T, Milgrom P, Prall CW, Spadafora A, Ramsay DS. CARL: a LabView 3 computer program for conducting exposure therapy for the treatment of dental injection fear. *Behaviour Research and Therapy* 1998;**36**(4):429-41.

Cronin 1994 {published data only}

* Cronin MW, Grice GL, Olsen RK. The effects of interactive video instruction in coping with speech fright. *Communication Education* 1994;**43**(1):42-53.

Crowder 1970 {published data only}

* Crowder JE, Thornton DW. Effects of systematic desensitization, programmed fantasy and bibliotherapy on a specific fear. *Behaviour Research and Therapy* 1970;**8**(1):35-41.

Currie 2010 {published data only}

* Currie SL, McGrath PJ, Day V. Development and usability of an online CBT program for symptoms of moderate depression, anxiety, and stress in post-secondary students. *Computers in Human Behavior* 2010; Vol. 26, issue 6:1419-26.

Dannon 2002 {published data only}

- * Dannon PN, Iancu I, Grunhaus L. Psychoeducation in panic disorder patients: the effect of a self-information booklet, a randomized, masked-rater study. *Depression and anxiety* 2002;**16**(2):71-6.

den Boer 2007 {published data only}

- * den Boer PC, Wiersma D, Ten Vaarwerk I, Span MM, Stant AD, Van den Bosch RJ. Cognitive self-therapy for chronic depression and anxiety: a multi-centre randomized controlled study. *Psychological Medicine* 2007;**37**(3):329-39.

Devine 1973 {published data only}

- * Devine DA, Fernald PS. Outcome effects of receiving a preferred, randomly assigned, or nonpreferred therapy. *Journal of Consulting and Clinical Psychology* 1973;**41**(1):104-7.

Dolezal-Wood 1998 {published data only}

- * Dolezal-Wood S, Belar CD, Snibbe J. A comparison of computer-assisted psychotherapy and cognitive-behavioral therapy in groups. *Journal of Clinical Psychology in Medical Settings* 1998;**5**(1):103-15.

Donner 1969 {published data only}

- Donner L. Automated group desensitization: a follow-up report. *Behaviour Research and Therapy* 1970 Aug;**8**(3):241-7.
- * Donner L, Guernsey BG. Automated group desensitization for test anxiety. *Behaviour Research and Therapy* 1969;**7**(1):1-13.

Furmark 2009 {published data only}

- * Furmark T, Carlbring P, Hedman E, Sonnenstein A, Clevberger P, Bohman B, et al. Guided and unguided self-help for social anxiety disorder: randomised controlled trial. *British Journal of Psychiatry* 2009;**195**(5):440-7.

Gatchel 1986 {published data only}

- * Gatchel RJ. Impact of a videotaped dental fear-reduction program on people who avoid dental treatment. *Journal of the American Dental Association* 1986;**112**(2):218-21.

Gross 1979 {published data only}

- * Gross AM, Brigham TA. A comparison of self-delivered consequences and desensitization in the treatment of fear of rats. *Journal of Clinical Psychology* 1979;**35**(2):384-90.

Grossman 1991 {published data only}

- * Grossman KS, McNamara J, Dudley K. Treatment effectiveness of a self-help manual for dating anxiety. *Journal of College Student Psychotherapy* 1991;**6**(1):85-106.

Gruber 2001 {published data only}

- * Gruber K, Moran PJ, Roth WT, Taylor C. Computer-assisted cognitive behavioral group therapy for social phobia. *Behavior Therapy* 2001;**32**(1):155-65.

Halpern 1995 {published data only}

- * Halpern RF. Mastery through self-efficacy. University of Detroit Mercy, 1995.

Hamberger 1986 {published data only}

- * Hamberger LK, Schuldt WJ. Live and taped relaxation instructions: effects of procedural variables. *Applied Psychophysiology and Biofeedback* 1986;**11**(1):31-46.

Hasson 2005 {published data only}

- * Hasson D, Anderberg UM, Theorell T, Arnetz BB. Psychophysiological effects of a web-based stress management system: a prospective, randomized controlled intervention study of IT and media workers. *BMC Public Health* 2005;**25**(5):78.

Hasson 2010 {published data only}

- * Hasson H, Brown C, Hasson D. Factors associated with high use of a workplace web-based stress management program in a randomized controlled intervention study. *Health Education Research* 2010;**25**(4):596-607.

Hazen 2009 {published data only}

- * Hazen RA, Vasey MW, Schmidt NB. Attentional retraining: a randomized clinical trial for pathological worry. *Journal of Psychiatric Research* 2009;**43**(6):627-33.

Heatherton 2004 {published data only}

- * Heatherton J, Matheson A, Robson M. Recruitment by GPs during consultations in a primary care randomized controlled trial comparing computerized psychological therapy with clinical psychology and routine GP care: problems and possible solutions. *Primary Health Care Research and Development* 2004;**5**:5-10.

Hecker 2004 {published data only}

- * Hecker JE, Losee MC, Roberson-Nay R, Maki K. Mastery of your anxiety and panic and brief therapist contact in the treatment of panic disorder. *Journal of Anxiety Disorders* 2004;**18**(2):111-26.

Hetherton 2004 {published data only}

- * Hetherton J, Matheson A, Robson M. Recruitment by GPs during consultations in a primary care randomized controlled trial comparing computerized psychological therapy with clinical psychology and routine GP care: problems and possible solutions. *Primary Health Care Research and Development* 2004;**5**(1):5-10.

Hickie 2010 {published data only}

- * Hickie IB, Davenport TA, Luscombe GM, Moore M, Griffiths KM, Christensen H. Practitioner-supported delivery of internet-based cognitive behaviour therapy: evaluation of the feasibility of conducting a cluster randomised trial. *Medical Journal of Australia* 2010;**192**:S31-5.

Hiebert 1983 {published data only}

- * Hiebert B, Cardinal J, Dumka L, Marx RW. Self-instructed relaxation: a therapeutic alternative. *Biofeedback & Self Regulation* 1983;**8**(4):601-17.

Hoek 2012 {published data only}

- * Hoek W, Schuurmans J, Koot HM, Cuijpers P. Effects of Internet-based guided self-help problem-solving therapy for adolescents with depression and anxiety: a randomized controlled trial. *PLoS One* 2012;**7**(8):e43485.

Hogan 1968 {published data only}

- * Hogan RA, Kirchner JH. Implosive, eclectic verbal and bibliotherapy in the treatment of fears of snakes. *Behaviour Research and Therapy* 1968 May;**6**(2):167-71.

Hopf 1995 {published data only}

- * Hopf T, Ayres J, Ayres F, Baker B. Does self-help material work? Testing a manual designed to help trainers construct public speaking apprehension reduction workshops. *Communication Research Reports* 1995;**12**(1):34-8.

Israel 1977 {published data only}

- * Israel E, Beiman I. Live versus recorded relaxation training: a controlled investigation. *Behavior Therapy* 1977;**8**(2):251-4.

Jannoun 1982 {published data only}

- * Jannoun L, Oppenheimer C, Gelder M. A self-help treatment program for anxiety state patients. *Behavior Therapy* 1982;**13**(1):103-11.

Jarmon 1972 {published data only}

- * Jarmon DG. Differential effectiveness of rational-emotive therapy, bibliotherapy, and attention-placebo in the treatment of speech anxiety (PhD). Southern Illinois University, 1972.

Jeffcoat 2012 {published data only}

- Jeffcoat T, Hayes SC. A randomized trial of ACT bibliotherapy on the mental health of K-12 teachers and staff. *Behaviour Research and Therapy* 2012;**50**:571-9.

Johansson 2012 {published data only}

- Johansson R, Hesser H, Ljótsson B, Frederick RJ, Andersson G. Transdiagnostic, affect-focused, psychodynamic, guided self-help for depression and anxiety through the internet: study protocol for a randomised controlled trial. *BMJ Open* 2012;**2**:e002167.

Jones 2002b {published data only}

- * Jones R, White J, Kamarzaman Z, Naven L, Morton R, Marriott C, et al. Cognitive behavioural computer therapy for anxiety: difficulties in carrying out a randomised trial and lessons learned. (Cited in Kaltenthaler E, Brazier J, De Nigris E, Tumor I, Ferriter M, Beverley C, et al Computerised cognitive behaviour therapy for depression and anxiety update: a systematic review and economic evaluation. *Health Technology Assessment* 2006;10(33):1-186) 2002.

Kahn 1968 {published data only}

- * Kahn M, Baker B. Desensitization with minimal therapist contact. *Journal of Abnormal Psychology* 1968;**73**(3):198-200.

Kassinove 1980 {published data only}

- * Kassinove H, Miller N, Kalin M. Effects of pretreatment with rational emotive bibliotherapy and rational emotive audiotherapy on clients waiting at community mental health center. *Psychological Reports* 1980;**46**(3):851-7.

Kenwright 2005 {published data only}

- * Kenwright M, Marks I, Graham C, Franses A, Mataix-Cols D. Brief scheduled phone support from a clinician to enhance computer-aided self-help for obsessive-compulsive disorder:

randomized controlled trial. *Journal of Clinical Psychology* 2005;**61**(12):1499-508.

Khanna 2010 {published data only}

- * Khanna MS, Kendall PC. Computer-assisted cognitive behavioral therapy for child anxiety: results of a randomized clinical trial. *Journal of Consulting and Clinical Psychology* 2010;**78**(5):737-45.

Kiely 1984 {published data only}

- * Kiely BG. Pilot evaluation of a stress self-help/educational package, as used in a primary care setting (MSc). University of Birmingham, 1984.

Kiely 1986 {published data only}

- * Kiely BG, McPherson IG. Stress self-help packages in primary care: a controlled trial evaluation. *The Journal of the Royal College of General Practitioners* 1986;**36**(288):307-9.

Kilfedder 2010 {published data only}

- * Kilfedder C, Power K, Karatzias T, McCafferty A, Niven K, Chouliara Z, et al. A randomized trial of face-to-face counselling versus telephone counselling versus bibliotherapy for occupational stress. *Psychology and Psychotherapy* 2010;**83**(3):223-42.

Kirsch 1979 {published data only}

- * Kirsch I, Henry D. Self-desensitization and meditation in the reduction of public speaking anxiety. *Journal of Consulting and Clinical Psychology* 1979;**47**(3):536-41.

Klein 2009 {published data only}

- * Klein B, Austin D, Pier C, Kiropoulos L, Shandley K, Mitchell J, et al. Internet-based treatment for panic disorder: does frequency of therapist contact make a difference?. *Cognitive Behaviour Therapy* 2009;**38**(2):100-13.

Kohutsek 1983 {published data only}

- * Kohutsek K. Bibliotherapy within a correctional setting. *Journal of Clinical Psychology* 1983;**39**:920-4.

Kupshik 1999 {published data only}

- * Kupshik GA, Fisher CR. Assisted bibliotherapy: effective, efficient treatment for moderate anxiety problems. *British Journal of General Practice* 1999;**49**(438):47-8.

Linder 1971 {published data only}

- * Linder LH, McGlynn F. Experimental desensitization of mouse-avoidance following two schedules of semi-automated relaxation training. *Behaviour Research and Therapy* 1971;**9**(2):131-6.

Lucock 2008 {published data only}

- * Lucock MP, Padgett K, Noble R, Westley A, Atha C, Horsefield C, et al. Controlled clinical trial of a self-help for anxiety intervention for patients waiting for psychological therapy. *Behavioural and Cognitive Psychotherapy* 2008;**36**(5):541-51.

Lucock 2011 {published data only}

- * Lucock M, Kirby R, Wainwright N. A pragmatic randomized controlled trial of a guided self-help intervention versus a waiting list control in a routine primary care mental health service. *British Journal of Clinical Psychiatry* 2011;**50**(3):298-309.

Lynch 1997 {published data only}

- * Lynch F. Progressive relaxation and guided imagery as audio cassette interventions to reduce teacher stress. United States International University, 1997.

Mann 1972 {published data only}

- * Mann J. Vicarious desensitization of test anxiety through observation of videotaped treatment. *Journal of Counseling Psychology* 1972;**19**(1):1-7.

Marchand 2007 {published data only}

- * Marchand A, Todorov C, Borgeat F, Pelland ME. Effectiveness of a brief cognitive behavioural therapy for panic disorder with agoraphobia and the impact of partner involvement. *Behavioural and Cognitive Psychotherapy* 2007; Vol. 35, issue 5.

Marshall 1976 {published data only}

- * Marshall W, Presse L, Andrews W. A self-administered program for public speaking anxiety. *Behaviour Research and Therapy* 1976;**14**(1):33-9.

Mataix-Cols 2006 {published data only}

- * Mataix-Cols D, Cameron R, Gega L, Kenwright M, Marks I. Effect of referral source on outcome with cognitive-behaviour therapy self-help. *Comprehensive Psychiatry* 2006;**47**(4):241-5.

Matthews 2010 {published data only}

- * Matthews AJ. Online exposure for spider fear: treatment completion and habituation outcomes. *Behaviour Change* 2010; Vol. 27, issue 4.

McGlynn 1976 {published data only}

- * McGlynn FD, Walls R. Credibility ratings for desensitization and pseudotherapy among moderately and mildly snake-avoidant college students. *Journal of Clinical Psychology* 1976; Vol. 32, issue 1:140-5.

Moritz 2011 {published data only}

- * Moritz S, Rufer M. Movement decoupling: a self-help intervention for the treatment of trichotillomania. *Journal of Behavior Therapy and Experimental Psychiatry* 2011;**42**(1):74-80.

Muroff 2012 {published data only}

- * Muroff J, Steketee G, Bratiliotis C, Ross A. Group cognitive and behavioral therapy and bibliotherapy for hoarding: a pilot trial. *Depression and Anxiety* 2012;**29**(7):597-604.

Newman 1997 {published data only}

- * Newman M, Kenardy J, Herman S, Taylor C. Comparison of palmtop-computer-assisted brief cognitive-behavioral treatment to cognitive-behavioral treatment for panic disorder. *Journal of Consulting and Clinical Psychology* 1997;**65**(1):178-83.

Ormrod 2010 {published data only}

- * Ormrod JA. Computerised cognitive behavioural therapy in an adult mental health service: a pilot study of outcomes and alliance. *Cognitive Behaviour Therapy* 2010; Vol. 39, issue 3.

Pier 2008 {published data only}

- * Pier C, Austin DW, Klein B, Mitchell J, Schattner P, Ciechomski L, et al. A controlled trial of internet-based cognitive-behavioural therapy for panic disorder with face-to-face support from a general practitioner or email support from a psychologist. *Mental Health in Family Medicine* 2008;**5**(1):29-39.

Pittaway 2009 {published data only}

- * Pittaway S, Cupitt C, Palmer D, Arowobusoye N, Milne R, Holtum S, et al. Comparative, clinical feasibility study of three tools for delivery of cognitive behavioural therapy for mild to moderate depression and anxiety provided on a self-help basis. *Mental Health in Family Medicine* 2009;**6**(3):145-54.

Pless 2010 {published data only}

- * Pless A. Treatment of test anxiety: a computerized approach. Central Michigan University, 2010.

Pleva 2007 {published data only}

- * Pleva J, Wade TD. Guided self-help versus pure self-help for perfectionism: a randomised controlled trial. *Behavioural Research and Therapy* 2007;**45**(5):849-61.

Powell 2012 {published data only}

- * Powell J, Hamborg T, Stallard N, Burls A, McSorley J, Bennett K, et al. Effectiveness of a Web-based cognitive-behavioral tool to improve mental well-being in the general population: randomized controlled trial. *Journal of Medical Internet Research* 2013;**15**(1):e2.

Rahe 2002 {published data only}

- * Rahe RH, Taylor CB, Tolles RL, Newhall LM, Veach TL, Bryson S. A novel stress and coping workplace program reduces illness and healthcare utilization. *Psychosomatic Medicine* 2002;**64**(2):278-86.

Register 1991 {published data only}

- * Register AC, Beckham JC, May JG, Gustafson DJ. Stress inoculation bibliotherapy in the treatment of test anxiety. *Journal of Counseling Psychology* 1991;**38**(2):115-9.

Rishel 1993 {published data only}

- * Rishel DJD. A comparison of videotaped behavioral interventions in reducing anxiety and increasing patient satisfaction. University of Texas Southwestern Medical Center, 1993.

Ruwaard 2007 {published data only}

- * Ruwaard J, Lange A, Bouwman M, Broeksteeg J, Schrieken B. E-mailed standardized cognitive behavioural treatment of work-related stress: a randomized controlled trial. *Cognitive Behaviour Therapy* 2007;**36**(3):179-92.

Saul 2007 {published data only}

- * Saul JE. A comparison of internet-based versus face-to-face cognitive behavioral therapy for snake phobia. Hofstra University, 2007.

Schelver 1983 {published data only}

- * Schelver SR, Gutsch KU. The effects of self-administered cognitive therapy on social-evaluative anxiety. *Journal of Clinical Psychology* 1983;**39**(5):658-66.

Schneider 1995 {published data only}

- * Schneider SJ, Schwartz MD, Fast J. Computerized, telephone-based health promotion: II. Stress management program. *Computers in Human Behavior* 1995;**11**:205-14.

Serrano 2009 {published data only}

- * Serrano-Aguilar P, Ramallo-Farina Y, Trujillo-Martin MM, Munoz-Navarro SR, Perestelo-Perez L, de las Cuevas-Castresana C. The relationship among mental health status (GHQ-12), health related quality of life (EQ-5D) and health-state utilities in a general population. *Epidemiologia e Psichiatria Sociale* 2009;**18**:229-39.

Smith 1987 {published data only}

- * Smith JJ. The effectiveness of a computerized self-help stress coping program with adult males. *Computers in Human Services* 1987;**2**(1-2):37-49.

Sportel 2013 {published data only}

- Sportel BE, de Hullu E, de Jong PJ, Nauta MH. Cognitive bias modification versus CBT in reducing adolescent social anxiety: a randomized controlled trial. *PLoS One* 2013;**8**(5):e64355.

Tillfors 2008 {published data only}

- * Tillfors M, Carlbring P, Furmark T, Lewenhaupt S, Spak M, Eriksson A, et al. Treating university students with social phobia and public speaking fears: Internet delivered self-help with or without live group exposure sessions. *Depression and Anxiety* 2008;**25**(8):708-17.

Titov 2009b {published data only}

- * Titov N, Andrews G, Choi I, Schwencke G, Johnston L. Randomized controlled trial of web-based treatment of social phobia without clinician guidance. *Australian and New Zealand Journal of Psychiatry* 2009;**43**(10):913-9.

Titov 2009c {published data only}

- * Titov N, Andrews G, Schwencke G, Solley K, Johnston L, Robinson E. An RCT comparing effect of two types of support on severity of symptoms for people completing Internet-based cognitive behaviour therapy for social phobia. *Australian and New Zealand Journal of Psychiatry* 2009;**43**(10):920-6.

van Straten 2008 {published data only}

- * van Straten A, Cuijpers P, Smits N. Effectiveness of a web-based self-help intervention for symptoms of depression, anxiety, and stress: randomized controlled trial. *Journal of Medical Internet Research* 2008;**10**(1):e7.

Vestre 1989 {published data only}

- * Vestre ND, Judge TJ. Evaluation of self-administered rational emotive therapy programs for interpersonal anxiety. *Journal of Rational-Emotive and Cognitive Behavior Therapy* 1989;**7**(3):141-54.

Weissberg 1977 {published data only}

- * Weissberg M. A comparison of direct and vicarious treatments of speech anxiety: desensitization, desensitization with coping imagery, and cognitive modification. *Behavior Therapy* 1977; Vol. 8, issue 4:606-20.

Wolitzky-Taylor 2010 {published data only}

- * Wolitzky-Taylor KB, Telch MJ. Efficacy of self-administered treatments for pathological academic worry: a randomized controlled trial. *Behaviour Research and Therapy* 2010;**48**(9):840-50.

Wright 1997 {published and unpublished data}

- Wright J, Clum G, Roodman A, Febbraro G. A bibliotherapy approach to relapse prevention in individuals with panic attacks. *Journal of Anxiety Disorders* 2000;**14**(5):483-99.

- * Wright JH. An investigation of a minimal-contact bibliotherapy approach to relapse prevention for individuals treated for panic attacks (PhD). Virginia Polytechnic Institute and State University, Virginia Polytechnic Inst And State U., US, 1997.

Zou 2012 {published data only}

- * Zou JB, Dear BF, Titov N, Lorian CN, Johnston L, Spence J, et al. Brief internet-delivered cognitive behavioral therapy for anxiety in older adults: a feasibility trial. *Journal of Anxiety Disorders* 2012;**26**(6):650-5.

References to studies awaiting assessment

ACTRN12610001058066 {published data only}

- ACTRN12610001058066. A randomized controlled trial of the effects of different levels of graded support on symptoms of anxiety and depression in adults using the Internet-based Wellbeing Course. www.anzctr.org.au/ACTRN12610001058066.aspx (accessed 26 February 2013).

ACTRN12611000116921 {published data only}

- ACTRN12611000116921. A randomised controlled trial of a clinician-guided education program for obsessive compulsive disorder comparing the efficacy of administration via the Internet with administration via Bibliotherapy. www.anzctr.org.au/ACTRN12611000116921.aspx (accessed 26 February 2013).

ACTRN12611000625976 {published data only}

- ACTRN12611000625976. A randomized controlled trial of Internet based education for social phobia, panic disorder (with or without agoraphobia), and generalised anxiety disorder, comparing immediate education versus delayed education groups on severity of symptoms of anxiety. www.anzctr.org.au/ACTRN12611000625976.aspx (accessed 26 February 2013).

ACTRN12611000626965 {published data only}

ACTRN12611000626965. Internet versus face to face cognitive behaviour therapy for social phobia. www.anzctr.org.au/ACTRN12611000626965.aspx (accessed 26 February 2013).

Andersson 2011 {published data only}

* Andersson G. Psychodynamic internet treatment versus cognitive behavioral therapy (CBT) for generalized anxiety disorder (NCT01312116), 2011. <http://clinicaltrials.gov/ct2/show/NCT01312116> (accessed 1 May 2013).

Bohus 2010 {published data only}

* Bohus M. Evaluation of the efficacy, mechanisms of change and efficiency of a stepped-care program with a computer-based self-help module and minimal therapist contact in comparison to a standard cognitive therapy for patients with social phobia, 2010. <http://clinicaltrials.gov/ct2/show/NCT00434759> (accessed 1 May 2013).

Christensen 2010 {published data only}

Christensen H, Guastella A, Mackinnon A, Griffiths KM, Eagleson C, Batterham P, et al. Protocol for a randomised controlled trial investigating the effectiveness of an online e-health application compared to attention placebo or sertraline in the treatment of generalised anxiety disorder. *Trials [electronic resource]* 2010;**11**:48.

Donker 2009 {published data only}

* Donker T, van Straten A, Riper H, Marks I, Andersson G, Cuijpers P. Implementation of Internet-based preventive interventions for depression and anxiety: role of support? The design of a randomized controlled trial. *Trials* 2009;**10**:59.

DRKS00004612 {published data only}

DRKS00004612. Internet-based treatment for obsessive-compulsive disorder—acceptance and effectiveness [Internetgestützte Behandlung von Zwangsstörungen—Akzeptanz und Wirksamkeit]. [German Clinical Trials Register] www.drks.de/DRKS00004612 (accessed 26 February 2013).

Greist 2008 {published data only}

Greist JH. Cost-effectiveness of adding Web-based cognitive-behavioral therapy (CBT) to Luvox CR for obsessive compulsive disorder (OCD). <http://clinicaltrials.gov/show/NCT00743834> (accessed 1 May 2013).

Griffiths 2010a {published data only}

* Griffiths K. The wellbeing project: internet interventions to improve mental health and wellbeing (ISRCTN65657330). <http://www.controlled-trials.com/ISRCTN65657330/> (accessed 1 May 2013).

Griffiths KM, Crisp D, Christensen H, Mackinnon AJ, Bennett K. The ANU WellBeing study: a protocol for a quasi-factorial randomised controlled trial of the effectiveness of an Internet support group and an automated Internet intervention for depression. *BMC Psychiatry* 2010;**10**:20.

Griffiths KM, Mackinnon AJ, Crisp DA, Christensen H, Bennett K, Farrer L. The effectiveness of an online support group for

members of the community with depression: a randomised controlled trial. *PLoS ONE* 2012;**7**(12):e53244.

ISRCTN36376369 {published data only}

ISRCTN36376369. Stepped care for panic disorder [Stepped care voor paniekstoornis]. <http://www.controlled-trials.com/ISRCTN36376369> (accessed 26 February 2013).

Kerkhof 2005 {published data only}

* Kerkhof AJFM. Stop worrying: the effectivity of a self-help brochure for excessive worrying, with or without telephonic support (ISRCTN99166327). <http://www.controlled-trials.com/ISRCTN99166327> (accessed 1 May 2013).

Kool 2007 {published data only}

* Kool L. Stepped care in depression and anxiety: from primary to secondary care (ISRCTN17831610). <http://www.controlled-trials.com/ISRCTN17831610/> (accessed 1 May 2013).

Lacasse 2010 {published data only}

Lacasse JR. Mindfulness and lifestyle interventions for depression and anxiety: a pilot study. <http://clinicaltrials.gov/ct2/show/NCT01230047> (accessed 1 May 2013) <http://clinicaltrials.gov/ct2/show/NCT01230047?term=NCT01230047&rank=1>.

Martinez 2008 {published data only}

* Martinez R. A pilot research project into the use of online resources for low mood and anxiety in the voluntary sector (ISRCTN71327173), 2008. <http://www.controlled-trials.com/ISRCTN71327173/> (accessed 1 May 2013).

Moore 1998 {published data only (unpublished sought but not used)}

* Moore A. An investigation of the use of self-help booklets for clients on a waiting list: pilot study. National Research Register 1998.

Muntingh 2009 {published data only}

* Muntingh ADT, van der Feltz-Cornelis CM, van Marwijk HW, Spinhoven P, Assendelft WJ, Hakkaart-van Roijen L, et al. Collaborative stepped care for anxiety disorders in primary care: aims and design of a randomized controlled trial. *BMC Health Services Research* 2009;**9**:159.

NTR1260 {published data only}

NTR1260. Phobias under control. Internet-based guided self-help for treatment of social, agoraphobia, or specific phobia(s): a pilot trial. [Netherlands Trial Register] www.trialregister.nl/trialreg/admin/rctview.asp?TC=1260 (accessed 26 February 2013).

NTR2573 {published data only}

NTR2573. A better picture of your Self: strengthening your self-image in order to counter feelings of anxiety and depression: a study on the effectiveness of a guided self-help intervention. [Netherlands Trial Register] <http://www.trialregister.nl/trialreg/admin/rctview.asp?TC=2573> (accessed 26 February 2013).

Schmidt 2008 {published data only}

- * Schmidt B. Interactive computer treatment for panic disorder (NCT00063375), 2008. <http://clinicaltrials.gov/ct2/show/NCT00063375> (accessed 1 May 2013).

Spooner 2011 {published data only}

- * Spooner D. Cognitive-behavioural self-help for depressed and anxious adults: a randomised controlled trial comparing computer based interactive self-help versus a self-help treatment manual. ISRCTN32111346, 2011. <http://www.controlled-trials.com/ISRCTN32111346/> (accessed 1 May 2013).

Titov 2011a {published data only}

- * Titov N. The Wellbeing Program: a randomized controlled trial of internet based treatment of anxiety and depression (ACTRN12610000247077), 2011. http://www.anzctr.org.au/trial_view.aspx?id=335292 (accessed 1 May 2013).

van Ballegooijen 2011 {published data only}

- * van Ballegooijen W. Don't Panic Online: a randomised trial to the effects of an internet based self-help course for reducing panic symptoms. <http://www.trialregister.nl/trialreg/admin/rctview.asp?TC=1639> (accessed 1 May 2013).
- * van Ballegooijen W. The effects of an Internet based self-help course for reducing panic symptoms — Don't Panic Online: study protocol for a randomised controlled trial. *Trials* 2011;**11**(12):75.

References to ongoing studies

ACTRN12610001081000 {published data only}

ACTRN12610001081000. A randomised control trial of guided self help cognitive behavioural therapy for clinical perfectionism versus a guided self help cognitive behavioural therapy for anxiety and depression. www.anzctr.org.au/ACTRN12610001081000.aspx (accessed 26 February 2013).

ACTRN12611000321943 {published data only}

ACTRN12611000321943. Internet-based cognitive-behavioural therapy for obsessive-compulsive disorder. www.anzctr.org.au/ACTRN12611000321943.aspx (accessed 26 February 2013).

ACTRN12611001052921 {published data only}

ACTRN12611001052921. Development, evaluation and implementation of an online metacognitive intervention programme for obsessive-compulsive disorder. www.anzctr.org.au/ACTRN12611001052921.aspx (accessed 26 February 2013).

ACTRN12611001055998 {published data only}

ACTRN12611001055998. The Wellbeing6 Program for Anxiety and Depression: a randomised controlled trial. www.anzctr.org.au/ACTRN12611001055998.aspx (accessed 26 February 2013).

ACTRN12612000430831 {published data only}

ACTRN12612000430831. Is internet-delivered treatment specifically for symptoms of social phobia as effective as internet-delivered treatment for symptoms of both anxiety and depression, and are self-guided versions of these as

effective as therapist-guided versions?. www.anzctr.org.au/ACTRN12612000430831.aspx (accessed 26 February 2013).

ACTRN12612000431820 {published data only}

ACTRN12612000431820. Is internet-delivered treatment specifically for symptoms of panic disorder as effective as internet-delivered treatment for symptoms of both anxiety and depression, and are self-guided versions of these as effective as therapist-guided versions?. www.anzctr.org.au/ACTRN12612000431820.aspx (accessed 26 February 2013).

ACTRN12612000433808 {published data only}

ACTRN12612000433808. Is internet-delivered treatment specifically for symptoms of generalised anxiety disorder as effective as internet-delivered treatment for symptoms of both anxiety and depression, and are self-guided versions of these as effective as therapist-guided versions?. www.anzctr.org.au/ACTRN12612000433808.aspx (accessed 26 February 2013).

ACTRN12612001073897 {published data only}

ACTRN12612001073897. The Obsessive Compulsive Disorder (OCD) program: a randomised controlled trial comparing clinician-assisted Internet based treatment for OCD vs. a waitlist control condition on severity of symptoms of OCD. www.anzctr.org.au/ACTRN12612001073897.aspx (accessed 26 February 2013).

ACTRN12612001306808 {published data only}

ACTRN12612001306808. The Obsessive Compulsive Disorder (OCD) program: a randomised controlled trial of online versus face-to-face cognitive behavioural therapy (CBT). www.anzctr.org.au/ACTRN12612001306808.aspx (accessed 26 February 2013).

Carlbring 2012a {published data only}

Per Carlbring. Internet-based treatment of generalized anxiety disorder. <http://clinicaltrials.gov/ct2/show/NCT01570374> (accessed 1 May 2013).

Carlbring 2012b {unpublished data only}

Per Carlbring. An Internet-administered, mindfulness training program for the treatment of anxiety. <http://clinicaltrials.gov/ct2/show/NCT01577290> (accessed 1 May 2013).

Clark 2012 {published data only}

- * Clark DM. A randomised controlled trial of internet-based cognitive therapy (iCT) and standard cognitive therapy (CT) for social anxiety disorder. <http://www.controlled-trials.com/ISRCTN95458747> (accessed 1 May 2013). [DOI: [10.1186/ISRCTN95458747](https://doi.org/10.1186/ISRCTN95458747)]

Havik 2012 {published data only}

- * Havik OE. Treatment of social phobia and panic disorder in adults (TSPPA). <http://clinicaltrials.gov/ct2/show/NCT00619138> (accessed 1 May 2013).

Hedman 2012 {unpublished data only}

Hedman E. Cognitive behavioral therapy, guided self help versus individual therapy. <http://clinicaltrials.gov/ct2/show/NCT01667822> (accessed 1 May 2013).

Hedman 2013 {unpublished data only}

Erik Hedman. Internet Treatment for Health Anxiety (HA-X). <http://clinicaltrials.gov/ct2/show/NCT01673035>.

ISRCTN73535163 {published data only}

Health Technology Assessment. Obsessive compulsive treatment efficacy trial (OCTET) (Project record). www.hta.ac.uk/protocols/200900810001.pdf (accessed 12 June 2013).

ISRCTN73535163. Obsessive compulsive treatment efficacy trial (OCTET). <http://www.controlled-trials.com/ISRCTN73535163/> (accessed 26 February 2013).

Kok 2012 {published data only}

* Kok RN, van Straten A, Beekman A, Bosmans J, de Neef M, Cuijpers P. Effectiveness and cost-effectiveness of web-based treatment for phobic outpatients on a waiting list for psychotherapy: protocol of a randomised controlled trial. *BMC Psychiatry* 2012;**12**:131.

Nordgren 2012 {published data only}

Carlbring P, Andersson G, Nordgren LB. Internet-administrated treatment of anxiety disorders (NOVA II) (NCT01390168). <http://clinicaltrials.gov/ct2/show/NCT01390168> (accessed 1 May 2013).

* Nordgren LB, Andersson G, Kadowaki Å, & Carlbring P. Tailored internet-administered treatment of anxiety disorders for primary care patients: study protocol for a randomised controlled trial. *Trials* 2012;**13**:16.

NTR1985 {published data only}

NTR1985. Acceptance and commitment therapy as a self-help intervention. [Netherlands Trial Register] <http://www.trialregister.nl/trialreg/admin/rctview.asp?TC=1985> (accessed 26 February 2013).

Proudfoot 2010 {published data only}

* Proudfoot J. Randomised controlled trial to evaluate the effect on mental health of a self-monitoring and self-management program delivered by mobile phone and computer for people with mild to moderate depression, anxiety or stress compared against an attention control program, and a waitlist control (ACTRN12610000625077), 2010. http://www.anzctr.org.au/trial_view.aspx?id=335772 (accessed 1 May 2013).

Rollman 2012 {published data only}

* Rollman B. Online treatments for mood and anxiety disorders in primary care. <http://clinicaltrials.gov/ct2/show/NCT01482806> (accessed 1 May 2013).

Tulbure 2012 {published data only}

* Tulbure BT, Månsson KN, Andersson G. Internet treatment for social anxiety disorder in Romania: study protocol for a randomized controlled trial. *Trials* 2012;**13**:202.

Additional references
Andrews 2010

Andrews G, Cuijpers P, Craske M, Andrews G. Computer therapy for the anxiety and depressive disorders is effective, acceptable and practical health care: a meta-analysis. *PLoS ONE* 2010; Vol. 5, issue 10.

APA 1994

American Psychiatric Association. Diagnostic and Statistical Manual of Mental Disorders (DSM-IV). 4th Edition. Washington, DC: American Psychiatric Association, 1994.

Beck 1988

Beck AT, Epstein N, Brown G, Steer RA. An inventory for measuring clinical anxiety: psychometric properties. *Journal of Consulting and Clinical Psychology* 1988;**56**(6):893-7.

Bennett-Levy 2004

Bennett-Levy J, Butler G, Fennell M, Hackmann A, Mueller M, Westbrook D. Oxford Guide to Behavioural Experiments in Cognitive Therapy (Cognitive Behaviour Therapy: Science and Practice). Oxford: Oxford University Press, 2004.

Bessell 2002

Bessell T, McDonald S, Silagy C, Anderson J, Hiller J, Sansom L. Do internet interventions for consumers cause more harm than good? A systematic review. *Health Expectations* 2002;**5**(1):28-37.

Bloom 2011

Bloom DE, Cafiero ET, Jané-Llopis E, Abrahams-Gessel S, Bloom LR, Fathima S, et al. The Global Economic Burden of Noncommunicable Diseases. Geneva: World Economic Forum, 2011.

Borenstein 2005 [Computer program]

Borenstein M, Hedges L, Higgins J, Rothstein H. Comprehensive Meta-analysis Version 2. Englewood NJ: Biostat, 2005.

Bower 2001

Bower P, Richards D, Lovell K. The clinical and cost-effectiveness of self-help treatments for anxiety and depressive disorders in primary care: a systematic review. *British Journal of General Practice* 2001;**51**(471):838-45.

Bruce 2005

Bruce SE, Yonkers KA, Otto MW, Eisen JL, Weisberg RB, Pagano M, et al. Influence of psychiatric comorbidity on recovery and recurrence in generalized anxiety disorder, social phobia, and panic disorder: a 12-year prospective study. *The American Journal of Psychiatry*. 2005/06/03 2005; Vol. 162, issue 6:1179-87. [0002-953X: (Print)]

Butler 2006

Butler A, Chapman J, Forman E, Beck A. The empirical status of cognitive behavioral therapy. *Clinical Psychology Review* 2006;**26**(1):17-31.

Chambless 1984

Chambless DL, Caputo GC, Bright P, Gallagher R. Assessment of fear of fear in agoraphobics: the Body Sensations Questionnaire

and the Agoraphobic Cognitions Questionnaire. *Journal of Consulting and Clinical Psychology* 1984;**52**(6):1090-7.

Clark 2006

Clark DM, Ehlers A, Hackmann A, McManus F, Fennell M, Grey N, et al. Cognitive therapy versus exposure and applied relaxation in social phobia: a randomized controlled trial. *Journal of Consulting and Clinical Psychology* 2006;**74**(3):568-78.

Clark 2009

Clark DM, Layard R, Smithies R, Richards DA, Suckling R, Wright B. Improving access to psychological therapy: initial evaluation of two UK demonstration sites. *Behaviour Research and Therapy*. 2009/08/04 2009; Vol. 47, issue 11:910-20. [1873-622X: (Electronic)]

Conrad 2007

Conrad A, Roth WT. Muscle relaxation therapy for anxiety disorders: it works but how?. *Journal of Anxiety Disorders* 2007; Vol. 21, issue 3:243-64.

Cramer 2005

Cramer V, Torgersen S, Kringlen E. Quality of life and anxiety disorders: a population study. *The Journal of Nervous and Mental Disease*. 2005/02/25 2005; Vol. 193, issue 3:196-202. [0022-3018: (Print)]

Cuijpers 2009

Cuijpers P, Marks IM, van Straten A, Cavanagh K, Gega L, Andersson G. Computer-aided psychotherapy for anxiety disorders: a meta-analytic review. *Cognitive Behaviour Therapy* 2009; Vol. 38, issue 2:66-82. [1650-6073]

Cuijpers 2010

Cuijpers PD. Is guided self-help as effective as face-to-face psychotherapy for depression and anxiety disorders? A systematic review and meta-analysis of comparative outcome studies. *Psychological Medicine* 2010; Vol. 40, issue 12:1943-57.

Demyttenaere 2004

Demyttenaere K, Bruffaerts R, Posada-Villa J, Gasquet I, Kovess V, Lepine JP, et al. Prevalence, severity, and unmet need for treatment of mental disorders in the World Health Organization world mental health surveys. *JAMA*. 2004/06/03 2004; Vol. 291, issue 21:2581-90. [1538-3598: (Electronic)]

den Boer 2004

den Boer PC, Wiersma D, van den Bosch RJ. Why is self-help neglected in the treatment of emotional disorders? A meta-analysis. *Psychological Medicine* 2004;**34**(6):959-71.

Emmelkamp 1975

Emmelkamp PM, Wessels H. Flooding in imagination vs flooding in vivo: a comparison with agoraphobics. *Behaviour Research and Therapy* 1975; Vol. 13, issue 1:7-15.

Foa 1986

Foa EB, Kozak MJ. Emotional processing of fear: exposure to corrective information. *Psychological Bulletin* 1986; Vol. 99, issue 1:20-35.

Glasgow 1978

Glasgow R, Rosen G. Behavioral bibliotherapy: a review of self-help behavior therapy manuals. *Psychological Bulletin* 1978;**85**(1):1-23.

Gould 1993b

Gould R, Clum G. A meta-analysis of self-help treatment approaches. *Clinical Psychology Review* 1993;**13**(2):169-86.

GRADE 2004

The GRADE Working Group. Grading quality of evidence and strength of recommendations. *BMJ* 2004;**328**:1490-4.

Grant 2013

Grant SP, Mayo-Wilson E, Melendez-Torres GJ, Montgomery P. Reporting quality of social and psychological intervention trials: a systematic review of reporting guidelines and trial publications. *PLoS ONE* 2013;**8**(5):e65442.

Greenberg 1999

Greenberg PE, Sisitsky T, Kessler RC, Finkelstein SN, Berndt ER, Davidson JR, et al. The economic burden of anxiety disorders in the 1990s. *The Journal of Clinical Psychiatry*. 1999/08/24 1999; Vol. 60, issue 7:427-35. [0160-6689: (Print)]

Griffiths 2006

Griffiths KM, Christensen H. Review of randomised controlled trials of internet interventions for mental disorders and related conditions. *Clinical Psychologist* 2006;**10**(1):16-29.

Griffiths 2010b

Griffiths KM, Farrer L, Christensen H. The efficacy of internet interventions for depression and anxiety disorders: a review of randomised controlled trials. *Medical Journal of Australia* 2010;**192**(11 Suppl):S4-11.

Haug 2012

Haug T, Nordgreen T, Ost LG, Havik OE. Self-help treatment of anxiety disorders: a meta-analysis and meta-regression of effects and potential moderators. *Clinical Psychology Review* 2012;**32**:425-45.

Higgins 2011

Higgins J, Greene S (editors). *Cochrane Handbook for Systematic Reviews of Interventions* Version 5.1.0. The Cochrane Collaboration, 2011.

Hilvert-Bruce 2012

Hilvert-Bruce Z, Rossouw PJ, Wong N, Sunderland M, Andrews G. Adherence as a determinant of effectiveness of internet cognitive behavioural therapy for anxiety and depressive disorders. *Behaviour Research and Therapy* 2012;**50**(7-8):463-8.

Hirai 2006

Hirai M, Hirai M. A meta-analytic study of self-help interventions for anxiety problems. *Behavior Therapy* 2006; Vol. 37, issue 2.

Imdad 2010

Imdad A, Herzer K, Mayo-Wilson E, Yakoob MY, Bhutta ZA. Vitamin A supplementation for preventing morbidity and

mortality in children from 6 months to 5 years of age. *Cochrane Database of Systematic Reviews* 2010, Issue 12. [DOI: [10.1002/14651858.CD008524.pub2](https://doi.org/10.1002/14651858.CD008524.pub2)]

Jacobson 1938

Jacobson E. *Progressive Relaxation*. Chicago: University of Chicago Press, 1938.

Kaltenthaler 2002

Kaltenthaler E, Shackley P, Stevens K, Beverley C, Parry G, Chilcott J. A systematic review and economic evaluation of computerised cognitive behaviour therapy for depression and anxiety. *Health Technology Assessment* 2002;**6**(22):1-89.

Kaltenthaler 2008

Kaltenthaler E, Parry G, Beverley C, Ferriter M. Computerised cognitive-behavioural therapy for depression: systematic review. *British Journal of Psychiatry*. 2008/09/02 2008; Vol. 193, issue 3:181-4. [0007-1250: (Print)]

Kapczinski 2003

Kapczinski F, Lima MS, Souza JS, Schmitt R. Antidepressants for generalized anxiety disorder. *Cochrane Database of Systematic Reviews* 2003;**2**:CD003592.

Keeley 2002

Keeley H, Williams C, Shapiro DA. A United Kingdom survey of accredited cognitive behaviour therapists' attitudes towards and use of structured self-help materials. *Behavioural and Cognitive Psychotherapy* 2002; Vol. 30, issue 2:193-203.

Kendall 2011

Kendall T, Cape J, Chan M, Taylor C. Management of generalised anxiety disorder in adults: summary of NICE guidance. *BMJ* 2011;**342**.

Kessler 2005a

Kessler R, Berglund P, Demler O, Jin R, Walters E. Lifetime prevalence and age-of-onset distributions of DSM-IV disorders in the National Comorbidity Survey Replication. *Archives of General Psychiatry* 2005;**62**(6):593-602.

Kessler 2005b

Kessler R, Chiu W, Demler O, Walters E. Prevalence, severity, and comorbidity of 12-month DSM-IV disorders in the National Comorbidity Survey Replication. *Archives of General Psychiatry* 2005;**62**(6):617-27.

Lewis 2012

Lewis C, Pearce J, Bisson JI. Efficacy, cost-effectiveness and acceptability of self-help interventions for anxiety disorders: systematic review. *The British Journal of Psychiatry*. 2012/01/05 2012; Vol. 200, issue 1:15-21. [1472-1465: (Electronic)]

Liebowitz 1987

Liebowitz MR. Social phobia. *Modern Problems of Pharmacopsychiatry* 1987;**22**:141-73.

Marks 1971

Marks I, Boulougouris J, Marset P. Flooding versus desensitization in the treatment of phobic patients: a crossover

study. *British Journal of Psychiatry* 1971; Vol. 119, issue 551:353-75.

Marrs 1995

Marrs R. A meta-analysis of bibliotherapy studies. *American Journal of Community Psychology* 1995;**23**(6):843-70.

Mayo-Wilson 2007

Mayo-Wilson E. Reporting implementation in randomized trials: proposed additions to the consolidated standards of reporting trials statement. *American Journal of Public Health* 2007;**97**(4):630-3.

Menchola 2007

Menchola M, Arkowitz HS, Burke BL. Efficacy of self-administered treatments for depression and anxiety. *Professional Psychology: Research and Practice* 2007; Vol. 38, issue 4:421-9.

Mewton 2012

Mewton L, Wong N, Andrews G. The effectiveness of internet cognitive behavioural therapy for generalized anxiety disorder in clinical practice. *Depression and Anxiety* 2012;**29**(10):843-9.

Mitte 2005a

Mitte K. Meta-analysis of cognitive-behavioral treatments for generalized anxiety disorder: a comparison with pharmacotherapy. *Psychological Bulletin* 2005;**131**(5):785-95.

Mitte 2005b

Mitte K. A meta-analysis of the efficacy of psycho- and pharmacotherapy in panic disorder with and without agoraphobia. *Journal of Affective Disorders* 2005;**88**(1):27-45.

NCCMH 2006

National Collaborating Centre for Mental Health. *Obsessive-compulsive Disorder: Core Interventions in the Treatment of Obsessive-compulsive Disorder and Body Dysmorphic Disorder*. Leicester and London: The British Psychological Society and the Royal College of Psychiatrists, 2006.

Newman 2003

Newman M, Erickson T, Przeworski A. Self-help and minimal-contact therapies for anxiety disorders: is human contact necessary for therapeutic efficacy?. *Journal of Clinical Psychology* 2003;**59**(3):251-74.

Norton 2007

Norton PJ, Price EC. A meta-analytic review of adult cognitive-behavioral treatment outcome across the anxiety disorders. *The Journal of Nervous and Mental Disease* 2007; Vol. 195, issue 6:521-31.

Ormel 1994

Ormel J, VonKorff M, Ustun TB, Pini S, Korten A, Oldehinkel T. Common mental disorders and disability across cultures: results from the WHO Collaborative Study on Psychological Problems in General Health Care. *JAMA*. 1994/12/14 1994; Vol. 272, issue 22:1741-8. [0098-7484: (Print)]

Pilling 2013

Pilling S, Mayo-Wilson E, Mavranouzou I, Kew K, Taylor C, Clark D. Recognition, assessment and treatment of social anxiety disorder: summary of NICE guidance. *BMJ* 2013;346:f2541. [DOI: [10.1136/bmj.f2541](https://doi.org/10.1136/bmj.f2541)]

Przeworski 2006

Przeworski A, Newman M. Efficacy and utility of computer-assisted cognitive behavioural therapy for anxiety disorders. *Clinical Psychologist* 2006;**10**(2):43-53.

Pull 2006

Pull CB. Self-help Internet interventions for mental disorders. *Current Opinion in Psychiatry* 2006;**19**(1):50-3.

Rapaport 2005

Rapaport MH, Clary C, Fayyad R, Endicott J. Quality-of-life impairment in depressive and anxiety disorders. The American Journal of Psychiatry. 2005/06/03 2005; Vol. 162, issue 6:1171-8. [0002-953X: (Print)]

Rapee 2009

Rapee RM, Gaston JE, Abbott MJ. Testing the efficacy of theoretically derived improvements in the treatment of social phobia. *Journal of Consulting and Clinical Psychology*. 2009/03/25 2009; Vol. 77, issue 2:317-27. [1939-2117: (Electronic)]

Reger 2009

Reger MAG. A meta-analysis of the effects of Internet- and computer-based cognitive-behavioral treatments for anxiety. *Journal of Clinical Psychology* 2009; Vol. 65, issue 1:53-75.

Review Manager 2011 [Computer program]

The Nordic Cochrane Centre, The Cochrane Collaboration. Review Manager (RevMan) Version 5.1. Copenhagen: The Nordic Cochrane Centre, The Cochrane Collaboration, 2011.

Richards 2003b

Richards J, Klein B, Carlbring P. Internet-based treatment for panic disorder. *Cognitive Behaviour Therapy* 2003;**32**(3):125-35.

Riordan 1989

Riordan R, Wilson L. Bibliotherapy: does it work?. *Journal of Counseling and Development* 1989;**67**(9):506-8.

Saarni 2007

Saarni SI, Suvisaari J, Sintonen H, Pirkola S, Koskinen S, Aromaa A, et al. Impact of psychiatric disorders on health-related quality of life: general population survey. The British Journal of Psychiatry. 2007/04/03 2007; Vol. 190:326-32. [0007-1250: (Print)]

Schneier 2001

F.R. Schneier. Treatment of social phobia with antidepressants. *Journal of Psychiatry* 2001;**62**(Suppl 1):43-8.

Schrank 1981

Schrank F, Engels D. Bibliotherapy as a counseling adjunct: research findings. *Personnel and Guidance Journal* 1981;**60**(3):143-7.

Scogin 1990

Scogin F, Bynum J, Stephens G, Calhoun S. Efficacy of self-administered treatment programs: meta-analytic review. *Professional Psychology: Research and Practice* 1990;**21**(1):42-7.

Spek 2007

Spek VC. Internet-based cognitive behaviour therapy for symptoms of depression and anxiety: a meta-analysis. *Psychological Medicine* 2007; Vol. 37, issue 3:319-28.

Steketee 1982

Steketee G, Foa EB, Grayson JB. Recent advances in the behavioral treatment of obsessive-compulsives. *Archives of General Psychiatry*. 1982/12/01 1982; Vol. 39, issue 12:1365-71. [0003-990X: (Print)]

Stevens 1982

Stevens M, Pfost K. Bibliotherapy: medicine for the soul?. *Psychology: A Journal of Human Behavior* 1982;**19**(4):21-5.

Stott 2013

Stott R, Wild J, Grey N, Liness S, Warnock-Parkes E, Commis S, et al. Internet-delivered cognitive therapy for social anxiety disorder: a development pilot series. *Behavioural and Cognitive Psychotherapy* 2013;**41**(4):383-97. [DOI: [10.1017/S1352465813000404](https://doi.org/10.1017/S1352465813000404)]

Sunderland 2012

Sunderland M, Wong N, Hilvert-Bruce Z, Andrews G. Investigating trajectories of change in psychological distress amongst patients with depression and generalised anxiety disorder treated with internet cognitive behavioural therapy. *Behaviour Research and Therapy* 2012;**50**(6):374-80.

van Boeijen 2005b

van Boeijen C, Van Balkom A, Van Oppen P, Blankenstein N, Cherpanath A, Van Dyck R. Efficacy of self-help manuals for anxiety disorders in primary care: a systematic review. *Family Practice* 2005;**22**(2):192-6.

Wade 2010

Wade AG. Use of the internet to assist in the treatment of depression and anxiety: a systematic review. *Primary Care Companion to the Journal of Clinical Psychiatry* 2010; Vol. 12, issue 4.

Wells 1997

Wells A. Cognitive Therapy of Anxiety Disorders: A Practice Manual and Conceptual Guide. New York: Wiley-Blackwell, 1997.

Wells 2006

Wells A, King P. Metacognitive therapy for generalized anxiety disorder: an open trial. *Journal of Behavior Therapy and Experimental Psychiatry*. 2005/08/30 2006; Vol. 37, issue 3:206-12. [0005-7916: (Print)]

Wittchen 2002

Wittchen H, Kessler R, Beesdo K, Krause P, Hoefler M, Hoyer J. Generalized anxiety and depression in primary care: prevalence, recognition, and management. *Journal of Clinical Psychiatry* 2002;**63**(Suppl 8):24-34.

Zaider 2003

Zaider TI, Heimberg RG, Fresco DM, Schneier FR, Liebowitz MR. Evaluation of the clinical global impression scale among individuals with social anxiety disorder. *Psychological Medicine* 2003;**33**(4):611-22.

Öst 1987

Öst LG. Applied relaxation: description of a coping technique and review of controlled studies. *Behaviour Research and*

Therapy. 1987/01/01 1987; Vol. 25, issue 5:397-409. [0005-7967: (Print)]

Öst 1988

Öst LG. Applied relaxation vs progressive relaxation in the treatment of panic disorder. *Behaviour Research and Therapy*. 1988/01/01 1988; Vol. 26, issue 1:13-22. [0005-7967: (Print)]

* Indicates the major publication for the study

CHARACTERISTICS OF STUDIES

Characteristics of included studies [ordered by study ID]

Abramowitz 2009

Methods	Location: Rochester (MN), USA Recruitment: Advertising and referral Exclusion rate: 25% Randomised (N): 21 (11 intervention; 10 comparison)
Participants	Disorder: Social Anxiety (Unspecified); MINI (DSM-IV) Duration (years): Unknown Age (years): 43.4 Sex (female): 76.19% Race (white): 57.14% Medication (using at baseline): 57.14% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Previous psychotherapy restricted Medication: "Individuals on psychotropic medication were instructed to remain on their current dose and were included as long as they had been at the same dosage for 3 months or more." Drug/Alcohol: "individuals were excluded if they reported substance abuse/dependence in last 6 months" Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: Shyness and Social Anxiety Workbook (Antony and Swinson, 2000) Days of treatment: 48 Author wrote: No Type of media: Book / Booklet

Abramowitz 2009 (Continued)

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face & Phone

Automated contact: N=0

Provider contact: N=7 (180 min)

Contact description: "At weeks 1, 2, 3, 6, and 8, a brief (<30 min) meeting with the therapist was held to review the chapters...No exposure or cognitive therapy took place; all CBT techniques were performed by the patient on his or her own. Questions for how to implement these techniques could be addressed...On weeks when no therapist contact was scheduled, a maximum 15-min telephone call was scheduled." "Individuals who inquired about the study were screened by telephone. If they appeared to meet entry criteria, they were invited to undergo a diagnostic interview..."

Intervention description: "Patients were provided with a copy of the SSAW free of charge. The 11 chapters in the workbook were divided into 5 sections to be read over the 8 weeks of therapy as shown in Table 1." "CBT-based self-help resource that includes instruction in how to implement cognitive restructuring and situational exposure..."

Compliance: "on the adherence question pertaining to completion of non-exposure therapy assignments in the SSAW was 6.38 (SD=0.80; range 5–7), suggesting relatively good self-reported adherence. On the question assessing adherence to exposure assignments, the mean score was 5.85 (SD= 1.32; range 2–7), also suggesting fairly good...." Adherence associated with better outcomes at follow-up though not at post-test.

Outcomes	Specific Symptoms: Brief Social Phobia Scale; Clinical Global Impression (CGI): Severity; Social Interaction Anxiety Scale (SIAS) General Symptoms: State-Trait Anxiety Inventory (STAI): Trait Depression: Beck Depression Inventory (BDI)
Notes	Funding: Mayo Clinic and Mayo Foundation. Grant awarded to Elizabeth Moore. Unpublished data: Abramowitz provided details about the generation of the allocation sequence, recruitment methods, and duration of contact for assessment. Unable to provide further outcome data or data about participants.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Participants were randomly assigned to 1 of 2 conditions" "We used a random number generator on a computer" (personal communication)
Allocation concealment (selection bias)	Low risk	"the allocation was not administered by a third party. The study coordinator simply generated the assignment on a computer." (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "...meeting with the therapist was held to review the chapters assigned for that particular week. The same therapist (ELM) worked with each patient and used a manual (available from the authors) to guide each meeting."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Assessor-rated outcomes: Yes Assessor blind: Yes

Abramowitz 2009 (Continued)

“Two independent evaluators (IEs) administered all posttest and follow-up assessments. IEs were trained in the administration of the BSPS and CGI-S and completely blind to treatment condition and assessment time.”

Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: No Dropout There were 3 dropouts at follow-up, but this occurred after the WL group received treatment and the results are not included in this review.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Al-Kubaisy 1992

Methods	Location: London, England Recruitment: Referral Exclusion rate: 36.13% Randomised (N): 99 (34 intervention; 31 comparison)
Participants	Disorder: Mixed (AG, SAD, SP); Clinical Interview (ICD-10) Duration (years): 18 Age (years): 35 Sex (female): 65.66% Race (white): 11.25% Medication (using at baseline): 11.25% Education (years): 12.02 EXCLUSIONS Depression: Yes Psychotherapy: Previous psychotherapy restricted Medication: "If medication exceeds more than the daily equivalent of 5mg of diazepam, 100mg imipramine, or 10mg propranolol, taken only at night, for at least 4 months" Drug/Alcohol: "no more than 2 units of alcohol a day from at least 3 weeks before entering the trial" Physical: "no severe organic disease"
Interventions	Comparison group(s): Applied relaxation (self-help)

Al-Kubaisy 1992 (Continued)

Comparison description: "patients were asked to carry out 90 minutes daily of r while lying or sitting comfortably and listening to one or more of three half-hour audiotapes of progressive muscle relaxation instructions (Jacobsen's) which were supplied....play the tapes of their choice intermittently to a total of 90 minutes daily and to record homework daily in a diary"

Intervention name: Living With Fear (Marks, 1980)

Days of treatment: 64

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=6 (360 min)

Contact description: "saw the clinician six times for an hour each time (at weeks 0, 1, 2, 3, 4, 6). In these six sessions the clinician helped the patient set relevant self-guided homework (e or r) to be carried out over weeks 0-8 for at least 90 minutes daily and be recorded in a daily diary. The clinician and patient reviewed problems and the homework diary together at each session except the first...Ee patients had a further 1.5 hours of clinician contact per session doing clinician-accompanied live exposure (E) (6 × 1.5 hours =9 hours in all)."

Intervention description: were asked to read the self-help chapter from Living With Fear (Marks, 1980) and to follow its guidelines. From the avoidance profile worked out at initial assessment the patient and clinician together planned the (e) (self- exposure) program together. Patients were asked to complete 90 minutes of e daily to their most phobic stimuli that they could tolerate without escape (crowds, shops, zoo, etc.), and to record completed exposure homework tasks in a daily diary."

Compliance: In the intervention group, participants reported 58 hours of homework out of 83 prescribed (72%). In the relaxation group, participants reported 62 of 84 hours (70%). Daily diary. Participants were not asked if they read the manual. Patient's compliance with homework was rated by the therapists based on a semi-structured interview.

Outcomes	Specific Symptoms: Fear Questionnaire (FQ) General Symptoms: Fear Questionnaire (FQ): Anxiety Depression Depression: Beck Depression Inventory (BDI); Hamilton Depression Rating Scale, clinician-rated (HAM-D) Disability: Work and Social Adjustment Scale (WSAS)	
Notes	Funding: Republic of Iraq grant awarded to TA-K, ORS grant, University of London PhD. Unpublished data: Located thesis. Unable to contact first author. Two eligible intervention groups differed only in amount of contact.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear

Al-Kubaisy 1992 (Continued)

		“Randomization was stratified according to 3 phobic types-agoraphobia, social phobia, or specific phobia” Dropouts were replaced (See Incomplete Outcome Data).
Allocation concealment (selection bias)	High risk	“At the end assessment the blind assessor made further assessment appointment for week 8, and handed the clinician a sealed envelope about the patient's treatment condition. Therapy then began (week 0) in the assigned treatment mode.” Dropouts were replaced (See Incomplete Outcome Data).
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Patients in all three conditions saw the clinician six times for an hour each time...” “The clinician helped the patient set relevant self-guided home-work...”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “...measures were self-rated (S) and rated by an assessor (A)- psychiatrists, psychologists and nurse therapists-kept blind to the treatment condition...”
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=12 (35.29%) <i>Comparison:</i> N=6 (19.35%) Method of analysis: Per protocol “The number of any patient who dropped out was returned to the pool of random numbers and reassigned to a fresh patient.” Int: 12/34 (4 treatment dropouts remained in the study, 4 refused, 4 dropouts). Con: 6/31 (2 refused, 4 dropouts).
Selective reporting (reporting bias)	Low risk	No trial registration number given, but data extracted from thesis likely complete. At 2-year follow-up “Only self-rated measures were analysed because assessor-rated measures were unavailable for patients who did not attend follow-up and so could be biased.”
Other bias	High risk	Dropouts were replaced (See Incomplete Outcome Data).

Al-Kubaisy 1992 (AG)

Methods	As above.	
Participants		
Interventions		
Outcomes		
Notes		
Risk of bias		
Bias	Authors' judgement	Support for judgement

Al-Kubaisy 1992 (AG) *(Continued)*

Allocation concealment (selection bias)	Low risk
Blinding of participants and personnel (performance bias) All outcomes	High risk
Blinding of outcome assessment (detection bias) All outcomes	Low risk
Incomplete outcome data (attrition bias) All outcomes	High risk
Selective reporting (reporting bias)	Low risk
Other bias	Low risk

Al-Kubaisy 1992 (SAD)

Methods	As above.
Participants	
Interventions	
Outcomes	
Notes	

Risk of bias

Bias	Authors' judgement	Support for judgement
Allocation concealment (selection bias)	Low risk	
Blinding of participants and personnel (performance bias) All outcomes	High risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	High risk	
Selective reporting (reporting bias)	Low risk	

Al-Kubaisy 1992 (SAD) (Continued)

Other bias Low risk

Al-Kubaisy 1992 (SP)

Methods As above.

Participants

Interventions

Outcomes

Notes

Risk of bias

Bias	Authors' judgement	Support for judgement
Allocation concealment (selection bias)	Low risk	
Blinding of participants and personnel (performance bias) All outcomes	High risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	High risk	
Selective reporting (reporting bias)	Low risk	
Other bias	Low risk	

Andersson 2009a

Methods **Location:** Stockholm, Sweden
Recruitment: Advertising/ Internet
Exclusion rate: 71.7%
Randomised (N): 30 (15 intervention; 15 comparison)

Participants **Disorder:** Specific Phobia; SCID (DSM-IV R)
Duration (years): Unknown
Age (years): 25.6

Andersson 2009a (Continued)

Sex (female): 84.8%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): 14.22

EXCLUSIONS

Depression: Yes

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Individual Exposure

Comparison description: "one-session treatment was delivered in two sessions, with a first brief orientation session and then the 3-hr exposure session, following Ost's (1997) guidelines...cognitions are challenged during the exposures...and four jars with spiders of increasing size were prepared...participants were given the video after the completion of the exposure. They were also given a maintenance program and instructed to confront spiders..."

Intervention name: Not named

Days of treatment: 28

Author wrote: Yes

Type of media: Internet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=4 (25min)

Contact description: "Participants were contacted if they failed to send in homework assignment reports." "When the homework was reported the following week, the participant had to complete a brief set of questions in order to be allowed to proceed to the next module." "contact with the therapist was 25 min in total per client"

Intervention description: "five text modules, which were presented on web pages and as downloadable PDF files...each participant had a therapist who was responsible for the whole treatment. E-mails were used in the contact, and the whole process was password-protected. In addition, a videotape illustrating exposure instruction was sent by post to the participants. Included psychoeducation, detailed exposure instructions, avoided situations and cognitions, exposure in real life, maintenance programme."

Compliance: "on average the participants' own estimate of how much time they had devoted to the treatment was 2.4 hr. per week (total time: 12 hr.)." "participant had to complete a brief set of questions

Andersson 2009a (Continued)

in order to be allowed to proceed to the next module" "participants were contacted if they failed to send in homework assignment reports"

Outcomes	Specific Symptoms: Fear Survey Schedule General Symptoms: Beck Anxiety Inventory (BAI) Response: Behavioural Approach Test: Completed Depression: Beck Depression Inventory (BDI)
Notes	Funding: Swedish Research Council

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "randomised by an independent person"
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "each participant had a therapist who was responsible for the whole treatment. E-mails were used in the contact..."
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) Clinically significant improvement was derived from the BAT.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (13.33%) <i>Comparison:</i> N=1 (6.67%) Method of analysis: Available Case because of computer problems (N=1) or lack of time (N=2). At follow-up, "one participant dropped out from the Internet treatment condition and two from the live-exposure treatment either because of lack of time or relocation." Percentages for the BAT indicate 7 missing.
Selective reporting (reporting bias)	High risk	No trial registration number given. The stated primary outcome (BAT) is not reported in full. Mean scores for each group appear in a line graph with no error bars.
Other bias	Low risk	N/A

Andersson 2012a

Methods	Location: Stockholm, Sweden
---------	------------------------------------

Andersson 2012a (Continued)

Recruitment: Advertising/ Internet

Exclusion rate: 52.36%

Randomised (N): 101 (50 intervention; 51 comparison)

Participants

Disorder: OCD; SCID (DSM-IV R)

Duration (years): 18

Age (years): 34.01

Sex (female): 66.34%

Race (white): 22.77%

Medication (using at baseline): 22.77%

Education (years): 13.89

EXCLUSIONS

Depression: No

Psychotherapy: Current and previous therapy restricted

Medication: "Concurrent use of psychotropic medication was permitted, if it had been stable for at least 2 months prior to inclusion, and if the participant agreed to maintain a constant dosage throughout the study."

Drug/Alcohol: "Current alcohol or drug abuse."

Physical: "Physical illness that could interfere with ICBT."

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: "...online non-directive supportive therapy, which consisted of access to an e-mail function integrated in the treatment platform...no active treatment components such as self-help texts or worksheets. Each week, the therapist contacted the participant through the treatment platform to enquire how the week had progressed and to encourage the participant to discuss current distressing life events..."

Intervention name: Not named (Andersson et al. 2011)

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=0

Provider contact: N=52 (129min)

Contact description: "provide feedback on homework assignments, grant consecutive access to the modules, and to support the participants in doing ERP...support or clarification...if they had not logged

Andersson 2012a (Continued)

on to the treatment platform for 7 days." "for the ICBT group was 129 min (SD=67.26) and the total number of messages sent from the therapist was 35 (SD=13.95)" 17 min/16 messages control. "... 17 telephone calls to the ICBT group reminding them to report ERP progression on the treatment platform. For the control condition, the therapists made 41 telephone calls..."

Intervention description: "text material (about 100 pages) and worksheets divided into 10 modules...accessible as an mp3 file (about 5 h of total listening)... read the same texts relating to general psychoeducation and rationale for the treatment, but tailored examples of obsessions and compulsions...Modules 1–4 consisted of psychoeducation, cognitive restructuring of meta-cognitions, and of establishing an individual ERP hierarchy...Modules 5–10 focused on doing daily in vivo ERP..."

Compliance: "The average number of completed modules in the ICBT group was 7.28...Six participants in the ICBT group were considered as non-completers, as they did not begin ERP"

Outcomes	Specific Symptoms: Clinical Global Impression (CGI): Improvement Response: Yale-Brown Obsessive-Compulsive Scale (Y-BOCS) Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Disability: Global Assessment of Functioning Scale (GAF)	
Notes	Funding: Swedish Research Council and the Swedish Society of Medicine. “support was provided through the regional agreement on medical training and clinical research (ALF) between Stockholm County Council and Karolinska Institutet.”	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “Participants were randomized (www.random.org) with a 1:1 ratio”
Allocation concealment (selection bias)	Low risk	“by an independent person who was not involved in the study.”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “This group used the same therapists as the ICBT group but this contact did not include any CBT interventions.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “Blinding was broken for five participants in the ICBT group and for one participant in the control condition during the post-treatment interview assessments.”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (4%) <i>Comparison:</i> N=0 (0%) Method of analysis: Available Case “One participant in the ICBT group was lost to the post-treatment telephone interview assessment, and two participants did not complete the internet self-

Andersson 2012a (Continued)

rating questionnaires...There was no loss of data for the control group at baseline or post-treatment."

Selective reporting (reporting bias)	High risk	Trial registration number: NCT01347099 Dimensional Obsessive Compulsive Scale and EuroQoL not reported. CGI and GAF were not registered. Dichotomous CGI results not reported.
Other bias	Low risk	N/A

Andersson 2012b

Methods	Location: Sweden Recruitment: Advertising/ Internet Exclusion rate: 44.1% Randomised (N): 204 (102 intervention; 102 comparison)
Participants	Disorder: Social Anxiety (Unspecified); SCID (DSM-IV) Duration (years): Unknown Age (years): 38.3 Sex (female): 60.20% Race (white): Unknown Medication (using at baseline): 13.73% Education (years): 12.96 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted; no previous restrictions reported Medication: "if on prescribed medication for anxiety/ depression, dosage had to be constant for 3 months before the treatment onset and kept constant throughout the study" Drug/Alcohol: "not admitting another serious or dominant disorder (e.g. psychosis, substance misuse) that could be expected to influence the outcome of the study" Physical: N/A
Interventions	Comparison group(s): Wait-list Intervention name: Social Fobi-Effektiv Hjalp med Kognitiv Beteendeterapi [Social Phobia - Effective Help with CBT] Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere

Andersson 2012b (Continued)

Type of contact: Email

Automated contact: N=9

Provider contact: N=9 (135 min)

Contact description: "Feedback was given each week by the therapists. Text messages were used to remind participants to log in or to complete weekly reports on the LSAS-SR." "The treatment group had access to an Internet therapist during the 9 week treatment period. E-mail correspondence occurred weekly (Sundays) and generally concerned the results of homework assignments as described in the self-help manual...On average, Internet therapists spent 15 min per week for each participant reading messages and providing feedback via the online contact handling system."

Intervention description: "186 pages divided into nine chapters (modules) adapted for use via the Word Wide Web...introductory module describing SAD and...CBT...a cognitive model for SAD...cognitive restructuring... exposure and attention shifting exercises...social skills and relapse prevention...complete one module every week...Each module consisted of information, exercises (home-work assignments) and ended with a short quiz..." "both groups had access to separate moderated online discussion forums."

Compliance: "46/102 (45%) failed to complete all nine modules...activity in the online discussion forums varied in line with previous investigations, which means that a majority made few comments and postings in addition to the postings linked to the "topic of the week", some were more active with discussions, and some mainly read were passive."

Outcomes	Specific Symptoms: Liebowitz Social Anxiety Scale (LSAS); Social Phobia Scale (SPS); Social Interaction Anxiety Scale (SIAS); Social Phobia Screening Questionnaire (SPSQ) Response: Clinical Global Impression (CGI); Liebowitz Social Anxiety Scale (LSAS) Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Swedish Research Council	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Randomization was performed by an independent third-party using an online true random-number service (www.random.org)."
Allocation concealment (selection bias)	Low risk	See "Sequence Generation."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Feedback was given each week by the therapists"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "independent"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N= 8 (7.84%) <i>Comparison:</i> N= 2 (1.96%)

Andersson 2012b (Continued)

Method of analysis: LOCF

"the last previous LSAS-SR score was used to replace missing data."

Selective reporting (reporting bias)	Low risk	Trial registration number: UMIN000001383 All outcomes reported.
Other bias	Low risk	N/A

Andersson 2012c

Methods	Location: Sweden Recruitment: Advertising/ Internet Exclusion rate: 54.49% Randomised (N): 54 (27 intervention; 27 comparison)
Participants	Disorder: GAD; SCID (DSM-IV) Duration (years): 20.05 Age (years): 42 Sex (female): 75.93% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted; no previous restrictions reported Medication: "(e) if taking prescribed medication for anxiety or depression, the duration had to be at least 12 weeks, and the participant had to be on a stable dosage for at least 6 weeks (and such participants were instructed not to change medication and/or dosage during the trial)" Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-list Intervention name: Not named (as in Paxling 2011) Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Internet forum

Andersson 2012c (Continued)

Automated contact: N=9

Provider contact: N=9 (90 min)

Contact description: "Except for the weekly online treatment contact and diagnostic procedures, no other contact took place between the therapists and participants...5 therapists provided the treatment for this group. Two were licensed psychologists...3 were psychology students...four supervision sessions were given of approximately 90 min each. Feedback from the therapist was provided...In addition, occasional reminders were sent. The mean therapist time devoted to each client during the overall treatment period was 92 min (SD = 61)."

Intervention description: "text-based treatment modules delivered on a weekly basis for 8 weeks... Introduction to GAD and the treatment...applied relaxation...applied relaxation, and worry time...cognitive restructuring...applied relaxation, cognitive distancing, and problem solving...exposure...interpersonal problem solving...sleep management...Relapse prevention and maintenance of progress."

Compliance: "Mean 5.1 (SD=2.5) modules completed."

Outcomes	Specific Symptoms: Generalized Anxiety Disorder Questionnaire–IV; Penn State Worry Questionnaire (PSQW); General Symptoms: Beck Anxiety Inventory (BAI); State-Trait Anxiety Inventory (STAI) Response: Generalized Anxiety Disorder Questionnaire–IV; Penn State Worry Questionnaire (PSWQ) Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI) Recovery: No longer met criteria for diagnosis Response: Generalized Anxiety Disorder Questionnaire–IV; Penn State Worry Questionnaire (PSQW); Clinical Global Impression (CGI)	
Notes	Funding: Swedish Council for Working and Life Research	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "unrestricted randomization"
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Feedback from the therapist was provided..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "participants were asked not to reveal whether they had received treatment."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N= 0 (0%) <i>Comparison:</i> N= 0 (0%)

Andersson 2012c (Continued)

Method of analysis: mixed effects

"Mixed effect models are able to accommodate missing data and integrate time-varying factors."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Andrews 2011a

Methods	Location: Sydney, Australia Recruitment: Referral Exclusion rate: 50.67% Randomised (N): 37 (23 intervention; 14 comparison)
Participants	Disorder: Social Anxiety (Unspecified); Clinical Interview (DSM-IV) Duration (years): Unknown Age (years): 31.9 Sex (female): 40.5% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "Change in medications during last 1 month or intended change during study duration." Drug/Alcohol: "Current substance abuse/dependence" Physical: N/A
Interventions	Comparison group(s): Individual CBT Comparison description: "Each group had a maximum of seven participants (not all of whom were research participants) who met weekly for seven weeks for 4 h under the guidance of the same clinician (M.D.). The content of the programme followed the programme outlined in a standard text. All participants were given a copy of the programme to use in the sessions and to revise between sessions." (Andrews G, Creamer M, Crino R, Hunt C, Lampe L, Page A (2003). The treatment of anxiety disorders.) Intervention name: Shyness Program (http://www.climateclinic.tv) Days of treatment: 56

Andrews 2011a (Continued)

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=11

Provider contact: N=7 (duration unknown)

Contact description: “automatic emails and fortnightly short message service (SMS)...Participants were telephoned or emailed each week by the clinician.”

Intervention description: “six online lessons; a summary/homework assignment for each lesson; comments by participants on a forum moderated by the clinician (M.D.)...provided education about the symptoms and treatment...how to develop an exposure hierarchy...cognitive restructuring...relapse prevention. Participants were encouraged to complete the first four lessons within the first two weeks, in order to provide more opportunity to practise the graded exposure, cognitive skills and other coping techniques.”

Compliance: 6 withdrew before treatment began. 1 terminated at lesson 3, 1 at lesson 4, 1 at lesson 6.

Outcomes	Specific Symptoms: Social Interaction Anxiety Scale (SIAS) Disability: WHO Disability Assessment Schedule (WHODAS-II)
Notes	Funding: None acknowledged Unpublished data: Personal correspondence.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “by NT via a true randomization process (www.random.org)”
Allocation concealment (selection bias)	Low risk	“by NT via a true randomization process (www.random.org)”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Participants were telephoned or emailed each week by the clinician.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=6 (26.09%) <i>Comparison:</i> N=0 (0%)

Andrews 2011a (Continued)

Method of analysis: LOCF

The pre-treatment scores of the 10 participants who did not complete the post-treatment questionnaires were replicated as their post-treatment scores but only those who started lesson 1 were "eligible for analysis."

Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12609000212257 PHQ-9, Kessler-10 and Sheehan Disability Scale all stated in protocol and omitted from write-up
Other bias	Low risk	N/A

Baillie 2002

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 77.06% Randomised (N): 117 (51 intervention; 66 comparison)
Participants	Disorder: Panic Attacks; CIDI (DSM-IV) Duration (years): 8.13 Age (years): 37.93 Sex (female): 76.07% Race (white): 59.83% Medication (using at baseline): 59.83% Education (years): 13.38 EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Treatment-as-usual Intervention name: Booklet based on Rapee, Craske and Barlow (1996) and "Panic Surfing" Days of treatment: 137 Author wrote: Yes Type of media: Book / Booklet

Baillie 2002 (Continued)

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=1 (duration unknown)

Contact description: People who did not complete assessments were reminded by post after two weeks, sent a second copy of the questionnaire after 4 weeks, and telephoned after 6 weeks.

Intervention description: Stepped-up care, moving up if panic attacks continued or MIA score 1.67 or greater. First: "posted a psychoeducational booklet." Second: a manual that "focuses on catastrophic misinterpretations of physical sensations...daily diary and described a five-week program of activities to challenge catastrophic thought, carry out exposure to situations and sensations related to panic, and develop a plan of relapse prevention."

Compliance: There was not evidence that treatment group, overall (est.=0.29, SE=0.27, Z=1.10) nor the self-help manual specifically (est.=0.04, SE=0.31, Z=0.13) influencing the frequency that participants used exposure.

Outcomes	Recovery: Clinically Significant Improvement (Composite)
Notes	Funding: National Health and Medical Research Council Public Health Postgraduate Research Scholarship. Macquarie University Postgraduate Research Fund. Division of Scientific Affairs of the Australian Psychological Society. Unpublished data: Baillie provided unpublished thesis and information about the trial design. Requiring written consent introduced a delay between assessment and posting the booklet of 18.5 (14.6) days. Data about duration of illness appear unreliable.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "A list of random assignments was constructed at the outset of the trial and participants were assigned in the order that their consent forms were received by post." (personal communication)
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "All telephone interviews were conducted by the first author..."
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "Participants were not blind to their allocation and all telephone assessments [were] conducted by the first author who was not blind to group allocation." "At baseline and at 6 weeks, 4.5 months (20 weeks), and 9 months (40 weeks) participants completed a telephone interview and a self report questionnaire sent by post."

Baillie 2002 (Continued)

Incomplete outcome data (attrition bias)
All outcomes

Low risk

Not analysed at post-treatment

Intervention: N=0 (0%)

Comparison: N=2 (3.03%)

Method of analysis: Available Case

Of 468 assessment points (117 participants x 4 collections), there were 437 valid responses. For Panic Disorder Severity Scale, group means were substituted for missing cases. Further details are not reported by group.

Selective reporting (reporting bias)

High risk

No trial registration number given.

Raw data are not reported and regressions based on 9 month outcomes cannot be incorporated in meta-analysis. The authors interpret the data to indicate no effect. Missing measures include the Depression, Anxiety, Stress Scales (DASS), Mobility Inventory for Agoraphobia (MIA), and panic diary.

Other bias

Low risk

N/A

Baker 1973

Methods

Location: Cambridge (Mass), USA

Recruitment: Advertising/ Internet

Exclusion rate: Unknown

Randomised (N): 30 (9 intervention; 9 comparison)

Participants

Disorder: Specific Phobia; Unclear

Duration (years): Unknown

Age (years): 32

Sex (female): 70%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Individual Exposure

Baker 1973 (Continued)

Comparison description: “standard systematic desensitization with a therapist (cf. Wolpe and Lazarus, 1966)...Following one session for hierarchy construction, three or four sessions were used for relaxation training; S was also encouraged to practice relaxation at home...median of eight desensitization sessions (range 5-10); each session included a median of 14 image presentations, each lasting about 30 set with 45 set rest between trials...continued until S completed his hierarchy.”

Intervention name: Based on Behavior Therapy Techniques: A Guide to the Treatment of Neurosis (Wolpe, J & A. A Lazarus; 1996)

Days of treatment: 49

Author wrote: Yes

Type of media: Audio

Type of therapy: Desensitization

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=6 (90 min)

Contact description: “seen by this therapist for only one full session, to construct a hierarchy. Thereafter, S came to an office to conduct, on his own, a tape recorded relaxation or desensitization session...every second session S would speak with his therapist for about 5 min to discuss progress or problems.”

Intervention description: “came to an office to conduct, on his own, a tape recorded relaxation or desensitization session, using a taped version of the self-directed desensitization record...” “...three or four relaxation sessions were followed by median of 7 desensitization sessions (range 5-14).”

Compliance: The number of sessions intended was not reported. The totals reported are the sessions actually attended.

Outcomes	Specific Symptoms: Acrophobia Questionnaire: Anxiety
Notes	Funding: Foundations' Fund for Research Unpublished data: Provided information about assignment of participants. Unable to provide further information about the results. It is unclear how previous reviewers calculated effect sizes for this study.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear “Wait-list group excluded: 'Ss who would be unable to schedule sessions throughout the following 2 months (but were available later) were arbitrarily placed on the WL. All other Ss were blocked by sex and randomly assigned to one of three conditions...' WL group excluded as clearly not random (12 participants not included).
Allocation concealment (selection bias)	High risk	After the initial interview and the polygraph session, S was scheduled to begin treatment or was placed on the Waiting List.

Baker 1973 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Provider contact: Yes Provider blind: No “assigned randomly to one of the authors, but was seen by this therapist for only one full session”
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) “this session was conducted by an E who was blind to S’s treatment condition. Since S’s treatment condition would become apparent to E during the interview, E first administered the objective tests, then gave S a self-report questionnaire to complete, and then conducted the interview.”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=2 (22.22%) Method of analysis: Available Case 9 were assigned to face-to-face therapy. 1 transferred to the wait-list and 1 dropped out after 3 relaxation and 2 desensitization sessions. After post-treatment assessment, 3 WL participants were assigned to each treatment group and 6 undertook treatment at home and were not followed.
Selective reporting (reporting bias)	High risk	No trial registration number given. Summary statistics are not reported and statistical tests are reported selectively. Authors report that changes on the Leary and MMPI were not significant. Data insufficient to analyse Slide-Series Discomfort Rating.
Other bias	High risk	May not be a RCT, and it is not indexed as such in electronic databases.

Barrera 1977

Methods	Location: Eugene (Oregon), USA Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 24 (16 intervention; 8 comparison)
Participants	Disorder: Specific Phobia; Questionnaire & BAT Duration (years): 25.6 Age (years): 31.6 Sex (female): 100% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown

Barrera 1977 (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Attention/Monitoring Comparison description: "self-administered placebo with self-reward contracting" Intervention name: Rosen, G. M. (1974). A manual for self-administering systematic desensitization in your home. University of Oregon. Days of treatment: 56 Author wrote: Yes Type of media: Book / Booklet Type of therapy: Desensitization Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: Unknown Contact description: The self-reward contingency contracting supplement received by subjects in SSDCC and PCC was explained during an individual session that followed pretesting. Intervention description: 1) self-administered desensitization 2) "self-administered desensitization with self-reward contracting" "adapted from Rosen et al. (1976)." Compliance: "While working on their programs, subjects had no contact with project staff, although they were to mail in weekly progress reports as previously described." "As in earlier studies, 50% of SSD subjects (n = 4) finished treatment, whereas not one of the subjects in SSDCC progressed beyond the relaxation training phase of the program."
Outcomes	Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR) Response: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR); Social Interaction Anxiety Scale (SIAS); Social Phobia Scale (SPS) Depression: Beck Depression Inventory (BDI)
Notes	Funding: None acknowledged Unpublished data: Programme duration not reported - assumed it was delivered as in Rosen 1976.
Risk of bias	
Bias	Authors' judgement Support for judgement
Random sequence generation (selection bias)	Unclear risk Method: Unclear

Barrera 1977 (Continued)

“Subjects were assigned on a random basis...”

Allocation concealment (selection bias)	Unclear risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “The self-reward contingency contracting supplement received by subjects in SSDCC and PCC was explained during an individual session that followed pretesting.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes included. “Assessment instruments included a standard behavioral avoidance test, fear thermometer (FT) ratings, a snake attitude questionnaire, the snake item from Geer's Fear Survey Schedule (FSS), discomfort ratings while viewing snake and neutral slides, and a general fear-change rating.”
Incomplete outcome data (attrition bias) All outcomes	Unclear risk	Not analysed at post-treatment <i>Intervention:</i> Unknown <i>Comparison:</i> Unknown Method of analysis: Unclear
Selective reporting (reporting bias)	High risk	No trial registration number given. Outcome data cannot be analysed. "Assessments occurred at pre- assessment, postassessment, and 1-month follow- up periods." "between-groups differences on posttest or follow-up scores were not revealed"
Other bias	Low risk	N/A

Bell 2012

Methods	Location: Christchurch, New Zealand Recruitment: Referral Exclusion rate: 92.73% Randomised (N): 83 (40 intervention; 43 comparison)
Participants	Disorder: Mixed (GAD, PD, SAD); SCID (DSM-IV) Duration (years): 18.01 Age (years): 35.31 Sex (female): 67.47% Race (white): Unknown Medication (using at baseline): 59.03 Education (years): 12

Bell 2012 (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted; no previous restrictions reported

Medication: "started on, or changed the dose of, an antidepressant in the previous 6 weeks."

Drug/Alcohol: "current diagnosis of substance dependence"

Physical: "significant or disabling cardiac, respiratory or neurological condition (which would make participation in the trial medically unwise)"

Interventions

Comparison group(s): Wait-list

Intervention name: Based on Andrews et al. 2003 (The Treatment of Anxiety Disorders: Clinician Guides and Patient Manuals)

Days of treatment: 89

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=6 (30 min)

Contact description: "The research assistant telephoned participants in the CCBT condition every 2 weeks over the treatment phase of the study (i.e. the first 12 weeks)...the WLC group were also contacted by the research assistant every 2 weeks over the same timeframe. The content of the telephone call followed a predetermined structure for both groups and was of about 5 minutes' duration. It involved checking if participants were having any technical problems with the programme and encouraged them to complete the sessions."

Intervention description: "(GAD: four lessons; panic disorder: six lessons; social phobia: six lessons). Part of the content of each lesson was presented in the form of an illustrated story...provided education...gave instructions about developing an exposure hierarchy and practising graded exposure, demonstrated principles of cognitive restructuring and concluded with information about relapse prevention."

Compliance: "Of the 40 patients randomised to CCBT, 14 (35%) did not complete the treatment (as defined above) (GAD = 2/7, 29%; social phobia = 5/17, 29%; panic disorder = 7/16, 44%)...In order to allow comparison with other CCBT studies using the same programme in different settings and with different design, the absolute completion rates of the CCBT (i.e. all sessions completed) were 4/7 (57%) patients with GAD, 9/17 (53%) patients with social phobia, and 5/16 (31%) patients with panic disorder."

Outcomes

Specific Symptoms: Global severity

General Symptoms: Beck Anxiety Inventory (BAI); Fear of Negative Evaluation (FNE); Fear Questionnaire (FQ); Generalised Anxiety Disorder Assessment Inventory (GADI); Liebowitz Social Anxiety Scale (LSAS)

Depression: Beck Depression Inventory (BDI)

Disability: Work and Social Adjustment Scale

Bell 2012 (Continued)

Notes

Funding: Canterbury District Health Board, Canterbury Medical Research Foundation, New Zealand Lottery Grants Board

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "randomised to either CCBT or the WLC, using a stratified block design for the primary diagnosis (GAD, panic disorder or social phobia). The stratified randomisation list was generated by a statistician (CF) who was not involved in implementation of the study. " "Participants were randomised in a 1:1 ratio to the computerised programme and WLC."
Allocation concealment (selection bias)	Low risk	"The allocation list and envelopes containing the allocations were held by the CRU coordinator (who was not involved directly in implementation of the study) to ensure that the clinicians and the research assistant involved in assessment and enrolment were not biased in the enrolment process"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "All participants in the trial (both those randomised to CCBT and those to WLC) were contacted by the research assistant at two-weekly intervals across the treatment phase of the study (six times over the 12 weeks)."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N= 0 (0%) <i>Comparison:</i> N= 0 (0%) Method of analysis: mixed effects "All data analysis was carried out according to a pre-established statistical analysis plan. Analyses of participants' self-rated questionnaire scores were undertaken using mixed model repeated measures (MMRM) controlling for baseline scores and testing for the between-group factors of group (CCBT, WLC) and primary diagnosis (GAD, panic disorder, social phobia)."
Selective reporting (reporting bias)	Unclear risk	Trial registration is incomplete and does not list measures used: AC-TRN12606000349549; URB/06/05/039
Other bias	Low risk	N/A

Berger 2009

Methods

Location: Bern, Switzerland

Recruitment: Advertising/ Internet

Exclusion rate: 70.45%

Randomised (N): 52 (31 intervention; 21 comparison)

Berger 2009 (Continued)

Participants

Disorder: Social Anxiety (Unspecified); SCID (DSM-IV)

Duration (years): 10.05

Age (years): 28.9

Sex (female): 55.8%

Race (white): 1.92%

Medication (using at baseline): 1.92%

Education (years): 13.77

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: Based on CBT (Clark and Wells 1995; Stangier, Heidenreich and Peitz 2003).

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=10.9 (duration unknown)

Contact description: "therapists have their own Internet-based platform where they have an overview of all their patients and an easy and quick access to the Web-based environment of each patient." "therapists were instructed to write a short motivating message to the clients once a week" Author estimates 120 minutes for SCID (personal communication), and "The average number of emails written by the participants was 6.01....The average number of emails written by the therapists was 10.9..."

Intervention description: "based on the established cognitive-behavioral approach by Clark and Wells (1995; Stangier, Heidenreich, & Peitz, 2003)...participants are able to freely navigate through the Web pages and repeat exercises...57 Web sites that are divided into five sessions." "module for establishing regular text-based contact with a therapist, a continuous monitoring and feedback system of patients' responses, as well as collaborative online group elements"

Compliance: 1 participant did not write to the therapist and completed only 15% of the guide, 2 completed 40% and 50% of the guide but not post-treatment (no reason, lack of time). Those who complet-

Berger 2009 (Continued)

ed post-treatment completed 85% of the self-help guide (16 completed 5 sessions, 6 completed 4 sessions, 5 completed 3 sessions, one completed only the first session).

Outcomes	Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR) Response: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR); Social Interaction Anxiety Scale (SIAS); Social Phobia Scale (SPS) Depression: Beck Depression Inventory (BDI)
Notes	Funding: Swiss National Science Foundation (SNF 100011-112345) Unpublished data: Berger provided demographic details, contact with participants, and details about randomisation and allocation concealment.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “According to a computer-generated randomization scheme, 31 of the 52 subjects included in the randomization were assigned to the treatment group and 21 to the control group.”
Allocation concealment (selection bias)	Unclear risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “When patients log into the program for the first time, they are redirected to the contact module, where a therapist has introduced her- or himself and informed the patients that they may contact her/him whenever they want to.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF “All statistical analyses used intent-to-treat principles in which pretreatment data were carried forward for noncompleters to replace missing values. However, there were few drop-outs in our study, with 90% of the participants completing the questionnaires at postassessment.”
Selective reporting (reporting bias)	High risk	Trial registration number: ISRCTN62304985 The trial registration lists the State-Trait Anxiety Inventory (STAI), but this is not reported.
Other bias	Low risk	The registered age range was 18-35 but the report includes participants 18-45.

Bergstrom 2010

Methods	Location: Stockholm, Sweden Recruitment: Referral Exclusion rate: 71.46% Randomised (N): 113 (<i>53 intervention; 60 comparison</i>)
Participants	Disorder: Panic disorder; MINI (DSM-IV) Duration (years): 6.68 Age (years): 34.22 Sex (female): 61.4% Race (white): 45.04% Medication (using at baseline): 45.04% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "If taking prescribed drugs for panic disorder, having had a constant dosage for 2 months prior to commencing treatment in the study" Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Group CBT Comparison description: The group treatment was led by two clinical psychologists who presented the self-help programme mentioned above during weekly 2-hour sessions, with the support of printed handouts of the modules given to the patients. "the 54 group patients were distributed over 10 different groups whose sessions were 2 hours each and led by 2 therapists, and that the actual average number of weekly group sessions attended in the group treatment was 8.1 (SD = 2.1)" Intervention name: Not named Days of treatment: 70 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Email Automated contact: N=0 Provider contact: N=11.3 (35 min)

Bergstrom 2010 (Continued)

Contact description: "Each module ended with a number of questions to be answered by the patient through interactive forms (e.g. homework assignments). After reviewing these answers, the psychologist gave access to the next module and provided feedback. At any moment the patient could post a message if he or she needed further help." "The total number of e-mails sent by the therapists during treatment was 555 (mean per patient: 11.3, SD = 4.3). The total average therapist time spent per patient in the Internet treatment was 35.4 minutes (SD = 19.0)."

Intervention description: "...psychoeducation (module 1), cognitive restructuring (modules 2 and 3), interoceptive exposure (modules 4 and 5), exposure in-vivo (for agoraphobic situations; modules 6 to 9), and relapse prevention (module 10)...The patient also had the opportunity to participate in an on-line discussion forum with other patients in treatment during the same time period. However, this was not mandatory."

Compliance: The average number of weekly modules completed in the Internet treatment was 6.7 (SD = 2.5).

Outcomes	Specific Symptoms: Panic Disorder Severity Scale (PDSS) General Symptoms: Anxiety Sensitivity Index (ASI) Response: Clinical Global Impression (CGI): Improvement; Panic Disorder Severity Scale (PDSS) Recovery: No longer met criteria for diagnosis Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Disability: Sheehan Disability Scale (SDS)
Notes	Funding: Stockholm County Council Unpublished data: Table 3 appears to contain an error - means and SEs are not reported for each group.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Unclear "The participants were divided into two groups, Internet or group treatment, by an independent random-number procedure..."
Allocation concealment (selection bias)	Low risk	"...each patient was assigned to either treatment by the opening of sealed numbered envelopes."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Each module ended with a number of questions to be answered by the patient through interactive forms (e.g. homework assignments). After reviewing these answers, the psychologist gave access to the next module and provided feedback."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "The psychiatrists performing the clinical interviews at post-treatment and follow-up were blind to treatment condition."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=9 (16.98%)

Bergstrom 2010 (Continued)

Comparison: N=11 (18.33%)

Method of analysis: Available Case

"Nine participants dropped out after randomisation but before commencing treatment. Various reasons were given for not starting treatment, but all pertained to different life circumstances of the individual participants and not to randomisation status. These initial dropouts were excluded from the statistical analyses." "A number of patients did not return for the clinical interview...a Mixed effects models approach was used in the statistical analysis to adjust for these missing values."

Selective reporting (reporting bias)	Unclear risk	Trial registration number: NCT00845260 The study was registered after it had been completed. All registered outcomes are reported, but the CGI is reported and not registered.
Other bias	Low risk	N/A

Bickel 2007

Methods	Location: Ohio, USA Recruitment: Advertising/ Internet Exclusion rate: 85.03% Randomised (N): 22 (11 intervention; 11 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-IV) Duration (years): Unknown Age (years): 42 Sex (female): 73.73% Race (white): 68.18% Medication (using at baseline): 68.18% Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current and previous therapy restricted Medication: "Patients who were currently taking medication for the treatment of panic disorder on a regular basis had to be willing to keep their dosage constant through the 6-week evaluation period." No "Initiation or reinitiation of tricyclic antidepressants, SSRIs, or MAO inhibitors during the past 6 months." nor "Initiation or reinitiation of benzodiazepines in the past 30 days." Drug/Alcohol: N/A

Bickel 2007 (Continued)

Interventions	Physical: "Lifetime history of severe medical disorders that could complicate panic disorder including cardiovascular, respiratory or renal disorders, stroke, epilepsy or hypertension." Comparison group(s): Wait-List Comparison description: Participants in both groups completed twice-weekly monitoring forms. Intervention name: CALM (Bickel) Days of treatment: 42 Author wrote: Yes Type of media: Computer Type of therapy: Exposure Location used: Clinic/ Surgery Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=10 (duration unknown) Contact description: "The treatment was administered twice weekly at scheduled appointments. At each appointment, a trained undergraduate administrator asked the participant to complete a symptom monitoring form for the intervening period between sessions. These forms were used to alert the experimenter to any drastic changes in symptoms." Intervention description: 10 sessions, twice weekly for 5 weeks. "includes video, audio, and text. Approximately two thirds of the treatment is comprised of audio accompanied by text. The rest of the treatment is video-based with supplementary text. The computer program is moderately interactive. In addition to the above, participants respond to onscreen quiz questions during the psychoeducational portion of treatment. Correct responses are required for the patient to proceed..." Compliance: "The quality of homework completed was rated either 'fair' or 'good' for all clients across sessions with little variability between or within groups. Compliance ratings were completed by K.W.B."
Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Anxiety Sensitivity Index (ASI); Beck Anxiety Inventory (BAI) Depression: Beck Depression Inventory (BDI)
Notes	Funding: Society for the Science of Clinical Psychology and the Coca Cola Foundation Unpublished data: Bickel provided dissertation but was unable to provide further details about the study.
Risk of bias	
Bias	Authors' judgement Support for judgement
Random sequence generation (selection bias)	Method: Unclear "Twenty-two participants (see power analysis below) were matched on severity of panic and agoraphobic symptoms and randomly assigned to one of two treatment conditions: treatment via CALM or a delayed treatment control condition, in a matched- pairs design." "Participants who qualified for the treatment study after the Time 1 interview were matched with another participant

Bickel 2007 (Continued)

		and were randomly assigned to receive immediate treatment or wait 5-weeks to begin a delayed treatment."
Allocation concealment (selection bias)	Low risk	"The initial assessment, as well as the post-treatment assessment, included the administration of the Anxiety Disorders Interview Schedule for the DSM-IV (ADIS-IV), conducted by a trained administrator who was blind to treatment condition." "All substantive interactions with clients were conducted by trained research assistants (assessments) or advanced undergraduate students (subsequent appointments) in order to reduce interactions with the therapist from the computer intervention (KWB)."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "At each appointment, a trained undergraduate administrator asked the participant to complete a symptom monitoring form for the intervening period between sessions." "By administering the treatment at regular appointments, we were able to monitor completion of the treatment."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes are reported, but the paper reports, "The initial assessment, as well as the post-treatment assessment.. conducted by a trained administrator who was blind to treatment condition." "Time 2 assessment was conducted subsequently by a trained interviewer who was blind to treatment condition"
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (27.27%) <i>Comparison:</i> N=6 (54.55%) Method of analysis: Available Case At the end of the study, there were only 5 complete matched-pairs. "we proceeded under the assumption that attrition was different between the two groups... For this review, data were extracted for all completers. The author's main analyses include only complete matched-pairs. The pattern of results was essentially the same, but the effects were smaller for the author's analysis. (Those remaining in the comparison group were the most distressed.) "at the time of recruitment, our clinic was in a transition phase and therapist-administered treatment was not available. Subsequent to the initiation of the study, therapists became available..."
Selective reporting (reporting bias)	High risk	No trial registration number given. Mobility Inventory for Agoraphobia (MI) and Fear Questionnaire (FQ) not reported, except as graphs. Safety Maneuver Questionnaire measured but not reported as authors found it problematic and in need of further study. Baseline values for the ACQ and the BSQ (Table 2) are either incorrect or transformed through a process that is not described.
Other bias	Low risk	N/A

Botella 2009

Methods
Location: Castelló, Spain

Recruitment: Advertising and referral

Exclusion rate: Unknown

Randomised (N): 52 (30 *intervention*; 22 *comparison*)

Participants
Disorder: Social Anxiety (Specific); ADIS (DSM-IV)

Duration (years): Unknown

Age (years): 24.4

Sex (female): Unknown

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): 13.85

EXCLUSIONS
Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: N/A

Drug/Alcohol: "Exclusion criteria were current alcohol or drug dependence"

Physical: N/A

Interventions
Comparison group(s): Individual CBT

Comparison description: "The therapist followed exactly the same steps as 'Talk to me': education, cognitive therapy, and exposure. The main difference was that instead of being a computer- based self-applied treatment delivered over the Internet, treatment was provided by an experienced therapist." "two sessions every week until they finished the treatment, receiving face-to-face therapy for no longer than two months"

Intervention name: Talk to Me

Days of treatment: 61

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "These participants could contact the psychologist by e-mail or telephone when they had any doubt or problem."

Botella 2009 (Continued)

Intervention description: “six scenarios which consist of real videotaped audiences...the system asks some questions to make sure that the patient has understood the content...rationale for cognitive restructuring, highlighting the role of negative thoughts...Each scenario offered by the program has several variables (e.g., number of people) in order to build an exposure hierarchy.” “if they had no Internet access they could self-apply the program at the university.”

Compliance: N/A

Outcomes	None
Notes	Funding: Ministerio de Educacion y Ciencia, Spain, PROYECTOS CONSOLIDER-C (SEJ2006-14301/PSIC), by Programa de Acciones Integradas con Sudafrica (HS2006-0001), and by Universitat Jaume I - Fundacio Caixa Castello (P1 1A2005-06)

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “All participants who wanted to take part in the study were randomly allocated to one of the two experimental conditions.”
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “These participants could contact the psychologist by e-mail or telephone when they had any doubt or problem. Another group received face-to-face therapy.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> Unknown <i>Comparison:</i> Unknown Method of analysis: Unclear No information given on dropout at any stage of the trial, only that there were more participants in the self-administered condition because it usually had a higher attrition rate.
Selective reporting (reporting bias)	High risk	No trial registration number given. Data could not be extracted. The paper reports only significant correlations.
Other bias	Unclear risk	Methods are poorly reported and the conduct of the study cannot be fully assessed.

Botella 2010

Methods

Location: Castelló, Spain

Recruitment: Advertising and referral

Exclusion rate: 27.84%

Randomised (N): 127 (62 *intervention*; 29, 36 *comparisons*)

Participants

Disorder: Social Anxiety (Unspecified); ADIS (DSM-IV)

Duration (years): Unknown

Age (years): 24.4

Sex (female): 79.22%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): 14.82

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: N/A

Drug/Alcohol: "not be diagnosed for substance abuse or dependence"

Physical: N/A

Interventions

Comparison group(s): Wait-List, Individual CBT

Comparison description: For two months, "Participants in the live therapy condition received therapy once or twice a week, depending on their availability. Treatment included individual sessions lasting 45 to 60 minutes. This treatment featured the same components as Talk to Me: education, cognitive therapy, and exposure." "wait-list control group were assessed when they asked for help and 1 month afterwards when they had the chance to be treated"

Intervention name: Talk to Me

Days of treatment: 61

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Botella 2010 (Continued)

Contact description: “Participants in the self- administered condition did not have any contact with the psychologist during the treatment, although they could e-mail or telephone her if they had any problem with the system.”

Intervention description: “information about the nature and physiological characteristics of anxiety, fear, and phobias. The cognitive restructuring component teaches users to identify and challenge negative thoughts...the program organizes a hierarchy of public speaking scenarios that were created during the initial assessment...When their level of fear has diminished 2 or more points, Talk to Me presents them with a new situation.”

Compliance: N/A

Outcomes	Specific Symptoms: Brief Fear of Negative Evaluation (BFNE) Behaviour Test (Level): Impromptu Speech Task: Performance Behaviour Test (Symptoms): Impromptu Speech Task: Anxiety; Impromptu Speech Task: Anxiety, assessor rated Recovery: No longer met criteria for diagnosis Depression: Beck Depression Inventory (BDI) Disability: Maladjustment Scale (MS)
Notes	Funding: Ministerio de Educacion y Ciencia, Spain, PROYECTOS CONSOLIDER-C (SEJ2006-14301/PSIC), by Programa de Acciones Integradas con Sudafrica (HS2006-0001), and by Universitat Jaume I - Fundacio Caixa Castello (P1 1A2005-06)

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “One hundred twenty-seven participants were randomly assigned to one of the following experimental conditions”
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Participants in the self- administered condition did not have any contact with the psychologist during the treatment, although they could e-mail or telephone her if they had any problem with the system. Another group received face-to-face therapy.
Blinding of outcome assessment (detection bias) All outcomes	Unclear risk	Included assessor-rated outcomes (blinding unclear) The participant reports his or her level of fear during the task, and the therapist also assesses it on a scale of 0, no fear at all, to 10, severe fear). "The therapist assesses the global severity of the participant on a 6-point scale..."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison (wait-list):</i> N=0 (0%) <i>Comparison (individual CBT):</i> N=0 (0%)

Botella 2010 (Continued)

Method of analysis: LOCF

"We have postbaseline data of 35 of the 50 dropouts...We used the last rating as a postbaseline for those participants...In the 15 cases where the postbaseline data were unavailable because they dropped out right after randomization, baseline data were carried forward..." "...obligations like university exams and having not enough time to dedicate to the treatment. Some participants also dropped out because they had doubts about the efficacy of the program, and some because they felt better after a few sessions." Global Impression may be an Available Case analysis.

Selective reporting (reporting bias)	High risk	No trial registration number given. Dichotomous data not reported for the control group. Data for the "Maladjustment Scale (MS): Work" and "Maladjustment Scale (MS): Social" appear to be incorrect - the values are identical.
Other bias	Unclear risk	Methods are poorly reported and the conduct of the study cannot be fully assessed.

Bowler 2012

Methods	Location: East Anglia, England Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 71 (24 intervention; 22 comparison; 25 other)
Participants	Disorder: Social Anxiety (unspecified); Questionnaire only Duration (years): Unknown Age (years): 21.91 Sex (female): 64.29% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current or previous restricted Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-list Intervention name: E-couch Days of treatment: 14

Bowler 2012 (Continued)

Author wrote: No

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=4

Contact description: "A researcher ensured participant attendance and general compliance with the module."

Intervention description: "...accessed the e-couch social anxiety program online over four sessions in the laboratory. A researcher ensured participant attendance and general compliance with the module...psychoeducational subsection, which contained information on the nature of social anxiety... 'exposure practice,' 'modifying your thinking,' 'attention practice,' and 'social skills training.'...During the course, participants completed two pieces of homework concerning exposure practice and social skills practice, respectively."

Compliance: "A researcher ensured participant attendance and general compliance with the module."

Outcomes	Specific Symptoms: Fear of Negative Evaluation (FNE); Social Phobia Inventory (SPIN) General Symptoms: State Trait Anxiety Inventory (STAI) - Trait Depression: Beck Depression Inventory (BDI)
Notes	Funding: Wellcome Trust Grant 083475

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	"Randomization was subject to the restriction that group sizes were approximately equal."
Allocation concealment (selection bias)	Unclear risk	"Randomization was subject to the restriction that group sizes were approximately equal."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "A researcher ensured participant attendance and general compliance with the module."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N= 3 (12.5%) <i>Comparison:</i> N= 2 (9.1%) Method of analysis: Per protocol "A complete-case analysis approach, where only participants with all data points complete are included, was the primary approach used to analyze the

Bowler 2012 (Continued)

data due to the small number of cases who dropped out (see Figure 1) and because it was reasonable to assume that the data were missing at random (Little & Rubin, 1987). In addition, for the symptom outcomes we also report intent-to-treat (ITT) analyses, and the pattern of results was unaltered." "Two participants (one in the cCBT group and one in the control group) were excluded from the analysis because they did not follow instructions correctly."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Bowman 1997

Methods	Location: Birmingham, AL, USA Recruitment: Advertising/ Internet Exclusion rate: 38.71% Randomised (N): 38 (19 intervention; 19 comparison)
Participants	Disorder: Generalised Anxiety Disorder; ADIS (DSM-III R) Duration (years): Unknown Age (years): 42.91 Sex (female): 73.68% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "not receiving pharmacotherapy unless stabilized on medication that one had taken for at least 2 months" Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: Self-Examination Therapy Days of treatment: 28 Author wrote: Yes Type of media: Book / Booklet

Bowman 1997 (Continued)

Type of therapy: SET

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=4 (20min)

Contact description: "Daniel Bowman called participants once each week to make sure that no one needed to be referred to a mental health professional for evidencing psychosis, suicidal risk, homicidal risk, or mania...During the calls, he also offered to clarify any questions about SET, but no therapy was provided. Calls were restricted to approximately 5 min."

Intervention description: "The booklet contained 28 identical worksheets. We encouraged participants to use a worksheet each day. On the worksheets, participants recorded what mattered to them. They also recorded any problems they were having. Then they compared what was bothering them with what mattered. If what was bothering them did not relate to what mattered, they crossed it out and left the problem alone..."

Compliance: "To be included in this study, participants receiving SET were required to complete at least seven worksheets in the SET booklet. All participants who received SET reported meeting the inclusion criterion." "Before treatment, participants averaged answering 2.54 out of 7...After completing treatment, participants averaged answering 5.16 out of 7 questions correctly..."

Outcomes	General Symptoms: Hamilton Anxiety Rating Scale, clinician-rated (HAM-A)	
Notes	Funding: None acknowledged Unpublished data: Authors confirmed that an equal number of participants were assigned to each group.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “randomly assigned to either the SET or the delayed-treatment group.”
Allocation concealment (selection bias)	Unclear risk	"Immediately after the screening, one of three trained clinicians used the HARS-R to assess participants who met the inclusion criteria. The clinicians were unaware of the treatment condition to which each participant was assigned. After the HARS-R interview, participants completed the SCL-90-R, STAI, and SET quiz and were randomly assigned..."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Daniel Bowman called participants once each week"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “Each participant spent 4 weeks in a treatment condition and then was re-assessed on the HARS-R by a clinician who was unaware of the participant's treatment condition.”

Bowman 1997 (Continued)

Incomplete outcome data (attrition bias)
All outcomes

Low risk

Not analysed at post-treatment

Intervention: N=2 (10.53%)

Comparison: N=1 (5.26%)

Method of analysis: Available Case

“Two participants from the SET group and 1 from the delayed-treatment group did not return for the second assessment. One participant moved, 1 did not return our calls, and 1 was excluded from the study because she decided to start psychotherapy.”

Selective reporting (reporting bias)

Unclear risk

No trial registration number given.

Other bias

Low risk

N/A

Carlbring 2001

Methods

Location: Uppsala, Sweden

Recruitment: Advertising/ Internet

Exclusion rate: 91.8%

Randomised (N): 41 (21 intervention; 20 comparison)

Participants

Disorder: Panic disorder; ADIS (DSM-IV)

Duration (years): Unknown

Age (years): 34

Sex (female): 70.73%

Race (white): 64%

Medication (using at baseline): 64%

Education (years): Unknown

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "if on prescribed drugs for panic disorder, (a) the dosage had to be constant for 3 months before the start of the treatment, and (b) the patient had to agree to keep the dosage constant throughout the study"

Drug/Alcohol: N/A

Physical: "no epilepsy, kidney problems, strokes, organic brain syndrome, emphysema, heart disorders, or chronic high blood pressure."

Interventions

Comparison group(s): Wait-List

Carlbring 2001 (Continued)

Comparison description: Study homepage included general information about CBT.

Intervention name: An End to Panic: Breakthrough Techniques for Overcoming Panic Disorder (Zuercher-White, 1998). 20% was Mastery of Your Anxiety and Panic II (Barlow and Craske, 1994), Overcoming Panic: A Complete Nine-Week Home-based Treatment Program for Panic Disorder

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=7.5 (90min)

Contact description: "The mean total time spent on each participant was approximately 90 minutes, including assessment, administration, and responding to the e-mails. The number of reciprocal contacts (receive and send) ranged from 6 to 15 (M = 7.5, SD = 1.2)." "Each module ended with five to eight questions...Individual feedback was given within 24 hours...an assessment was made to judge whether the participant was ready to continue; if so, the password to the next module was sent."

Intervention description: "...six modules...psychoeducation...Clark's cognitive model (Clark, 1986)...breathing retraining...reveal catastrophic interpretations of physical symptoms and then to produce alternative interpretations... interoceptive exposure... exposure in vivo... assertiveness skills and how to deal with setbacks."

Compliance: All participants, except the dropouts, managed to complete all modules. Extensions permitted for business trips and illness.

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Beck Anxiety Inventory (BAI) Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)
Notes	Funding: Swedish Foundation for Health Care Sciences and Allergy Research, the Boëthius Foundation, the Swedish Council for Research in the Humanities and Social Sciences, and the Swedish Medical Council Unpublished data: Carlbring provided details about randomisation, demographic measures, and number assigned to each group.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "Participants were divided into two groups by the drawing of lots. These were drawn for the two treatment groupings pairwise for participants who had completed their baseline measurements. In other words, as soon as two partici-

Carlbring 2001 (Continued)

pants had completed their baseline measurements, one was allocated to the treatment group and the other to the waiting-list group.”		
Allocation concealment (selection bias)	Low risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Individual feedback was given within 24 hours... an assessment was made to judge whether the participant was ready to continue”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF “In the treatment group, lack of time was given as the main reason for discontinuing (n = 3). One patient dropped out because of a newly discovered cancer. The person who left the waiting-list group gave no reason. "In the statistical analysis, preassessment data for applicants who were allocated to one of the two study groupings but who did not complete the treatment were brought forward and used as postassessment data. This was done on the basis of an intention-to-treat evaluation of the results, which is a more conservative approach compared to only including completers of treatment.”
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Carlbring 2003

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 63.33% Randomised (N): 22 (11 intervention; 11 comparison)
Participants	Disorder: Panic disorder; SCID (DSM-IV) Duration (years): 10.4 Age (years): 37.95 Sex (female): 68.18% Race (white): 99.9%

Carlbring 2003 (Continued)

Medication (using at baseline): 99.9%

Education (years): Unknown

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "if on prescribed drugs for PD, (a) the dosage had to be constant for 3 months before the start of the treatment, and (b) the patient had to agree to keep the dosage constant throughout the study"

Drug/Alcohol: N/A

Physical: "no epilepsy, kidney problems, strokes, organic brain syndrome, emphysema, heart disorders, or chronic high blood pressure."

Interventions

Comparison group(s): Applied relaxation (self-help)

Comparison description: In the AR group Ost's applied relaxation book(Ost, 1997) was adapted for self-help use via the WWW. A compact disc (CD) with three relaxation instructions was also sent to the participants. The treatment was divided into 9 modules: (1) psychoeducation, (2) rational, (3) progressive muscle relaxation: long version, (4) progressive muscle relaxation: short version, (5) conditioned relaxation, (6) differential relaxation, (7) quick relaxation, (8) applied relaxation, and (9) relapse prevention.

Intervention name: An End to Panic: Breakthrough Techniques for Overcoming Panic Disorder (Zuercher-White, 1998). 20% was Mastery of Your Anxiety and Panic II (Barlow and Craske, 1994), Overcoming Panic: A Complete Nine-Week Home-based Treatment Program for Panic Disorder

Days of treatment: 183

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & SMS

Automated contact: N=0

Provider contact: N=7 (30min)

Contact description: An e-mail message to inquire how work with the current module was progressing was sent out once... "Participants had to answer the questions at the end of each module before they received the password to the next module. The mean total time spent by the therapist on each participant was approximately 30 min, including administration, and responding to the e-mails. Most answers were fully standardized. Often only the participants name was altered in the greeting phrase of the e-mails."

Intervention description: The main treatment component in the CBT group was a self-help manual that was adapted for use via the WWW, and to be suitable for Swedish conditions. It consisted of 197 pages of text and exercises divided into 6 modules: psychoeducation, breathing retraining, cognitive restructuring, interoceptive exposure, exposure in vivo, relapse prevention and assertiveness training (for details see Carlbring et al., 2001).

Carlbring 2003 (Continued)

Compliance: The intervention group completed 56.1% (17.1) of modules, the control group completed 56.6% (30.0).

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Beck Anxiety Inventory (BAI) Response: Panic Free Depression: Beck Depression Inventory (BDI) Quality of Life: Quality of Life Inventory (QOLI)
Notes	Funding: Swedish Foundation for Health Care Sciences and Allergy Research, the Boëthius Foundation, the Swedish Council for Research in the Humanities and Social Sciences, and the Söderström-Köniska Foundation Unpublished data: Carlbring provided details about randomisation and demographic measures.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online Participants were divided into two groups by a true random number service (http://www.random.org).
Allocation concealment (selection bias)	Low risk	We thank Per Forslin and Carola Strandlund for the SCI interviews. "When it had been decided on what people to include all participants were randomized by a third party" (personal communication).
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Standardized feedback was given within 7 days of the participants sending their answers via e-mail. On the basis of these e-mails, an assessment was made of whether the participant was ready to continue, if so, the password to the next module was sent.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF After randomization, five people dropped out during the course of the study. There were three dropouts from the CBT group and two from the AR group. Lack of time was given as the main reason for discontinuing. In the statistical analysis, preassessment data for applicants who were allocated to one of the two study groupings but who did not complete the treatment were brought forward and used as postassessment data (n=5).

Carlbring 2003 (Continued)

Selective reporting (re-reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Carlbring 2005

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 88.52% Randomised (N): 49 (25 intervention; 24 comparison)
Participants	Disorder: Panic disorder; SCID (DSM-IV) Duration (years): 9 Age (years): 34.98 Sex (female): 71.43% Race (white): 51% Medication (using at baseline): 51% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "if on prescribed drugs for PD: (a) the dosage had to be constant for 3 months before the start of the treatment and (b) the patient had to agree to keep the dosage constant throughout the study" Drug/Alcohol: N/A Physical: "to rule out general medical conditions the participant must have had a previous contact with a physician, psychologist, or other health professional as a consequence of panic attacks; no epilepsy, kidney problems, strokes, organic brain syndrome, emphysema, or heart disorders."
Interventions	Comparison group(s): Individual CBT Comparison description: Participants in the live therapy condition received 10 weekly individual sessions lasting 45–60 min. Between each session the participant was expected to do homework, which included reading a module handout identical to the text in the IT condition. Furthermore, each session was tape recorded and the participant was instructed to listen to it after the sessions to consolidate learning (Clark,1989). Intervention name: (Similar to Carlbring 2001.) Days of treatment: 70 Author wrote: Yes

Carlbring 2005 (Continued)

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=15.4 (150min)

Contact description: "All contact was exclusively via e-mail. The participants were encouraged to come up with questions or reflections during treatment, and they were free to send an unlimited number of e-mails. The total number of reciprocal contacts (receive and send) ranged from 4 to 31 (M=15.4; SD=5.5). As the e-mail response to the participants often were very similar much text could be recycled. The mean total time spent on each participant was approximately 150 min, including administration, and responding to the e-mails."

Intervention description: The treatment was manualized and divided into 10 modules: (1–2) psychoeducation and socialization, (3) breathing retraining and hyperventilation test, (4–5) cognitive restructuring, (6–7) interoceptive exposure, (8–9) exposure in vivo, and finally (10) relapse prevention and assertiveness training. Each module consisted of approximately 25 pages, and was in large part similar to the previously tested self-help material by Carlbring et al. (2001).

Compliance: The intervention and control groups completed 7.4 (2.2) and 9.0 (2.7) modules (See Table 1). 28% intervention group finished all modules; 88% of comparison completed all sessions. Sessions completed did not correlate to change on any measure. "each module ended with an interactive multiple-choice quiz that the participants needed to get 100% correct in order to proceed"

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Beck Anxiety Inventory (BAI) Response: Agoraphobic Cognitions Questionnaire (ACQ); Body Sensations Questionnaire (BSQ); Mobility Inventory for Agoraphobia (MIA) Recovery: No longer met criteria for diagnosis Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Swedish Foundation for Health Care Sciences and Allergy Research, the Boëthius Foundation, the Swedish Council for Research in the Humanities and Social Sciences, and the Soderstrom Konigska Foundation. Unpublished data: Carlbring provided details about randomisation and demographic measures.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Participants were divided into two groups, live therapy (LIVE) or Internet-based (IT) by a true random-number-service (http://www.random.org)."
Allocation concealment (selection bias)	Low risk	We thank Anders Wiberg for conducting nine SCID-interviews and Katharina Johansson for the CBT-supervision. "When it had been decided on what people to include all participants were randomized by a third party" (personal communication).

Carlbring 2005 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "On the basis of these e-mails, a subjective assessment was made by the therapist of whether the participant was ready to continue..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "To determine the clinical significance of the treatment a clinical reinterview (SCID) was administered by an independent psychologist blind for treatment condition. This was done 1 month after the treatment ended, and at a 1-year follow-up. To ensure the reliability of the diagnosis made, 10% of the tapes was randomly selected and reassessed by an independent psychiatrist with extensive SCID training."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "Lack of time was given as the main reason for discontinuing. However, in accordance with the intention to treat paradigm (e.g., Nich & Carroll, 2002; Newell, 1992) post-treatment data were collected from all dropouts. Six participants did not return their follow-up questionnaires, and their post-treatment scores were carried forward to the follow-up assessment point. Hence, all 49 participants that were randomized to one of the two conditions are included in the statistical analysis."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Carlbring 2006a

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 83.24% Randomised (N): 60 (30 intervention; 30 comparison)
Participants	Disorder: Panic disorder; SCID (DSM-IV) Duration (years): Unknown Age (years): 36.7 Sex (female): 60% Race (white): 54% Medication (using at baseline): 54% Education (years): 13.09

Carlbring 2006a (Continued)

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "If the participant was taking prescribed drugs for panic disorder, a) the dosage had to be constant for 3 months before starting treatment, and b) the participant had to agree to keep the dosage constant throughout the study"

Drug/Alcohol: N/A

Physical: "To rule out general medical conditions, the participant must have consulted a physician, psychologist, or other health professional"

Interventions

Comparison group(s): Wait-List

Intervention name: Similar to Carlbring 2001.

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=0

Provider contact: N=23.5 (238min)

Contact description: "The total number of reciprocal contacts (received and sent) ranged from seven to 29 (mean=13.5, SD=4.4)...The mean time spent on each participant per week was approximately 12 minutes, including administration and responding to e-mail. Weekly telephone calls were...timed and lasted an average of 11.8 minutes per week (range=9.6–15.6)." "In addition to the written material, all participants received one telephone call per week by the same therapist, each lasting, on average, 17.8 min (SD=4.2). The median total telephone time was 188 min." Data about contact from 2006 and 2011 paper are contradictory.

Intervention description: received either the full book at the start of the treatment or one module per week during the 10 weeks "modules 1–2, psychoeducation and socialization; module 3, breathing re-training and hyperventilation test; modules 4–5, cognitive restructuring; modules 6–7, interoceptive exposure; modules 8–9, exposure in vivo; and finally, module 10, relapse prevention and assertiveness training. Each module consisted of approximately 25 pages of written text...converted into interactive web pages..."

Compliance: "The questions were included to promote learning and to enable the research supervisors to assess whether the participants had understood the material and completed their homework." "Of 30 participants, 24 (80%) finished all of the modules within the intended 10-week time limit. Hence, the average number of modules completed in this study was nine (mean=8.9, SD=2.6)."

Outcomes

Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ)

General Symptoms: Beck Anxiety Inventory (BAI)

Recovery: No longer met criteria for diagnosis

Carlbring 2006a (Continued)

Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS)

Quality of Life: Quality of Life Inventory (QOLI)

Notes

Funding: Swedish Foundation for Health Care Sciences and Allergy Research, the Boëthius Foundation, Swedish Research Council and the Soderstrom Konigska Foundation

Unpublished data: Carlbring provided details about randomisation and demographic measures.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “The participants were divided into two groups, treatment or a waiting list, by a true random-number service.” (www.random.org)
Allocation concealment (selection bias)	Low risk	“The authors thank Tommy Maurin and Josefin Nilsson for conducting the blind clinical telephone interviews.” “When it had been decided on what people to include all participants were randomized by a third party” (personal communication).
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Weekly telephone calls were made by the therapists to each participant. The purpose was to provide positive feedback and answer questions about the modules.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “The clinical significance of the treatment was assessed in two ways: one with a statistical method, the other with clinical judgment in a reinterview administered by an independent psychologist who was blind to treatment condition.”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF “posttreatment data were also collected from the participant who dropped out. Two participants in the treatment condition and one on the waiting list did not return their posttreatment questionnaires. Therefore, their pretreatment scores were carried forward to the posttreatment assessment point. Hence, all 60 participants who were randomly assigned to one of the two conditions were included in the statistical analysis.”
Selective reporting (reporting bias)	Unclear risk	No trial registration number given. Anxiety diary reported in other papers but does not appear here.
Other bias	Unclear risk	N/A

Carlbring 2007

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 75.31% Randomised (N): 60 (30 intervention; 30 comparison)
Participants	Disorder: Social Anxiety (Unspecified); SCID (DSM-IV R) Duration (years): Unknown Age (years): 32.65 Sex (female): 61.67% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 13.75 EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: "if taking prescribed drugs for anxiety or depression, the dosage had to be constant for 3 months before the start of the treatment, and the participants had to agree to keep the dosage constant throughout the study" Drug/Alcohol: "... or substance misuse." Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: Social Phobia-Effective Treatment with Cognitive-Behavioural Therapy (Furmark et al. 2006) Days of treatment: 63 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Email & Phone Automated contact: N=0 Provider contact: N=19 (198min) Contact description: One weekly telephone call was made by the therapists to each participant in the treatment group. "Feedback on homework assignments was usually given within 24h after participants had sent their answers by email." "The mean total time per week spent on each participant

Carlbring 2007 (Continued)

was approximately 22 min, including telephone calls, administration, and reading and responding to emails. Hence, the total human contact time per participant including screening was over 2.5 h."

Intervention description: "based on established cognitive-behavioural methods as described in self-help books (e.g. Rapee, 1998; Antony & Swinson, 2000). The text, consisting of 186 pages, was taken from an existing manual (Furmark et al, 2006), divided into nine modules...included information, exercises and an interactive quiz, and ended with three to eight essay questions. Participants were asked to explain in their own words the most important sections of the module they had just completed, provide thought records..."

Compliance: "The questions were intended to promote learning and to enable the online therapists to assess whether the participants had assimilated the material. For each module participants were required to post at least one message in an online discussion group about a predetermined topic." "A total of 27 of the 29 people in the treatment group completed all nine modules within the intended 9-week time frame. Lack of time was provided as the explanation for terminating treatment prematurely."

Outcomes	Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR): Avoidance General Symptoms: Beck Anxiety Inventory (BAI) Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Swedish Research Council and the Soderstrom Konigska Foundation Unpublished data: Carlbring provided details about randomisation, demographic measures, recruitment, and contact with participants.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “divided into two groups (treatment or waiting-list control) by an online true random-number service...”
Allocation concealment (selection bias)	Low risk	"independent of the investigators and therapists. This service is run by the Department of Computer Science at the University of Dublin..." "When it had been decided on what people to include all participants were randomized by a third party" (personal communication).
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “One weekly telephone call was made by the therapists to each participant in the treatment group. The purpose was to provide positive feedback and to answer any questions the participant might have regarding the modules.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (6.67%) <i>Comparison:</i> N=2 (6.67%)

Carlbring 2007 (Continued)

Method of analysis: Available Case

"Prior to completing baseline measures, one control participant was excluded for lack of access to a computer. One participant in each condition was excluded because they started another treatment, and they do not appear in the analyses. One treatment-completer did not provide post-treatment measures." "A total of 27 of the 29 people in the treatment group completed all nine modules within the intended 9-week time frame. Lack of time was provided as the explanation for terminating treatment prematurely. One of them did not send in post-treatment measures, which explains why intention-to-treat analysis was used."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Unclear risk	N/A

Carlbring 2011

Methods	Location: Umeå, Sweden Recruitment: Advertising/ Internet Exclusion rate: 60.58% Randomised (N): 54 (27 intervention; 27 comparison)
Participants	Disorder: Mixed (any anxiety disorder); SCID (DSM-IV) Duration (years): Unknown Age (years): 38.8 Sex (female): 75.93% Race (white): 25.93% Medication (using at baseline): 25.93% Education (years): 13.43 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "taking no current medication for psychological problems or, if on medication, have an unchanged dosage during the last three months." Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Attention/Monitoring Comparison description: "invited to join a new confidential online support group specifically targeting anxiety problems. Each participant was asked to make at least one posting a week about a pre-determined topic...presented by a therapist every eighth day, and participants were encouraged to com-

Carlbring 2011 (Continued)

ment on one another's postings. The mean number of postings per participant, per topic were $M=1.23$ ($SD=0.42$) and total therapist time for writing and monitoring the online discussion area was approximately 1 h a week."

Intervention name: Not named

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=10 (150min)

Contact description: "Eight clinical psychology M.Sc. students...responsible for 7-10 participants...All contact with the participants, once they had been interviewed and included in the study, was via e-mail. "If the therapist had not heard from the participant after a week, a reminder was sent by e-mail." "Individual feedback was usually given within 24 h...an assessment was made to judge whether the participant was ready to continue...total time per week spent on each participant in this study was approximately 15 min, including administration as well as reading and responding to e-mails."

Intervention description: "combination of previous Internet-based programs...All of these were, in turn, based on established CBT methods. From a total of 16 different modules, each participant was prescribed 6 to 10 modules to work with during the 10 week treatment period. First (intro) and last (relapse prevention) modules fixed. Others included cognitive restructuring (2), SAD (2), GAD (3), PD (2), AG, behavioural activation (2), applied relaxation, and sleep." "modules were available for download in PDF format."

Compliance: "On average, participants were prescribed $M=8.86$ ($SD=1.20$, range 6-10) modules, and typically completed $M=7.96$ modules ($SD=2.37$, range 2-10) during the ten weeks of treatment. Fifty nine per cent of the participants finished all prescribed modules within the 10 week time limit."

Outcomes	General Symptoms: Beck Anxiety Inventory (BAI) Response: Clinical Global Impression (CGI): Improvement Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Swedish Council for Working Life and Social Research	
<i>Risk of bias</i>		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “by an online true random-number service...”
Allocation concealment (selection bias)	Low risk	"...independent of the investigators and therapists."

Carlbring 2011 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "on the basis of these emails an assessment was made to judge whether the participant was ready to continue. If not, participant received instructions on what needed to be completed to be able to get to the next step..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "after a telephone interview by a blind assessor who had no earlier contact with the participants and no knowledge of to which group they had been randomly allocated."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "Since the missing data at post-treatment was only in the treatment group, repeated ANOVAs with conservative imputation according to the last observation-carried-forward method in case of missing data was used in the analysis of the immediate results."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Chung 2008

Methods	Location: Seoul, South Korea Recruitment: Referral Exclusion rate: Unknown Randomised (N): 45 (27 intervention; 15 comparison)
Participants	Disorder: Social Anxiety (Unspecified); ADIS (DSM-IV) Duration (years): Unknown Age (years): 26.36 Sex (female): 39.5% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 15.03 EXCLUSIONS

Chung 2008 (Continued)

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: Too much shyness: A cognitive therapy for social phobia (Kwon, Cho, and Lee, 1998)

Days of treatment: 42

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Home/ Anywhere

Type of contact: Email

Automated contact: N= 0

Provider contact: N=6 (duration unknown)

Contact description: Group 1: "did their homework weekly, which was based on the contents of the book, and received feedback about their homework via e-mail from the first author...Moreover, the BT 1 group could con- tact the therapist via e-mail whenever they had a question about the homework." Group 2: "just read the book for about 6 weeks without any feed- backs and were then debriefed with a semi- structured interview, in order to determine if they had correctly understood the content of the book at the end of the BT."

Intervention description: "information about social phobia...how to monitor social anxiety and record events in the Social Anxiety Record Form... identifying cognitive biases (e.g., catastrophic thinking) and changing dysfunctional beliefs...identify one's underlying core beliefs and modify them... exposure techniques."

Compliance: "did their homework weekly, which was based on the contents of the book, and received feedback about their homework via e-mail from the first author."

Outcomes

Specific Symptoms: Social Interaction Anxiety Scale (SIAS); Social Phobia Scale (SPS)

Response: Social Interaction Anxiety Scale (SIAS)

Notes

Funding: Korea Science and Engineering Foundation funded by the Korean government (Ministry of Education, Science, & Technology) (No. R01-2008-000-11782-0)

Risk of bias

Bias

Authors' judgement

Support for judgement

Random sequence genera-
tion (selection bias)

Unclear risk

Method: Unclear

"participants were randomly assigned"

Chung 2008 (Continued)

Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Provider contact: Yes Provider blind: No "received feedback about their homework via e-mail from the first author"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (11.11%) <i>Comparison:</i> N=4 (26.67%) Method of analysis: Available Case "the remaining 7 participants (two in BT 1, one in BT 2, and four in WL) dropped out"
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Donnan 1990

Methods	Location: Northumberland, England Recruitment: Referral Exclusion rate: Unknown Randomised (N): 103 (51 intervention; 52 comparison)
Participants	Disorder: Mixed (not specified); Unclear Duration (years): Unknown Age (years): 41.49 Sex (female): 74.26% Race (white): 58.25% Medication (using at baseline): 58.25% Education (years): Unknown EXCLUSIONS Depression: No

Donnan 1990 (Continued)

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Treatment-as-usual Intervention name: Not named Days of treatment: 42 Author wrote: Yes Type of media: Book / Booklet Type of therapy: Relaxation Location used: Anywhere Type of contact: None Automated contact: N=0 Provider contact: Unknown Contact description: No information was collected about the frequency of consultation. The initial questionnaire was followed by postal questionnaires at six weeks and three months. Intervention description: printed illustrated booklet (27 pages, approximately 4000 words) Psychoeducation, "...stopping anxiety developing...ways in which anxiety can be coped with better, including understanding the problem, dealing with the causes of the anxiety where possible, using relaxation, coping with worry, planning better coping, having realistic expectations, letting other people help, and dealing with panic." Relaxation tape. Compliance: N/A
Outcomes	None
Notes	Funding: None acknowledged

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "The self-help package was evaluated using a randomized controlled trial...Forty general practitioners in Northumberland agreed to recruit up to six patients each, randomly selecting three for the intervention group and three for the control group."
Allocation concealment (selection bias)	High risk	"Each patient received an envelope at the end of the consultation with their general practitioner, which at random contained either the self-help materials and a questionnaire or the questionnaire alone. The envelopes were similar but those containing the self-help material were heavier."

Donnan 1990 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Each patient received an envelope at the end of the consultation with their general practitioner, which at random contained either the self-help materials and a questionnaire or the questionnaire alone. The envelopes were similar but those containing the self-help material were heavier."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=18 (35.29%) <i>Comparison:</i> N=9 (17.31%) Method of analysis: Available Case "Patients who dropped out of the intervention group had lower baseline scores than those who dropped out of the control group but there were no statistically significant differences. The characteristics of patients prior to loss to follow up at each time period were also examined and no statistically significant differences were found between the intervention and control groups."
Selective reporting (reporting bias)	High risk	No trial registration number given. No data could be extracted for analysis.
Other bias	High risk	The study is very poorly described.

Febbraro 1997a

Methods	Location: Virginia, West Virginia and Pennsylvania, USA Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 98 (50 intervention; 48 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-IV) Duration (years): 6.94 Age (years): 44.4 Sex (female): 74.6% Race (white): 65.1% Medication (using at baseline): 65.1% Education (years): 14.1

Febbraio 1997a (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: "Participants taking medication for anxiety or depression were allowed to participate provided they had been stabilised on the medication and dosage for at least four weeks and continued to experience full-blown or limited symptom attacks..."

Drug/Alcohol: "In addition, participants who were alcohol or substance dependent or had any type of psychotic disorder were also excluded from data analyses."

Physical: "seizure disorder, kidney disease, stroke...emphysema or heart problems."

Interventions	Comparison group(s): Wait-list, Monitoring alone Intervention name: Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks (G Clum; 1990) Days of treatment: 70 Author wrote: Yes Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: None Automated contact: N=0 Provider contact: N=0 (0 min) Contact description: All participants had been exposed to assessment and feedback as part of an earlier study. The amount of contact listed here is unique to this part of the study and thus underestimates contact with this research team over the course of several years. "There was no contact with researchers at pretreatment assessment unless the pretreatment measures were not received by the experimenter within 7 days of their mailing. In this case, participants were contacted by telephone to ascertain if they had received the measures." Intervention description: 1) Self-help alone, 2) Self-help & monitoring. Included "(a) educating individuals about the etiology and nature of panic disorder; (b) teaching individuals a variety of cognitive and behavioral strategies that include relaxation, cognitive restructuring, breathing retraining, and exposure...Advice on implementing coping strategies is provided and homework assignments are incorporated to encourage active practice of these techniques." Compliance: Treatment knowledge was defined as being able to successfully answer at least 7 of 10 questions about principles discussed in the book. All participants were able to answer at least 70% of questions about the booklet correctly.
Outcomes	Specific Symptoms: Panic Attack Cognitions Questionnaire (PACQ) General Symptoms: State-Trait Anxiety Inventory (STAI): State Response: Panic Free Depression: Beck Depression Inventory (BDI)
Notes	Funding: None acknowledged

Febbraro 1997a (Continued)

Unpublished data: Author provided details about study methods.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "Stratified assignment was utilized in order to ensure as much as possible that an equal number of participants from each condition in Phase 1 (i.e., assessment only, assessment with face-to-face feedback, assessment with mailed feedback, and wait-list) were assigned to each condition in the present study." "the best way to describe the method I used to assign participants to groups was to try and match groups as closely as possible" (personal communication)
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	Low risk	Provider contact: No Provider blind: N/A Contact appears to be for assessment only and by post.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=18 (36%) <i>Comparison:</i> N=17 (35.42%) Method of analysis: Per protocol 98 assigned; 63 completers (17 booklet; 15 booklet plus monitoring; 13 monitoring; 18 wait list). 31 dropouts (11 No longer interested, 5 Project taking too long, 2 Moved, 13 Unknown). "Four additional participants were removed from the study for various reasons as well (e.g., beginning or changing medications during treatment [n = 2], beginning therapy during treatment [n = 1], or experiencing heart problems [n = 1])." "In addition, participants who were alcohol or substance dependent or had any type of psychotic disorder were also excluded from data analyses."
Selective reporting (reporting bias)	Low risk	No trial registration number given. Thesis likely includes all outcomes.
Other bias	Low risk	N/A

Febbraro 1997b

Methods	As above.
Participants	

Febbraro 1997b (Continued)

Interventions

Outcomes

Notes

Risk of bias

Bias	Authors' judgement	Support for judgement
Allocation concealment (selection bias)	High risk	
Blinding of participants and personnel (performance bias) All outcomes	Low risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	High risk	
Selective reporting (reporting bias)	Low risk	
Other bias	Low risk	

Febbraro 2005

Methods	Location: Iowa, USA Recruitment: Advertising/ Internet Exclusion rate: 46.07% Randomised (N): 48 (32 intervention; 16 comparison)
Participants	Disorder: Panic Attacks; ADIS (DSM-IV) Duration (years): 8.66 Age (years): 47.1 Sex (female): 73.3% Race (white): 53.33% Medication (using at baseline): 53.33% Education (years): 14.6 EXCLUSIONS

Febbraio 2005 (Continued)

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Participants taking medications to cope with panic symptoms and/or other psychological problems needed to be stabilized on a particular dosage per their self-report for four weeks prior to beginning this study."

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: Phone contact alone: like the intervention group, participants in the control group received phone calls at weeks 2, 4, 6 and 8.

Intervention name: Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks (G Clum; 1990)

Days of treatment: 56

Author wrote: No

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=2 (30min)

Contact description: "approximately one hour of phone contact with the principal investigator during the pre-treatment assessment session...." Contact averaged; in one group "in addition to utilizing the book, participants were contacted via telephone at weeks 2, 4, 6, and 8...contact did not ever involve discussing how to implement coping strategies... If participants ever asked for such information, they would be referred to the book...The phone contact lasted no more than 15 minutes, as it was meant to be brief..."

Intervention description: Two groups. Both received book "(1) educating individuals about the etiology and nature of panic attacks and panic disorder; (2) teaching individuals a variety of cognitive and behavioral strategies that include relaxation, cognitive restructuring, breathing retraining, and exposure to panic-related situations; and (3) advising individuals on how to implement the above cognitive and behavioral strategies." One group received phone calls.

Compliance: In one group "To monitor compliance, participants were asked how they were doing in reading the book, whether they had any questions about what they had read, and whether they were using any of the coping strategies suggested."

Outcomes

Specific Symptoms: Panic Attack Cognitions Questionnaire (PACQ)

Response: Panic Free

Notes

Funding: None acknowledged

Unpublished data: Febbraio provided details of randomisation process, number assigned to each group, and number of dropouts per group. Did not provide data for one person excluded from analyses for protocol violations.

Febbraro 2005 (Continued)

Assumed equal allocation of 48 participants.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "participants meeting study inclusion criteria were randomly assigned to one of the following conditions..." "choosing group assignments out of a hat" (personal communication)
Allocation concealment (selection bias)	Low risk	See Sequence Generation. "briefly screened over the telephone by the principal investigator to determine eligibility... moved into the pre-treatment phase of the study."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "to monitor their compliance in reading the book, provide some support, and answer any questions about the study"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=14 (43.75%) <i>Comparison:</i> N=2 (12.5%) Method of analysis: Per protocol Of these 48 participants, 30 completed the study, and 18 dropped out for various reasons... "If participants began some other form of psychological treatment during the study, they were allowed to continue in the study, but their data was removed from the final analyses. There was only one participant who began therapy during the course of the study." Author reports "Given that only one participant began therapy during the study thus resulting in their exclusion from the analyses, it is not possible to conduct some of the additional analyses you suggest" (personal communication).
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Fletcher 2005

Methods	Location: Manchester, England Recruitment: Referral Exclusion rate: 91.71% Randomised (N): 30 (15 intervention; 15 comparison)
---------	---

Fletcher 2005 (Continued)

Participants

Disorder: Mixed (Mean HADS Anxiety score over the minimal clinical cutoff of 8 (average >10)); No diagnostic interview (questionnaire only)

Duration (years): Unknown

Age (years): 38.6

Sex (female): 76.67%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): 12.27

EXCLUSIONS

Depression: Yes

Psychotherapy: No restrictions reported

Medication: "Use of antidepressant medication was not an exclusion criterion, but data gathered about patient use of antidepressants was incomplete, mainly due to patient recall issues."

Drug/Alcohol: "not primary diagnosis of substance misuse"

Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: Kennedy and Lovell (2002). A Handy Guide to Managing Depression and Anxiety: what should I do?

Days of treatment: 84

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: Continued to receive treatment by a GP, but no support using the booklet.

Intervention description: Pocket sized. "...general advice about lifestyle, professional help and treatments available for anxiety and depression. Part two uses cognitive behavioural techniques...helps the reader to recognize thoughts, physical symptoms and behaviour and also to identify problems and set goals. The second part also teaches self-help interventions such as behavioural activation, relaxation, problem solving, exposure and cognitive therapy."

Compliance: "Process/satisfaction data were collected from the experimental group. There was a high level of satisfaction with the self-help, with all parts of the book being rated as useful or highly useful by over 50% of the patients (except for 'evaluating your progress' for which several patients gave feedback that 12 weeks was too soon to evaluate progress)."

Fletcher 2005 (Continued)

Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety Depression: Hospital Anxiety and Depression Scale (HADS): Depression	
Notes	Funding: None acknowledged. Authors wrote the booklet but do not derive income from it. Unpublished data: Fletcher (now Archer) provided SPSS file with anonymous data, but was unable to locate mid-point data.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical “The 30 patients were randomly allocated to one of two groups, self-help intervention (n=15) or waiting list control (n=15). Randomization was conducted using sealed plain opaque envelopes inside 15 of which was a slip of paper stating 'control group', whilst the other 15 contained a slip stating 'experimental group'. All the envelopes were equal in terms of size, design and weight.”
Allocation concealment (selection bias)	Low risk	“The 30 envelopes were then mixed up and given to the administrator of the Primary Care Mental Health Team, who randomly slotted an envelope into each of the 30 consecutively numbered study packs. The packs were then allocated by the principal investigator.”
Blinding of participants and personnel (performance bias) All outcomes	Low risk	Provider contact: No Provider blind: N/A “The patients were given a brief description of the book and advised to read it at their own pace.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF ""The main analysis used an independent samples t-test to detect significant differences between the intervention group and the control group on week 12 scores using intention-to-treat analysis, with missing data replaced by last observation carried forward (LOCF). Therefore, data collected at weeks 0 or 6 were used in the analyses when data at week 12 were not available." "The two significant differences found between completers and those lost-to-follow-up were (1) age, which was higher in the latter group (t=3.62; df, 28; p = .001), and (2) CORE total score, which was also higher (t=2.74; df, 28, p = .011)."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Unclear risk	N/A

Fraser 2001

Methods	Location: Tasmania, Australia Recruitment: Advertising/ Internet Exclusion rate: 41.18% Randomised (N): 30 (15 intervention; 15 comparison)
Participants	Disorder: Specific Phobia; CIDI (DSM-III R) Duration (years): Unknown Age (years): 32.55 Sex (female): 93.33% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: "not taking psychotropic medication" Drug/Alcohol: "alcohol or illicit drug abuse problem" Physical: N/A
Interventions	Comparison group(s): Self-help (Exposure) Comparison description: 3 sessions of the same programme. Intervention name: CAVE (Smith 1997) Days of treatment: 21 Author wrote: Yes Type of media: Computer Type of therapy: Exposure Location used: Clinic/ Surgery Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=6 (30min) Contact description: "The therapist stayed with the participant for the first five minutes of the treatment to answer any questions. No self-exposure homework instructions were given by the therapist, the program or by handout."

Fraser 2001 (Continued)

Intervention description: 6 sessions received twice weekly. "...assist an animated screen figure to overcome his/her fear of spiders...There are four levels on the program, which model exposure to a spider picture, a plastic spider, a dead spider and a live spider, in a household setting. An on-screen 'anxiety thermometer' models the screen figure's initial anxiety on approach of spider stimuli, and habituation with the accumulation of approach behaviours."

Compliance: "Process measures analysed were: (a) the time spent with a spider stimulus visible, on each level; (b) the number of enactments of vicarious touching of the spider at each level; and (c) the score rate per 5-minute epoch as a summary measure of overall vicarious exposure activity." "Mann-Whitney U tests showed no significant differences between the groups' final session on any variable." Homework activities 6-session versus 3-session: PT=2.3 (1.3) vs. 2.2 (1.9), FU=2.7 (1.5) vs. 2.3 (1.8).

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Global Phobia Behaviour Test (Level): Behavioural Approach Test: Level Behaviour Test (Symptoms): Behavioural Approach Test: Subjective Units of Distress Disability: Work and Adjustment Rating Scale (WARS)	
Notes	Funding: None acknowledged	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear “The participant was randomly allocated to one of the two treatment conditions...” See Allocation Concealment.
Allocation concealment (selection bias)	High risk	“Participants who did not complete the treatment program were replaced by new participants allocated to the same treatment group.”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “The therapist stayed with the participant for the first five minutes of the treatment to answer any questions. No self-exposure homework instructions were given by the therapist, the program or by handout.”
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) “A replication study with blind assessment and a control group is warranted.”
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (20%) <i>Comparison (other):</i> N=4 (26.67%) Method of analysis: Per protocol “Seven participants (three-session group n=4; six-session group n=3) did not complete the full treatment program, reportedly due to: work commitments (n=1); travel commitments (n=3); the participant’s opinion that they were cured (n=1); and because they felt the program was not helpful (n=1).”

Fraser 2001 (Continued)

Selective reporting (reporting bias)	Unclear risk	No trial registration number given. "Assessments were repeated immediately following the final treatment session (posttreatment) and four weeks after treatment (follow-up). These comprised BAT, SUDS, SPQ, FQ Main, FQ Global, PT Problem, PT Total, WARS Total, and HWQ."
Other bias	Low risk	"Participants who did not complete the treatment program were replaced by new participants allocated to the same treatment group"

Furmark 2009a

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 59.48% Randomised (N): 120 (80 intervention; 40 comparison)
Participants	Disorder: Social Anxiety (Unspecified); SCID (DSM-IV) Duration (years): Unknown Age (years): 36.13 Sex (female): 67.5% Race (white): 6.67% Medication (using at baseline): 6.67% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "if prescribed drugs for anxiety or depression, the dosage had to be constant for 3 months before the treatment onset and kept constant throughout the study" Drug/Alcohol: "substance misuse" Physical: N/A
Interventions	Comparison group(s): Wait-List Comparison description: "The waiting-list control (WLC) group controlled for time and repeated assessments during the initial 9-week period. Participants had no contact with each other or with the study team during their waiting period except for conditional reminders by email or SMS to complete the weekly assessment of the LSAS-SR." Intervention name: Social Fobi-Effektiv Hjalp med Kognitiv Beteendeterapi [Social Phobia - Effective Help with CBT] Days of treatment: 63

Furmark 2009a (Continued)

Author wrote: Yes

Type of media: Mixed

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=Unknown

Provider contact: N=4.5 (67.5min)

Contact description: "in all groups, some participants received neutral reminders, sent by email or SMS, to complete the LSAS-SR ICBT:" "15min per week giving email feedback to each participant"

Intervention description: 1) The "pure" bibliotherapy (Bib) arm (40/80 intervention) received the manual by post. 2) 186 pages divided into nine modules for use over the internet. "Modules were also sent by post on a weekly basis..." Psychoeducation, cognitive restructuring, exposure, attention training, social skills, relapse prevention. "had access to an online discussion forum...Discussions were surveyed but the study personnel did not take part in them."

Compliance: "each module...ended with a short quiz to check adherence" "in cases of missing data, brief, neutral reminder was sent 24h later by email, and if necessary followed by another reminder as autogenerated short text message" For Furmark 2009a and 2009b: completed modules 1 (100%), 2 (95%), 3 (92.5%), 4 (85%), 5 (80%), 6 (77.5%), 7 (72.5%), 8 (70%), 9 (62.5%).

Outcomes

Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR)

General Symptoms: Beck Anxiety Inventory (BAI)

Response: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR); Social Interaction Anxiety Scale (SIAS); Social Phobia Scale (SPS); Social Phobia Screening Questionnaire (SPSQ)

Depression: Montgomery and Asberg Depression Rating Scale (MADRS)

Quality of Life: Quality of Life Inventory (QOLI)

Notes

Funding: Swedish Research Council. "The manual was released as a self-help book for the Swedish market after completion of the study." T.F., A.H., E.S., P.Ca. and G.A. receive royalties for authoring the self-help book

Trial 1: Combined ICBT and Bib groups to form the intervention group. Averaged outcomes and treatment characteristics, including number and duration of contact. 5-year follow-up includes data only for the treatment group.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Randomisation was performed by an independent third party using an online true random-number service (www.random.org)."
Allocation concealment (selection bias)	Low risk	See "Sequence Generation"

Furmark 2009a (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "...giving email feedback to each participant. There were 13 internet therapists in the study, of whom 6 were licensed clinical psychologists and 7 were clinical psychology students in their last semester of the 5-year master's degree programme. The students had clinical supervision during the trial. Participants were randomly allocated to their therapist."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "one each from the pure bibliotherapy and waiting-list groups, withdrew immediately after randomisation because of personal reasons and one additional participant (ICBT group) did not provide post-treatment data. The waiting-list control group received delayed treatment after 9 weeks (see below) but 3 individuals withdrew..." "missing data were replaced by the last obtained score (pre- or post-treatment), i.e. last observation carried forward." "all participants were asked to complete post-treatment and follow-up assessments, regardless of how many treatment modules they had completed."
Selective reporting (reporting bias)	Low risk	Trial registration number: NCT01145690 All outcomes reported.
Other bias	Low risk	N/A

Furmark 2009b

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 59.48% Randomised (N): 115 (86 intervention; 29 comparison)
Participants	Disorder: Social Anxiety (Unspecified); SCID (DSM-IV) Duration (years): Unknown Age (years): 34.7 Sex (female): 67.83% Race (white): 13.91% Medication (using at baseline): 13.91%

Furmark 2009b (Continued)

Education (years): Unknown

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "if prescribed drugs for anxiety or depression, the dosage had to be constant for 3 months before the treatment onset and kept constant throughout the study"

Drug/Alcohol: "substance misuse"

Physical: N/A

Interventions
Comparison group(s): Applied relaxation (self-help)

Comparison description: "Participants in the internet-delivered applied relaxation (IAR) group followed an applied relaxation manual, which had been adapted for self-help use via the internet and evaluated (e.g. for panic disorder)...The structure of the manual was the same as in the ICBT and Bib groups, i.e. the text was divided into nine modules containing information and exercises, and participants completed one module per week." "had access to an internet therapist during the 9-week treatment period."

Intervention name: Social Fobi-Effektiv Hjalp med Kognitiv Beteendeterapi [Social Phobia - Effective Help with CBT]

Days of treatment: 63

Author wrote: Yes

Type of media: Mixed

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=Unknown

Provider contact: N=3 (45min)

Contact description: "in all groups, some participants received neutral reminders, sent by email or SMS, to complete the LSAS-SR." One third "had no contact with the study team except for the usual on-line assessments before, immediately after, and 1 year after treatment" One third "had access to an internet therapist during the 9-week treatment period." "Email correspondence occurred weekly...instruction on what needed to be completed to proceed to the next module...therapists spent approximately 15 min per week..."

Intervention description: 1 AND 2) Two bibliotherapy groups received the manual by post. 57/86 had online discussion group. "had access to an online discussion forum...Discussions were surveyed but the study personnel did not take part in them. 3) The other arm received 186 pages divided into nine modules for use over the internet." "Modules were also sent by post on a weekly basis..." Psychoeducation, cognitive restructuring, exposure, attention training, social skills, relapse prevention.

Compliance: "each module...ended with a short quiz to check adherence" "in cases of missing data, brief, neutral reminder was sent 24h later by email, and if necessary followed by another reminder as autogenerated short text message" For Furmark 2009a and 2009b: completed modules 1 (100%), 2 (95%), 3 (92.5%), 4 (85%), 5 (80%), 6 (77.5%), 7 (72.5%), 8 (70%), 9 (62.5%).

Furmark 2009b (Continued)

Outcomes

Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR)

General Symptoms: Beck Anxiety Inventory (BAI)

Response: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR); Social Interaction Anxiety Scale (SIAS); Social Phobia Scale (SPS); Social Phobia Screening Questionnaire (SPSQ)

Depression: Montgomery and Asberg Depression Rating Scale (MADRS)

Quality of Life: Quality of Life Inventory (QOLI)

Notes

Funding: Swedish Research Council. "The manual was released as a self-help book for the Swedish market after completion of the study." T.F., A.H., E.S., P.Ca. and G.A. receive royalties for authoring the self-help book

Trial 2: Combined ICBT, Bib, BibDG groups to form the intervention group. Averaged outcomes and treatment characteristics, including number and duration of contact.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Randomisation was performed by an independent third party using an online true random-number service (www.random.org)."
Allocation concealment (selection bias)	Low risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "...giving email feedback to each participant. There were 13 internet therapists in the study, of whom 6 were licensed clinical psychologists and 7 were clinical psychology students in their last semester of the 5-year master's degree programme. The students had clinical supervision during the trial. Participants were randomly allocated to their therapist."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF 0 lost to post-treatment; 14 intervention 2 control lost to follow-up. Applies to Furmark 2009a and 2009b: "Ten participants (4.3%) withdrew from the study after the first (n = 6) or second (n = 4) treatment week, the main reasons being lack of time or motivation and personal problems..." "For all randomised participants, missing data were replaced by the last obtained score (pre- or post-treatment), i.e. last observation carried forward." "In accordance with the in-

Furmark 2009b (Continued)

tention-to-treat principle, all participants were asked to complete post-treatment and follow-up assessments, regardless of how many treatment modules they had completed."

Selective reporting (reporting bias)	Low risk	Trial registration number: NCT01145690 All outcomes reported.
Other bias	Low risk	N/A

Ghosh 1984

Methods	Location: London, England Recruitment: Referral Exclusion rate: 42.11% Randomised (N): 88 (59 intervention; 29 comparison)
Participants	Disorder: Mixed (AG, SAD, SP); Clinical Interview (ICD-9) Duration (years): 11 Age (years): 36 Sex (female): 74.65% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: If patients were on drugs they were asked to gradually come off them before being included in the trial." Drug/Alcohol: N/A Physical: "uncontrolled hypertension, cardiac or severe respiratory conditions"
Interventions	Comparison group(s): Individual Exposure Comparison description: "Therapist-instructed patients had the same treatment approach and worked only in discussion with a therapist (AG)." Intervention name: Living With Fear (Marks, 1980) Days of treatment: 70 Author wrote: Yes Type of media: Mixed

Ghosh 1984 (Continued)

Type of therapy: Exposure

Location used: Other

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=6 (67min)

Contact description: 40 minutes, 4.2 hours and 3.2 hours. "book-instructed patients were only seen three times during the treatment phase and thrice during the follow-up period; computer- and therapist-instructed patients were seen at weekly intervals during the treatment phase and thrice during follow up..." "Participants could contact the psychiatrist at any time, though none did so. All patients were treated individually." All received an explanation of avoidance, exposure, and homework, and they were told to enlist relatives and friends to help.

Intervention description: 2 groups combined: (i) "given a copy of the book 'Living with Fear' (Marks 1978), which explains step by step how patients can help themselves by slowly coming off sedatives and alcohol and devising a detailed programme of graded exposure..." (ii) "3-10 treatment sessions of 40-60 minutes..." "computer was programmed to 'discuss' exposure tasks from a hierarchical list of patients' phobic situations. At the end of the 'interview' it printed out an 'exposure homework' diary..."

Compliance: Homework diaries. Diary data are recorded and analysed.

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Agoraphobia General Symptoms: Fear Questionnaire (FQ): Anxiety Depression; Fear Questionnaire (FQ): Anxiety Depression, assessor rated Disability: Work and Adjustment Rating Scale (WARS)
Notes	Funding: Medical Research Council Project Grant Unpublished data: Marks identified Ghosh 1984 as a preliminary report of the participants in the 1987 and 1988 papers. Unable to provide further information. The 1988 paper says the therapist group was added 3 months into the study.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear "Patients were randomly assigned to one of three self-exposure treatment groups" "imbalance in numbers arose because (a) originally the study had been planned to compare only computer and book-instructed self-exposure, and the therapist-instructed group was added 3 months into the trial"
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "book-instructed patients were only seen three times during the treatment phase and thrice during the follow-up period; computer- and therapist-instructed patients were seen at weekly intervals during the treatment phase and thrice during follow up..."

Ghosh 1984 (Continued)

Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “Patients were assessed before and after treatment and at 3 and 6 months follow-up by an experienced behavioural psychotherapist, who was blind to the treatment condition.”
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=7 (11.86%) <i>Comparison:</i> N=10 (34.48%) Method of analysis: Unclear Dropouts occurred "before completing an adequate trial of treatment." "Seventy-five patients completed treatment, but included 4 who had not been randomized and so these were omitted from the analysis. Of the 71 patients who completed the randomized trial, all except one in the computer-instructed group completed six months' follow-up."
Selective reporting (reporting bias)	High risk	No trial registration number given. “Most statistical tests are not reported for the whole sample. The only results are for the subgroup with AG.”
Other bias	High risk	Numbers randomised and analysed differ among reports. A 1987 report is presented as a stand-alone trial, but the author confirms it reports data for a subgroup of the earlier trial. One group was added 3 months into the study. Dropout figures assume equal allocation, but allocation may not have been equivalent.

Gilroy 2000

Methods	Location: Tasmania, Australia Recruitment: Advertising/ Internet Exclusion rate: 13.46% Randomised (N): 45 (15 intervention; 15, 15 comparisons)
Participants	Disorder: Specific Phobia; CIDI (DSM-III R) Duration (years): Unknown Age (years): 33.11 Sex (female): 100% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS

Gilroy 2000 (Continued)

	Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Muscle relaxation (self-help), Individual exposure Comparison description: 1) "An audiotape of progressive muscle relaxation...The long and short versions of Jacobson's complete deep muscle relaxation were repeated twice to fill the 45-minute..." 2) "five pictorial representations of spiders and then moving onto a sequence of steps of increasing fearfulness similar to those in the BAT. Throughout treatment, progression to a higher step was based on three successful attempts at the previous step, a SUDS rating of below 20...." Intervention name: CAVE (Smith 1997) Days of treatment: 42 Author wrote: Yes Type of media: Computer Type of therapy: Exposure Location used: Clinic/ Surgery Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=3 (15min) Contact description: Pretreatment assessment comprised two sessions. The first session involved obtaining written consent, demographic details, and completion of the CIDI... "Each participant then received three 45-minute treatment sessions..." "In the initial session the therapist stayed with the participant for approximately 5 minutes to ensure the participant was comfortable using the program. The therapist then left the participant to work through the program." Intervention description: The program instructed participants in vicarious exposure for spider phobia. Participants used a point-and-click method with the computer mouse to guide a person" (a screen figure) through a house. The participant could direct the screen figure into scenarios depicting a spider picture, a plastic spider, a dead spider, and a live spider in various rooms." 3 x 45-min sessions every two weeks. Compliance: Measured exposure homework at 33 months.
Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Global Phobia Behaviour Test (Level): Behavioural Approach Test: Level Behaviour Test (Symptoms): Behavioural Approach Test: Symptom Rating Disability: Work and Adjustment Rating Scale (WARS)
Notes	Funding: None acknowledged Unpublished data: Gilroy described the method of randomisation, demographic details and estimated contact with participants, including time required for consent and assessment (email, 19 May 2009).

Gilroy 2000 (Continued)

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Computer/Online The first 45 participants were randomly assigned to one of the three treatment conditions. See Allocation Concealment.
Allocation concealment (selection bias)	High risk	"Five participants (4 from relaxation condition, 1 from live exposure condition) did not complete treatment due to time constraints (3), moving interstate (1), and having already undergone prior relaxation treatment (1). These participants were replaced with new participants who were allocated to the same treatment groups as those who had not completed treatment."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) The assessor, a master's student in psychology (LG), delivered both the assessment and treatment sessions.
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) The assessor, a master's student in psychology (LG), delivered both the assessment and treatment sessions.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (6.67%) <i>Comparison (muscle relaxation):</i> N=0 (0%) <i>Comparison (individual exposure):</i> N=2 (13.33%) Method of analysis: Per protocol Dropouts replaced. 14 intervention, 13 face-to-face and 15 relaxation subjects completed questionnaires at 147 week follow-up. 13 intervention 11 face-to-face and 8 relaxation subjects completed the BAT at 147 week follow-up.
Selective reporting (reporting bias)	High risk	No trial registration number given. BAT results for the relaxation group at 33 months are missing, but other outcomes appear to be reported clearly and completely.
Other bias	High risk	Dropouts were replaced, thus violating the randomisation.

Gould 1993a

Methods	Location: Virginia, USA Recruitment: Advertising/ Internet Exclusion rate: 69.16% Randomised (N): 33 (12 intervention; 12, 9 comparison)
---------	---

Gould 1993a (Continued)

Participants
Disorder: Panic disorder w/ AG; ADIS (DSM-III R)

Duration (years): Unknown

Age (years): 35.7

Sex (female): 64.52%

Race (white): 18.18%

Medication (using at baseline): 18.18%

Education (years): 14.26

EXCLUSIONS
Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: Subjects taking medication for anxiety or depression were allowed to participate if they had been stabilized on the medication for at least four weeks and continued to have panic symptoms (n = 6).

Drug/Alcohol: N/A

Physical: "having been diagnosed by a physician as having seizure disorder, kidney disease, stroke...emphysema, heart attack, or chronic hypertension."

Interventions
Comparison group(s): Wait-list, Individual CBT

Comparison description: 1) "WL subjects were told that they would have to wait 7 weeks for their treatment to begin." "Subjects who did not mail in their weekly data were reminded to do so by telephone." 2) Individual therapy "plans were derived primarily from material in Coping with Panic." "Subjects met with a therapist twice per week for 4 weeks for a total of eight 1-hour individual therapy sessions targeting their panic attacks. There were four graduate-level therapists in this study."

Intervention name: Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks (G Clum; 1990)

Days of treatment: 28

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=2 (30min)

Contact description: Total experimenter contact time for subjects in the WL and BT groups was for assessment purposes only and was 2.5 and 3.0 hours, respectively; ITGIC subjects required 10.5 hours. "In all groups, subjects who did not complete weekly measures were reminded by phone. "contacted at weeks 2 and 4 by the experimenter to assess their progress in reading the book. Each phone call lasted

Gould 1993a (Continued)

approximately 10 minutes, during which the experimenter followed a written protocol and did not answer questions..."

Intervention description: 2-week monitoring and assessment (common to all groups, beginning in week 5 for the wait-list). "...the etiology and nature of panic disorder; (2) teaching them a variety of cognitive and behavioral strategies that include relaxation, cognitive restructuring, breathing retraining, and exposure; and (3) advising them on how to implement these strategies."

Compliance: Experimenters questioned subjects about specific material covered in the book to ascertain if subjects were reading the book and to determine their level of comprehension. A score of 70% on this assessment was considered evidence that the subject was reading the book.

Outcomes	Specific Symptoms: Mobility Inventory for Agoraphobia (MIA) General Symptoms: Anxiety Sensitivity Index (ASI) Response: Panic Free Depression: Beck Depression Inventory (BDI)
Notes	Funding: None acknowledged Unpublished data: Author provided information about the conduct of the study.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear The paper explains: "Subjects were matched on level of avoidance and then randomly assigned to one of the three experimental conditions." The author explains: "individuals who agreed to be in the study and were eligible were alternately assigned to groups" (personal communication).
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Experimenters questioned subjects about specific material covered in the book to ascertain if subjects were reading the book"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (8.33%) <i>Comparison (wait-list):</i> N=1 (8.33%) <i>Comparison (individual CBT):</i> N=0 (0%) Method of analysis: Per protocol Only 2 subjects dropped out: 1 from the WL group and 1 from the BT group. The subject from the WL group reported that she had to move to a different state to seek employment and could no longer continue in the study. The sub-

Gould 1993a (Continued)

		ject from the BT condition completed all the dependent measures of the study, but was not included in the analyses because she had failed to read the book.
Selective reporting (reporting bias)	High risk	No trial registration number given. Limited symptom panic attacks not reported.
Other bias	Low risk	N/A

Gould 1995

Methods	Location: Virginia, USA Recruitment: Advertising/ Internet Exclusion rate: 76.56% Randomised (N): 33 (17 intervention; 16 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-III R) Duration (years): 10.3 Age (years): 36.2 Sex (female): 84% Race (white): 44% Medication (using at baseline): 44% Education (years): 13.8 EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "Subjects taking medication for anxiety or depression were allowed to participate if they had been stabilized on the medication for at least 4 weeks and continued to have panic symptoms, and if they recorded their medication usage throughout the study (n Drug/Alcohol: "chronic use of alcohol, drug dependence or abuse.." Physical: "any of the following conditions: seizure disorder, kidney disease, stroke...emphysema, myocardial infarction, or chronic hypertension."
Interventions	Comparison group(s): Wait-List Intervention name: Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks (G Clum; 1990) Days of treatment: 28 Author wrote: Yes Type of media: Book / Booklet

Gould 1995 (Continued)

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=2 (30min)

Contact description: Subjects who failed to mail in their weekly data were reminded to do so by telephone contact from one of the study's personnel. "The total time spent with researchers/therapists for each group of subjects was 2.5 hours for the WL group and 3.0 hours for the SH group, including the initial diagnostic evaluation." "SH subjects were told that their progress in reading the book would be assessed at mid- and posttreatment by telephone."

Intervention description: 2-week monitoring and assessment (common to all groups, beginning in week 5 for the wait-list). "...the etiology and nature of panic disorder; (2) teaching them a variety of cognitive and behavioral strategies that include relaxation, cognitive restructuring, breathing retraining, and exposure, and (3) advising them on how to implement these strategies." "videotape was a 15-minute role-played therapy session" "an audiotape that taught the progressive muscle relaxation"

Compliance: "SH subjects were instructed to complete and mail in the same four weekly measures as WL subjects. In addition, they were instructed to fill out a weekly Practice Record..." "Subjects also kept a weekly log of the amount of time they spent practicing specific coping techniques, including self-exposure. This self-monitoring procedure was not included as a dependent measure but rather was included as an inducement to practice coping strategies."

Outcomes	Specific Symptoms: Mobility Inventory for Agoraphobia (MIA) Response: Panic Free	
Notes	Funding: None acknowledged Unpublished data: Author provided information about the conduct of the study.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear At the end of the baseline period, all subjects were matched on their pretreatment level of avoidance score and randomly assigned to either the SH or WL groups. The author explains: "subjects were alternately placed in the two groups" (personal communication)
Allocation concealment (selection bias)	High risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "SH subjects were told that their progress in reading the book would be assessed at mid- and posttreatment by telephone."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (29.41%)

Gould 1995 (Continued)

Comparison: N=3 (18.75%)

Method of analysis: Per protocol

"...the baseline period, 3 subjects decided not to be in the study...prior to their learning their treatment assignment." "Among the WL dropouts, 1 subject reported not having the time to keep records and the other gave no reason for termination. Among SH subjects, 1 decided he needed to see a psychiatrist and wanted medication for his problems, 1 terminated for no discernible reason and did not return our telephone calls, and 1 never read the book."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given. A similar study by the same authors (Gould 1993) included the BDI.
Other bias	Low risk	N/A

Granado 2007

Methods	Location: Sao Paulo, Brazil Recruitment: Advertising/ Internet Exclusion rate: 84.38% Randomised (N): 25 (13 intervention; 12 comparison)
Participants	Disorder: Specific Phobia; SCID (DSM-IV) Duration (years): 23 Age (years): 31.3 Sex (female): Unknown Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Attention/Monitoring Comparison description: Placebo

Granado 2007 (Continued)

Intervention name: SLAT: Spiderless Arachnophobia Therapy

Days of treatment: 28

Author wrote: Yes

Type of media: Internet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=4 (duration unknown)

Contact description: To assess progress during the treatment, placebo and treatment subjects underwent the BAT (including the SUDS) each week.

Intervention description: "patients attend a computer presentation of images that, while not being spiders, have a subset of the characteristics of spiders" "patient was instructed to run the presentation twice a day at home

preferably during moments in which she/he was not tired or under stress." "To assess progress during the treatment, placebo and treatment subjects underwent the BAT (including the SUDS) each week."

Compliance: Prior to each presentation run, the patient was given one question to answer at the end of the run. Every week these data were checked out in order to verify the rate of cooperation of patients and to encourage noncooperative patients, if any. In all subjects, the cooperation was satisfactory and no statistics were deemed necessary to measure the rate of cooperation.

Outcomes	Specific Symptoms: Fear of Spider Questionnaire (FSQ) Behaviour Test (Level): Behavioural Approach Test: Level Behaviour Test (Symptoms): Behavioural Approach Test: Subjective Units of Distress
Notes	Funding: supported by a CNPq Fellowship 154342/2006-8 and, previously, by a FAPESP Fellowship 03/08804-0

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "randomly divided into two groups"
Allocation concealment (selection bias)	Unclear risk	"randomly divided into two groups"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "To assess progress during the treatment, placebo and treatment subjects underwent the BAT (including the SUDS) each week."
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind)

Granado 2007 (Continued)

“To assess progress during the treatment, placebo and treatment subjects underwent the BAT (including the SUDS) each week.”

Incomplete outcome data (attrition bias) All outcomes	Low risk	No dropout.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Greist 2002

Methods	Location: Salt Lake City, UT; Wheat Ridge, CO; Gainesville, FL; Houston, TX; Toronto, ON; Raleigh, NC; Worcester, MS; Atlanta, GA, USA/Canada Recruitment: Advertising and referral Exclusion rate: Unknown Randomised (N): 218 (74 intervention; 75, 69 comparisons)
Participants	Disorder: OCD; SCID (DSM-IV) Duration (years): 22 Age (years): 39 Sex (female): 42% Race (white): 49% Medication (using at baseline): 49% Education (years): 14.63 EXCLUSIONS Depression: Yes Psychotherapy: No restrictions reported Medication: "51% had not taken an SRI for at least 2 weeks prior to prescreening... The remainder were taking an SRI at or above an adequate minimum stable dose...The study did not allow dose adjustments or other prescription psychotropics." Drug/Alcohol: "in the past 6 months, alcohol or substance abuse" Physical: "unstable medical conditions"
Interventions	Comparison group(s): Relaxation, Individual Exposure Comparison description: 1) "asked to perform progressive relaxation exercises for at least an hour daily and to keep daily relaxation diaries..." 2) 11 weekly, 1-hour (or longer) sessions. Intervention name: BT STEPS

Greist 2002 (Continued)

Days of treatment: 70

Author wrote: Yes

Type of media: Computerised Phone

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=12

Provider contact: N=3 (45min)

Contact description: All participants met with a clinician for 15 minutes at the end of weeks 2, 6, and 10 after starting treatment to assess improvement, safety, and appropriateness of continued participation. "Patients get feedback on their progress via the calls and reports mailed to them weekly".

Intervention description: 9-step, computer-driven IVR [interactive voice response] system that allows patients with OCD to telephone from home and progress through a self-paced workbook." Education and assessment, daily self-exposure, relapse prevention.

Compliance: The mean length of telephone calls to BT STEPS was 8.6 minutes...YBOCS improvement correlated significantly with more calls (mean $\pm$ SD = 22.5 $\pm$ 71.6... "The number of self-exposure home-work sessions completed by patients in the BT STEPS groups (mean $\pm$ SD = 9.9 $\pm$ 35.2) correlated significantly with mean YBOCS improvement..." "61% of calls were made between 5:00 p.m. and 9:00 a.m. or on weekends."

Outcomes	Specific Symptoms: Yale-Brown Obsessive-Compulsive Scale (Y-BOCS) Response: Clinical Global Impression (CGI): Improvement; Patient Global Impression (PGI): Improvement Depression: Hamilton Depression Rating Scale, self-rated (HAM-D S) Disability: Work and Social Adjustment Scale (WSAS)
Notes	Funding: Cathryn Clary was employed by Pfizer. Isaac Marks and Lee Baer received royalties from BT STEPS, owned by Healthcare Technology Systems. Greist, Kobak, Wenzel, Hirsch and Mantle were employed by HTS. Follow-up paper notes that severity moderates response.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "After screening, subjects were randomly assigned..."
Allocation concealment (selection bias)	Unclear risk	"After screening, subjects were randomly assigned..."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind)

Greist 2002 (Continued)

In addition to contact in the face-to-face therapy group, "All participants met with a clinician for 15 minutes at the end of weeks 2, 6, and 10...."		
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "Ratings were determined by an interviewing clinician (not the therapist) using paper-and pencil scales (CGI) and by the patient using paper-and-pencil (TRT-X, PSQ) or telephone interactive voice response (IVR) scales* (YBOCS, PGI, WSAS HAM-D). A subsample of 90 patients was also rated on the clinician-administered YBOCS at baseline and end-point by a rater blind to treatment condition."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention</i>: N=19 (25.68%) <i>Comparison (relaxation)</i>: N=9 (12%) <i>Comparison (exposure)</i>: N=14 (20.29%) Method of analysis: Per protocol Subjects whose total YBOCS scores fell by 25% or more during assessment (placebo responders, N=16) or who did not complete assessment tasks (N=5), violated the protocol (N=12), or withdrew (N=2) were excluded. "176 had at least 1 evaluable post-week 0 visit and were included in the end-point intent-to-treat analyses..." "the last available postrandomization rating was input to endpoint for subjects who stopped prematurely."
Selective reporting (reporting bias)	High risk	No trial registration number given. "A subsample of 90 patients was also rated on the clinician-administered YBOCS..." Data for goals completed not reported. Expectation (TRT-X) and satisfaction (PSQ) not reported (but available from author on request).
Other bias	High risk	A run-in undermined the randomisation; 24% of the intervention group were excluded from further analyses: "One hundred eighty-three reached week 0 after 2 weeks' assessment (baseline visit: 57 in the BT STEPS group, 59 in the clinician guided behavior therapy group, and 67 in the relaxation group)."

Grime 2004

Methods	Location: London, England Recruitment: Workplace Exclusion rate: 64.85% Randomised (N): 48 (24 <i>intervention</i>; 24 <i>comparison</i>)
Participants	Disorder: Mixed (Mean HADS Anxiety score in the moderate range (i.e. mean >11)); No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 39 Sex (female): 58.33%

Grime 2004 (Continued)

Race (white): 20.83%

Medication (using at baseline): 20.83%

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: "All participants continued whatever care they were otherwise receiving (conventional care), details of which were recorded"

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Treatment-as-usual

Comparison description: 21% received the intervention outside the trial.

Intervention name: Beating the Blues (Ultrasis; <http://www.ultrasis.com>)

Days of treatment: 56

Author wrote: Yes

Type of media: Computer

Type of therapy: CBT

Location used: Workplace

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=8 (125min)

Contact description: Author estimates that the first interaction was 30-45 minutes and subsequent visits were 5-10 minutes (as in Proudfoot, and as estimated for other clinic-based studies). When participants failed to attend, the researcher contacted them by phone. "A weekly progress report of distress self-ratings and suicidal ideation is generated for the user and for the supervising clinician."

Intervention description: "Beating The Blues' is an interactive computerized CBT programme. Its cognitive components explore automatic thoughts, thinking errors and distraction, challenging unhelpful thinking, core beliefs and attributional style...activity scheduling, task breakdown, problem solving, sleep management, relaxation training and biofeedback, planning and prioritizing and graded exposure. Cognitive and behavioural exercises are prescribed at the end of each module, and debriefed at the start of the next."

Compliance: "Of those randomized to 'Beating The Blues', 16 completed all eight sessions of the programme, one completed six sessions, two completed four sessions, one completed three sessions and four completed two sessions. Reasons for not completing the programme included attrition, inability to commit the necessary time, and a desire not to continue."

Outcomes

General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety

Depression: Hospital Anxiety and Depression Scale (HADS): Depression

Grime 2004 (Continued)

Notes

Funding: None acknowledged

Unpublished data: Grime provided unpublished manuscript, demographics, qualitative feedback.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online Eligible participants giving written, informed consent, were randomized according to a sequence generated by a random number table
Allocation concealment (selection bias)	Low risk	Randomisation codes in sealed envelopes were opened sequentially, after collection of baseline demographic, absence and psychological data.
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Participants were seen at the start and end of each session.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=8 (33.33%) <i>Comparison:</i> N=1 (4.167%) Method of analysis: Available Case Analysis was performed on an intention to treat basis... "Of the eight who did not complete the programme, four completed follow up questionnaires and four were lost to follow up. One participant completed all eight sessions of 'Beating The Blues', but none of the follow up questionnaires." Four control participants were excluded from absenteeism analyses as they were absent for (i) unemployment and (ii) maternity leave.
Selective reporting (reporting bias)	Low risk	No trial registration number given. Author provided unpublished dissertation.
Other bias	Low risk	N/A

Hassan 1992

Methods

Location: Leeds, England

Recruitment: Advertising/ Internet

Exclusion rate: 5%

Randomised (N): 42 (11 intervention; 9, 22 comparison)

Hassan 1992 (Continued)

Participants

Disorder: Specific Phobia; Clinical Interview (DSM-III R)

Duration (years): 20.34

Age (years): 28.74

Sex (female): 78.95%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS
Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Wait-list, Individual exposure

Comparison description: (i) Exposure with modelling and live spiders. When all photos were viewed with little or no fear, live spiders were introduced. Concluded when participants could perform target behaviours with clear ease and confidence. Tarantulas used. (N=10.5). (ii) Exposure plus relaxation (N=10.5). (iii) Wait-list (N=10). [First two groups combined for analysis.] (Intervention strategies similar, method of delivery varied.)

Intervention name: Computer-Based Symbolic Modelling

Days of treatment: 18

Author wrote: Yes

Type of media: Computer

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=5 (20min)

Contact description: Therapist was present in the treatment room, but no help was offered or requested by clients. 20 minute session followed completion of computer modelling; researcher had live spiders in boxes and explained the BAT verbally without demonstration.

Intervention description: Computerised exposure. Information, relaxation. Video and photographs of a woman approaching spiders (like BAT). Rate controlled by participants. Twice weekly 40-min sessions. Anxiety Rating Scale displayed when "Stop" pressed, and subjects saw relaxation instructions or listened to classical music. Session terminated if stopped twice for an item. Resumed where previous session ended. Debriefing preceded post-treatment assessment.

Hassan 1992 (Continued)

Compliance: Intervention participants completed 5 sessions on average, duration of 3.34 hours. By comparison, graded exposure and live modeling (combined for analysis) lasted 4.78 and 4.45 sessions and involved 3.19 and 2.97 hours of therapist contact.

Outcomes	Specific Symptoms: Spider Phobia Questionnaire (SPQ) Behaviour Test (Level): Behavioural Approach Test: Level Response: Behavioural Approach Test: Completed; Spider Phobia Questionnaire (SPQ)
Notes	Funding: None acknowledged Unpublished data: Contacted Leeds, which provided the dissertation, but unable to contact the author directly.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear Using a matched-group design technique, clients were first stratified into groups on four on the basis of their BAT scores. This was done by ranking all clients in terms of their BAT scores in ascending order, the first group of four in the list were taken...then randomly assigned to the four conditions of the study.
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Participants were seen at the start and end of each session.
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) Participants were seen at the start and end of each session.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (9.09%) <i>Comparison (wait-list):</i> N=1 (11.11%) <i>Comparison (individual exposure):</i> N=2 (9.09%) Method of analysis: Available Case Three clients who attended assessment but did not attend any further sessions were not included in the analyses. One client withdrew who thought her condition was "beyond treatment." Estimated number of participants assuming equivalent group sizes.
Selective reporting (reporting bias)	Low risk	No trial registration number given. The composite score is not reported, but other measures appear to be reported completely and clearly. Data extracted from thesis likely complete.

Hassan 1992 (Continued)

Other bias	Low risk	N/A
------------	----------	-----

Hazen 1996

Methods	Location: Winnipeg, Canada Recruitment: Referral Exclusion rate: 9.4% Randomised (N): 117 (32 intervention; 28, 28 comparison)
---------	---

Participants	Disorder: Panic disorder; SCID (DSM III) Duration (years): 8.6 Age (years): 37.12 Sex (female): 72.6% Race (white): 46.99% Medication (using at baseline): 46.99% Education (years): 11.36 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "current pharmacological treatment for panic disorder, with the exception of low doses of benzodiazepines (equivalent of 20 mg diazepam or less) or stable doses of antidepressants (i.e., prescribed for at least 6 months and stable dose for at least 3 mont Drug/Alcohol: "substance abuse" Physical: "physician agreement regarding participation. Exclusion criteria included: (1) presence of organic disease which might be related to panic disorder or interfere with participation in the study"
--------------	---

Interventions	Comparison group(s): Wait-list, Group CBT Comparison description: 1) Wait-list. 2) "thirteen 1.5 hour sessions were conducted over a four-teen-week period. Sessions were structured around the content of the self-help manual, and included discussion of reading and practice homework assignments." Intervention name: Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks (G Clum; 1990) Days of treatment: 98 Author wrote: No Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere
---------------	--

Hazen 1996 (Continued)

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "Potential participants were initially assessed with a 30-minute structured telephone interview. Respondents who appeared to meet the inclusion criteria were then interviewed in person by a clinician experienced in assessing anxiety disorders."

Intervention description: "The content of the treatment program included psycho-educational information about anxiety, and cognitive-behavioral treatment strategies, including relaxed breathing, progressive muscle relaxation, cognitive restructuring, and graduated exposure to feared situations. Subjects assigned to the self-help manual condition were instructed to complete one section of the self-help manual weekly for 14 weeks."

Compliance: N/A

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Agoraphobia General Symptoms: Anxiety Sensitivity Index (ASI) Response: Clinical Global Impression (CGI): Improvement
Notes	Funding: Manitoba Health Research Council Unpublished data: Walker provided slides from presentations showing number randomised to each group. Described randomisation process and demographics.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "...we would recruit a block of participants – say eight or ten – then that block would be assigned to one of the four conditions...we would have four slips of paper with the four conditions, and blindly pull one from a hat...We would then recruit for the next block. When that block was ready, we would pull a strip randomly (for the remaining three conditions) and assign to that condition."
Allocation concealment (selection bias)	Low risk	"If we were screening a lot of people at once, we would also try to match up the groups a bit in terms of gender and severity (before they were randomized)."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Comparison group received face-to-face treatment.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "Subjects also attended a post-treatment evaluation interview, during which several rating scales, including the Clinical Global Improvement scale, were administered by the first author, who served as the independent assessor for the treatment study. Over the course of the study, the assessor remained blind to subjects' treatment group status in order to ensure that unbiased ratings were made."
Incomplete outcome data (attrition bias)	High risk	Not analysed at post-treatment

Hazen 1996 (Continued)

All outcomes

Intervention: N=5 (15.63%)

Comparison (wait-list): N=1 (3.57%)

Comparison (group CBT): N=2 (7.14%)

Method of analysis: Available Case

Unpublished presentation clarifies that 11 participants were assigned and not analysed.

Selective reporting (reporting bias)	High risk	No trial registration number given. Unable to extract Frequency of PAs, BDI.
Other bias	Low risk	N/A

Heading 2001

Methods	Location: Tasmania, Australia Recruitment: Advertising/ Internet Exclusion rate: 22.4% Randomised (N): 45 (15 intervention; 16, 14 comparison)
Participants	Disorder: Specific Phobia; CIDI (DSM-IV) Duration (years): Unknown Age (years): 34.9 Sex (female): 95% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: No restrictions reported Medication: "not taking psychotropic medication" Drug/Alcohol: "substance-abuse problem" Physical: N/A
Interventions	Comparison group(s): Wait-list, Individual exposure Comparison description: 1) Wait list, 2) "The treatment rationale focused on habituation/extinction of the anxiety response (Gilroy et al., 2000)...No relaxation exercises, modelling, or behavioural experiments aimed at disconfirming specific beliefs were used in the exposure sessions. No exposure home-work instructions were given to the participant."

Heading 2001 (Continued)

Intervention name: CAVE (Smith 1997)

Days of treatment: 1

Author wrote: Yes

Type of media: Computer

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=1 (20min)

Contact description: The therapist stayed with the participant for the first 5 minutes of the treatment, to answer any questions. The program was reset by the therapist every 45 minutes to total 3 hours.

Intervention description: Participants are asked to assist an animated screen figure to overcome his/her fear of spiders, using a computer mouse to direct the figure through different exposure scenarios. An on-screen "anxiety thermometer" models habituation with the accumulation of approach behaviours, while a score displays the figure's progress towards a target score of 2000."

Compliance: There were no significant differences in scores on the HWQ for self-directed homework between the LGE, CAVE, and waiting-list groups at both post-treatment and follow-up assessment. Overall, participants reported few self-initiated homework activities: mean for LGE 2.8 (SD 1.2), CAVE 2.0 (SD .8), and waiting list 2.0 (SD 1.8).

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Global Phobia Behaviour Test (Level): Behavioural Approach Test: Level Behaviour Test (Symptoms): Behavioural Approach Test: Subjective Units of Distress Disability: Work and Social Adjustment Scale (WSAS)
Notes	Funding: None acknowledged Unpublished data: Heading, Kirkby and Gilroy contacted. Gilroy described the baseline measures and estimated contact with participants.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear The participant was randomised to one of the three conditions, with the treatment session 1 week after the pre-treatment assessment.
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind)

Heading 2001 (Continued)

		The therapist stayed with the participant for the first 5 minutes of the treatment.
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "The researcher accompanied the participant during all steps to advise of the next step, to record SUDS, and to score steps. Participants were debriefed following the BAT. BATs are considered highly sensitive measures of treatment change (de Beurs, Lange, Van Dyck, Blonk, & Pieter, 1991)."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention</i> : N=3 (20%) <i>Comparison (wait-list)</i> : N=1 (6.25%) <i>Comparison (individual exposure)</i> : N=1 (7.14%) Method of analysis : Available Case Five participants with- drew from the study (CAVE = 2, waiting list = 3) due to time commitments, before receiving treatment. "One participant (LGE) with- drew following the post-treatment assessment."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Hedman 2011a

Methods	Location : Stockholm, Sweden Recruitment : Advertising and referral Exclusion rate : 30.77% Randomised (N) : 81 (40 intervention; 41 comparison)
Participants	Disorder : Health Anxiety; MINI (DSM-IV) Duration (years) : 20.99 Age (years) : 39.05 Sex (female) : 74.07% Race (white) : 38.27% Medication (using at baseline) : 38.27% Education (years) : Unknown EXCLUSIONS Depression : Yes Psychotherapy : Current and previous therapy restricted

Hedman 2011a (Continued)

Medication: "constant dosage 2 months prior to treatment if on prescribed medication for anxiety or depression and agree to keep dosage constant throughout the study"

Drug/Alcohol: "not currently fulfilling the diagnostic criteria for substance misuse according to the MINI"

Physical: "no serious somatic disease as assessed by a physician based on a review of medical records and the anamnesis from the diagnostic assessment interview"

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: "The control condition consisted of an online discussion forum where participants could send messages anonymously to each other over a period of 12 weeks...the rationale for employing this control condition design was to ensure a basic control for attention and the possible anxiety-alleviating effect of sharing one's distress with others. Participants were encouraged to discuss their health anxiety and helpful ways of coping with it, and to provide support to others..."

Intervention name: Not named

Days of treatment: 84

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=12 (108min)

Contact description: Estimated 1 weekly contact based on the following: "The role of the therapist was mainly to provide feedback regarding all homework and to grant access to the succeeding treatment modules; however, the participant could contact the therapist at any time and expect a reply within 24h. In addition, therapists encouraged inactive participants to continue the treatment work. The participant and therapist had no face-to-face or telephone contact during the treatment. On average therapists spent 9min (s.d. = 5.6) weekly per participant."

Intervention description: "118 pages, divided into 12 modules" with homework exercise...emphasising the role of avoidance and safety behaviours, internal focus and interpretations of bodily sensations as signs of serious illness as maintaining factors of hypochondriasis. In addition, the treatment included mindfulness training as a means of acquiring skills to help participants expose themselves... "an online discussion forum that enabled anonymous contact with other participants receiving internet-based CBT."

Compliance: "The average number of completed modules in internet-based CBT was 9.1 (s.d. = 3.3). Six participants completed fewer than six modules and were considered non-completers...The most common reason reported for not completing treatment was lack of time. In the control group, the average number of postings was 16.1 (s.d. = 19.3). Thirty-nine of 41 participants in the control group posted at least one message."

Outcomes

Specific Symptoms: Illness Attitude Scale (IAS)

General Symptoms: Anxiety Sensitivity Index (ASI); Beck Anxiety Inventory (BAI)

Response: Clinical Global Impression (CGI): Improvement

Recovery: No longer met criteria for diagnosis

Hedman 2011a (Continued)

Depression: Montgomery and Asberg Depression Rating Scale (MADRS)

Disability: Global Assessment of Functioning Scale (GAF)

Quality of Life:

Quality of Life Inventory (QOLI)

Notes

Funding: Karolinska Institutet

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The randomisation procedure was managed by an external person not involved in the study. A true random number service (www.random.org) was used to ensure complete randomness."
Allocation concealment (selection bias)	Low risk	"The random sequence was generated after inclusion of participants to ensure that assignment of intervention was concealed from the assessing psychologists and researchers involved in the study."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "The role of the therapist was mainly to provide feedback regarding all homework and to grant access to the succeeding treatment modules..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "The clinical psychologists performing the assessments were masked to treatment status and when summoned to clinician assessment all participants were instructed not to mention which intervention they had received." "In two instances masking was broken...no significant association between assessors' guess and actual treatment allocation (P50.58–0.052)."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: No Dropout
Selective reporting (reporting bias)	Low risk	Trial registration number: NCT00828152 All outcomes reported.
Other bias	Low risk	N/A

Hedman 2011b

Methods

Location: Stockholm, Sweden

Media-delivered cognitive behavioural therapy and behavioural therapy (self-help) for anxiety disorders in adults (Review)

Copyright © 2013 The Cochrane Collaboration. Published by John Wiley & Sons, Ltd.

Hedman 2011b (Continued)

Recruitment: Advertising and referral

Exclusion rate: 45.22%

Randomised (N): 126 (64 intervention; 62 comparison)

Participants

Disorder: Social Anxiety (Unspecified); SCID (DSM-IV)

Duration (years): 20.96

Age (years): 35.35

Sex (female): 35.71%

Race (white): 24.6%

Medication (using at baseline): 24.6%

Education (years): Unknown

EXCLUSIONS
Depression: Yes

Psychotherapy: Current and previous therapy restricted

Medication: "have constant dosage two months prior to treatment of any prescribed medication for anxiety or depression and agree to keep dosage constant throughout the study"

Drug/Alcohol: "not currently meet the diagnostic criteria for substance abuse"

Physical: N/A

Interventions

Comparison group(s): Group CBT

Comparison description: Heimberg RG, Becker RE (2002) Cognitive-behavioral group therapy for social phobia. Basic mechanisms and clinical strategies. New York: Guilford Press. "an initial individual session followed by 14 group sessions over 15 weeks. The individual session prepared the participant to begin group treatment sessions and included a rationale for group treatment. Each group session was 2.5 hours long, including a 15 minute break. Groups were lead by two therapists and had six to seven participants."

Intervention name: Not named

Days of treatment: 105

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=17.4 (82.5min)

Contact description: "...mainly to provide feedback regarding home work and to grant access to the treatment modules...could contact the therapist at any time and expect a reply within 24 hours during

Hedman 2011b (Continued)

week days...no face-to-face or telephone contact ...instruction to the internet therapists was to have the ambition to restrict time spent on each patient to less than 10 minutes per week." "spent 5.5 minutes (SD = 3.6) weekly per patient. The corresponding amount of time in CBGT was 50 minutes (2.5 h sessions with two therapists and 6 patients)....ICBT therapists sent 17.4 messages..."

Intervention description: "stresses the importance of avoidance and safety behaviors as well as misinterpretations of social events and internal focus as maintaining factors of SAD...15 text modules, each covering a specific theme (e.g., exposure or cognitive restructuring) completed with a homework component"

Compliance: "The average number of completed modules in ICBT was 9.33 (SD=4.95) of 15. Fifty- one participants in ICBT (80%) completed at least 5 modules and 19 (29.7%) completed all..." Comparison: "all reviewed sessions were judged to have been conducted in accordance with the treatment manual." "9.40 (SD=4.87) out of a possible total of 15. Fifty participants in CBGT (81%) attended at least five sessions and 17 (27%) attended all..."

Outcomes	Specific Symptoms: Liebowitz Social Anxiety Scale, self-rated (LSAS-SR) General Symptoms: Anxiety Sensitivity Index (ASI); Beck Anxiety Inventory (BAI) Response: Clinical Global Impression (CGI): Improvement; Liebowitz Social Anxiety Scale, self-rated (LSAS-SR) Recovery: No longer met criteria for diagnosis Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Disability: Global Assessment of Functioning Scale (GAF) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Stockholm County Council (Nils Lindefors) and the Bror Gadelius Fund (Erik Hedman)	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “A true random number service (http://www.random.org) was used to ensure randomization.”
Allocation concealment (selection bias)	Low risk	"The randomization procedure involved two external persons not involved in the study; one provided randomization data and the other monitored that no manipulation of treatment allocation was performed by the research group...The random sequence was generated after inclusion of participants to ensure that assignment of intervention was concealed from assessing psychiatrists and researchers of the study."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Provider contact: Yes Provider blind: No “most messages to patients are very brief entailing the core feed-back that the homework was successfully completed and the next treatment module is accessible”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "...participants were instructed not to mention which treatment they had received during the post- treatment and follow-up interviews. After completing the interviews, the assessing psychiatrists guessed allocation..." "On two occa

Hedman 2011b (Continued)

sions participants accidentally mentioned their treatment allocation status to the assessor, and in other two occasions it was deemed necessary to break the blinding because of the need to assess increased depressive symptoms during treatment."

Incomplete outcome data
(attrition bias)
All outcomes

Low risk

Not analysed at post-treatment

Intervention: N=1 (1.57%)

Comparison: N=0 (0%)

Method of analysis: Available Case

"On the CGI-I scale, missing values were replaced with 'no change'. As a complement, the social anxiety measures were also analyzed based on the sample of completers only." At post-treatment, 1 intervention and 0 controls failed to provide self-report data, 5 and 10 failed to complete clinician assessment. At follow-up, 4 intervention and 3 controls failed to provide self-report data, 10 and 11 failed to complete clinician assessment.

Selective reporting (re-
porting bias)

High risk

Trial registration number: NCT00564967

The protocol lists three measures (WQ, TIC-P, SSP) that are not reported. "During treatment, the LSAS-SR and the suicide ideation item of MADRS-S were administered on a weekly basis."

Other bias

Low risk

N/A

Hellström 1995

Methods

Location: Uppsala, Sweden

Recruitment: Advertising and referral

Exclusion rate: 0%

Randomised (N): 52 (42 intervention; 10 comparison)

Participants

Disorder: Specific Phobia; ADIS (DSM-III R)

Duration (years): 19.7

Age (years): 26.7

Sex (female): 100%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Hellström 1995 (Continued)

Medication: N/A

Drug/Alcohol: N/A

Physical: "no disease of the heart or lungs"

Interventions	Comparison group(s): Individual Exposure Comparison description: Based on the description given by Ost (1989a), this method includes a combination of gradual exposure and modelling. The treatment is described in detail in Ost et al. (1991). The previous trial included 126.2 (30.5) minutes contact. Included modeling and live spiders. Sessions videotaped and replayed to patients. (Intervention strategies similar, method of delivery varied.) Intervention name: 1) Living with Fear (Marks, translated) or 2) Not named (also used in Ost 1991) Days of treatment: 14 Author wrote: Yes Type of media: Book / Booklet Type of therapy: Exposure Location used: Other Type of contact: Phone Automated contact: N=0 Provider contact: N=1 (duration unknown) Contact description: Averaged clinic visits across groups. Unspecified phone contact (all groups) between treatment and 1-year assessment. "The program involved 7 time periods during 6 months with specific forms that the patients should fill in and send to the therapist at certain intervals. Upon receiving the form the therapist either phoned the patient or wrote a letter to her in order to provide feedback." Intervention description: Group 1 "instructed to follow a 30 page manual specifically written for spider phobics used in an earlier study (Ost et al., 1991)...based on the one-session therapist-directed exposure..." Group 2 "general manual (Marks, 1978) consists of 23 pages beginning with questions for the patient and a description of the exposure procedure. Both groups were split into those who used the book at (a) home or (b) clinic. For clinic groups, spiders were provided. Compliance: N/A
Outcomes	Specific Symptoms: Fear Survey Schedule General Symptoms: Beck Anxiety Inventory (BAI); State-Trait Anxiety Inventory (STAI) Response: Behavioural Approach Test: Completed Depression: Beck Depression Inventory (BDI)
Notes	Funding: Swedish Medical Research Council Unpublished data: Contacted Hellstrom and Ost. The lead author did not respond. Ost provided data for another study.
Risk of bias	
Bias	Authors' judgement Support for judgement

Hellström 1995 (Continued)

Random sequence generation (selection bias)	Unclear risk	Method: Unclear "After the pre-treatment assessment, the patients were randomly assigned to 5 different treatment groups"
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Half the intervention group interacted with researcher/clinician (not blind) throughout the trial. Therapists treated participants in each group."
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) The therapists who conducted the treatments were the authors of this paper. "The patient's degree of phobic severity was rated by the clinician"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=4 (9.52%) <i>Comparison:</i> N=0 (0%) Method of analysis: Imputed "Four patients in the manual groups at the clinic (3 in S-C and 1 in G-C) stopped treatment prior to the post-treatment assessment, but since this was in effect comparable to patients in the two manual at home groups who performed less than two 2-hour sessions prior to the post-treatment assessment, it was not counted as attrition. For ethical reasons, we offered these four patients therapist-directed one session treatments and therefore their results are included in the follow-up data, but as estimates based on the post-treatment data."
Selective reporting (reporting bias)	High risk	No trial registration number given. Behavioural test, self-rated anxiety, and clinician-rated anxiety are presented as graphs, and significance tests are reported. However, there is not enough information to include these outcomes in meta-analysis. Variance not reported for physiological measures.
Other bias	Low risk	N/A

Holdsworth 1996

Methods	Location: Northumberland, England Recruitment: Referral Exclusion rate: Unknown Randomised (N): 106 (53 intervention; 53 comparison)
Participants	Disorder: Mixed (Mean scores on the GHQ-12 were above accepted threshold for caseness, and anxiety scores were higher than depression scores); Other

Holdsworth 1996 (Continued)

Duration (years): Unknown

Age (years): Unknown

Sex (female): Unknown

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Treatment-as-usual Intervention name: Managing Anxiety and Depression: A Self-help Guide (Holdsworth and Paxton, 1999) Days of treatment: 30 Author wrote: Yes Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=0 (0 min) Contact description: “no contact by participants with researchers.” Intervention description: “a 42 page manual containing a range of treatment techniques for anxiety, depression and related complaints. The material is arranged and presented within the theoretical framework of the Three-Systems-Model (Lang, 1968; Rachman, 1978; Hugdahl, 1981). This model analyses psychological functioning in terms of thought, feeling and behaviour...selection of exercises designed to change what they have identified as most troubling them.” Compliance: Asked about use of the booklet at 3 months.
Outcomes	None included.
Notes	Funding: The Mental Health Foundation, The Design and Communications Department of Northumberland Mental Health NHS Trust. Unpublished data: Paxton had no information. Holdsworth provided details of randomisation, but was unable to provide details about baseline or outcomes. The Mental Health Foundation was con-

Holdsworth 1996 (Continued)

tacted: "I have received a response from our Head of Research. Unfortunately, they have both said that they do not recognise this report. It was suggested that this could be a report the Foundation supported but it wasn't catalogued, which is very unusual, as all of our publications are recorded with The British Library and can be located using their ISBN number."

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Unclear "a randomised controlled trial in the primary care setting." "Equivalent levels of stress were controlled for at the outset in the experimental and control groups using the Social Readjustment Rating Scale." It is not clear how this was done, but the author indicates that a random assignment list was administered using numbered sealed envelopes that were indistinguishable on weight and handling (personal communication).
Allocation concealment (selection bias)	Low risk	"recruits were handed a sealed envelope which contained either a questionnaire and SAE only or these items together with a copy of Managing Anxiety and Depression: A Self-Help Guide. The general practitioner did not know what each envelope contained." It is not clear how these could have been indistinguishable, but the author indicates that they were (personal communication).
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) GPs recruited and treated patients.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=23 (43.39%) <i>Comparison:</i> N=21 (39.62%) Method of analysis: Per protocol "At the outset of the trial, 106 patients were recruited by 40 general practitioners. At the close of the trial, 62 patients had successfully completed all questionnaires: 32 in the control group and 30 in the intervention group." Only participants who completed the first questionnaire were sent questionnaires at 1 and 3 months.
Selective reporting (reporting bias)	High risk	No trial registration number given. It was not possible to extract most outcome measures because means are reported without measures of variance and only significant <i>p</i> values are reported.
Other bias	Unclear risk	It is unclear how many participants were assigned to each group; we assumed equal allocation.

Houghton 2008

Methods	Location: Internet, USA Recruitment: Advertising/ Internet Exclusion rate: 77.98% Randomised (N): 231 (116 intervention; 115 comparison)
Participants	Disorder: Generalised Anxiety Disorder; Unclear Duration (years): Unknown Age (years): 44 Sex (female): 100% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 15.24 EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: "currently in good health as acknowledged by their primary care physician"
Interventions	Comparison group(s): Wait-List Intervention name: Mindfulness-Based Stress Reduction Program (J Kabat-Zinn) Days of treatment: 56 Author wrote: No Type of media: Internet Type of therapy: Muscle Relaxation Location used: Anywhere Type of contact: None Automated contact: N=0 Provider contact: N=0 (0 min) Contact description: None reported. Intervention description: breathing exercises, mindfulness meditation, and yoga stretching Compliance: N/A
Outcomes	General Symptoms: State-Trait Anxiety Inventory (STAI)

Houghton 2008 (Continued)

Notes

Funding: None acknowledged

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "The participants were then randomized to either the treatment or the wait list control group."
Allocation concealment (selection bias)	Unclear risk	"Participants were notified as to which group they were randomized to after they completed the pretests."
Blinding of participants and personnel (performance bias) All outcomes	Unclear risk	Unclear if there was contact with researchers or clinicians.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=66 (%) <i>Comparison:</i> N=65 (%) Method of analysis: Unclear "At the end of eight weeks all 116 participants in the experimental group were sent the post-tests and 84 returned the tests. The data was randomly selected for analysis from the participants who completed the post-tests (n = 50 experimental group, n = 50 wait list control group, (See Figure 1). Also at the completion all 57 wait list control participants who returned the post-tests were given access to the MBSR information."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Johnston 2011

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 48.22% Randomised (N): 139 (93 intervention; 46 comparison)
Participants	Disorder: Mixed (GAD, PD, SAD); MINI (DSM-IV) Duration (years): Unknown

Johnston 2011 (Continued)

Age (years): 41.62

Sex (female): 58.8%

Race (white): 29%

Medication (using at baseline): 29%

Education (years): 13.91

EXCLUSIONS
Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "if taking medication (people taking benzodiazepines were excluded), had been taking the same dose for at least 1 month and did not intend to change that dose during the course of the program"

Drug/Alcohol: "not using illicit drugs or consuming more than three standard drinks a day"

Physical: N/A

Interventions
Comparison group(s): Wait-List

Comparison description: Wait list.

Intervention name: Anxiety Program (enhanced)

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=20.01

Provider contact: N=16.4 (69.39min)

Contact description: weekly telephone or email/asynchronous messaging contact with the Clinician or Coach, and regular automated reminder and notification emails Coach/Clinician: 7.56/7.54 phone calls, 8.88/8.83 mutual written contacts, 19.37/20.43 automated contacts, 69.09/69.59 minutes.

Intervention description: 1) Clinician-assisted, 2) Coach-assisted. "Eight online lessons; a summary/homework assignment for each lesson...access to additional written resources that included guidelines about managing low mood, improving sleep, and answers to frequently asked questions...also provided with access to de-identified vignettes written by participants in previous iCBT programs covering topics relevant to each of the eight lessons."

Compliance: 4% withdrew before the programme began or did not start. 72% completed all lessons.

Outcomes
General Symptoms: Depression Anxiety Stress Scales - 21 Item (DASS-21)

Depression: Patient Health Questionnaire (PHQ-9)

Johnston 2011 (Continued)

Disability: Sheehan Disability Scale (SDS)

Notes

Funding: National Health and Medical Research Council of Australia: Dora Lush Priority PhD scholarship, the New South Wales Institute of Psychiatry, and the Australian Rotary Health Research Fund

Request disaggregated data by disorder and data for each intervention. Did not use disorder-specific measures reported only for the total sample.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "randomized via a true randomization process (www.random.org)"
Allocation concealment (selection bias)	Low risk	"generated by an independent person, to either CL, CO or Control groups. The allocation sequence preceded pre-treatment diagnostic interviews and was concealed from LJ and JS"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "weekly telephone or email/asynchronous messaging contact with the Clinician or Coach, and regular automated reminder and notification emails"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=4 (4.30%) <i>Comparison:</i> N=4 (8.70%) Method of analysis: LOCF "Thirty-two of 43 (74%) CO and 35/46 CL (76%) group participants completed all eight lessons within the 10-week program; intention-to-treat (ITT) design and missing data was addressed by carrying forward the first available data (baseline-observation carried-forward; BOCF)"
Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12610000242022 NEO-5 and Agoraphobia Cognitions Questionnaire not reported. Treatment groups collapsed for analyses. Outcomes not reported for specific diagnoses.
Other bias	Low risk	N/A

Jones 2002a

Methods

Location: Manchester, England

Jones 2002a (Continued)

	Recruitment: Referral Exclusion rate: Unknown Randomised (N): 40 (20 intervention; 20 comparison)
Participants	Disorder: Health Anxiety; Other Duration (years): Unknown Age (years): 50.3 Sex (female): Unknown Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: "Half of the patients involved were also suffering from a medically diagnosed, physical-based problem"
Interventions	Comparison group(s): Treatment-as-usual Intervention name: Understanding Health Anxiety: A Self-help Guide for Suffers and their Families (Kuchemann, C & D Sanders; 1999) Days of treatment: 28 Author wrote: No Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=4 (180min) Contact description: Assessment and weekly sessions 1-hour (personal communication). 45 min intervention and 15 min feedback. Intervention description: "...consistent with the cognitive-behavioural model of health anxiety. The aim of the booklet is to educate the reader on how health worries come about, what keeps them going and to provide guidelines on what can be done."

Jones 2002a (Continued)

Compliance: After four weeks, the intervention group "confirmed that they had read the self-help booklet and completed the exercises."

Outcomes	Specific Symptoms: Health Anxiety Questionnaire (HAQ) General Symptoms: State-Trait Anxiety Inventory (STAI)
Notes	Funding: None acknowledged Unpublished data: Jones was contacted and provided information about the conduct of the study. She was unable to describe the randomisation process (see the Risk of Bias table).

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear They were then randomly allocated to two groups Asked about the number assigned to each group, Jones replied "I think it was 50-75", but 40 participants are described in the paper.
Allocation concealment (selection bias)	Unclear risk	"At the first interview all participants were given information about the study, then asked to complete the questionnaires assessing their health anxiety." Author reports "The participants were assigned to each group as they were referred to me they were assigned a number and assigned to each group by numbers." (personal communication.) "The participants were randomly assigned to each group as they were referred and before I had looked at the referral." (personal communication.)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Weekly sessions 1-hour (personal communication)
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Unclear risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (10%) <i>Comparison:</i> N=1 (5%) Method of analysis: Unclear Statistical tests indicate that 37 people are included in each of the post-treatment analyses. Details for dropouts are not reported; assumed groups were initially equal in size.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	High risk	Description of randomisation and allocation very poor. Number assigned unclear; we assumed participants were allocated evenly to groups.

Kenardy 2003

Methods	Location: Fife, SCO and Brisbane, AU, UK/Australia Recruitment: Advertising and referral Exclusion rate: 55.71% Randomised (N): 186 (50 intervention; 45 comparison)
Participants	Disorder: Panic disorder; SCID (DSM-IV) Duration (years): 5.95 Age (years): 36.8 Sex (female): 75.5% Race (white): 30.35% Medication (using at baseline): 30.35% Education (years): 11.9 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "All patients taking medication at the time of entry must have been on a stable dose for 3 months and must have been willing and able to remain on a stable regime for 3 months during the course of treatment." Drug/Alcohol: "no evidence of alcohol or drug dependence" Physical: "...no evidence of cardiovascular disease, asthma, epilepsy, or pregnancy or intention to become pregnant during the course of the study."
Interventions	Comparison group(s): SH + Individual CBT versus CBT alone Comparison description: Six 1-hr sessions. "The manuals provided specific detail of content to be covered within each session, including session agendas, information to be presented, homework exercises, and specific therapeutic procedures (e.g., interoceptive exposure, relapse prevention). The CBT approach was based on panic control treatment (Barlow, Cerny, & Klosko, 1989) that incorporated the cognitive and behavioral theories of panic (e.g., Barlow et al., 1989; Clark, 1986)..." Intervention name: Based on Panic Control Treatment (Barlow, Cerny and Klosko 1989) Days of treatment: 42 Author wrote: Yes Type of media: Computer Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0

Kenardy 2003 (Continued)

Provider contact: N=5.9 (354 min, common to both groups)

Contact description: 90 min assessment (personal communication). "On average, patients completed 5.9 therapy sessions in CBT6-CA and 5.9 sessions in CBT6. In the CBT6-CA condition, patients continued to carry the palmtop computer for the remaining 6 weeks, after which it was returned."

Intervention description: Participants in both group received the same face-to-face therapy. Intervention group also "The palmtop computer used in the CBT6-CA was the Hewlett Packard 200LX, which weighs about 300g...programmed to signal the participant five times daily...to prompt the practice of the therapy components. The computer program included a self-statement module, a breathing control module, and a new exposure module incorporating both situational exposure and interoceptive exposure."

Compliance: On average, participants in each group completed 5.9 therapy sessions. "Adherence data was complex but overall we achieved 96.8% adherence to treatment protocol. Scotland had 98.5% and Australia had 95.1%." (personal communication)

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: State-Trait Anxiety Inventory (STAI): Trait Response: Panic Free Depression: Beck Depression Inventory (BDI) Quality of Life: Medical Outcomes Study Short Form (SF-36)
Notes	Funding: National Health Service and Medical Research Council, Australia, and from the Chief Scientist's office, Scottish Executive, Scotland Unpublished data: Kenardy provided details about recruitment, demographics, randomisation, allocation concealment, numbers of participants in analyses, contact with participants. He also provided data for the BDI and the SF-36.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear "Patients who dropped out were replaced." "Additional cases were randomly assigned in the next "cohort" to make up the shortfall, we had four cohorts which recruited and randomised about 20 each but this number varied depending on the dropouts. In the last cohort we just went with what we had." (Personal communication)
Allocation concealment (selection bias)	High risk	"Patients who dropped out were replaced." Administered by a third party (research assistant). "Additional cases were randomly assigned in the next "cohort" to make up the shortfall, we had four cohorts which recruited and randomised about 20 each but this number varied depending on the dropouts. In the last cohort we just went with what we had." (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "each therapist treating approximately equal proportions of patients in each of the three treatment conditions."
Blinding of outcome assessment (detection bias)	Low risk	No assessor-rated outcomes.

Kenardy 2003 (Continued)

All outcomes

Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=15 (30%) <i>Comparison:</i> N=11 (24.44%) Method of analysis: Per protocol 12% did not complete at least three sessions of treatment or provide "adequate data." "and were defined as dropouts. An additional 13 participants were lost to follow-up, leaving 93 participants with 9-month follow-up analysis." "An intent-to-treat analysis was also conducted in which all cases with available pretreatment data were carried forward...The intent-to-treat analysis was limited to 3-months posttreatment because it was judged that extrapolation of pretreatment data to the 6-month follow-up was untenable."
Selective reporting (reporting bias)	Low risk	No trial registration number given. BDI and SF-36 are not reported in the paper, but received from the author.
Other bias	High risk	"Patients who failed to receive at least three sessions of their respective course of treatment or who failed to provide adequate pretreatment and endpoint data were defined as dropouts. Patients who dropped out were replaced."

Kiely 2002

Methods	Location: Herefordshire, England Recruitment: Referral Exclusion rate: 0% Randomised (N): 60 (20 intervention; 20, 20 comparison)
Participants	Disorder: Generalised Anxiety Disorder; ADIS (DSM-III R) Duration (years): 7.6 Age (years): 41.1 Sex (female): 54.35% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current and previous therapy restricted Medication: N/A

Kiely 2002 (Continued)

Drug/Alcohol: "alcohol or substance misuse"

Physical: N/A

Interventions

Comparison group(s): Wait-list, Psychoeducation

Comparison description: 1) Wait-list. 2) "The IOP comprised two booklets of 27 pages total length containing general information, about the causes and consequences of anxiety, including the common symptoms of anxiety, Lazarus and Folkman's (1984) model of stress and Cannon's (1929) Fight-Flight survival mechanism."

Intervention name: Not named

Days of treatment: 91

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=1 (10 min)

Contact description: The total assessment interview took up to 120 minutes. "Those in the treatment group were introduced to the intervention through a 10 minute presentation following assessment."

Intervention description: "two booklets of 27 pages total length...common symptoms...plus six others, totalling 106 pages and an audio cassette detailing relaxation exercises." "detect internal and external anxiety cues and to learn to apply new coping strategies...self-monitoring of thoughts, feelings and behaviours (Durham and Turvey, 1987), cognitive change (Beck and Emery, 1985)..."

Compliance: Author asked about use of material and strategies implemented in addition to amount read and frequency of reading. Measures of advice implemented are compared between groups.

Outcomes

Specific Symptoms: Penn State Worry Questionnaire (PSWQ)

General Symptoms: Beck Anxiety Inventory (BAI); Hospital Anxiety and Depression Scale (HADS): Anxiety

Depression: Hospital Anxiety and Depression Scale (HADS): Depression

Notes

Funding: "part of my post in an NHS psychology service" (personal communication)

Unpublished data:

Kiely provided unpublished papers with study details, demographics, outcome data.

Risk of bias
Bias
Authors' judgement
Support for judgement

Random sequence generation (selection bias)

Low risk

Method: Other

"with reference to a prepared list of 60 spaces, numbered 1-60 consecutively, each number being allocated in advance to one of the 3 conditions via a random number table."

Kiely 2002 (Continued)

Allocation concealment (selection bias)	Low risk	“The assessor only referred to the randomised list once a patient was allocated to the next available space on the main list –hence they were unaware of the relevant condition in advance of allocation.”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Those receiving the IOP or FP were introduced to the respective programmes by the assessor using a scripted presentation lasting up to 10 minutes.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention</i> : N=4 (20%) <i>Comparison (wait-list)</i> : N=6 (30%) <i>Comparison (psychoeducation)</i> : N=4 (20%) Method of analysis : Available Case “independent Clinical Psychologist...blind to treatment condition and patient’s scores...read the initial assessments on each patient, including the ADIS-R interview and their biographical self-completion questionnaire. They then assigned a diagnosis, primary and secondary, to each patient. If the independent assessor decided a patient did not meet the criteria for the study, the patient’s results were excluded from the subsequent analysis. 12 were excluded on this basis. 2 failed to attend follow-up. Participants with complete pre- and post-treatment data were included in the analyses.”
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Unclear risk	“12 patients included (and randomised) were then excluded from analyses because didn’t have primary diagnosis of GAD - and unclear how many in each group.”

Kiropoulos 2008

Methods	Location : Victoria, Australia Recruitment : Advertising/ Internet Exclusion rate : 89.24% Randomised (N) : 86 (46 intervention; 40 comparison)
Participants	Disorder : Panic disorder; ADIS (DSM-IV) Duration (years) : Unknown Age (years) : 38.96 Sex (female) : 72.09% Race (white) : 48.24%

Kiropoulos 2008 (Continued)

Medication (using at baseline): 48.24%

Education (years): 12.53

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Those participants who were taking medication for anxiety or depression were accepted if they had been stabilised on their medication for at least 12 weeks but continued to experience panic symptoms and met a diagnosis of PD."

Drug/Alcohol: "alcohol or drug dependency"

Physical: "heart condition...chronic hypertension"

Interventions

Comparison group(s): Individual CBT

Comparison description: "Participants attended one hour weekly treatment sessions with their allocated psychologist...given a copy of the manual 'Mastery of Your Anxiety and Panic – Third Edition' (Barlow & Craske, 2000)...focuses on teaching participants a variety of cognitive and behavioral strategies that include controlled breathing, cognitive restructuring, and interoceptive and situational exposure...weekly reading and were instructed to work through this manual with the assistance of the treating psychologist."

Intervention name: Panic Online (Klein and Richards)

Days of treatment: 84

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=0

Provider contact: N=18.6 (352min)

Contact description: See Table 4. Participants received 10.64 emails and sent 18.24. Therapists made .36 phone calls per participant. Author estimated assessment lasted 60 to 90 min (personal communication). "All psychologists interacted with their participant via email, which allowed the psychologist to provide individualized support and feedback to the participant, according to the participants' individual needs...maintained a log of the time they spent..."

Intervention description: "introductory module, four learning modules, and a relapse prevention module...instructions for controlled breathing, cognitive restructuring, and interoceptive and situational exposure...participants had access to the stress and benzodiazepine reduction modules...did not vary according to participant input. Participants were instructed to read one specified module per week and to practice the described activities in this module."

Compliance: "Participants in the face-to-face CBT treatment group completed a mean 10.96 sessions of the 12-week treatment program. Unfortunately, no data is available about how many modules the PO participants completed." "Therapists treating participants in the face-to-face CBT rated their par-

Kiropoulos 2008 (Continued)

ticipants as having higher compliance to treatment and having greater understanding of the treatment material given to them compared to the PO participants."

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Anxiety Sensitivity Index (ASI); Depression, Anxiety, Stress Scales (DASS): Anxiety Response: Panic Free Recovery: High End-State Functioning Depression: Depression, Anxiety, Stress Scales (DASS): Depression Quality of Life: WHO Quality of Life - BREF
Notes	Funding: National Health and Medical Research Council. Panic Online developed by Jeffrey Richards with funds from the Australian Rotary Health Research Fund. Unpublished data: Kiropoulos confirmed contact, duration of follow-up, was aware of the allocation sequence. Further baseline measures not recorded, unable to reconcile discrepancy between education in text (p. 1275) and Table 1. Unable to provide weekly panic attacks or to recall assignment of 22 missing cases.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Other "After all pre-assessment questionnaires were completed by the participants, they were randomly allocated using a random numbers table to either the PO or face-to-face CBT treatment condition."
Allocation concealment (selection bias)	Low risk	All assessors were blind to treatment allocation of eligible participants into the study. "I had access and knowledge of the sequence table only. The manager of the project (me) allocated participants into the groups based on the random numbers table."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "All psychologists interacted with their participant via email"
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "Additionally the ADIS-IV was also conducted during a face-to-face interview by the assessor. Clinicians providing treatment to participants also completed the TAQ at post-assessment for each participant they were allocated."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "Data analysis involved intention-to-treat analyses. Participants who had missing post-assessment questionnaire(s) (n=22) or who discontinued treat-

Kiropoulos 2008 (Continued)

ment (n = 7) were treated as 'intention to treat' (ITT). That is, participants' pre-assessment scores were carried forward and used in post treatment. 5 intervention and 2 controls discontinued treatment. 22 had missing post-assessment questionnaires."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Unclear risk	N/A

Klein 2001

Methods	Location: Melbourne or Victoria, Australia Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 23 (11 intervention; 12 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-IV) Duration (years): Unknown Age (years): 40.82 Sex (female): 86.36% Race (white): 40.91% Medication (using at baseline): 40.91% Education (years): 11.7 EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "Nine participants (41%) reported the use of anxiety medication but subsequently all reported no alteration in their dosage levels throughout the 3 weeks." Drug/Alcohol: Measured by the Prime MD Physical: "absence of any significant physical health problems"
Interventions	Comparison group(s): Wait-List Comparison description: Self-monitoring only Intervention name: Panic Online (Klein and Richards) Days of treatment: 21 Author wrote: Yes Type of media: Internet

Klein 2001 (Continued)

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: Unknown

Contact description: Provided assistance loading programme. Random telephone checks conducted to ensure compliance. Average length of calls was 5 minutes and is included in the time reported for assessment. Assessment took 2.5 to 3 hours (personal communication) and the lower estimate was extracted.

Intervention description: 2 x 45-min sessions: "nature, effects and causes of panic, and the second on useful and non-useful ways of managing panic. Negative self- statements were discussed as were errors in thinking. Brief techniques on how to overcome these cognitive errors were explained...considerable use of colour, animated illustrations, hyperlinks between sections of the text, and self- assessment quizzes with immediate feedback."

Compliance: None reported, though the study indicates that information about progress was obtained from participants.

Outcomes	Specific Symptoms: Body Vigilance Scale (BVS) General Symptoms: Anxiety Sensitivity Index (ASI)
Notes	Funding: None acknowledged Unpublished data: Klein provided unpublished outcome data and details of contact with participants.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "Participants were randomly assigned..."
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Random telephone checks conducted to ensure compliance" (personal communication)
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (9.09%) <i>Comparison:</i> N=0 (0%)

Method of analysis: Available Case

Klein 2001 (Continued)

		“One participant failed to complete the study and subsequently her data were excluded.”
Selective reporting (reporting bias)	Low risk	No trial registration number given. Full outcome data were provided by the author.
Other bias	Low risk	N/A

Klein 2006

Methods	Location: Australia Recruitment: Advertising/ Internet Exclusion rate: 57.69% Randomised (N): 55 (37 intervention; 18 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-IV) Duration (years): Unknown Age (years): Unknown Sex (female): 80% Race (white): 52.73% Medication (using at baseline): 52.73% Education (years): 12.5 EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "Those participants who were taking medication for anxiety or depression were accepted if they had been stabilised on their medication for at least 4 weeks but continued to experience panic symptoms and met a diagnosis of PD." Drug/Alcohol: "alcohol or drug dependency" Physical: "stroke...heart condition...or chronic hypertension"
Interventions	Comparison group(s): Attention/Monitoring Comparison description: participated in the same telephone and online assessments as those in the PO and MAN conditions...contacted the IC participants each week and inquired about their current panic status. She provided minimal support (e.g., empathic listening) and asked participants whether they had been doing their daily monitoring and advised them to re-read the information on an internet-based informational program... Intervention name: Panic Online (Klein and Richards) Days of treatment: 54

Klein 2006 (Continued)

Author wrote: Yes

Type of media: Mixed

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=0

Provider contact: N=12.45 (292.85min)

Contact description: Internet: "Therapist interaction occurred via email, enabling the therapist to provide individualised support and feedback to the participant, according to their requests and needs." Mean 16.39 emails sent. Book: "informed that a therapist would telephone them at home once weekly to assist them and monitor their progress...The therapist was required to keep a log of amount of time spent..." Mean 7.73 calls.

Intervention description: Two groups combined 1) "6-week structured program comprised of an introductory module, four learning modules, and a relapse prevention module...controlled breathing, cognitive restructuring, and interoceptive and situational exposure." "did not vary according to participant input." 2) "MAP-3 (Barlow & Craske, 2000) free of charge...cognitive and behavioural strategies that include controlled breathing, cognitive restructuring, and interoceptive and situational exposure as in PO."

Compliance: Participants reported using the program mostly from home and rarely at their work-places."

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Anxiety Sensitivity Profile (ASP); Depression, Anxiety, Stress Scales (DASS): Anxiety Recovery: High End-State Functioning Depression: Depression, Anxiety, Stress Scales (DASS): Depression
----------	--

Notes	Funding: Australian Rotary Health Research Fund
-------	--

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Other After assessment, all participants were randomly assigned sequentially (i.e., ABC, ABC) using a block design to internet-based treatment, treatment with a self-help manual with limited therapist assistance, or an internet-based information control (IC) condition.
Allocation concealment (selection bias)	Low risk	The assessors were blind to which treatment the participant would be assigned to until after the pre-assessment was completed.
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) ...enabling the therapist to provide individualised support and feedback to the participant, according to their requests and needs
Blinding of outcome assessment (detection bias)	Low risk	Included assessor-rated outcomes (blind)

Klein 2006 (Continued)

All outcomes		“The clinician who conducted the post- and follow-up assessments did not provide the treatment.”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF Data analysis involved intention-to-treat analyses. That is, for those participants who discontinued their involvement during treatment (n= 9), their pre-assessment scores were carried forward and used in both the post-treatment (PO, MAN, and IC) and follow-up assessments (PO and MAN).
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Koszycki 2011

Methods	Location: Canada Recruitment: Advertising and referral Exclusion rate: 13.1% Randomised (N): 251 (126 intervention; 125 comparison)
Participants	Disorder: Panic disorder; SCID (DSM-IV) Duration (years): 9.83 Age (years): 36.15 Sex (female): 40.3% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: "use of any psychotropics within 14 days of the baseline visit (6 weeks for fluoxetine)" Drug/Alcohol: Other primary comorbid disorder like generalized anxiety disorder, social phobia, somatization disorder and specific phobia. Other psychiatric disorders; electroconvulsive therapy in the past 6 months; if on Oxazepam, a maximum daily dose of 15 mg, 60 mg

Koszycki 2011 (Continued)

Physical: "a history of psychosurgery; significant medical conditions; abnormal laboratory findings; a hypersensitivity to serotonergic agents; a history of non-response to sertraline; lactose intolerance"

Interventions	Comparison group(s): Sertraline only, Placebo Only Intervention name: Not named Days of treatment: 84 Author wrote: Yes Type of media: Audio Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=8 (duration unknown) Contact description: Tapes were distributed weekly during acute treatment by a research coordinator and a standard format was adopted for instructions to be given to patients. Compliance was assessed at each visit by asking patients how much time they spent listening to the tape, whether they attempted the suggested homework and whether they recorded their homework in the workbook. Patients who entered the 12-week extension phase were given the CBT package to use at their own discretion and no particular instructions were given. Intervention description: SCBT+ Sertraline and SCBT +Placebo "SCBT consisted of 12 audiotapes and a workbook that contained monitoring forms for homework. The tapes and workbook were developed for this study by psychologists with expertise in CBT, psychoeducation about anxiety and the cognitive model of PD, breathing retraining and relaxation skills, cognitive restructuring that addressed misappraisal of panic symptoms, interoceptive and situational exposure, and relapse prevention." Compliance: "Compliance was assessed at each visit by asking patients how much time they spent listening to the tape, whether they attempted the suggested homework and whether they recorded their homework in the workbook."
Outcomes	None included.
Notes	Funding: Canadian Institutes of Health Research (POP-15247) and Pfizer Canada.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Patients were randomly allocated to one of four groups by a computer generated randomization code"
Allocation concealment (selection bias)	Low risk	"Investigators at each site were provided with a sealed envelope that contained the identification of the study drug being administered to the patient. In a medical emergency, the investigator was authorized to break the code for that subject only."
Blinding of participants and personnel (performance bias)	Low risk	Provider contact: Yes Provider blind: Yes

Koszycki 2011 (Continued)

All outcomes		Placebo and sertraline were provided as matching capsules and administered double-blind. "Patients were instructed not to divulge their SCBT assignment to the investigators."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "Outcome assessments were made by investigators who were blind to allocation of the drug and who were not told whether the patient was assigned to SCBT. Patients were instructed not to divulge their SCBT assignment to the investigators."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (2.38%) <i>Comparison:</i> N=1 (0.80%) Method of analysis: Available Case "The mixed models were estimated by means of Restricted Maximum Likelihood (REML), and degrees of freedom were computed using the Kenward-Roger approach (Littell et al. 2006)."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Lewis 1978

Methods	Location: Eugene (Oregon), USA Recruitment: College only Exclusion rate: 19.74% Randomised (N): 62 (50 intervention; 12 comparison)
Participants	Disorder: Mixed (not specified); No diagnostic interview (questionnaire only) Duration (years): 4.9 Age (years): 23.88 Sex (female): 62.3% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: No restrictions reported

Lewis 1978 (Continued)

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Attention/Monitoring Comparison description: Subjects in this condition were asked to continue monitoring their daily anxiety experiences for an additional 2 weeks. In addition, they were reminded that there is evidence that self-monitoring helps some people change their behavior. Intervention name: Don't be afraid: A self-instructional program for reducing fears and phobia (Rosen, 1975) Days of treatment: 21 Author wrote: No Type of media: Book / Booklet Type of therapy: Muscle Relaxation Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=1 (40min) Contact description: Phone call, then "During the first appointment the study was described...subjects filled out all pretreatment assessment instruments, were assisted in identifying situations that made them anxious (Situations A and B), and were given instructions on self-monitoring their anxiety. After 1 week of self-monitoring, subjects returned to the clinic and met with the experimenter..." "contact with the subject over all three sessions was typically 1 hour and 40 minutes. The bulk of this time (approximately 60 minutes) was spent in subject's completing the relevant questionnaires." Intervention description: 1) "manual on deep muscle relaxation, which was written by Rosen (1975, 1976)...The manual suggested that the client practice each day in two 15- to 20-minute sessions". 2) Pamphlet suggesting 15- to 20-min daily using any method in anxiety provoking situations. Half of the participants in each group "were asked to deposit an amount of money that they "wouldn't want to lose." Compliance: Groups reported 12.75 to 17.64 mean practice sessions of 15.81 to 24.24 minutes over two weeks. Expectancy correlated with compliance, ease relaxing improvement on the STAI and on daily rating of tension. Adherence did not predict outcomes. Several clients said the manual was unclear and difficult to follow and some practice sessions would have helped.
---------------	---

Outcomes	General Symptoms: State-Trait Anxiety Inventory (STAI): State
Notes	Funding: National Institute of Mental Health. Grant MH27559-01. Unpublished data: Unable to locate Lewis. Biglan had no further information.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear

Lewis 1978 (Continued)

Following a week of self-monitoring, subjects were randomly assigned to one of five conditions. Assignments were done separately for men and women in order to equate the proportion of men and women in each condition.		
Allocation concealment (selection bias)	Unclear risk	Randomisation followed assessment and a week of self-monitoring.
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) All participants were treated by the same person, who was aware of assignment.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=8 (16%) <i>Comparison:</i> N=2 (16.67%) Method of analysis: Unclear 10 dropouts. Assumed participants were randomised in equal numbers and assumed equal dropout between groups.
Selective reporting (reporting bias)	High risk	No trial registration number given. Several outcome measures are not reported: Ability to relax, Sense of self-control, Benefit derived from programme, Likelihood they would use the techniques, Internal locus of control, How easy it was to get relaxed during sessions.
Other bias	Unclear risk	N/A

Lidren 1994

Methods	Location: Washington and Idaho, USA Recruitment: Advertising and referral Exclusion rate: 76% Randomised (N): 36 (12 intervention; 12, 12 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-III R) Duration (years): 6.23 Age (years): 34.85 Sex (female): 69.44% Race (white): 38.89% Medication (using at baseline): 38.89%

Lidren 1994 (Continued)

Education (years): 14.1

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "subjects taking medication for anxiety were allowed to participate in the study if they still suffered from panic symptoms after at least six weeks of stabilization of their medication; if they maintained the same dosages throughout treatment; and if the recorded daily both type and amount of medication usage"

Drug/Alcohol: "...chronic use of alcohol, drug dependence or abuse..."

Physical: "Before the initiation of the study, a complete physical by a physician was required to rule out any organic cause for panic symptoms and/or complicating physical ailments. Researchers excluded: seizure disorders, kidney disease, stroke...emphysema, heart"

Interventions

Comparison group(s): Wait-list, Individual CBT

Comparison description: 1) Wait-list; 2) "met once a week for eight weeks for an hour and a half each session. All sessions were conducted by the same advanced clinical graduate student...Each participant in this condition was also responsible for reading the entire book *Coping with Panic*."

Intervention name: *Coping with Panic: A Drug-Free Approach to Dealing with Anxiety Attacks* (G Clum; 1990)

Days of treatment: 56

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=3 (75 min)

Contact description: Three calls lasting 25 minutes each following written protocol and did not answer questions or discuss treatment. Quizzed about the chapters in the book. (personal communication)

Intervention description: responsible for reading the entire book, *Coping with Panic*, and mailing in weekly homework assignments...cognitive restructuring, breathing retraining, and exposure as well as advise individuals how to implement these strategies...exploring faulty cognitions, reattributing, seeking alternatives, decatastrophizing symptoms, and testing faulty hypothesis. Subjects purchased the book for \$15 from the bookstore and paid \$30 (half was refunded upon completion of the study).

Compliance: Compliance measured through telephone interviews, diary and questionnaire. Intervention group practiced 2.8 (4.1) hours per week, group therapy participants practiced 3.6 (3.7). All participants in the intervention group got 70% or more of the answers correct at each of the three calling periods.

Outcomes

Specific Symptoms: Mobility Inventory for Agoraphobia (MIA)

Lidren 1994 (Continued)

Response: Panic Attacks: Frequency

Depression: Beck Depression Inventory (BDI)

Notes

Funding: None acknowledged

Unpublished data: Watkins provided information about contact with participants and confirmed that assessment was not blind and there was no attrition. No further information was available from the authors.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	"Individuals who met criteria for PD and agreed to participate in the study were randomly assigned to one of three conditions..."
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Interacted with researcher/clinician throughout the trial, who was not blind (Personal communication).
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: No dropout
Selective reporting (reporting bias)	Unclear risk	Main outcomes reported clearly in published study, but unable to extract data about medication usage.
Other bias	Low risk	N/A

Marks 2004

Methods

Location: London, England

Recruitment: Advertising and referral

Exclusion rate: 27.91%

Randomised (N): 93 (37 intervention; 17, 39 comparison)

Participants

Disorder: Mixed (AG, PD, SAD, SP); Clinical Interview (DSM-IV)

Duration (years): 17.13

Age (years): 38.3

Marks 2004 (Continued)

Sex (female): 68.89%

Race (white): 22.22%

Medication (using at baseline): 22.22%

Education (years): 11.24

EXCLUSIONS

Depression: Yes

Psychotherapy: No restrictions reported

Medication: "not on a benzodiazepine or a diazepam-equivalent dose of >5 mg/day...had not begun or changed dose or type of anti-depressant medication within the last 4 weeks."

Drug/Alcohol: "not on >21 units (men) or >14 units (women) of alcohol a week"

Physical: "disabling cardiac or respiratory disease"

Interventions

Comparison group(s): Relaxation, Individual Exposure

Comparison description: 1) "guided in self-relaxation techniques by a computer which explained the relaxation rationale, taught the practice of relaxation exercises with a biofeedback relaxation-training program (de-STRESS, 1997) and advised daily relaxation homework for 40 min daily between sessions. To facilitate homework, patients were given relaxation audio-tapes." 2) "similar self-exposure instruction, but guided entirely face-to-face by a clinician".

Intervention name: Fear Fighter (Marks 2004)

Days of treatment: 70

Author wrote: Yes

Type of media: Computer

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=6 (76min)

Contact description: They were screened in a 25-min semi-structured interview to confirm a phobia/panic diagnosis via a checklist of relevant DSM-IV criteria. "each hour-long session also included brief face-to-face help from a clinician : a mean of up to 5 min coaching and reviewing progress beforehand and up to 15 min discussing progress and giving extra treatment advice at the end." Mean therapist time 76 (43) minutes during treatment, 17 (5.4) during 1-month follow-up.

Intervention description: six hour-long individual treatment sessions "...self-exposure therapy. Step 3 explained how to recruit and work with a co-therapist. Step 4 helped clients identify the triggers...identify and set individualized exposure homework...coping tactics to remain in panic-evoking situations. Step 7 gave clients coping exercises to practise during exposure. Step 8 reviewed exposure homework, gave feedback and helped users extend gains...Step 9 involved troubleshooting..."

Compliance: [Relaxation] patients attended more treatment sessions than [face-to-face therapy] patients (R mean=5.7, SD=0.9; F mean=4.2, SD=2.2 ; Mann-Whitney U test= 186, Z=x2.26, P=0.008) and did more homework sessions than [face-to-face therapy] and [computerised] patients (R mean= 46, SD=24 ;

Marks 2004 (Continued)

F mean=20, SD=17 ; C mean= 24, SD=19; F(2,86)=11.02, P<0.001), but total homework time did not differ among F, C and R."

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Global Phobia Disability: Work and Social Adjustment Scale (WSAS)
Notes	Funding: EU Marie Curie Fellowship. Marks owns Fear Fighter. Unpublished data: Marks provided an unpublished manuscript. Mataix-Cols provided unpublished follow-up data. Unable to provide disaggregated data for different disorders.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "... based on a computer-generated set of random numbers. Suitable patients were randomized to one of three treatment groups in a 2F : 2C : 1R ratio..."
Allocation concealment (selection bias)	Low risk	"At the end of the screening interview the screening clinician obtained the patient's randomization by asking the unit administrator to open the next opaque, sealed envelope from one of three sets (one per phobia type)..."
Blinding of participants and personnel (performance bias) All outcomes	High risk	"Patients had six hour-long individual treatment sessions over 10 weeks and follow-up 1 and 3 months later. The three therapists (two nurses and a psychiatrist) were experienced behaviour therapists. They standardized each treatment condition by using printed session-by-session guides."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) Assessors blindness was tested by asking them at post-treatment to guess the treatment group of the patient.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=16 (43.24%) <i>Comparison (Relaxation):</i> N=1 (5.88%) <i>Comparison (Individual Exposure):</i> N=10 (25.64%) Method of analysis: Available Case "All analyses were intention-to-treat by analysing subjects in the group to which they were originally randomized (Everitt, 1994). Longitudinal data were analysed for all randomized patients for whom post-baseline data were available. Where post-baseline data were unavailable, baseline data were not carried forward in the manner often done..." "Two patients who had been randomized to F (one completer, one drop-out) could not be included in the analysis as their files disappeared when the team moved to a new site on 28.9.00, as did data from one patient who was withdrawn on becoming severely depressed..."
Selective reporting (reporting bias)	Low risk	No trial registration number given.

Marks 2004 (Continued)

The author provided follow-up data. "Many patients' self-ratings posted at follow-up went astray when most of the team moved from the Maudsley to Charing Cross Hospital and staff changed."

Other bias	Low risk	N/A
------------	----------	-----

Matthews 2011

Methods	Location: Hobart, Australia Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 26 (13 intervention; 13 comparison)
Participants	Disorder: Specific Phobia; Clinical Interview (DSM-IV) Duration (years): Unknown Age (years): 38 Sex (female): 100% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 11.5 EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: N/A Drug/Alcohol: N/A Physical: "any serious medical condition"
Interventions	Comparison group(s): Self-help (Exposure) Comparison description: Intermittent exposure using feardrop.com. "The intermittent stage involved exposure to the same fear-relevant spider stimulus, alternating every 2 minutes with an irrelevant stimulus (beach ball), resulting in four 2-minute fear-relevant exposure trials and three 2-minute fear-irrelevant exposure trials." Intervention name: feardrop.com Days of treatment: 1 Author wrote: Yes Type of media: Internet Type of therapy: Exposure

Matthews 2011 (Continued)

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=1 (duration unknown)

Contact description: SUDS ratings were completed prior to exposure (0 minutes), at 2-minute intervals during the task (2, 4, 6, 8, 10, and 12 minutes) and at completion of the task (14 minutes).

Intervention description: Continuous exposure using feardrop.com. "exposure to an image of a fear-relevant stimulus (Huntsman spider) for seven 2-minute trials." "Photographs of huntsmen spiders were taken by the researchers and associates or were obtained from online sources with a creative commons licence...All home-based stages were continuous. Each stage lasted 6 minutes. SUDS ratings were completed prior to each stage (0 minutes), during each stage (at 2 and 4 minutes), and at the end of each stage (6 minutes)."

Compliance: Lab-based. Measured "Time spent outside the tracking circle"

Outcomes	None included.
----------	----------------

Notes	Funding: None acknowledged
-------	-----------------------------------

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear sequentially allocated to two exposure groups (intermittent exposure or continuous exposure)
Allocation concealment (selection bias)	Unclear risk	recruited following the screening interview and were sequentially allocated
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) For both tasks SUDS ratings were completed prior to exposure (0 minutes), at 2-minute intervals during the task (2, 4, 6, 8, 10, and 12 minutes) and at completion of the task (14 minutes).
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (15.38%) <i>Comparison (other):</i> N=1 (7.69%)

Method of analysis: Available Case

The data for three participants were excluded due to technical issues with the program "Six participants did not complete the 1-week post-treatment assessment and the home-based exposure task, and a further three participants did not complete the one-month post treatment assessment. Thus full

Matthews 2011 (Continued)

		follow-up data were available for 14 participants." "Outcome measures at all three assessment times were available for 14 participants."
Selective reporting (reporting bias)	High risk	No trial registration number given. "These data were not analysed by exposure group, due to small numbers in the intermittent (n = 6) and continuous (n = 8) groups."
Other bias	Low risk	N/A

Maunder 2009

Methods	Location: Northumberland, England Recruitment: Prison Exclusion rate: 43.902439024% Randomised (N): 38 (20 intervention; 18 comparison)
Participants	Disorder: Mixed, but due to the small number of participants recruited with depression, only results from the anxiety group were analysed ; No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 35.22 Sex (female): 0% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 11.53 EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: "During the study both groups also experienced treatment as usual from the primary health care teams and referral to the mental health in-reach team as appropriate. This included medication, support, counselling, or other psychological therapies." Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-List Comparison description: "All received other mental health treatment as usual." Intervention name: Not named Days of treatment: 28 Author wrote: Yes

Maunder 2009 (Continued)

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: there was little trouble with literacy during the study, though occasional help was needed. "It is also unclear whether there would have been added benefit from guidance on the self-help materials from a health or other worker. About half of the participants in the study indicated that this might have been useful."

Intervention description: They are based on the Northumberland, Tyne and Wear NHS Trust booklets, and use CBT principles. "The materials had been redrafted and re-designed, with the aid of prisoners, to make them suitable for this population (Maunder et al., 2004)."

Compliance: N/A

Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety
Notes	Funding: The study was supported by a small research grant from Northumberland Care Trust. Depression groups excluded from this review

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "Blocked randomization using computer-generated number sequences was used to allocate participants to the intervention (group B) or control (group A) in both the anxiety and depression groups."
Allocation concealment (selection bias)	Low risk	"The researcher, blind to the allocation of groups, administered the HADS and BSI" "participants assigned to the intervention group received the self-help booklet for anxiety or depression in an opaque envelope (for privacy and for the purpose of randomisation)."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "there was little trouble with literacy during the study, though occasional help was needed."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "The study was a pilot single-blind randomized controlled trial (RCT)." "The researcher, blind to the allocation of groups, administered the HADS and BSI" ""The post-intervention evaluative questionnaire (conducted in the form of a semi-structured interview) was completed with the intervention group at the second assessment and the control group at the third assessment."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (25%)

Maunder 2009 (Continued)

Comparison: N=0 (0%)

Method of analysis: Available Case

"As a result of DNA's, withdrawals, discharges and transfers only 82.6% completed the second assessment and 71.7% completed the third assessment."

Selective reporting (reporting bias)	Unclear risk	Trial registration number: ISRCTN54760645 Results for the BSI are not reported in full. Results for the depression group are not reported at all, but these would not have been included in this review. Trial registration only lists the primary outcome (HADS).
Other bias	Low risk	N/A

McNamee 1989

Methods	Location: London, England Recruitment: Advertising/ Internet Exclusion rate: 37.84% Randomised (N): 23 (13 intervention; 10 comparison)
Participants	Disorder: Panic disorder w/ AG; SCID (DSM III) Duration (years): 12 Age (years): 45 Sex (female): 88.89% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current and previous therapy restricted Medication: "no more than 10mg diazepam or its equivalent" Drug/Alcohol: "...no more than 2 pints of beer daily." Physical: "no serious cardiac or pulmonary disease"
Interventions	Comparison group(s): Applied relaxation (self-help) Comparison description: "relaxation tapes and had the same format of telephone consultations over 12 weeks. The rationale given to them was: 'You become anxious and panicky in feared situations and so avoid them. Over the next 12 weeks we will teach you how to relax so that you may overcome your phobia.'...If control subjects asked whether to enter phobic situations they were told: 'It is entirely up to you to do whatever you feel comfortable with.'"

McNamee 1989 (Continued)

Intervention name: Living With Fear (Marks, 1980)

Days of treatment: 84

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Phone

Automated contact: N=0

Provider contact: N=10 (120 min)

Contact description: "They spoke on the phone with a therapist for 12 mins weekly for 8 weeks and then 2-weekly for a further 4 weeks i.e. for 10 times in all for a total of 2 hours over 12 weeks. During telephone contact new goals were set each week." "Total time spent in telephone contact did not differ in the 2 groups."

Intervention description: "posted to them a self-help manual (Living with Fear, Marks 1978) with the rationale: Only by facing the situations that evoke fear and enduring the ensuing discomfort until it subsides will you overcome your problem...They were encouraged to have feelings of discomfort...breathing control and positive self-statements."

Compliance: 46% intervention and 80% controls completed 12 week treatment. "Subjects were asked to practice self-exposure of these goals at least 4 times a week for 2 hours at a time, and to record these self-exposure activities in homework diaries."

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Global Phobia, assessor rated Response: Target Phobias Disability: Work and Adjustment Rating Scale (WARS), assessor rated
Notes	Funding: Upjohn Company Unpublished data: Requested further information. Lelliott replied that data may be lost.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "Of these, 23 agreed to self-treatment guided by phone and were randomized..."
Allocation concealment (selection bias)	Unclear risk	"Evaluation at baseline was by a blind evaluator..."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "One therapist (GM) gave the telephoned guidance to both exposure and control groups throughout the study."

McNamee 1989 (Continued)

Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “further telephoned blind evaluations were done at weeks 2,4,6,8,10,12,20 and 32.”
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=7 (53.85%) <i>Comparison:</i> N=2 (20%) Method of analysis: Available Case “Before week six, 5 patients dropped out leaving no data for outcome analysis (4e, 1r). At week six, 18 cases were evaluated (9e, 9r). Thereafter, 4 ceased contact (3e - 1 had too many panics, 1 became depressed and needed antidepressants, 1 died; 1r - was 'fed up'). The 14 patients (6e, 8r) were assessed at week 32 i.e. follow-up at 5 months posttreatment. One exposure case at week 6 developed complications of pregnancy precluding her doing exposure homework, but was retained in the analysis. The 18 evaluable subjects...”
Selective reporting (reporting bias)	High risk	No trial registration number given. Self-ratings were of the Fear Questionnaire (Marks & Mathews 1979), 4 phobic targets (each 0-8), social adjustment (as above), Beck Depression Inventory (21-item). Homework, phobic outings and panics were recorded in weekly diaries which patients were asked to return by post fortnightly. "Assessor ratings were used in the analysis as subjects had not consistently returned self-ratings."
Other bias	Low risk	N/A

Mead 2005

Methods	Location: North West, England Recruitment: Referral Exclusion rate: 81.81% Randomised (N): 114 (57 intervention; 57 comparison)
Participants	Disorder: Mixed (presenting complaint for 69% of participants was an anxiety disorder or mixed anxiety with depression; 51% anxiety alone); No diagnostic interview (questionnaire and GP report) Duration (years): Unknown Age (years): 39.75 Sex (female): 67.54% Race (white): 66.5% Medication (using at baseline): 66.5% Education (years): 13.52

Mead 2005 (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: "current involvement with other statutory specialist mental health services (e.g. substance abuse)"

Physical: N/A

Interventions

Comparison group(s): Wait-List

Comparison description: The WLC group involved routine care from primary-care professionals, which may involve general support, antidepressant medication or referral to community agencies.

Intervention name: Self-Help in Anxiety and Depression: SHADE (Richards and Lovell).

Days of treatment: 91

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=3.16 (101.12min)

Contact description: 50 (88%) attended at least one session, and 31 (54%) attended all four. The mean number of sessions attended was 3.16 (SD=1.94) and the mean session length was 32 minutes (SD=6.3).

Intervention description: "...information about anxiety and depression and available intervention-s...descriptions of patients' experiences, derived from focus groups with service users, who were also involved in evaluating the draft manual. Part two was a therapeutic section based on cognitive-behavioural principles and containing specific exercises: behavioural activation, exposure, problem solving, cognitive restructuring and lifestyle strategies, with case vignettes and details of how to apply the interventions."

Compliance: "88% reported reading 'at least half the manual', although only 52% reported conducting self-help exercises 'at least weekly'."

Outcomes

General Symptoms: Hospital Anxiety and Depression Scale (HADS)

Depression: Beck Depression Inventory (BDI)

Disability: Social Adjustment Scale (SAS)

Notes

Funding: National Primary Care Research and Development Centre, University of Manchester (funded by the Department of Health). Authors publish similar manuals commercially.

Unpublished data: Mead provided unpublished outcome data, details of assessment, duration of intervention, and contact details.

Risk of bias

Mead 2005 (Continued)

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “Allocation to treatment was undertaken using a dynamic form of random allocation called minimization conducted by an external specialist clinical trials unit...minimized by BDI score (0–22, or 23+), estimated time remaining to first formal therapy appointment (3–5 months, 6–8 months, or 9 months or over) and type of therapy for which patients had been referred (CBT/psycho-dynamic therapy or counselling).”
Allocation concealment (selection bias)	Low risk	“Allocation was thus effectively concealed from the researchers making judgments of eligibility...”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “GSH intervention involved...one-to-one sessions with an AP”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=7 (12.28%) <i>Comparison:</i> N=4 (7.018%) Method of analysis: Available Case “Data missing because of loss to follow-up were not imputed, but stepwise logistic regression (with backward selection) was used initially to investigate predictors of loss to follow-up.”
Selective reporting (reporting bias)	Low risk	Trial registration number: ISRCTN33308608 All outcomes reported.
Other bias	Low risk	N/A

Milne 1988

Methods	Location: Wakefield, England Recruitment: Referral Exclusion rate: Unknown Randomised (N): 22 (8 intervention; 7 comparison)
Participants	Disorder: Generalised Anxiety Disorder; Other Duration (years): Unknown Age (years): 53

Milne 1988 (Continued)

Sex (female): 72.22%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Wait-List Intervention name: Anxiety: How to understand and control your nerves (Milne and Linford, 1984) Days of treatment: 28 Author wrote: Yes Type of media: Book / Booklet Type of therapy: Muscle Relaxation Location used: Anywhere Type of contact: None Automated contact: N=0 Provider contact: N=0 (0 min) Contact description: Both groups received treatment-as-usual by GP unaware of group assignment. Intervention description: “35 page account of anxiety in terms of the nervous system, its prevalence and causes, and ways of managing it by behaviour therapy. For example, it detailed how to practise progressive muscular relaxation. Self-test quizzes were provided after each section of the manual so as to provide feedback on understanding. Diagrams and illustrations were included.” Compliance: Not collected.
Outcomes	General Symptoms: Fear Questionnaire (FQ): Total Phobia
Notes	Funding: None acknowledged Unpublished data: Milne was unable to provide further information but suggested how randomisation may have been done and what race participants might have been.
Risk of bias	
Bias	Authors' judgement Support for judgement

Milne 1988 (Continued)

Random sequence generation (selection bias)	Unclear risk	Method: Unclear “The 22 referrals were randomly allocated to one of three groups...” The author cannot recall but suggests it may have been names in a hat (personal communication).
Allocation concealment (selection bias)	Low risk	“The referring GP was unaware of group assignment.”
Blinding of participants and personnel (performance bias) All outcomes	Low risk	Provider contact: Yes Provider blind: Yes The referring GP was unaware of group assignment. "There was no concurrent modification of the routine GP management of the patient's anxiety, for example, medication."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (12.5%) <i>Comparison:</i> N=1 (14.29%) Method of analysis: Unclear “During the evaluation period, 22 patients with clinical anxiety were diagnosed and referred to the study by a GP All had agreed to participate in the study Four patients did not return their questionnaire, leaving a sample of 18." Dropout estimates assuming equal allocation of 22 participants.
Selective reporting (reporting bias)	High risk	No trial registration number given. Some outcomes not fully reported, including Coping and Change Questionnaire, consultations, and use of medication.
Other bias	Low risk	N/A

Moritz 2010

Methods	Location: Germany/ Switzerland Recruitment: Advertising/ Internet Exclusion rate: 37.23% Randomised (N): 86 (43 intervention; 43 comparison)
Participants	Disorder: OCD; "participants confirmed that a diagnosis of OCD was previously determined..." Duration (years): Unknown Age (years): 34.52 Sex (female): 67.44%

Moritz 2010 (Continued)

Race (white): Unknown
Medication (using at baseline): Unknown
Education (years): 11.6

EXCLUSIONS

Depression: No
Psychotherapy: No restrictions reported
Medication: N/A
Drug/Alcohol: N/A
Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: My metacongitive Training for OCD (myMCT)

Days of treatment: 30

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: It was made clear that participation would not require personal or telephone contact and that it was strictly anonymous. "Patients were provided with the e-mail address of the first author in case of questions. E-mails were responded to within 24 hours. However, only three participants turned to the first author, whereby questions were solely related to the handling of the PDF file."

Intervention description: "self-help manual starts with an introduction which defines core features of OCD symptomatology, demonstrates its most prevalent subtypes and requests patients to identify their own core problems and dysfunctional coping strategies...then, the aims of the program are explained. the myMCT consists of 14 sections dealing with prevalent cognitive biases in OCD"

Compliance: "36 patients who received the manual, nine stated that they had not read the manual..." "half of the patients stated that they did not find the time to study the manual intensively. 25.9% performed exercises over a time-span of at least 14 days, whereby only two patients (7.4%) performed the exercises every day. The largest group (55.5%) performed the exercises for 7 to 14 days. The rest (18.5%) spend less than seven days..."

Outcomes

None included.

Notes

Funding: None acknowledged

Risk of bias

Moritz 2010 (Continued)

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear Participants who left e-mail addresses were allocated to the experimental or waitlist group according to a random plan.
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	Low risk	No contact with researchers or clinicians.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=7 (16.28%) <i>Comparison:</i> N=5 (11.63%) Method of analysis: Per protocol "...data from participants in the experimental group (myMCT) who after the third and final reminder still did not disclose whether or not they had read the manual were removed from the analyses...we retained patients in the myMCT group who had read (part of) the manual but did not perform any of the exercises according to self-report."
Selective reporting (reporting bias)	High risk	No trial registration number given. T-tests reported, but these cannot be analysed as it is unclear how many participants are in each group and if they were analysed in the groups to which they were assigned.
Other bias	High risk	"Unclear if methods used for the figures were used for all analyses: 'Figures 1 and 2 show the results of the pre-post assessment calculated for completers. For this analysis, we added subjects from the myMCT group who did not read the manual to the waitlist group.'"

Muller 2011

Methods	Location: Basel, Switzerland Recruitment: College only Exclusion rate: 83.64% Randomised (N): 36 (18 intervention; 18 comparison)
Participants	Disorder: Specific Phobia; No diagnostic interview (questionnaire only) Duration (years): Unknown

Muller 2011 (Continued)

Age (years): 23.17

Sex (female): 100%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: "The spider-fearful control participants underwent the same procedure with presentation of nine neutral pictures and an equal exposure time of 27 min."

Intervention name: Not named

Days of treatment: 7

Author wrote: Yes

Type of media: Internet

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=1 (duration unknown)

Contact description: "Participants were tested individually in a sound-attenuated room."

Intervention description: "When participants had completed the pretreatment FSQ and BAT they were seated in front of a computer, 60 cm from the screen, to perform the CBE...All instructions were presented on the computer screen. Each spider picture in the CBE group was presented on the computer screen for 3 min, with a total standardised exposure time of 27 min."

Compliance: Lab-based, but compliance data not reported.

Outcomes

Specific Symptoms: Fear of Spider Questionnaire (FSQ)

Behaviour Test (Level): Behavioural Approach Test: Level

Notes

Funding: None acknowledged

Risk of bias

Muller 2011 (Continued)

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “The experimenter randomly assigned participants meeting the inclusion criteria to the CBE condition (n=18) or the control condition (n=18).”
Allocation concealment (selection bias)	Unclear risk	“The experimenter randomly assigned participants meeting the inclusion criteria to the CBE condition (n=18) or the control condition (n=18).”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Participants were tested individually in a sound-attenuated room.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: No Dropout
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Nordin 2010

Methods	Location: Sweden Recruitment: Advertising/ Internet Exclusion rate: 80% Randomised (N): 40 (20 intervention; 20comparison)
Participants	Disorder: Panic disorder; No diagnostic interview (questionnaire only) Duration (years): 10.1 Age (years): 37.7 Sex (female): 72.5% Race (white): 37.5% Medication (using at baseline): 37.5% Education (years): 13.1

Nordin 2010 (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Participants taking medication were allowed to participate if the dose of medication was stable for 3 months prior to commencing the self-help treatment. Participants in the treatment group were asked not to increase or decrease the dosage during the study without consulting their prescribing physician."

Drug/Alcohol: N/A

Physical: "any of the following somatic complaints: epilepsy, kidney problems, stroke, organic heart syndrome, emphysema, heart attack, or hypertension"

Interventions

Comparison group(s): Wait-List

Intervention name: Ingen panik: Fri från panik- och ångestattacker i 10 steg med kognitiv beteendeterapi. [No panic: Free from panic and anxiety]

Days of treatment: 70

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "In the written letter, the participants were also encouraged to contact the person responsible for the study if they had any further questions during the duration of the treatment. However, none of the participants did so."

Intervention description: "308 pages...psychoeducation...breathing retraining and a hyperventilation test. Chapters 4–5 contain information and exercises regarding cognitive restructuring, and chapters 6–7 give instructions on how to do interoceptive exposure. Chapters 8–9 deal with exposure in vivo, and the book ends with a chapter on relapse prevention and assertiveness training. At the end of each chapter, the reader is presented with a quiz...the reader was also given homework assignments..."

Compliance: N/A

Outcomes

Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ)

General Symptoms: Beck Anxiety Inventory (BAI)

Response: Panic Disorder Severity Scale (PDSS)

Recovery: No longer met criteria for diagnosis

Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS)

Quality of Life: Quality of Life Inventory (QOLI)

Nordin 2010 (Continued)

Notes

Funding: Swedish Research Council "Dr. Carlbring shares intellectual property rights in the tested book."

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "used the www. random.org website to generate a list that was then sent to the researchers."
Allocation concealment (selection bias)	Low risk	"Randomization was arranged by an independent person without contact with the participants."
Blinding of participants and personnel (performance bias) All outcomes	Low risk	No contact with researchers or clinicians.
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "Along with the PDSS, the interviewer also formed a clinical global impression (CGI; Guy, 1976) of each participant at posttreatment and follow-up. The interviewer was not uninformed regarding the treatment status"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "Only one individual, belonging to the waiting list group, dropped out of the study. This participant failed to complete the online self- report instruments at posttreatment and was not reachable for the telephone interview. Intention-to- treat analysis was used for this single case (Newell, 1992), with the last observation carried forward procedure assuming no change in scores."
Selective reporting (reporting bias)	High risk	No trial registration number given. "The mobility inventory...one version incorporating pictures (data not included here)" CGI data are not reported."
Other bias	Low risk	N/A

Ost 1991

Methods

Location: Oxford (UK) and Uppsala (Sweden)

Recruitment: Advertising and referral

Exclusion rate: 2.86%

Randomised (N): 34 (17 intervention; 17 comparison)

Ost 1991 (Continued)

Participants

Disorder: Specific Phobia; ADIS (DSM-III R)

Duration (years): 25.2

Age (years): 31.5

Sex (female): 100%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS
Depression: No

Psychotherapy: No restrictions reported

Medication: N/A

Drug/Alcohol: N/A

Physical: "have no heart or lung disease"

Interventions

Comparison group(s): Individual Exposure

Comparison description: "...detailed rationale for exposure, incorporating ideas of habituation, self-control, and being able to control and predict the spider, and the patient's own reactions... Four or five spiders in increasing sizes were used during single therapy session that lasted a max of 3 hours. Each step was modeled by the therapist before the patient carried it out. Mean 126.2 (30.5) minutes. Sessions videotaped and replayed to patients. (Intervention strategies similar, method of delivery varied.)"

Intervention name: Not named (also used in Hellstrom 1995)

Days of treatment: 14

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "The interview took about 60 min, and its main purpose was to ascertain the patient's suitability for the study and to conduct the behavioral test." "At the end of the screening interview the patient had to agree to schedule at least two 2 hr. self-exposure sessions during the next 2 weeks..."

Intervention description: "had to agree to schedule at least two 2 hr. self-exposure sessions during the next 2 weeks...get the help of a relative or friend to collect 4-5 spiders...If getting this help was impossible, the patient was to call the experimenter who would give her the necessary spiders...30-page manual...based on the procedures of the therapist-directed one-session treatment...same general information as that in Marks (1978)"

Ost 1991 (Continued)

Compliance: Mean number sessions 3.2 (1.9), range 0 to 6. One patient did not try the exposure due to anxiety. 16 participants completed a mean of 3.4 exposure sessions each. Mean exposure time was 282.8 minutes (157.5), range 0 to 535. Mean exposure time per session 80.4 (18.4) minutes.

Outcomes	Specific Symptoms: Spider Phobia Questionnaire (SPQ) Behaviour Test (Symptoms): Behavioural Approach Test: Symptom Rating Response: Behavioural Approach Test: Completed; Clinically Significant Improvement (Composite)
Notes	Funding: Swedish Medical Research Council. Medical Research Council (UK). Unpublished data: Ost provided unpublished outcome data and information about participant characteristics.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear At both sites the patients were randomly assigned to treatment conditions
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "At the end of the screening interview the patient had to agree to schedule at least two 2 hr. self-exposure sessions during the next 2 weeks, i.e. the interval between the pre- and post-treatment assessment for the therapist-directed group. It was emphasized that more would be preferred."
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "The authors functioned as therapists"
Incomplete outcome data (attrition bias) All outcomes	Low risk	No Dropout
Selective reporting (reporting bias)	Low risk	No trial registration number given. Several outcomes do not appear in the published report, but data were provided by the author.
Other bias	Low risk	N/A

Paxling 2011

Methods	Location: Linköping, Sweden Recruitment: Advertising/ Internet Exclusion rate: 66.54% Randomised (N): 89 (44 intervention; 45 comparison)
Participants	Disorder: Generalised Anxiety Disorder; SCID (DSM-IV)

Paxling 2011 (Continued)

Duration (years): 20

Age (years): 39.3

Sex (female): 79.78%

Race (white): Unknown

Medication (using at baseline): 37.1%

Education (years): Unknown

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Participants on medication (e.g. anti-depressants) had to be stabilized for at least 1 month and were asked to maintain a stable dose level during the study "

Drug/Alcohol: "An AUDIT score of 19 or higher has been reported to indicate severe alcohol problems and was chosen as a cutoff in this study."

Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: Not named

Days of treatment: 56

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=8 (97 min)

Contact description: "Feedback from the therapist was provided on a weekly basis in association with the homework assignments. This feedback usually ranged in length from one-half written page to three written pages. In addition, occasional reminders were sent via e-mail...therapist time devoted to each client ranged from 10 to 15 min per week, totalling 71 h and 5 min for all patients in the trial and a mean of 97 min (SD=52, range=12–215) per patient."

Intervention description: "text-based treatment modules delivered on a weekly basis for 8 week-s...psychoeducation...applied relaxation...applied relaxation and worry time...applied relaxation and cognitive restructuring...applied relaxation, more on cognitive distancing, and problem solving...applied relaxation and worry exposure...applied relaxation, interpersonal problem solving, and sleep management...relapse prevention...CD with audio-recorded instructions..."

Compliance: "The mean number of treatment modules completed was 4.8 (SD=2.1), and only 10.5% (4/38) completed all modules in time."

Outcomes

Specific Symptoms: Generalized Anxiety Disorder Questionnaire–IV

Paxling 2011 (Continued)

General Symptoms: Beck Anxiety Inventory (BAI); State-Trait Anxiety Inventory (STAI)

Response: Generalized Anxiety Disorder Questionnaire-IV; Penn State Worry Questionnaire (PSWQ)

Depression: Beck Depression Inventory (BDI); Montgomery and Asberg Depression Rating Scale (MADRS)

Quality of Life: Quality of Life Inventory (QOLI)

Notes

Funding: Swedish Council for Working and Life Research

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/ Online "A computer-generated random list was obtained via www.random.org, which utilizes atmospheric noise to create random sequences of numbers."
Allocation concealment (selection bias)	Low risk	"by an independent person not involved in the study"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Feedback from the therapist was provided on a weekly basis in association with the homework assignments."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "The interviewers were blinded concerning participant status"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=6 (13.64%) <i>Comparison:</i> N=1 (2.22%) Method of analysis: Mixed effects "To compare the Internet treatment and control group according to the outcome measures at baseline and posttreatment, an unstructured covariance pattern model was used. The restricted maximum likelihood model was used as estimation method."
Selective reporting (reporting bias)	High risk	CGI-Improvement not reported.
Other bias	Low risk	N/A

Proudfoot 2004a

Methods

Location: London and SE England

Recruitment: Referral

Proudfoot 2004a (Continued)

Exclusion rate: 45.42%

Randomised (N): 274 (146 intervention; 128 comparison)

Participants
Disorder: Mixed (presenting complaint for 66% was an anxiety disorder or mixed anxiety and depression; 14% had anxiety alone); No diagnostic interview (questionnaire only)

Duration (years): Unknown

Age (years): 43.51

Sex (female): 73.72%

Race (white): 68.39%

Medication (using at baseline): 68.39%

Education (years): 13.76

EXCLUSIONS
Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "had been taking medication for anxiety and/or depression continuously for 6 months or more immediately prior to entry" "the general practitioner, who administered the inclusion and exclusion criteria, decided whether medication was to be prescribed, and indicated the treatment the patient was to receive if randomly allocated to treatment as usual"

Drug/Alcohol: "patients were excluded if they had alcohol and/or drug dependence"

Physical: N/A

Interventions
Comparison group(s): Treatment-as-usual

Comparison description: GPs could prescribe medication or refer to counselling. Usual care included discussions with the doctor, practical and social help, further physical investigation.

Intervention name: Beating the Blues (Ultrasis; <http://www.ultrasis.com>)

Days of treatment: 63

Author wrote: Yes

Type of media: Computer

Type of therapy: CBT

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=8 (80min)

Contact description: Nurses were instructed to spend no more than 5 min with each patient at the start and at the end of each session (i.e. up to a total of 80 min over the eight sessions).

Intervention description: interactive, multimedia, computerised cognitive-behavioural therapy package consisting of a 15min introductory videotape, followed by eight therapy sessions (Fig. 1). Each

Proudfoot 2004a (Continued)

weekly session lasts about 50 min, with 'homework' projects between the sessions. Goal setting automatic thoughts, thinking errors, distraction, attributional styles, activity scheduling, problem solving, graded exposure, task breakdown.

Compliance: Sessions attended reported for "Phase 1", which is a partial report. Data not found for Phase 2 or the full sample.

Outcomes	Anxiety was reduced only in participants with BAI scores over 18. General Symptoms: Beck Anxiety Inventory (BAI) Depression: Beck Depression Inventory (BDI) Disability: Work and Social Adjustment Scale (WSAS)
Notes	Funding: NHS Executive London, Ultrasis (owner Beating the Blues). Authors declare financial conflict. Unpublished data: Proudfoot provided details about participants and conduct of the study.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "Within each of the seven surgeries, patients who fitted the inclusion criteria and consented to participate were stratified according to whether they were prescribed medication or not (yes, no), and the duration of current episode of episode (≤6 months, >6 months). They were then randomly allocated, within each of the two strata, to receive BtB or TAU."
Allocation concealment (selection bias)	Low risk	"Randomly sorted cards naming one of the two treatments (computer therapy/treatment as usual) were placed by the first author in individual envelopes, which were sealed and numbered."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Nurses were instructed to spend no more than 5 min with each patient at the start and at the end of each session..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=54 (36.99%) <i>Comparison:</i> N=35 (27.34%) Method of analysis: Available Case Means and SD do not appear to include imputed values. "All analyses were intention-to-treat, by which we mean that patients were analysed as randomised rather than by treatment actually received. Within each intention-to-treat group, patients were not included in the model-fitting process described above if they had missing values on any of the covariates used (pre-treatment value, drugs, duration of episode) or if all four post-randomisation values of

Proudfoot 2004a (Continued)

		the response being analysed were missing. Consequently, all the available post-randomisation values of the response were included, rather than only those obtained from patients with all four values recorded."
Selective reporting (reporting bias)	High risk	No trial registration number given. Data about receipt of other treatments are reported for Phase 1, but not for Phase 2. Compliance data are also reported only for Phase 1.
Other bias	Low risk	N/A

Rapee 2007

Methods	Location: Sydney, Australia Recruitment: Advertising and referral Exclusion rate: Unknown Randomised (N): 224 (56 intervention; 52, 59 comparison)
Participants	Disorder: Social Anxiety (General); ADIS (DSM-IV) Duration (years): Unknown Age (years): 35.81 Sex (female): 51.5% Race (white): 31.74% Medication (using at baseline): 31.74% Education (years): 13.4 EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: "Concurrent pharmacotherapy or psychotherapy was allowed as long as dosages had been consistent for 3 months and there was no plan to change." Drug/Alcohol: "severe substance misuse or dependence" Physical: N/A
Interventions	Comparison group(s): Wait-List, Group CBT Comparison description: 1) Wait-list. 2) "groups of approximately six participants with two graduate psychology student therapists...ten 2h sessions across 12 weeks. Treatment was manualised, based on principles and components described in a book by Rapee & Sanderson (1998)...cognitive restructuring of negative evaluation beliefs, exposure to feared social situations, realistic feedback of social performance, and attention training. Participants engaged in home exercises and received various handouts as relevant." Intervention name: Overcoming Shyness and Social Phobia: A Step by Step Guide (Rapee, 1998)

Rapee 2007 (Continued)

Days of treatment: 84

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: Participants in the pure self-help group were given a copy of the book and the cover letter and were then simply contacted again after 12 weeks for a second assessment. Duration of assessment estimated (personal communication).

Intervention description: "The strategies outlined paralleled those in the standard group treatment, and practice sheets and exercises formed part of the book...given a cover letter with the book welcoming them to the programme and providing a suggested rate of progress in order to complete it in 12 weeks."

Compliance: Pure self-help read 4.11 (2.95) chapters. Those with more severe social phobia read more chapters, and they experienced the greatest improvement. However, improvement was not related to the group x time x chapters interaction: differences in the number of chapters read did not account for differences in improvement between groups.

Outcomes	Specific Symptoms: Brief Fear of Negative Evaluation (BFNE) Recovery: No longer met criteria for diagnosis
Notes	Funding: None acknowledged Unpublished data: Rapee provided study details and referred to Baillie, who was on sabbatical.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Unclear Those who met inclusion criteria were randomly allocated to one of the four conditions. Randomisation was done using a pre-assigned random number generator in blocks of eight to allow for group delivery.
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Standard treatment was included to represent the 'gold standard' treatment effect. Treatment was conducted in groups of approximately six participants with two graduate psychology student therapists who received minimal supervision from an experienced clinical psychologist."
Blinding of outcome assessment (detection bias) All outcomes	Unclear risk	Included assessor-rated outcomes (blinding unclear)

Rapee 2007 (Continued)

"The number and percentage of participants from each treatment condition who no longer met criteria for a diagnosis of social phobia at the post-treatment and follow-up assessments are shown in Table 2."

Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison (wait-list):</i> N=0 (0%) <i>Comparison (group CBT):</i> N=0 (0%) Method of analysis: LOCF "The number of participants who provided no data at post-treatment and at 3-month follow-up is shown in Fig. 1. The last value carried forward strategy was used to substitute missing data if data were not available at the 3-month follow-up or at both post-treatment and 3-month follow-up. Interpolation was used if posttreatment data only were not available. As a precaution against biasing effects of these methods of handling missing data, analyses were conducted with and without missing data substituted. Analyses with missing data substituted are equivalent to intent-to-treat analyses"
Selective reporting (reporting bias)	High risk	No trial registration number given. Outcomes are not reported for the Albany Panic and Phobia Questionnaire or the Self Consciousness Scale.
Other bias	Low risk	N/A

Richards 2003a

Methods	Location: Leeds, England Recruitment: Referral Exclusion rate: 44.4% Randomised (N): 139 (75 intervention; 64comparison)
Participants	Disorder: Mixed (not specified); No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 39.2 Sex (female): 84% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: Yes

Richards 2003a (Continued)

Psychotherapy: No restrictions reported

Medication: "judged to require immediate treatment with a new course of anti-depressants"

Drug/Alcohol: N/A

Physical: N/A

Interventions	Comparison group(s): Treatment-as-usual Comparison description: GPs treated patients assigned to them in their 'usual' way. There were no restrictions on the type of care delivered. Intervention name: Managing Anxiety and Depression: A Self-help Guide (Holdsworth and Paxton, 1999) Days of treatment: 28 Author wrote: No Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=2 (duration unknown) Contact description: Practice nurses assisted patients in using the booklet both within and between sessions. The programme comprised up to three sessions based on a previous trial evaluating a 'two-plus-one'... The first two appointments, offered 1 week apart, focused on familiarisation with the booklet and applying it to samples of the patients' problems. Patients then used the booklet at home and were offered a third review appointment ('plus-one') 3 months later. Intervention description: Booklet CBT. 42 pages on A5 targeting anxiety, depression and related problems. Designed to be consulted, not read through. "Practice nurse-facilitated self-help. Practice nurses assisted patients in using a self-help book — Managing Anxiety and Depression—a booklet developed for primary care, based on cognitive behavioural therapy techniques and previously subject to an outcome study." Compliance: 47 of those assigned to the intervention attended one or more appointments.
Outcomes	Quality of Life: EuroQol Thermometer
Notes	Funding: NHS: Northern and Yorkshire NHS Executive. Research and Development Levy from the Leeds Community and Mental Health Teaching Trust. Unpublished data: Barkham was unable to provide details at the time but would check if further information was available.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "carried out using a computer-generated random numbers table with practices blind to the randomisation strategy. Block randomisation was utilised in

Richards 2003a (Continued)

		blocks of four patients per practice to prevent practice nurse workload becoming unmanageable. Patients were informed of their randomly allocated group within 24 hours and were requested to make the appropriate appointment at their surgery."
Allocation concealment (selection bias)	Low risk	"Patients were randomised once consent had been given. Randomisation was independent of the research team..."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Practice nurses assisted patients in using the booklet both within and between sessions."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=16 (21.33%) <i>Comparison:</i> N=17 (26.56%) Method of analysis: LOCF Of those randomised 16/75 (21%) and 17/64 (27%) did not return consent forms. Observations carried forward for other participants, except for improvement (dichotomous) data.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	High risk	Some participants who were randomised to usual care were given an appointment to receive the intervention and "it was considered unethical to rectify the error once the patient had commenced treatment." It is unclear if and how these participants appear

Richards 2006

Methods	Location: Melbourne, Australia Recruitment: Advertising/ Internet Exclusion rate: 52.94% Randomised (N): 32 (23 intervention; 9 comparison)
Participants	Disorder: Panic disorder; ADIS (DSM-IV) Duration (years): 12.7 Age (years): 36.59 Sex (female): 68.75% Race (white): 37.5%

Richards 2006 (Continued)

Medication (using at baseline): 37.5%

Education (years): 13.16

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Participants taking medication for anxiety or depression were included if they had been stabilised on it for at least 4 weeks but continued to experience panic symptoms and met a diagnosis of PD."

Drug/Alcohol: "alcohol or drug dependency"

Physical: "stroke...heart condition...chronic hypertension"

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: These participants were required to participate in the same telephone and online assessments as those in PO1 and PO2. A Doctor of Psychology (Clinical) student contacted the IC participants each week by email and inquired about their current panic status. She provided minimal support and questioned participants as to whether they had been doing their daily monitoring and advised them to re-read the information on this panic resource program that contained no active CBT information.

Intervention name: Panic Online (Klein and Richards)

Days of treatment: 68

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=13.45 (343 min)

Contact description: a full clinical assessment using the ADIS-IV and PDSS was conducted. This typically required 90 min. Sent 15.45 (5.32) emails and received 13.45 (13.05) emails. Combined treatment groups received 342.8 (135.97) minutes of therapist support.

Intervention description: 1) "PO1 comprised four learning modules and introductory and relapse prevention modules that were designed to be completed over 8 weeks...instructions for controlled breathing, progressive muscle relaxation, cognitive restructuring and interoceptive and situational exposure..." 2) As before with stress management component.

Compliance: Measured days in treatment, emails sent.

Outcomes

Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ)

General Symptoms: Anxiety Sensitivity Profile (ASP); Depression, Anxiety, Stress Scales (DASS): Anxiety

Recovery: High End-State Functioning

Richards 2006 (Continued)

Depression: Depression, Anxiety, Stress Scales (DASS): Depression

Quality of Life: WHO Quality of Life - BREF

Notes

Funding: Australian Rotary Health Research Fund

Unpublished data: Barkham was unable to provide details at the time but would check if further information was available.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear After all pre-treatment assessment measures were completed by the participants, they were randomly allocated to one of the three conditions.
Allocation concealment (selection bias)	Unclear risk	The two assessors were the second author of the present study and a probationary registered psychologist/ PhD candidate. The second author was not blind to treatment allocation, although the other assessor was.
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "all experienced in CBT for PD made email contact with the Internet-based treatment (Panic Online: PO1) participants assigned to them and guided each through the program."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "The two clinicians who conducted the assessments did not provide any treatment."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (13.04%) <i>Comparison:</i> N=3 (33.33%) Method of analysis: LOCF Of the 32 people who commenced the study, five discontinued during treatment. The overall attrition rate for the present study was therefore 15.6%. The attrition rate for PO1 was 16.7% (2/12), 9% (1/11) in PO2 and 22% (2/9) in IC. Reasons given for discontinuing the treatment in the PO1 condition were a lack of motivation or an episode of major depression. The PO2 person discontinued because of a wish to commence selective serotonin reuptake inhibitor medication halfway through the study. Of the two IC participants, no reason for discontinuing was given.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Unclear risk	N/A

Robinson 2010

Methods	Location: National, Australia Recruitment: Advertising/ Internet Exclusion rate: 27.18% Randomised (N): 150 (101 intervention; 49 comparison)
Participants	Disorder: GAD; MINI (DSM-IV) Duration (years): N/A Age (years): 47 Sex (female): 68.28% Race (white): N/A% Medication (using at baseline): 32.4% Education (years): 14.38 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "if taking medication, had been taking the same dose for less than 1 month or intending to change that dose during the course of the program" Drug/Alcohol: "using illicit drugs or consuming more than three standard drinks/day" Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: The Worry Programme Days of treatment: 70 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Home/ Anywhere Type of contact: Email & Phone Automated contact: N= 11 Provider contact: N=11 (78 min) Contact description: "Automatic emails are sent to congratulate participants for completing each lesson, to remind them..." Group 1 (75min): "weekly email or telephone...given a guideline script...provide encouragement and support, and where possible to respond to participants' general questions." Group 2 (81 min): "weekly email or telephone...access to an online discussion forum...provided with the same guideline script as the technician but was also instructed to answer participants' questions via forum, email, or telephone. The clinician was instructed to actively engage with each participant in treatment..."

Robinson 2010 (Continued)

Intervention description: "... six online lessons...cognitive therapy, challenging meta-beliefs about worry, graded exposure, challenging core beliefs, and relapse prevention...printable summary and homework assignments...illustrated story about a woman with GAD..."

Compliance "Forty (40/50, 80%) TA and 35/47 CA (74%) group participants completed all 6 lessons within the required time. Reasons for not completing all lessons were not collected."

Outcomes	Specific Symptoms: Generalised Anxiety Disorder Assessment (GAD 7); Penn State Worry Questionnaire (PSWQ) General Symptoms: Social Interaction Anxiety Scale (SIAS); Penn State Worry Questionnaire (PSWQ) Response: Generalised Anxiety Disorder Assessment (GAD 7) Depression: Patient Health Questionnaire (PHQ-9) Disability: Sheehan Disability Scale (SDS) Psychiatric Distress: Kessler 10 (K-10)	
Notes	Funding: Australian Rotary Health Research Fund	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "NT via a true randomisation process (www.random.org)"
Allocation concealment (selection bias)	Low risk	"Allocation preceded the diagnostic telephone call." "by contacting the holder of the allocation schedule who is off-site."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Provider contact: Yes Provider blind: No "weekly email or telephone"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=4 (3.96%) <i>Comparison:</i> N=0 (0.00%) Method of analysis: LOCF
Selective reporting (reporting bias)	Low risk	Trial registration number: ACTRN12609000563268 All outcomes reported.
Other bias	Low risk	N/A

Rosen 1976

Methods	Location: Eugene (Oregon), USA Recruitment: Advertising/ Internet Exclusion rate: Unknown Randomised (N): 55 (22 intervention; 11, 11 comparison)
Participants	Disorder: Specific Phobia; Other Duration (years): 25.9 Age (years): 33.5 Sex (female): 90.7% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-list, Individual exposure Comparison description: 1) Wait list 2) "Subjects in this group met twice a week with one of five graduate student therapists...weekly group meetings that were held over a 3-month period prior to treatment. A common set of instructional materials was used as part of the training program." Intervention name: A manual for self-administering systematic desensitization in your home (GM Rosen, 1974) Days of treatment: 56 Author wrote: Yes Type of media: Book / Booklet Type of therapy: Muscle Relaxation Location used: Anywhere Type of contact: Phone Automated contact: N=0 Provider contact: N=8 (45.6min) Contact description: Two groups combined for analysis. 1) No contact. 2) "During these phone calls, therapists served as consultants who answered questions and made procedural suggestions. Calls ranged from 4 to 49 minutes with an average time per call of 11.4 minutes."

Rosen 1976 (Continued)

Intervention description: 2 groups combined: 1) "A log sheet accompanied the program...In addition to written materials, subjects were given a record of relaxation instructions (Kahn, cited in Kahn & Baker, 1968, p. 198) to help them learn varied instructional phrases. Subjects were instructed to always conduct practice sessions on their own and to not rely on recorded procedures." 2) "In addition, subjects received weekly phone calls from the same therapists who worked with clients in the clinic group."

Compliance: 47% completed at least half of their hierarchy. 67% of the face-to-face comparison completed their entire programme.

Outcomes	Specific Symptoms: Fear Survey Schedule: Snake Behaviour Test (Level): Behavioural Approach Test: Level Behaviour Test (Symptoms): Behavioural Approach Test: Symptom Rating Response: Behavioural Approach Test: Completed
Notes	Funding: National Institute of Mental Health. "Grant MH256S7-01" Unpublished data: Contacted Barrera and Rosen. They were unable to provide further information. Did not extract data for the "placebo" control, information about snakes (11 participants not included).

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "After pretreatment assessments had been completed, the subjects were matched on behavior approach scores and assigned by block randomization to one of four treatment groups or a no-treatment control."
Allocation concealment (selection bias)	Unclear risk	"Pretreatment to posttreatment and follow-up assessments of subjects' attitudes and reactions toward snakes were conducted by assistants blind to subjects' group assignment."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "During these phone calls, therapists served as consultants who answered questions and made procedural suggestions."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "Pretreatment to posttreatment and follow-up assessments of subjects' attitudes and reactions toward snakes were conducted by assistants blind to subjects' group assignment."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (22.73%) <i>Comparison (no-treatment):</i> N=4 (36.36%) <i>Comparison (face-to-face):</i> N=2 (18.18%) Method of analysis: Available Case

Rosen 1976 (Continued)

Assumed equal allocation to groups. All participants in the original study sent a follow-up questionnaire. Because measures included a desensitisation technique involving a four foot snake, data for 42% of desensitization subjects who were unable to be in a room with the snake could not be used. 23 of desensitisation subjects and 11 of 17 controls responded.

Selective reporting (reporting bias)	High risk	No trial registration number given. In addition to the scales extracted, the report suggests that other measures may have been used. Follow-up data are not reported in sufficient detail to include in meta-analysis.
Other bias	Low risk	N/A

Rosmarin 2010

Methods	Location: Jewish communities, International Recruitment: Advertising/ Internet Exclusion rate: 46.3% Randomised (N): 261 (106 intervention; 72, N/A, 83 comparison)
Participants	Disorder: Mixed (not specified); No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 41.8 Sex (female): 76.6% Race (white): Unknown Medication (using at baseline): Unknown Education (years): 15.32 EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: "no changes in psychotropic medication (type or dosage) during the 8-weeks before treatment" Drug/Alcohol: "no past or current diagnosis of substance abuse" Physical: "traumatic brain injury, current life threatening illnesses"
Interventions	Comparison group(s): Wait-List Intervention name: Not named Days of treatment: 14 Author wrote: Yes

Rosmarin 2010 (Continued)

Type of media: Internet

Type of therapy: Muscle Relaxation

Location used: Anywhere

Type of contact: Email

Automated contact: N=14

Provider contact: Unknown

Contact description: To encourage participation in the treatment programs, participants in the active treatment groups received a daily email reminder about their treatment period "To facilitate compliance, email messages prompting participants to complete the post-treatment and follow-up assessments were sent on a daily basis until the assessments were completed."

Intervention description: Progressive muscle relaxation. Relaxation: tensing and relaxing 16 different muscle groups. Instructions presented audibly, asked to practice relaxation throughout daily life. Provided with link so that they could print summary of PMR program. 10 minute information-orientation video. Use techniques once per day.

Compliance: Completion of daily treatment was tracked electronically to provide information about how many sessions each participant completed.

Outcomes	Specific Symptoms: Penn State Worry Questionnaire (PSWQ) Depression: Center for Epidemiologic Studies Depressed Mood Scale (CESD)
Notes	Despite the title, this does not appear to be CBT. Included only the relaxation condition as a treatment group. "The SI-CBT contained four components: (1) an introduction; (2) a cognitive component consisting of stories and readings, in which participants were encouraged to think about what it means to trust in God, and the role of God in their lives; (3) a behavioral component consisting of spiritually integrated strategies designed to increase awareness of God's benevolence and care; and (4) a brief prayer for Divine assistance..." Funding: "This study was supported by a generous grant from a private Jewish community foundation in Toronto, Canada." Unpublished data: Extracted outcomes and characteristics from thesis. The text of the spiritual intervention is included in the thesis.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "After completing the screening procedure, eligible participants were automatically randomly assigned to one of the three study conditions using a computer-generated allocation sequence and hence no blinding/masking was necessary."
Allocation concealment (selection bias)	Low risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias)	Unclear risk	Unclear if there was any contact with researchers or clinicians.

Rosmarin 2010 (Continued)

All outcomes

Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=65 (61.32%) <i>Comparison (wait-list):</i> N=24 (33.33%) Method of analysis: Available Case "The remaining 261 participants were randomized to the SI-CBT group (n= 83), the PMR group (n = 106) or the WLC group (n = 72). Contact was lost with 65 participants who did not complete the pre-treatment (baseline) assessment. Of the remaining 202 participants, 77 participants did not complete the post treatment assessment, and an additional 29 participants did not complete the follow-up assessment. Thus, analyses were conducted with 125 participants who completed the pre- and post-treatment assessments, of which 96 participants also completed the follow-up assessment."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Ruwaard 2010

Methods	Location: Amsterdam, Holland Recruitment: Advertising/ Internet Exclusion rate: 64.63% Randomised (N): 58 (27 intervention; 31 comparison)
Participants	Disorder: PAs; Clinical Interview (DSM-IV) Duration (years): 9.40 Age (years): 38.74 Sex (female): 72.41% Race (white): Unknown Medication (using at baseline): 29.31% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported

Ruwaard 2010 (Continued)

Medication: "no use of neuroleptic medication; no use of anxiolytics or antidepressants for less than three months or use of unstable doses (i.e., clients took medications as prescribed and in the targeted dosage)"

Drug/Alcohol: "no drug abuse"

Physical: "absence of a medical condition that might explain panic symptoms"

Interventions	Comparison group(s): Wait-List	
	Intervention name: Interapy	
	Days of treatment: 77	
	Author wrote: Yes	
	Type of media: Internet	
	Type of therapy: CBT	
	Location used: Home/ Anywhere	
	Type of contact: Email	
	Automated contact: N= 0	
	Provider contact: N=14 (450min)	
	Contact description: "At specific occasions indicated in the manual, therapists post feedback and further instructions on the basis of the contents of this workbook. Therapists take about 20–40 min to read a client’s assignment, and to prepare feedback. The manual includes 14 of these feedback moments, so that a full treatment requires between 5 and 9 h of therapist time."	
	Intervention description: “seven treatment modules...attacks are explained...Panic Diary...breathing retraining...encouraged to expose...educated about different forms of exposure (in vitro/in vivo/ interoceptive)...cognitive restructuring...formulate a ‘relapse prevention toolkit’”	
	Compliance: "read a client’s assignment, and to prepare feedback"	
Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ); Body Sensations Questionnaire (BSQ); Mobility Inventory for Agoraphobia (MIA): Alone; Panic Attacks: Frequency (Full Attacks); Panic Attacks: Intensity (Full); Panic Disorder Severity Scale, self-rated (PDSS-SR)	
	General Symptoms: Penn State Worry Questionnaire (PSWQ)	
	Response: Panic Attacks: Frequency	
	Depression: Depression, Anxiety, Stress Scales (DASS): Depression	
Notes	Funding: The Netherlands Organization for Health Research and Development and Interapy	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "The participants were randomly assigned to two groups."
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation."

Ruwaard 2010 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Provider contact: Yes Provider blind: No "read a client's assignment, and to prepare feedback."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0.00%) <i>Comparison:</i> N=0 (0.00%) Method of analysis: LOCF
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Salaberria 1995

Methods	Location: Basque Country, Spain Recruitment: Advertising and referral Exclusion rate: 23.81% Randomised (N): 80 (27 intervention; 27 comparison)
Participants	Disorder: Social Anxiety (Specific); ADIS (DSM-III R) Duration (years): 10 Age (years): 31 Sex (female): 48% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: "substance abuse"

Salaberria 1995 (Continued)

Physical: No "incapacitating illness."

Interventions	Comparison group(s): SH + Group CBT versus CBT alone Comparison description: Group therapy without self-help. Participants in all groups received group therapy focusing on self-exposure homework (8 x 2.5 hours). One group received cognitive therapy. Intervention name: Managing Anxiety (Butler, 1985) [translated] Days of treatment: 56 Author wrote: Yes Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=8 (1200 min, common to both groups) Contact description: All groups received group therapy with approximately 1200 minutes during treatment. Intervention description: Within two group therapy comparisons, participants were randomised to self-help or no self-help. Taught to identify and question automatic thoughts and to suggest alternative responses. Cognitive restructuring exercises. Compliance: Compliance with the booklet not reported. "record keeping sheets are distributed so that patients may take notes at home on said exercises"
Outcomes	None included.
Notes	Funding: University of Basque Country (UPV 006.230-H067/90 Research Grant) Unpublished data: Echeburúa replied that they have no further information but estimated that "participants were Caucasian, they had about 15 years of education and the disorder lasted for more than 10 years. At the start of the trial nobody was on medication."

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "Patients were randomly assigned to the three groups, after they had been preliminarily stratified according to age and gender...At the same time, and given that another of the aims of this study was to assess the additional efficacy of a self-help manual for management of anxiety, a factorial 2x2 design was used..."
Allocation concealment (selection bias)	Unclear risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias)	High risk	Included contact with researcher/clinician (not blind). All groups received group therapy.

Salaberria 1995 (Continued)

All outcomes

Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (18.52%) <i>Comparison:</i> N=4 (14.81%) Method of analysis: Per protocol 9 lost before treatment began (7 failed to attend the evaluation, 2 failed to initiate treatment). "The number of dropouts in all phases was 15 (3 of them control group participants). Thus, the total number of refusals, dropouts, and loss of patients in all groups comes to 17..." The "control group" was not part of the factorial study and was not included; assumed equal allocation to groups.
Selective reporting (reporting bias)	High risk	No trial registration number given. The main comparison in this trial is not eligible. Within the two main comparisons, the self help groups can both be included, though only one outcome is reported. Authors note that all others were non-significant.
Other bias	Low risk	N/A

Schneider 2005

Methods	Location: London, England Recruitment: Advertising and referral Exclusion rate: 27.66% Randomised (N): 68 (45 intervention; 23 comparison)
Participants	Disorder: Mixed (AG, SAD, SP); Clinical Interview (ICD-10) Duration (years): 14 Age (years): 39 Sex (female): 73.53% Race (white): 61.76% Medication (using at baseline): 61.76% Education (years): 12 EXCLUSIONS Depression: Yes Psychotherapy: Previous psychotherapy restricted

Schneider 2005 (Continued)

Medication: "not on a benzodiazepine or diazepam-equivalent dose of 15 mg/day" "not begun or changed dose or type of antidepressant within the last 4 week"

Drug/Alcohol: "drank <21 units (men) or <14 units (women) of alcohol a week"

Physical: "disabling cardiac or respiratory disease"

Interventions

Comparison group(s): Muscle relaxation (self-help)

Comparison description: Internet CBT without exposure. Book (Managing Anxiety) plus audio CD with progressive muscle relaxation. "...7 rather than 9 modules and encouraged a similar amount of time spent at the computer and on homework (but of a non-exposure kind)." "...asked them to do at least a week of homework between each of the 6 sessions..keep a mood diary...note anxiety triggers...taught the reason for practising relaxation..positive thoughts...healthy habit diary...coping responses..."

Intervention name: Fear Fighter (Marks 2004)

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: Exposure

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=0

Provider contact: N=6 (115min)

Contact description: "Immediately after randomisation, patients were e-mailed (n = 59) or posted (n = 9): (a) their assigned 3 pages of user instructions..." "All patients had 6 scheduled brief therapist contacts by phone or e-mail" "40 min on the telephone screening interview. Excluding screening time, their mean 8 SD time spent per patient supporting and rating patients by phone totalled: (a) during treatment weeks 0-10: FF 115 ± 44 min, MA 87 ± 28 min, F(1,46)=5.0, p=0.03; of this time, 80% were spent giving support, 17% obtaining ratings and 3% giving technical advice.."

Intervention description: Net-accessed FF had 9 self-exposure steps to be completed in 6 sessions Daily self-exposure homework diaries.

Compliance: 5.9 (0.4) treatment sessions intervention, 5.7 (1.0) control. 44(43) hours doing homework, 29(58) control.

Outcomes

Specific Symptoms: Fear Questionnaire (FQ): Global Phobia, assessor rated

General Symptoms: Fear Questionnaire (FQ): Anxiety Depression; Fear Questionnaire (FQ): Total Phobia

Disability: Work and Social Adjustment Scale (WSAS); Work and Social Adjustment Scale (WSAS), assessor rated

Notes

Funding: EU Marie Curie Fellowships supported Dr. Schneider and Dr. Mataix-Cols. The Leeds Community and Mental health Services NHS Trust gave a grant towards and referred patients for the trial.

Unpublished data: Marks forwarded queries to Andreas J. Schneider who did not reply.

Risk of bias

Schneider 2005 (Continued)

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "Stratifying for phobia type (agoraphobia, social and specific phobia), patients were randomised..." See allocation concealment.
Allocation concealment (selection bias)	Low risk	"At the end of the telephone screening interview, the interviewer found the randomly assigned treatment condition for the suitable patients by opening the next opaque, sealed envelope in a previously mixed up pile of trial-numbered envelopes in the required 2:1 ratio. Each envelope contained a card noting the randomised treatment for a given patient's trial number."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) 6 scheduled brief therapist contacts by phone
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "During rating interviews, the treatment group was in-advertently revealed to the rater by 1 patient at week 0, 6 patients at week 10 and 4 patients at week 14. Excluding those, the blinded rater's guesses as to the patient's treatment condition were no better than chance at weeks 0 (kappa = 0.12, p = 0.22) and 10 (kappa = -0.02, p = 0.81) but slightly better than chance at week 14 (kappa = -0.3, p = 0.026)."
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=12 (26.67%) <i>Comparison:</i> N=8 (34.78%) Method of analysis: Available Case "Intention-to-treat outcome analysed all randomised patients in the group to which they were originally randomised [16]. Where postbaseline data were unavailable, baseline data were not carried forward in the manner often done, as it is unlikely that scores remained frozen at their last observed value [17]." "At week 14, data of 44 (91%) patients were available (31 FF, 13 MA)."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Seekles 2011

Methods	Location: Amsterdam, Holland Recruitment: Referral Exclusion rate: 89.14% Randomised (N): 120 (60 intervention; 60 comparison)
---------	---

Seekles 2011 (Continued)

Participants

Disorder: Mixed (GAD, PD, SAD; 93% had an anxiety disorder, 44% had only an anxiety disorder); CIDI (DSM-IV)

Duration (years): 21.5

Age (years): 50.6

Sex (female): 66.67%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "receiving current (less than 2 months) treatment (medical/psychotherapy) for psychological problems" "Benzodiazepines were allowed in both study groups. Patients in the stepped care group were only allowed to receive antidepressants in later phase of the treatment protocol."

Drug/Alcohol: "severe alcohol problems (N20 on the Alcohol Use Disorders Identification Test (AUDIT))"

Physical: N/A

Interventions

Comparison group(s): Treatment-as-usual

Comparison description: Treatment as usual (wait-list). "Patients in the care as usual who responded (n=38) received the following care: 21 (55%) went to their GP, 10 (26%) received mental health care (psychiatric nurse, specialized mental health care, a psychiatrist or a psychotherapist), 2 (5%) went to a medical specialist, 1 (3%) went to a social worker and 4 (11%) went to some form of complementary and alternative medicine. Nine patients (24%) received benzodiazepines but none of them received antidepressants."

Intervention name: Problem Solving Treatment (van Straten et al., 2008) and exposure (Cuijpers and De Neef, 2007)

Days of treatment: 35

Author wrote: Yes

Type of media: Mixed

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=1 (duration unknown)

Contact description: For PST: "If they applied for feedback, they were supported by email (the Internet group) or by telephone (the book group). ..." For exposure: "Feedback is therefore provided by telephone." "For both: "During the first session, the psychiatric nurse decides together with the patient,

Seekles 2011 (Continued)

and based on the symptoms, which self-help course is most suitable." "None of the patients who received a self-help intervention via book (PST or Phobias) contacted the coach for feedback."

Intervention description: "The first is a generic intervention based on Problem Solving Treatment (PST)...(van Straten et al., 2008)... could choose to follow this course via the Internet or by using a book... The second self-help intervention is aimed specifically at patients with phobias and is based on exposure therapy (Cuijpers and De Neef, 2007)... make a plan to practice exposure...takes six weeks to complete and is only available as a book."

Compliance: 28% self-help via internet, 32% self-help via book, 15% self-help for phobias, 10% refused. "The 16 patients who performed the PST online completed 1.13 (SD=1.4) out of 6 lessons on average. It is however not possible to draw any conclusions from this number because all patients with a book or internet and who did not fill in their homework online are not included."

Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS) Response: Clinically Significant Improvement (Composite) Depression: Inventory of Depressive Symptomatology (IDS) Quality of Life: Medical Outcomes Study Short Form (SF-36)
Notes	Funding: "We would like to thank GGZ-inGeest and Mentrum for providing specialized mental health care and for supervising psychiatric nurses/psychologists in the intervention." Some results are reported separately for people with anxiety and depression. For anxiety and depression, we analysed those with anxiety only and those with comorbid anxiety/depression.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "computer-generated block randomization"
Allocation concealment (selection bias)	Low risk	"An independent researcher, not involved in the current project, used computer-generated block randomization to produce sealed envelopes. After every inclusion the researcher opened a sealed envelope."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "GPs were informed which patients were randomized to the stepped care program to ensure that the patient received treatment from the GP and to ensure adherence to the stepped care protocol."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (8.33%) <i>Comparison:</i> N=7 (11.67%)
Method of analysis: Available Case		

Seekles 2011 (Continued)

"Twelve (10.0%) of the 120 randomized participants reported severe dysfunctioning on the WSAS...were referred to the last step of our stepped care model (psychotherapy or medication) and...are excluded from the current analysis."
"Missing items on questionnaires were imputed using a Two-Way Imputation of Missing Test Data (Sijtsma and Van der Ark, 2003)...Missing total scores on questionnaires (because respondents did not return the questionnaire) were imputed using regression imputation."

Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Sharp 2000

Methods	Location: Fort Valley, UK, Scotland Recruitment: Referral Exclusion rate: 21.21% Randomised (N): 104 (35 intervention; 36 comparison)
Participants	Disorder: Panic disorder; Clinical Interview (DSM-III R) Duration (years): 2.67 Age (years): 39.4 Sex (female): 68.12% Race (white): 46.67% Medication (using at baseline): 46.67% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: "Any patients taking concurrent psychotropic medications were required to have taken them for two months prior to study entry, and to continue taking them as prescribed throughout the study period." Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Individual CBT Comparison description: All patients received the same CBT and all received identical treatment instructions, the same written treatment manual being supplied to all patients. "Patients in the standard contact condition received the standard treatment manual and eight sessions of 45 minutes' duration over 12 weeks with sessions at Days 0, 7, 14, 28, 42, 56, 70, and 84; a total of six hours' therapist contact."

Sharp 2000 (Continued)

Intervention name: Similar to: Psychological Treatment of Panic (Barlow and Cerny, 1988)

Days of treatment: 84

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=2 (60min)

Contact description: “Patients in the bibliotherapy condition received the treatment manual and assessment sessions at Days 0, 42, and 84. The one hour and 30 minutes of therapist contact in this condition was for assessment only, with treatment instruction provided solely by the treatment manual.”

Intervention description: “...emphasised the alteration of the action tendencies associated with panic attacks and the hypervigilant and avoidant information processing and behaviours typical of panic disorder and agoraphobia. Patients were encouraged to confront their panic attacks—during exposure outings if necessary—and to attempt to replace behavioural and cognitive avoidances with more approach-centred actions. This approach to CBT was similar to that of Barlow and co-workers...”

Compliance: None reported.

Outcomes	Specific Symptoms: Fear Questionnaire (FQ): Agoraphobia General Symptoms: Hamilton Anxiety Rating Scale, clinician-rated (HAM-A) Response: Panic Free Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Disability: Sheehan Disability Scale (SDS): Family/Home; Sheehan Disability Scale (SDS): Social/Leisure	
Notes	Funding: None acknowledged Unpublished data: Author provided details of the randomisation, allocation concealment and baseline characteristics. Confirmed no measures of compliance. Unable to provide further information about dropouts.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “Randomisation was conducted according to a computer-generated randomisation schedule, with individual allocations unavailable until after initial assessment.”
Allocation concealment (selection bias)	Low risk	“Administered using opaque envelopes.”

Sharp 2000 (Continued)

Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “patients were seen for assessment prior to treatment group randomisation and thereafter for treatment by the same clinical psychologist” “Patients in the standard contact condition received the standard treatment manual and eight sessions of 45 minutes...”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “Treatment end-point assessments were conducted by an independent clinical psychologist (KP) and patients were instructed not to discuss their treatment with the blind endpoint assessor.”
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=6 (17.14%) <i>Comparison:</i> N=5 (13.89%) Method of analysis: Available Case “Patients withdrawing early owing to effectiveness or ineffectiveness of treatment, receiving at least 42 days treatment, and who provided full endpoint data, were included in the final analysis as ‘defined completers’...A total of 91 ‘full completers’ and ‘defined completers’ comprised the study group.”
Selective reporting (reporting bias)	High risk	No trial registration number given. Data from panic diaries are partially reported. 6-week data are not reported in one paper, though they appear in the other. 6-month follow-up are not reported in full.
Other bias	High risk	Data are reported in two papers that do not reference each other and which report partial outcomes and time points. 6-month outcomes are reported for a subgroup of participants, and these outcomes were not extracted.

Shoenberger 2008

Methods	Location: Reno, Nevada, USA Recruitment: College only Exclusion rate: Unknown Randomised (N): 57 (30 <i>intervention</i> ; 27 <i>comparison</i>)
Participants	Disorder: Generalised Anxiety Disorder; No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 28 Sex (female): 100% Race (white): 22% Medication (using at baseline): 22% Education (years): 14

Shoenberger 2008 (Continued)

EXCLUSIONS

Depression: No

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Participants had not initiated any psychiatric medications 30 days prior to enrolling in the study, and did not change doses of psychiatric medications that she was taking for longer than 30 days, during the course of the

treatment."

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Treatment-as-usual

Comparison description: "The control condition consisted of 27 participants who did not receive the self-help book, but could be seeking services from other healthcare providers such as physicians and the university counseling center."

Intervention name: Women Who Worry Too Much

Days of treatment: 28

Author wrote: No

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=4 (duration unknown)

Contact description: "Intervention participants were contacted weekly via e-mail by the co-investigator. The weekly e-mail contacts were non-directive check-ins which were designed to provide intervention participants with an opportunity to ask questions about specific techniques or exercises. This opportunity was provided in order to minimally address concerns about treatment accuracy."

Intervention description: "The book contained information about chronic worry and evidence-based CBT skills including: behavioral activation, relaxation, problem-solving, perspective taking, identifying and managing unhelpful thoughts, and assertiveness skills targeting reduction of the frequency and intensity of chronic worry. Participants were instructed to practice these techniques and exercises as frequently as possible throughout the four-week intervention."

Compliance: "Intervention compliance was assessed using a self-report continuum with 0 being 'did not use at all' and 100 being 'used completely.' Overall, intervention participants (N = 30) reported moderate utilization of the self-help materials (M = 49.33, SD = 27.16)."

Outcomes

Specific Symptoms: Generalized Anxiety Disorder Questionnaire–IV

General Symptoms: State-Trait Anxiety Inventory (STAI)

Depression: Center for Epidemiologic Studies Depressed Mood Scale (CESD)

Shoenberger 2008 (Continued)

Quality of Life: Quality of Life Inventory (QOLI)

Notes	Funding: None acknowledged	
<i>Risk of bias</i>		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Other “Participants were randomly assigned to one of two conditions before the intervention using a table of random numbers.”
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “Intervention participants were contacted weekly via e-mail by the co-investigator.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	No dropout.
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Silfvernagel 2012

Methods	Location: Stockholm, Sweden Recruitment: Advertising/ Internet Exclusion rate: 61.74% Randomised (N): 57 (29 intervention; 28 comparison)
Participants	Disorder: Panic Attacks; SCID (DSM-IV) Duration (years): N/A Age (years): 32.4 Sex (female): 64.91% Race (white): 47.37% Medication (using at baseline): 47.37% Education (years): 13.44

Silfvernagel 2012 (Continued)

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "if on medication it had to be on stable dosage for the last three months"

Drug/Alcohol: "not be at risk of alcohol abuse or fulfilling the criteria for current alcohol addiction"

Physical: N/A

Interventions	Comparison group(s): Wait-List Intervention name: Not named Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Email Automated contact: N=0 Provider contact: N=19 (120min) Contact description: "If the participant had not sent in their homework...therapist would send out an email reminder. The therapists were instructed to check whether the participants had completed the assignments...and to provide individual feedback within 24 hours. If the therapist believed that the participant was ready to move on with their treatment the next module was made available...approximately 15 min per week per participant (estimated), which included administration as well as reading and responding to emails (estimated 19 emails between therapist and participant throughout the treatment)..." Intervention description: "The treatment consisted of 19 CBT modules derived from previous iCBT trials on panic disorder [31], generalized anxiety disorder, social phobia [32], depression [33] and tailored iCBT for anxiety [17, 18] and depression. In this trial the first module (introduction) and the last module (relapse prevention) were fixed and the following 17 were optional for the therapists to prescribe..." Compliance: "Seven (24.1 %) out of the 29 participants completed all prescribed modules (6-8) within the eight week treatment period. A total of 17 participants (59.0 %) completed 50 per cent of prescribed modules and 14 (48.3 %) completed 75 per cent of prescribed modules. Mean for number of completed modules for the whole group was M=5.0"
Outcomes	Specific Symptoms: Panic Disorder Severity Scale (PDSS) General Symptoms: Beck Anxiety Inventory (BAI) Response: Panic Disorder Severity Scale (PDSS) Depression: Montgomery and Asberg Depression Rating Scale (MADRS) Quality of Life: Quality of Life Inventory (QOLI)
Notes	Funding: Swedish Council for Working Life and Social Research

Silfvernagel 2012 (Continued)

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The blocked randomization process was conducted through an online true random-number service (random.org)"
Allocation concealment (selection bias)	Low risk	"independent of the investigators and therapists."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "The therapists were instructed to check whether the participants had completed the assignments..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "At post treatment participants were instructed via email to complete the follow-up questionnaires and to participate in a semi-structured telephone interview carried out by a blinded assessor who had no earlier contact with the participants. The same procedure was repeated at twelve months after treatment completion. We used the PDSS as the primary outcome measure in the trial."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: Mixed effects "A mixed models approach with an unstructured covariance structure was endorsed as a way to handle missing data at post treatment and at the 1 year follow up."
Selective reporting (reporting bias)	Low risk	Trial registration number: NCT01296321 Protocol suggests follow-up analyses should be forthcoming.
Other bias	Low risk	N/A

Smith 1997

Methods	Location: Tasmania, Australia Recruitment: Advertising/ Internet Exclusion rate: 0% Randomised (N): 49 (33 intervention; 16 comparison)
Participants	Disorder: Specific Phobia; CIDI Auto (DSM-III R) Duration (years): 22.7

Smith 1997 (Continued)

Age (years): 34.8

Sex (female): 97.96%

Race (white): 0%

Medication (using at baseline): 0%

Education (years): Unknown

EXCLUSIONS

Depression: No

Psychotherapy: No restrictions reported

Medication: "none were taking anxiolytic medication"

Drug/Alcohol: N/A

Physical: N/A

Interventions

Comparison group(s): Attention/Monitoring

Comparison description: "The vignette portrays a phobia of elevators. The scenario ranges from at home, along a street, and into a building of seven floors with an elevator. The anxiety thermometer is operating and the score system rewards approach and riding in the elevator."

Intervention name: CAVE (Smith 1997)

Days of treatment: 42

Author wrote: Yes

Type of media: Computer

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=3 (15min)

Contact description: Author estimated 45 minutes for the CIDI for duration of face-to-face assessment. For treatment, therapist stayed 5 min to answer questions and remained available in another room for assistance. Estimated 15 min contact over three sessions.

Intervention description: 1) "you are asked to help a person on the screen to overcome a specific anxiety problem - fear of x (i.e., spiders/lifts [elevators])... The vignette in the introductory section refers to spider phobia. Subjects have the choice of systematically exposing the screen figure to a variety of anxiety-evoking situations, ranging from mild to severe (spider picture, plastic spider, dead spider, or live spider). The anxiety thermometer is operating." 2) "The anxiety thermometer is omitted."

Compliance: "subjects in the RENO group reported fewer new activities than the other two groups...but the main effect failed to reach statistical significance...homework activities correlated significantly with clinical improvement as indexed by the SPQ ($r = -0.441$, $p < 0.01$), but was not significantly correlated with improvements on the SQ or problem and target ratings...more new homework activities showed a greater fall in SPQ scores."

Smith 1997 (Continued)

Outcomes	Specific Symptoms: Phobic Targets: Phobic Problem Disability: Work and Adjustment Rating Scale (WARS)	
Notes	Funding: None acknowledged Unpublished data: Gilroy did not have further information about missing participants. Provided details about contact and methods.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “Subjects were randomized to one of three treatment groups...”
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “During treatment, the therapist stayed 5 min to answer questions and remained available in another room for assistance.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (9.09%) <i>Comparison:</i> N=1 (6.25%) Method of analysis: Per protocol Three subjects dropped out before completing all treatment sessions and a fourth subject was excluded from the analysis due to missing data. "Seven of 45 subjects did not complete the 6- to 12 month follow-up (3 from IF group, 3 from REF and 1 from RENoF) due to 1 subject receiving treatment for spider phobia following the posttreatment sessions, and others experiencing changes in personal circumstances, such as moving from the state. No additional treatment was sought by the remaining subjects during the follow-up."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Sorby 1991

Methods	Location: Southampton & Portsmouth, England Recruitment: Referral Exclusion rate: Unknown	
---------	--	--

Sorby 1991 (Continued)

Randomised (N): 64 (33 intervention; 31 comparison)

Participants
Disorder: Mixed (GAD, PD); SCID (DSM III R)

Duration (years): Unknown

Age (years): Unknown

Sex (female): 81.25%

Race (white): 67%

Medication (using at baseline): 67%

Education (years): Unknown

EXCLUSIONS
Depression: Yes

Psychotherapy: No restrictions reported

Medication: "to minimize the effects of medication, it was agreed that no changes in psychotropic medication were to be made at the start or within the first two weeks of the study..."

Drug/Alcohol: "...alcohol or substance misuse were excluded"

Physical: N/A

Interventions
Comparison group(s): Treatment-as-usual

Comparison description: "Patients in the control group were not made aware of the booklet."

Intervention name: Anxiety-An explanation and self-help manual (Sorby and Reavley).

Days of treatment: 56

Author wrote: Yes

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=4 (90min)

Contact description: At the end of the interview, members of the experimental group were given a copy of the booklet Anxiety- an explanation and self help manual (written by NS and WR) and up to 10 minutes were spent explaining the contents. "Patients were asked to return to their doctor two, four and eight weeks after the initial consultation. Directly after each visit the patients completed the questionnaires used at the first session, at home, and returned them to the surgery by post or by hand." About 30 minutes of contact with a GP and 60 minutes of contact with a researcher (personal communication).

Intervention description: "The booklet describes how anxiety operates in terms of the three factor theory of physical, mental and avoidance components. How these components relate to each other and operate to create and maintain anxiety is outlined. Advice is given on how readers can intervene at

Sorby 1991 (Continued)

different stages in the anxiety cycle and develop confidence in coping strategies which they are encouraged to apply in a range of situations, and on how they can monitor their progress."

Compliance: Other treatments received were not measured.

Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety Depression: Hospital Anxiety and Depression Scale (HADS): Depression
Notes	Funding: None acknowledged Unpublished data: Sorby provided details about randomisation and demographic characteristics. "Looks like I calculated the depression values from the total HADS score and the anxiety score."

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "After agreeing to take part in the study patients were randomly allocated...using a random card pack." "The random card pack was literally a series of cards in brown envelopes that were shuffled and picked at random."
Allocation concealment (selection bias)	High risk	"One of the authors (NS) arranged to call on each patient within 72 hours to collect the completed questionnaire and to carryout a structured clinical interview for DSM II, to confirm the subjects' suitability for the trial." "Following the structured clinical interview, two women and one man in the control group, and one woman in the booklet group were excluded as they failed to meet the inclusion criteria."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Participants in the control group were not aware of the booklet (the intervention). "Patients were asked to return to their doctor two, four and eight weeks after the initial consultation."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (9.09%) <i>Comparison:</i> N=12 (38.71%) Method of analysis: Available Case "Three men and one woman in the control group dropped out but there were no dropouts from the booklet group. Five women in the control group and two women in the booklet group failed to complete the questionnaires satisfactorily. Following the structured clinical interview, two women and one man in the control group, and one woman in the booklet group were excluded as they failed to meet the inclusion criteria. Thus, 49 patients (76.6%) successfully completed the trial- 30 in the booklet group (90.9%) (23 women and seven men) and 19 in the control group (61.3%) (18 women and one man)."

Sorby 1991 (Continued)

Selective reporting (re-reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Tarrier 1986

Methods	Location: Manchester, England Recruitment: Referral Exclusion rate: Unknown Randomised (N): 40 (20 intervention; 10 comparison)
Participants	Disorder: Generalised Anxiety Disorder; Unclear Duration (years): 5.34 Age (years): 40.5 Sex (female): 60% Race (white): 68% Medication (using at baseline): 68% Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: No restrictions reported Medication: "those who thought their medication was no longer producing any improvement were included, and were asked to continue taking their medication." Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Wait-list, Relaxation Comparison description: 1) Wait-list, 2) Relaxation therapy taught by demonstration. Intervention name: Not named Days of treatment: 42 Author wrote: Yes Type of media: Mixed Type of therapy: Applied Relaxation Location used: Clinic/ Surgery Type of contact: None

Tarrier 1986 (Continued)

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: Contacted for referral and assessment. No contact during the trial reported.

Intervention description: Two eligible groups: 1) Booklet relaxation. 2) Taped relaxation. "All patients received a rationale for applied relaxation training...(a) self-monitoring of anxiety levels; (b) correct breathing monitoring of breathing and the use of exercises to encourage diaphragmatic breathing; (c) progressive muscle relaxation, consisting of tension release exercises; (d) positive mental imagery."

Compliance: "One third of the group had complied with the instructions on 14 days or less; about 25% practised on 29 days or more. The mean number of days practice was 23.1 (s.d. 15.7)...Compliance was also calculated as the percentage of times practised, where once per day would indicate 100% compliance; 24% of patients had a compliance of 76% or more, and there was a mean compliance of 68% s.d. 42.0, range 0-200%)."

Outcomes	None included.	
Notes	Funding: None acknowledged Unpublished data: Author provided information about randomisation and number of participants allocated. Unable to provide further information about outcomes.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “...were randomly allocated to one of four treatment conditions or to a waiting list control ...”
Allocation concealment (selection bias)	Low risk	Independent, "i.e. not by me as I carried out the treatment" (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	Unclear risk	Unclear if there was contact with researchers or providers.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	High risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (15%) <i>Comparison:</i> N=0 (0%) Method of analysis: Unclear 4 dropouts, none from the wait-list control group. Assumed dropout was equivalent across groups.
Selective reporting (reporting bias)	High risk	No trial registration number given.

Tarrier 1986 (Continued)

Groups were combined post-hoc for analysis. None of the data can be extracted.

Other bias	Low risk	N/A
------------	----------	-----

Titov 2008a

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 68.47% Randomised (N): 105 (50 intervention; 55 comparison)
Participants	Disorder: Social Anxiety (Unspecified); CIDI (DSM-IV R) Duration (years): 23.6 Age (years): 38.13 Sex (female): 58.59% Race (white): 30.3% Medication (using at baseline): 30.3% Education (years): 13.01 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "had been taking the same dose for at least 1 month and did not intend to change that dose during the course of the programme (if taking medication for social phobia)" Drug/Alcohol: "not using illicit drugs or consuming more than three standard drinks/day" Physical: N/A
Interventions	Comparison group(s): Wait-List Comparison description: The control group was asked to post a greeting to other participants in an online forum (to reduce attrition). Intervention name: Shyness Program (http://www.climateclinic.tv) Days of treatment: 70 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere

Titov 2008a (Continued)

Type of contact: Email

Automated contact: N=16

Provider contact: N=6 (125min)

Contact description: Average 22 emails and responses on the forum (personal communication). "After completing each lesson participants were emailed by the therapist. The themes of the therapist's emails varied from reinforcement for continued participation and efforts, encouragement to practise..." "It was estimated that the mean amount of therapist time per participant was 125 min (SD=25 min), including monitoring of the discussion forum and providing email...An additional average 25min per patient was required for administrative purposes..."

Intervention description: "...six online lessons were based on the online social phobia lessons from the CLIMATEGP programme (www.climategp.tv)...illustrated story about a young man with social phobia...education...exposure hierarchy...cognitive restructuring...relapse prevention."

Compliance: Eleven "failed to complete all six lessons...two formally withdrew citing lack of time and motivation after experiencing a death or illness...exposure exercises were too anxiety provoking...did not find the programme helpful...overseas holiday; three cited a change in work or study...one reported complications due to her pregnancy and two did not give a reason. The mean number of lessons completed was 5.2..." 2 terminated at lesson 1, 3 at lesson 2, 4 at lesson 3, and 2 at lesson 4.

Outcomes	Specific Symptoms: Social Interaction Anxiety Scale (SIAS) Depression: Patient Health Questionnaire (PHQ-9) Disability: WHO Disability Assessment Schedule (WHODAS-II)
Notes	Funding: Faculty of Medicine, University of New South Wales Unpublished data: Titov provided details about baseline characteristics, allocation concealment, contact with participants and trial registration. Titov contacted a colleague who has GAD-7 results, but data have not been provided.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The 105 people accepted into the Shyness programme were randomized via a true randomization process (www.random.org)..."
Allocation concealment (selection bias)	Low risk	"We used a series of envelopes to conceal allocation from interviewers, and they were instructed to open each envelope only after a person they were interviewing had met criteria." (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) The themes of the therapist's emails varied from reinforcement for continued participation and efforts, encouragement to practise "One clinical psychologist (NT) provided all clinical contact with participants."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias)	Low risk	Not analysed at post-treatment

Titov 2008a (Continued)

All outcomes

Intervention: N=0 (0%)

Comparison: N=6 (10.91%)

Method of analysis: LOCF

6 control participants dropped out before baseline measures were collected.

"Post-treatment data were collected from 93 participants (44/50 treatment group participants and 49/49 waitlist control group participants). In accordance with the intention-to-treat paradigm, the pre-treatment scores of these six participants who did not complete the post-treatment questionnaires were replicated as their post-treatment scores."

Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12607000235404 Outcomes are reported clearly except the GAD-7. Subsequent reports include the combined GAD data for the intervention and control groups, but these could not be extracted for meta-analysis. (See also the trial registration and personal communication.)
Other bias	Low risk	N/A

Titov 2008b

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 68.57% Randomised (N): 88 (43 intervention; 45 comparison)
Participants	Disorder: Social Anxiety (Unspecified); CIDI (DSM-IV R) Duration (years): Unknown Age (years): 36.79 Sex (female): 62.96% Race (white): 25.93% Medication (using at baseline): 25.93% Education (years): 13.14 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "No significant changes were made either to the inclusion criteria, treatment procedure, or materials used in the original Shyness programme." Drug/Alcohol: "No significant changes were made either to the inclusion criteria, treatment procedure, or materials used in the original Shyness programme."

Titov 2008b (Continued)

Physical: N/A

Interventions	Comparison group(s): Wait-List	
	Intervention name: Shyness Program (http://www.climateclinic.tv)	
	Days of treatment: 70	
	Author wrote: Yes	
	Type of media: Internet	
	Type of therapy: CBT	
	Location used: Anywhere	
	Type of contact: Email	
	Automated contact: N=2.36	
	Provider contact: N=22.32 (126.76min)	
	Contact description: The average therapist time per treatment group participant was 126.76 min (SD=30.89 min) including monitoring of the discussion forum and feedback. An additional average 25 min per patient was required for administrative purposes, including the diagnostic telephone interview. A review of email and forum contact showed that a total of 915 emails were sent to treatment group participants (mean=22.32, SD=4.06), with a mean of 2.36 (SD=2.06) forum postings in response to each participant's posts (additional postings were made to the entire group).	
	Intervention description: "The treatment programme was the same used in the original Shyness programme: six online lessons; homework assignments; participation in an online discussion forum; and regular email contact with a therapist."	
	Compliance: "Of these eight noncompleters, one said the programme was not helpful, and one reported they had improved sufficiently. The mean number of lessons completed was 5.5, and 33 participants completed all six lessons. The overall completion rate was 80%." 2 terminated at lesson 1, 1 at lesson 3, and 2 at lesson 4, 3 at lesson 5.	
Outcomes	Specific Symptoms: Social Interaction Anxiety Scale (SIAS)	
	Depression: Patient Health Questionnaire (PHQ-9)	
	Disability: WHO Disability Assessment Schedule (WHODAS-II)	
Notes	Funding: Faculty of Medicine, University of New South Wales	
	Unpublished data: Titov provided details about baseline characteristics, allocation concealment, contact with participants and trial registration. Titov contacted a colleague who has GAD-7 results, but data have not been provided.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The 88 people accepted into the programme were randomized via a true randomization process (www.random.org)..."

Titov 2008b (Continued)

Allocation concealment (selection bias)	Low risk	"We used a series of envelopes to conceal allocation from interviewers, and they were instructed to open each envelope only after a person they were interviewing had met criteria." (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "The treatment programme was the same used in the original Shyness programme: six online lessons; homework assignments; participation in an online discussion forum; and regular email contact with a therapist." "One clinical psychologist (NT) provided all clinical contact with participants. A research assistant (GS) provided administrative support to collate data, follow progress and assist in the telephone interviews."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (11.63%) <i>Comparison:</i> N=5 (11.11%) Method of analysis: LOCF "Although eight members of the treatment group did not complete all six lessons, post-treatment data were collected from 78 participants (38/41 treatment group participants and 40/40 waitlist control group participants). In accordance with the intention-to-treat paradigm, the pre-treatment scores of the three treatment group participants who did not complete the post-treatment questionnaires were replicated as their post-treatment scores."
Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12608000465358 Outcomes are reported clearly except the GAD-7. Subsequent reports include the combined GAD data for the intervention and control groups, but these could not be extracted for meta-analysis. (See also the trial registration and personal communication).
Other bias	Low risk	N/A

Titov 2008c

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 40.24% Randomised (N): 98 (63 intervention; 35comparison)
Participants	Disorder: Social Anxiety (Unspecified); MINI (DSM-IV R) Duration (years): Unknown Age (years): 37.97 Sex (female): 61.05%

Titov 2008c (Continued)

Race (white): 21.74%

Medication (using at baseline): 21.74%

Education (years): 13.3

EXCLUSIONS

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "No significant changes were made either to the inclusion criteria or materials used in the previous Shyness programmes."

Drug/Alcohol: "No significant changes were made either to the inclusion criteria or materials used in the previous Shyness programmes."

Physical: N/A

Interventions

Comparison group(s): Wait-List

Intervention name: Shyness Program (<http://www.climateclinic.tv>)

Days of treatment: 70

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email & Phone

Automated contact: N=Unknown

Provider contact: N=18.32 (84min)

Contact description: "per CaCCBT group participant was 168 min (SD=40min)...25 min per patient was required for administrative purposes, including the diagnostic telephone interview. A review of email contact showed that a total of 568 emails were sent to treatment group participants (mean=18.32, SD=4.84). Additional contacts included telephone calls to participants who were inactive...and regular responses to forum postings..."

Intervention description: Group 1 "received the usual Shyness treatment programme." Group 2 "received the Shyness treatment programme as described in the previous section, but without regular emails or forum responses from the therapist."

Compliance: "The mean number of lessons completed was 5.39 (SD=1.31) and 3.97 (SD=1.87), for the CaCCBT and CCBT groups, respectively. In the therapist-assisted and no-email intervention groups, 75% and 32% completed all lessons. There was a significant relationship between number of lessons completed and changes in SPS scores, events avoided, and events attended."

Outcomes

Specific Symptoms: Social Interaction Anxiety Scale (SIAS)

Depression: Patient Health Questionnaire (PHQ-9)

Disability: Sheehan Disability Scale (SDS)

Titov 2008c (Continued)

Notes

Funding: None acknowledged

Unpublished data: Titov provided details about baseline characteristics, allocation concealment, contact with participants and trial registration. Titov contacted a colleague who has GAD-7 results, but data have not been provided.

Two intervention groups differ in amount of contact.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The 98 people accepted into the programme were randomized via a true randomization process (www.random.org)..."
Allocation concealment (selection bias)	Low risk	"We used a series of envelopes to conceal allocation from interviewers, and they were instructed to open each envelope only after a person they were interviewing had met criteria." (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) Two groups were combined for analysis. One group interacted with researcher/clinician (not blind) throughout the trial.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=2 (3.17%) <i>Comparison:</i> N=0 (0%) Method of analysis: LOCF "2 treatment and 1 control withdrew before beginning treatment." 4 "withdrew during the programme (two citing work commitments, one citing relationship breakdown, one citing death in the family), one did not complete, citing exam pressures, one did not complete, citing work commitments, and one could not be contacted. Of the non-completers from the CCBT group one participant formally withdrew during the programme (citing work commitments), three did not complete, citing work commitments, 11 reported that they were unwilling to complete the exposure component of treatment independently, while five either could not be contacted..." "analyses involve an intention-to-treat (ITT) design."
Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12608000192381 Outcomes are reported clearly except the GAD-7. Subsequent reports include the combined GAD data for the intervention and control groups, but these could not be extracted for meta-analysis. (See also the trial registration and personal communication).
Other bias	Low risk	N/A

Titov 2009a

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 28.36% Randomised (N): 48 (25 intervention; 23comparison)
Participants	Disorder: Generalised Anxiety Disorder; MINI (DSM-IV R) Duration (years): Unknown Age (years): 44 Sex (female): 75.56% Race (white): 28.89% Medication (using at baseline): 28.89% Education (years): 13.52 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "if taking medication, had been taking the same dose for at least 1 month and did not intend to change that dose during the course of the programme." Drug/Alcohol: "not using illicit drugs or consuming more than three standard drinks day" Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: The Worry Programme Days of treatment: 63 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Email & Phone Automated contact: N=49.7 Provider contact: N=9.6 (130min) Contact description: "The mean total therapist time per treatment group participant was 130 min...additional average 30 min per patient was required for administrative purposes, including the diagnostic telephone interview. During the programme the software sent a total of 568 automatic emails to treatment group participants (mean=23.7 per participant), and the clinician sent 132 person-

Titov 2009a (Continued)

al instant messages (mean=5.5 per participant). The clinician also made a total of 98 telephone calls (mean=4.1 telephone calls per participant) and made 26 forum postings to the entire group."

Intervention description: "... six online lessons; a summary/homework assignment for each lesson...illustrated story about a woman with GAD...education...challenging positive and negative beliefs about excessive worry... graded exposure...challenging core beliefs... relapse prevention. Participants were encouraged to complete the first four lessons within the first 4 weeks, in order to provide more opportunity to practise the graded exposure..."

Compliance: "That is, 75% of participants completed the programme..." 1 terminated at lesson 3, 5 terminated at lesson 5.

Outcomes	Specific Symptoms: Generalised Anxiety Disorder Assessment (GAD 7) Response: Generalised Anxiety Disorder Assessment (GAD 7) Depression: Patient Health Questionnaire (PHQ-9) Disability: Sheehan Disability Scale (SDS)	
Notes	Funding: Australian Rotary Health Research Fund Unpublished data: Titov provided unpublished manuscript, details about baseline characteristics, allocation concealment, contact with participants and trial registration.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “The 48 people accepted into the programme were randomized by NT via a true randomization process (www.random.org) to either the treatment (n=25) or control (n=23) groups (Table 1).”
Allocation concealment (selection bias)	Low risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) The mean total therapist time per treatment group participant was 130 min including monitoring of the discussion forum, instant messages, and telephone calls. "One clinical psychologist (ER) provided all clinical contact with participants."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=1 (4%) <i>Comparison:</i> N=2 (8.70%) Method of analysis: LOCF “Post-treatment data were collected from 21/24 (88%) treatment group participants and 19/21 (90%) control group participants. In accordance with the ITT

Titov 2009a (Continued)

paradigm, the pre-treatment scores of the five participants who did not complete the post-treatment questionnaires were replicated as their post-treatment scores.”

Selective reporting (reporting bias)	Low risk	Trial registration number: ACTRN12609000136202 All outcomes reported.
Other bias	Low risk	N/A

Titov 2010a

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 43.42% Randomised (N): 86 (42 intervention; 44 comparison)
Participants	Disorder: Mixed (GAD, PD, SAD); MINI (DSM-IV) Duration (years): Unknown Age (years): 39.5 Sex (female): 67.95% Race (white): 47.44% Medication (using at baseline): 47.44% Education (years): 13.15 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: Participants on medication were on a stable dose for 1 month. No benzodiazepines. Drug/Alcohol: No illicit drug use, no more than 3 drinks per day. Physical: N/A
Interventions	Comparison group(s): Wait-List Intervention name: Anxiety Program Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere

Titov 2010a (Continued)

Type of contact: Email & Phone

Automated contact: N=41.1

Provider contact: N=6 (46min)

Contact description: "46 min (SD=16min) including monitoring of the discussion forum, sending and reading instant messages and telephoning participants. An additional average 30 min per participant was required for administrative purposes, including the diagnostic telephone interview." "treatment group participants received a total of 765 automatic emails (mean=19.1 per participant), with the clinician sending a mean of 4.5 additional personal instant messages per participant. The clinician also made a total of 215 telephone calls (mean = 5 per participant, and made 22 forum postings to the entire group"

Intervention description: Six lessons, homework, online discussion forum, automatic emails, instant messaging with clinician. Psychoeducation, controlling physical symptoms lifestyle, principles of CBT, exposure, communication/assertiveness skills, relapse prevention. Participants encouraged to complete one each week, and to complete the six lessons within 8 weeks. Additional written resources. Illustrated stories about people who successfully begin to gain mastery over their symptoms.

Compliance: Thirty (75%) treatment group participants completed the six lessons within the eight weeks of the program. "30 participants completed all 6 Lessons (1 withdrew at lesson 2, 2 withdrew at Lesson 4), 34 completed Lesson 5, 38 completed Lesson 4, 39 completed Lesson 3, and all participants completed Lessons 1 and 2."

Outcomes	Specific Symptoms: Panic Disorder Severity Scale, self-rated (PDSS-SR); Penn State Worry Questionnaire (PSWQ); Social Phobia Screening Questionnaire (SPSQ) General Symptoms: Depression Anxiety Stress Scales - 21 Item (DASS-21) Depression: Patient Health Questionnaire (PHQ-9) Disability: Sheehan Disability Scale (SDS)	
Notes	Funding: Australian Rotary Health Research Fund; New South Wales Institute of Psychiatry	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “Eighty six applicants met all inclusion criteria and were randomized by NT via a true randomization process (www.random.org) to either the treatment (n=42) or control (n=44) groups (Table 1).”
Allocation concealment (selection bias)	Low risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "One clinical psychologist (ER) provided all clinical contact with treatment group participants and another clinical psychologist (JS) provided all clinical contact with the control group....Therapists were encouraged to actively engage with participants in treatment..."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias)	Low risk	Not analysed at post-treatment

Titov 2010a (Continued)

All outcomes

Intervention: N=2 (4.76%)

Comparison: N=6 (13.63%)

Method of analysis: LOCF

Intervention (6 dropouts): 2 withdrew before the intervention began, 4 (observations carried forward) did not complete post-treatment questionnaires. Control (8 dropouts): 5 withdrew before the program began, 1 did not complete pre-treatment questionnaire, 2 (carried forward) did not complete post-treatment questionnaires. "the pre-treatment scores of participants who did not complete the post-treatment or follow-up questionnaires were replicated as their post-treatment or follow-up scores" (intention-to-treat)

Selective reporting (reporting bias)	High risk	Trial registration number: ACTRN12609000501246 The MINI, BAI, SPS, SIAS, BVS, ACQ, and WHODAS-II are not reported. The GAD-7 and PSWQ are not reported by group.
Other bias	Low risk	N/A

Titov 2010a (GAD)

Methods	As above.
Participants	
Interventions	
Outcomes	
Notes	

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	
Allocation concealment (selection bias)	Low risk	
Blinding of participants and personnel (performance bias) All outcomes	High risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	Low risk	

Titov 2010a (GAD) *(Continued)*

Selective reporting (reporting bias)	High risk
Other bias	Low risk

Titov 2010a (PD)

Methods	See above.
Participants	
Interventions	
Outcomes	
Notes	

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	
Allocation concealment (selection bias)	Low risk	
Blinding of participants and personnel (performance bias) All outcomes	High risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	Low risk	
Selective reporting (reporting bias)	High risk	
Other bias	Low risk	

Titov 2010a (SAD)

Methods	See above.
Participants	
Interventions	

Titov 2010a (SAD) (Continued)

Outcomes

Notes

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	
Allocation concealment (selection bias)	Low risk	
Blinding of participants and personnel (performance bias) All outcomes	High risk	
Blinding of outcome assessment (detection bias) All outcomes	Low risk	
Incomplete outcome data (attrition bias) All outcomes	Low risk	
Selective reporting (reporting bias)	High risk	
Other bias	Low risk	

Titov 2010b

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 13.74% Randomised (N): 113 (57 intervention; 56 comparison)
Participants	Disorder: Social Anxiety (Unspecified); MINI (DSM-IV) Duration (years): Unknown Age (years): 43.6 Sex (female): 47.22% Race (white): 15.74% Medication (using at baseline): 15.74% Education (years): 13.56 EXCLUSIONS

Titov 2010b (Continued)

Depression: Yes

Psychotherapy: Current restricted, no previous restrictions reported

Medication: "Change in medications during last 1 month or intended change during study duration." (protocol)

Drug/Alcohol: "Current substance abuse/dependence" (protocol)

Physical: N/A

Interventions

Comparison group(s): Self-help (CBT)

Comparison description: The intervention was the same except for the reflective questions in the first 2 lessons.

Intervention name: Shyness Program (<http://www.climateclinic.tv>)

Days of treatment: 77

Author wrote: Yes

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=25

Provider contact: N=0 (0 min)

Contact description: "Participants did not receive any contact with the researchers once the programme began, except for advice about browser connectivity. All participants received approximately 25 automatic email messages during the 11 week programme. These were triggered when they completed a lesson, to remind them to complete a lesson, or to inform them about materials that had become available."

Intervention description: Six online lessons, illustrated story, education, graded exposure, cognitive restructuring, relapse prevention. Reflective questions about "costs and benefits of participating... values and how their symptoms affect attainment... reasons for participating, iv) how their life might be in five years if they didn't participate... personal strengths... goals for participating, ii) obstacles... techniques they would use to overcome obstacles... personal strengths."

Compliance: Motivation group: "40 participants (75%) completed all lessons (1 terminated at Lesson 1, 1 terminated at Lesson 4, 2 at Lesson 6, 4 at Lesson 7, 5 at Lesson 8)" No-contact group: 31 participants (56%) completed all lessons (1 terminated at Lesson 4, 1 at Lesson 5, 3 at Lesson 6, 5 at Lesson 7, 14 at Lesson 8)

Outcomes

Specific Symptoms: Social Interaction Anxiety Scale (SIAS)

Response: Social Interaction Anxiety Scale (SIAS)

Depression: Patient Health Questionnaire (PHQ-9)

Disability: Sheehan Disability Scale (SDS)

Notes

Funding: None acknowledged

Risk of bias

Titov 2010b (Continued)

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The 113 people accepted into the programme were randomized by NT via a true randomization process (www.random.org)..."
Allocation concealment (selection bias)	Low risk	Trial registration reports "Allocation concealment will occur by contacting the holder of the allocation schedule who is off-site."
Blinding of participants and personnel (performance bias) All outcomes	Low risk	"Participants did not receive any contact with the researchers once the programme began, except for advice about browser connectivity."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=4 (7.018%) <i>Comparison (other):</i> N=1 (1.79%) Method of analysis: LOCF Analyses include those "Eligible for analysis (started Lesson 1)" "Reasons for participants not completing lessons or questionnaires were not available. In accordance with the ITT and LOCF models, missing data was replaced with the last observed score on that measure."
Selective reporting (reporting bias)	Low risk	Trial registration number: ACTRN12609001082291 All outcomes reported.
Other bias	Low risk	N/A

Tolin 2007

Methods	Location: Hartford, CT, USA Recruitment: Referral Exclusion rate: 83.47% Randomised (N): 41 (20 intervention; 21 comparison)
Participants	Disorder: OCD; SCID (DSM-IV) Duration (years): 18.67 Age (years): 38.18 Sex (female): 36.59% Race (white): 56.1%

Tolin 2007 (Continued)

Medication (using at baseline): 56.1%

Education (years): 14.95

EXCLUSIONS

Depression: No

Psychotherapy: Current and previous therapy restricted

Medication: "only if they could demonstrate a current or previous adequate trial of an SRI medication...the medication had to be maintained within the dose range typically used for OCD (see Tolin et al., 2004, for specific dose ranges) for at least 10 weeks...we confirmed with patients that they subjectively experienced no more than minimal effects..." Excluded those who "made any changes to psychiatric medications within 2 months of entering the study."

Drug/Alcohol: "met criteria for a substance use disorder within the past 3 months"

Physical: N/A

Interventions

Comparison group(s): Individual CBT

Comparison description: "Treatment was provided by an experienced doctoral-level psychologist or postdoctoral fellow. ERP was based on a published manual (Foa and Kozak, 1997b)... Patients received 15 sessions, delivered twice weekly over 7 1/2 weeks. ERP treatment consisted of the following components: Information gathering and treatment planning...Exposure and response prevention...Relapse prevention..." Other CBT interventions allowed as needed.

Intervention name: Stop Obsessing! (Foa and Wilson, 2001).

Days of treatment: 42

Author wrote: No

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "following the pretreatment independent evaluation, patients in the self-administered ERP condition met with a study clinician. The clinician provided the patient with a brief overview of this treatment condition, including the instruction that they would be expected to conduct ERP exercises on their own...no further planned contact with the therapist for the next 6 weeks, although they were encouraged to contact the therapist in case of emergency or acute worsening of symptoms."

Intervention description: ERP: "the book begins with a general orientation, psychoeducation, and a motivational discussion; next, rationale and instructions for ERP are provided; finally (and for the bulk of the program), patients are led through specific ERP exercises aimed to reduce obsessive thoughts and compulsive behaviors. Patients were instructed to read the book and follow its instructions over a 6-week period, and were given a written schedule of suggested chapters to read during each of the 6 weeks."

Tolin 2007 (Continued)

Compliance: “At the end of the 6-week period, patients met again with the therapist to review their experiences with self-administered ERP. The therapist queried the patient about whether they read the entire book, the frequency and duration of exposure exercises, and the degree of abstinence from compulsive behaviors.”

Outcomes	Specific Symptoms: Clinical Global Impression (CGI): Severity General Symptoms: State-Trait Anxiety Inventory (STAI): Trait Response: Clinical Global Impression (CGI): Improvement; Yale-Brown Obsessive-Compulsive Scale (Y-BOCS) Depression: Beck Depression Inventory (BDI); Hamilton Depression Rating Scale, clinician-rated (HAM-D) Disability: Sheehan Disability Scale (SDS) Quality of Life: Quality of Life Inventory (QOLI)	
Notes	Funding: Patrick and Catherine Weldon Donaghue Medical Research Foundation Unpublished data: Author provided demographic details and outcome data. While the published data are analysed using ITT, the data provided are for available cases. The ITT data were used for analysis.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “Patients were then randomly assigned to receive either therapist-administered ERP or self-administered ERP according to a computerized random allocation table.”
Allocation concealment (selection bias)	Low risk	“Administered by a research assistant and not available to other members of the research team.” Assessment conducted by a blind rater. (personal communication)
Blinding of participants and personnel (performance bias) All outcomes	High risk	Control group "received 15 sessions, delivered twice weekly over 7.5 weeks. ERP treatment consisted of the following components"
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) the therapists, who were not blind to treatment condition or to treatment outcome, assigned effort ratings. "independent evaluator who was unaware of treatment condition conducted all clinical assessments at pretreatment, post-treatment and 10, 3-, and 6-month follow-up"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (15%) <i>Comparison:</i> N=4 (19.05%) Method of analysis: Available Case

Tolin 2007 (Continued)

“Participants who changed their medication (contrary to instructions) were included in the analyses. One participant went to the emergency room with 'hyperventilation syndrome'. The participant was discontinued from the study and last observations were carried forward. Most outcomes are not reported in the published paper, which reports data using LOCF. Data from the author are available cases; ITT analyses used when both were available (YBOCS).”		
Selective reporting (reporting bias)	Low risk	Trial registration number: NCT00316355 Not all outcomes included in paper; unpublished data received from the authors, though analysed using available cases.
Other bias	Low risk	N/A

Tolin 2011

Methods	Location: Connecticut, USA Recruitment: Unclear Exclusion rate: 85.65% Randomised (N): 34 (19 intervention; 15 comparison)
Participants	Disorder: OCD; ADIS (DSM-IV) Duration (years): N/A Age (years): 33.91 Sex (female): 58.82% Race (white): 47.06% Medication (using at baseline): 47.06% Education (years): N/A EXCLUSIONS Depression: Yes Psychotherapy: Current and previous therapy restricted Medication: "Participants who were taking psychiatric medications were stable on medication for at least 1 month and agreed to maintain their current type and dose of medication, with their physician's approval." Drug/Alcohol: "substance abuse/dependence" Physical: N/A
Interventions	Comparison group(s): Individual CBT Comparison description: "...17 outpatient treatment sessions lasting 90–120 min each. Treatment began with information gathering and treatment planning, during which the patient's symptoms were assessed in detail and an exposure hierarchy was created. ERP was then provided twice weekly... consisted of graded exposure to the items on the hierarchy, combined with strict abstinence from compulsive behavior. In the final session, relapse prevention was reviewed..."

Tolin 2011 (Continued)

Intervention name: Stop Obsessing! (Foa and Wilson, 2001).

Days of treatment: 42

Author wrote: No

Type of media: Book / Booklet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=3 (105 min)

Contact description: “During step 1, participants also met with the therapist three times (one 30–40 min visit every 2 weeks). The therapist’s function was to provide support and examine sources of life stress. In addition, the therapist used motivational interviewing techniques to address ambivalence about change. The therapist answered questions regarding ERP, and provided suggestions for implementing ERP; however, no ERP was performed or modeled within these sessions.”

Intervention description: “...three sections: Understanding Your Problem, The Initial Self-Help Program, and The Intensive Three-Week Program (which focuses on ERP). Based on our experience in the open trials, we organized the reading schedule to emphasize self-administered ERP.”

Compliance: N/A

Outcomes	None included.
Notes	Funding: National Institute of Mental Health. Grant R34 MH071464

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear “randomized to receive stepped care or standard ERP”
Allocation concealment (selection bias)	Low risk	“Patients meeting study inclusion criteria were then referred to a study therapist, who informed the patient of their randomly assigned treatment condition.”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) “During step 1, participants also met with the therapist three times (one 30–40 min visit every 2 weeks). The therapist’s function was to provide support...”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes included in this review. (See Selective Outcome Reporting.)
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%)

Tolin 2011 (Continued)

Method of analysis: Mixed effects

Only data on clinical response were analysed.

Selective reporting (reporting bias)	High risk	No trial registration number given. Data are presented after "Step 2" when participants receiving self-help had received face-to-face therapy if they did not improve. For that time point, means and SDs are reported for the YBOCS but for none of the other outcome measures.
Other bias	Low risk	N/A

Tortella-Feliu 2011

Methods	Location: Mallorca, Spain Recruitment: Advertising/ Internet Exclusion rate: 30.23% Randomised (N): 60 (21 intervention; 20 comparison)
Participants	Disorder: Specific Phobia; ADIS (DSM-IV) Duration (years): 3.97 Age (years): 37.1 Sex (female): 60.98% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "taking anxiolytic or antidepressive medication" Drug/Alcohol: N/A Physical: "cardiovascular or respiratory illness, and current pregnancy"
Interventions	Comparison group(s): Self-help (Exposure) Comparison description: "...buying a ticket, packing at home, going to the airport and boarding, taking off, flying, landing, and hearing news about an aircraft accident. At the end of each sequence, the program requested fear ratings; based on the replies, the exposure continued with the same sequence or proceeded to the next sequence in the hierarchy...Therapists attended all CAE-T sessions...If patients asked about the treatment process, they were invited to utilize the program's online help system."

Tortella-Feliu 2011 (Continued)

Intervention name: Not named

Days of treatment: 21

Author wrote: Yes

Type of media: Computer

Type of therapy: Exposure

Location used: Clinic/ Surgery

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=2 (30min)

Contact description: The first exposure trials were begun with a therapist present, to make sure the patients had completely understood the instructions. "At the last session, each patient had a personal 20-min meeting with a therapist."

Intervention description: "essentially self-administered ...The first exposure trials were begun with a therapist present, to make sure the patients had completely understood the instructions. Then, the therapist left the clinic while the user was exposed to images and sounds. In the following sessions, the user simply asked the receptionist for access to the session room. At the last session, each patient had a personal 20-min meeting with a therapist."

Compliance: "The mean number of sessions for each treatment was M = 4.70 (SD = 1.34) in the CAE-T [control], M = 4.10 (SD = 1.51) in the CAE-SA [intervention]..."

Outcomes	Specific Symptoms: Anxiety Disorders Interview Schedule (ADIS-IV): Specific Phobia, Avoidance Behaviour Test (Level): Flights Taken Response: Fear of Flying Questionnaire (FFQ) Recovery: No longer met criteria for diagnosis
Notes	Funding: Grants SEJ2005- 00899/PSIC, SEJ-2006-14301/PSIC, PROMETEO/2008/157, and CIBER Fisiopatología de la Obesidad y Nutrición—ISCIII CB06/03/0052 from the Spanish Government.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Unclear risk	Method: Unclear "Suitable candidates for treatment (n = 60) were randomized to one of the three treatment conditions"
Allocation concealment (selection bias)	Unclear risk	"All the assessments were carried out by independent evaluators who were not involved in the randomization process."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "The first exposure trials were begun with a therapist present..."
Blinding of outcome assessment (detection bias)	Low risk	Included assessor-rated outcomes (blind)

Tortella-Feliu 2011 (Continued)

All outcomes		“independent assessors rated, both at baseline and at posttreatment...”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison (other):</i> N=0 (0%) Method of analysis: LOCF “Analyses included an intent-to-treat (ITT) using the last value carried forward (LVCF) method. Missing values at posttreatment and/or at 1-year follow-up were imputed with the participant’s pretreatment score.”
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Tyrer 1988

Methods	Location: Nottingham, England Recruitment: Referral Exclusion rate: 5.41% Randomised (N): 154 (42 intervention; 28, 84 comparison)
Participants	Disorder: Mixed (9% had GAD or PD); SCID (DSM III) Duration (years): Unknown Age (years): 35 Sex (female): 69.05% Race (white): 0% Medication (using at baseline): 0% Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: No restrictions reported Medication: "They were currently receiving no psychotropic drugs (and had not been taking benzodiazepines, neuroleptic drugs, or antidepressants in regular dosage for at least 4 weeks before entry to the study)" Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): Pill placebo, Individual CBT

Tyrer 1988 (Continued)

Comparison description: 1) Pill placebo. 2) "Patients receiving cognitive and behaviour therapy had five one-hour treatments at 0, 1, 2, 4, and 6 weeks after randomisation"

Intervention name: Not named

Days of treatment: 42

Author wrote: Yes

Type of media: Audio

Type of therapy: Muscle Relaxation

Location used: Anywhere

Type of contact: Face-to-face

Automated contact: N=0

Provider contact: N=5 (75min)

Contact description: "saw the same team of community nurses at the same treatment intervals, but on each occasion were seen for 15 minutes only" At weeks 0, 1, 2, 4 and 6 for 15 minutes (5 sessions).

Intervention description: "given a relaxation tape without any specific training, and given a list of self-help organisations and groups for people with anxiety and depression. Patients in this group were told initially that drugs were potentially dangerous because of dependence and that individual psychotherapy could lead to similar dependence on a therapist, therefore it was wise to avoid these where possible and to work out one's own treatment programme...completed after these five sessions."

Compliance: Pill counts for the medication group.

Outcomes	None included.
Notes	Funding: Trent Regional Health Authority, Jessie Spencer Trust, Boots Pure Drug Company (which provided diazepam, dothiepin and placebo capsules). Unpublished data: Emailed Tyrer for further information, but the author did not reply.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Unclear "A randomised parallel-group design" (See "Allocation Concealment")
Allocation concealment (selection bias)	Low risk	"The randomised treatments were indicated by opening a sealed envelope once patients satisfied the inclusion criteria for the study."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "seen at weeks 0, 1, 2, 4 and 6 for 15 minutes."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) "All trained psychiatrists were masked with respect to treatment and diagnosis"

Tyrer 1988 (Continued)

Incomplete outcome data (attrition bias)
All outcomes

Unclear risk

Not analysed at post-treatment

Intervention: N=4 (9.52%)

Comparison (pill placebo): N=2 (7.14%)

Comparison (individual CBT): N=7 (8.33%)

Method of analysis: Available Case

18 dropped-out, though the two-year follow-up paper lists only 2 dropouts in Table 2 (1 from the drug/placebo group; 1 CBT). 4 patients had no baseline ratings and 5 provided only baseline measures. Assumed equal dropout between groups. "The 17 missing ratings were estimated statistically by equating residuals to zero in a repeated measures ANOVA and the residual and to degrees of freedom reduced by 17"

Selective reporting (reporting bias)

High risk

No trial registration number given.

"The main outcomes are not reported in a manner that can be extracted for meta-analysis."

Other bias

Low risk

N/A

van Boeijen 2005a

Methods

Location: Rural and urban areas, Holland

Recruitment: Referral

Exclusion rate: 46.34%

Randomised (N): 154 (59 intervention; 28, 67 comparison)

Participants

Disorder: Mixed (GAD, PD); SCID (DSM-IV)

Duration (years): 8.75

Age (years): 38.8

Sex (female): 62.68%

Race (white): 28.87%

Medication (using at baseline): 28.87%

Education (years): 12.73

EXCLUSIONS

Depression: No

Psychotherapy: Current and previous therapy restricted

Medication: "use of antidepressants or the use of more than 30 mg oxazepam equivalents daily"

Drug/Alcohol: N/A

van Boeijen 2005a (Continued)

Physical: N/A

Interventions	Comparison group(s): Treatment-as-usual, Individual CBT Comparison description: 1) The Netherlands College of General Practitioners Anxiety Disorder Guidelines implemented by a GP. "GP was free to choose the number of sessions and the type of intervention...relaxation exercises or psychiatric treatment and to prescribe antidepressants or benzodiazepines." 2) "12 45-minute sessions. The participants practiced for 3 hours a week. They received a CBT handbook explaining the basic facts about anxiety and the rationale of CBT" Intervention name: Not named Days of treatment: 84 Author wrote: Yes Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=5 (100 min) Contact description: "Self-help treatment consisted of five 20-minute sessions. In the first session, the GP informed the participant about the contents of the manual, which comprised an explanation of the basic facts about anxiety, simple cognitive techniques, and relaxation and in vivo exposure exercises. The participants put the self-help techniques learnt from the manual into practice for 3 hours a week. In subsequent sessions, the GP reinforced the patient's achievements and motivated him or her to continue with the use of the manual." Intervention description: "12 weeks based on cognitive-behavioural principles." Compliance: "the GP reinforced the patient's achievements and motivated him or her to continue with the use of the manual."	
Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) General Symptoms: Lehrer Woolfolk Anxiety Symptom Questionnaire (LWASQ); State-Trait Anxiety Inventory (STAI) Depression: Beck Depression Inventory (BDI) Disability: Sheehan Disability Scale (SDS)	
Notes	Funding: Nederlandse Organisatie voor Wetenschappelijk Onderzoek Unpublished data: van Boeijen provided data about participant characteristics and treatment provided by GPs.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "The participating practices (n=46) were first randomly assigned in a ratio of 1:1 to 'manual' or 'guidelines' as a primary care method. Next, within each

van Boeijen 2005a (Continued)

practice, patients were randomly assigned to primary or secondary care in a ratio of 2:1. For both randomisations a computerised randomisation scheme was used. Randomisation ratio changed halfway through the trial."		
Allocation concealment (selection bias)	Low risk	"In both randomisation procedures (primary care practices and participants) the assignments were put in sequentially numbered, sealed, opaque envelopes by an independent statistician. At the start of the study the first author opened the envelopes for the participating practices. During the study the first author opened the envelopes after participants had given their informed consent."
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "Self-help treatment consisted of five 20-minute sessions."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention</i> : N=0 (0%) <i>Comparison (treatment-as-usual)</i> : N=2 (7.14%) <i>Comparison (individual CBT)</i> : N=4 (5.97%) Method of analysis : LOCF "no significant differences were found between dropouts and completers at pre-test" 12 drop-outs in self-help group at post-test, 13 drop-outs in CBT treatment at post-test, 4 drop-outs in TaU group at post-test. 24 dropouts in self-help group at follow-up, 14 drop-outs in CBT at follow-up, 7 drop-outs in TaU at follow-up."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	High risk	GPs felt unable to conduct CBT for TaU condition. "practices randomly assigned to 'manual' treatment had on average twice as many patients as those assigned to 'guidelines'. As a consequence, there was an unintended difference in size between the three study groups. Having consulted an independent statistician, the authors decided halfway through the study to change the primary care: secondary care randomisation ratio in the practices offering 'manual'"

Varley 2011

Methods	Location : Sheffield, England Recruitment : Advertising/ Internet Exclusion rate : 98.95% Randomised (N) : 262 (176 intervention; 86 comparison)
---------	---

Varley 2011 (Continued)

Participants	Disorder: Mixed (not specified); No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 29.65 Sex (female): 72.19% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: No Psychotherapy: Current restricted, no previous restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: N/A
Interventions	Comparison group(s): No-Treatment Comparison description: "participants in the control condition did not receive any intervention" Intervention name: Feeling Less Worried: A Three-Step Plan to Help You Manage Your Anxiety Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: Relaxation Location used: Anywhere Type of contact: None Automated contact: N=0 Provider contact: N=0 (0 min) Contact description: "following the baseline questionnaire, participants downloaded the booklets on-line. The next contact that they received from the research team was eight-weeks later when they received an email request to complete a follow-up questionnaire." (personal communication) Intervention description: "eight pages long and included psychoeducation...diary sheets...two relaxation techniques...a short breathing exercise that could be undertaken whenever it was needed...a longer, deep muscle relaxation..." "participants in the augmented self-help condition received the same booklet but also additional materials that asked them to form an implementation intention specifying when they would use the techniques...taken to a Web page where they could download..." Compliance: Frequency of strategy use was measured by three items (5-point scales ranging from never to once or more a day) Standard=3.02, Augmented 3.65.
Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety

Varley 2011 (Continued)

Depression: Hospital Anxiety and Depression Scale (HADS): Depression

Notes

Funding: ESRC. "The contributions of Thomas L. Webb and Paschal Sheeran were supported by ESRC grant RES-060-25-0044: Emotion regulation of others and self (EROS)."

Unpublished data: "Conducted as part of a DCLinPsy student's (Rachel Varley) thesis work."

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online "randomly allocated via an html program to one of three conditions (participants were unaware of the randomization procedure)"
Allocation concealment (selection bias)	Low risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	Low risk	No provider contact.
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=5 (2.84%) <i>Comparison:</i> N=8 (9.30%) Method of analysis: Available Case See Figure 1. 249 of 262 (95%) analysed
Selective reporting (reporting bias)	Unclear risk	"No, we did not register the trial." (personal communication)
Other bias	Low risk	N/A

Walker 2001

Methods

Location: Winnipeg, Canada

Recruitment: Advertising/ Internet

Exclusion rate: Unknown

Randomised (N): 128 (32 *intervention*; 32, 32 *comparison*)

Participants

Disorder: Social Anxiety (General); SCID (DSM-IV)

Duration (years): Unknown

Walker 2001 (Continued)

Age (years): 37.73

Sex (female): 60.27%

Race (white): Unknown

Medication (using at baseline): Unknown

Education (years): 13.84

EXCLUSIONS

Depression: Yes

Psychotherapy: No restrictions reported

Medication: "Individuals who were taking psychiatric medication (most often low dose benzodiazepines or SSRI antidepressants) were allowed to participate providing that they had been on a stable dose of medication for the previous 3 months and that the symptoms were sufficient in severity..."

Drug/Alcohol: "substance abuse"

Physical: N/A

Interventions

Comparison group(s): Wait list, Group CBT

Comparison description: 1) Wait-list. 2) Group therapy: participants attended weekly group meetings lead by a psychologist or a senior psychology graduate student experienced in CBT of anxiety disorders. Participants received the self-help materials.

Intervention name: Dying of Embarrassment (Markway, Carmin, Pollard, and Flynn, 1992). Audiotape "Letting Go of Stress".

Days of treatment: 71

Author wrote: No

Type of media: Mixed

Type of therapy: CBT

Location used: Anywhere

Type of contact: None

Automated contact: N=0

Provider contact: N=0 (0 min)

Contact description: "Individuals who called in response to the advertisement were screened over the telephone. Those who seemed likely after the screening...were seen for a SCID with clinician." No therapist contact during the thirteen week treatment period in self-administered treatment condition.

Intervention description: Book CBT. Workbook emphasised putting reading into practice. All groups used the same book, workbook and relaxation tape. The group therapy conditions used the same therapist manual. Treatment duration unclear (described as 12, 13 and 14 weeks on the same page).

Compliance: Weekly time logs were returned by post. The author reports that some participants completed few assignments "preferring to wait for the opportunity to participate in the group program which came at the end of the study."

Walker 2001 (Continued)

Outcomes	Response: Clinical Global Impression (CGI): Improvement	
Notes	Funding: Authors report that the program was developed with the Anxiety Disorders Association of Manitoba, a consumer-run self-help association, and they acknowledge the Medical Research Council. Unpublished data: Author provided details about randomisation process and sent presentations with outcome data.	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical Generated and administered by drawing names from a hat (personal communication). "Individuals accepted in the study were accumulated into blocks of eight. When a block has been assembled, participants in that block were randomly assigned to one of four treatment conditions."
Allocation concealment (selection bias)	High risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	Low risk	“Participants used the treatment program in a self-administered format with no therapist contact during the thirteen week treatment period.”
Blinding of outcome assessment (detection bias) All outcomes	Low risk	Included assessor-rated outcomes (blind) “independent assessor remained blind to the group assignment of the participants”
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (9.375%) <i>Comparison (wait-list):</i> N=3 (9.375%) <i>Comparison (group CBT):</i> N=4 (12.5%) Method of analysis: Available Case “a number of respondents (less than 10%) were lost to follow up when they dropped out of the study”
Selective reporting (reporting bias)	High risk	No trial registration number given. Clinical global impression scores are reported as a bar graph and could not be extracted. For continuous measures, variance is missing and most data could not be included in meta-analysis for: Social Anxiety and Distress Scale (SADS), Social Phobia Scale (SPS), Social Phobia Anxiety Inventory (SPAI), Social Interaction Anxiety Scale (SIAS), Fear of Negative Evaluation Scale (FNE), Fear Questionnaire (FQ): Social Phobia, Clinical Global Impression (CGI): Severity.
Other bias	Unclear risk	N/A

White 1995

Methods	Location: Glasgow, UK, Scotland Recruitment: Referral Exclusion rate: 36.08% Randomised (N): 62 (21 intervention; 20 comparison)
Participants	Disorder: Mixed (any anxiety disorder); ADIS (DSM-III R) Duration (years): 3 Age (years): 38.95 Sex (female): 58.54% Race (white): 58.54% Medication (using at baseline): 58.54% Education (years): 12.3 EXCLUSIONS Depression: No Psychotherapy: Current and previous therapy restricted Medication: "no recent change in psychotropic medication" Drug/Alcohol: "no evidence of...alcohol or drug problems" Physical: N/A
Interventions	Comparison group(s): Attention/Monitoring Comparison description: Advice Only: "The procedure for AO was the same as for SP, except that in the final 30 minutes patients were offered specific verbal advice on ways of coping, while on the waiting list, with specific problems elicited during assessment e.g. noting the importance of exposure, the value of setting aside time to relax, the value of challenging negative thinking etc. No written or taped material was offered. As with SP patients, they were given an appointment time in three months." Intervention name: Stresspac Days of treatment: 91 Author wrote: Yes Type of media: Book / Booklet Type of therapy: CBT Location used: Anywhere Type of contact: Face-to-face Automated contact: N=0 Provider contact: N=2 (30min) Contact description: All patients were given a two hour initial interview. This principally involved the ADIS-R assessment. The Hamilton Scales were omitted. This took, on average, 90 minutes. The remain-

White 1995 (Continued)

ing 30 minutes varied across conditions (see below). "Following ADIS-R, 30 minutes were given to describing Stresspac and how to use it."

Intervention description: "...79 page booklet, a four page introduction and a double-sided progressive relaxation tape ('Deep' and 'Rapid')...'Controlling your body'...'Controlling your thoughts'...'Controlling your actions' which emphasizes the importance of exposure and offers other behavioural advice. 'Controlling your future' (relapse prevention) ends the treatment section. Flesch reading formula scores (Flesch, 1948) for the booklet are 73 ('fairly easy')."

Compliance: Not reported.

Outcomes	Specific Symptoms: Anxiety Disorders Interview Schedule (ADIS-IV): Global Severity General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety Response: Global Improvement Depression: Hospital Anxiety and Depression Scale (HADS): Depression
Notes	Funding: Lanarkshire Health Board Unpublished data: See "Sequence Generation"

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	High risk	Method: Unclear "The remaining 62 patients were randomly allocated" Author says it was "alternating" (personal communication)
Allocation concealment (selection bias)	Unclear risk	N/A
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "author carried out all assessments and treatments"
Blinding of outcome assessment (detection bias) All outcomes	High risk	Included assessor-rated outcomes (NOT blind) "author carried out all assessments and treatments"
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=0 (0%) <i>Comparison:</i> N=0 (0%) Method of analysis: Imputed "There was very little missing data (<5%). Where necessary, a mean score was substituted (Winer, 1962)."
Selective reporting (reporting bias)	Unclear risk	No trial registration number given.
Other bias	Low risk	N/A

Wims 2010

Methods	Location: Sydney, Australia Recruitment: Advertising/ Internet Exclusion rate: 42.16% Randomised (N): 59 (32 <i>intervention</i>; 27<i>comparison</i>)
Participants	Disorder: Panic disorder; MINI (DSM-IV) Duration (years): Unknown Age (years): 42.08 Sex (female): 75.93% Race (white): 38.89% Medication (using at baseline): 38.89% Education (years): 13.63 EXCLUSIONS Depression: Yes Psychotherapy: Current restricted, no previous restrictions reported Medication: "if taking medication, had been taking the same dose for at least 1 month and did not intend to change that dose during the course of the program; (viii) and were not taking benzodiazepines." Drug/Alcohol: "not using illicit drugs or consuming more than three standard drinks/day" Physical:N/A
Interventions	Comparison group(s): Wait-List Intervention name: The Panic Program Days of treatment: 56 Author wrote: Yes Type of media: Internet Type of therapy: CBT Location used: Anywhere Type of contact: Email Automated contact: N=41 Provider contact: N=7.5 (75min) Contact description: "Participants who had not logged into the program for 2 weeks and had not returned emails were telephoned... "The mean therapist time per participant was 75 minutes including monitoring of the discussion forum and feedback. An additional average 60 minutes per patient was required for administrative purposes....A review of email and forum contact showed that a total of 218

Wims 2010 (Continued)

emails were sent to treatment group participants (mean=7.5 emails per participant), with 41 forum postings made to the entire group by the therapist."

Intervention description: "...four components: six online lessons, homework assignments...illustrated story about a woman with panic and agoraphobia who, with the help of a therapist, learn show to gain mastery over her symptoms...psychoeducation, graded exposure, cognitive restructuring, physiological de-arousal, and relapse prevention." "...complete the first three lessons within a week...Thereafter they were given access to the following lessons at fortnightly intervals"

Compliance: "Twenty-three members of the treatment group (79%) completed all six lessons within the required time frame."

Outcomes	Specific Symptoms: Agoraphobic Cognitions Questionnaire (ACQ) Response: Panic Disorder Severity Scale (PDSS) Recovery: No longer met criteria for diagnosis Depression: Patient Health Questionnaire (PHQ-9) Disability: Sheehan Disability Scale (SDS)	
Notes	Funding: None acknowledged	
Risk of bias		
Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Computer/Online “The 59 people accepted into the program were randomized via a true randomization process (www.random.org)”
Allocation concealment (selection bias)	Low risk	See “Sequence Generation”
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "One psychiatry registrar (EW) provided all clinical contact with participants. A research assistant provided administrative support to collate data." "After completing each lesson participants were emailed by the therapist...reinforcement for continued participation and efforts, encouragement to practice..."
Blinding of outcome assessment (detection bias) All outcomes	Unclear risk	Included assessor-rated outcomes (blinding unclear) “Participants were also telephoned by the clinician who administered the Panic Disorder Severity Scale, which also provided estimates of the frequency of panic attacks."
Incomplete outcome data (attrition bias) All outcomes	Low risk	Not analysed at post-treatment <i>Intervention:</i> N=3 (9.38%) <i>Comparison:</i> N=2 (7.41%) Method of analysis: LOCF In accordance with the intention-to-treat paradigm, the pre-treatment scores of the participants who did not complete the post-treatment questionnaires were replicated as their post-treatment scores. Participants who dropped out before lesson 1 and before completing pre-treatment questionnaires were ex-

Wims 2010 (Continued)

cluded. "Two participants found the program required more time than they were able to set aside, one person dropped out due to increased severity of their anxiety which required inpatient admission, another became ill, another found the course too difficult and the final participant moved house during the program and no longer had internet access."

Selective reporting (reporting bias)	Low risk	Trial registration number: ACTRN12608000548336 All outcomes reported.
Other bias	Low risk	N/A

Zetterqvist 2003

Methods	Location: Uppsala, Sweden Recruitment: Advertising/ Internet Exclusion rate: 39.02% Randomised (N): 100 (50 intervention; 50 comparison)
Participants	Disorder: Mixed (mean HADS Anxiety score in the moderate to severe range (i.e. mean >13).); No diagnostic interview (questionnaire only) Duration (years): Unknown Age (years): 39.17 Sex (female): 61.9% Race (white): Unknown Medication (using at baseline): Unknown Education (years): Unknown EXCLUSIONS Depression: Yes Psychotherapy: No restrictions reported Medication: N/A Drug/Alcohol: N/A Physical: "if they perceived that they had any obstacles that prevented them from completing the study (including medical, social and psychological reasons)"
Interventions	Comparison group(s): Wait-List Intervention name: Not named (Similar to interventions by Andersson and Carlbring) Days of treatment: 91 Author wrote: Yes

Zetterqvist 2003 (Continued)

Type of media: Internet

Type of therapy: CBT

Location used: Anywhere

Type of contact: Email

Automated contact: N=0

Provider contact: N=6 (duration unknown)

Contact description: "Participants registered the frequency and duration of practising relaxation in a diary and results were reported once each week. The rationale for this registration was mainly to keep regular contact with the participants, thereby promoting compliance. Duration of contact with participants not measured (personal communication), if two weeks passed without a report, participants were contacted by the researchers by email."

Intervention description: "6 treatment modules were each divided into the 3 sections: relaxation, additional exercises and information...problem-solving module presented step-by-step instructions with illustrating examples...time management exercise...cognitive and behavioural restructuring exercise...Techniques for analysing and changing negative automatic thoughts and stress enhancing behaviours were also presented..." "The actual treatment for the self-help group started in late April 2000 and ended in June/July"

Compliance: Participants registered the frequency and duration of practising relaxation in a diary and results were reported once each week. The rationale for this registration was mainly to keep regular contact with the participants, thereby promoting compliance. "At the time of the post-treatment data collection the self-help group had completed an average of 4.2 out of the 6 treatment modules. Twelve participants completed all homework assignments and treatment modules."

Outcomes	General Symptoms: Hospital Anxiety and Depression Scale (HADS): Anxiety Depression: Hospital Anxiety and Depression Scale (HADS): Depression
Notes	Funding: Swedish Council for Social Research Unpublished data: Authors provided details about randomisation and concealment. Unable to provide baseline data or outcome data.

Risk of bias

Bias	Authors' judgement	Support for judgement
Random sequence generation (selection bias)	Low risk	Method: Mechanical "assigned randomly to one of two conditions" Participants were assigned by coin toss (personal communication).
Allocation concealment (selection bias)	High risk	See "Sequence Generation"
Blinding of participants and personnel (performance bias) All outcomes	High risk	Included contact with researcher/clinician (not blind) "They were given the option to use a coded e-mail identity for contact with the project leaders. None of the participants used this alternative. All contact with the participants was conducted through e-mail."
Blinding of outcome assessment (detection bias) All outcomes	Low risk	No assessor-rated outcomes.

Zetterqvist 2003 (Continued)

Incomplete outcome data (attrition bias)
All outcomes

High risk

Not analysed at post-treatment

Intervention: N=27 (54%)

Comparison: N=10 (20%)

Method of analysis: Available Case

“3 participants in the control group and 12 in the self-help group declared that they were no longer interested and withdrew before the study proper had started. Hence, the actual number of participants was 85, who by then already had been randomized to each respective condition. Of the participants, 26% (15 in the treatment group and 7 in the control group) dropped out at various stages without providing follow-up data, leaving a final sample of 63 participants (23 in the self-help condition and 40 in the waiting list condition)” Authors report that an ITT analysis was not significant for the perceived stress scale, but data presented include 63 of 85 participants.

Selective reporting (reporting bias)

Unclear risk

No trial registration number given.

Other bias

Low risk

N/A

Characteristics of excluded studies [ordered by study ID]

Study	Reason for exclusion
Allen 1973	Participants not clinically anxious (test anxiety)
Altmaier 1981	Participants not clinically anxious (test anxiety)
Andersson 2006	No eligible comparison (contact varied). The intervention included two 3-hour group exposure sessions that were not common to both groups.
Arpin-Cribbie 2008	Participants not clinically anxious (perfectionism)
Ayres 1993	Participants not clinically anxious (public speaking)
Bennett 1985	Not BT/CBT (educational only)
Berger 2011	No eligible comparison (contact varied)
Billings 2008	Participants not clinically anxious (stress)
Boettcher 2012	Not BT/CBT (attention training)
Bornas 2001	Not stand-alone (virtual reality)
Botella 1999	No eligible comparison (contact varied)
Carpenter 1994	Participants not clinically anxious (dental anxiety)
Chiauszi 2008	Participants not clinically anxious (stress)

Study	Reason for exclusion
Coldwell 1998	Participants not clinically anxious (dental anxiety)
Cronin 1994	Participants not clinically anxious (public speaking)
Crowder 1970	Not BT/CBT (book about snakes)
Currie 2010	Not a RCT
Dannon 2002	Not BT/CBT (educational only)
den Boer 2007	Not stand-alone (self-help groups)
Devine 1973	No eligible comparison (preferred versus not preferred therapy)
Dolezal-Wood 1998	Not stand-alone (computer assisted therapy)
Donner 1969	Participants not clinically anxious (test anxiety)
Furmark 2009	No eligible comparison (contact varied)
Gatchel 1986	Participants not clinically anxious (dental anxiety)
Gross 1979	Participants not clinically anxious (specific fear). "undergraduated served as Ss. All were enrolled in the introductory psychology classes and were given credit for participating in the study. Ss were selected on the basis of their behavioral responses in the presence of five live rats."
Grossman 1991	Participants not clinically anxious (dating anxiety)
Gruber 2001	No eligible comparison (contact varied)
Halpern 1995	Participants not clinically anxious (college volunteers)
Hamberger 1986	Participants not clinically anxious (public speaking)
Hasson 2005	Participants not clinically anxious (stress)
Hasson 2010	Participants not clinically anxious (stress)
Hazen 2009	Not BT/CBT (attention training)
Heatherton 2004	Trial discontinued
Hecker 2004	No eligible comparison (contact varied)
Hetherton 2004	Trial discontinued
Hickie 2010	Participants not clinically anxious (psychological distress)
Hiebert 1983	Participants not clinically anxious (stress)
Hoek 2012	Participants <18 years
Hogan 1968	Not BT/CBT (book about snakes)
Hopf 1995	Not BT/CBT (for group leaders)

Study	Reason for exclusion
Israel 1977	Participants not clinically anxious (stress)
Jannoun 1982	No eligible comparison (follow-up times differ)
Jarmon 1972	Participants not clinically anxious (public speaking)
Jeffcoat 2012	Participants not clinically anxious
Johansson 2012	Not BT/CBT (psychodynamic)
Jones 2002b	Trial discontinued
Kahn 1968	Participants not clinically anxious. "students who were suffering from 'subclinical phobias'"
Kassinove 1980	Participants not clinically anxious (neurotic)
Kenwright 2005	No eligible comparison (contact varied)
Khanna 2010	Participants <18 years
Kiely 1984	Participants not clinically anxious (stress)
Kiely 1986	Participants not clinically anxious (stress)
Kilfedder 2010	Participants not clinically anxious (stress)
Kirsch 1979	Participants not clinically anxious (public speaking)
Klein 2009	No eligible comparison (contact varied)
Kohutsek 1983	Participants not clinically anxious (stress)
Kupshik 1999	No eligible comparison (contact varied)
Linder 1971	Participants not clinically anxious (specific fear, mice). "white, female, college freshmen and sophomores drawn from classes...partially fulfilled a course requirement for all Ss...randomly chosen from a large population of female students who did not touch a caged mouse..."
Lucock 2008	Not a RCT (alternating assignment)
Lucock 2011	No eligible comparison (contact varied)
Lynch 1997	Participants not clinically anxious (stress)
Mann 1972	Participants not clinically anxious (test anxiety)
Marchand 2007	No eligible comparison (contact varied)
Marshall 1976	Participants not clinically anxious (public speaking)
Mataix-Cols 2006	Not a RCT
Matthews 2010	Not a RCT
McGlynn 1976	Participants not clinically anxious (specific fear, snakes). "female college undergraduates who were selected from introductory psychology classes and whose participation partially fulfilled a course

Study	Reason for exclusion
	requirement. They were selected in two steps. First, a Preliminary Screening Questionnaire was administered during class to all students. Then females who reported fear of nonpoisonous snakes were invited to perform a Behavioral Avoidance Test. The 40 Ss were selected randomly from a population of female students who did not touch the snake..."
Moritz 2011	Participants not clinically anxious (trichotillomania). Currently classified as an impulse control disorder.
Muroff 2012	Participants not clinically anxious (hoarding). Hoarding was not included in DSM-IV and the study excluded "hoarding symptoms resulting from OCD symptoms".
Newman 1997	No eligible comparison (contact varied)
Ormrod 2010	Not a RCT
Pier 2008	No eligible comparison (contact varied)
Pittaway 2009	Not a RCT (alternating assignment)
Pless 2010	Participants not clinically anxious (test anxiety)
Pleva 2007	Participants not clinically anxious (perfectionism)
Powell 2012	Participants not clinically anxious (mental health promotion)
Rahe 2002	Participants not clinically anxious (stress)
Register 1991	Participants not clinically anxious (test anxiety)
Rishel 1993	Participants not clinically anxious (dental anxiety)
Ruwaard 2007	Not stand-alone (email therapy)
Saul 2007	Not stand-alone (internet chat)
Schelver 1983	Participants not clinically anxious (top 15% of undergraduates)
Schneider 1995	Participants not clinically anxious (stress)
Serrano 2009	Participants not clinically anxious (preventive or subthreshold)
Smith 1987	Participants not clinically anxious (stress)
Spotel 2013	Participants <18 years
Tillfors 2008	No eligible comparison (contact varied)
Titov 2009b	No eligible comparison (contact varied)
Titov 2009c	No eligible comparison (contact varied)
van Straten 2008	Participants not clinically anxious (no inclusion criteria)
Vestre 1989	Participants not clinically anxious (undergraduates, course credit)
Weissberg 1977	Participants not clinically anxious (public speaking)

Study	Reason for exclusion
Wolitzky-Taylor 2010	Not stand-alone (therapist led)
Wright 1997	Relapse prevention.
Zou 2012	Not a RCT

Characteristics of studies awaiting assessment *[ordered by study ID]*

[ACTRN12610001058066](#)

Methods	"Study type: interventional Allocation: randomized Endpoint: efficacy Assignment: parallel Masking: blinded Primary purpose: educational/counselling/training"
Participants	"Key inclusion criteria - Elevated symptoms of depression or anxiety as indicated by total scores on at least one of the following measures: PHQ-9 > 9; GAD-7 > 7; Mini SPIN > 5; ANSQ > 1. Scores above these cutoffs indicate likelihood of meeting diagnostic criteria for at least one of depression, generalized anxiety disorder, social phobia, panic disorder, respectively - Self-identifying as having difficulties with [either] depression, social phobia, panic disorder or generalized anxiety disorder. Note that meeting diagnostic criteria for a formal anxiety disorder or depression is NOT an inclusion [criterion] — instead, participants need to have elevated symptoms of anxiety or depression as indicated [in bullet just above] - No current participation in cognitive behavioural treatment - No change in medications in the period 1 month prior to this study, and no change in medications during the study - Access to the Internet and a printer 18 years to 64 years old - Resident of Australia - Prepared to provide name, phone number and address and to provide the name and address of a local general practitioner - Prepared to provide written informed consent Age: 18-64 years Gender: both males and females Key exclusion criteria - Severe depression (score of 23 or greater on PHQ-9) - Suicidal intent or plan (either reporting suicidal intent or responding > 2 to Question 9 [suicidal ideation] on the PHQ-9) - Scoring below cutoffs on the measures of anxiety and depression - Current substance abuse - Psychosis - Primary presenting problem not depression, generalized anxiety disorder, social phobia or panic disorder"

ACTRN12610001058066 (Continued)

Interventions	Group 1: Wait-list control (who will begin the programme 9 weeks after the other groups). This group will receive the same intervention as Group 3 Group 2: Self-guided Internet-based education programme for anxiety and depression Group 3: Self-guided Internet-based education programme for anxiety and depression + Automatic e-mails about new materials, upcoming lessons and reminders to complete lessons
Outcomes	Primary outcomes • Depression and Anxiety Stress Scales, 21 item (DASS-21) • Patient Health Questionnaire-9 (PHQ-9) • Generalized Anxiety Disorder-7 Item (GAD-7) Secondary outcomes • Penn State Worry Questionnaire (PSWQ) • Autonomic Nervous System Questionnaire (ANSQ) • MINI-Social Phobia Inventory (MINI-SPIN) • Sheehan Disability Scales (SDS) • NEO-Five Factor Inventory Neuroticism Scale (NEO-FFI-N) • Health status will be measured by the EQ-5D • Health service use will be measured by the Service Use Questionnaire
Notes	

ACTRN12611000116921

Methods	"Study type: interventional Allocation: randomized Endpoint(s): not listed Assignment: parallel Masking: open (masking not used) Primary purpose: educational/counselling/training"
Participants	"Key inclusion criteria: an Australian resident • Aged 18-79 years • Have access to a computer with Internet access • Primary diagnosis of obsessive compulsive disorder • YBOCS score greater than 16 • Fear-based obsessions Key exclusion criteria • Currently participating in a cognitive and behavioural therapy (CBT) intervention for obsessive compulsive disorder (OCD) • Current drug and/or alcohol abuse or dependence • History of or current psychosis/mania • Current suicidal plans or severe depression • Commenced or changed dose of medication over past 3 months

ACTRN12611000116921 (Continued)

	Primary hoarding concerns Suitable trial of CBT for OCD in the past (Exposure and Response Prevention (ERP) performed in a graded fashion of at least once a week with in-session exposure)"
Interventions	"Participants in the treatment groups will complete 5 lessons in 8 weeks of a guided program for OCD The Internet group will complete these lessons online, the bibliotherapy group will complete these lessons from a hard copy self-help workbook Both groups receive the same information The lessons for both groups take approximately 30 minutes to read through, with homework exercises [taking] a further hour per day The lessons are based on current best practice cognitive and behavioural techniques and include exposure and response prevention and cognitive techniques The participant is phoned twice weekly by a clinical psychologist (these days are determined by the participant) and may also be contacted at other times by the participant by e-mail or phone. A printable information summary will also be provided, and some homework assignments will be set Comparator: The wait-list control group receive[s] their choice of the two methods of administration (Internet or bibliotherapy) with weekly contact from a clinical psychologist once the other groups have completed the program (week 9)"
Outcomes	Primary outcomes • Yale Brown Obsessive Compulsive Disorder Scale (YBOCS) • Obsessive Compulsive Inventory (OCI) • Obsessive Beliefs Questionnaire (OBQ) Secondary outcomes • Sheehan Disability Scale (SDS) • Kessler-10 (K-10) • Patient Health Questionnaire (PHQ-9) • Generalised Anxiety Disorder-7 Item (GAD-7) • Treatment Expectancy Questionnaire • OCD Diagnosis via Structured Interview-Mini International Neuropsychiatric Interview (MINI)
Notes	Trial acronym: The OCD Program: RCT 1

ACTRN12611000625976

Methods	"Study type: interventional Allocation: randomized Endpoint: efficacy Assignment: parallel Masking: blinded Primary purpose: educational/counselling/training"
---------	--

ACTRN12611000625976 (Continued)

Participants	"Key inclusion criteria: meet <i>DSM-IV</i> criteria for social phobia and/or panic disorder (with or without agoraphobia) and/or generalised anxiety disorder— Internet access + printer access Australian citizen— males and females Minimum age: 18 years Maximum age: no limit Key exclusion criteria • Current substance abuse/dependence • Psychotic disorder • Current or planned psychological treatment during study duration • Change in medications during last 1 month or intended change during study duration • Use of benzodiazepines or beta-blockers"
Interventions	Group 1: immediate Internet education group (6 lessons; one lesson will be completed every 7 to 10 days) Group 2 (control): delayed Internet education group (no contact during the 10 weeks, then will be reassessed and then provided with Internet-based education for anxiety disorders) "The educational materials are based on the Shyness, Panic and Worry Program, which applies cognitive behavioural techniques Participants in both groups will receive weekly reminder telephone calls, and have access to a moderated online forum (the duration of phone calls and forum participation is expected to be 2-7 mins) Duration of the program is 9 weeks Participants will be contacted 3 months post program and asked to complete follow-up questionnaires. Study questionnaires will be administered at application, pre-education, post-education, and at 3-months post-education"
Outcomes	Primary outcomes • Mini International Psychiatric Interview • Beck Anxiety Inventory • Neuroticism is measured using the NEO Five Factor Inventory Secondary outcomes • Patient Health Questionnaire-9 (PHQ-9) • Sheehan Disability Scale • World Health Organisation Disability Assessment Schedule II (WHODAS II) • Social Interaction Anxiety Scale • Panic disorder is measured using the Body Vigilance Scale • Agoraphobic Questionnaire • Generalised Anxiety Disorder Questionnaire (GAD-7)
Notes	

ACTRN12611000626965

Methods	"Study type: interventional Allocation: randomized
---------	--

ACTRN12611000626965 (Continued)

	Endpoint: efficacy Assignment: parallel Masking: open (masking not used) Primary purpose: treatment"
Participants	"Key inclusion criteria: accepted for treatment for social phobia at the St Vincent's Anxiety Disorders Clinic Minimum age: 18 years Maximum age: 99 years Key exclusion criteria: Intake clinicians at the clinic do not offer treatment to people with comorbid psychosis, substance use disorder or severe personality disorder"
Interventions	Group 1: cognitive behaviour therapy delivered by the Internet Group 2 (control): cognitive behaviour therapy delivered in the clinic on a face-to-face basis Both groups receive six sessions/lessons of less than 50 minutes on a 7-10 day schedule following the content outlined in Andrews et al, Treatment of anxiety disorders, Cambridge University Press, 2003 (psychoeducation, de-arousal strategies, cognitive therapy, graded exposure)
Outcomes	"Primary outcome: reduction in social phobia as demonstrated from scores on Social Interaction Anxiety Scale (SIAS) and Social Phobia Scale (SPS) Secondary outcome: reduction in comorbid depression and anxiety as demonstrated from changes in the PHQ-9 and GAD-7 Timepoints: at 12 weeks after beginning treatment"
Notes	

Andersson 2011

Methods	"Study type: interventional Study design: allocation: randomized Endpoint classification: efficacy study Intervention model: parallel assignment Masking: open label Primary purpose: treatment"
Participants	"Inclusion criteria • Fulfill the diagnostic criteria for GAD according to <i>DSM-IV</i> • Have access to the Internet • Have good knowledge of the Swedish language Exclusion criteria • Recent (during last 6 weeks) change in psychiatric medication

Andersson 2011 (Continued)

- Presently in any other psychological treatment
- Severe depression
- Suicidal ideation"

Interventions	Active treatment: Internet-delivered cognitive behavior therapy, 8 weeks' treatment, guided self-help Active treatment: Internet-delivered psychodynamic therapy active treatment: Internet-delivered psychodynamic therapy, 8 weeks' treatment, guided self-help Wait-list condition, received treatment 3 months after initial treatment period
Outcomes	Penn State Worry Questionnaire (PSWQ) Montgomery Åsberg Depression Rating Scale-Self Rated (MADRS-S) Quality of Life Inventory (QOLI) Beck Depression Inventory (BDI) Beck Anxiety Inventory (BAI) Trimbos and iMTA Questionnaire on Costs Associated with Psychiatric illness (TIC-P) State Trait Anxiety Inventory (STAI) Generalized Anxiety Disorder Questionnaire IV (GAD-Q IV)
Notes	

Bohus 2010

Methods	Study type: interventional Study design: allocation: randomised Endpoint classification: efficacy study Intervention model: parallel assignment Masking: single blind (outcomes assessor) Primary purpose: treatment
Participants	"Inclusion criteria • Diagnosis of Social Phobia according to the criteria of <i>DSM-IV</i> • Duration of the disorder at least 1 year • Severity of symptomatology according to at least one of two self-report measures for social anxiety above cutoff: Social Interaction Anxiety Scale (SIAS: value of 26 or more) and/or Social Phobia Scale (SPS: value of 17 or more) • Age: 18-60 • Written informed consent Exclusion criteria • Social phobia is not the main diagnosis • Mental retardation

Bohus 2010 (Continued)

	• Lifetime diagnoses: schizophreniform disorder, bipolar affective disorder • Acute diagnoses: severe major depression episode, severe anorexia nervosa, substance-, drug - or alcohol-dependence, life-threatening suicidal crisis • Psychotropic medication (except stable treatment with antidepressant since 3 months before study start: medication and dosage has to be stable during the entire study duration)"
Interventions	"Active comparator: SCP stepped-care program with self-help module with minimal therapist contact as first step, followed by therapist-guided intervention depending on status of remission" "Active comparator: ST standard therapy (therapist-guided intervention only)"
Outcomes	Social anxiety (Self-ratings: SIAS, SPS, SPAI. Clinician rating: LSAS) Global severity of psychopathology (Brief Symptom Inventory; BSI) Depressive symptomatology (German Version of the CES-D) Cognitive, affective, behavioral and autonomic indicators of anxiety in social situations
Notes	

Christensen 2010

Methods	Study type: interventional Study design: multicentre randomised controlled trial Primary purpose: treatment Duration: 10 week treatment phase with 12 month follow-up (measures administered at screening, baseline, post-test and 6 and 12 months after post-test)
Participants	Inclusion criteria: • Aged 18-30 years, either sex • Access to the Internet • Score 5 or higher for GAD (as assessed through GAD-7 scale) • Consent to participate • Provide active e-mail address and phone number • Sufficient English language literacy
Interventions	Treatment trial Group 1: E-couch Website (which covers psychoeducation; cognitive behaviour therapy; relaxation; and physical activity) Group 2: sertraline (25 mg-50 mg/d) Group 3: attention control All conditions will also involve four face-to-face clinician monitoring sessions
Outcomes	Primary outcome measure: Generalised Anxiety Disorder Questionnaire (GAD-7)

Christensen 2010 (Continued)

Notes	There are two separate studies within one trial registration [ISRCTN76298775]. The treatment trial will use participants for whom a diagnosis of GAD has been confirmed, whereas the prevention trial will involve participants who score at a subclinical yet elevated level for GAD
-------	---

Donker 2009

Methods	"Randomised controlled trial"
Participants	"500 subjects (18 year and older) from the general population with mild to moderate depression and/or anxiety"
Interventions	"(1) Web-based problem solving through the Internet (self-examination therapy) without a coach (2) the same as 1, but with the possibility to ask help from a coach on the initiative of the respondent (on demand, by e-mail) (3) the same as 1, but with weekly scheduled contacts initiated by a coach (once per week, by e-mail) (4) weekly scheduled contacts initiated by a coach, but no Web-based intervention (5) information only (through the Internet)"
Outcomes	"Primary outcome measures are symptoms of depression and anxiety. Secondary outcome measures are dropout from the intervention, quality of life and economic costs. Other secondary outcome measures that may predict outcome are also studied (e.g. client satisfaction and problem-solving skills)"
Notes	

DRKS00004612

Methods	"Study type: interventional Allocation: randomized controlled trial Masking: open (masking not used) Control: no treatment Assignment: parallel Study design purpose: treatment"
Participants	"Inclusion criteria: current obsessive-compulsive disorder according to <i>DSM-IV</i> criteria (self-rating scale Y-BOCS-SR and OCI-R, as well as phone interview); informed consent to participate in the study; fluent in spoken and written German; access to Internet; agreement from their outpatient psychiatrist for crisis intervention Target sample: 40 Age: 18-65 years"
Interventions	"Intervention 1: experimental intervention: Internet-based cognitive-behavioral psychotherapy for adults with OCD Intervention 2: control intervention: waiting-list control condition"

DRKS00004612 (Continued)

Outcomes	"Yale-Brown Obsessive Compulsive Scale Obsessive Compulsive Inventory-Revised"
Notes	

Greist 2008

Methods	Randomised controlled trial
Participants	"Men and women aged 18 years, with OCD of at least 1 year's duration, meeting <i>DSM-IV</i> diagnostic criteria, and having a baseline Yale-Brown Obsessive-Compulsive Scale (Y-BOCS) score of 18"
Interventions	"The Web-based CBT (CT STEPS) includes an introductory session explaining the treatment, with videotapes of OCD patients who used an earlier version, along with 9 subsequent treatment modules. The CBT will include weekly e-mail feedback from a psychologist in response to questions asked about the CBT by the subjects assigned to this treatment arm"
Outcomes	Y-BOCS Work and Social Adjustment Scale Work Productivity and Activity Impairment Questionnaire Specific Health Problem Scale Cost
Notes	NCT00743834

Griffiths 2010a

Methods	Randomised controlled trial
Participants	Inclusion criteria "Aged 18-65 years, either sex - Evidence of psychological distress determined through screening questionnaire - Access to the Internet at home - Willingness to participate in the trial" Exclusion criteria "Current/prior participation in a project conducted by the Centre for Mental Health Research - Report that they are currently receiving treatment from a mental health professional or support group"
Interventions	Intervention group one: D-couch Intervention group two: Talking Point Control group: HealthWatch "Participants receive one of the above conditions or a combination of D-couch and Talking Point"
Outcomes	Centre for Epidemiology Studies-Depression (CES-D)

Griffiths 2010a (Continued)

- Anxiety symptoms: Penn State Worry Questionnaire (PSWQ)
- Quality of life: EUROHIS-QOL-8-item index
- Disability: 'Days Out of Role' questions adapted from US National Comorbidity Survey
- Depression knowledge:
 - D-Lit 21-item measure of depression literacy
 - An additional measure of depression treatment knowledge
- Perceived social support:
 - Medical Outcomes Study Social Support Survey (MOS)
 - UCLA Loneliness Scale
- Stigma: Depression Stigma Scale
- Help seeking: purpose developed items
- Self-esteem: Rosenberg Self-esteem Scale
- Empowerment:
 - Empowerment scale: Power-powerlessness subscale
 - Participation in healthcare measure
- Satisfaction and user-perceived benefits

Notes

ISRCTN36376369

Methods	"Randomised trial"
Participants	"Inclusion criteria: • Participants have a present, primary diagnosis of panic disorder according to the <i>DSM-IV</i> • Age between 18-70 • Patients give their informed consent • Patients are able to read and write the Dutch language" "Target number of participants: 130"
Interventions	"130 patients (across different treatment centres) will be randomly allocated to one of two treatment conditions: • A brief CBT intervention (guided self-help) of 10 weeks followed by manualised CBT (13 sessions) only when necessary • Treatment as usual according to the (NICE) guidelines"
Outcomes	"Primary outcome measures: • Panic and Agoraphobia Scale (PAS) • Outcome Questionnaire (OQ-45)"
Notes	

Kerkhof 2005

Methods	"Randomised, active controlled, crossover trial"
Participants	Inclusion criteria

Kerkhof 2005 (Continued)

	"Normal population 18+ years - Self-defined excessive worrying" Exclusion criteria "Under the age of 18 years - Undergoing psychiatric or psychological treatment - Using antidepressants - Clinical levels of anxiety and or depression"
Interventions	"A self-help brochure, with or without telephonic support. These exercises take one month's time. In the support condition, participants are contacted by telephone on a weekly basis for (minimal) support"
Outcomes	"Decrease in worrying intensity" "Decrease in anxiety, depression, sleeplessness"
Notes	

Kool 2007

Methods	"Randomised, controlled, parallel-group multicentre trial"
Participants	"They are recruited through screening (all patients who visited their General Practitioner (GP)). They have to meet the following criteria: - Between 18 to 65 years - A <i>Diagnostic and Statistical Manual of Mental Disorders (DSM)</i> diagnosis of minor depression, major depression, dysthymia, panic disorder (with or without agoraphobia), generalised anxiety disorder or social phobia. Patients with minor anxiety (not fulfilling any <i>DSM</i> criteria of an anxiety disorder) will also be included" Exclusion criteria "Have psychotic or bipolar symptoms - Have a high suicide risk - Are currently under treatment or received treatment for depression/anxiety in the past twelve months - Cannot read or write Dutch sufficiently enough to complete the questionnaires"
Interventions	"A stepped care program is characterised by different steps of treatment that are arranged in order of increasing intensity. After each step, the patient will be monitored, to determine if symptoms have been sufficiently reduced. The program consists of evidence-based interventions - Watchful waiting - Bibliotherapy - Problem-solving treatment - Medication and/or an evidence-based treatment in specialised mental health care The control condition is care as usual"
Outcomes	Quick Inventory of Depressive Symptomatology (QIDS) Hospital Anxiety and Depression Scale (HADS-A) for Anxiety <i>DSM</i> diagnosis (Composite International Diagnostic Interview (CIDI))

Kool 2007 (Continued)

Quality of life (Short Form health survey [SF36] and EuroQol questionnaire)

The use of health care services (Trimbos/iMTA Questionnaire for Costs Associated with Psychiatric Illness (TIC-P))

The use of medication (TIC-P)

Productivity losses (TIC-P)

Satisfaction with delivered care/continuity of care (Quote)

Notes

Lacasse 2010

Methods	"Randomised controlled trial"
Participants	"Participants self-report as having depression or anxiety Adults (age 18 or above)"
Interventions	"Psychoeducational course on mindfulness and lifestyle interventions for depression and anxiety"
Outcomes	Beck Depression Inventory (BDI) Beck Anxiety Inventory (BAI) Medication use Currently seeing therapist

Notes

Martinez 2008

Methods	"Randomised controlled trial"
Participants	"Inclusion criteria • Aged 16 and above • Living in the UK • Currently experiencing significant anxiety and or low mood as indicated by a nine-item Patient Health Questionnaire (PHQ-9) score above 10 or a seven-item PHQ (PHQ-7) score above 10 • Have online access and an active e-mail address • Wish to work in this way to address their difficulties" Exclusion criteria • No access to broadband Internet, soundcard and speakers either at home or the local library • Cannot read/understand the written and audio content • Active suicidal intent (as defined by a score of 2 or more on the suicide item questionnaire)"
Interventions	"This is a pilot randomised controlled trial comparing an online life skills package, Living Life To The Full (LLTFF), which is based on the cognitive behaviour therapy model, delivered with brief e-mail support (at least one e-mail per week), compared with matched e-mail support in the other arm of the study"

Martinez 2008 (Continued)

Outcomes	Beck Anxiety Inventory (BAI)
	Beck Depression Inventory (BDI-II)
	Work And Social Adjustment Scale (WASAS)
	Single-item satisfaction scale
	Four items assessing mental health literacy
Notes	

Moore 1998

Methods	Title only
Participants	
Interventions	
Outcomes	
Notes	

Muntingh 2009

Methods	"Two armed, cluster randomised controlled trial"
Participants	"Primary care patients with a <i>DSM-IV</i> diagnosis of PD and/or GAD will be included"
Interventions	"In the CSC group, a general practitioner, care manager and psychiatrist work together in a collaborative care framework. Stepped care is provided in three steps: (1) guided self-help, (2) cognitive behavioral therapy and (3) antidepressant medication"
Outcomes	Beck Anxiety Inventory (BAI) "Data on anxiety symptoms, mental and physical health, quality of life, health resource use and productivity"
Notes	

NTR1260

Methods	Randomised, single-blind, parallel-group intervention trial
Participants	"Inclusion criteria • Diagnosed with one or more phobias and waiting for treatment • Being able to work with the Internet Exclusion criteria • Not on waiting list for treatment of phobia at GGZ Buitendamstel Geestgronden"

NTR1260 (Continued)

	Target number of participants: 60
Interventions	"Internet-based guided self-help for patients suffering from a phobia (program based on cognitive behavioural therapy with a focus on exposure, with minimal guidance) The intervention will take about 12 weeks consisting of 6 sessions and home work in between sessions" "In this pilot trial we want to treat patients who are waiting for cognitive-behavioural therapy in a specialised mental health care institute with the guided self-help. We expect the patients to evaluate the treatment as feasible and satisfying" Control: unclear
Outcomes	Feasability with and satisfaction about use of the treatment Decrease in anxiety level
Notes	

NTR2573

Methods	Randomised, open-label, parallel-group, active-controlled, intervention trial
Participants	"Inclusion criteria • The presence of clinically relevant anxiety and/or depressive symptoms as established by a score of 7 or higher on the HADS-A and/or a score of 16 or higher on the CES-D • The participant is suffering from low self-esteem as established by administration of the Rosenberg Self-esteem Scale (Rosenberg 1965) • The participant is troubled and/or limited by the aforementioned symptoms Exclusion criteria • Insufficient understanding of the Dutch language" Target number of participants: 128
Interventions	"Experimental group: self-help book (which entails 6 different techniques, based on elements from cognitive behavioural therapy) Control group: waiting-list (no self-help book during 3 month wait-list period, but participants are free to seek help through the common channels in this time) Duration of the intervention may vary from 6 to 10 weeks, depending on the pace of the individual respondent Guidance by specially trained students and research assistants is mainly focused on motivating the respondent to work through the entire book, whilst providing feedback and information in case of any misunderstanding of the assignments described in the book"
Outcomes	Rosenberg Self-esteem Scale
Notes	

Schmidt 2008

Methods	"Study type: interventional Study design: allocation: randomized Endpoint classification: efficacy study Intervention model: parallel assignment Masking: open label Primary purpose: treatment"
Participants	Inclusion criteria • <i>DSM-IV</i> diagnosis of panic disorder, with or without agoraphobia Exclusion criteria • Change in medication type or dose 12 weeks prior to study entry • Suicidal • Current substance abuse • Current or past schizophrenia, bipolar disorder, or organic mental disorder. Patients on medications must meet stability requirements such that initial administration of the medication and dose has been maintained for at least 3 months prior to treatment and will be maintained until post-treatment.
Interventions	"Participants in this study will be randomly assigned to receive computer based treatment, book based treatment, or no treatment for 10 weeks"
Outcomes	"After these 10 weeks, symptoms of anxiety and depression will be measured with patient interviews, scales, and questionnaires"
Notes	

Spooner 2011

Methods	Randomised controlled trial
Participants	Not described
Interventions	• "Computer based cognitive behaviour therapy (CBT) self-help — immediate start • Computer based CBT self-help — delayed start (8 weeks) • Manual based CBT self-help — immediate start • Manual based CBT self-help — delayed start (8 weeks)"
Outcomes	Not described
Notes	

Titov 2011a

Methods	Randomised controlled trial
---------	-----------------------------

Titov 2011a (Continued)

Participants	"All participants will meet <i>Diagnostic and Statistical Manual of Mental Disorders 4th Edition (DSM-IV)</i> criteria for depression and/or social phobia and/or panic disorder (with or without agoraphobia) and/or generalised anxiety disorder (GAD)"
Interventions	"Participants will be randomly assigned to one of two groups: (1) an immediate treatment group, or (2) a delayed treatment group, also known as a wait-list control group"
Outcomes	Generalised Anxiety Disorder-7 Item Scale (GAD-7) Social Phobia Scale (SPS) Social Interaction Anxiety Scale (SIAS) NEO-Five Factor Inventory (NEO-FFI) Neuroticism Scale Sheehan Disability Scale (SDS) Kessler 10-Item Scale (K-10) Panic Disorder Severity Rating Scale — Self-Report Version (PDSS-SR) Penn State Worry Questionnaire (PSWQ) Patient Health Questionnaire-9 (PHQ-9)
Notes	

van Ballegooijen 2011

Methods	Randomised controlled trial
Participants	"Participants with mild or sub-clinical PD"
Interventions	Don't Panic Online (DPO) Wait list
Outcomes	"Severity of panic and anxiety symptoms are the primary outcome measures. Secondary outcomes include depressive symptoms, quality of life, loss of production and health care consumption"
Notes	

Characteristics of ongoing studies [ordered by study ID]

ACTRN12610001081000

Trial name or title	Cognitive Behavioural, Guided Self-Help for Clinical Perfectionism Versus Guided Self-Help for Anxiety and Depression
Methods	"Study type: interventional Allocation: randomized Primary purpose: treatment"
Participants	"Key inclusion criteria

ACTRN12610001081000 (Continued)

- Age 18 years of age or older
- Must have an elevated score (> 22.2) on the CM subscale of the FMPS
- A primary diagnosis of either a depressive disorder and/or an anxiety disorder
- If on antidepressant medication, must have been stabilised on this medication for at least three months prior to the baseline assessment

Key exclusion criteria

- A primary diagnosis of OCD or PTSD
- *DSM-IV* diagnosis of schizophrenia/psychosis
- Serious suicide ideation.
- Alcohol/Substance dependence.
- Anyone currently in psychological therapy.
- Anyone meeting the *DSM-IV* criteria for anorexia nervosa and/or a seriously low body weight (BMI < 17.5)"

Target sample: 75

Interventions	Duration trial: 8 weeks Group 1: guided self-help book for clinical perfectionism Group 2: guided self-help book for anxiety and depression Group 3: wait-list-control (randomly allocated to Group 1 or 2 after 8 weeks)
Outcomes	Primary outcomes • Clinical Perfectionism Questionnaire (CPQ) • Depression Anxiety Stress Scales (DASS) • Frost Multidimensional Perfectionism Scale (FMPS) Secondary outcomes • Dichotomous Thinking in Eating Disorders Scale (DTEDS) • Dysfunctional Attitudes Scale (DAS)
Starting date	01/2011
Contact information	s.egan@curtin.edu.au
Notes	

ACTRN12611000321943

Trial name or title	Internet-Based Cognitive-Behavioural Treatment for Obsessive-Compulsive Disorder
Methods	"Study type: interventional Allocation: randomized Masking: blinded Assignment: parallel Primary purpose: treatment Endpoint: efficacy"

ACTRN12611000321943 (Continued)

Participants	"Key inclusion criteria • Individuals must be 18 years or older and must reside in Australia • Individuals with a primary diagnosis of OCD and/or secondary diagnoses with symptoms not in the severe range are considered to fulfil participation criteria Key exclusion criteria • Individuals with psychotic symptoms, acute distress, or suicidal risk are considered ineligible for participation and will be referred for appropriate treatment to a mental health professional" Target sample: 212
Interventions	Duration: 12 weeks; 1 module (of 1 hour duration) completed each week Group 1: Therapist-assisted, Internet-based treatment (adapted from a manualised face-to-face CBT programme: OCD-STOP) Group 2: Progressive relaxation training for anxiety (PRT) Upon completing the PRT and post-treatment measures, individuals will complete the therapist-assisted OCD STOP free of charge
Outcomes	Primary outcomes • Yale-Brown Obsessive-Compulsive Scale • Hamilton Depression Rating Scale • Hamilton Anxiety Rating Scale Secondary outcomes • Australian Quality of Life Scale • Revised Obsessional Beliefs Questionnaire • Brief Disability Questionnaire • Self-Ambivalence Measure
Starting date	04/2011
Contact information	mkyrios@swin.edu.au
Notes	

ACTRN12611001052921

Trial name or title	Online Self-Help Meta-Cognitive Therapy for Obsessive-Compulsive Disorder
Methods	"Study type: interventional Allocation: randomized Masking: open (masking not used) Assignment: parallel Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria

ACTRN12611001052921 (Continued)

- Have a total score on the OCI-R of 21 or greater
- Be aged 18 years or over
- If participants are taking medication such as antidepressants or other mood stabilisers, they must be stable on this medication one month prior to their baseline assessment
- Participants must be willing to remain on the same dosage of the same medication throughout the entire treatment period including follow-up
- Be currently living in Australia

Key exclusion criteria

- They show moderate to high suicidal risk as measured by the suicidality section of the MINI
- They meet the diagnosis for psychosis and/or schizophrenia, according to the *DSM-IV-TR* diagnostic criteria, as measured by the PDSQ
- They are currently undergoing other psychological treatment"

Target sample size: 40

Interventions	Group 1: online meta-cognitive therapy for OCD (10 modules) Group 2 (active control): online (audio) progressive muscle relaxation training (weekly exercises of 15-30 minutes duration) Duration of trial: 6 weeks After implementation and evaluation of the programme is complete, participants in Group 2 will be provided with access to the online MCT programme if they wish
Outcomes	"Primary outcomes • Obsessive-compulsive Inventory — Revised (OCI-R) Secondary outcomes • Yale-Brown Obsessive-Compulsive Symptom Checklist • Depression, Anxiety, Stress Scales-21 (DASS-21) • Metacognitions Questionnaire-30 (MCQ-30) • Quality of Life Enjoyment and Satisfaction Questionnaire-18 (Q-LES-Q-18) • Psychiatric Diagnostic Screening Questionnaire (PDSQ) • Suicidality section of the Mini International Neuropsychiatric Interview (MINI)"
Starting date	07/2012
Contact information	C.Rees@curtin.edu.au
Notes	Trial Website: http://ocd-doctor-online.squarespace.com/

ACTRN12611001055998

Trial name or title	The Wellbeing6 Program for Generalized Anxiety Disorder (GAD) and/or Depression
Methods	"Study type: interventional Allocation: randomized Masking: open (masking not used) Assignment: parallel

ACTRN12611001055998 (Continued)

	Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria • Meet <i>Diagnostic and Statistical Manual of the American Psychiatric Association-4th Edition (DSM-IV)</i> criteria for generalised anxiety disorder and/or major depressive disorder, Internet access + printer access, Australian citizen • Ages: 18 years and older Key exclusion criteria • Current substance abuse/dependence, psychotic mental illness (bipolar or schizophrenia), current or planned psychological treatment during study duration, change in medications during last 1 month or intended change during study duration, use of benzodiazepines, suicidal Target sample size: 100"
Interventions	Group 1: clinician-assisted Internet-based cognitive behavioural therapy for generalised anxiety disorder (GAD) and/or depression Group 2: Wait-list control group Duration of trial: 10 weeks 6 online lessons (of 15 minutes' duration) to be completed (consecutively) every 7-14 days After 10 weeks, participants in the wait-list control group will receive the same treatment as Group 1
Outcomes	"Primary outcomes • Patient Health Questionnaire-9 (PHQ-9) • Generalized Anxiety Disorder-7 Item (GAD-7) Questionnaire Secondary outcomes • Psychological distress measured by the Kessler-10 (K-10) • Beck Depression Inventory-II (BDI-II) • NEO-Five Factor Inventory, Neuroticism Scale (NEO-FF-I, N Scale) • Penn State Worry Questionnaire (PSWQ) • WHO Disability Assessment Scale (WHODASS-II) • Repetitive Thinking Questionnaire (RTF) • Positive Beliefs about Rumination Scale-Adapted Version (PSRS-A) • Patient satisfaction questionnaire • Adherence to lessons and homework"
Starting date	10/2011
Contact information	gavina@unsw.edu.au
Notes	Trial acronym: Wellbeing6. Trial Website: www.virtualclinic.org.au

ACTRN12612000430831

Trial name or title	Factorial, Internet-Delivered Self-Guided versus Therapist-Guided Treatment (disorder-specific or trans-diagnostic) for Social Phobia
---------------------	---

ACTRN12612000430831 (Continued)

Methods	"Study type: interventional Allocation: randomized Masking: blinded Assignment: factorial Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria • Self-identifying as having difficulties with symptoms of social phobia • Elevated symptoms of social phobia as indicated by a score of 5 or more on the MINI-SPIN, indicating at least mild symptoms • No current participation in cognitive behavioural treatment for social phobia • No change in medications in the period 1 month prior to this study • Access to the Internet and a printer • 18 years to 64 years old • Resident of Australia • Prepared to provide name, phone number and address, and to provide the name and address of a local general practitioner • Prepared to provide written informed consent Key exclusion criteria • Severe depression at application (score of 23 or greater on PHQ-9) • Suicidal intent or plan at application (either reporting suicidal intent or responding > 2 to Question 9 (suicidal ideation) on the PHQ-9) • Scoring below cutoffs on PHQ-9 • Current psychosis • Principal presenting problem not symptoms of social phobia Target population: 208"
Interventions	Factorial, 2 (self-guided vs guided) × 2 (trans-diagnostic vs disorder-specific) treatment for social phobia Duration of trial: 8 weeks
Outcomes	"Primary outcomes • MINI-Social Phobia Inventory (MINI-SPIN) Secondary outcomes • Generalised Anxiety Disorder-7 item scale (GAD-7) • Patient Health Questionnaire-9 (PHQ-9) • Kessler-10 item (K-10) • Sheehan Disability Scales (SDS) • Things You Do Questionnaire (TYD) • Working Alliance Inventory (WAI) • Service Use Questionnaire • Panic Disorder Severity Rating Scale-Self-Report (PDSS-SR)"
Starting date	04/2012

ACTRN12612000430831 (Continued)

Contact information	nick.titov@mq.edu.au
Notes	Trial acronym: WELLRCT5

ACTRN12612000431820

Trial name or title	Factorial, Internet-Delivered Self-Guided versus Therapist-Guided Treatment (disorder-specific or trans-diagnostic) for Panic Disorder
Methods	"Study type: interventional Allocation: randomized Masking: blinded Assignment: factorial Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria • Self-identifying as having difficulties with symptoms of panic disorder • Elevated symptoms of panic as indicated by a score of 1 or more on the Autonomic Nervous System Questionnaire, indicating at least mild symptoms • No current participation in cognitive behavioural treatment for panic disorder • No change in medications in the period 1 month prior to this study • Access to the Internet and a printer • 18 years to 64 years old • Resident of Australia • Prepared to provide name, phone number and address, and to provide the name and address of a local general practitioner • Prepared to provide written informed consent Key exclusion • Severe depression at application (score of 23 or greater on PHQ-9) • Suicidal intent or plan at application (either reporting suicidal intent or responding > 2 to Question 9 (suicidal ideation) on the PHQ-9) • Scoring below cutoffs on PHQ-9 • Current psychosis • Principal presenting problem not symptoms of panic disorder Target sample size: 208"
Interventions	Factorial, 2 (self-guided vs guided) × 2 (trans-diagnostic vs disorder-specific) treatment for panic disorder Duration of trial: 8 weeks
Outcomes	"Primary outcomes • Panic Disorder Severity Rating Scale-Self-Report (PDSS-SR) Secondary outcomes • Generalised Anxiety Disorder-7 item scale (GAD-7)

ACTRN12612000431820 (Continued)

- Patient Health Questionnaire-9 (PHQ-9)
- Kessler-10 Item (K-10)
- Sheehan Disability Scales (SDS)
- Things You Do Questionnaire (TYD)
- Working Alliance Inventory (WAI)
- Service Use Questionnaire
- MINI-Social Phobia Inventory (MINI-SPIN)"

Starting date	10/2011
Contact information	nick.titov@mq.edu.au
Notes	Trial acronym: WELLRCT6

ACTRN12612000433808

Trial name or title	Factorial, Internet-Delivered Self-Guided versus Therapist-Guided Treatment (disorder-specific or trans-diagnostic) for Generalised Anxiety Disorder
Methods	"Study type: interventional Allocation: randomized Masking: blinded Assignment: factorial Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria • Self-identifying as having difficulties with symptoms of generalised anxiety disorder • Elevated symptoms of generalised anxiety disorder as indicated by a score of 5 or more on the GAD-7, indicating at least mild symptoms • No current participation in cognitive behavioural treatment for generalised anxiety disorder • No change in medications in the period 1 month prior to this study • Access to the Internet and a printer • 18 years to 64 years old • Resident of Australia • Prepared to provide name, phone number and address, and to provide the name and address of a local general practitioner • Prepared to provide written informed consent Key exclusion criteria • Severe depression at application (score of 23 or greater on PHQ-9) • Suicidal intent or plan at application (either reporting suicidal intent or responding > 2 to Question 9 (suicidal ideation) on the PHQ-9) • Scoring below cutoffs on PHQ-9 • Current psychosis • Principal presenting problem not symptoms of generalised anxiety disorder Target sample size: 208"

ACTRN12612000433808 (Continued)

Interventions	Factorial, 2 (self-guided vs guided) × 2 (trans-diagnostic vs disorder-specific) treatment for generalised anxiety disorder Duration of trial: 8 weeks
Outcomes	"Primary outcomes - Generalised Anxiety Disorder-7 item scale (GAD-7) Secondary outcomes - Patient Health Questionnaire-9 (PHQ-9) - European Quality of Life-5 Dimensions-5 Level (EQ-5D-5L) - Kessler-10 item (K-10) - Sheehan Disability Scales (SDS) - Things You Do Questionnaire (TYD) - Working Alliance Inventory (WAI) - Service Use Questionnaire - MINI-Social Phobia Inventory (MINI-SPIN) - Panic Disorder Severity Rating Scale-Self-Report (PDSS-SR)"
Starting date	04/2012
Contact information	nick.titov@mq.edu.au
Notes	Trial acronym: WELLRCT4

ACTRN12612001073897

Trial name or title	Internet-Based Cognitive Behavioural Therapy for Obsessive Compulsive Disorder (OCD)
Methods	"Study type: interventional Allocation: randomized Masking: open (masking not used) Assignment: parallel Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria - Meet <i>Diagnostic and Statistical Manual of the American Psychiatric Association-4th Edition (DSM-IV)</i> criteria for obsessive compulsive disorder, Internet access + printer access, Australian citizen - Ages: 18 years and older Key exclusion criteria - Current substance abuse/dependence, psychotic mental illness (bipolar or schizophrenia), current or planned psychological treatment during study duration, previous iCBT for OCD, change in medications during last 1 month or intended change during study duration, use of benzodiazepines, suicidal Target sample size: 50"

ACTRN12612001073897 (Continued)

Interventions	Group 1: clinician-assisted Internet based cognitive behavioural therapy for obsessive compulsive disorder (OCD) Group 2: Wait-list control group Duration of trial: 10 weeks 6 online lessons (of 15 minutes' duration) to be completed (consecutively) every 7-14 days After 10 weeks, participants in the wait-list control group will receive the same treatment as Group 1
Outcomes	"Primary outcomes • Obsessive Beliefs Questionnaire (OBQ) • Dimensional Obsessive Compulsive Scale (DOCS) Secondary outcomes • Kessler-10 (K-10) • Patient Health Questionnaire-9 (PHQ-9) • Generalized Anxiety Disorder-7 item (GAD-7) • Sheehan Disability Scale (SDS) • Credibility/Expectancy Questionnaire (CEQ) • Adherence to lessons and homework"
Starting date	10/2012
Contact information	gavina@unsw.edu.au
Notes	Trial acronym: The OCD Program. Trial Website: www.virtualclinic.org.au

ACTRN12612001306808

Trial name or title	Internet-Based Cognitive Behavioural Therapy for Obsessive Compulsive Disorder (OCD)
Methods	"Study type: interventional Allocation: randomized Masking: open (masking not used) Assignment: parallel Primary purpose: treatment Endpoint: efficacy"
Participants	"Key inclusion criteria • Meet <i>Diagnostic and Statistical Manual of the American Psychiatric Association-4th Edition (DSM-IV)</i> criteria for obsessive compulsive disorder, Internet access + printer access, Australian citizen, can access face-to-face sessions in Sydney for 12 weeks and referred to the Anxiety Disorders Clinic, St Vincent's Hospital, Sydney • Ages: 18 years and older Key exclusion criteria

ACTRN12612001306808 (Continued)

- Current substance abuse/dependence, psychotic mental illness (bipolar or schizophrenia), current or planned psychological treatment during study duration, previous iCBT for OCD, change in medications during last 1 month or intended change during study duration, use of benzodiazepines, suicidal

Target sample size: 50"

Interventions	Group 1: Internet-based CBT (iCBT) Group 2: clinician-guided, face-to-face CBT (treatment manual) Duration of trial: 12 weeks
Outcomes	"Primary outcomes • Obsessive Beliefs Questionnaire (OBQ) • Dimensional Obsessive Compulsive Scale (DOCS) Secondary outcomes • Kessler-10 (K-10) • Patient Health Questionnaire-9 (PHQ-9) • Generalized Anxiety Disorder-7 item (GAD-7) Questionnaire • Sheehan Disability Scale (SDS) • Credibility/Expectancy Questionnaire (CEQ) • Adherence to lessons and homework"
Starting date	01/2013
Contact information	gavina@unsw.edu.au
Notes	Trial Website: www.virtualclinic.org.au

Carlbring 2012a

Trial name or title	Internet-Based Treatment of Generalized Anxiety Disorder
Methods	Randomised controlled trial
Participants	Satisfy <i>DSM-IV</i> criteria for generalised anxiety disorder
Interventions	iCBT
Outcomes	Penn State Worry Questionnaire (PSWQ) Beck Anxiety Inventory (BAI) Quality of Life Inventory (QOLI) Montgomery-Åsberg Depression Rating Scale (MADRS) Patient Health Questionnaire (PHQ-9) Generalized Anxiety Disorder-7 (GAD-7)
Starting date	31/03/2012

Carlbring 2012a (Continued)

Contact information	per.carlbring@psy.umu.se
Notes	NCT01570374

Carlbring 2012b

Trial name or title	An Internet-Administered, Mindfulness Training Program for the Treatment of Anxiety
Methods	Randomised controlled trial
Participants	Satisfy <i>DSM-IV</i> criteria for an anxiety disorder
Interventions	"A 10 week, Internet-administered mindfulness training program, featuring no therapist-support. 10 minutes of training, twice per day"
Outcomes	Beck Anxiety Inventory (BAI) Beck Depression Inventory (BDI) Quality of Life Inventory (QOLI) Insomnia Severity Index (ISI)
Starting date	03/2012
Contact information	per.carlbring@psy.umu.se
Notes	NCT01577290

Clark 2012

Trial name or title	Internet-Based Cognitive Therapy for Social Anxiety Disorder
Methods	Randomised controlled trial
Participants	Adults with social anxiety
Interventions	Internet-based self-help
Outcomes	ADIS Social Phobia Weekly Summary Scale Liebowitz Social Anxiety Scale Fear of Negative Evaluation Scale Social Phobia Scale Social Interaction and Anxiety Scale PHQ-9 Sheehan Disability Scale Social Cognitions Questionnaire

Clark 2012 (Continued)

	Social Attitudes Questionnaire
	Treatment Credibility Scale
	Working Alliance Inventory
Starting date	01/01/2013
Contact information	david.clark@psy.ox.ac.uk
Notes	ISRCTN95458747

Havik 2012

Trial name or title	Treatment of Social Phobia and Panic Disorder in Adults (TSPPA)
Methods	Randomised
Participants	Clinical diagnosis of panic disorder or social phobia
Interventions	individual cognitive-behavioural therapy Stepped-care: short psycho-education, a 10 week Internet-based self-help programme, 12 weeks' individual cognitive-behavioural therapy
Outcomes	Clinical Severity Ratings Panic Attack Scale Body Sensations Questionnaire Agoraphobic Cognitions Questionnaire Mobility Inventory for Agoraphobia Social Phobia Scale Social Interaction Anxiety Scale
Starting date	16/01/2008
Contact information	Odd E Havik, University of Bergen
Notes	NCT00619138

Hedman 2012

Trial name or title	Cognitive Behavioral Therapy, Guided Self-Help Versus Individual Therapy
Methods	Randomised controlled trial
Participants	"Have an anxiety disorder diagnosis (obsessive compulsive disorder, social phobia, panic disorder, generalized anxiety disorder), or/and major depression, or/and maladaptive stress reaction, or/and primary insomnia. Clinician severity rating scale 2-6"

Hedman 2012 *(Continued)*

Interventions	Group 1: "After the initial guided self-help, patients in this arm continue the same self-help program with 1 additional guiding session" Group 2: "After the initial face of self-help CBT, patients in this arm receive individual CBT. The cognitive behavior therapy used in the study will be based on the protocols with best empirical support. The therapy is delivered by the same psychologist as in the first face, and the second face builds on the learnings from the first face"
Outcomes	Clinician Severity Rating (CSR) Montgomery-Åsberg Depression Rating Scale — Self-report (MADRS-S) Work Ability Index (WAI) Insomnia Severity Index (ISI) Health Anxiety Inventory (HAI) Perceived Stress Scale (PSS) Trimbos and Institute of Technology Cost Questionnaire for Psychiatry (TIC-P) Quality of Life Inventory (QOLI) EuroQol-5 Dimension (EQ5D) Sheehan Disability Scales (SDS) Self-Rated Health 5 (SRH-5) Obsessive Compulsive Inventory — Revised (OCI-R) Liebowitz Social Anxiety Scale Self-Report (LSAS-SR) Panic Disorder Severity Scale Self-Rated (PDSS-SR) Penn State Worry Questionnaire (PSWQ)
Starting date	08/2012
Contact information	kire.hedman@ki.se ; sigrid.salomonsson@sl.se
Notes	NCT01667822

Hedman 2013

Trial name or title	Internet Treatment for Health Anxiety (HA-X)
Methods	Randomised controlled trial
Participants	Adults with health anxiety
Interventions	Group 1: CBT, exposure and response prevention Group 2: BSM, stress management and applied relaxation
Outcomes	Health Anxiety Inventory (HAI) Illness Attitude Scale (IAS)

Hedman 2013 (Continued)

Whiteley Index (WI)

Montgomery-Åsberg Depression Rating Scale — Self-report (MADRS-S)

Beck Anxiety Inventory (BAI)

Anxiety Sensitivity Index (ASI)

Insomnia Severity Index (ISI)

Sheehan Disability Scale (SDS)

Trimbos and Institute of Medical Technology Assessment Cost Questionnaire (TIC-P)

EuroQol-5D (EQ-5D)

Obsessive Compulsive Inventory Revised (OCI-R)

Yale-Brown Obsessive Compulsive Scale (YBOCS)

AUDIT (alcohol use)

Starting date	09/2012
Contact information	kire.hedman@ki.se
Notes	NCT01673035

ISRCTN73535163

Trial name or title	Obsessive Compulsive Treatment Efficacy Trial
Methods	Randomised controlled trial
Participants	"Inclusion criteria: - Adults of 18 and above meeting <i>Diagnostic and Statistical Manual of Mental Disorders (DSM)-IV</i> criteria for obsessive compulsive disorder (assessed using the Structured Clinical Interview for <i>DSM-IV</i> (SCID-IV)) - Scoring 16 or over on the Yale Brown Obsessive Compulsive Checklist (YBOCS) - On a waiting list for therapist-led cognitive behavioral therapy (CBT) in either primary or secondary mental health care settings - Able to read English at a level of 11 years and above" Target number of participants: 432
Interventions	"Computer-aided cognitive-based therapy (cCBT) package (OC Fighter) over 12 weeks - Supported by six, 10-minute brief scheduled telephone calls, face to face or e-mail contact (depending on patient preference) from a mental health professional (total direct clinical input, 60 minutes) - Bibliotherapy (guided self-help) will consist of a self-help book <i>Overcoming OCD: A Workbook</i>, written by Karina Lovell - Participants will receive weekly guidance from a mental health professional for an initial session of 60 minutes (either face to face or telephone dependent on patient preference) - This is followed by up to 10 brief (30-minute) scheduled telephone, e-mail or face-to-face (dependent on patient preference) sessions over a 12-week period (total direct clinical input, 6 hours) - Comparator — patients on a waiting list for CBT"

ISRCTN73535163 (Continued)

Outcomes	Yale-Brown Obsessive Compulsive Scale (YBOCS) checklist
Starting date	09/2011
Contact information	Karina.Lovell@manchester.ac.uk
Notes	Trial acronym: OCTET

Kok 2012

Trial name or title	Internet-Based Treatment of Phobias in Specialized Mental Health Care: A Randomised Trial—FocD (Fobieën onder controle Doelmatigheid)
Methods	Randomised controlled trial
Participants	Adults with a social-, agora-, or specific phobia in a mental health care institute are eligible
Interventions	"Fobieën onder controle" by Manja de Neef and Pim Cuijpers
Outcomes	Fear Questionnaire (FQ) Beck Anxiety Inventory (BAI) Center for Epidemiologic Studies-Depression (Ces-D) Client Satisfaction Questionnaire (CSQ) Short Form-Health and Labour Questionnaire (SF-HLQ) Quality of Life in Economic Evaluations (EQ-5d) Trimbos/Imta Tic-P Questionnaire for Costs Associated With Psychiatric Illness (Tic-P) Cut-Annoyed-Guilty-Eye opener (CAGE-alcohol questionnaire)
Starting date	1/10/2010
Contact information	ga.schreuders@psy.vu.nl
Notes	NTR2233

Nordgren 2012

Trial name or title	Tailored Internet-Administered Treatment of Anxiety Disorders for Primary Care Patients
Methods	Randomised controlled trial
Participants	"Inclusion criteria • Over 18 years • Fulfill the criteria for at least one anxiety syndrome Exclusion criteria

Nordgren 2012 (Continued)

	• Ongoing psychological treatment • Suicidal prone • Ongoing alcohol addiction"
Interventions	"Behavioral: Tailored Internet-administrated CBT Treatment Behavioral: Wait-list"
Outcomes	Clinical Outcome in Routine Evaluation-Outcome Measure (CORE-OM) Montgomery-Åsberg Depression Rating Scale-Self-rated (MADRS-S) Quality of Life Inventory (QOLI)
Starting date	
Contact information	
Notes	

NTR1985

Trial name or title	Self-Help Acceptance and Commitment Therapy for Psychological Distress
Methods	Study type: interventional Allocation: randomised Masking: open label Assignment: parallel
Participants	"Inclusion criteria: people of 18 years and older with mild to moderate psychological distress (anxiety, depression), fatigue and/or with low mental health"
Interventions	• "Experimental condition: the self-help intervention, "Living to the Fullest" with process monitoring by a counselor with e-mail contact. The intervention is a self-help book that consists of 9 modules. The intervention is based on six core processes of ACT for creating more psychological flexibility and is based on mindfulness exercises • Experimental condition: the self-help intervention with process monitoring and personal support by a counselor with e-mail contact • Control condition: a wait-list group; the participants receive the self-help intervention after three months"
Outcomes	"Primary outcome: decreased psychological distress (depression measured with the CES-D and anxiety measured with the HADS-A) Secondary outcome: increased mental health, measured with the MHC-SF"
Starting date	03/2009
Contact information	e.t.bohlmeijer@gw.utwente.nl
Notes	Trial Website: www.cursusmindfulness.nl

Proudfoot 2010

Trial name or title	Evaluation of Electronic Program to Help People Self-Monitor and Self-Manage of Mild to Moderate Mental Health Problems
Methods	Randomised controlled trial
Participants	Key inclusion criteria • "Have a current episode of mild to moderate depression, anxiety or stress • Own a mobile phone and have access to a computer and the Internet • Have a current, valid e-mail address • Live in Australia • Be able to read and write English • Agree to use the program for 6 weeks and to complete questionnaires on 3 occasions" Key exclusion criteria • "Severe anxiety, stress or depression • Suicidal ideation • Psychosis"
Interventions	Participants receiving the intervention will use an electronic self-monitoring and self-management programme for 6 weeks, accessed via their mobile phone and computer. The programme consists of real-time monitoring of symptoms with SMS prompts, therapeutic modules, fact sheets, tips, motivational messages and a journal There are two control groups: (1) an online attention control providing minimal information about depression, anxiety and stress, together with weekly motivational messages for 6 weeks; (2) a wait-list control, who will receive the full electronic self-monitoring and self-management programme immediately after the intervention group concludes their treatment (6 weeks after the start of the programme)
Outcomes	Mean Depression Anxiety Stress Scales (DASS) score Subjective Units of Distress Scales (SUDS) Work & Social Adjustment Scale Utilisation of Health Services: This will be assessed using a questionnaire asking for details about participants' use of varying health services and medications Self-efficacy for coping with depression, anxiety and stress: This will be assessed using our new six-item questionnaire developed using Bandura's (2006) guidelines
Starting date	17/09/2010
Contact information	v.harrison@unsw.edu.au, j.proudfoot@unsw.edu.au
Notes	

Rollman 2012

Trial name or title	Online Treatments for Mood and Anxiety Disorders in Primary Care
Methods	Randomised
Participants	18-75 years of age

Rollman 2012 *(Continued)*

	Current major depression, panic and/or generalised anxiety disorder on PRIME-MD
Interventions	Computerised CBT + Internet support group Computerised CBT alone Usual care
Outcomes	SF-12 MCS Hamilton Rating Scale for Depression Hamilton Rating Scale for Anxiety WHO Health and Work Performance Questionnaire Health Services Utilization and Costs Attitudes to Computerized CBT Computerized CBT and Internet Support Group Usage
Starting date	22/11/11
Contact information	Bruce Rollman, University of Pittsburgh
Notes	NCT01482806

Tulbure 2012

Trial name or title	Internet Treatment for Social Phobia (iSOFIE-Ro)
Methods	Randomised controlled trial
Participants	Adults with social anxiety
Interventions	"Internet-administrated CBT for Social Phobia that contains 9 self-help text modules and exercises (i.e. behavioral experiments)"
Outcomes	Leibowitz Social Anxiety Scale-Self-Report (LSAS-SR) Social Phobia Inventory (SPIN) Beck Depression Inventory-II (BDI-II) Attitude and Belief Scale-II (ABS-II) Anxiety Sensitivity Index (ASI) Social Interaction and Anxiety Scale (SIAS) Social Phobia Screening Questionnaire (SPSQ)
Starting date	04/12
Contact information	Gerhard.Andersson@liu.se ; btulbure@gmail.com , bogdan.tulbure@ubbcluj.ro
Notes	NCT01557894

DATA AND ANALYSES

Comparison 1. Media-delivered interventions compared to no intervention

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
1 Anxiety (any measure), self-rated at post-treatment	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
2 Anxiety (any measure), self-rated at post-treatment (type of control)	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
2.1 Adjunct	1	93	Std. Mean Difference (Random, 95% CI)	0.33 [-0.08, 0.75]
2.2 Attention controls	22	1028	Std. Mean Difference (Random, 95% CI)	0.65 [0.48, 0.82]
2.3 Treatment as Usual	7	745	Std. Mean Difference (Random, 95% CI)	0.21 [0.02, 0.40]
2.4 Wait-List	46	2671	Std. Mean Difference (Random, 95% CI)	0.77 [0.61, 0.92]
3 Anxiety (any measure), self-rated at post-treatment (by disorder)	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
3.1 GAD	10	649	Std. Mean Difference (Random, 95% CI)	0.95 [0.44, 1.45]
3.2 Health	2	120	Std. Mean Difference (Random, 95% CI)	1.01 [0.24, 1.78]
3.3 Mixed	18	1376	Std. Mean Difference (Random, 95% CI)	0.36 [0.19, 0.52]
3.4 OCD	2	220	Std. Mean Difference (Random, 95% CI)	0.68 [0.41, 0.95]
3.5 Panic	21	797	Std. Mean Difference (Random, 95% CI)	0.62 [0.45, 0.79]
3.6 Social	15	1152	Std. Mean Difference (Random, 95% CI)	0.73 [0.59, 0.87]
3.7 Specific	8	223	Std. Mean Difference (Random, 95% CI)	0.99 [0.50, 1.48]
4 Anxiety (any measure), self-rated at post-treatment (by media)	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
4.1 Book	27	1046	Std. Mean Difference (Random, 95% CI)	0.48 [0.30, 0.65]
4.2 Computer	8	458	Std. Mean Difference (Random, 95% CI)	0.69 [0.31, 1.07]
4.3 Internet	36	2522	Std. Mean Difference (Random, 95% CI)	0.79 [0.62, 0.96]
4.4 Other	5	511	Std. Mean Difference (Random, 95% CI)	0.51 [0.32, 0.71]
5 Anxiety (any measure), self-rated at post-treatment (by therapy)	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
5.1 CBT	56	3550	Std. Mean Difference (Random, 95% CI)	0.62 [0.51, 0.73]
5.2 Exposure/ Desensitization	13	448	Std. Mean Difference (Random, 95% CI)	0.79 [0.49, 1.09]
5.3 Relaxation	6	504	Std. Mean Difference (Random, 95% CI)	0.84 [-0.06, 1.73]
5.4 Self-examination	1	35	Std. Mean Difference (Random, 95% CI)	0.92 [0.22, 1.62]
6 Anxiety (any measure), self-rated at post-treatment (by setting)	75	4506	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
6.1 Advertising	55	3438	Std. Mean Difference (Random, 95% CI)	0.77 [0.63, 0.90]
6.2 Referral Only	16	884	Std. Mean Difference (Random, 95% CI)	0.32 [0.14, 0.51]
6.3 Other	4	184	Std. Mean Difference (Random, 95% CI)	0.46 [0.10, 0.82]
7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias)	76	4537	Std. Mean Difference (Random, 95% CI)	0.67 [0.55, 0.78]
7.1 High or Unclear Risk for Incomplete Outcome Data	30	1481	Std. Mean Difference (Random, 95% CI)	0.68 [0.46, 0.90]
7.2 Low Risk for Incomplete Outcome Data	46	3056	Std. Mean Difference (Random, 95% CI)	0.66 [0.53, 0.79]
8 Specific symptoms, self-rated at post-treatment	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
9 Specific symptoms, self-rated at post-treatment (type of control)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
9.1 Adjunct	1	93	Std. Mean Difference (Random, 95% CI)	0.31 [-0.10, 0.73]
9.2 Attention controls	19	873	Std. Mean Difference (Random, 95% CI)	0.68 [0.47, 0.89]
9.3 Treatment as Usual	2	118	Std. Mean Difference (Random, 95% CI)	0.02 [-0.35, 0.40]
9.4 Wait-List	37	2108	Std. Mean Difference (Random, 95% CI)	0.74 [0.60, 0.88]
10 Specific symptoms, self-rated at post-treatment (by disorder)	58	3201	Std. Mean Difference (Random, 95% CI)	0.69 [0.57, 0.80]
10.1 GAD	8	514	Std. Mean Difference (Random, 95% CI)	0.68 [0.29, 1.07]
10.2 Health	2	120	Std. Mean Difference (Random, 95% CI)	0.85 [-0.50, 2.21]
10.3 Mixed	4	231	Std. Mean Difference (Random, 95% CI)	0.21 [-0.15, 0.57]
10.4 OCD	2	220	Std. Mean Difference (Random, 95% CI)	0.68 [0.41, 0.95]
10.5 Panic	20	740	Std. Mean Difference (Random, 95% CI)	0.61 [0.43, 0.79]
10.6 Social	15	1153	Std. Mean Difference (Random, 95% CI)	0.76 [0.63, 0.89]
10.7 Specific	8	223	Std. Mean Difference (Random, 95% CI)	0.99 [0.50, 1.48]
11 Specific symptoms, self-rated at post-treatment (by media)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
11.1 Book	19	691	Std. Mean Difference (Random, 95% CI)	0.53 [0.32, 0.74]
11.2 Computer	7	261	Std. Mean Difference (Random, 95% CI)	0.79 [0.29, 1.28]
11.3 Internet	29	1829	Std. Mean Difference (Random, 95% CI)	0.77 [0.61, 0.92]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
11.4 Other	4	411	Std. Mean Difference (Random, 95% CI)	0.60 [0.39, 0.82]
12 Specific symptoms, self-rated at post-treatment (by therapy)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
12.1 CBT	44	2662	Std. Mean Difference (Random, 95% CI)	0.67 [0.54, 0.80]
12.2 Exposure/ Desensitization	13	440	Std. Mean Difference (Random, 95% CI)	0.77 [0.46, 1.08]
12.3 Muscle relaxation	2	90	Std. Mean Difference (Random, 95% CI)	0.66 [0.22, 1.09]
13 Specific symptoms, self-rated at post-treatment (by setting)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
13.1 Advertising	48	2749	Std. Mean Difference (Random, 95% CI)	0.74 [0.62, 0.86]
13.2 Referral Only	9	350	Std. Mean Difference (Random, 95% CI)	0.38 [0.09, 0.67]
13.3 Other	2	93	Std. Mean Difference (Random, 95% CI)	0.53 [-0.25, 1.31]
14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
14.1 High or Unclear Risk for Incomplete Outcome Data	23	949	Std. Mean Difference (Random, 95% CI)	0.60 [0.44, 0.76]
14.2 Low Risk for Incomplete Outcome Data	36	2243	Std. Mean Difference (Random, 95% CI)	0.73 [0.57, 0.88]
15 Specific symptoms, self-rated at post-treatment (studies also reporting clinician rated)	59	3192	Std. Mean Difference (Random, 95% CI)	0.68 [0.57, 0.80]
15.1 Not reporting clinician rated	47	2674	Std. Mean Difference (Random, 95% CI)	0.68 [0.55, 0.81]
15.2 Also reporting clinician rated	12	518	Std. Mean Difference (Random, 95% CI)	0.70 [0.46, 0.95]
16 Specific symptoms, clinician-rated at post-treatment	15	629	Std. Mean Difference (Random, 95% CI)	1.04 [0.50, 1.58]
17 Specific symptoms, clinician-rated at post-treatment	15	629	Std. Mean Difference (Random, 95% CI)	1.04 [0.50, 1.58]

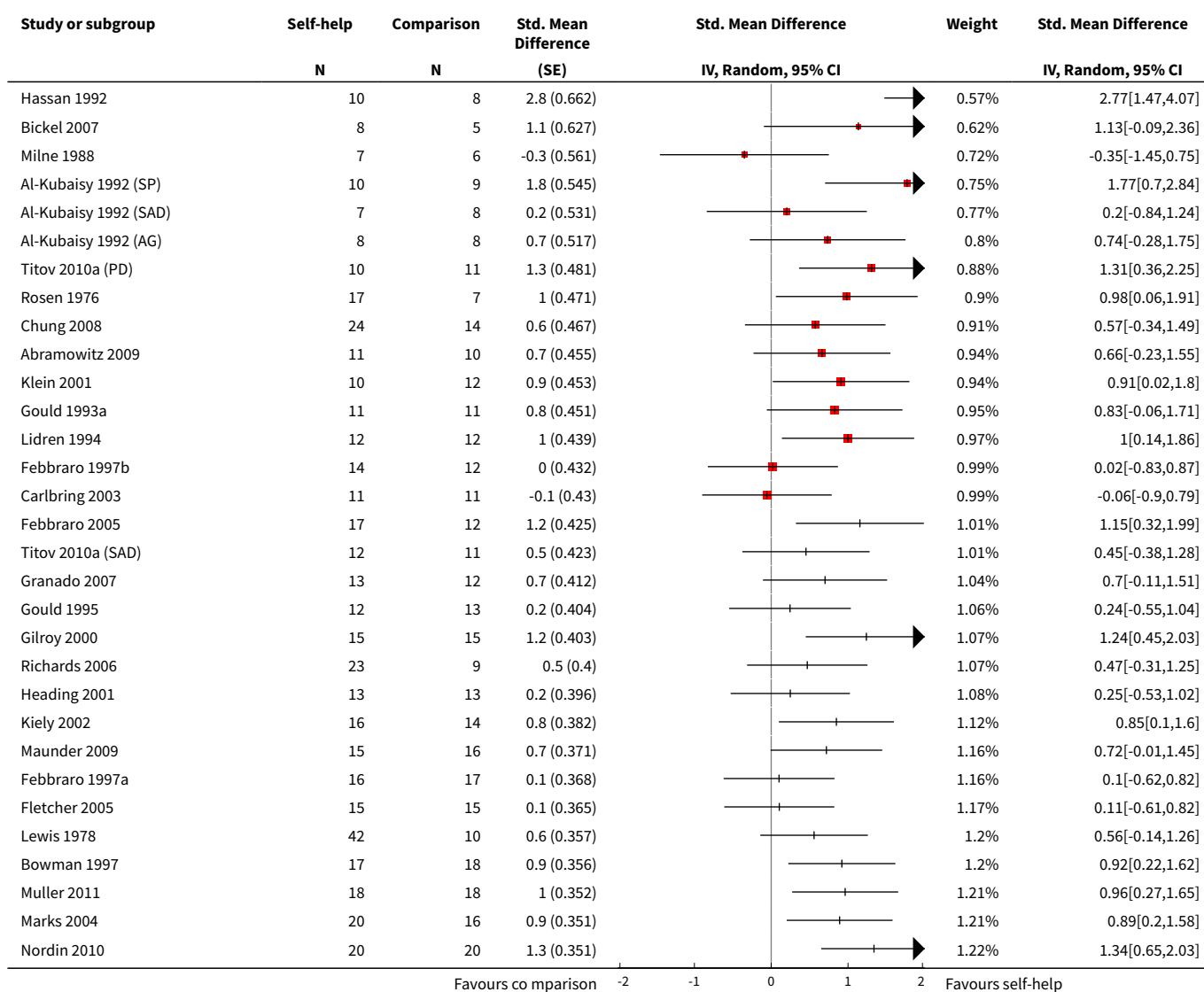
Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
17.1 Not reporting self-rated	3	112	Std. Mean Difference (Random, 95% CI)	1.39 [0.97, 1.80]
17.2 Also reporting self-rated	12	517	Std. Mean Difference (Random, 95% CI)	0.97 [0.32, 1.61]
18 General symptoms, self-rated at post-treatment	48	3101	Std. Mean Difference (Random, 95% CI)	0.58 [0.43, 0.73]
19 Behavioural Test (level), clinician-rated at post-treatment	7	250	Std. Mean Difference (Random, 95% CI)	1.11 [0.36, 1.87]
20 Behavioural Test (symptoms), self-rated at post-treatment	5	196	Std. Mean Difference (Random, 95% CI)	0.59 [-0.33, 1.51]
21 Response (specific), self-rated at post-treatment	21	1547	Risk Ratio (M-H, Random, 95% CI)	2.34 [1.81, 3.03]
22 Response (specific), self-rated at post-treatment (type of control)	21	1547	Risk Ratio (M-H, Random, 95% CI)	2.34 [1.81, 3.03]
22.1 Adjunct	1	95	Risk Ratio (M-H, Random, 95% CI)	1.43 [0.91, 2.25]
22.2 Attention controls	7	526	Risk Ratio (M-H, Random, 95% CI)	1.85 [1.39, 2.45]
22.3 Treatment as Usual	1	120	Risk Ratio (M-H, Random, 95% CI)	1.33 [0.49, 3.61]
22.4 Wait-List	12	806	Risk Ratio (M-H, Random, 95% CI)	3.18 [2.16, 4.68]
23 Response (specific), self-rated at post-treatment (by disorder)	21	1751	Risk Ratio (M-H, Random, 95% CI)	2.42 [1.88, 3.12]
23.1 GAD	4	342	Risk Ratio (M-H, Random, 95% CI)	4.60 [2.75, 7.68]
23.2 Mixed	2	161	Risk Ratio (M-H, Random, 95% CI)	2.00 [1.23, 3.24]
23.3 OCD	1	149	Risk Ratio (M-H, Random, 95% CI)	2.36 [1.16, 4.82]
23.4 Panic	9	384	Risk Ratio (M-H, Random, 95% CI)	1.73 [1.29, 2.30]
23.5 Social	4	695	Risk Ratio (M-H, Random, 95% CI)	2.85 [1.59, 5.11]
23.6 Specific	1	20	Risk Ratio (M-H, Random, 95% CI)	17.50 [1.16, 263.03]
24 Response (specific), self-rated at post-treatment (by media)	21	1751	Risk Ratio (M-H, Random, 95% CI)	2.42 [1.88, 3.12]
24.1 Book	7	250	Risk Ratio (M-H, Random, 95% CI)	1.85 [1.36, 2.52]
24.2 Computer	2	115	Risk Ratio (M-H, Random, 95% CI)	3.75 [0.25, 56.38]

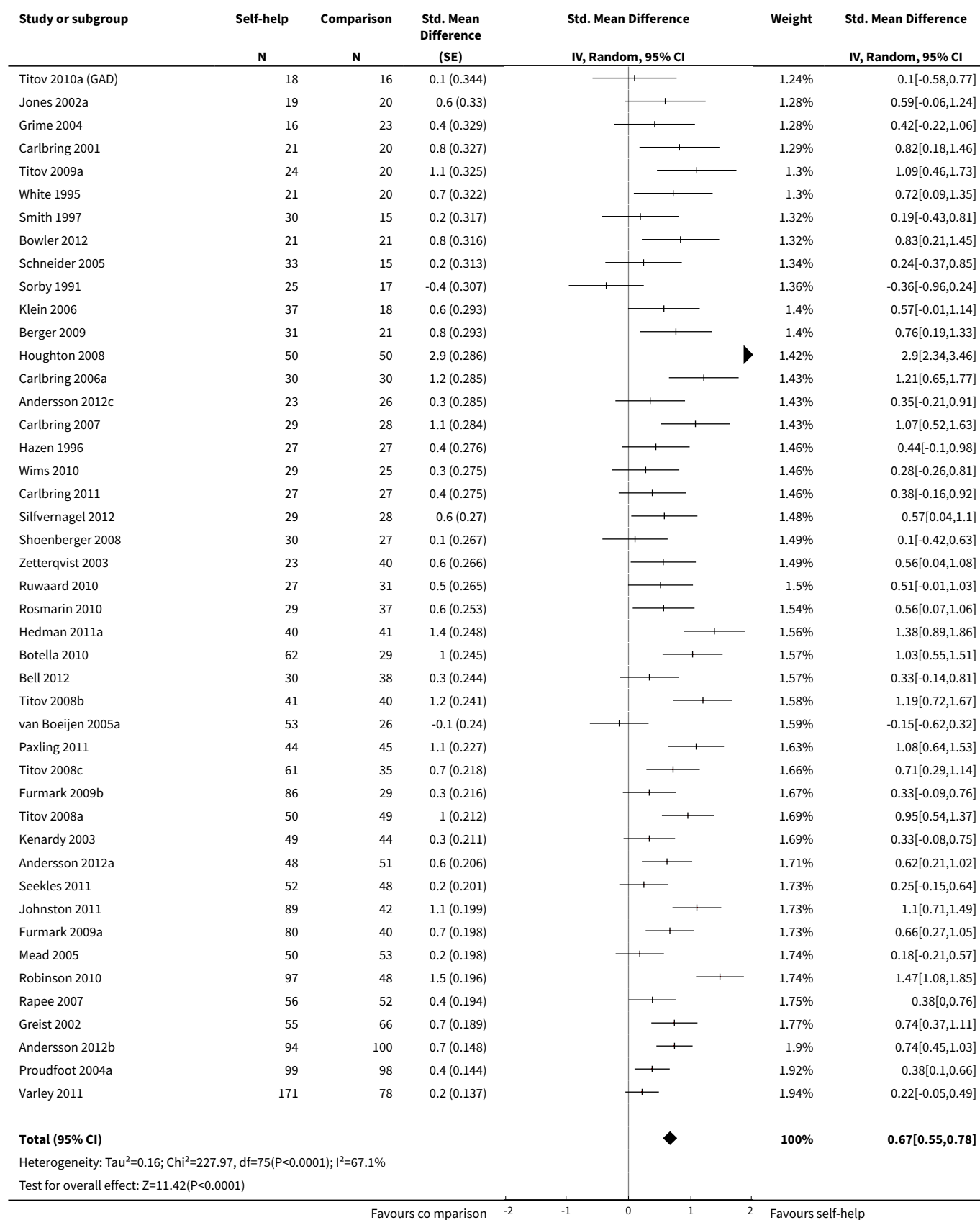
Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
24.3 Internet	8	882	Risk Ratio (M-H, Random, 95% CI)	3.76 [2.84, 4.97]
24.4 Other	4	504	Risk Ratio (M-H, Random, 95% CI)	1.79 [1.08, 2.99]
25 Response (specific), self-rated at post-treatment (by therapy)	21	1547	Risk Ratio (M-H, Random, 95% CI)	2.34 [1.81, 3.03]
25.1 Applied relaxation	0	0	Risk Ratio (M-H, Random, 95% CI)	0.0 [0.0, 0.0]
25.2 CBT	19	1378	Risk Ratio (M-H, Random, 95% CI)	2.30 [1.76, 3.01]
25.3 Exposure/ Desensitization	2	169	Risk Ratio (M-H, Random, 95% CI)	4.36 [0.63, 29.98]
25.4 Muscle relaxation	0	0	Risk Ratio (M-H, Random, 95% CI)	0.0 [0.0, 0.0]
25.5 Self-examination	0	0	Risk Ratio (M-H, Random, 95% CI)	0.0 [0.0, 0.0]
26 Response (specific), self-rated at post-treatment (by setting)	21	1547	Risk Ratio (M-H, Random, 95% CI)	2.34 [1.81, 3.03]
26.1 Advertising	19	1386	Risk Ratio (M-H, Random, 95% CI)	2.43 [1.82, 3.25]
26.2 Referral Only	2	161	Risk Ratio (M-H, Random, 95% CI)	2.00 [1.23, 3.24]
26.3 Other	0	0	Risk Ratio (M-H, Random, 95% CI)	0.0 [0.0, 0.0]
27 Response (specific), clinician-rated at post-treatment	11	922	Risk Ratio (M-H, Random, 95% CI)	4.90 [3.12, 7.68]
28 Response (general), self-rated at post-treatment	8	622	Risk Ratio (M-H, Random, 95% CI)	2.11 [1.13, 3.93]
29 Recovery, clinician-rated at post-treatment	9	605	Risk Difference (M-H, Random, 95% CI)	0.40 [0.20, 0.60]
30 Depression, self-rated at post-treatment	51	3682	Std. Mean Difference (Random, 95% CI)	0.47 [0.36, 0.58]
31 Depression, clinician-rated at post-treatment	1	50	Std. Mean Difference (Random, 95% CI)	0.06 [-0.49, 0.60]
32 Depression improvement, self-rated at post-treatment	5	224	Risk Ratio (M-H, Random, 95% CI)	2.64 [1.36, 5.14]
33 Disability, self-rated at post-treatment	20	1629	Std. Mean Difference (Random, 95% CI)	0.48 [0.33, 0.63]
34 Disability, clinician-rated at post-treatment	5	293	Std. Mean Difference (Random, 95% CI)	0.56 [-0.01, 1.14]
35 Quality of Life, self-rated at post-treatment	18	1344	Std. Mean Difference (Random, 95% CI)	0.36 [0.22, 0.49]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
36 Dropout at post-treatment	78	6059	Risk Ratio (M-H, Random, 95% CI)	0.96 [0.94, 0.99]
37 Anxiety (any measure), self-rated at follow-up	15	684	Std. Mean Difference (Random, 95% CI)	0.49 [0.24, 0.75]
38 Specific symptoms, self-rated at follow-up	12	439	Std. Mean Difference (Random, 95% CI)	0.61 [0.27, 0.94]
39 Specific symptoms, clinician-rated at follow-up	5	98	Std. Mean Difference (Random, 95% CI)	1.12 [0.70, 1.53]
40 General symptoms, self-rated at follow-up	9	569	Std. Mean Difference (Random, 95% CI)	0.22 [0.06, 0.39]
41 Behavioural Test (level), clinician-rated at follow-up	1	30	Std. Mean Difference (Random, 95% CI)	1.23 [0.47, 1.99]
42 Behavioural Test (symptoms), self-rated at follow-up	1	30	Std. Mean Difference (Random, 95% CI)	0.60 [-0.11, 1.31]
43 Response (specific), self-rated at follow-up	2	128	Risk Ratio (M-H, Random, 95% CI)	2.12 [1.18, 3.79]
44 Response (specific), clinician-rated at follow-up	1	23	Risk Ratio (M-H, Random, 95% CI)	3.93 [0.21, 73.71]
45 Response (general), self-rated at follow-up	1	95	Risk Ratio (M-H, Random, 95% CI)	0.97 [0.51, 1.84]
46 Recovery, clinician-rated at follow-up	1	117	Risk Ratio (M-H, Random, 95% CI)	0.91 [0.64, 1.28]
47 Depression, self-rated at follow-up	7	526	Std. Mean Difference (Random, 95% CI)	0.28 [0.05, 0.52]
48 Depression, clinician-rated at follow-up	1	50	Std. Mean Difference (Random, 95% CI)	0.32 [-0.23, 0.87]
49 Disability, self-rated at follow-up	7	513	Std. Mean Difference (Random, 95% CI)	0.21 [-0.15, 0.57]
50 Disability, clinician-rated at follow-up	3	108	Std. Mean Difference (Random, 95% CI)	0.53 [0.14, 0.93]
51 Quality of Life, self-rated at follow-up	3	211	Std. Mean Difference (Random, 95% CI)	0.08 [-0.19, 0.35]
52 Anxiety (any measure), self-rated at >6m	5	287	Std. Mean Difference (Random, 95% CI)	0.18 [-0.15, 0.52]
53 Specific symptoms, self-rated at >6m	5	287	Std. Mean Difference (Random, 95% CI)	0.20 [-0.11, 0.51]
54 General symptoms, self-rated at >6m	2	194	Std. Mean Difference (Random, 95% CI)	-0.07 [-0.63, 0.49]

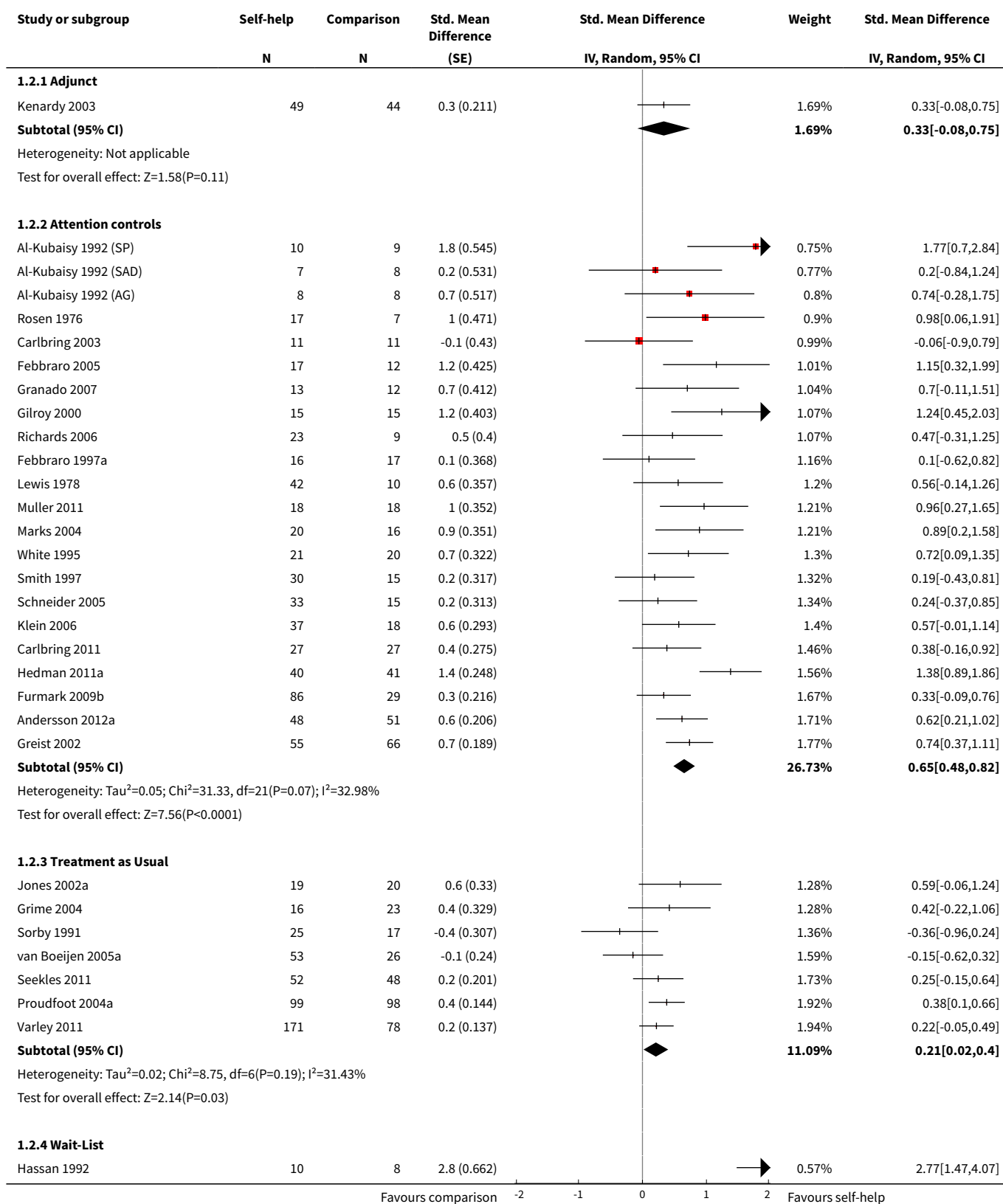
Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
55 Response (specific), self-rated at >6m	2	148	Risk Ratio (Random, 95% CI)	1.51 [0.67, 3.36]
56 Depression, self-rated at >6m	2	194	Std. Mean Difference (Random, 95% CI)	-0.25 [-0.58, 0.08]
57 Disability, self-rated at >6m	3	147	Std. Mean Difference (Random, 95% CI)	0.14 [-0.33, 0.60]
58 Quality of Life, self-rated at >6m	1	115	Std. Mean Difference (Random, 95% CI)	-0.06 [-0.48, 0.36]

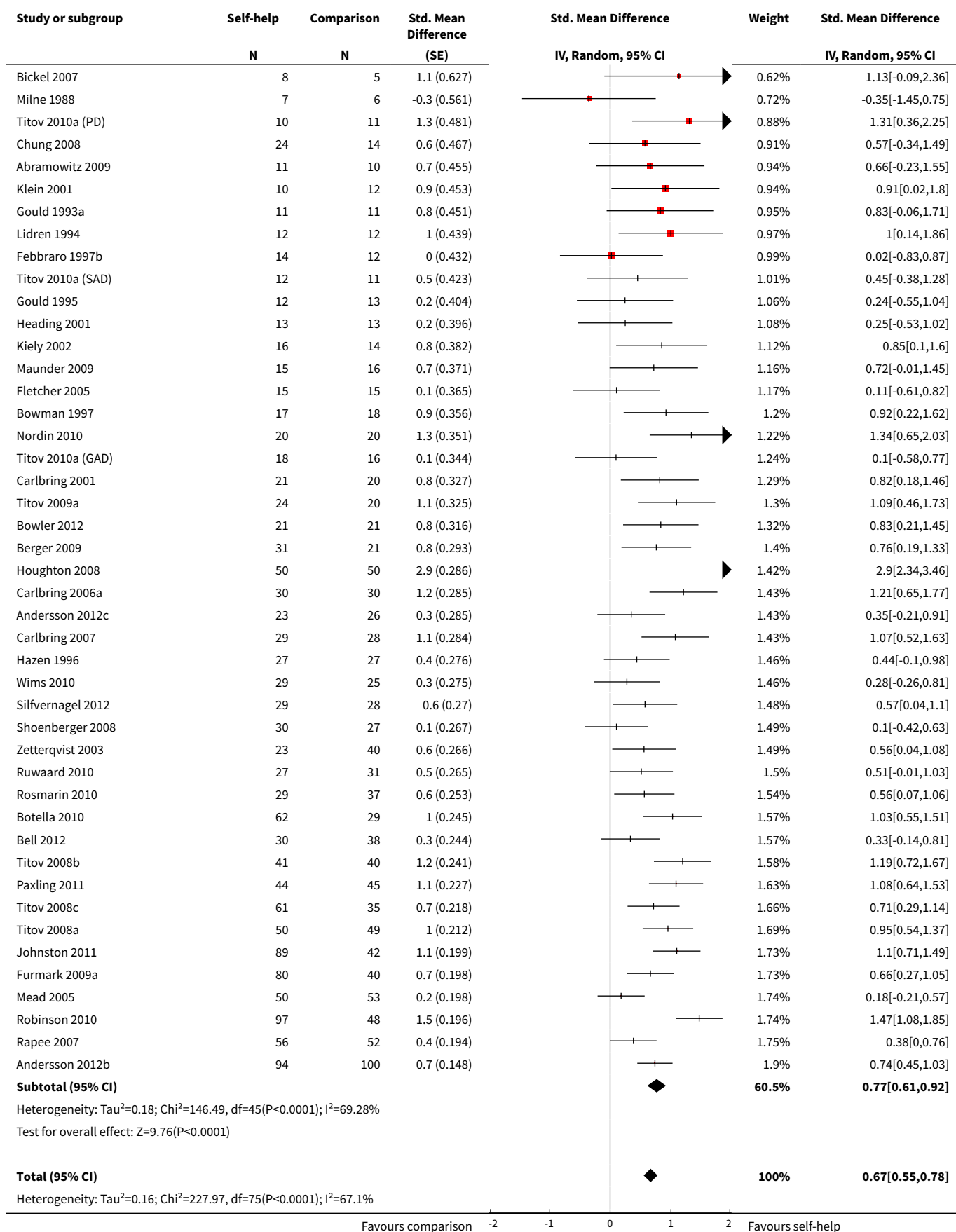
Analysis 1.1. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 1 Anxiety (any measure), self-rated at post-treatment.





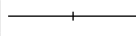
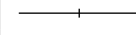
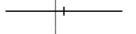
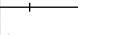
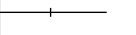
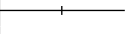
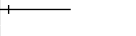
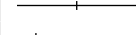
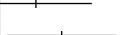
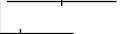
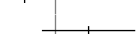
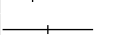
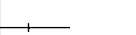
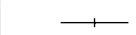
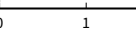
Analysis 1.2. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 2 Anxiety (any measure), self-rated at post-treatment (type of control).

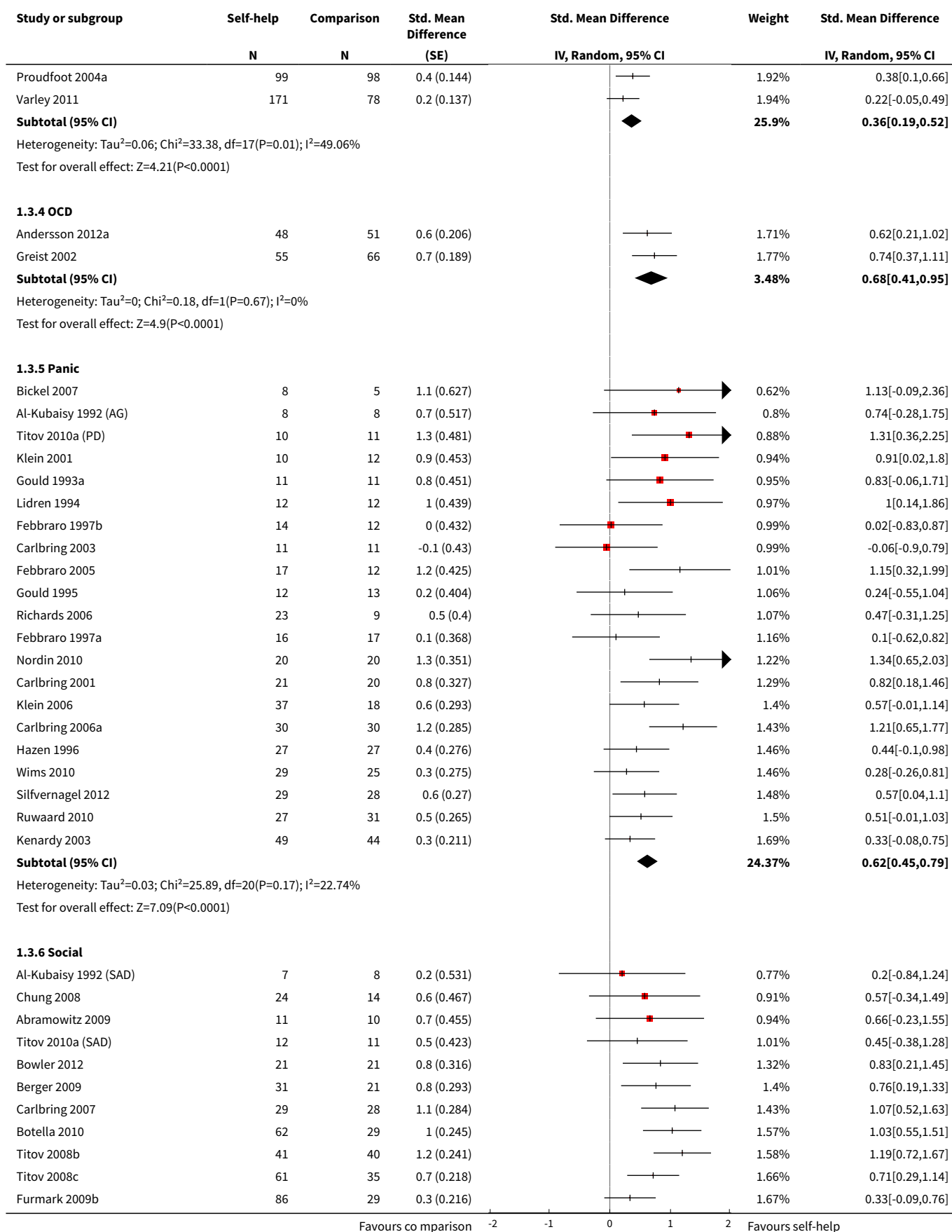


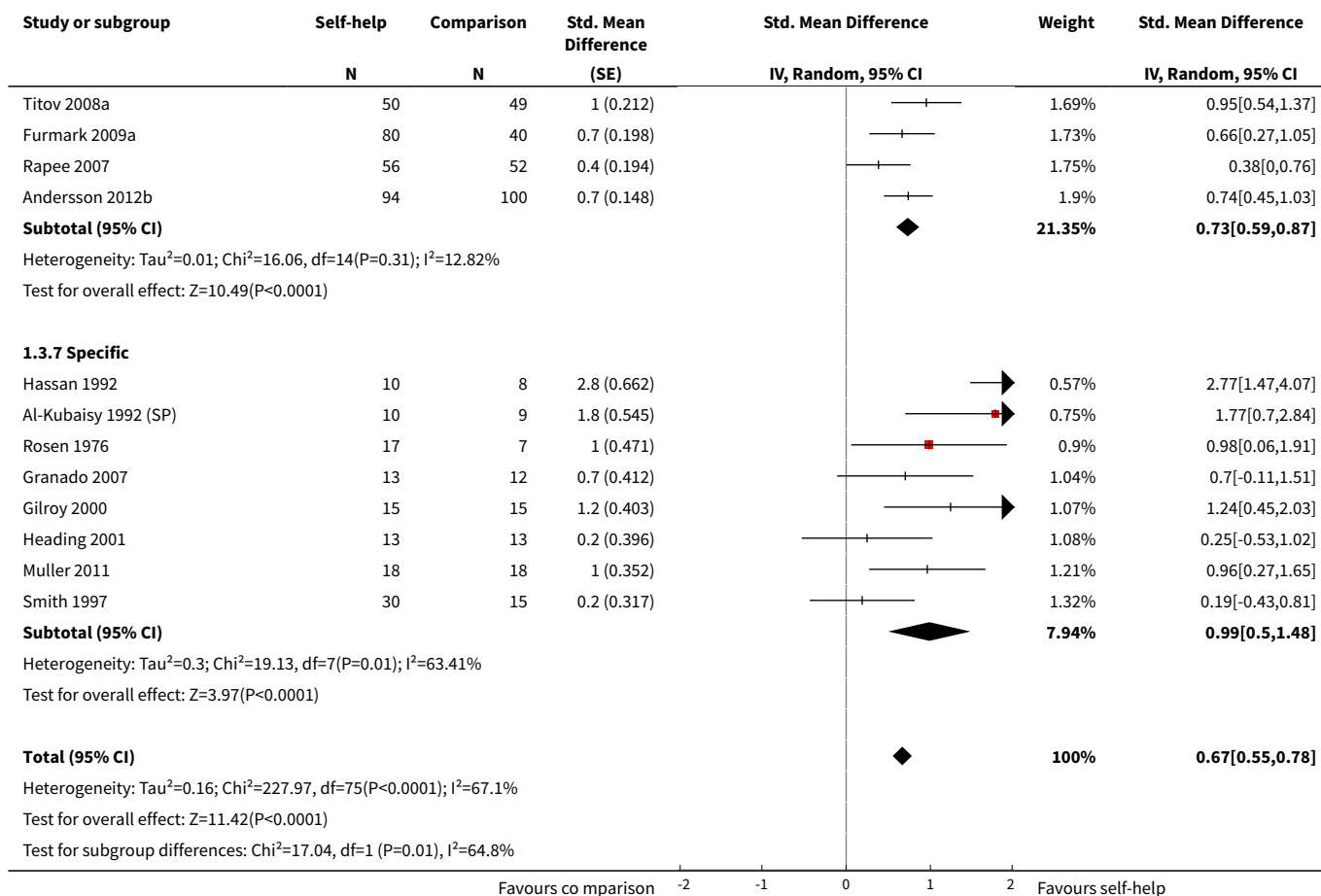


Study or subgroup	Self-help	Comparison	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
Test for overall effect: $Z=11.42(P<0.0001)$						
Test for subgroup differences: $\chi^2=21.84, df=1 (P<0.0001), I^2=86.26\%$						
Favours comparison -2 -1 0 1 2 Favours self-help						

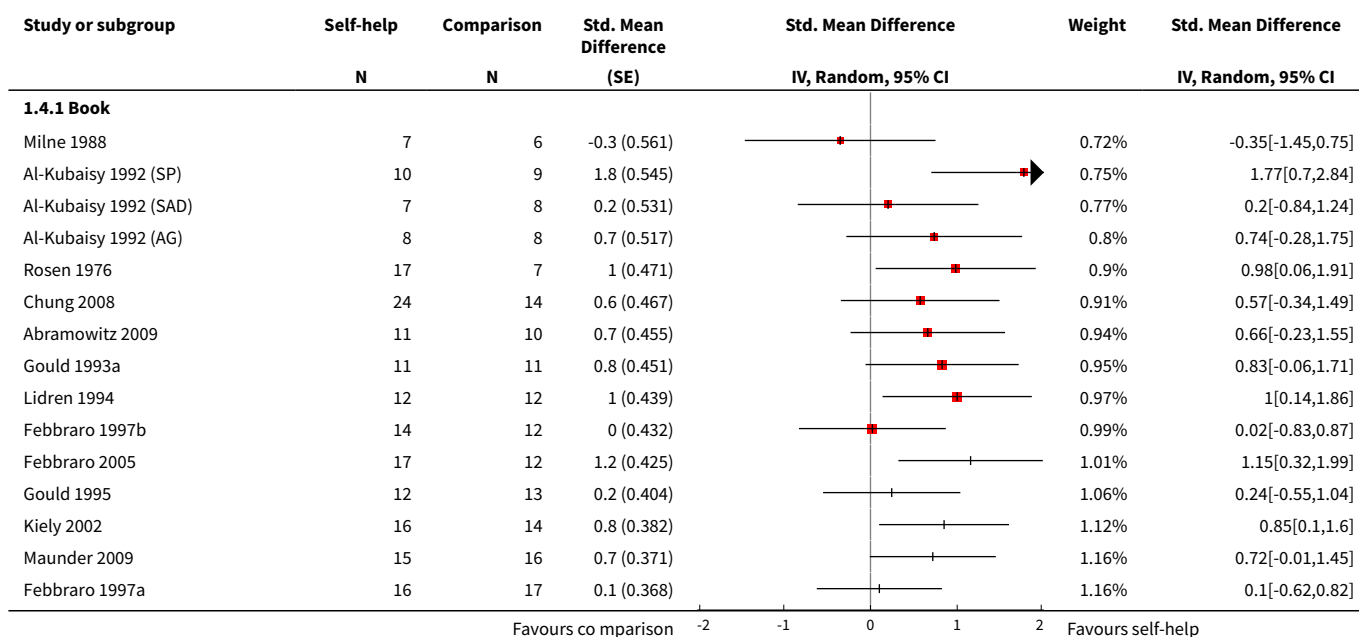
Analysis 1.3. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 3 Anxiety (any measure), self-rated at post-treatment (by disorder).

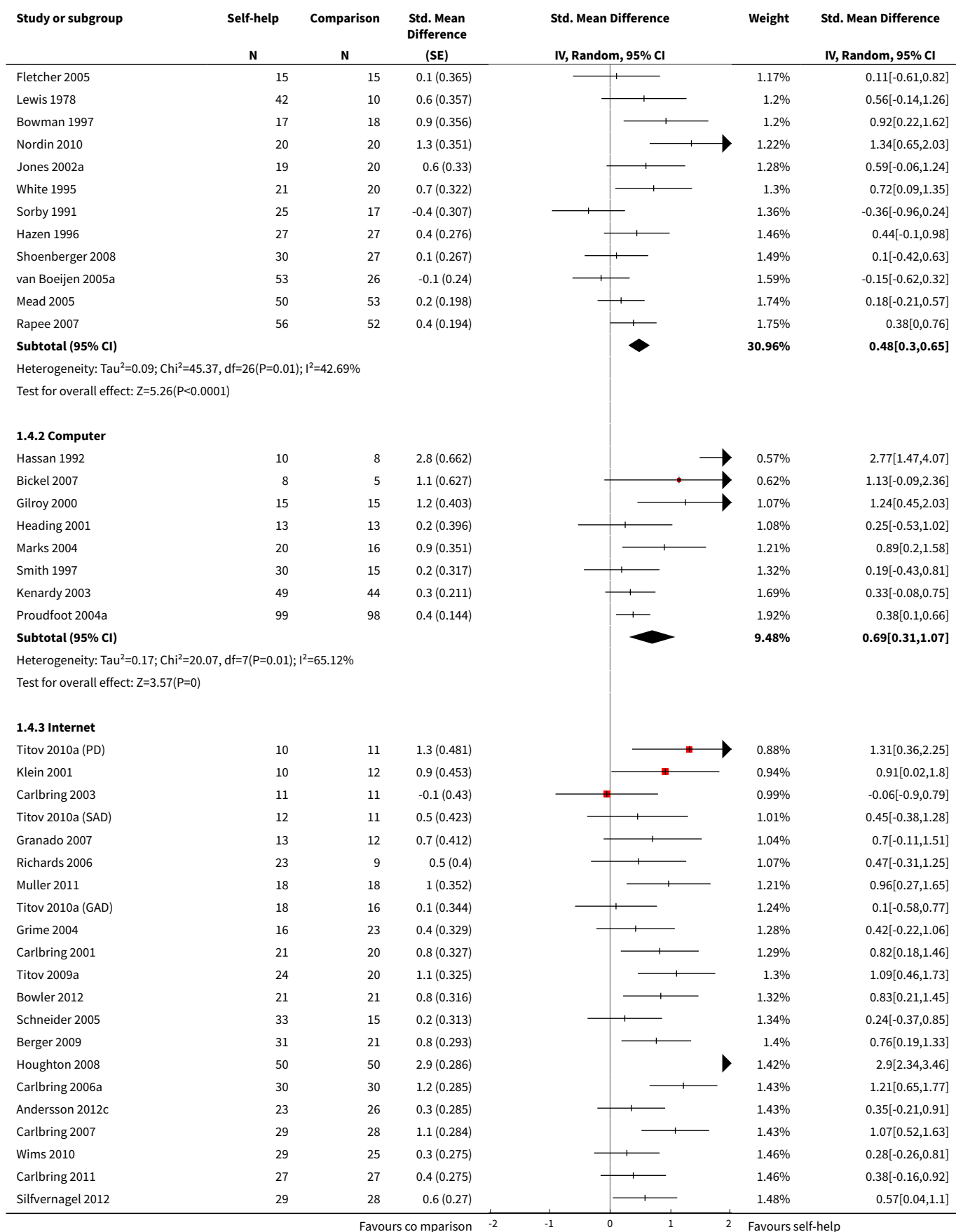
Study or subgroup	Self-help	Comparison	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
1.3.1 GAD						
Kiely 2002	16	14	0.8 (0.382)		1.12%	0.85[0.1,1.6]
Bowman 1997	17	18	0.9 (0.356)		1.2%	0.92[0.22,1.62]
Titov 2010a (GAD)	18	16	0.1 (0.344)		1.24%	0.1[-0.58,0.77]
Titov 2009a	24	20	1.1 (0.325)		1.3%	1.09[0.46,1.73]
Houghton 2008	50	50	2.9 (0.286)		1.42%	2.9[2.34,3.46]
Andersson 2012c	23	26	0.3 (0.285)		1.43%	0.35[-0.21,0.91]
Shoenberger 2008	30	27	0.1 (0.267)		1.49%	0.1[-0.42,0.63]
Rosmarin 2010	29	37	0.6 (0.253)		1.54%	0.56[0.07,1.06]
Paxling 2011	44	45	1.1 (0.227)		1.63%	1.08[0.64,1.53]
Robinson 2010	97	48	1.5 (0.196)		1.74%	1.47[1.08,1.85]
Subtotal (95% CI)					14.12%	0.95[0.44,1.45]
Heterogeneity: $\tau^2=0.58$; $\chi^2=76.4, df=9(P<0.0001)$; $I^2=88.22\%$						
Test for overall effect: $Z=3.68(P=0)$						
1.3.2 Health						
Jones 2002a	19	20	0.6 (0.33)		1.28%	0.59[-0.06,1.24]
Hedman 2011a	40	41	1.4 (0.248)		1.56%	1.38[0.89,1.86]
Subtotal (95% CI)					2.84%	1.01[0.24,1.78]
Heterogeneity: $\tau^2=0.22$; $\chi^2=3.64, df=1(P=0.06)$; $I^2=72.54\%$						
Test for overall effect: $Z=2.58(P=0.01)$						
1.3.3 Mixed						
Milne 1988	7	6	-0.3 (0.561)		0.72%	-0.35[-1.45,0.75]
Maunder 2009	15	16	0.7 (0.371)		1.16%	0.72[-0.01,1.45]
Fletcher 2005	15	15	0.1 (0.365)		1.17%	0.11[-0.61,0.82]
Lewis 1978	42	10	0.6 (0.357)		1.2%	0.56[-0.14,1.26]
Marks 2004	20	16	0.9 (0.351)		1.21%	0.89[0.2,1.58]
Grime 2004	16	23	0.4 (0.329)		1.28%	0.42[-0.22,1.06]
White 1995	21	20	0.7 (0.322)		1.3%	0.72[0.09,1.35]
Schneider 2005	33	15	0.2 (0.313)		1.34%	0.24[-0.37,0.85]
Sorby 1991	25	17	-0.4 (0.307)		1.36%	-0.36[-0.96,0.24]
Carlbring 2011	27	27	0.4 (0.275)		1.46%	0.38[-0.16,0.92]
Zetterqvist 2003	23	40	0.6 (0.266)		1.49%	0.56[0.04,1.08]
Bell 2012	30	38	0.3 (0.244)		1.57%	0.33[-0.14,0.81]
van Boeijen 2005a	53	26	-0.1 (0.24)		1.59%	-0.15[-0.62,0.32]
Seekles 2011	52	48	0.2 (0.201)		1.73%	0.25[-0.15,0.64]
Johnston 2011	89	42	1.1 (0.199)		1.73%	1.1[0.71,1.49]
Mead 2005	50	53	0.2 (0.198)		1.74%	0.18[-0.21,0.57]
Favours comparison -2 -1 0 1 2 Favours self-help						

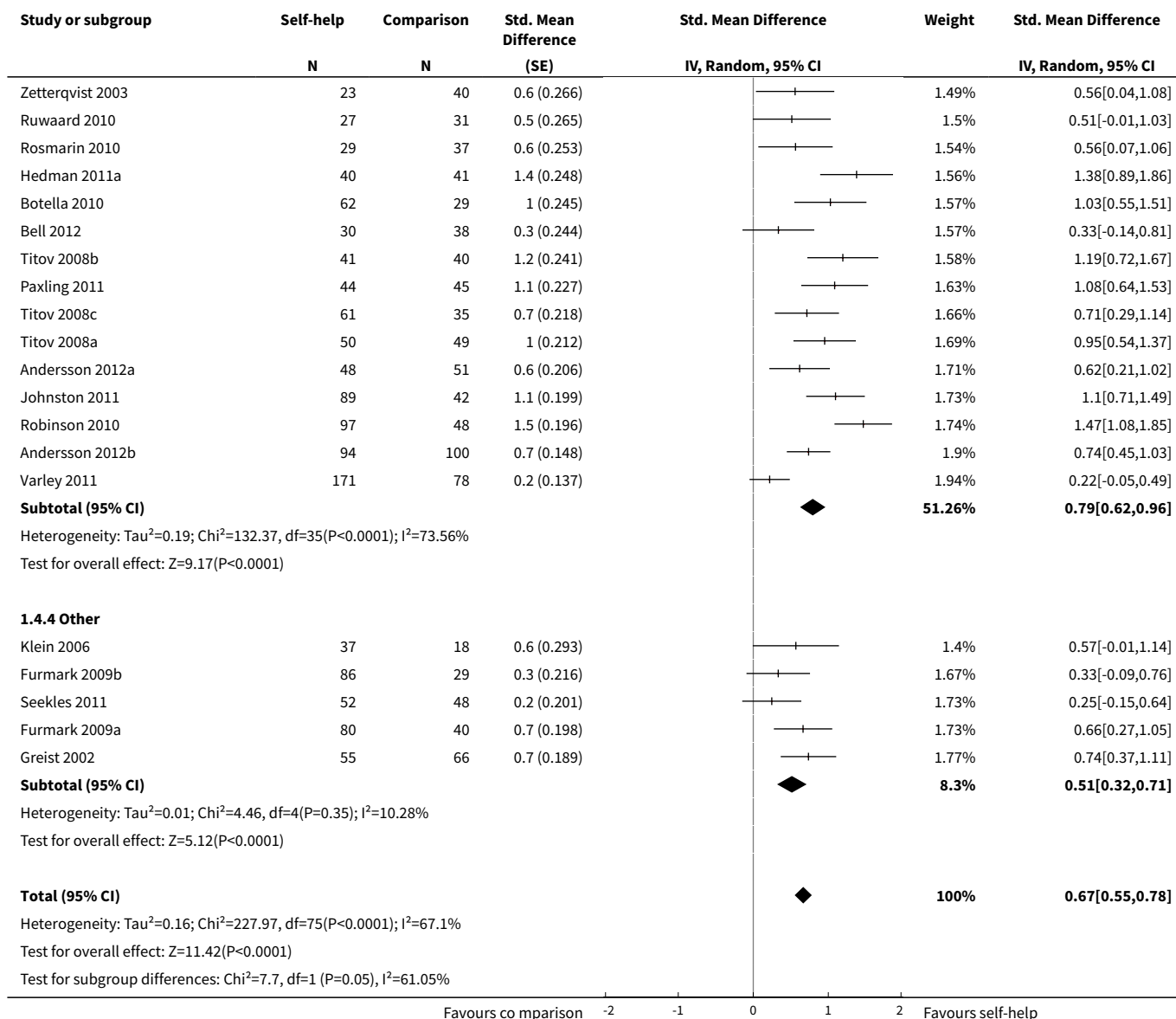




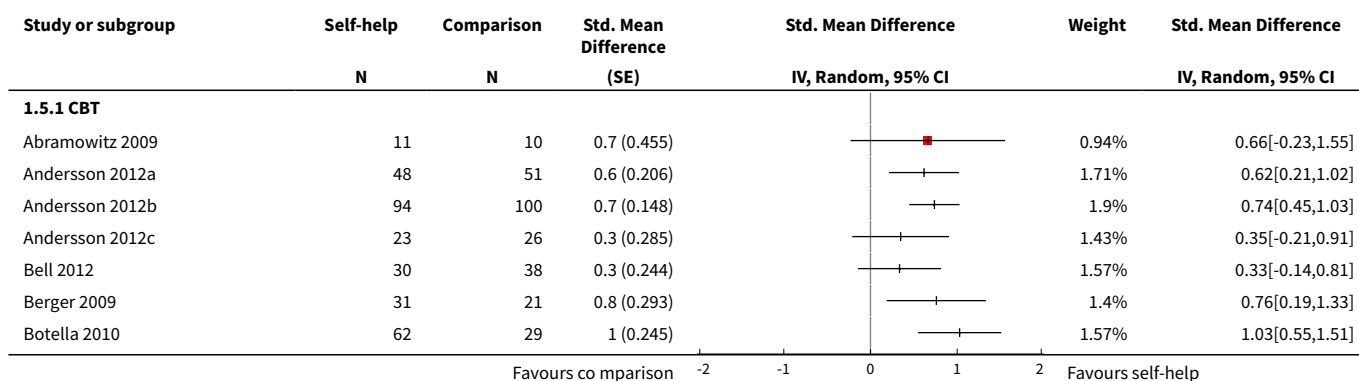
Analysis 1.4. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 4 Anxiety (any measure), self-rated at post-treatment (by media).

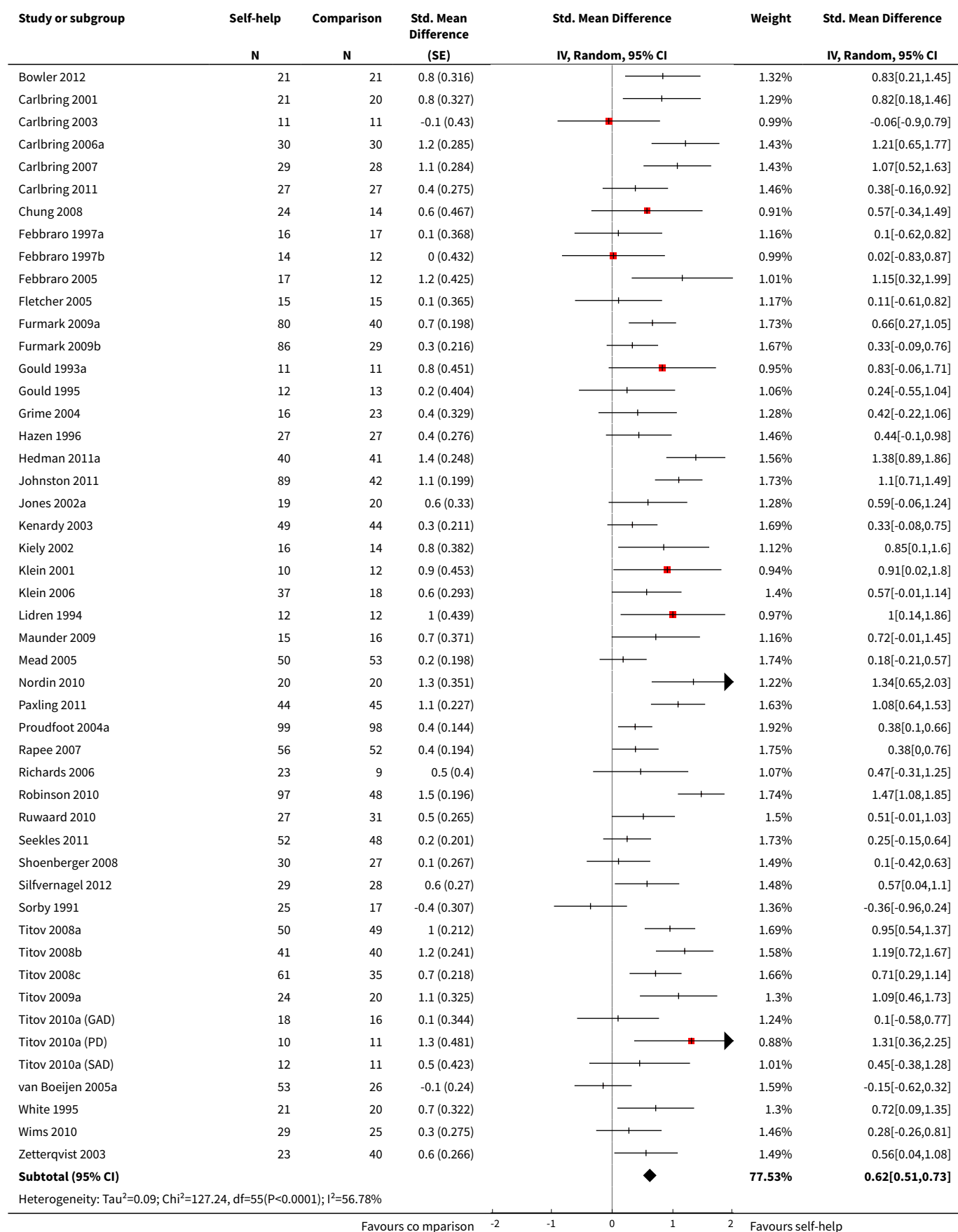


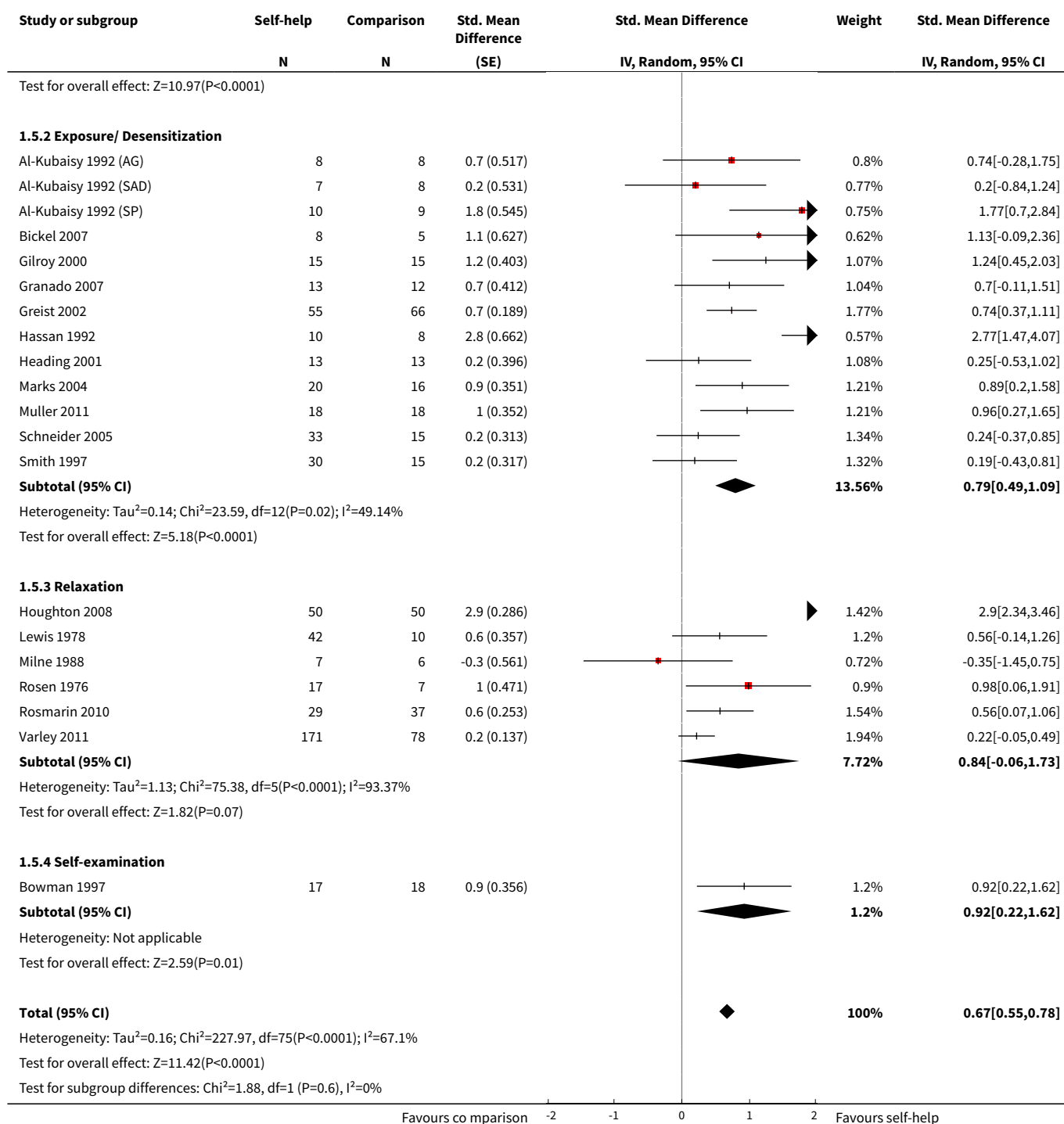




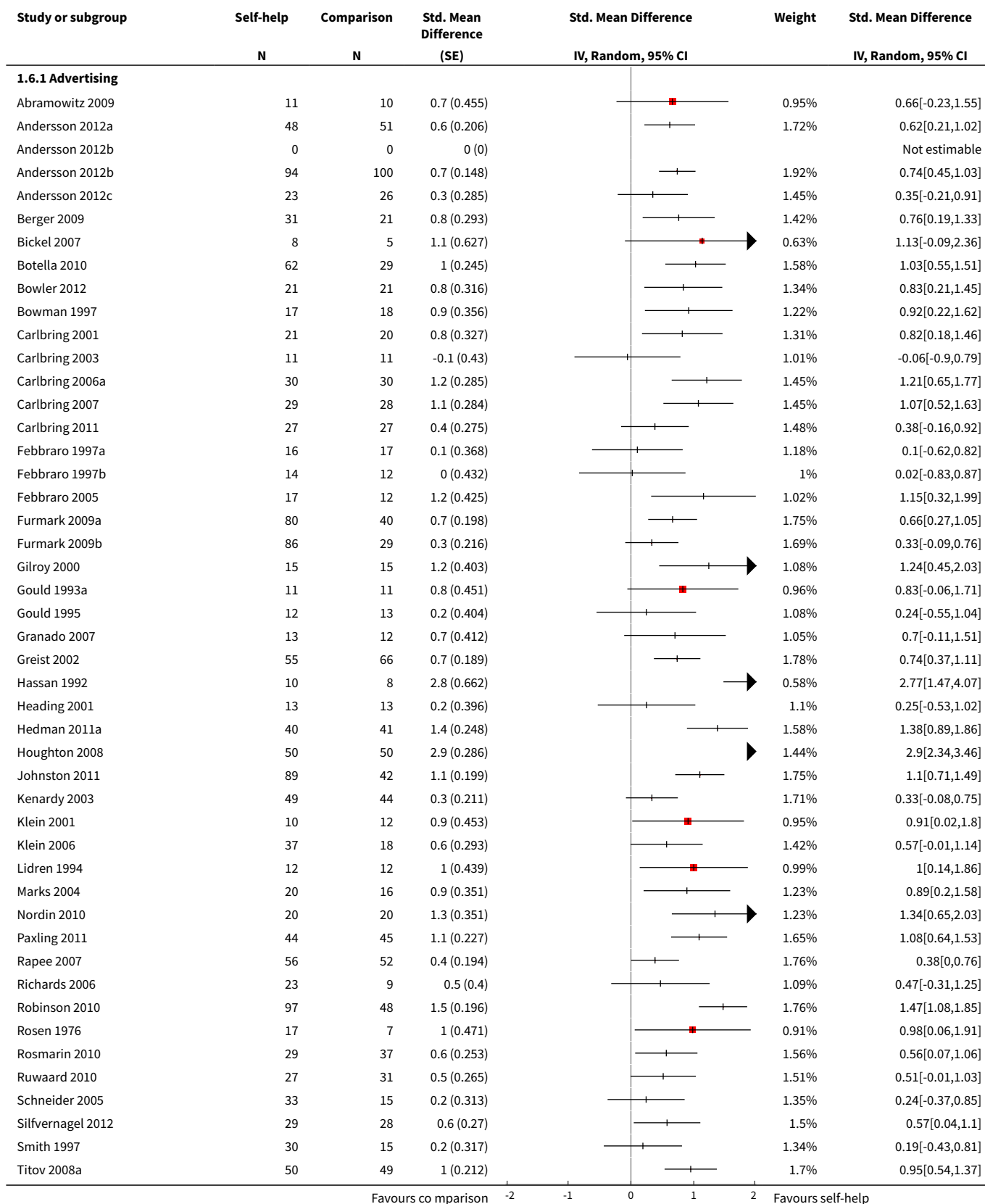
Analysis 1.5. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 5 Anxiety (any measure), self-rated at post-treatment (by therapy).

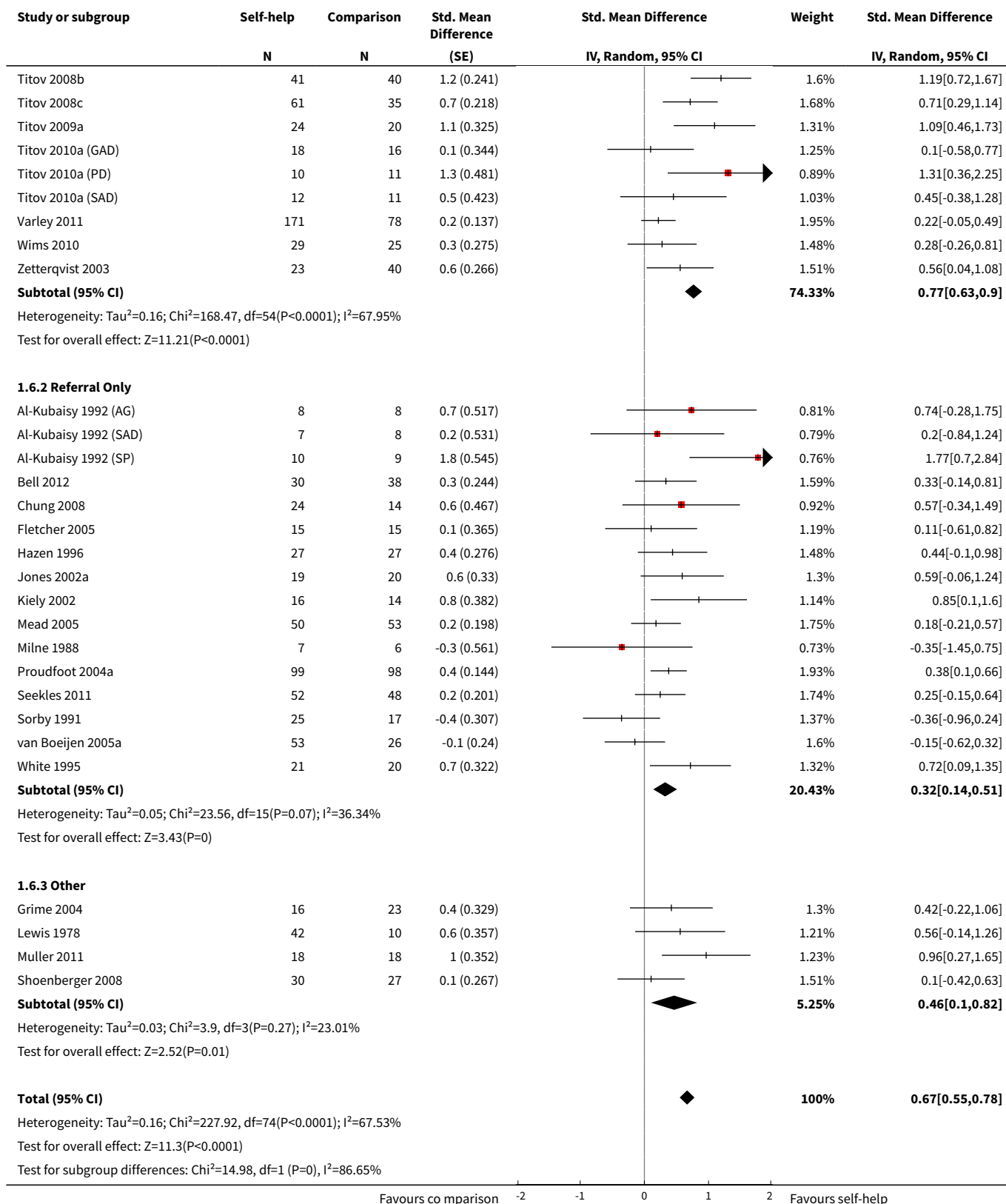




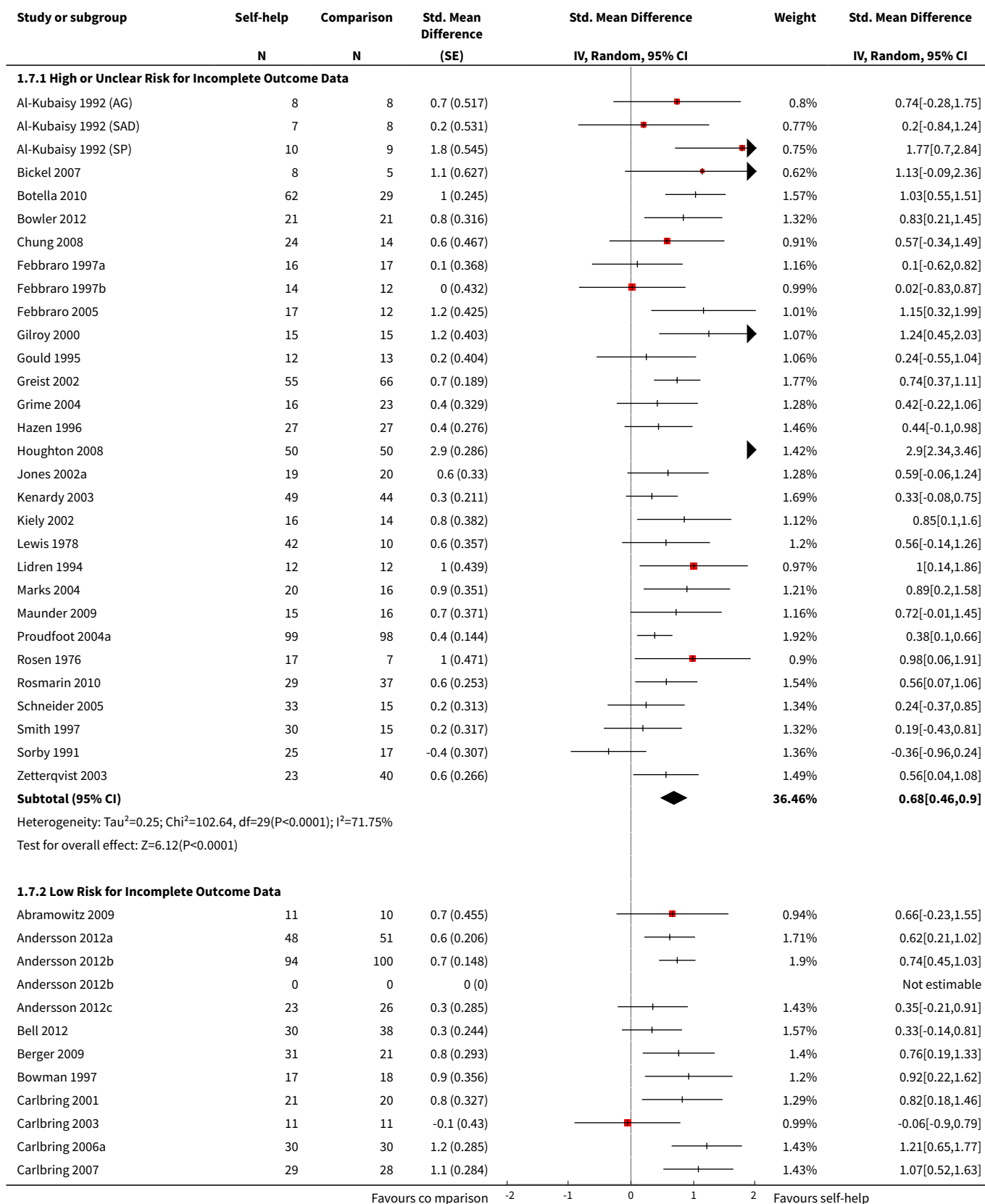


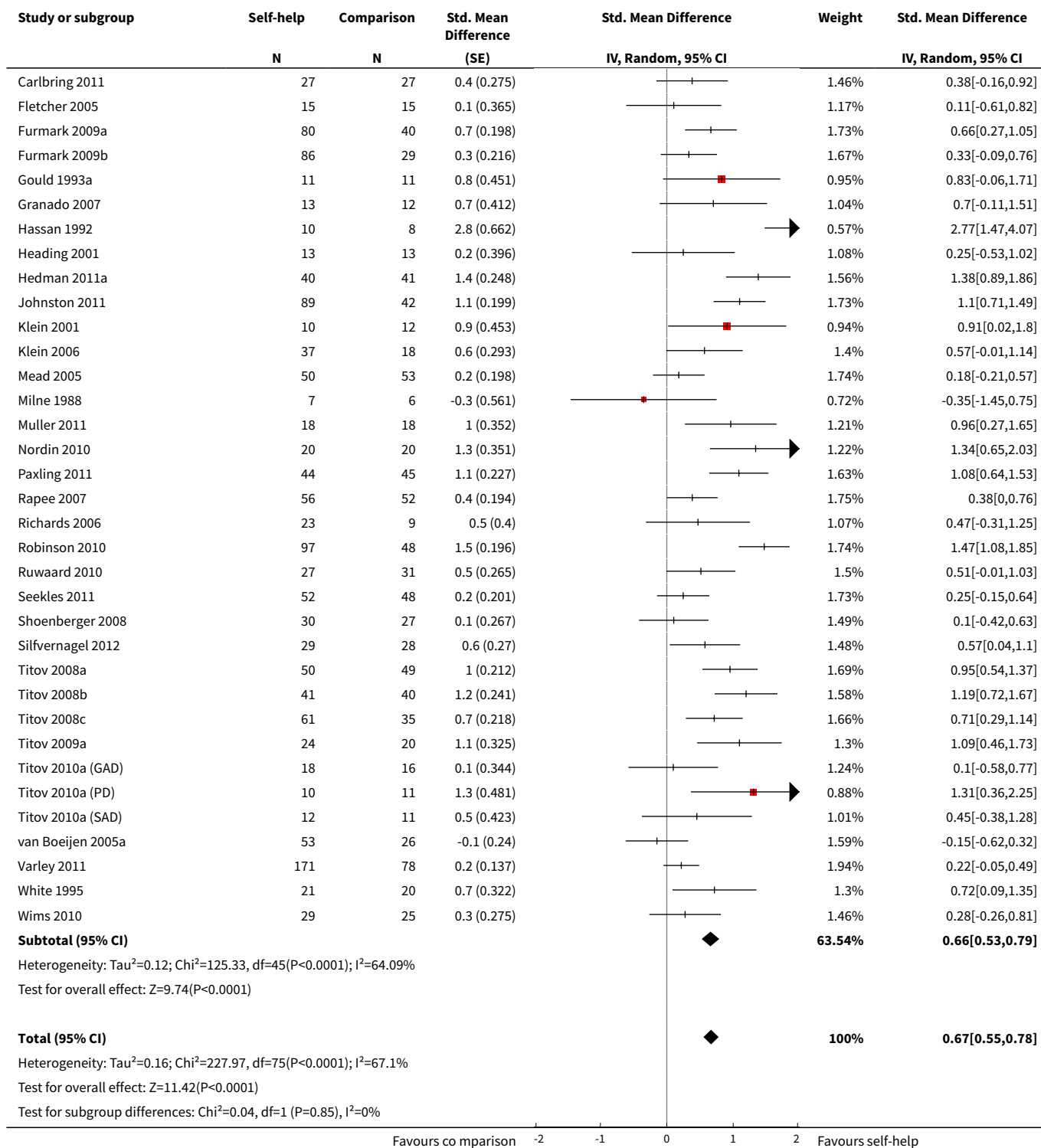
Analysis 1.6. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 6 Anxiety (any measure), self-rated at post-treatment (by setting).



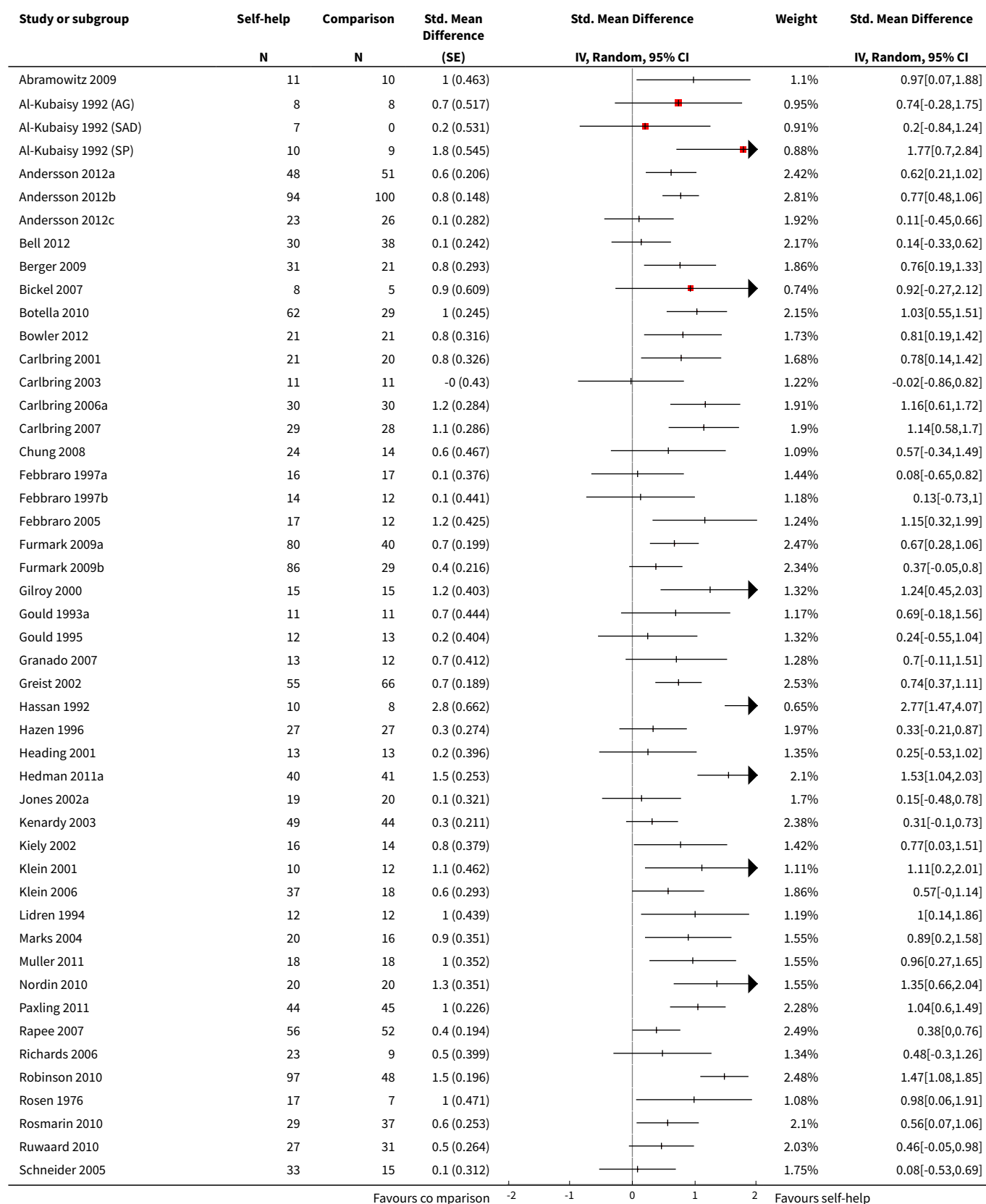


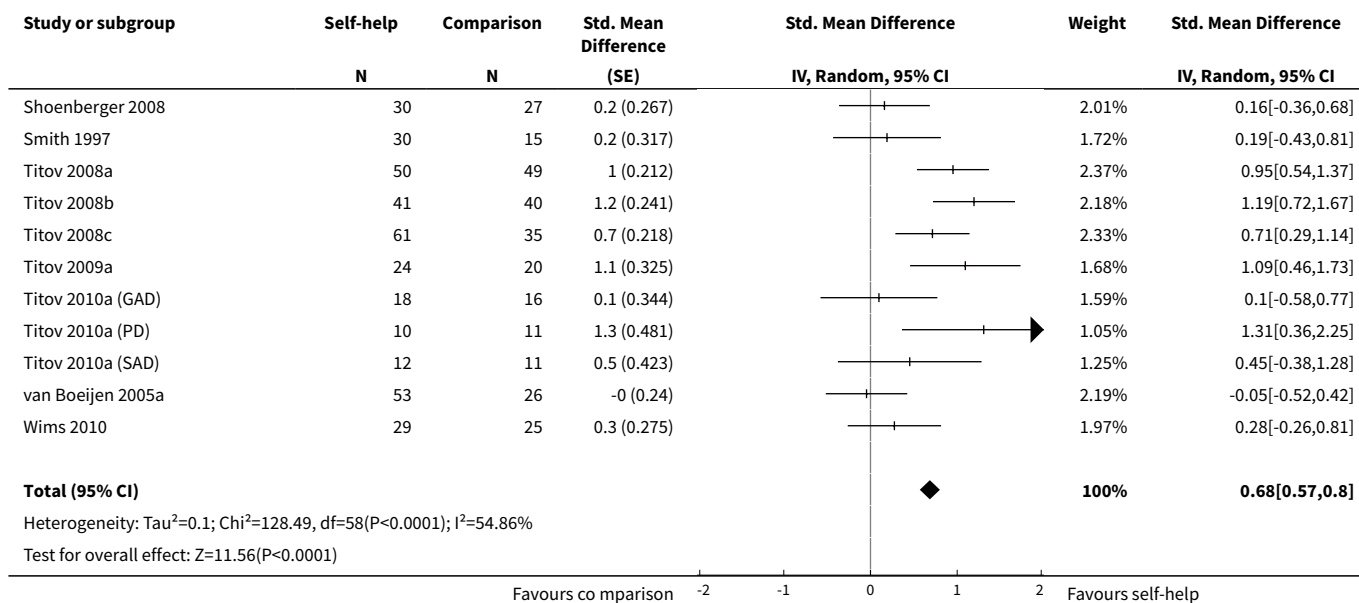
Analysis 1.7. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias).



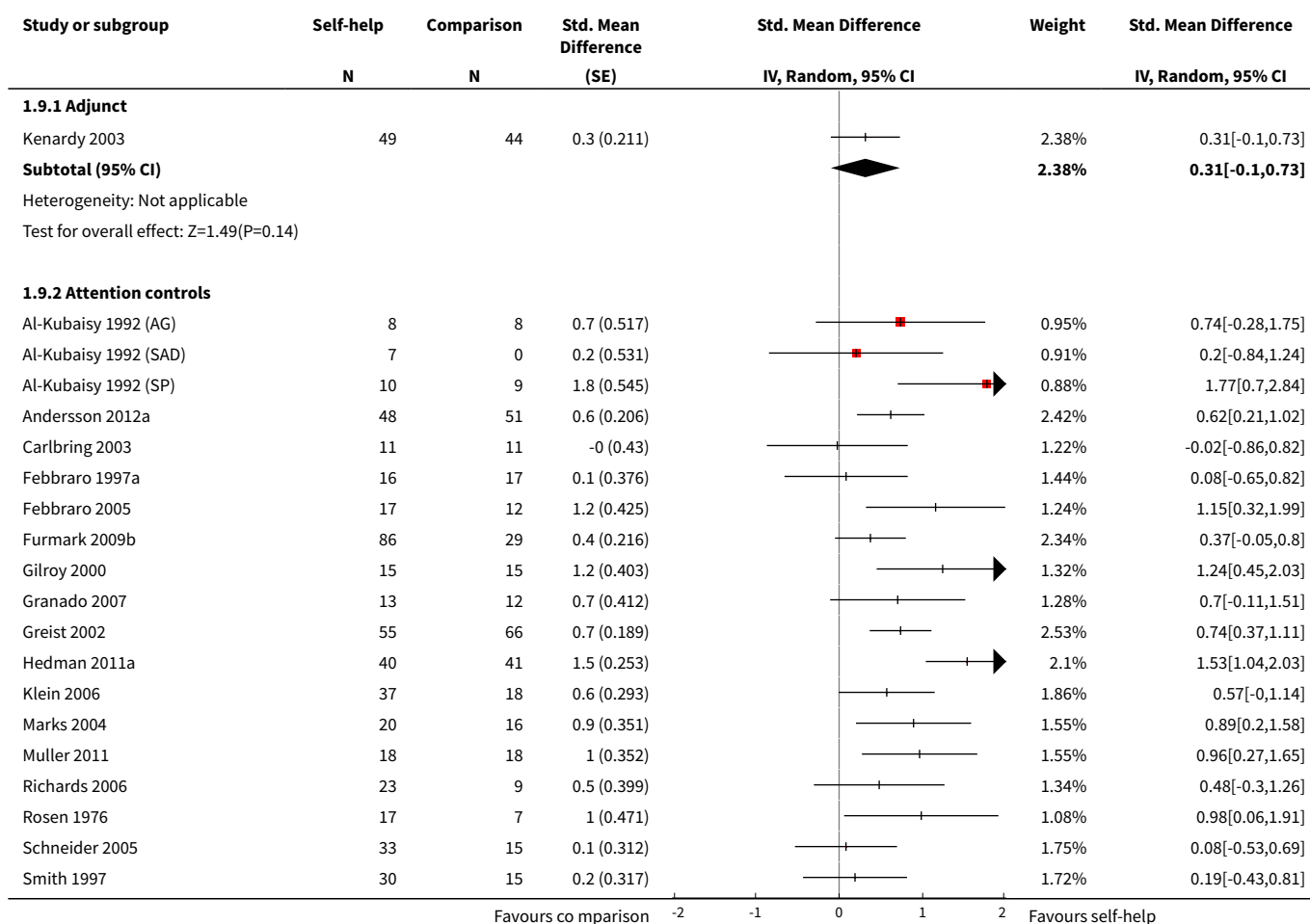


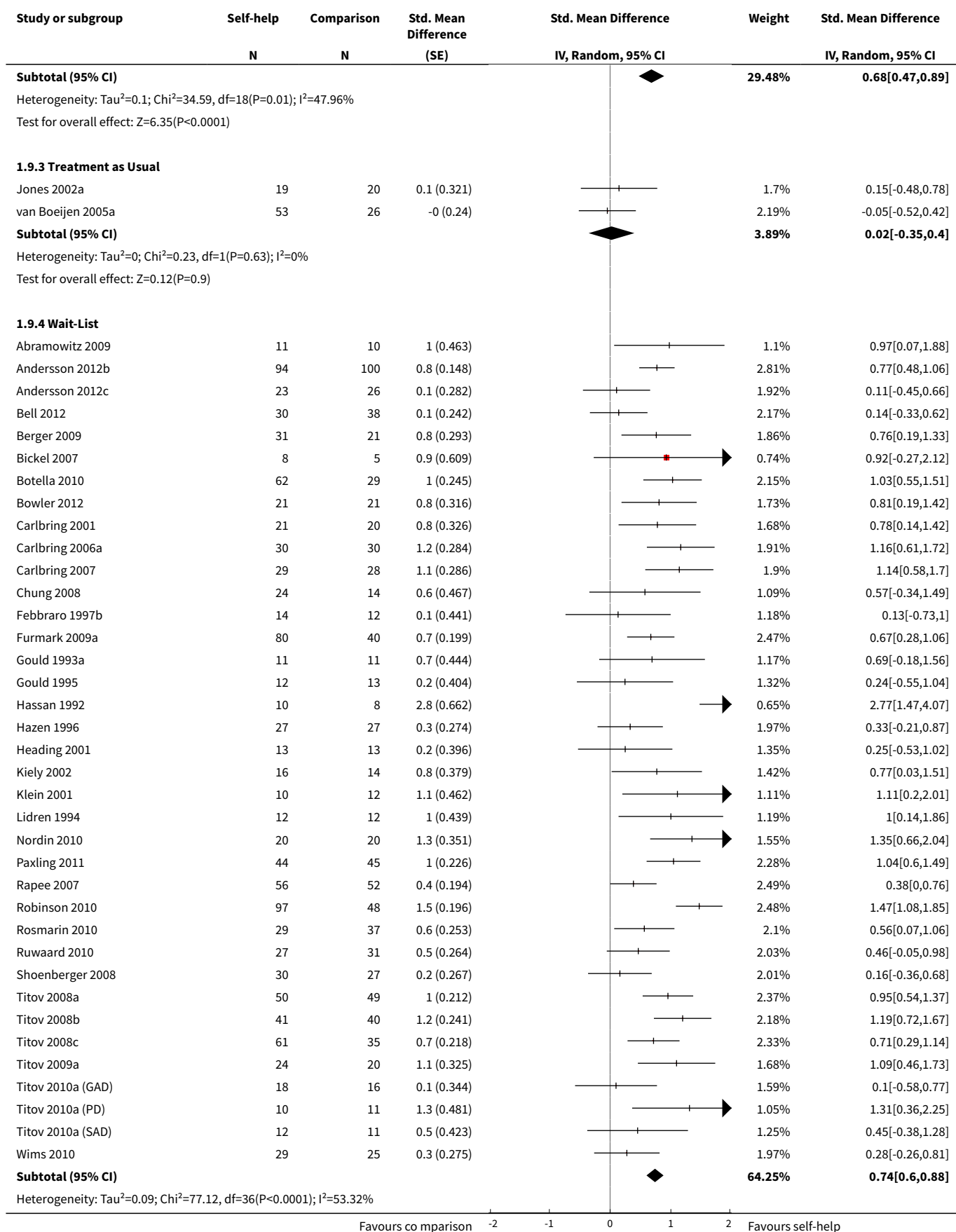
Analysis 1.8. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 8 Specific symptoms, self-rated at post-treatment.

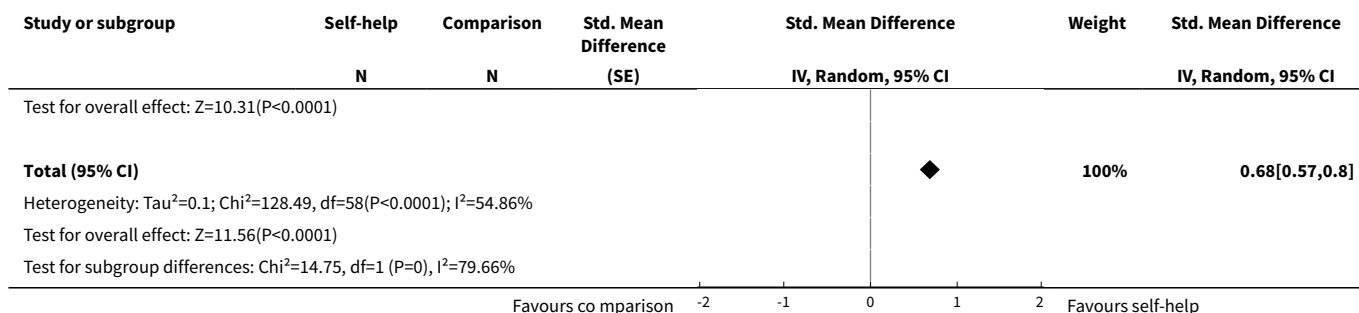




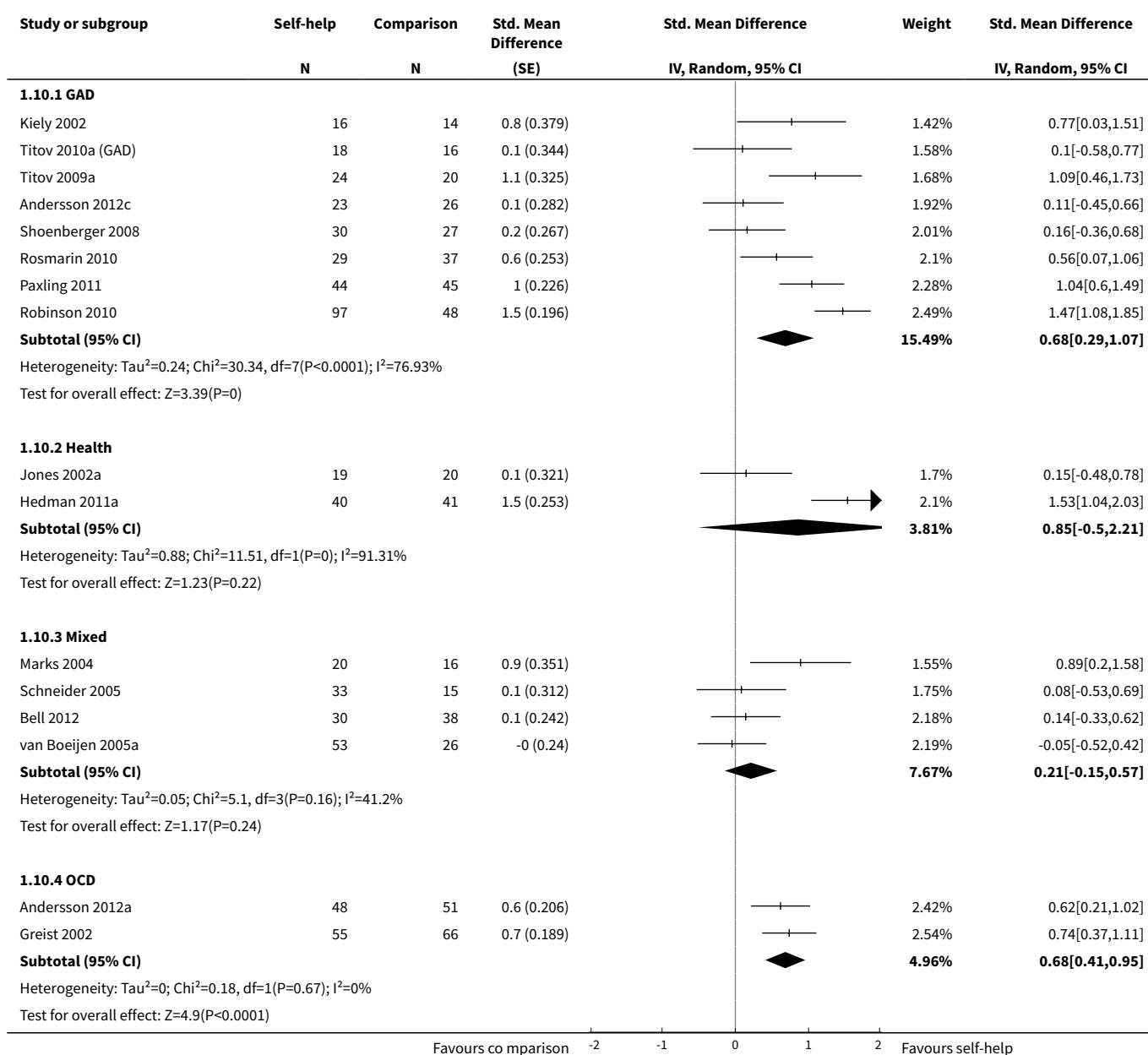
Analysis 1.9. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 9 Specific symptoms, self-rated at post-treatment (type of control).

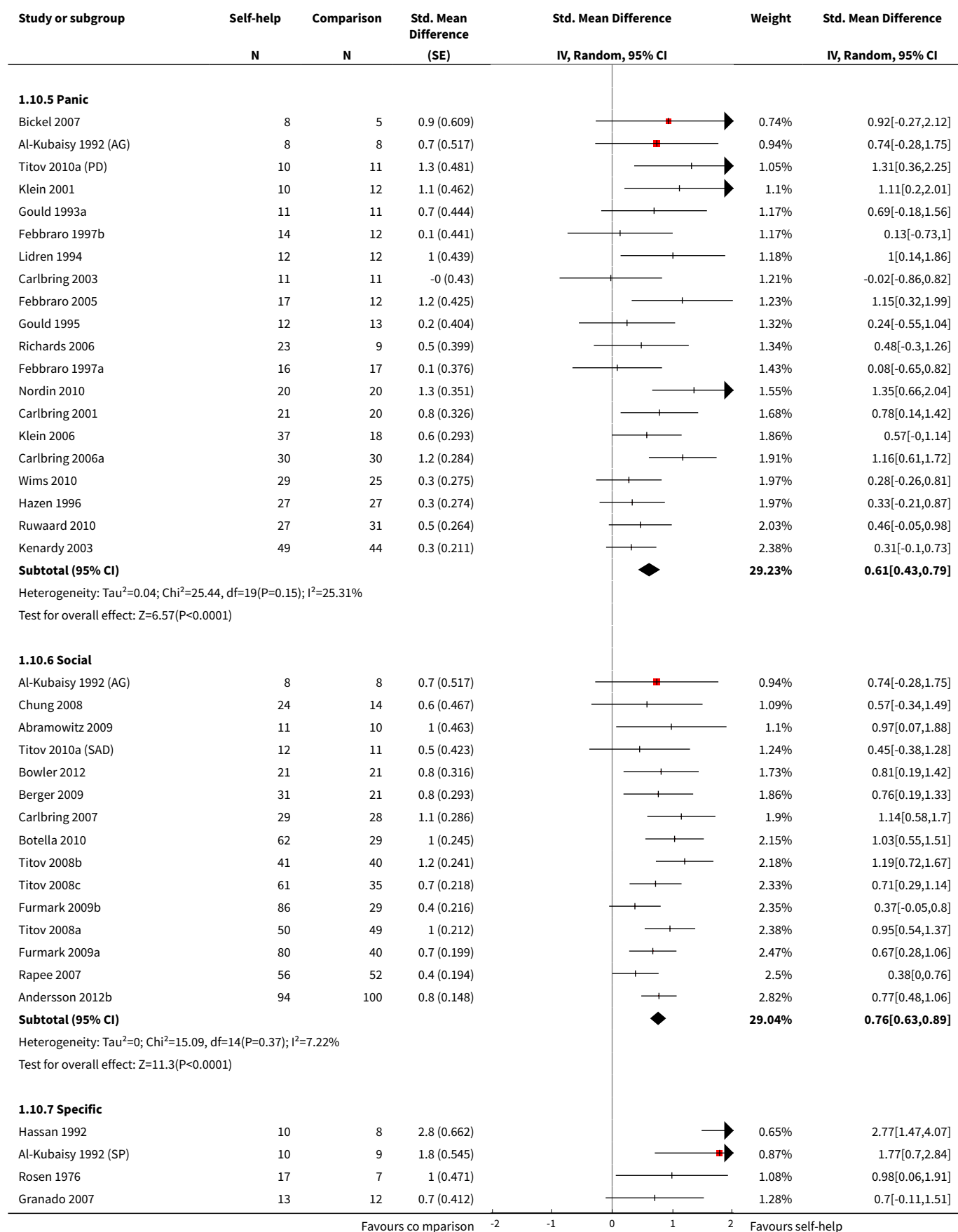


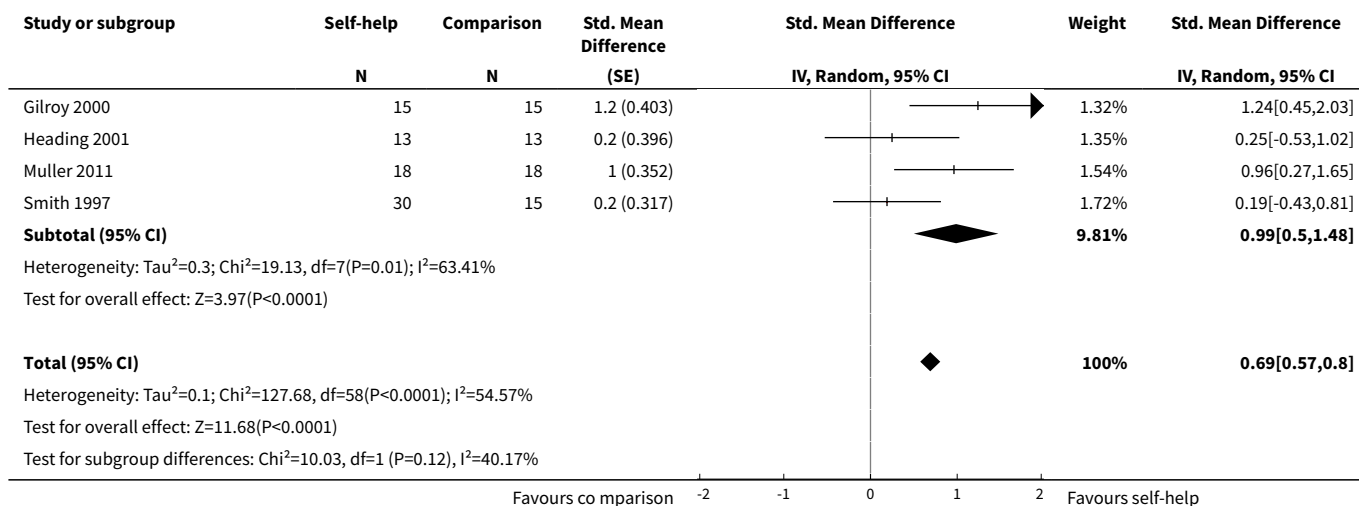




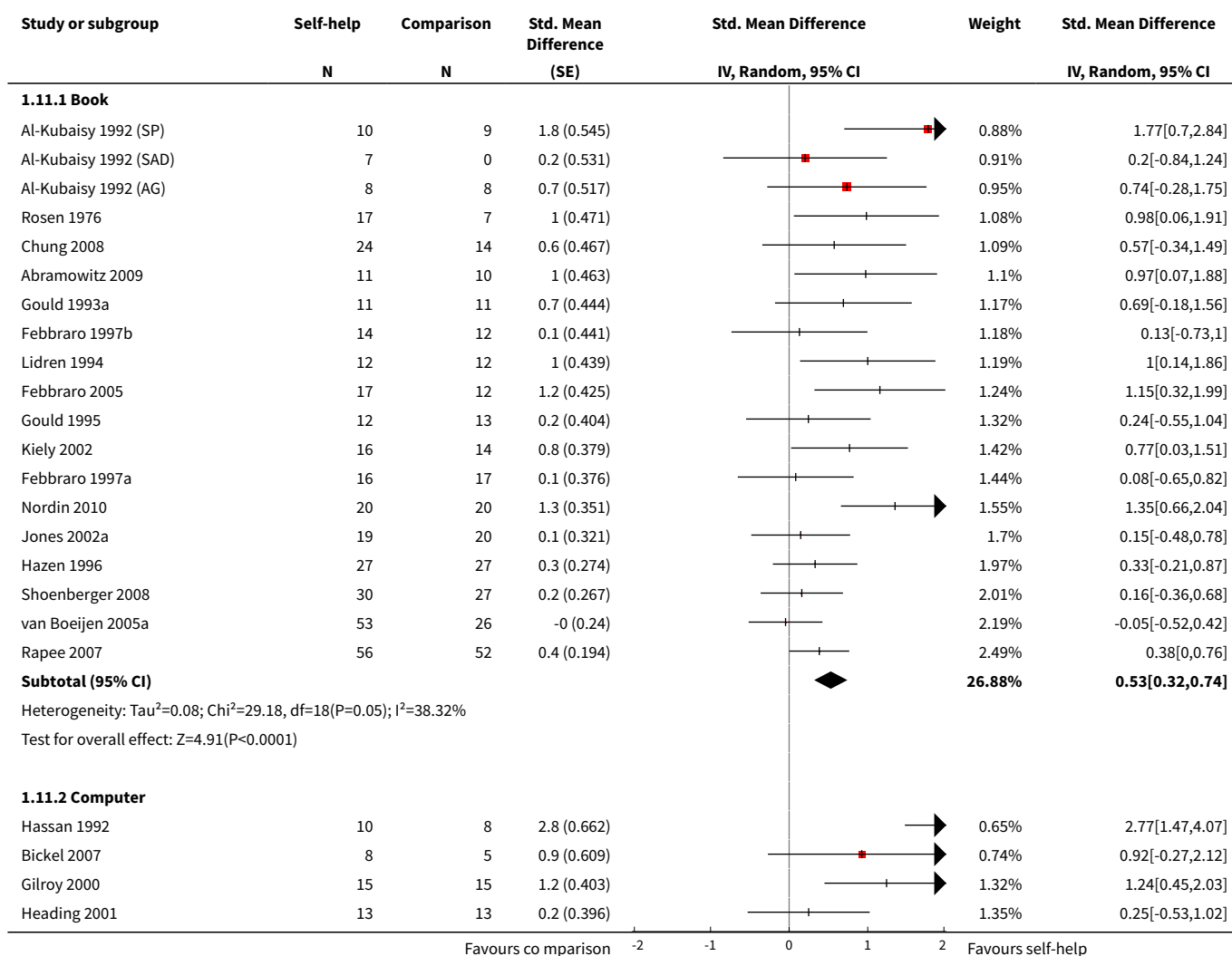
Analysis 1.10. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 10 Specific symptoms, self-rated at post-treatment (by disorder).

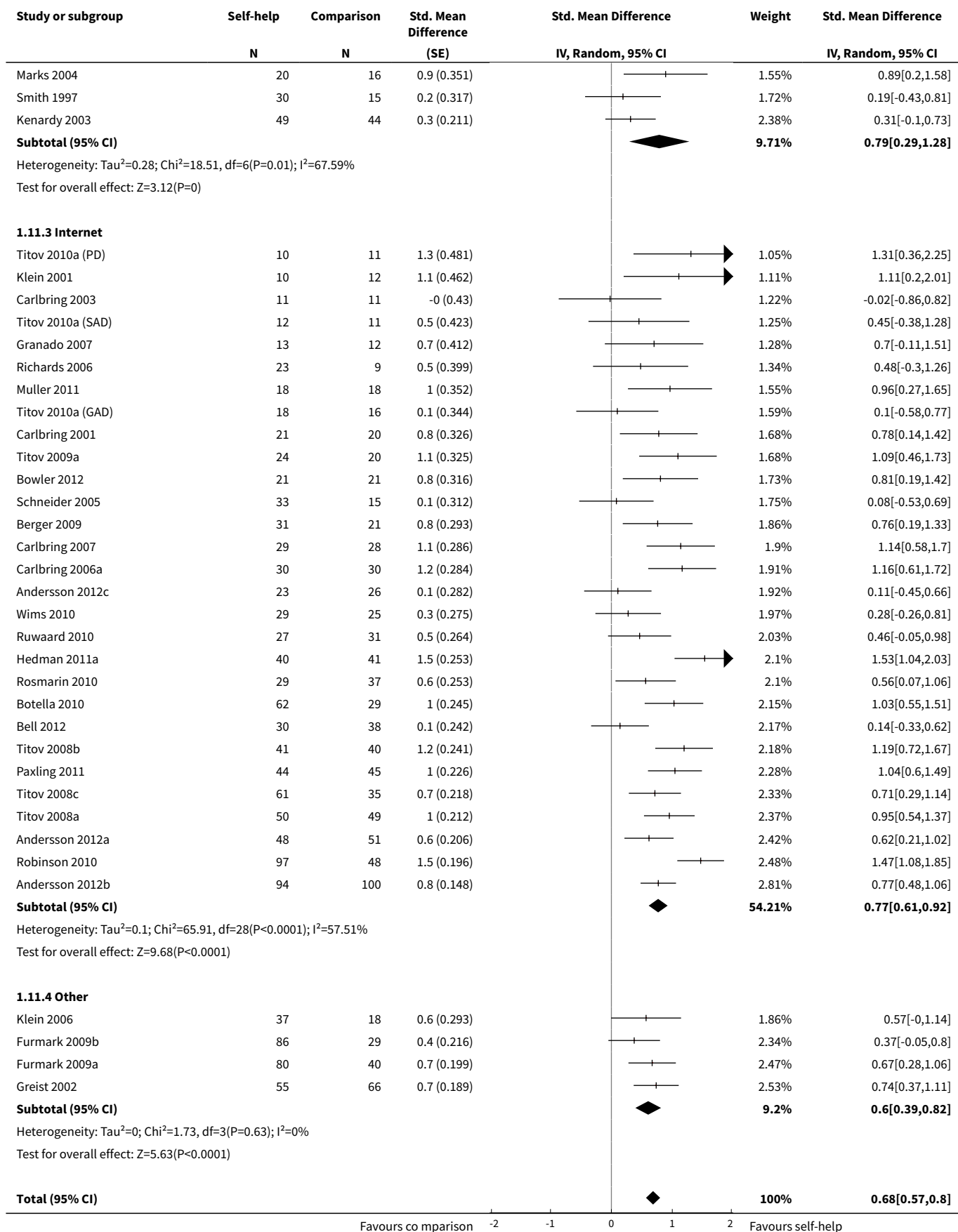






Analysis 1.11. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 11 Specific symptoms, self-rated at post-treatment (by media).

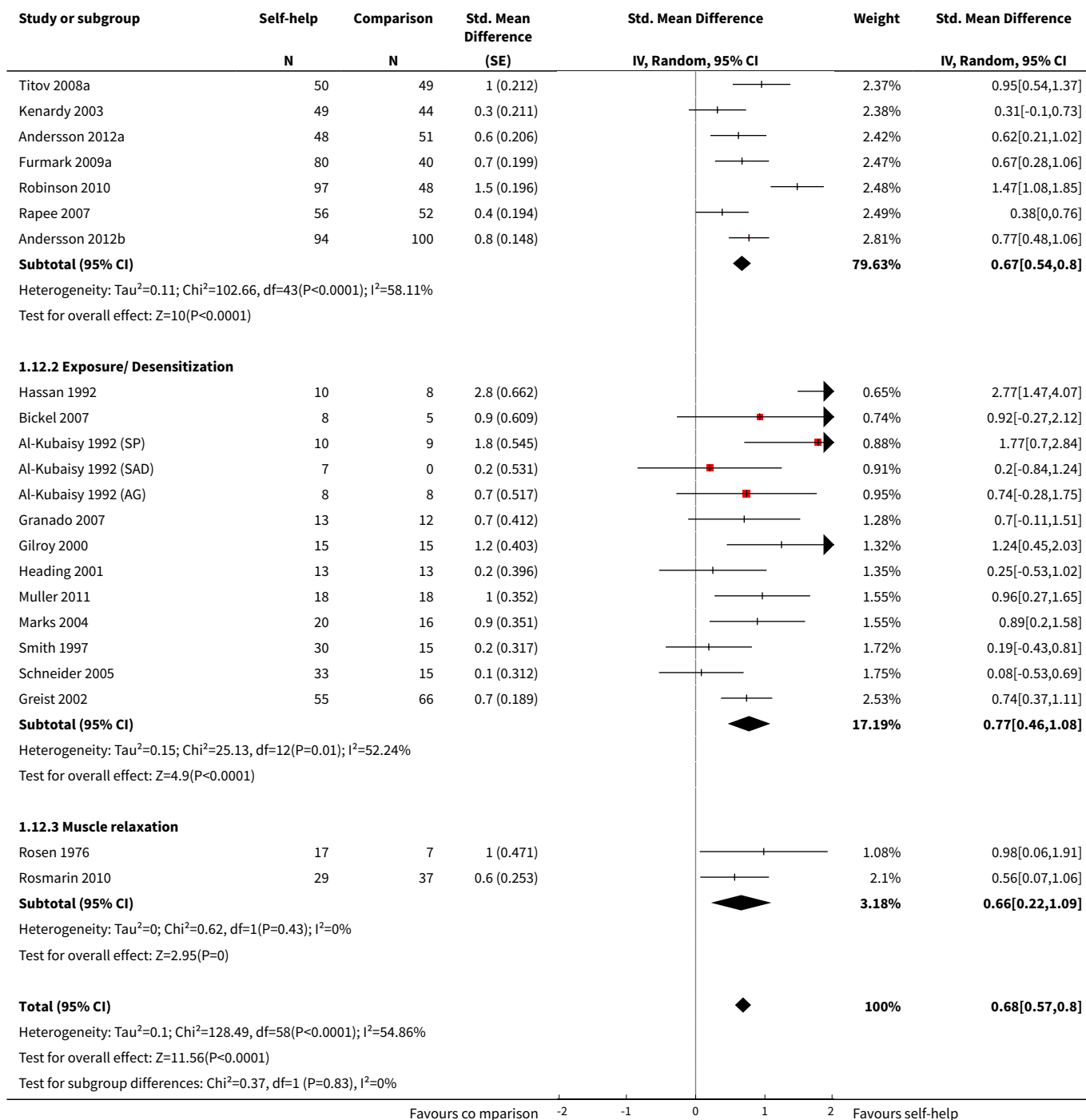




Study or subgroup	Self-help	Comparison	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
Heterogeneity: $\tau^2=0.1$; $\chi^2=128.49$, $df=58$ ($P<0.0001$); $I^2=54.86\%$						
Test for overall effect: $Z=11.56$ ($P<0.0001$)						
Test for subgroup differences: $\chi^2=3.8$, $df=1$ ($P=0.28$), $I^2=21.13\%$						
				-2 -1 0 1 2		Favours self-help

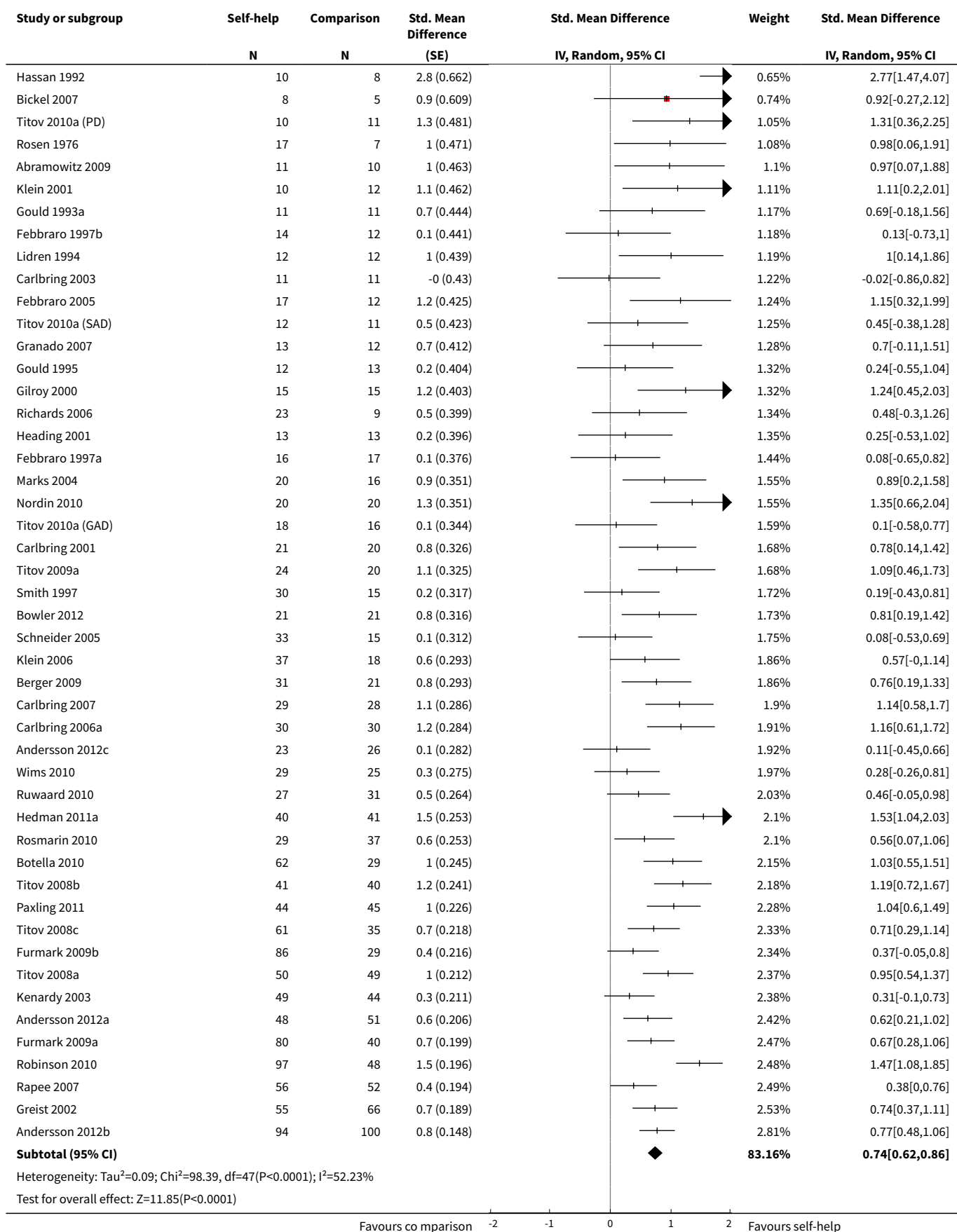
Analysis 1.12. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 12 Specific symptoms, self-rated at post-treatment (by therapy).

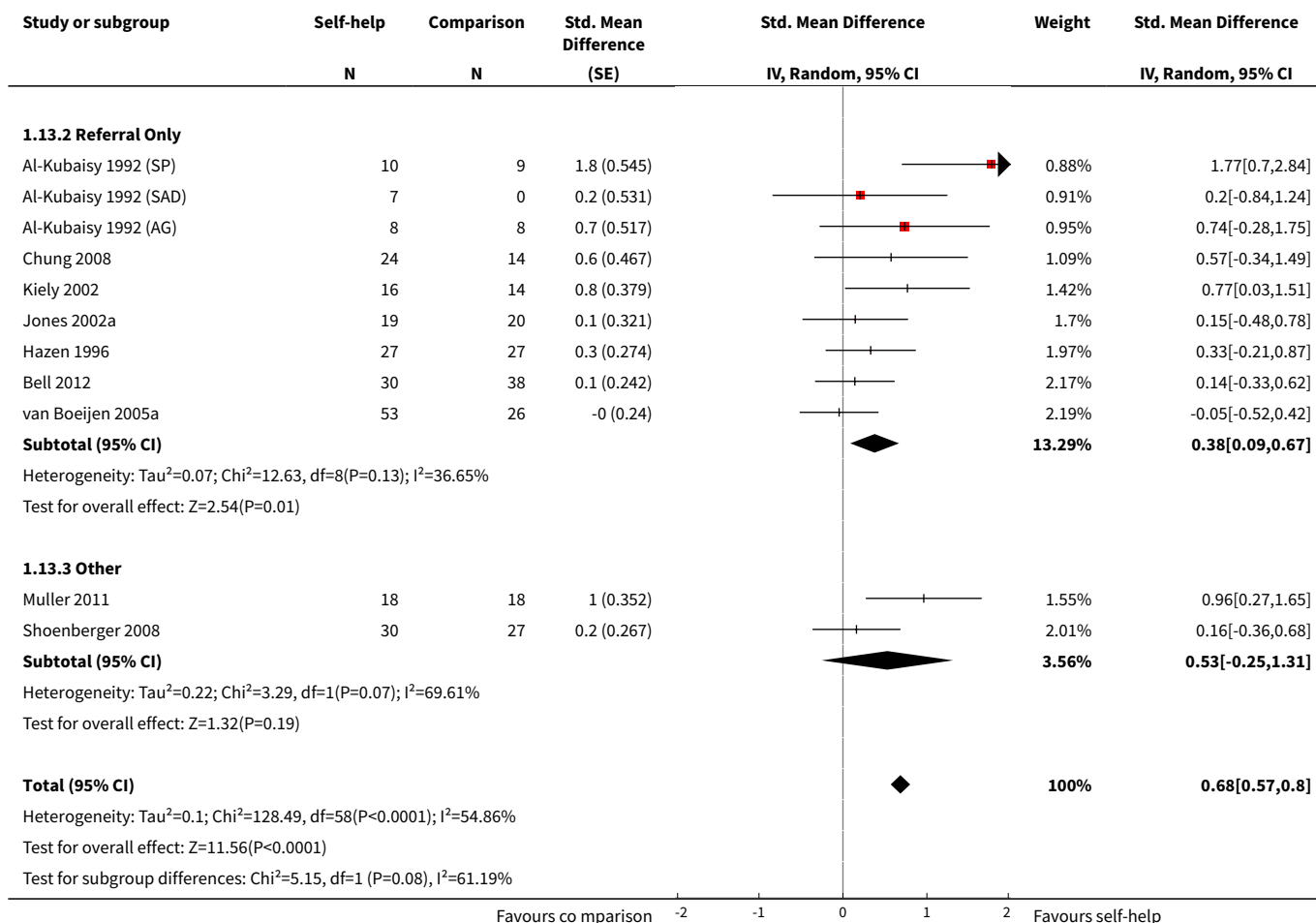
Study or subgroup	Self-help	Comparison	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
1.12.1 CBT						
Titov 2010a (PD)	10	11	1.3 (0.481)		1.05%	1.31[0.36,2.25]
Chung 2008	24	14	0.6 (0.467)		1.09%	0.57[-0.34,1.49]
Abramowitz 2009	11	10	1 (0.463)		1.1%	0.97[0.07,1.88]
Klein 2001	10	12	1.1 (0.462)		1.11%	1.11[0.2,2.01]
Gould 1993a	11	11	0.7 (0.444)		1.17%	0.69[-0.18,1.56]
Febbraro 1997b	14	12	0.1 (0.441)		1.18%	0.13[-0.73,1]
Lidren 1994	12	12	1 (0.439)		1.19%	1[0.14,1.86]
Carlbring 2003	11	11	-0 (0.43)		1.22%	-0.02[-0.86,0.82]
Febbraro 2005	17	12	1.2 (0.425)		1.24%	1.15[0.32,1.99]
Titov 2010a (SAD)	12	11	0.5 (0.423)		1.25%	0.45[-0.38,1.28]
Gould 1995	12	13	0.2 (0.404)		1.32%	0.24[-0.55,1.04]
Richards 2006	23	9	0.5 (0.399)		1.34%	0.48[-0.3,1.26]
Kiely 2002	16	14	0.8 (0.379)		1.42%	0.77[0.03,1.51]
Febbraro 1997a	16	17	0.1 (0.376)		1.44%	0.08[-0.65,0.82]
Nordin 2010	20	20	1.3 (0.351)		1.55%	1.35[0.66,2.04]
Titov 2010a (GAD)	18	16	0.1 (0.344)		1.59%	0.1[-0.58,0.77]
Carlbring 2001	21	20	0.8 (0.326)		1.68%	0.78[0.14,1.42]
Titov 2009a	24	20	1.1 (0.325)		1.68%	1.09[0.46,1.73]
Jones 2002a	19	20	0.1 (0.321)		1.7%	0.15[-0.48,0.78]
Bowler 2012	21	21	0.8 (0.316)		1.73%	0.81[0.19,1.42]
Klein 2006	37	18	0.6 (0.293)		1.86%	0.57[-0.1,1.24]
Berger 2009	31	21	0.8 (0.293)		1.86%	0.76[0.19,1.33]
Carlbring 2007	29	28	1.1 (0.286)		1.9%	1.14[0.58,1.7]
Carlbring 2006a	30	30	1.2 (0.284)		1.91%	1.16[0.61,1.72]
Andersson 2012c	23	26	0.1 (0.282)		1.92%	0.11[-0.45,0.66]
Wims 2010	29	25	0.3 (0.275)		1.97%	0.28[-0.26,0.81]
Hazen 1996	27	27	0.3 (0.274)		1.97%	0.33[-0.21,0.87]
Shoenberger 2008	30	27	0.2 (0.267)		2.01%	0.16[-0.36,0.68]
Ruwaard 2010	27	31	0.5 (0.264)		2.03%	0.46[-0.05,0.98]
Hedman 2011a	40	41	1.5 (0.253)		2.1%	1.53[1.04,2.03]
Botella 2010	62	29	1 (0.245)		2.15%	1.03[0.55,1.51]
Bell 2012	30	38	0.1 (0.242)		2.17%	0.14[-0.33,0.62]
Titov 2008b	41	40	1.2 (0.241)		2.18%	1.19[0.72,1.67]
van Boeijen 2005a	53	26	-0 (0.24)		2.19%	-0.05[-0.52,0.42]
Paxling 2011	44	45	1 (0.226)		2.28%	1.04[0.6,1.49]
Titov 2008c	61	35	0.7 (0.218)		2.33%	0.71[0.29,1.14]
Furmark 2009b	86	29	0.4 (0.216)		2.34%	0.37[-0.05,0.8]
				-2 -1 0 1 2		Favours self-help



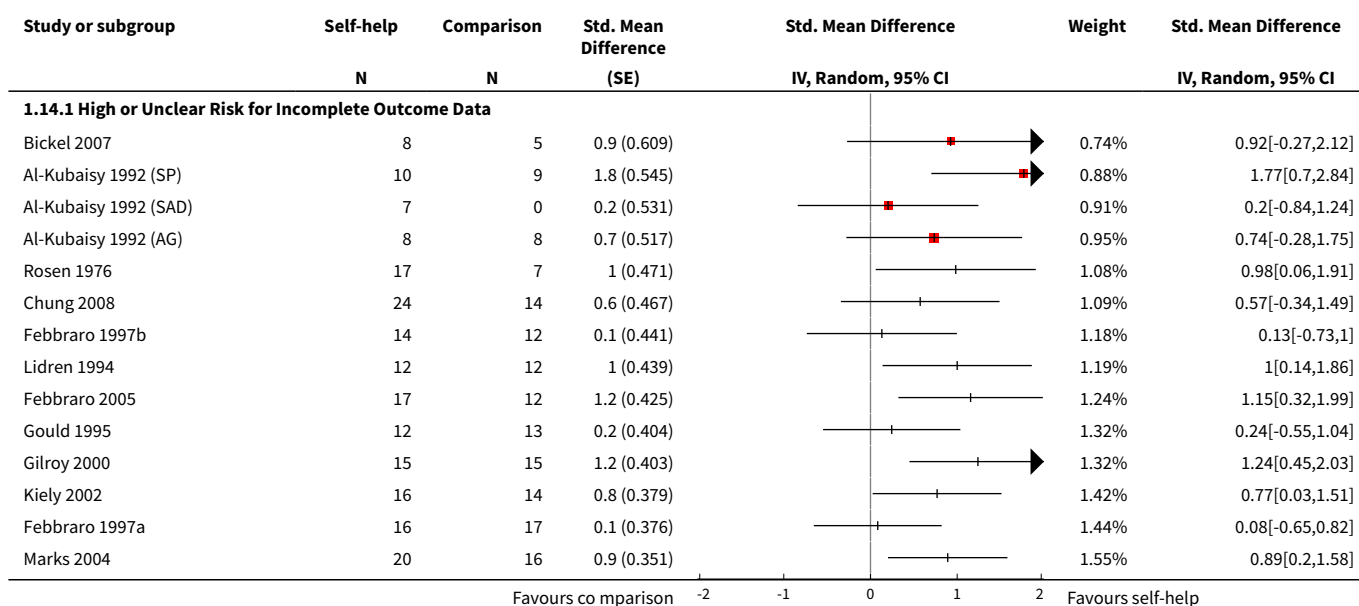
Analysis 1.13. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 13 Specific symptoms, self-rated at post-treatment (by setting).

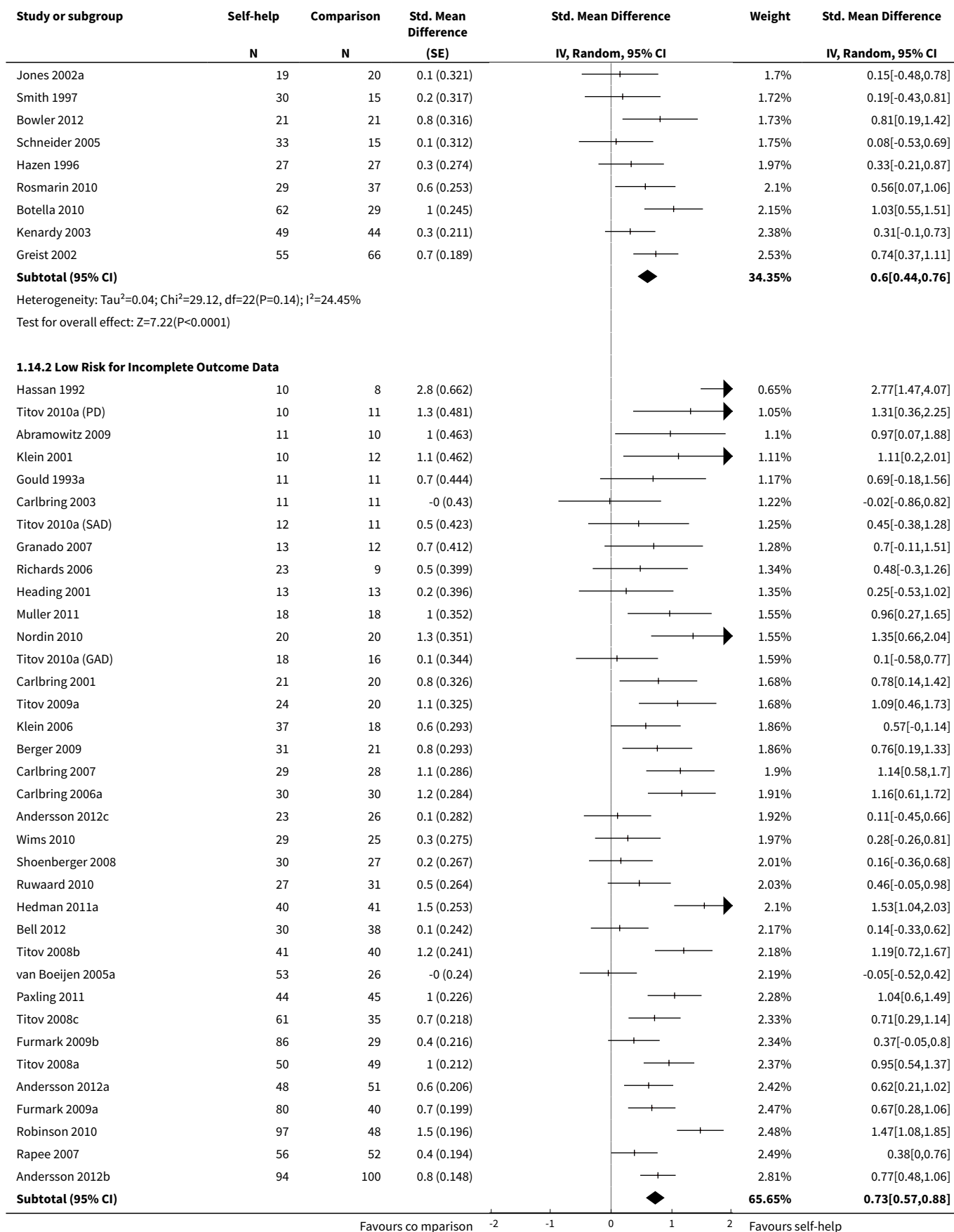


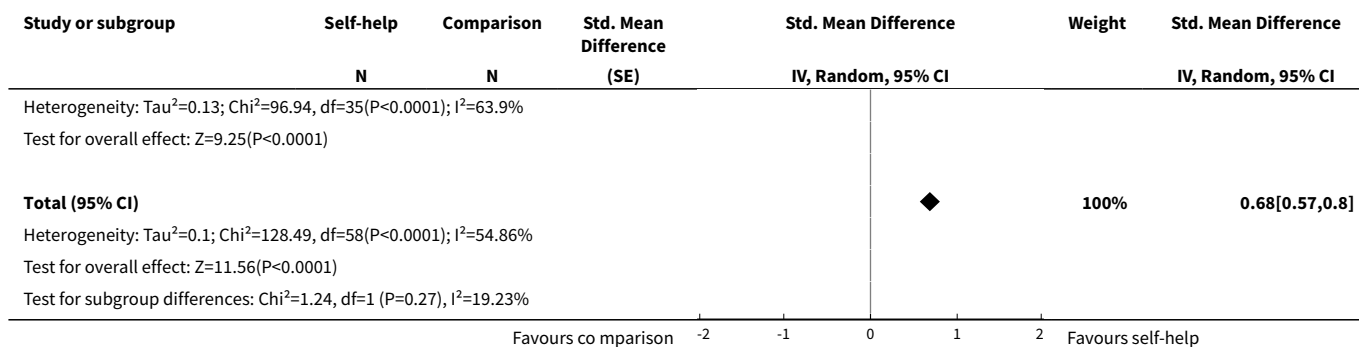




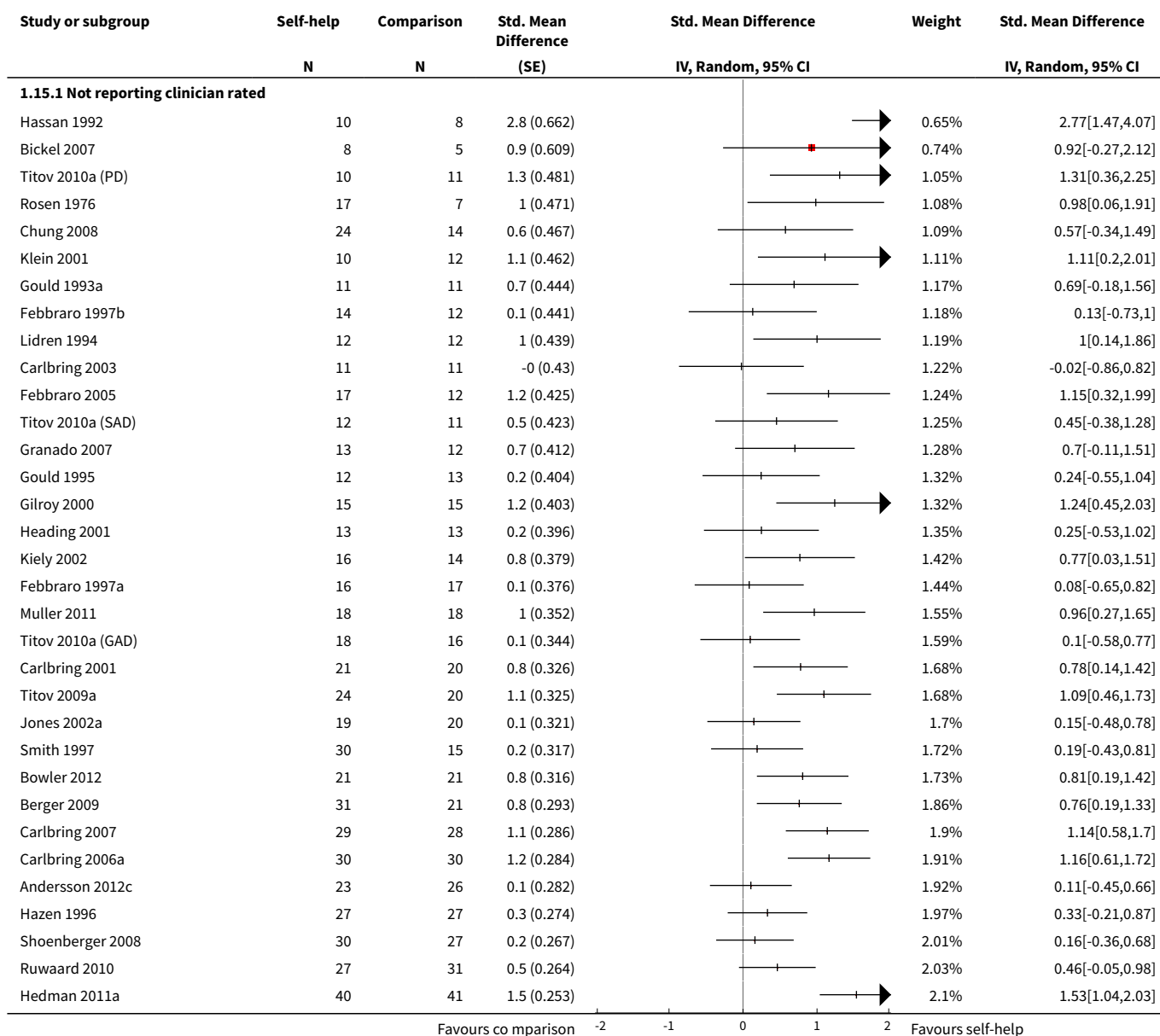
Analysis 1.14. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias).

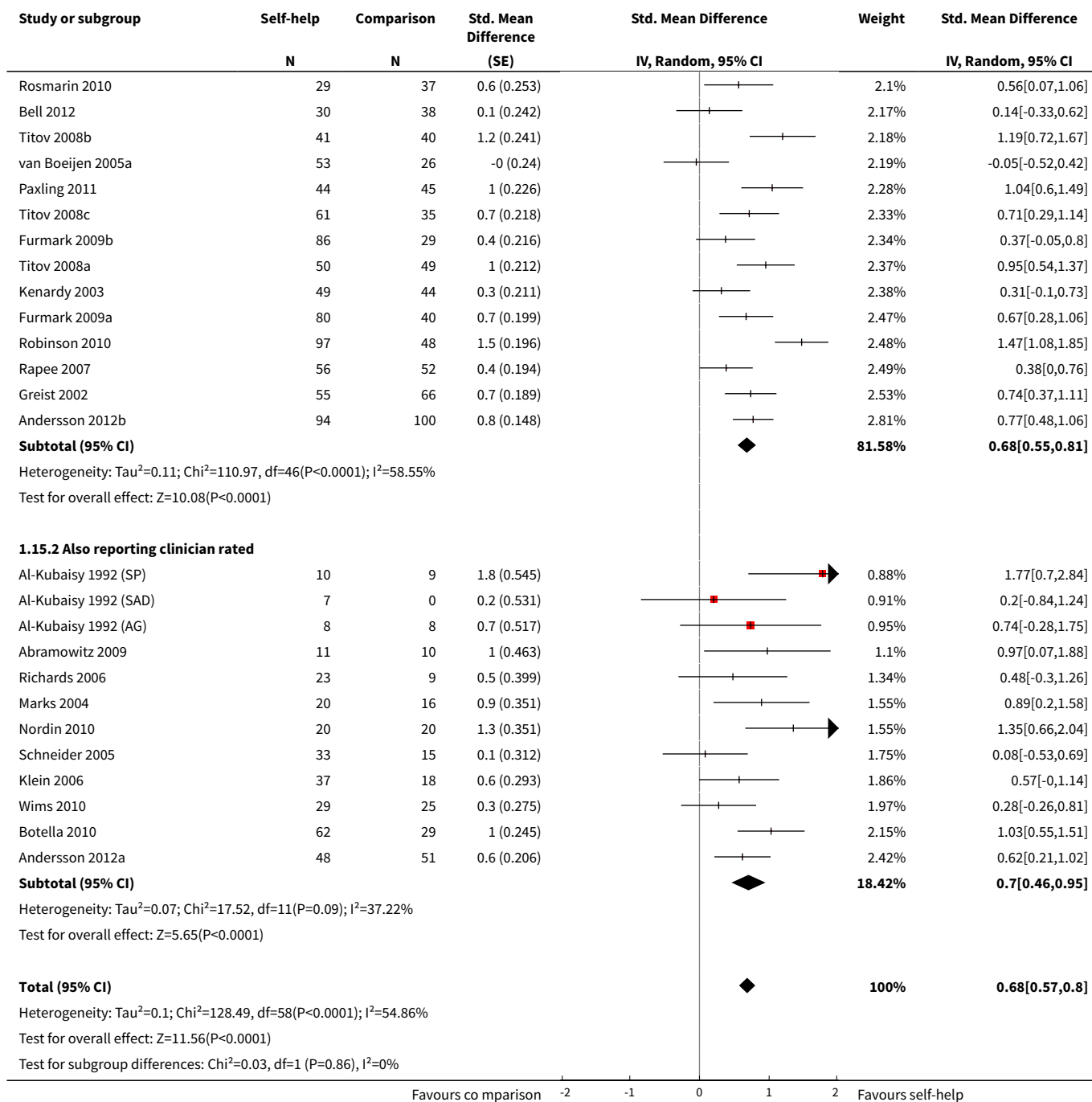




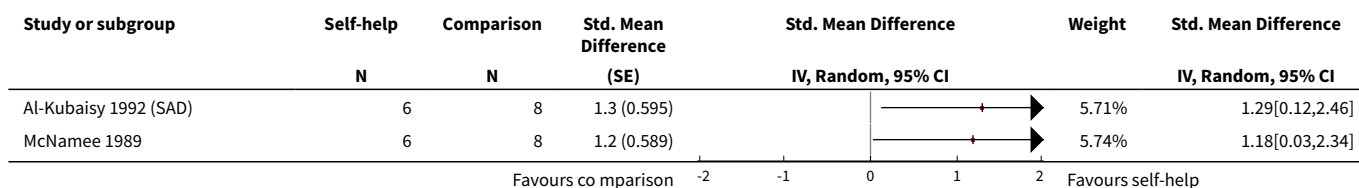


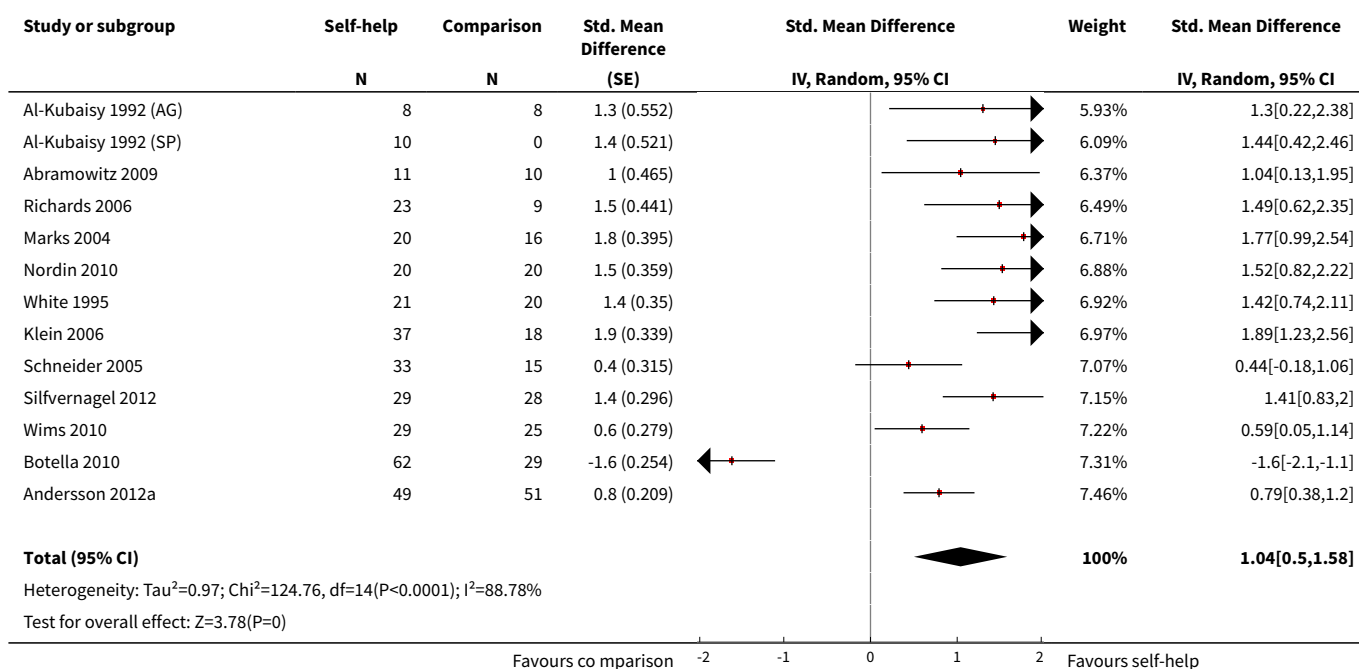
Analysis 1.15. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 15 Specific symptoms, self-rated at post-treatment (studies also reporting clinician rated).



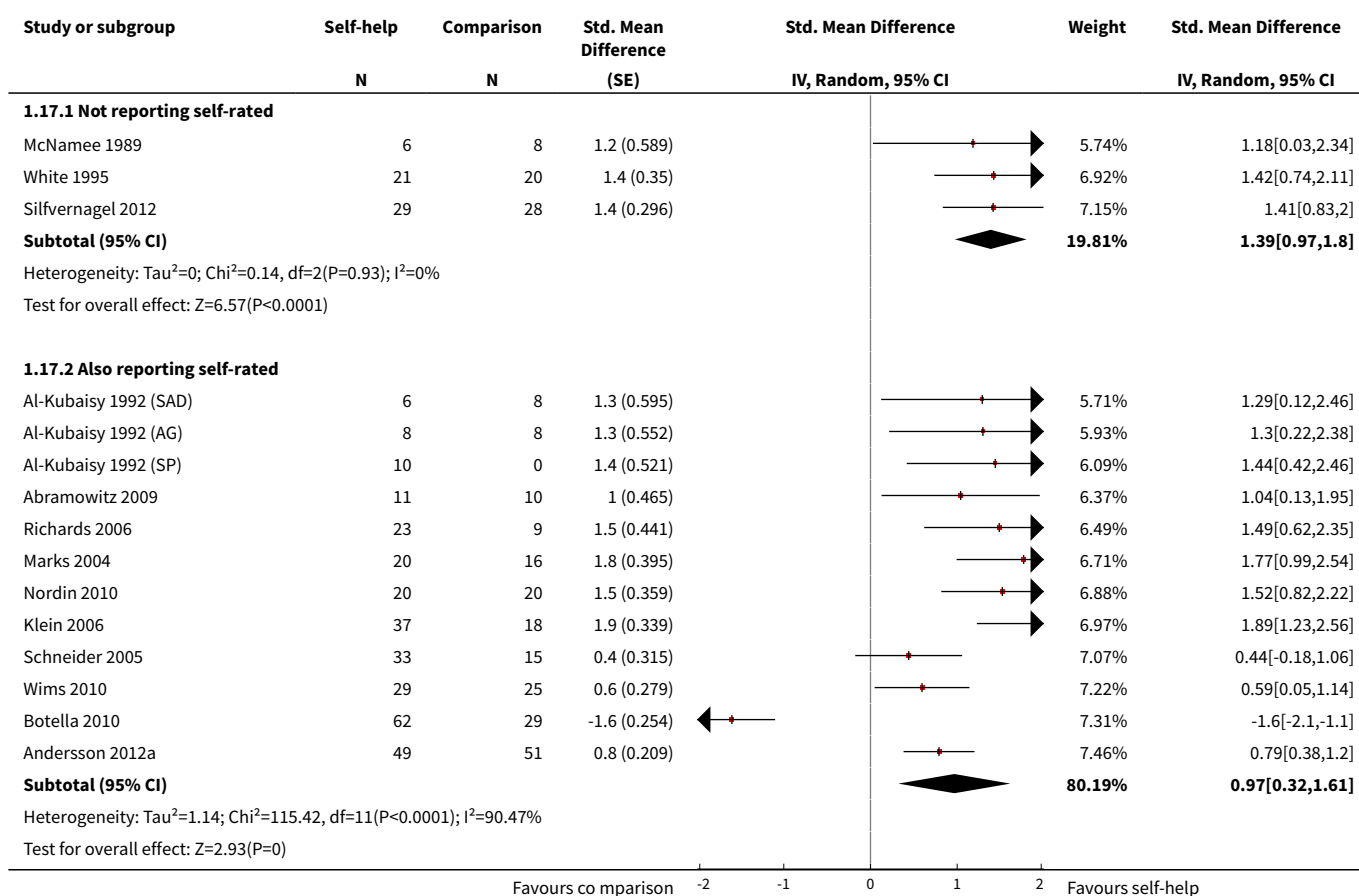


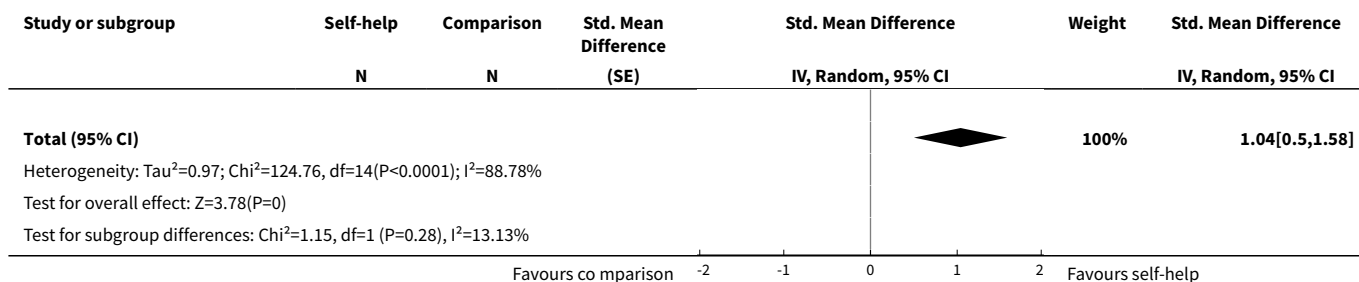
Analysis 1.16. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 16 Specific symptoms, clinician-rated at post-treatment.



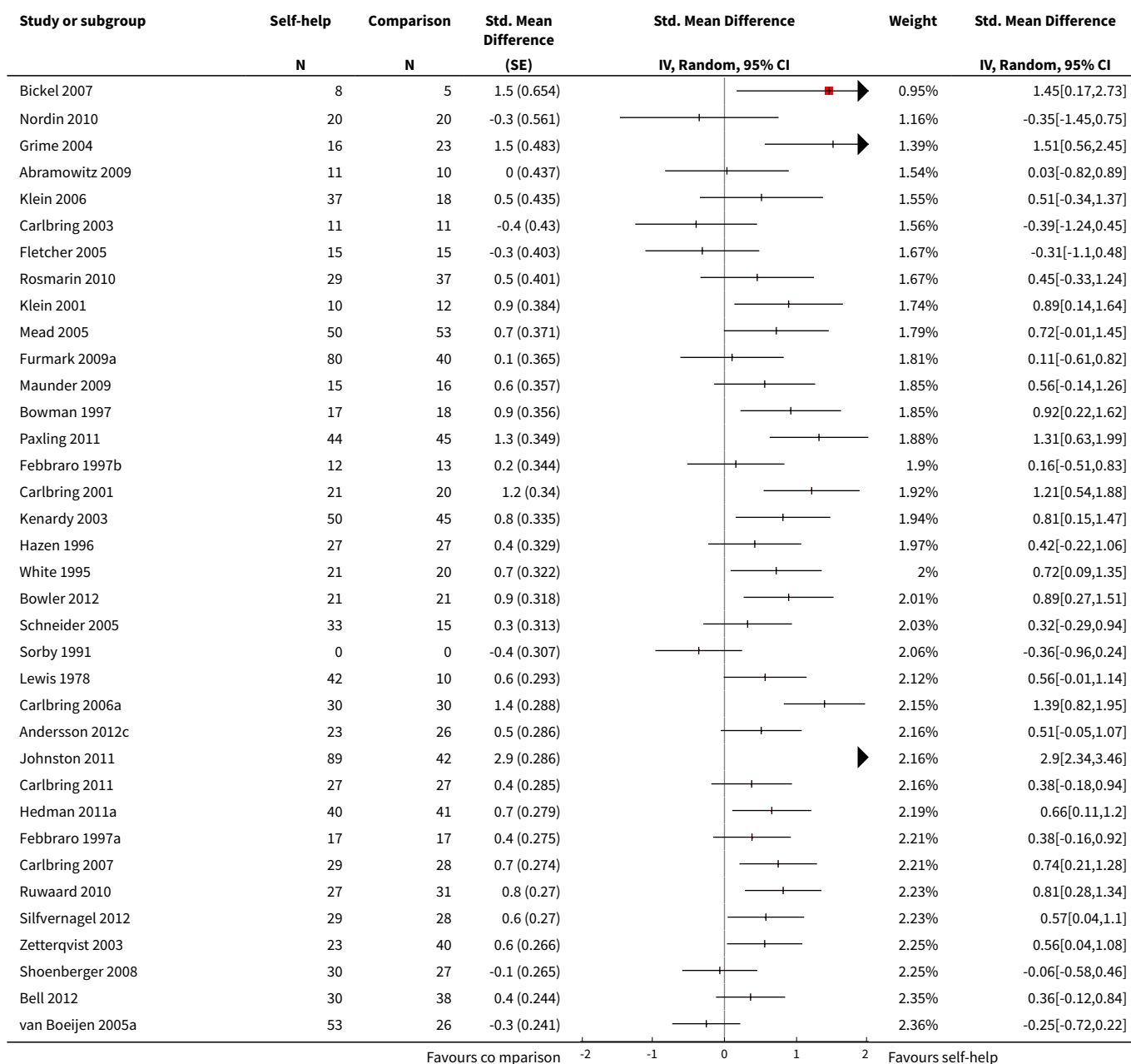


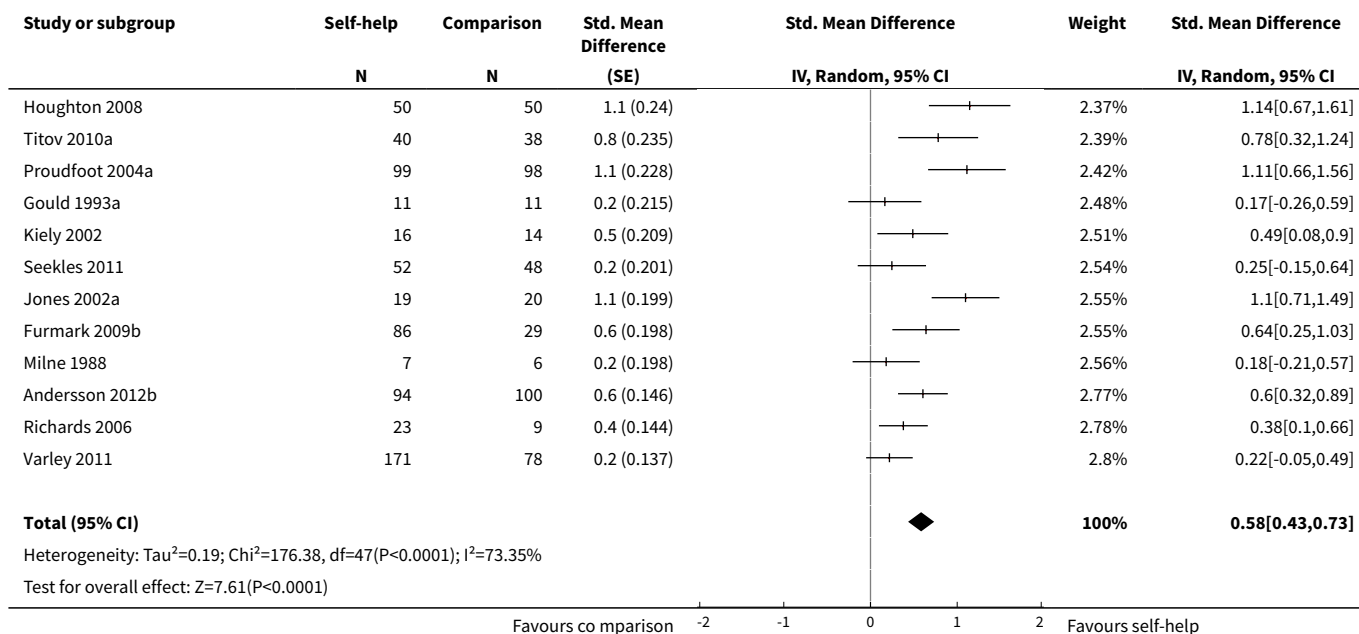
Analysis 1.17. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 17 Specific symptoms, clinician-rated at post-treatment.



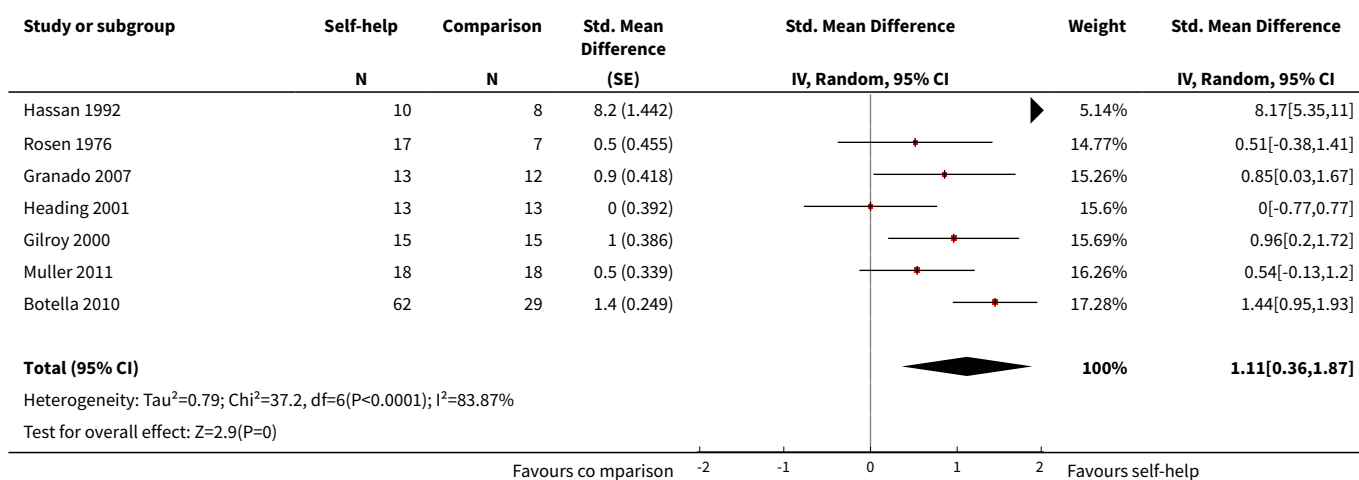


Analysis 1.18. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 18 General symptoms, self-rated at post-treatment.

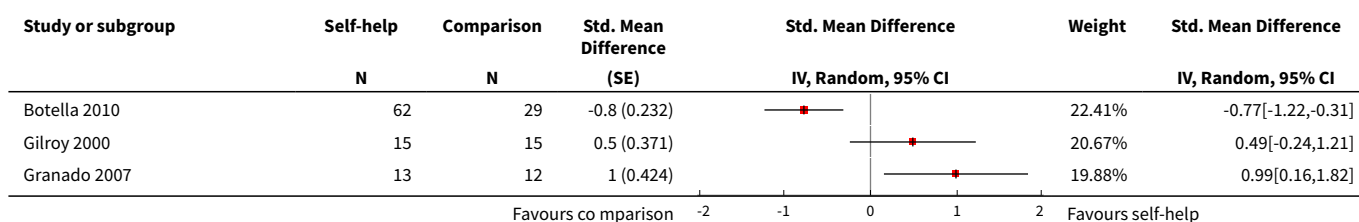


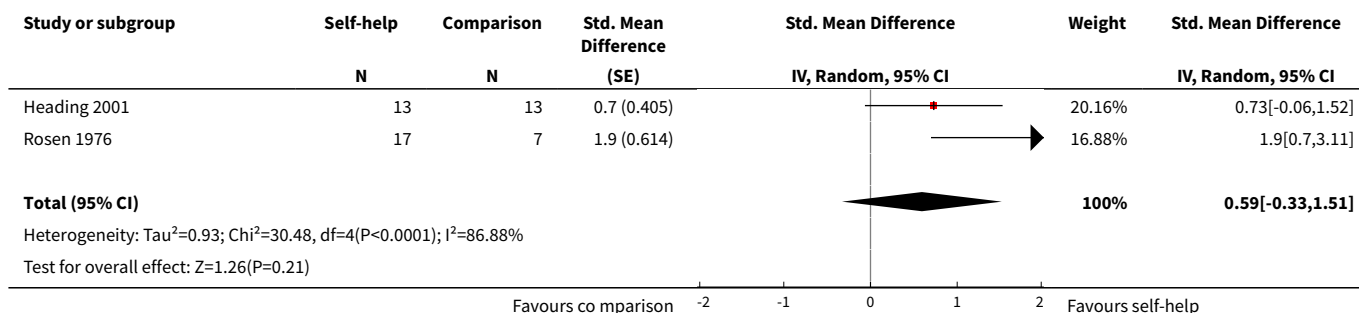


Analysis 1.19. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 19 Behavioural Test (level), clinician-rated at post-treatment.

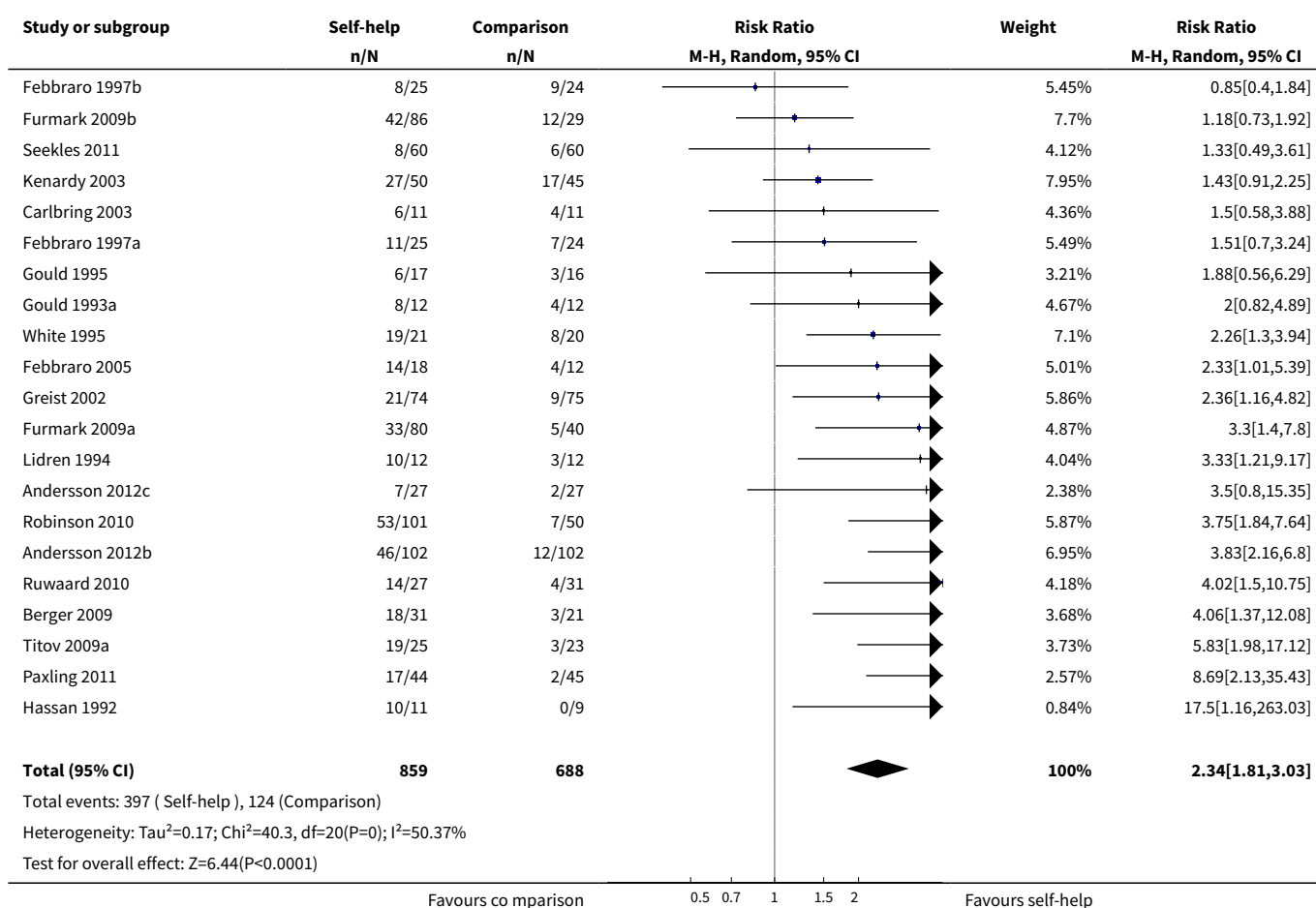


Analysis 1.20. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 20 Behavioural Test (symptoms), self-rated at post-treatment.

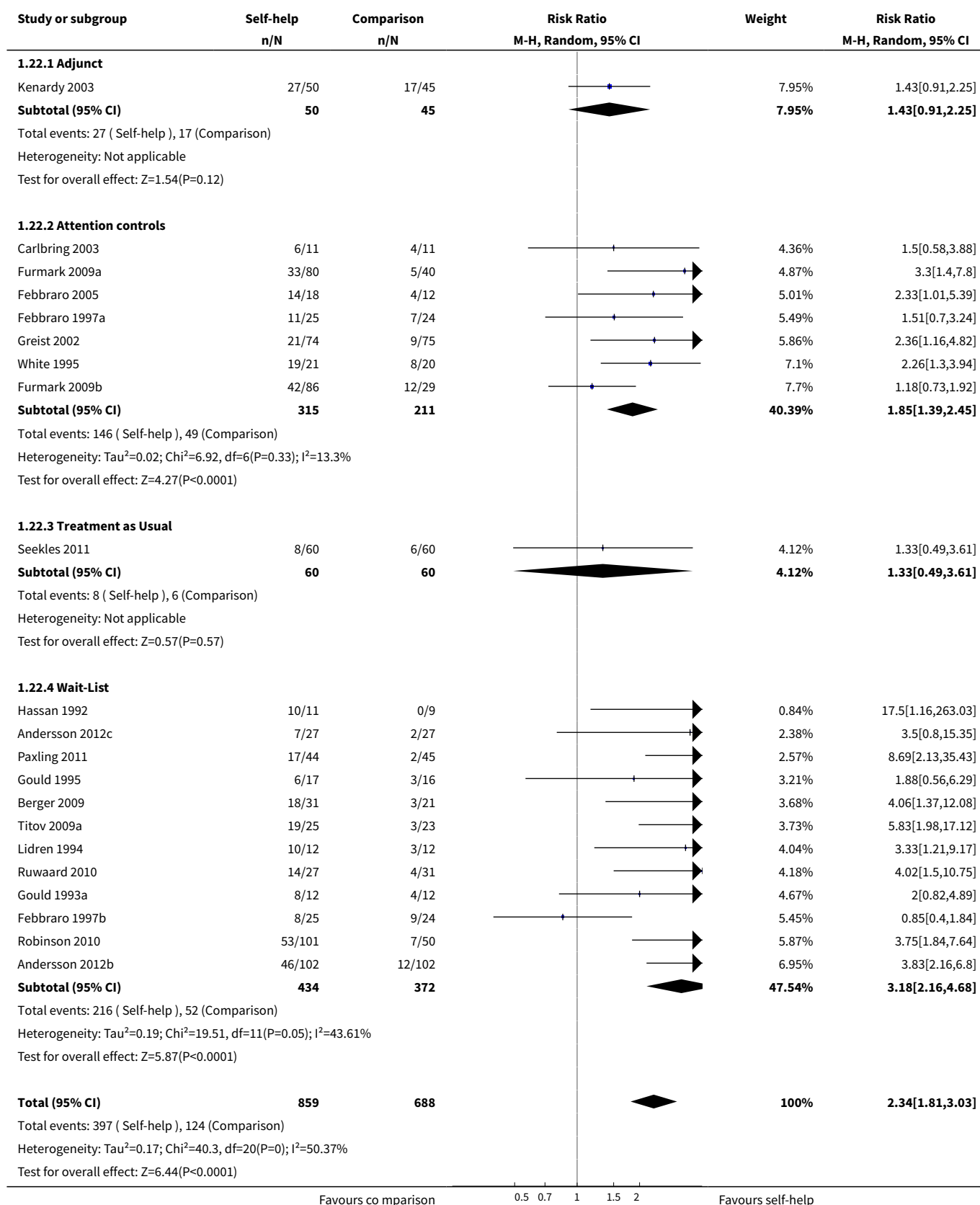




Analysis 1.21. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 21 Response (specific), self-rated at post-treatment.

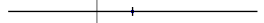
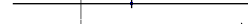


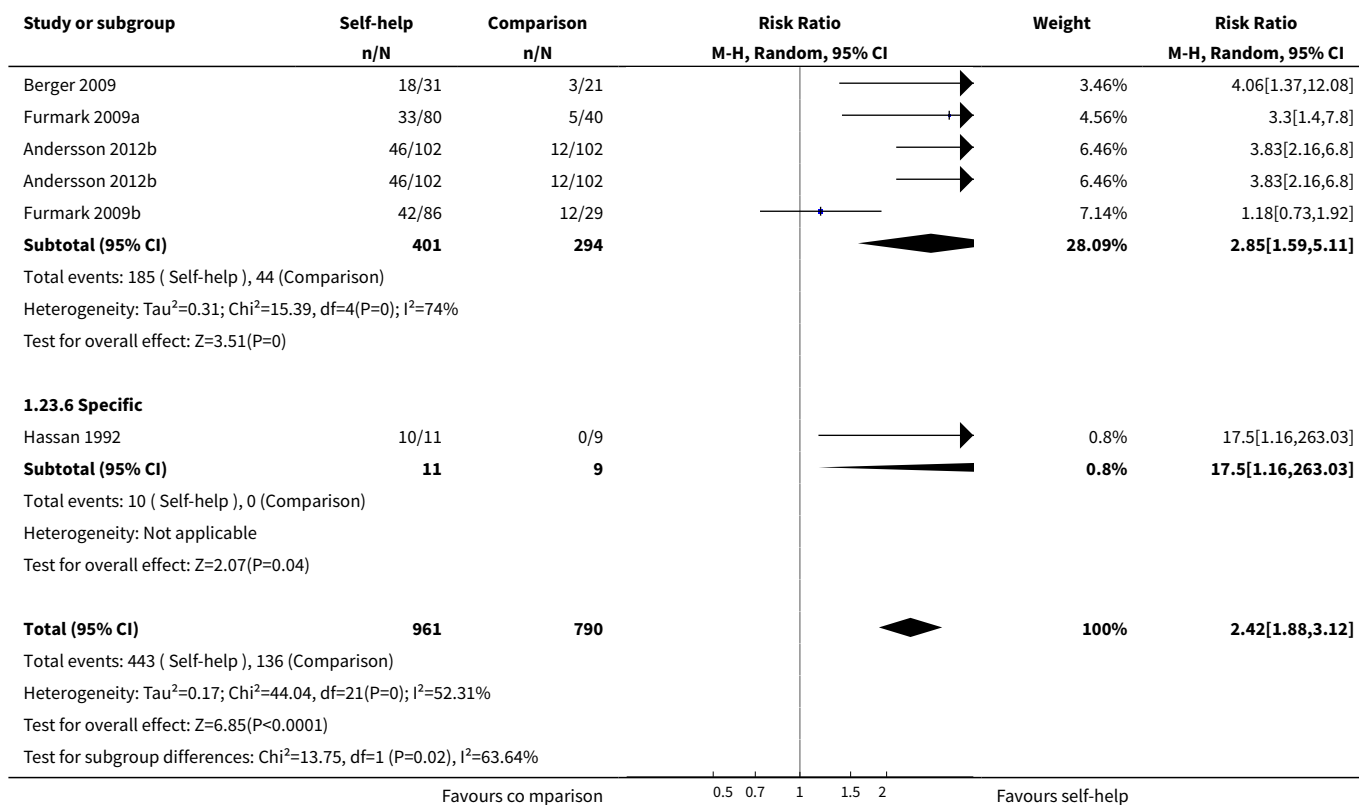
Analysis 1.22. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 22 Response (specific), self-rated at post-treatment (type of control).



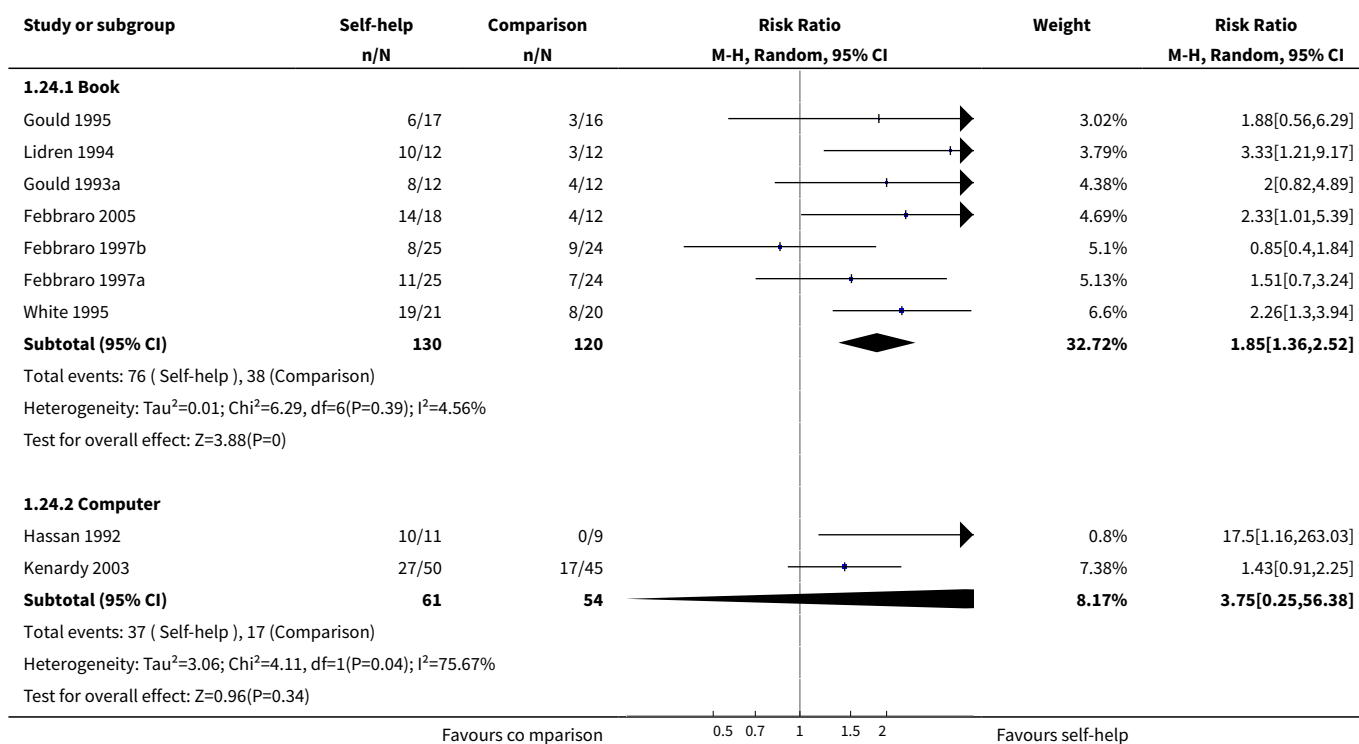
Study or subgroup	Self-help n/N	Comparison n/N	Risk Ratio M-H, Random, 95% CI	Weight	Risk Ratio M-H, Random, 95% CI
Test for subgroup differences: $\chi^2=8.58$, $df=1$ ($P=0.04$), $I^2=65.05\%$					
Favours co mparison			0.5 0.7 1 1.5 2	Favours self-help	

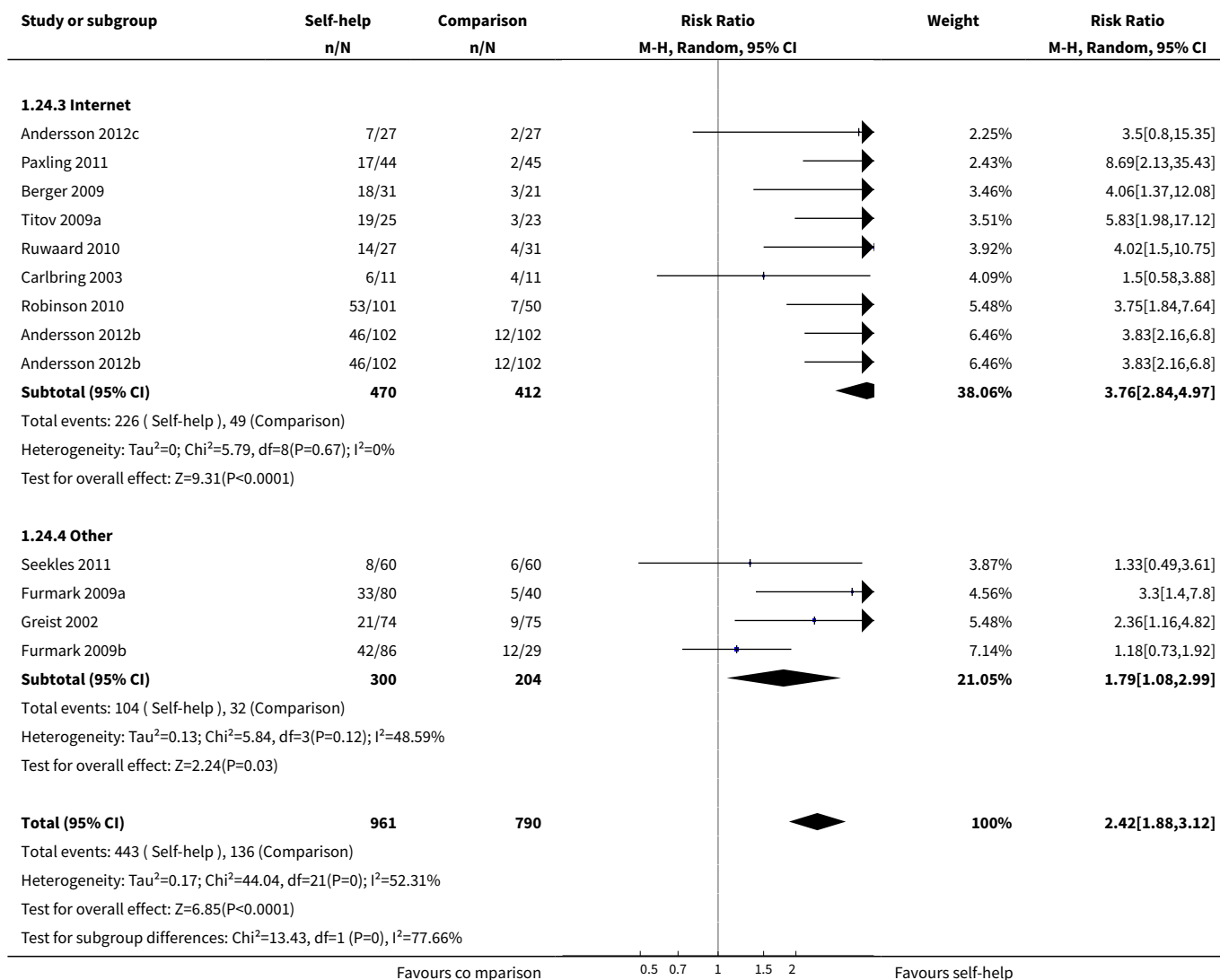
Analysis 1.23. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 23 Response (specific), self-rated at post-treatment (by disorder).

Study or subgroup	Self-help n/N	Comparison n/N	Risk Ratio M-H, Random, 95% CI	Weight	Risk Ratio M-H, Random, 95% CI
1.23.1 GAD					
Andersson 2012c	7/27	2/27		2.25%	3.5[0.8,15.35]
Paxling 2011	17/44	2/45		2.43%	8.69[2.13,35.43]
Titov 2009a	19/25	3/23		3.51%	5.83[1.98,17.12]
Robinson 2010	53/101	7/50		5.48%	3.75[1.84,7.64]
Subtotal (95% CI)	197	145		13.66%	4.6[2.75,7.68]
Total events: 96 (Self-help), 14 (Comparison)					
Heterogeneity: $\tau^2=0$; $\chi^2=1.43$, $df=3$ ($P=0.7$); $I^2=0\%$					
Test for overall effect: $Z=5.83$ ($P<0.0001$)					
1.23.2 Mixed					
Seekles 2011	8/60	6/60		3.87%	1.33[0.49,3.61]
White 1995	19/21	8/20		6.6%	2.26[1.3,3.94]
Subtotal (95% CI)	81	80		10.47%	2[1.23,3.24]
Total events: 27 (Self-help), 14 (Comparison)					
Heterogeneity: $\tau^2=0$; $\chi^2=0.9$, $df=1$ ($P=0.34$); $I^2=0\%$					
Test for overall effect: $Z=2.8$ ($P=0.01$)					
1.23.3 OCD					
Greist 2002	21/74	9/75		5.48%	2.36[1.16,4.82]
Subtotal (95% CI)	74	75		5.48%	2.36[1.16,4.82]
Total events: 21 (Self-help), 9 (Comparison)					
Heterogeneity: Not applicable					
Test for overall effect: $Z=2.37$ ($P=0.02$)					
1.23.4 Panic					
Gould 1995	6/17	3/16		3.02%	1.88[0.56,6.29]
Lidren 1994	10/12	3/12		3.79%	3.33[1.21,9.17]
Ruwaard 2010	14/27	4/31		3.92%	4.02[1.5,10.75]
Carlbring 2003	6/11	4/11		4.09%	1.5[0.58,3.88]
Gould 1993a	8/12	4/12		4.38%	2[0.82,4.89]
Febbraro 2005	14/18	4/12		4.69%	2.33[1.01,5.39]
Febbraro 1997b	8/25	9/24		5.1%	0.85[0.4,1.84]
Febbraro 1997a	11/25	7/24		5.13%	1.51[0.7,3.24]
Kenardy 2003	27/50	17/45		7.38%	1.43[0.91,2.25]
Subtotal (95% CI)	197	187		41.51%	1.73[1.29,2.3]
Total events: 104 (Self-help), 55 (Comparison)					
Heterogeneity: $\tau^2=0.03$; $\chi^2=9.23$, $df=8$ ($P=0.32$); $I^2=13.33\%$					
Test for overall effect: $Z=3.71$ ($P=0$)					
1.23.5 Social					
Favours co mparison			0.5 0.7 1 1.5 2	Favours self-help	

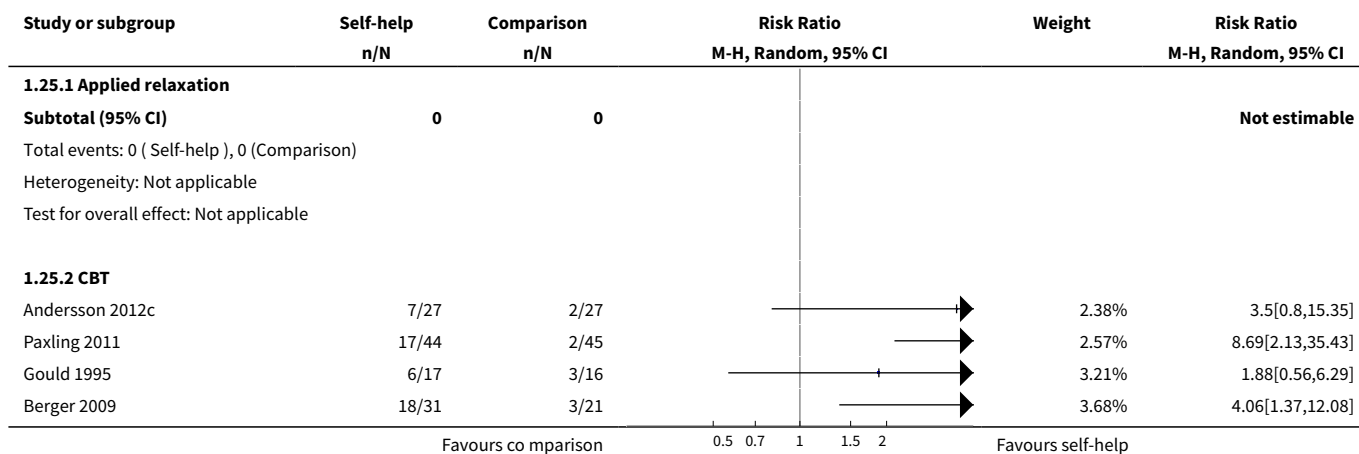


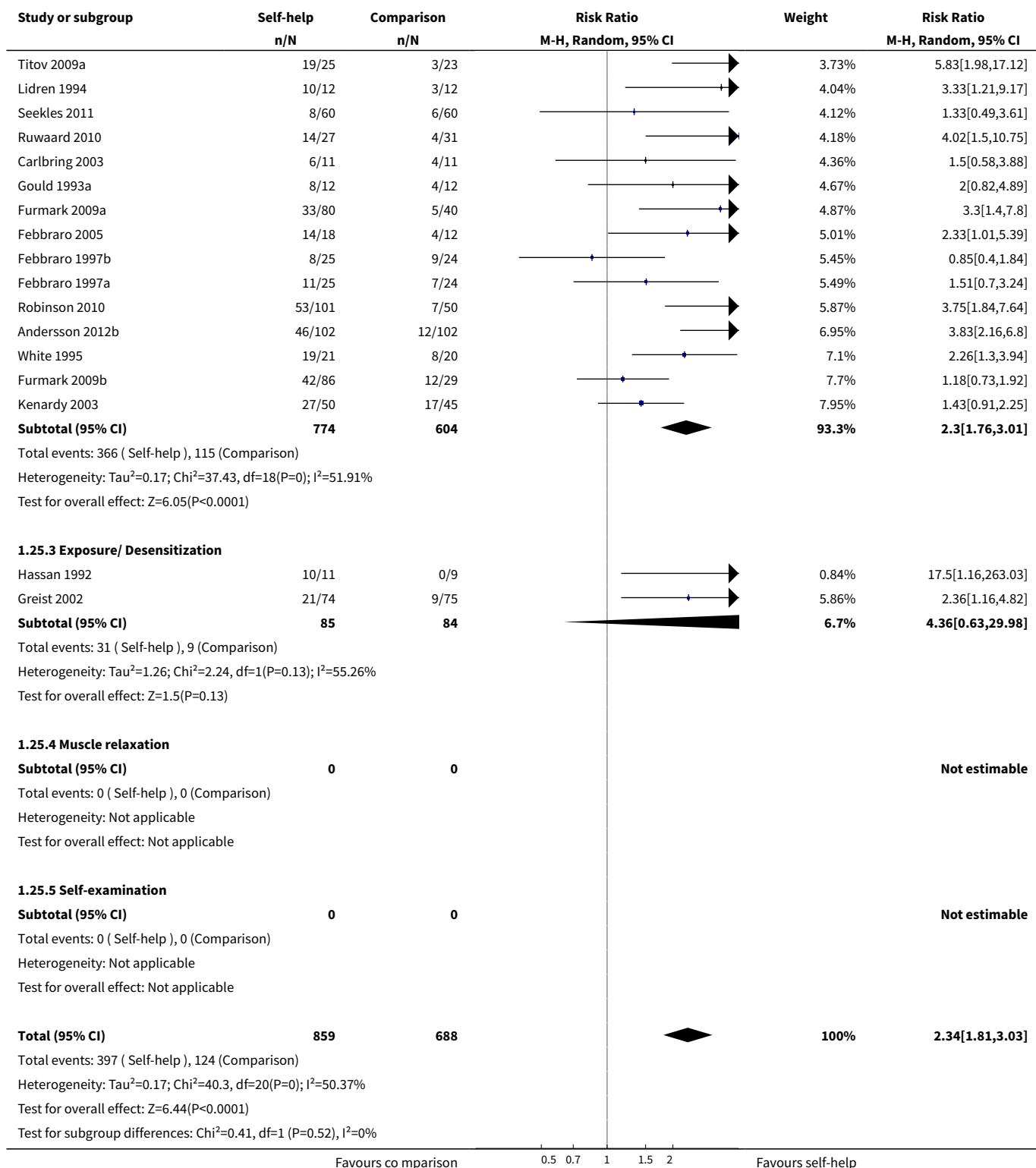
Analysis 1.24. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 24 Response (specific), self-rated at post-treatment (by media).



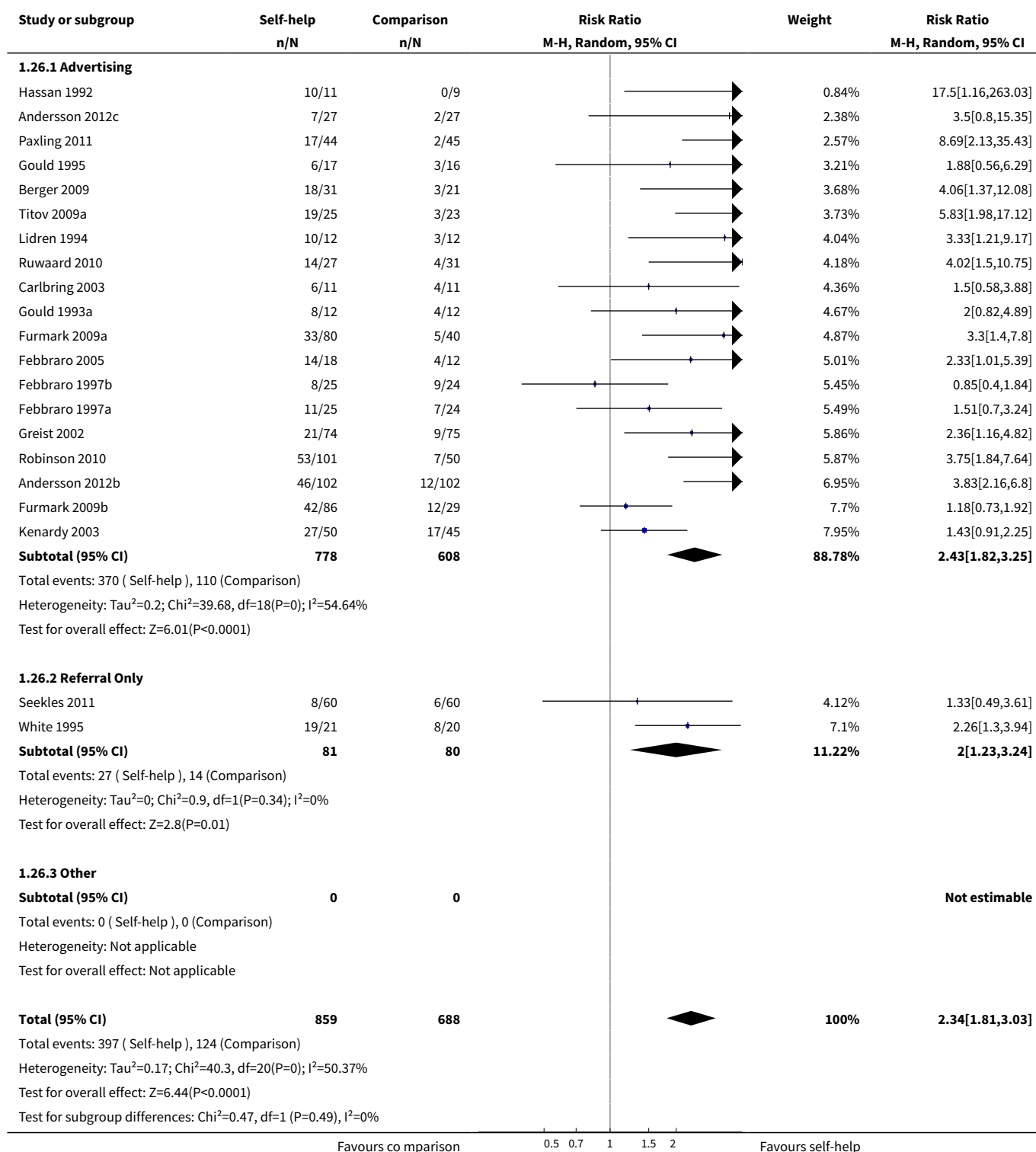


Analysis 1.25. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 25 Response (specific), self-rated at post-treatment (by therapy).

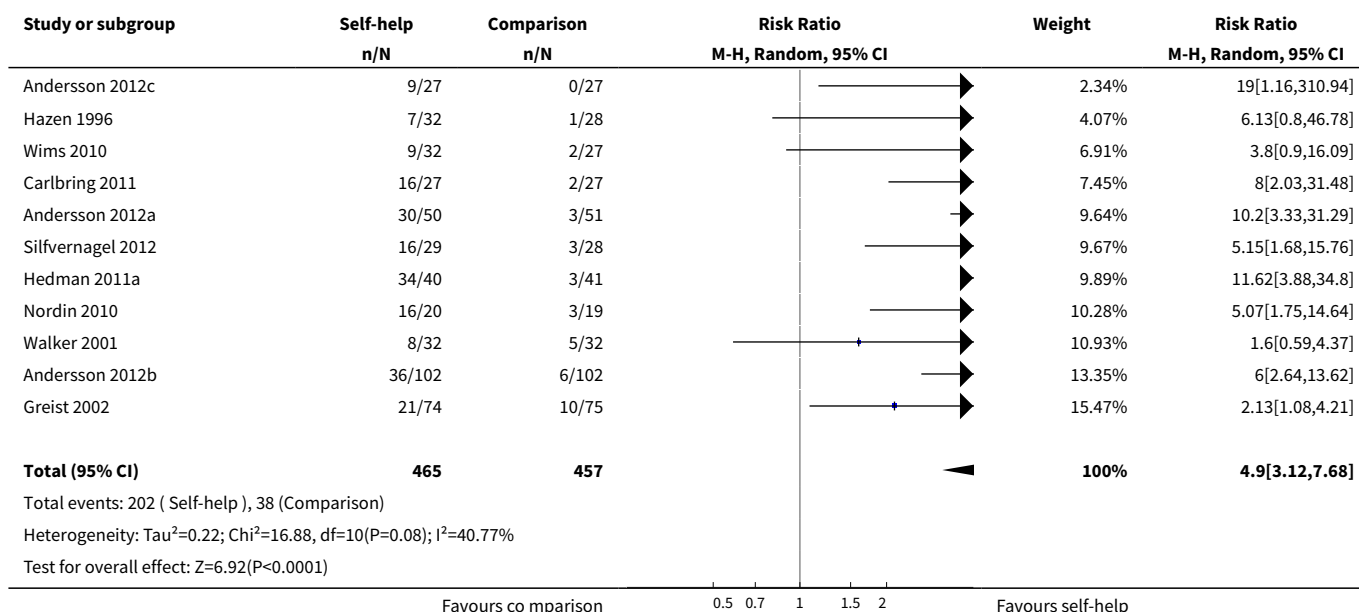




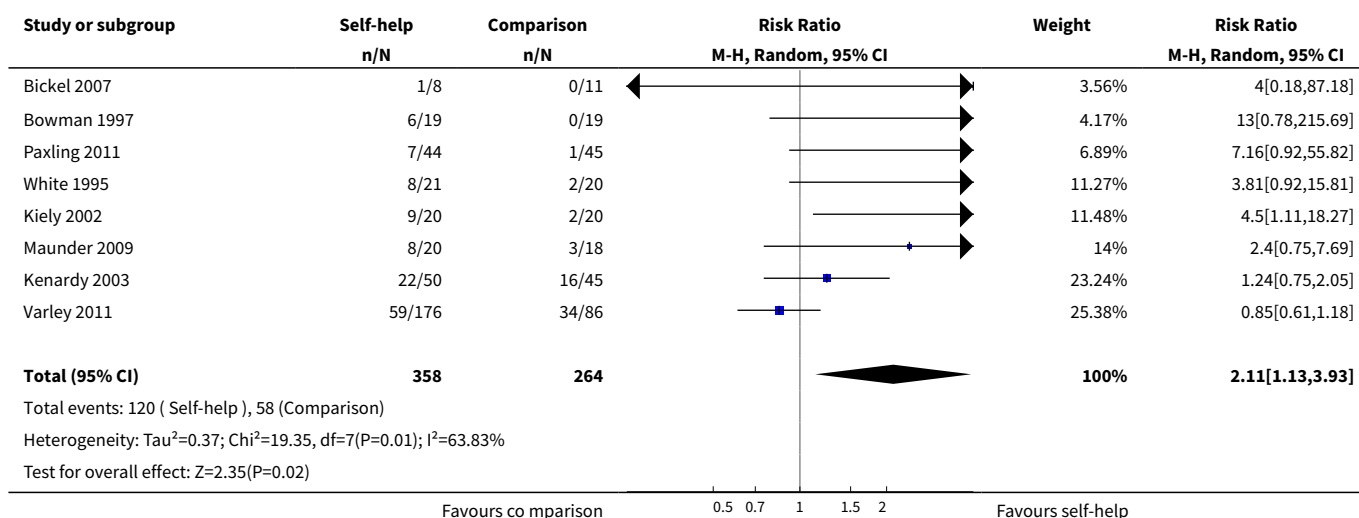
Analysis 1.26. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 26 Response (specific), self-rated at post-treatment (by setting).



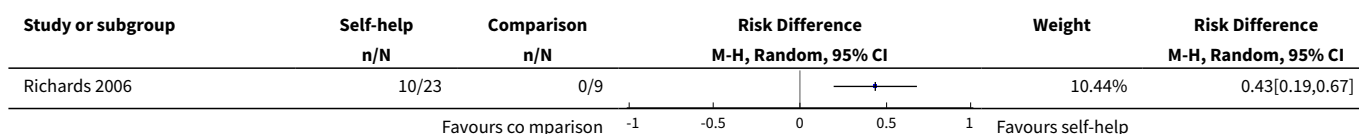
Analysis 1.27. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 27 Response (specific), clinician-rated at post-treatment.

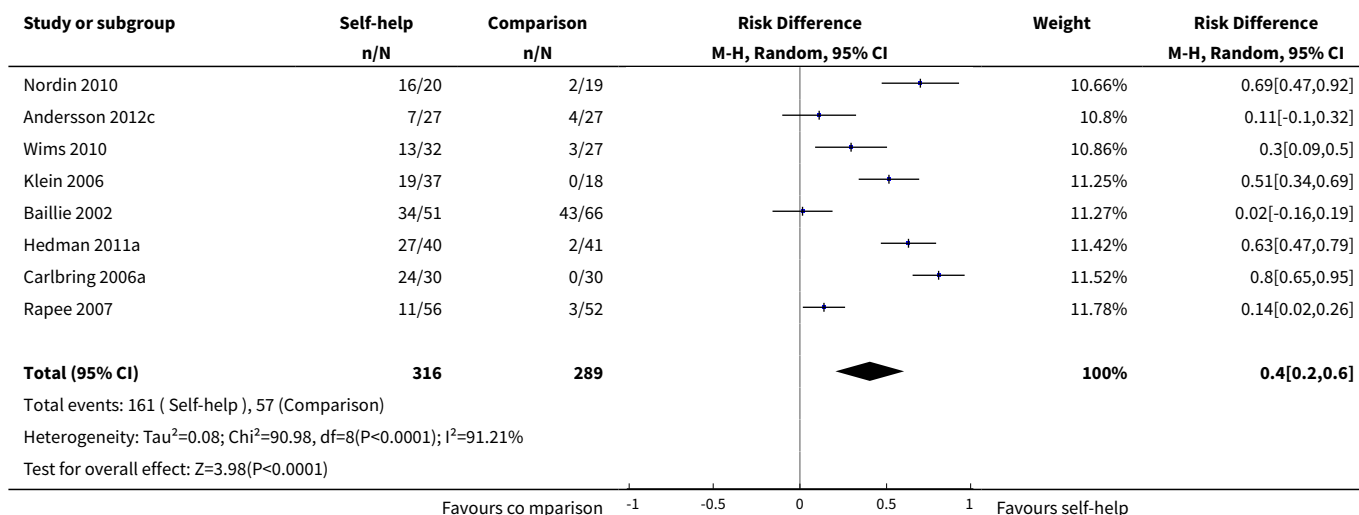


Analysis 1.28. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 28 Response (general), self-rated at post-treatment.

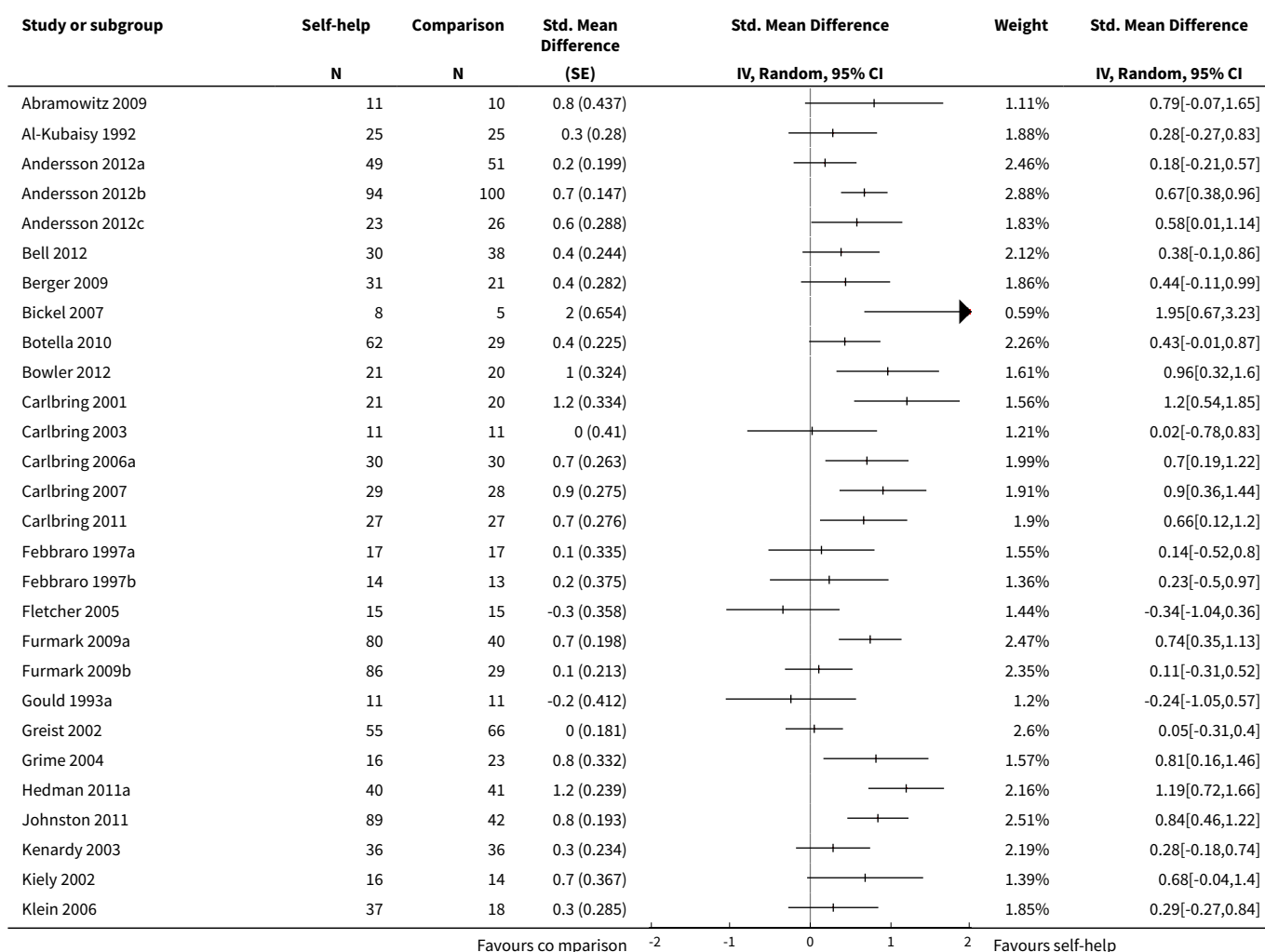


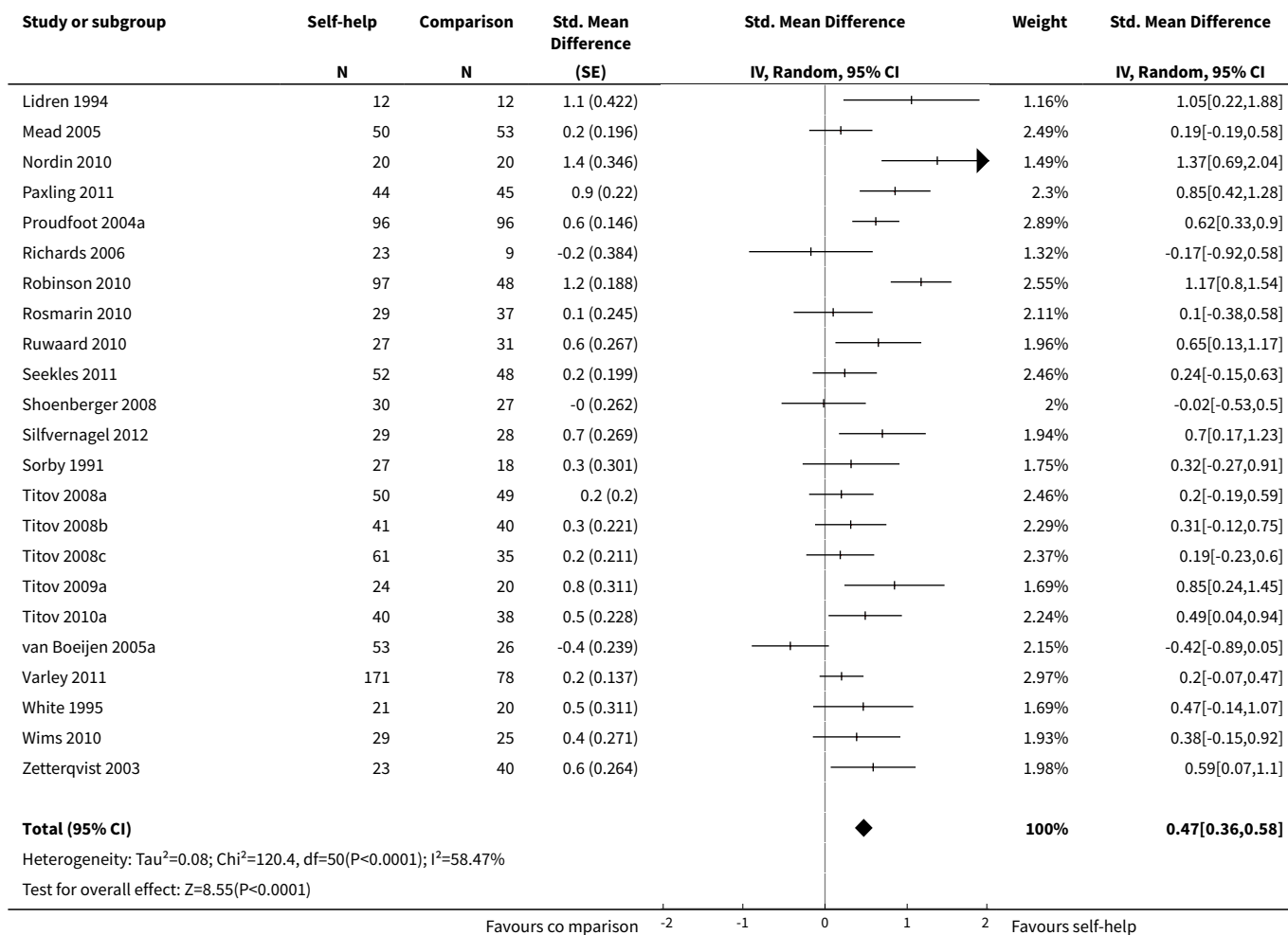
Analysis 1.29. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 29 Recovery, clinician-rated at post-treatment.



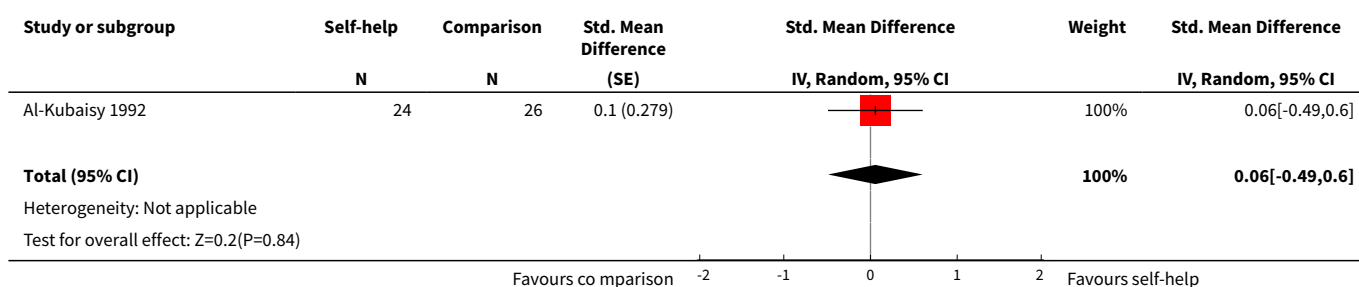


Analysis 1.30. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 30 Depression, self-rated at post-treatment.

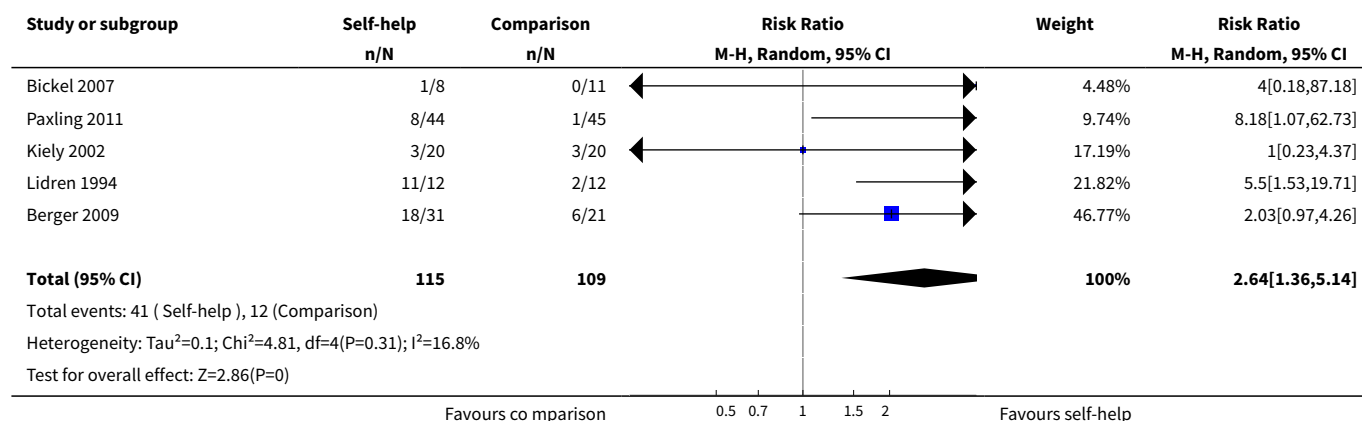




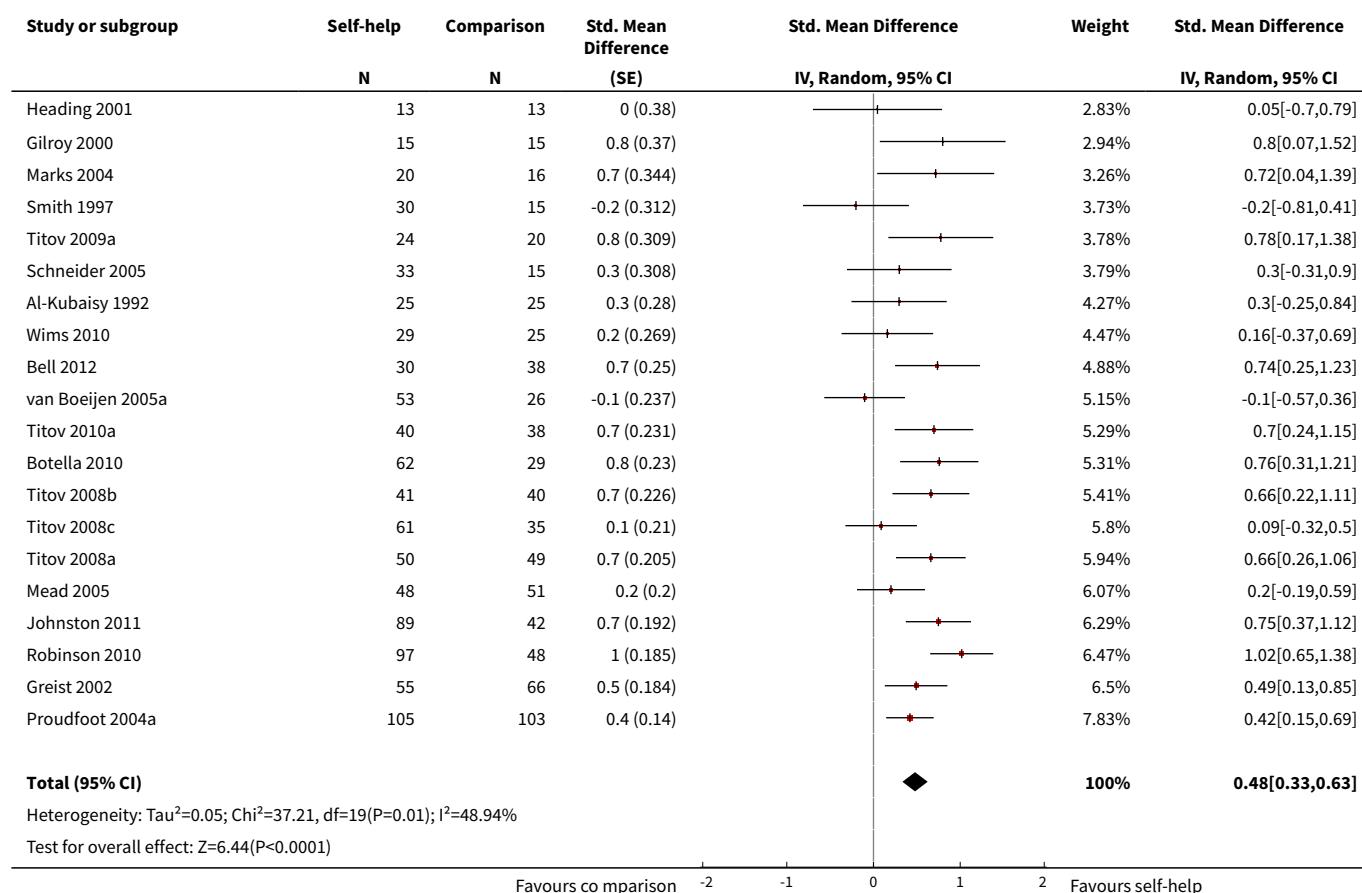
Analysis 1.31. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 31 Depression, clinician-rated at post-treatment.



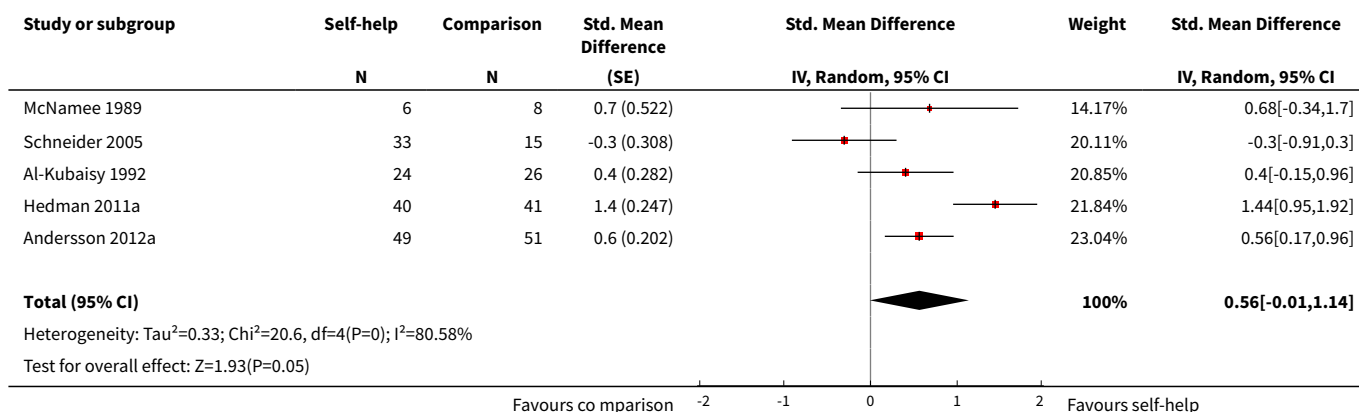
Analysis 1.32. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 32 Depression improvement, self-rated at post-treatment.



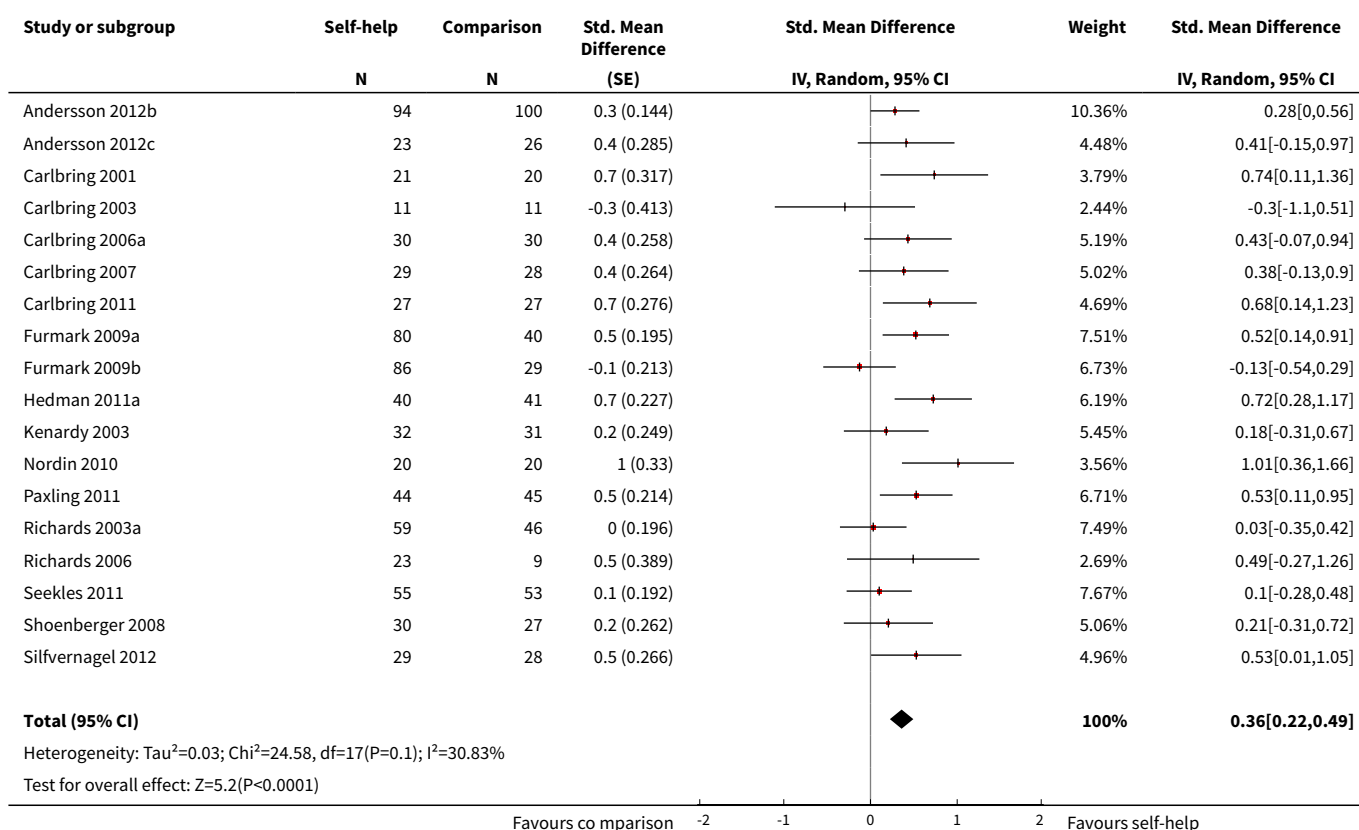
Analysis 1.33. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 33 Disability, self-rated at post-treatment.



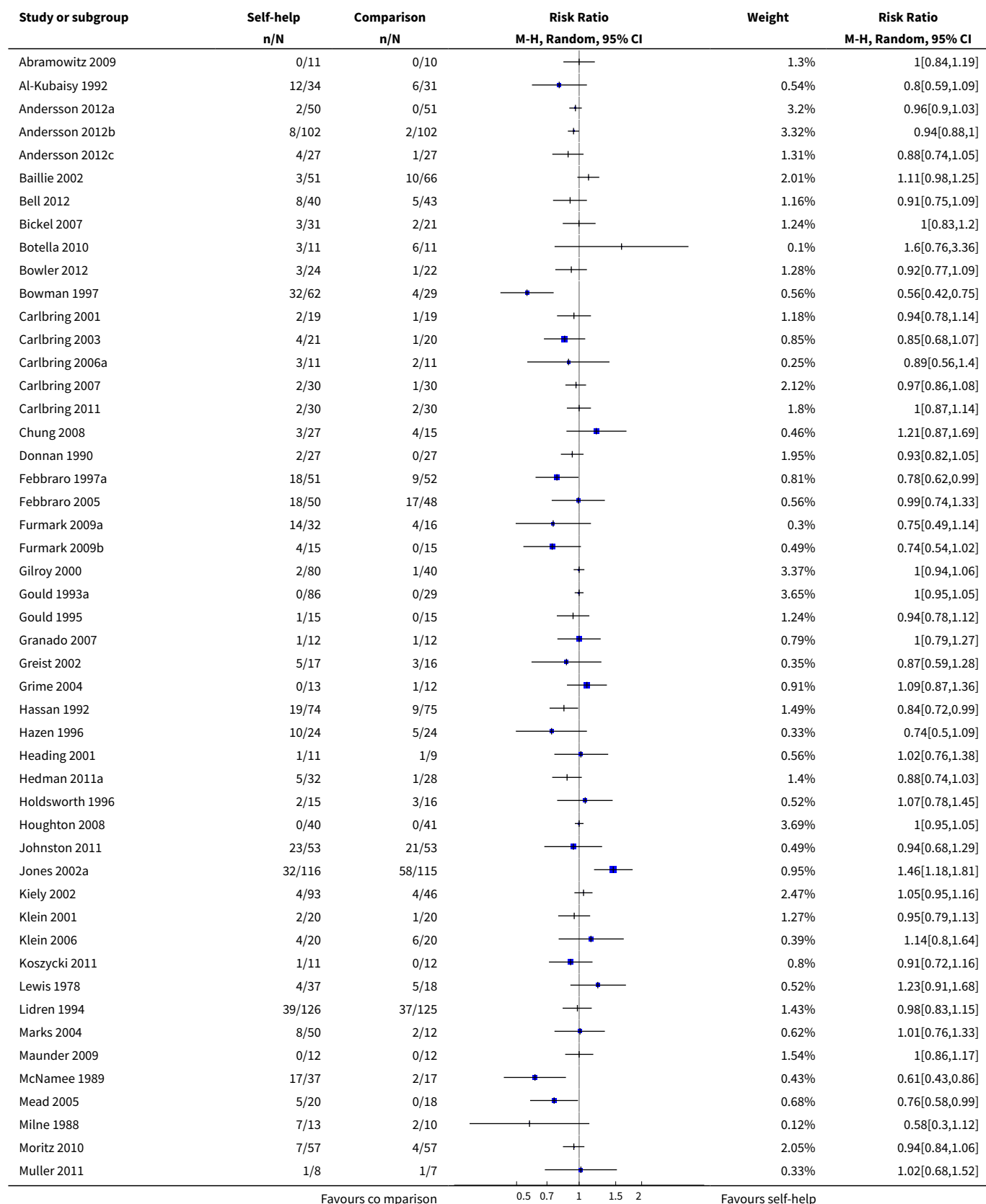
Analysis 1.34. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 34 Disability, clinician-rated at post-treatment.

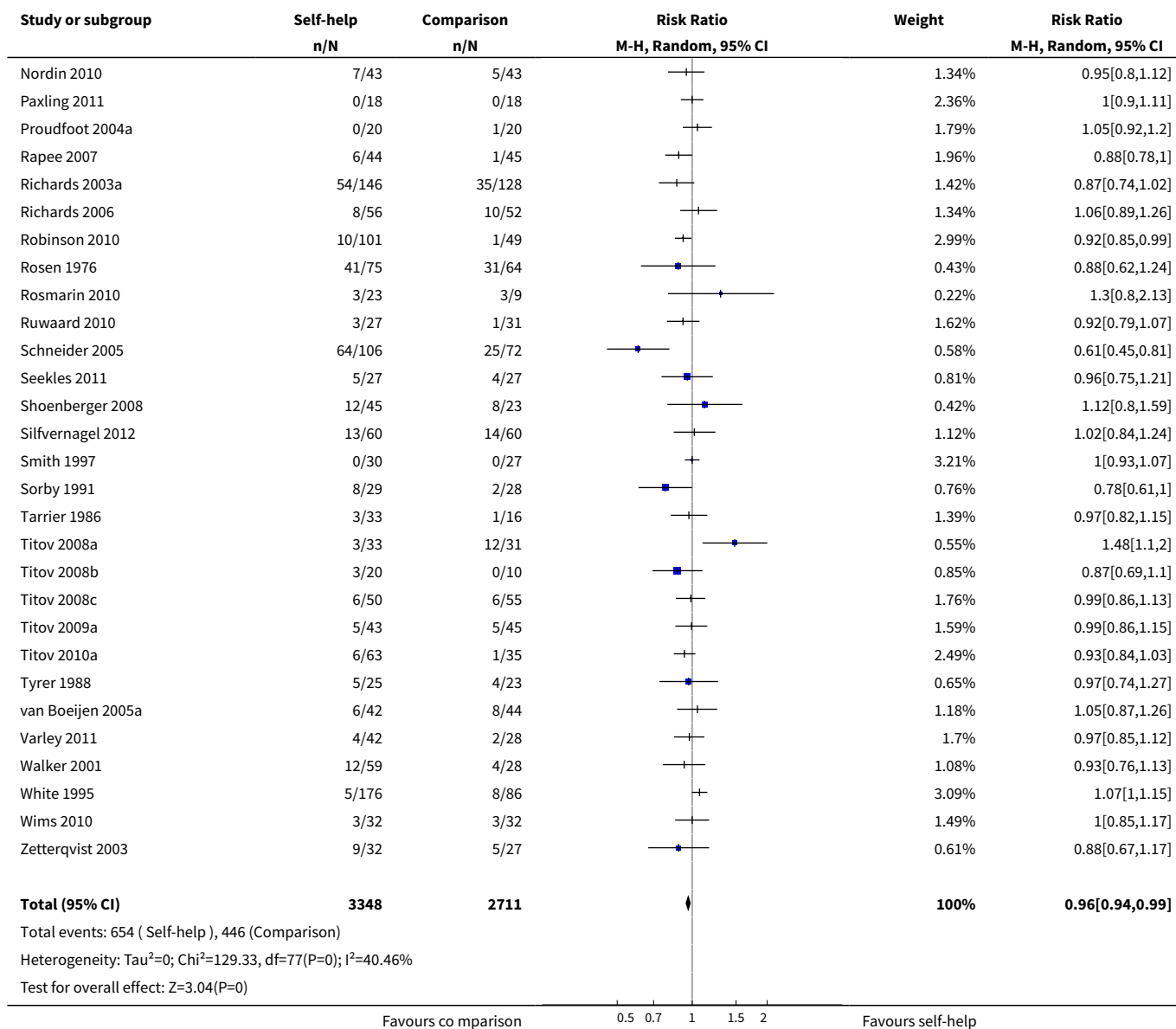


Analysis 1.35. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 35 Quality of Life, self-rated at post-treatment.

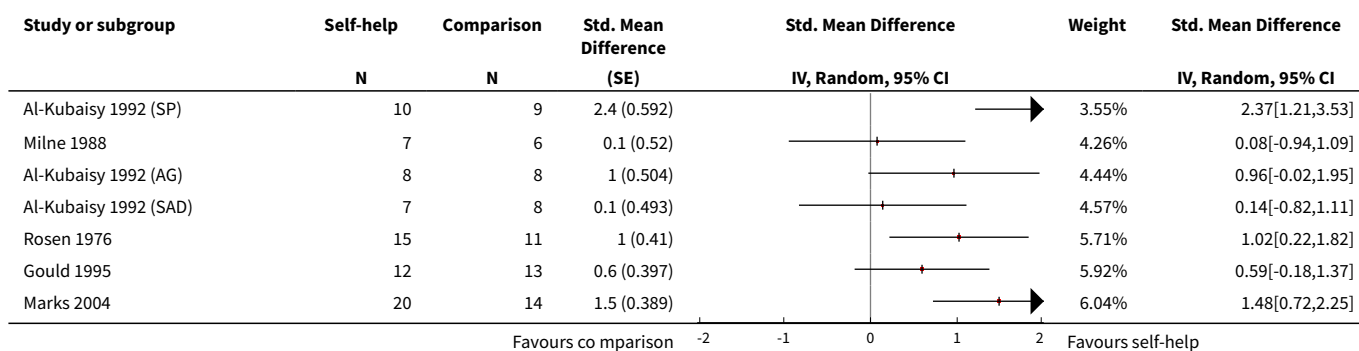


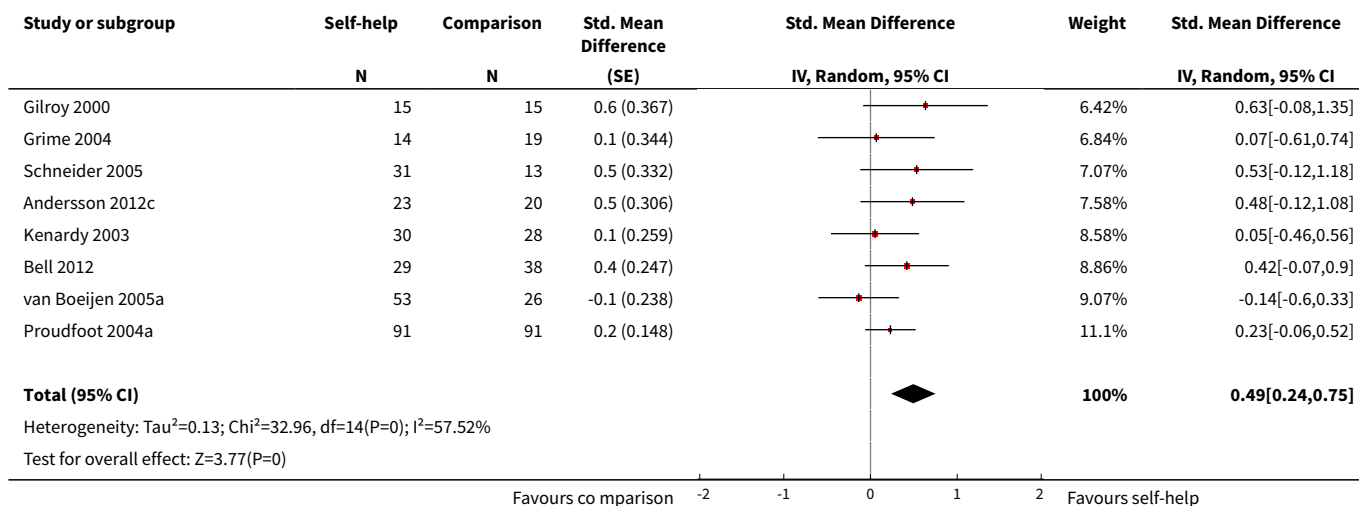
Analysis 1.36. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 36 Dropout at post-treatment.



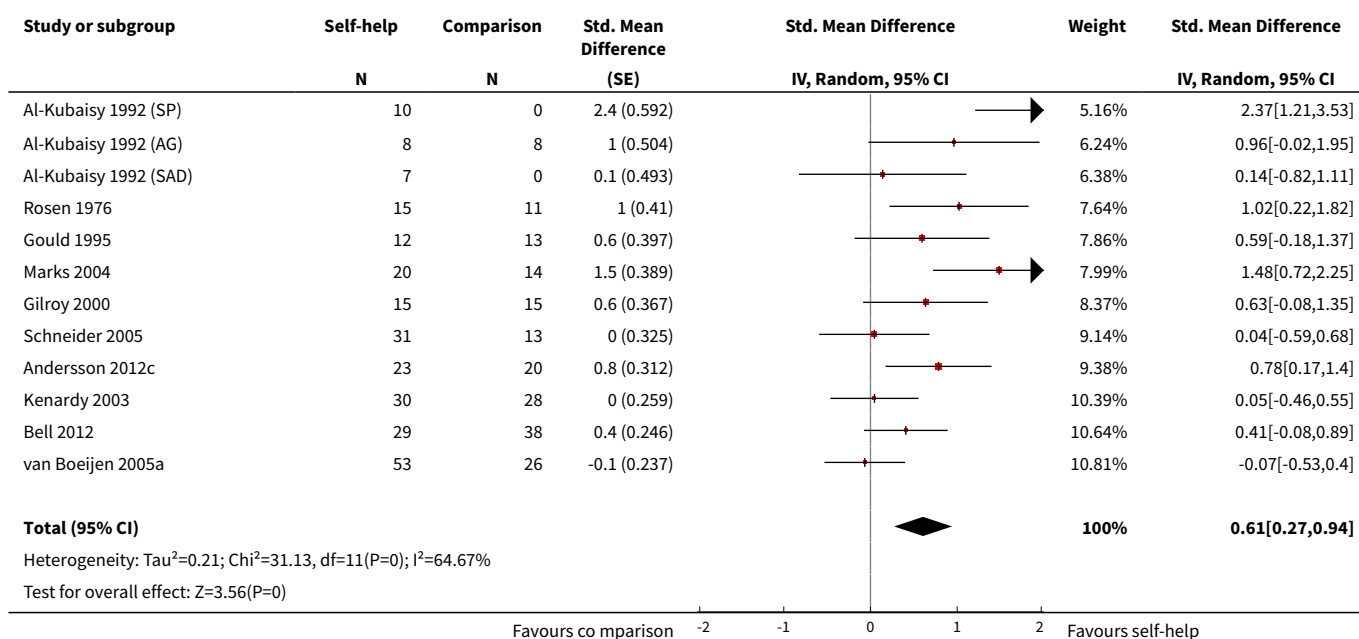


Analysis 1.37. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 37 Anxiety (any measure), self-rated at follow-up.

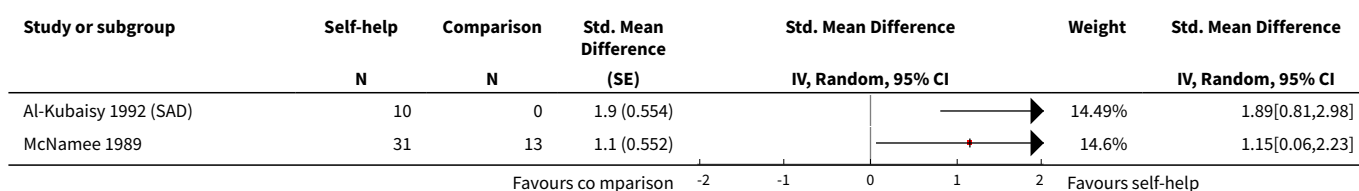


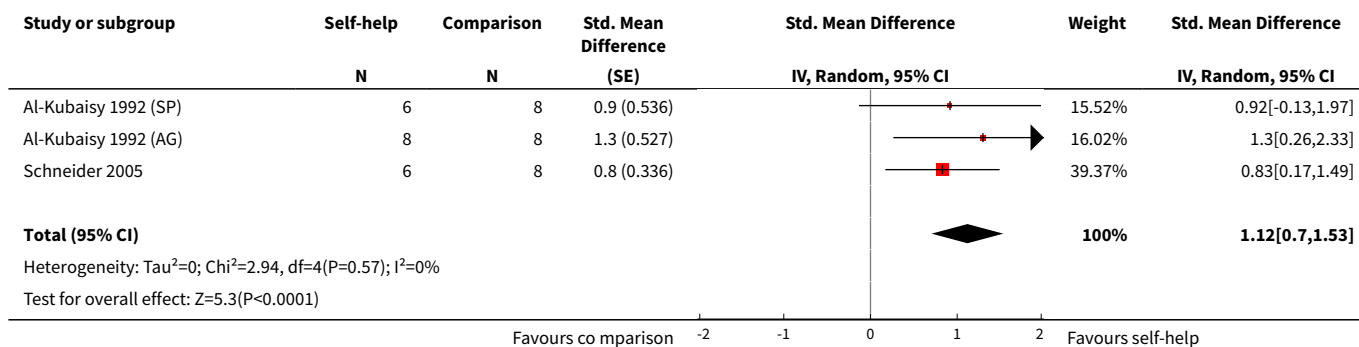


Analysis 1.38. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 38 Specific symptoms, self-rated at follow-up.

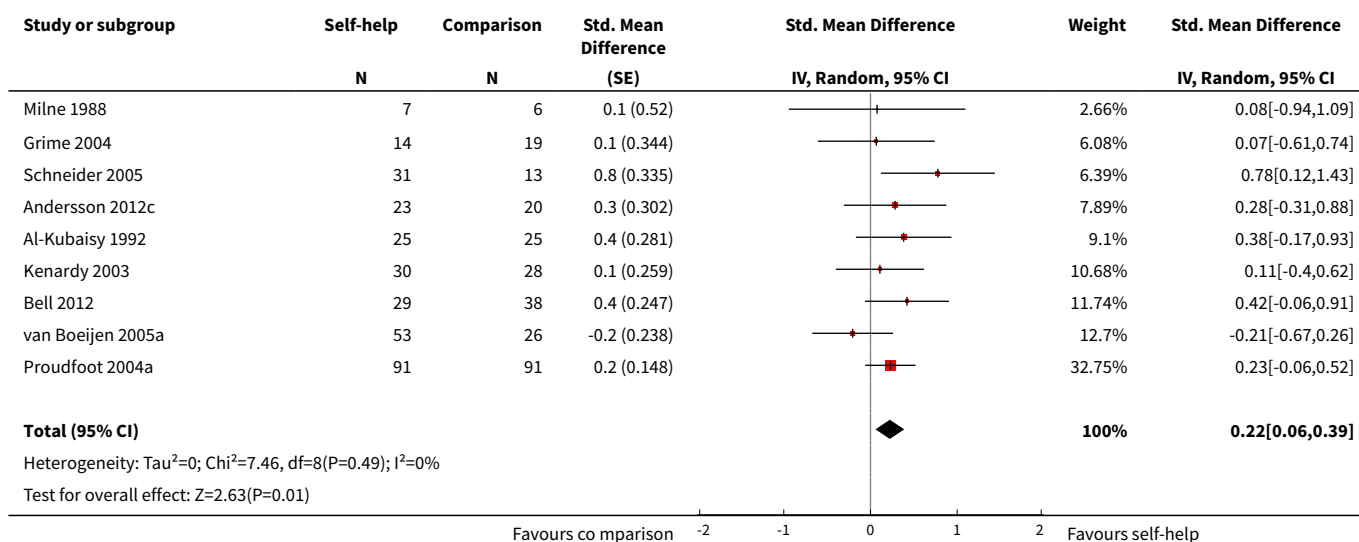


Analysis 1.39. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 39 Specific symptoms, clinician-rated at follow-up.

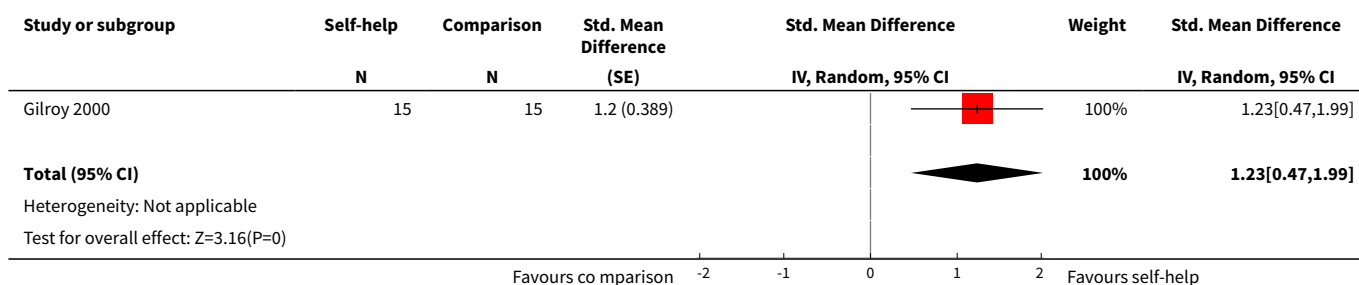




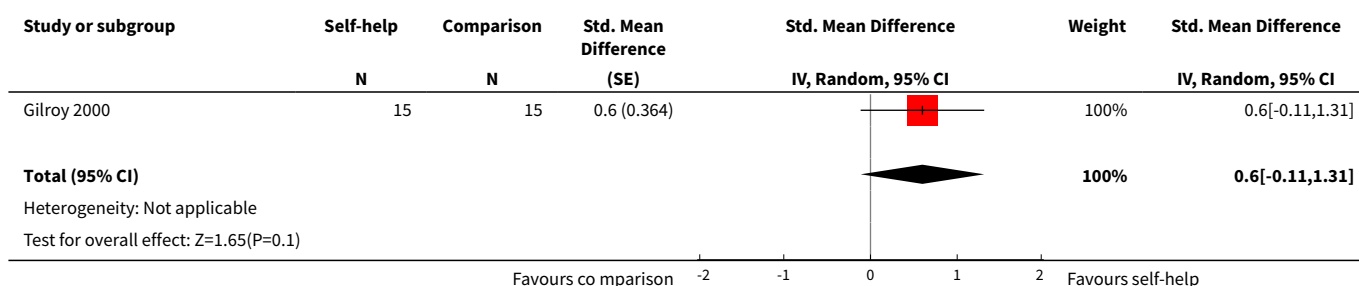
Analysis 1.40. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 40 General symptoms, self-rated at follow-up.



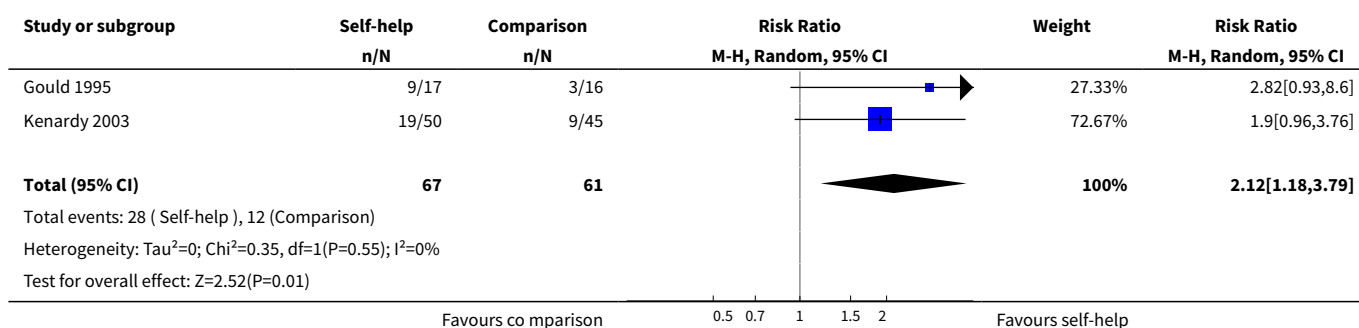
Analysis 1.41. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 41 Behavioural Test (level), clinician-rated at follow-up.



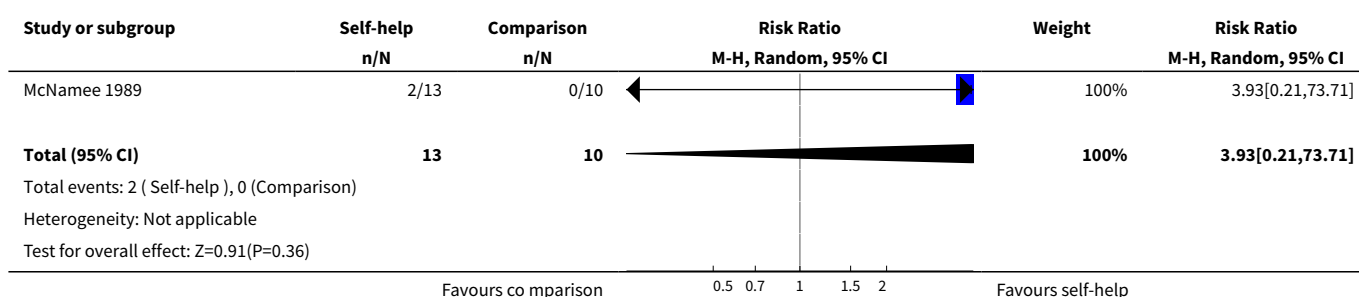
Analysis 1.42. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 42 Behavioural Test (symptoms), self-rated at follow-up.



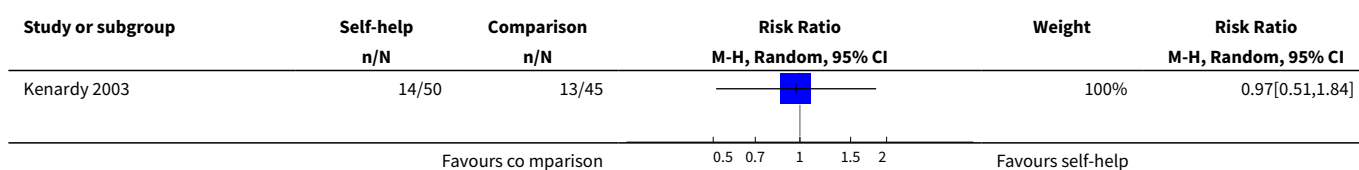
Analysis 1.43. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 43 Response (specific), self-rated at follow-up.

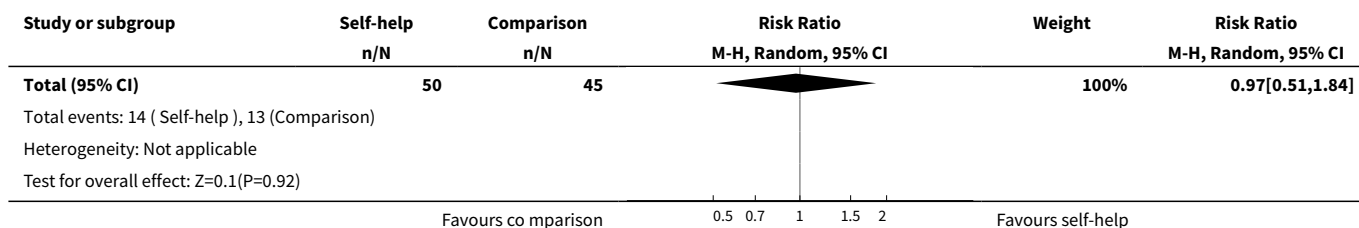


Analysis 1.44. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 44 Response (specific), clinician-rated at follow-up.

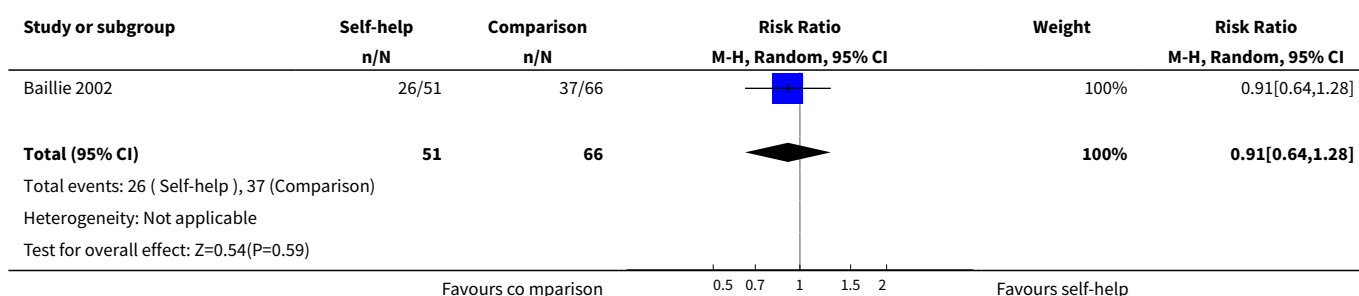


Analysis 1.45. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 45 Response (general), self-rated at follow-up.

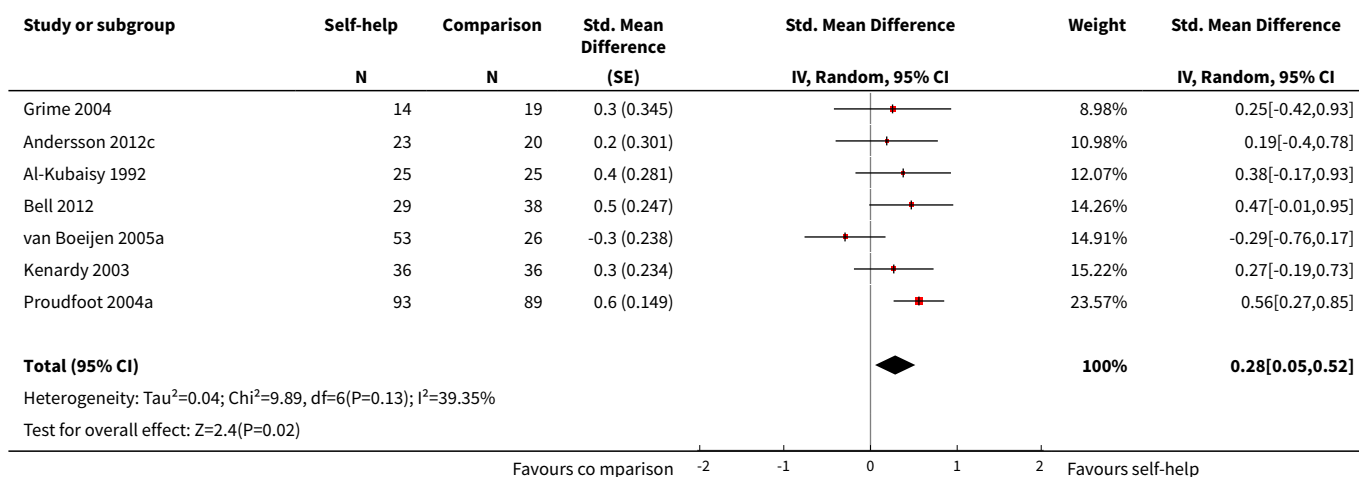




Analysis 1.46. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 46 Recovery, clinician-rated at follow-up.

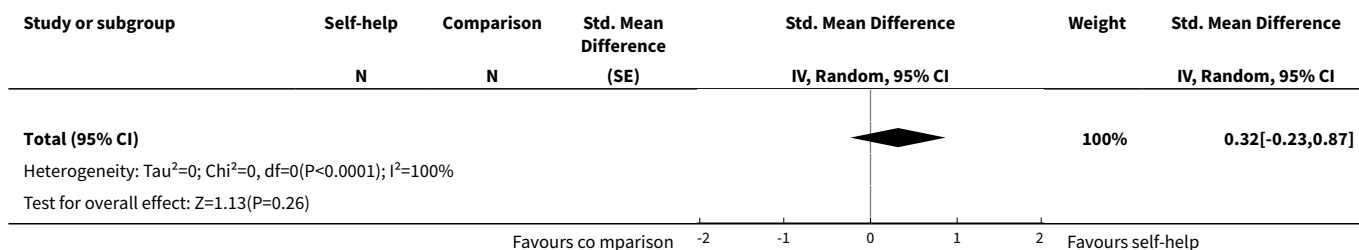


Analysis 1.47. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 47 Depression, self-rated at follow-up.

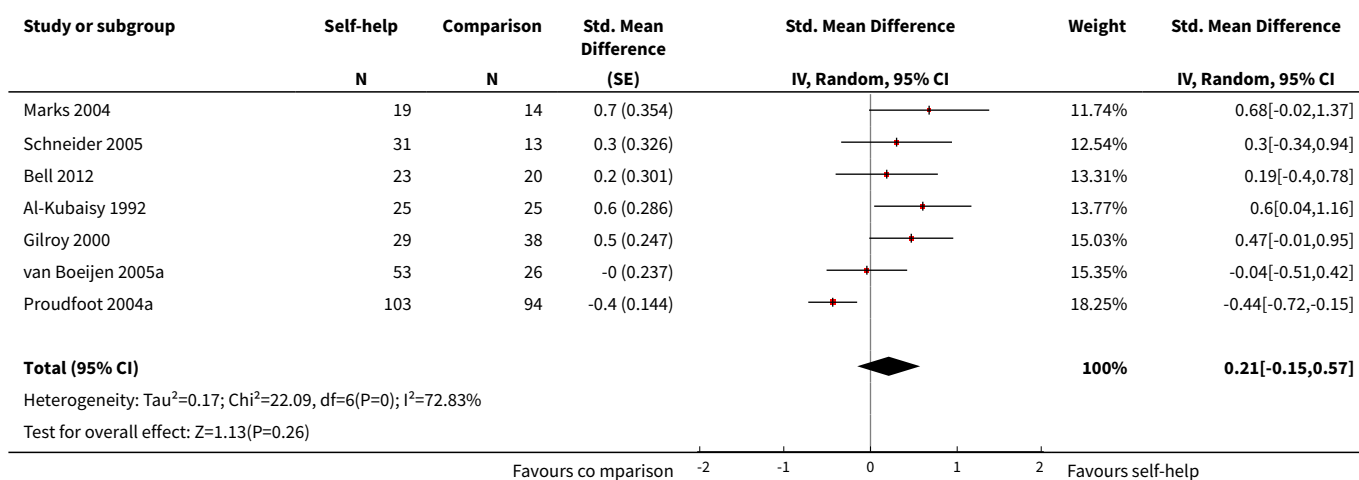


Analysis 1.48. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 48 Depression, clinician-rated at follow-up.

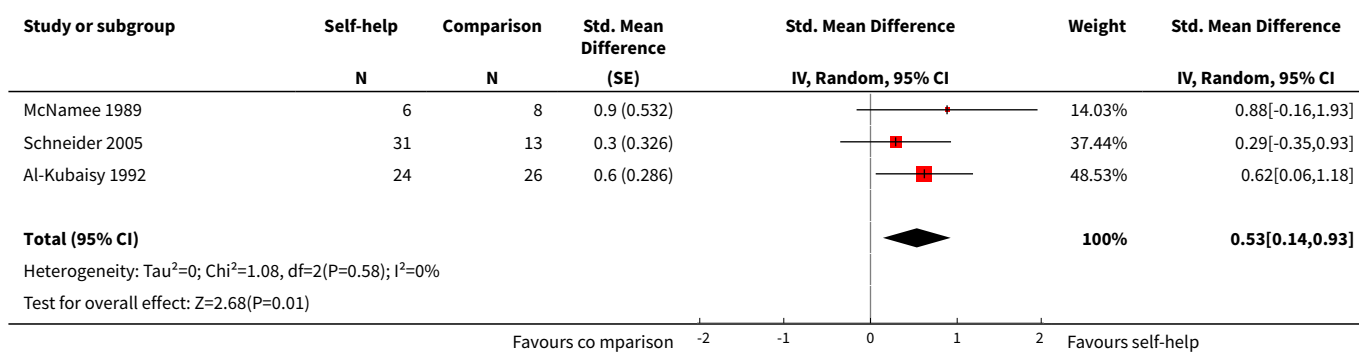




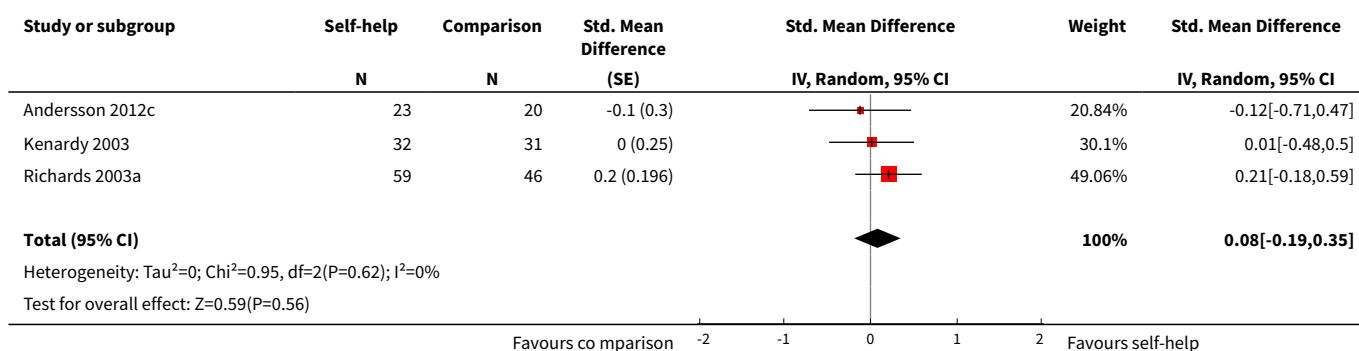
Analysis 1.49. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 49 Disability, self-rated at follow-up.



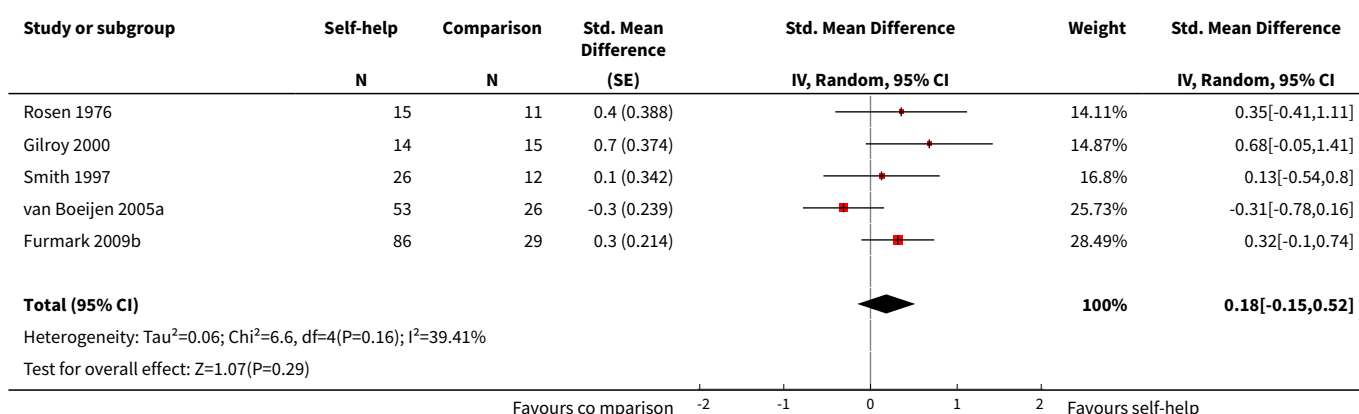
Analysis 1.50. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 50 Disability, clinician-rated at follow-up.



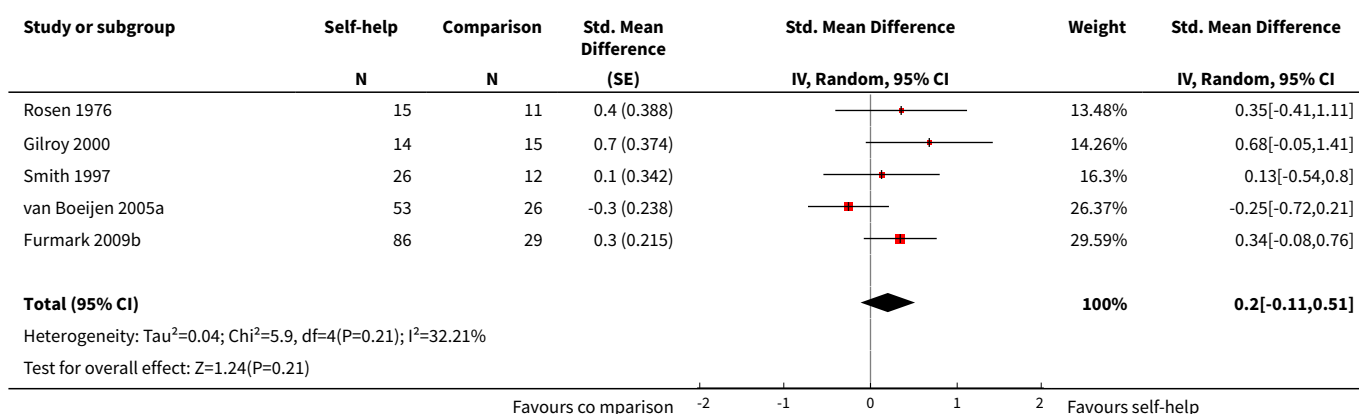
Analysis 1.51. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 51 Quality of Life, self-rated at follow-up.



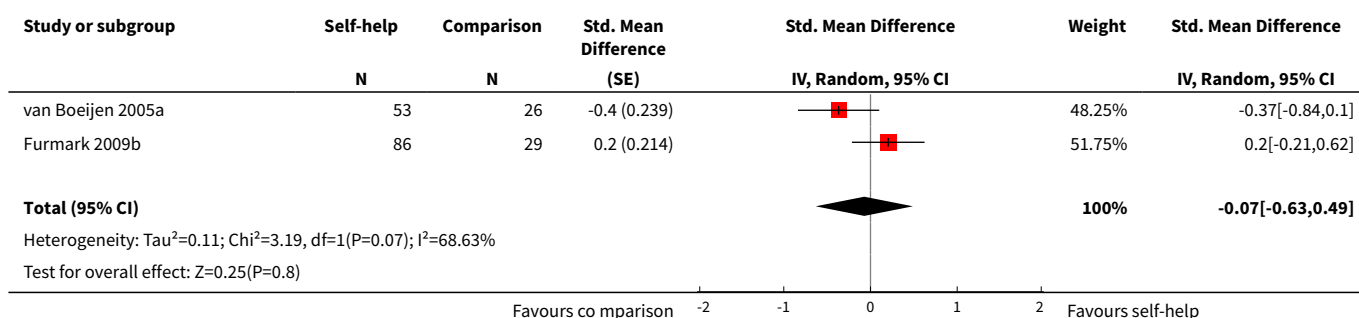
Analysis 1.52. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 52 Anxiety (any measure), self-rated at >6m.



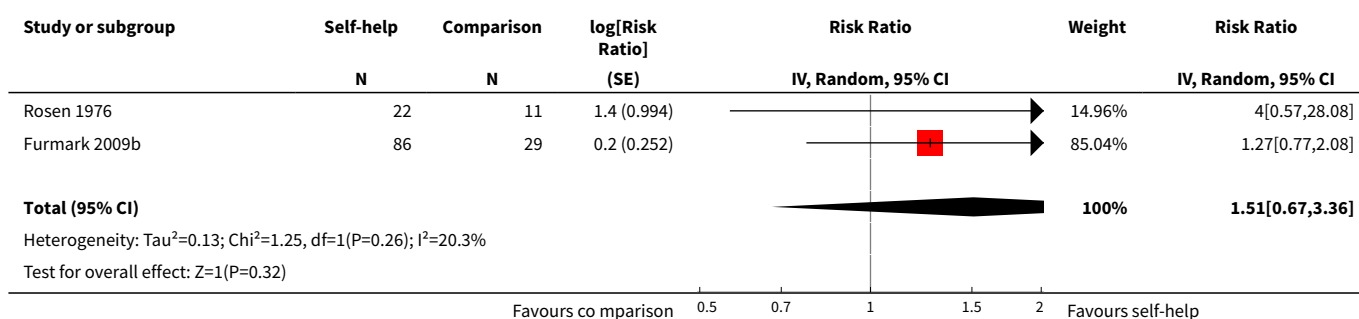
Analysis 1.53. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 53 Specific symptoms, self-rated at >6m.



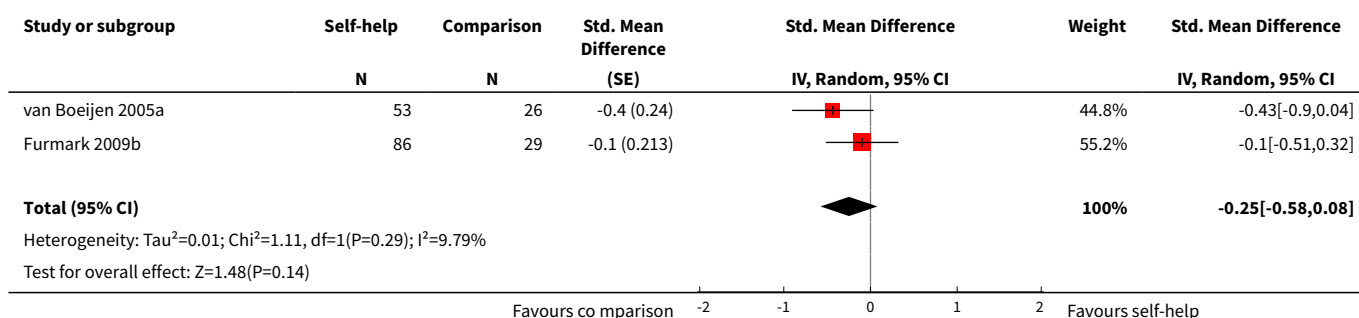
Analysis 1.54. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 54 General symptoms, self-rated at >6m.



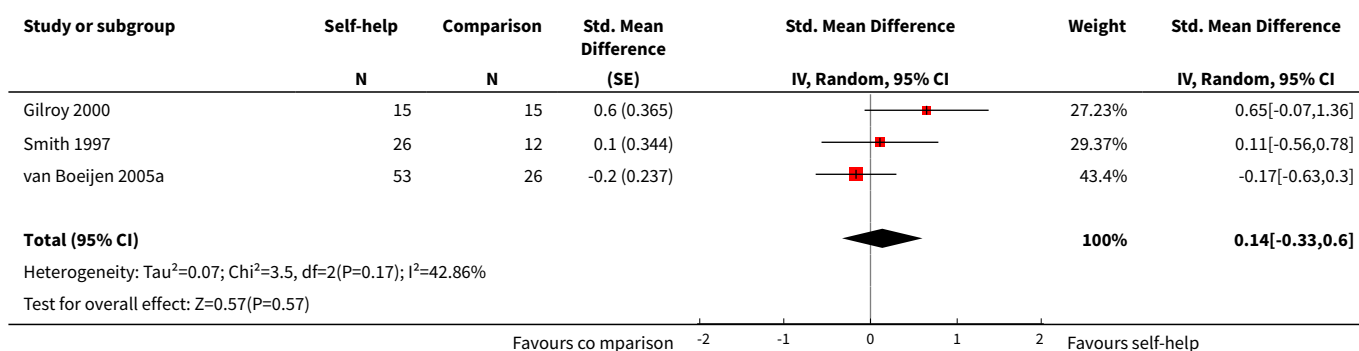
Analysis 1.55. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 55 Response (specific), self-rated at >6m.



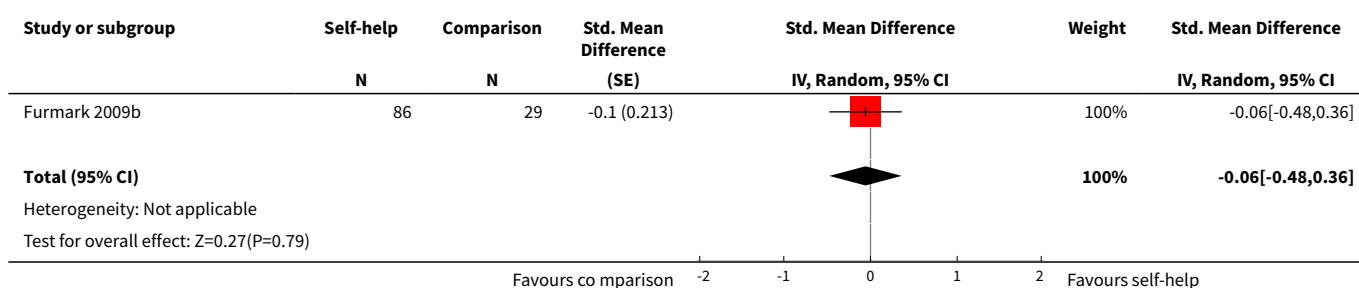
Analysis 1.56. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 56 Depression, self-rated at >6m.



Analysis 1.57. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 57 Disability, self-rated at >6m.



Analysis 1.58. Comparison 1 Media-delivered interventions compared to no intervention, Outcome 58 Quality of Life, self-rated at >6m.



Comparison 2. Media-delivered interventions compared to face-to-face interventions

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
1 Anxiety (any measure), self-rated at post-treatment	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]
2 Anxiety (any measure), self-rated at post-treatment (group versus individual)	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]
2.1 Group CBT	5	477	Std. Mean Difference (Random, 95% CI)	-0.18 [-0.44, 0.09]
2.2 Individual CBT	9	549	Std. Mean Difference (Random, 95% CI)	-0.17 [-0.34, 0.00]
2.3 Individual Exposure	10	334	Std. Mean Difference (Random, 95% CI)	-0.39 [-0.65, -0.13]
3 Anxiety (any measure), self-rated at post-treatment (by disorder)	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
3.1 Mixed	2	165	Std. Mean Difference (Random, 95% CI)	-0.25 [-0.56, 0.05]
3.2 OCD	2	147	Std. Mean Difference (Random, 95% CI)	-0.26 [-0.59, 0.06]
3.3 Panic	8	430	Std. Mean Difference (Random, 95% CI)	-0.27 [-0.51, -0.03]
3.4 Social	4	373	Std. Mean Difference (Random, 95% CI)	0.02 [-0.18, 0.22]
3.5 Specific	8	245	Std. Mean Difference (Random, 95% CI)	-0.39 [-0.73, -0.06]
4 Anxiety (any measure), self-rated at post-treatment (by media)	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]
4.1 Book	10	535	Std. Mean Difference (Random, 95% CI)	-0.34 [-0.60, -0.08]
4.2 Computer	4	136	Std. Mean Difference (Random, 95% CI)	-0.38 [-0.71, -0.04]
4.3 Internet	7	519	Std. Mean Difference (Random, 95% CI)	-0.05 [-0.22, 0.12]
4.4 Other	3	170	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.53, 0.07]
5 Anxiety (any measure), self-rated at post-treatment (by therapy)	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]
5.1 CBT	13	916	Std. Mean Difference (Random, 95% CI)	-0.16 [-0.31, -0.00]
5.2 Exposure/ Desensitization	10	419	Std. Mean Difference (Random, 95% CI)	-0.37 [-0.62, -0.13]
5.3 Muscle relaxation	1	25	Std. Mean Difference (Random, 95% CI)	-0.14 [-0.92, 0.65]
6 Anxiety (any measure), self-rated at post-treatment (by setting)	24	1360	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]
6.1 Advertising	17	916	Std. Mean Difference (Random, 95% CI)	-0.15 [-0.32, 0.02]
6.2 Referral Only	7	444	Std. Mean Difference (Random, 95% CI)	-0.37 [-0.56, -0.18]
7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias)	24		Std. Mean Difference (Random, 95% CI)	-0.23 [-0.36, -0.09]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
7.1 High or Unclear Risk for Incomplete Outcome Data	11		Std. Mean Difference (Random, 95% CI)	-0.28 [-0.44, -0.13]
7.2 Low Risk for Incomplete Outcome Data	13		Std. Mean Difference (Random, 95% CI)	-0.18 [-0.40, 0.04]
8 Specific symptoms, self-rated at post-treatment	23	1255	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
9 Specific symptoms, self-rated at post-treatment (group versus individual)	23		Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
9.1 Group CBT	4		Std. Mean Difference (Random, 95% CI)	-0.17 [-0.52, 0.19]
9.2 Individual CBT	9		Std. Mean Difference (Random, 95% CI)	-0.18 [-0.35, -0.02]
9.3 Individual Exposure	10		Std. Mean Difference (Random, 95% CI)	-0.40 [-0.65, -0.14]
10 Specific symptoms, self-rated at post-treatment (by disorder)	23	1255	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
10.1 Mixed	2	165	Std. Mean Difference (Random, 95% CI)	-0.24 [-0.54, 0.07]
10.2 OCD	2	145	Std. Mean Difference (Random, 95% CI)	-0.29 [-0.62, 0.03]
10.3 Panic	7	327	Std. Mean Difference (Random, 95% CI)	-0.30 [-0.56, -0.04]
10.4 Social	4	373	Std. Mean Difference (Random, 95% CI)	0.03 [-0.18, 0.23]
10.5 Specific	8	245	Std. Mean Difference (Random, 95% CI)	-0.40 [-0.74, -0.06]
11 Specific symptoms, self-rated at post-treatment (by media)	22	1275	Std. Mean Difference (Random, 95% CI)	-0.26 [-0.39, -0.12]
11.1 Book	10	553	Std. Mean Difference (Random, 95% CI)	-0.42 [-0.65, -0.20]
11.2 Computer	4	136	Std. Mean Difference (Random, 95% CI)	-0.38 [-0.71, -0.04]
11.3 Internet	6	416	Std. Mean Difference (Random, 95% CI)	0.01 [-0.18, 0.20]
11.4 Other	3	170	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.54, 0.07]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
12 Specific symptoms, self-rated at post-treatment (by therapy)	23	1255	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
12.1 CBT	12	811	Std. Mean Difference (Random, 95% CI)	-0.16 [-0.33, 0.01]
12.2 Exposure/ Desensitization	10	419	Std. Mean Difference (Random, 95% CI)	-0.38 [-0.62, -0.14]
12.3 Muscle relaxation	1	25	Std. Mean Difference (Random, 95% CI)	-0.14 [-0.92, 0.65]
13 Specific symptoms, self-rated at post-treatment (by setting)	23	1255	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
13.1 Advertising	17	917	Std. Mean Difference (Random, 95% CI)	-0.16 [-0.32, 0.00]
13.2 Referral Only	6	338	Std. Mean Difference (Random, 95% CI)	-0.42 [-0.64, -0.21]
14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias)	23	1255	Std. Mean Difference (Random, 95% CI)	-0.23 [-0.37, -0.10]
14.1 High or Unclear Risk for Incomplete Outcome Data	10	523	Std. Mean Difference (Random, 95% CI)	-0.29 [-0.47, -0.12]
14.2 Low Risk for Incomplete Outcome Data	13	732	Std. Mean Difference (Random, 95% CI)	-0.20 [-0.41, 0.02]
15 Specific symptoms, clinician-rated at post-treatment	6	418	Std. Mean Difference (Random, 95% CI)	-0.13 [-0.33, 0.06]
16 General symptoms, self-rated at post-treatment	11	627	Std. Mean Difference (Random, 95% CI)	-0.08 [-0.36, 0.20]
17 Behavioural Test (level), clinician-rated at post-treatment	5	211	Std. Mean Difference (Random, 95% CI)	-0.44 [-0.76, -0.13]
18 Behavioural Test (symptoms), self-rated at post-treatment	5	209	Std. Mean Difference (Random, 95% CI)	-0.57 [-1.35, 0.21]
19 Response (specific), self-rated at post-treatment	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
20 Response (specific), self-rated at post-treatment (group versus individual)	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
20.1 Group CBT	1	126	Risk Ratio (M-H, Random, 95% CI)	1.61 [1.07, 2.44]
20.2 Individual CBT	7	382	Risk Ratio (M-H, Random, 95% CI)	0.73 [0.53, 1.01]

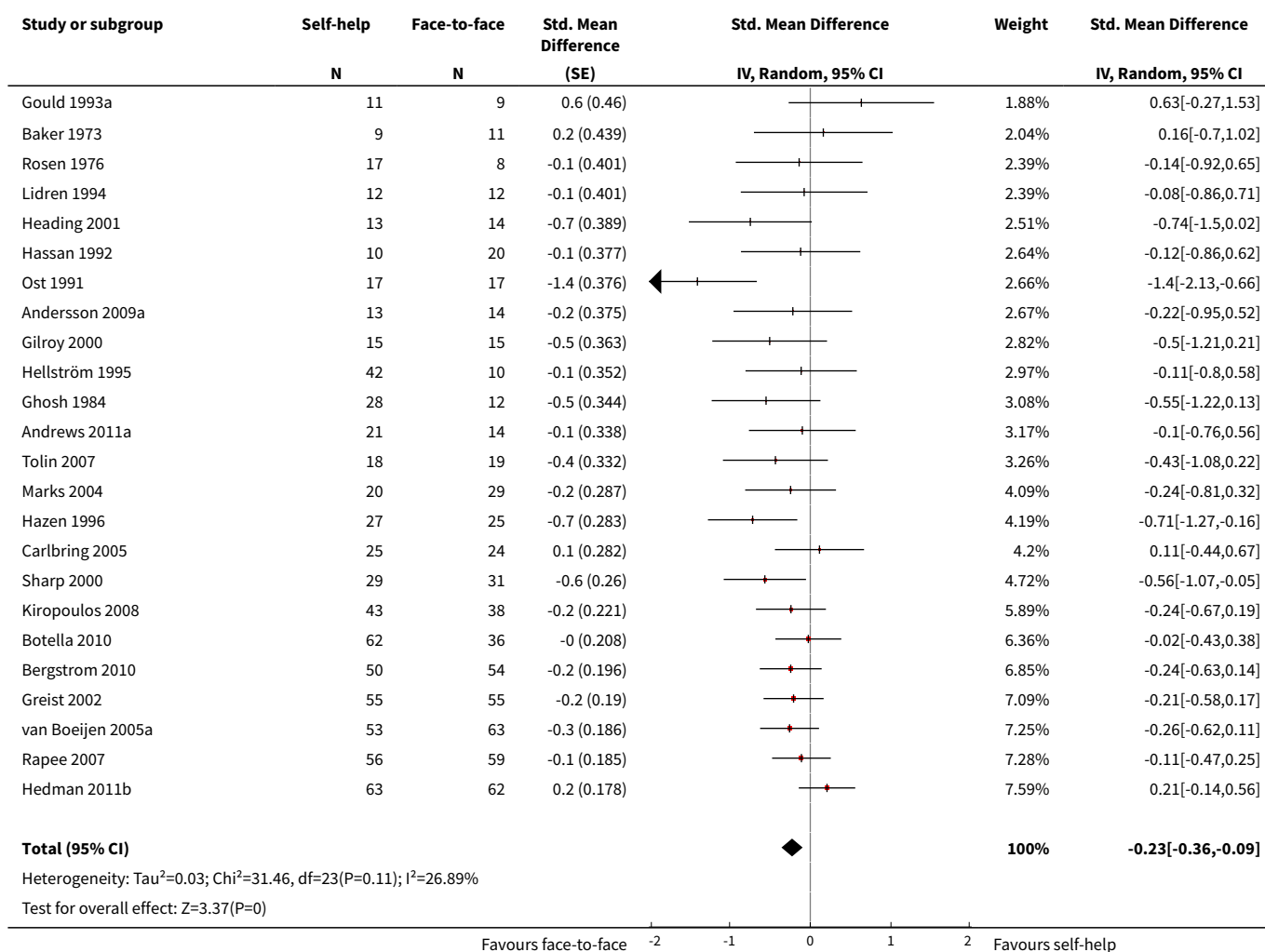
Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
20.3 Individual Exposure	2	67	Risk Ratio (M-H, Random, 95% CI)	0.39 [0.02, 7.29]
21 Response (specific), self-rated at post-treatment (by disorder)	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
21.1 OCD	3	217	Risk Ratio (M-H, Random, 95% CI)	0.59 [0.40, 0.87]
21.2 Panic	4	165	Risk Ratio (M-H, Random, 95% CI)	0.82 [0.55, 1.23]
21.3 Social	1	126	Risk Ratio (M-H, Random, 95% CI)	1.61 [1.07, 2.44]
21.4 Specific	2	67	Risk Ratio (M-H, Random, 95% CI)	0.39 [0.02, 7.29]
22 Response (specific), self-rated at post-treatment (by media)	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
22.1 Book	6	224	Risk Ratio (M-H, Random, 95% CI)	0.58 [0.30, 1.10]
22.2 Computer	1	33	Risk Ratio (M-H, Random, 95% CI)	1.06 [0.74, 1.52]
22.3 Internet	2	175	Risk Ratio (M-H, Random, 95% CI)	1.17 [0.61, 2.25]
22.4 Other	1	143	Risk Ratio (M-H, Random, 95% CI)	0.61 [0.39, 0.95]
23 Response (specific), self-rated at post-treatment (by therapy)	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
23.1 CBT	7	365	Risk Ratio (M-H, Random, 95% CI)	0.87 [0.60, 1.26]
23.2 Exposure/ Desensitization	3	210	Risk Ratio (M-H, Random, 95% CI)	0.54 [0.22, 1.34]
24 Response (specific), self-rated at post-treatment (by setting)	10	575	Risk Ratio (M-H, Random, 95% CI)	0.78 [0.56, 1.09]
24.1 Advertising	7	430	Risk Ratio (M-H, Random, 95% CI)	0.90 [0.64, 1.27]
24.2 Referral Only	3	145	Risk Ratio (M-H, Random, 95% CI)	0.48 [0.30, 0.76]
25 Response (specific), clinician-rated at post-treatment	9	663	Risk Ratio (M-H, Random, 95% CI)	0.49 [0.31, 0.77]

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
26 Response (general), self-rated at post-treatment	1		Risk Ratio (M-H, Random, 95% CI)	Totals not selected
27 Recovery, clinician-rated at post-treatment	6	587	Risk Ratio (M-H, Random, 95% CI)	1.05 [0.88, 1.24]
28 Depression, self-rated at post-treatment	13	906	Std. Mean Difference (Random, 95% CI)	-0.01 [-0.22, 0.21]
29 Depression, clinician-rated at post-treatment	1		Std. Mean Difference (Random, 95% CI)	Totals not selected
30 Depression improvement, self-rated at post-treatment	2	73	Risk Ratio (M-H, Random, 95% CI)	1.21 [0.74, 1.97]
31 Disability, self-rated at post-treatment	10	670	Std. Mean Difference (Random, 95% CI)	-0.07 [-0.33, 0.20]
32 Disability, clinician-rated at post-treatment	3	200	Std. Mean Difference (Random, 95% CI)	0.24 [-0.04, 0.52]
33 Quality of Life, self-rated at post-treatment	4	282	Std. Mean Difference (Random, 95% CI)	0.08 [-0.23, 0.38]
34 Dropout at post-treatment	28	1852	Risk Ratio (M-H, Random, 95% CI)	0.99 [0.95, 1.03]
35 Anxiety (any measure), self-rated at follow-up	11	677	Std. Mean Difference (Random, 95% CI)	-0.14 [-0.30, 0.01]
36 Specific symptoms, self-rated at follow-up	10	573	Std. Mean Difference (Random, 95% CI)	-0.15 [-0.31, 0.02]
37 Specific symptoms, clinician-rated at follow-up	3	185	Std. Mean Difference (Random, 95% CI)	-0.19 [-0.69, 0.30]
38 General symptoms, self-rated at follow-up	5	308	Std. Mean Difference (Random, 95% CI)	-0.10 [-0.30, 0.09]
39 Behavioural Test (level), clinician-rated at follow-up	2	60	Std. Mean Difference (Random, 95% CI)	-0.02 [-0.74, 0.71]
40 Behavioural Test (symptoms), self-rated at follow-up	1		Std. Mean Difference (Random, 95% CI)	Totals not selected
41 Response (specific), self-rated at follow-up	4	224	Risk Ratio (M-H, Random, 95% CI)	0.99 [0.73, 1.35]
42 Response (specific), clinician-rated at follow-up	3	280	Risk Ratio (M-H, Random, 95% CI)	1.10 [0.83, 1.45]
43 Recovery, clinician-rated at follow-up	3	354	Risk Ratio (M-H, Random, 95% CI)	1.04 [0.70, 1.56]

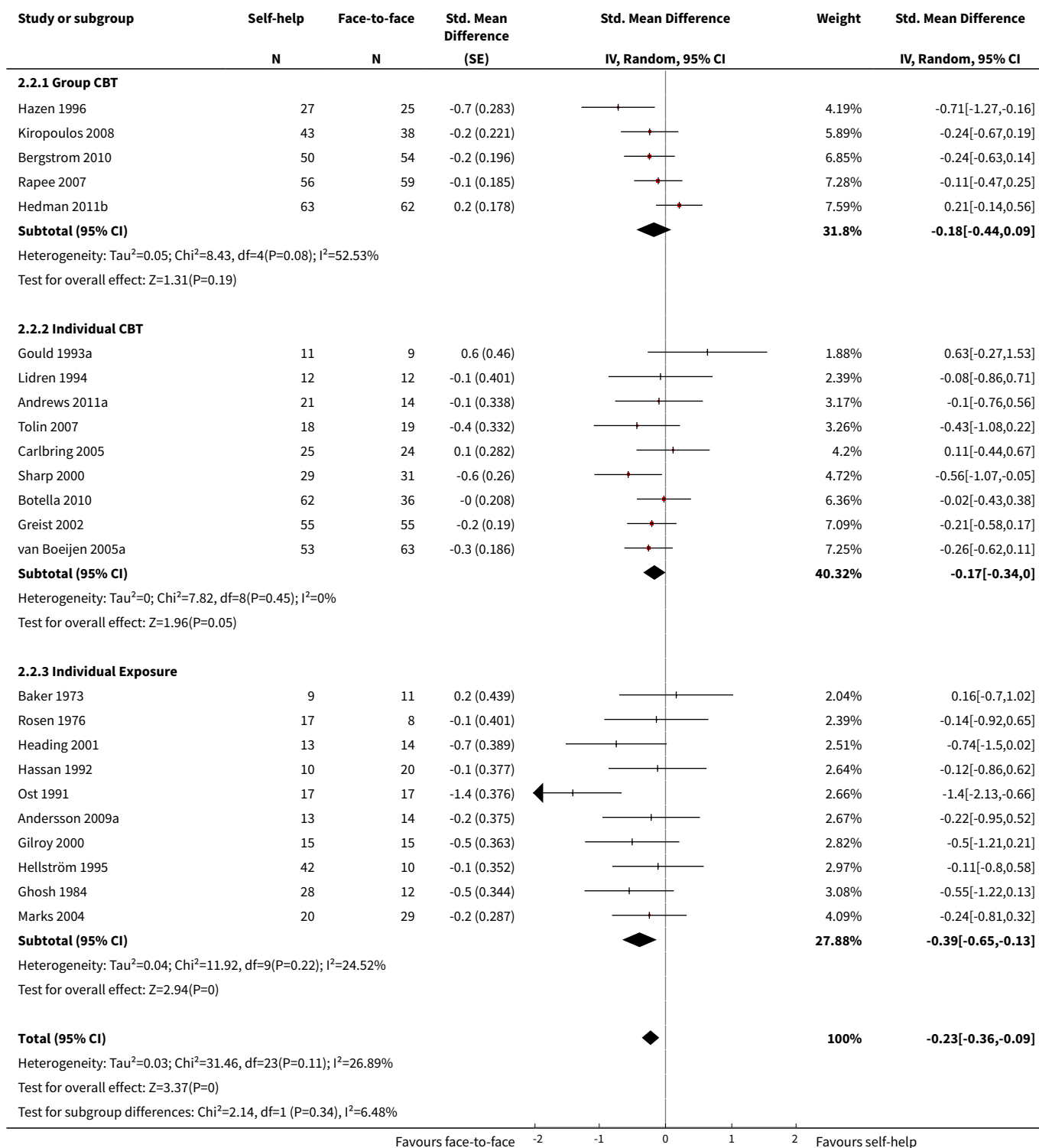
Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
44 Depression, self-rated at follow-up	5	403	Std. Mean Difference (Random, 95% CI)	0.09 [-0.19, 0.36]
45 Depression, clinician-rated at follow-up	1		Std. Mean Difference (Random, 95% CI)	Totals not selected
46 Depression improvement, self-rated at follow-up	1		Risk Ratio (M-H, Random, 95% CI)	Totals not selected
47 Disability, self-rated at follow-up	5	337	Std. Mean Difference (Random, 95% CI)	-0.10 [-0.39, 0.19]
48 Disability, clinician-rated at follow-up	2	145	Std. Mean Difference (Random, 95% CI)	0.18 [-0.30, 0.65]
49 Quality of Life, self-rated at follow-up	2	143	Std. Mean Difference (Random, 95% CI)	0.40 [0.07, 0.73]
50 Anxiety (any measure), self-rated at >6m	8	423	Std. Mean Difference (Random, 95% CI)	-0.16 [-0.50, 0.18]
51 Specific symptoms, self-rated at >6m	8	423	Std. Mean Difference (Random, 95% CI)	-0.20 [-0.54, 0.14]
52 Specific symptoms, clinician-rated at >6m	1		Std. Mean Difference (Random, 95% CI)	Totals not selected
53 General symptoms, self-rated at >6m	4	241	Std. Mean Difference (Random, 95% CI)	0.04 [-0.22, 0.30]
54 Behavioural Test (level), clinician-rated at >6m	2	122	Std. Mean Difference (Random, 95% CI)	-0.11 [-0.97, 0.76]
55 Behavioural Test (symptoms), self-rated at >6m	3	162	Std. Mean Difference (Random, 95% CI)	-0.46 [-1.19, 0.27]
56 Response (specific), self-rated at >6m	3	116	Risk Ratio (M-H, Random, 95% CI)	0.58 [0.20, 1.64]
57 Response (specific), clinician-rated at >6m	2	82	Risk Ratio (M-H, Random, 95% CI)	0.54 [0.16, 1.80]
58 Response (general), self-rated at >6m	1		Risk Ratio (M-H, Random, 95% CI)	Totals not selected
59 Recovery, clinician-rated at >6m	2	147	Risk Ratio (M-H, Random, 95% CI)	0.90 [0.64, 1.25]
60 Depression, self-rated at >6m	5	339	Std. Mean Difference (Random, 95% CI)	0.08 [-0.14, 0.30]
61 Depression improvement, self-rated at >6m	1		Risk Ratio (M-H, Random, 95% CI)	Totals not selected

Outcome or subgroup title	No. of studies	No. of participants	Statistical method	Effect size
62 Disability, self-rated at >6m	3	242	Std. Mean Difference (Random, 95% CI)	-0.21 [-0.47, 0.04]
63 Quality of Life, self-rated at >6m	1		Std. Mean Difference (Random, 95% CI)	Totals not selected

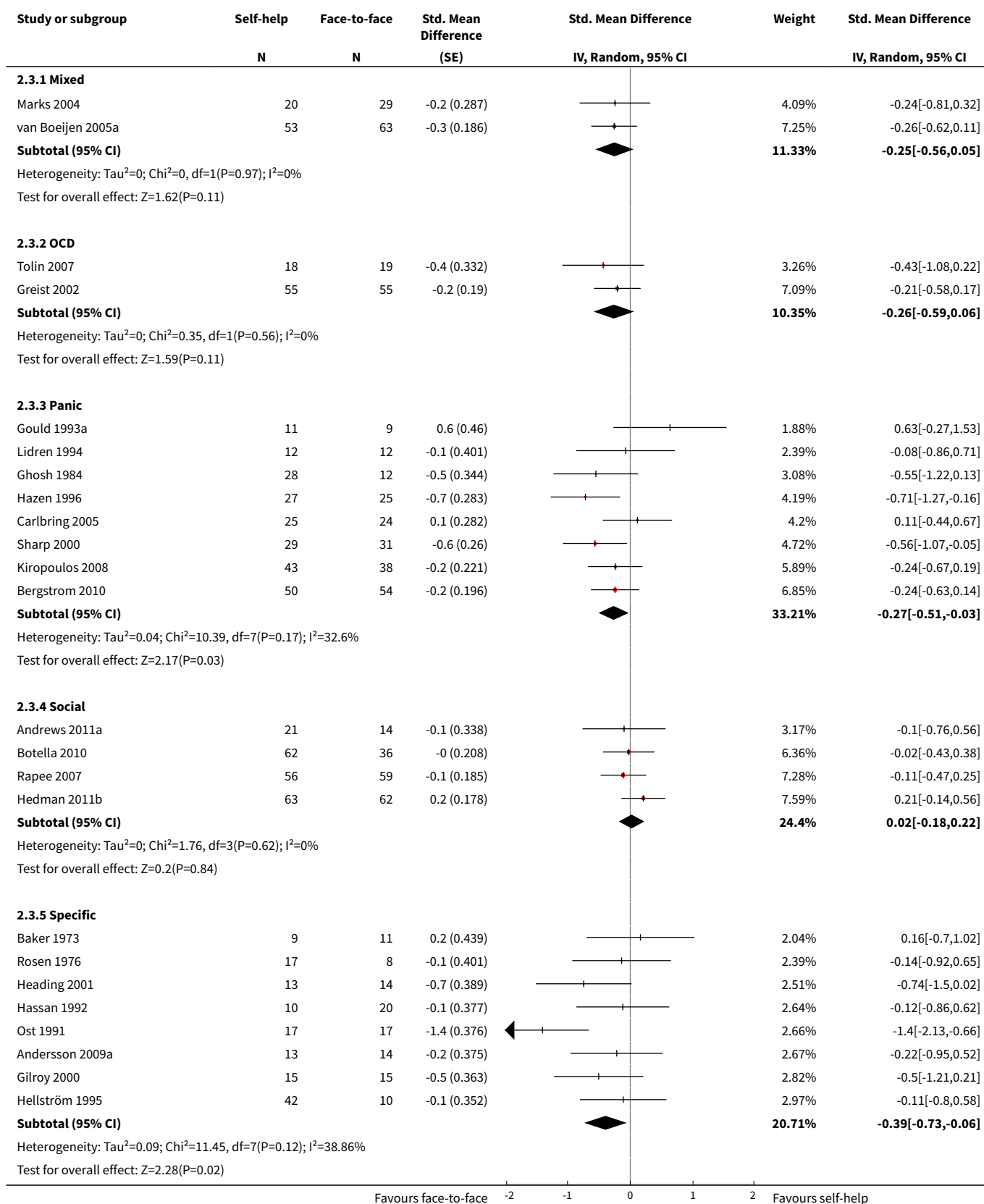
Analysis 2.1. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 1 Anxiety (any measure), self-rated at post-treatment.

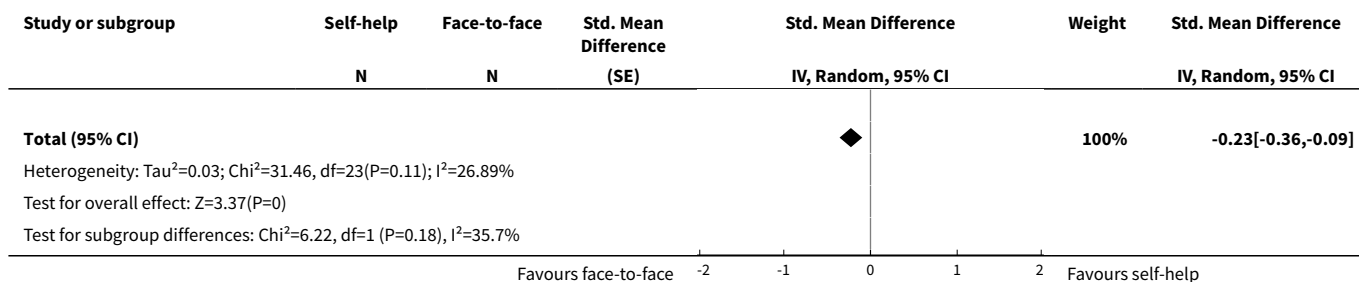


Analysis 2.2. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 2 Anxiety (any measure), self-rated at post-treatment (group versus individual).

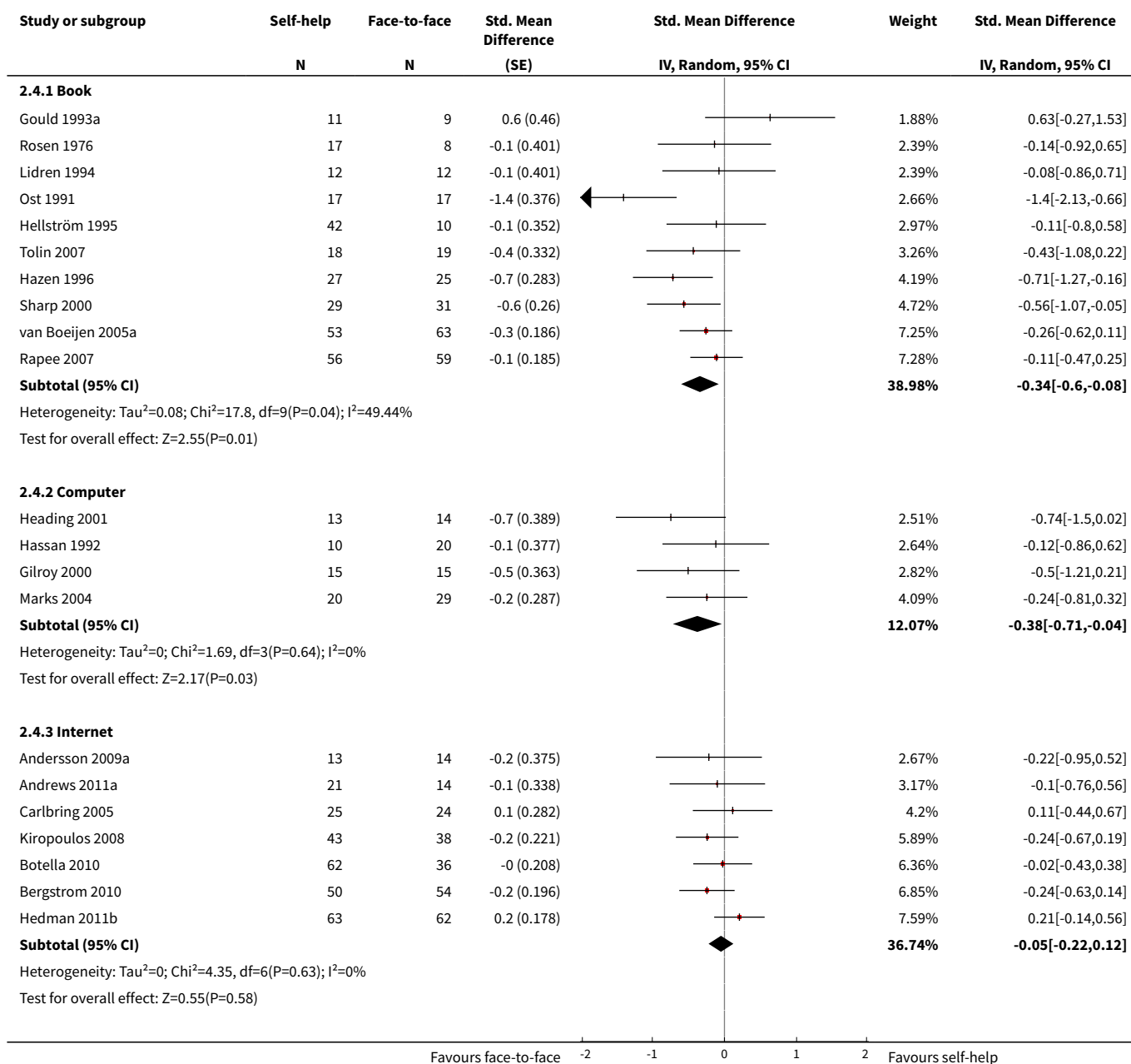


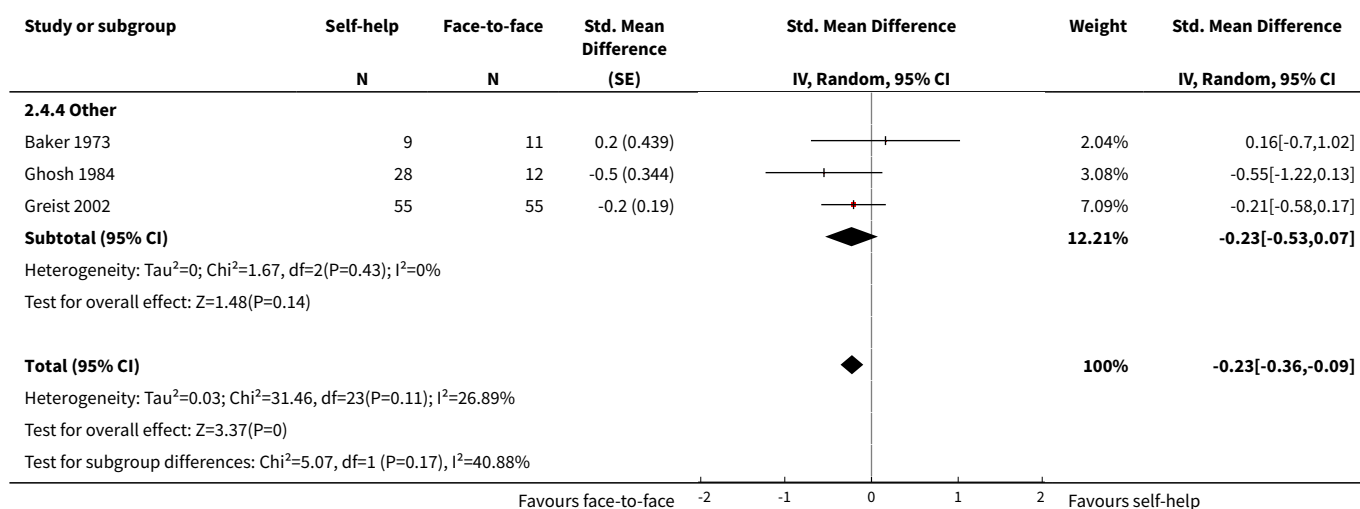
Analysis 2.3. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 3 Anxiety (any measure), self-rated at post-treatment (by disorder).



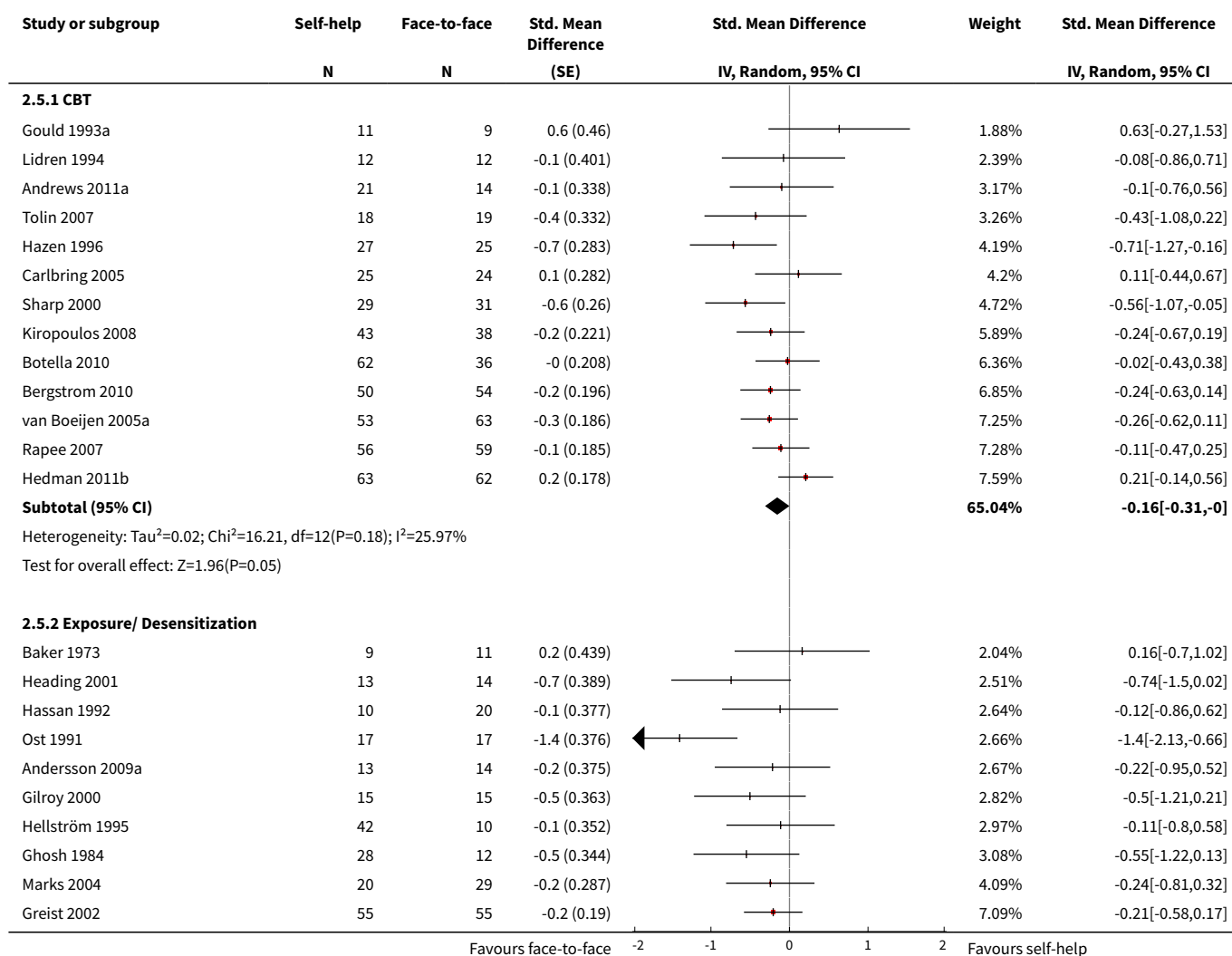


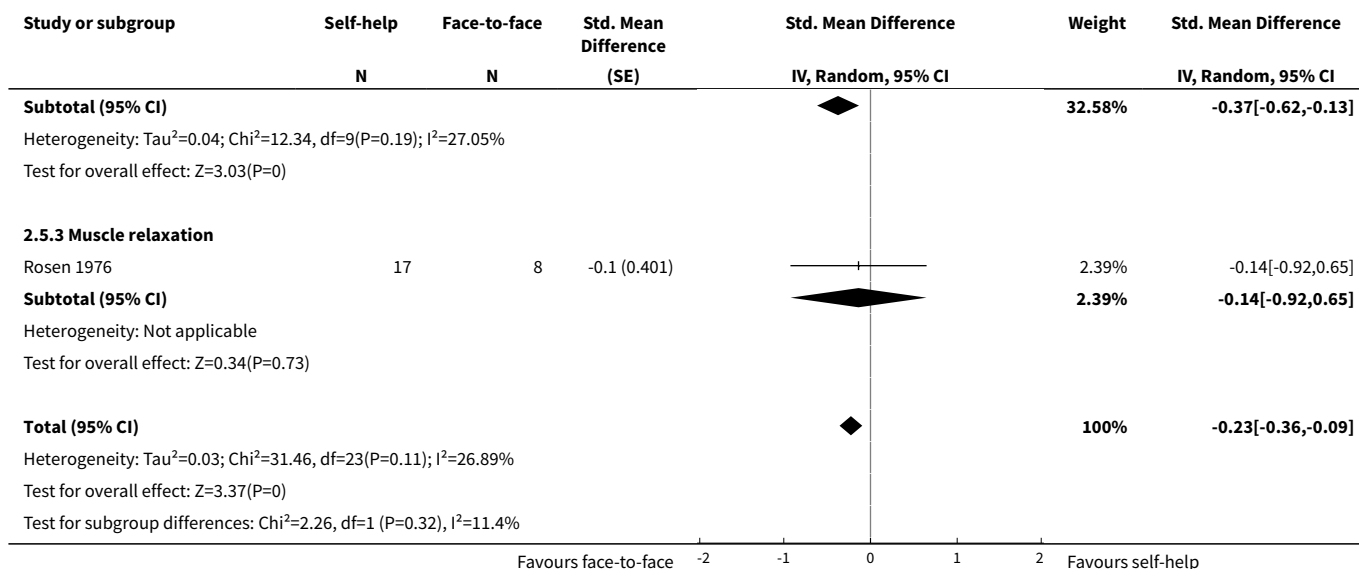
Analysis 2.4. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 4 Anxiety (any measure), self-rated at post-treatment (by media).



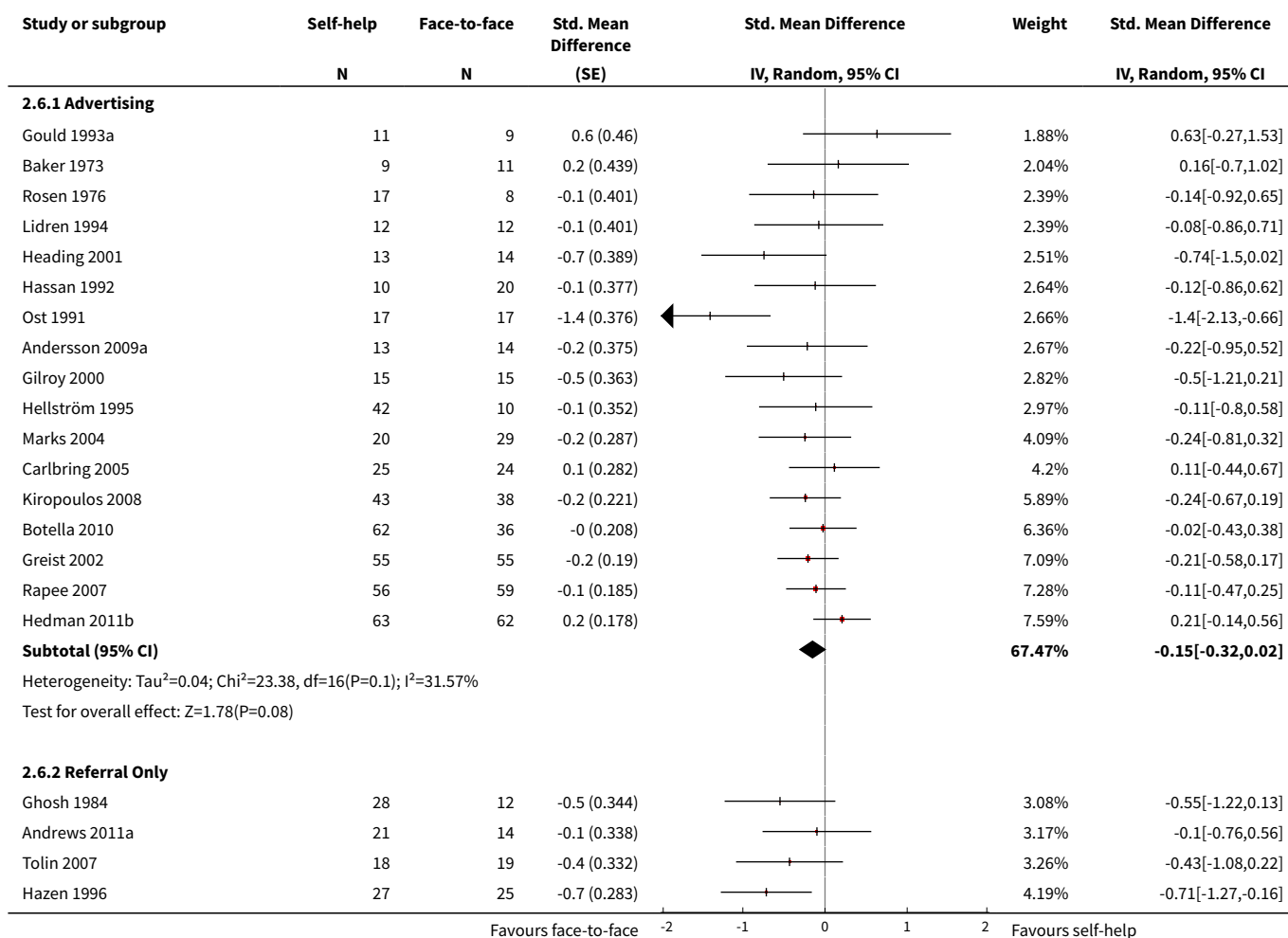


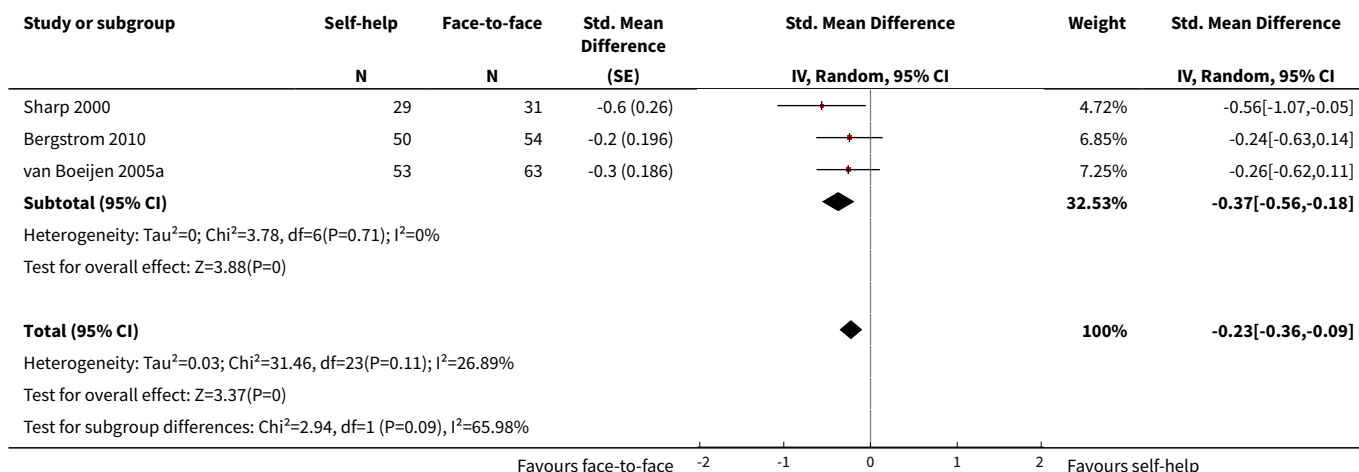
Analysis 2.5. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 5 Anxiety (any measure), self-rated at post-treatment (by therapy).



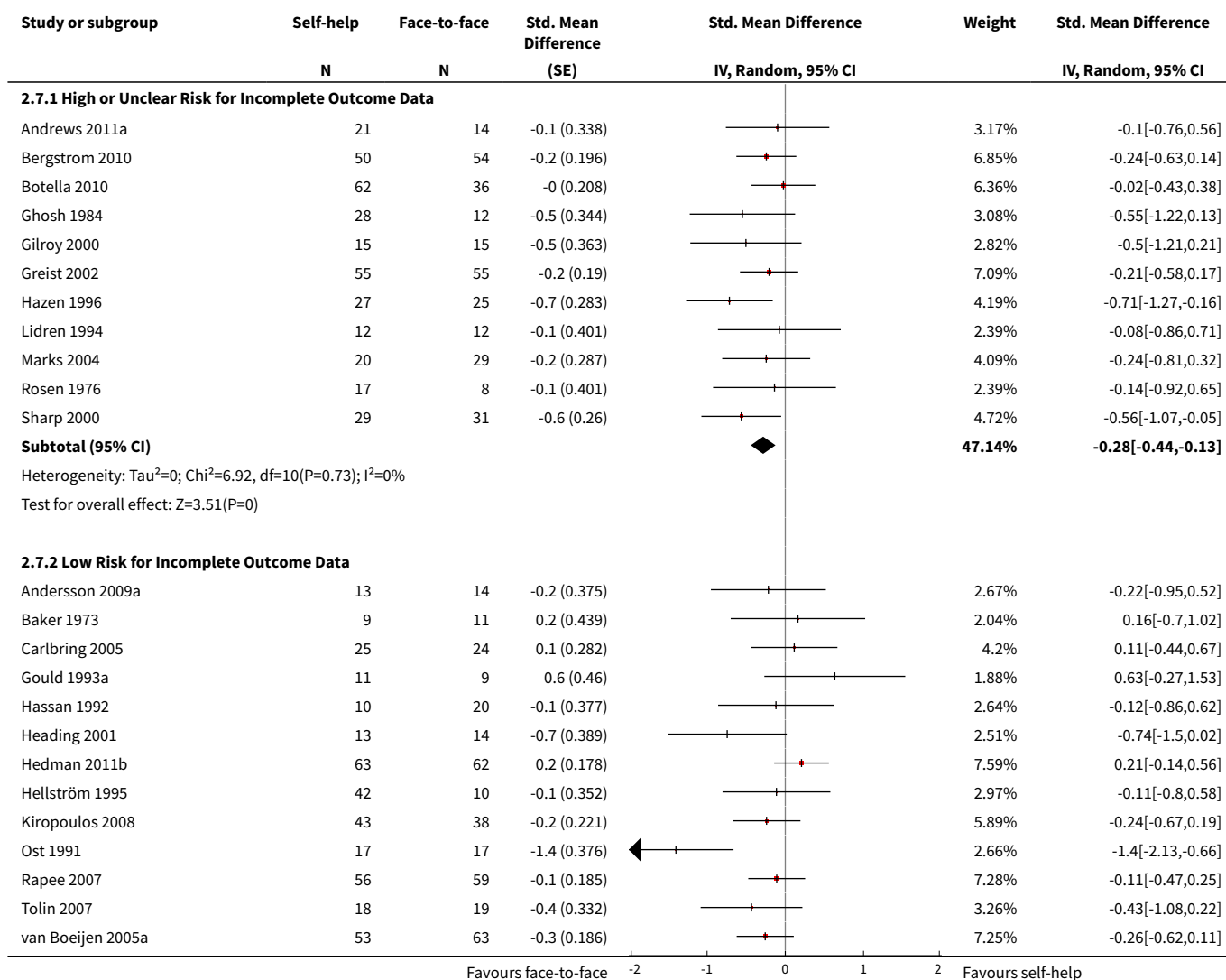


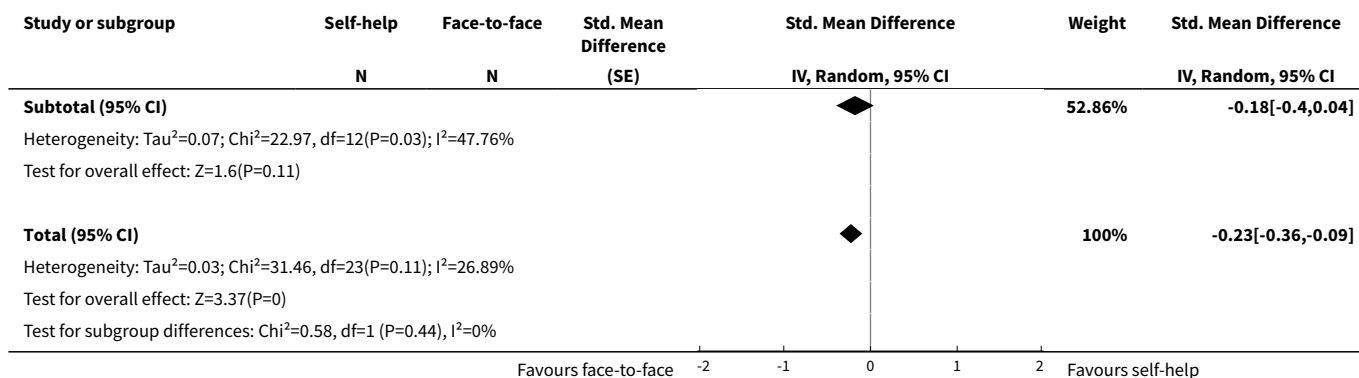
Analysis 2.6. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 6 Anxiety (any measure), self-rated at post-treatment (by setting).



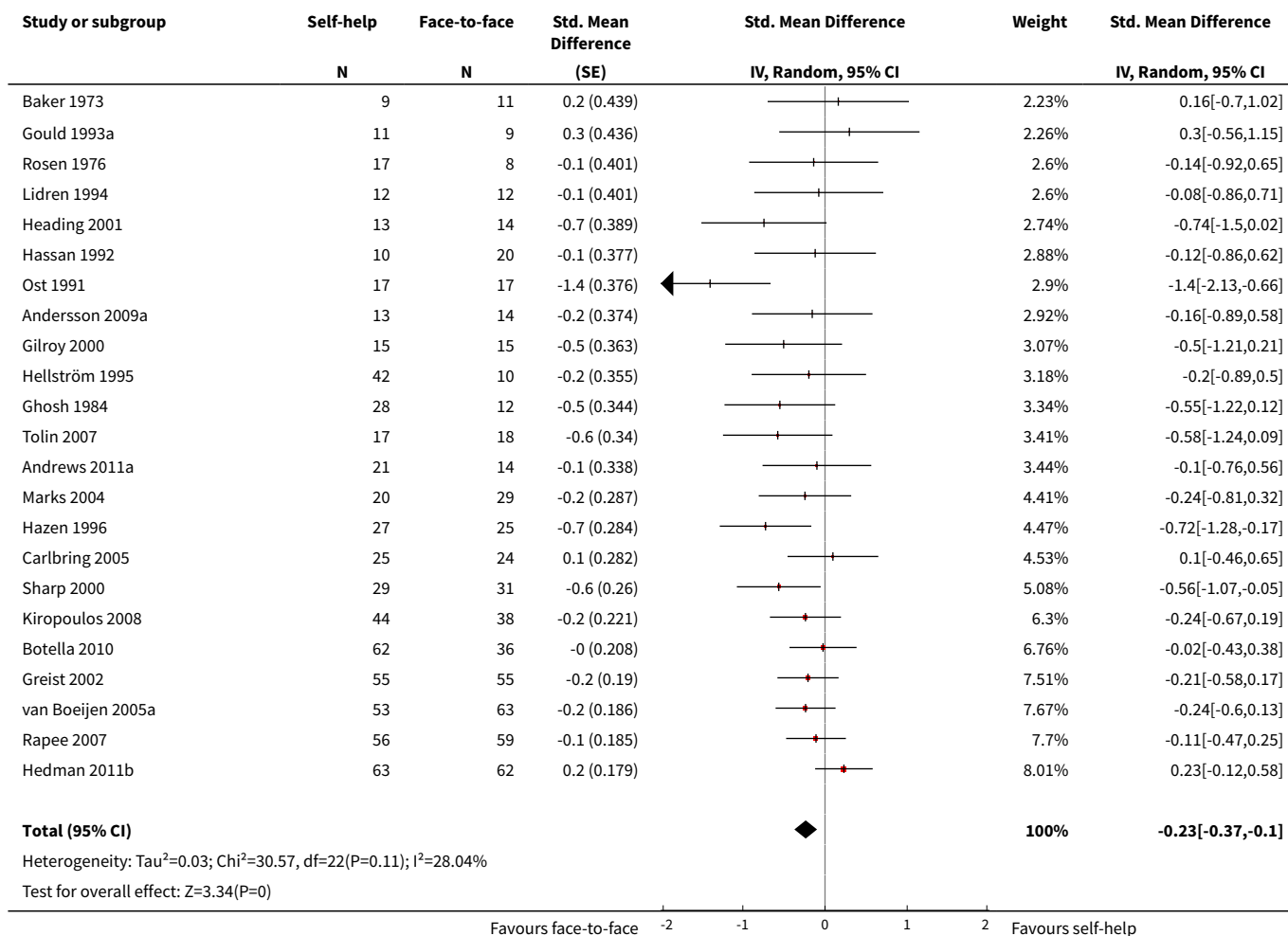


Analysis 2.7. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 7 Anxiety (any measure), self-rated at post-treatment (sensitivity analysis for bias).

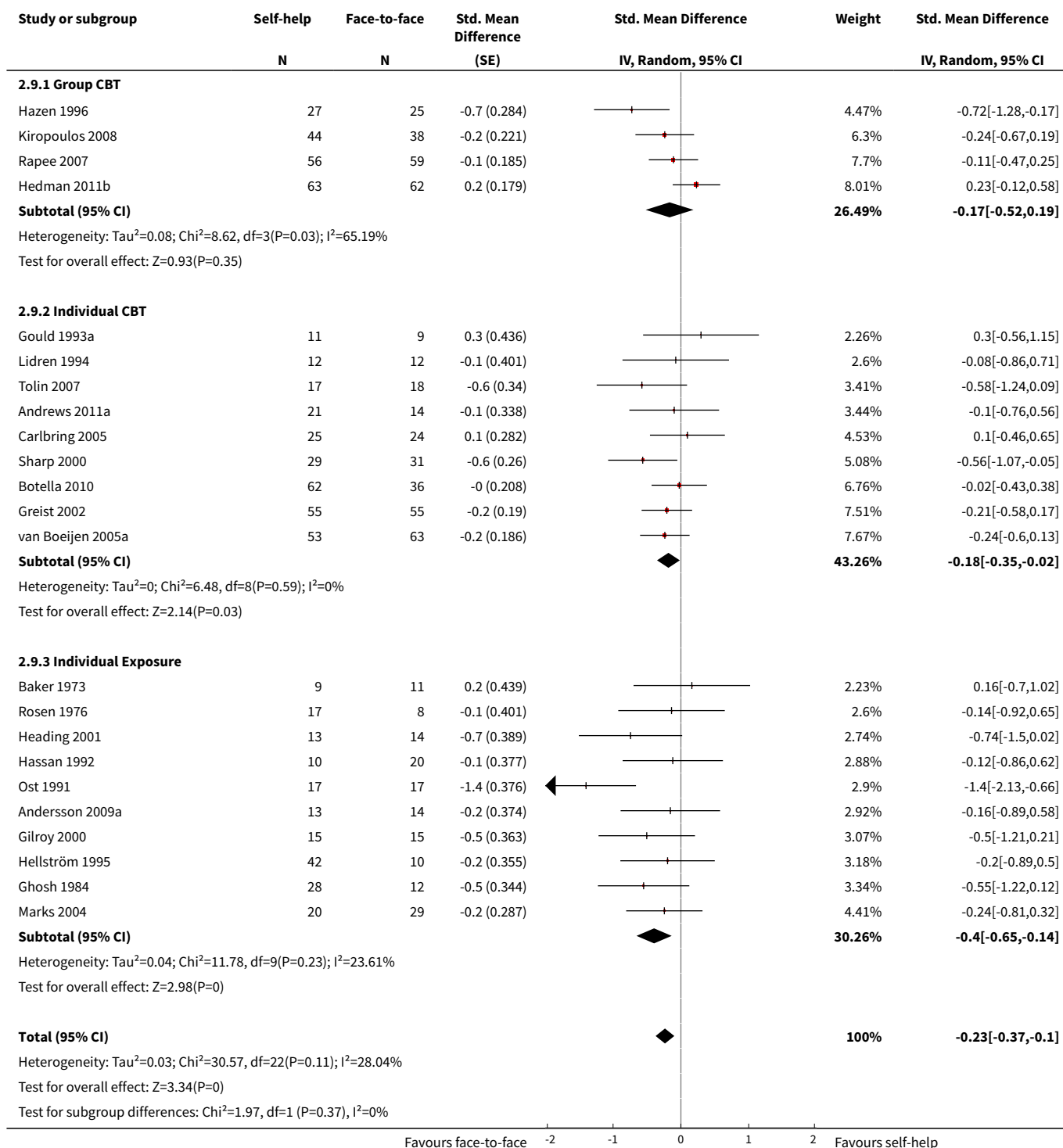




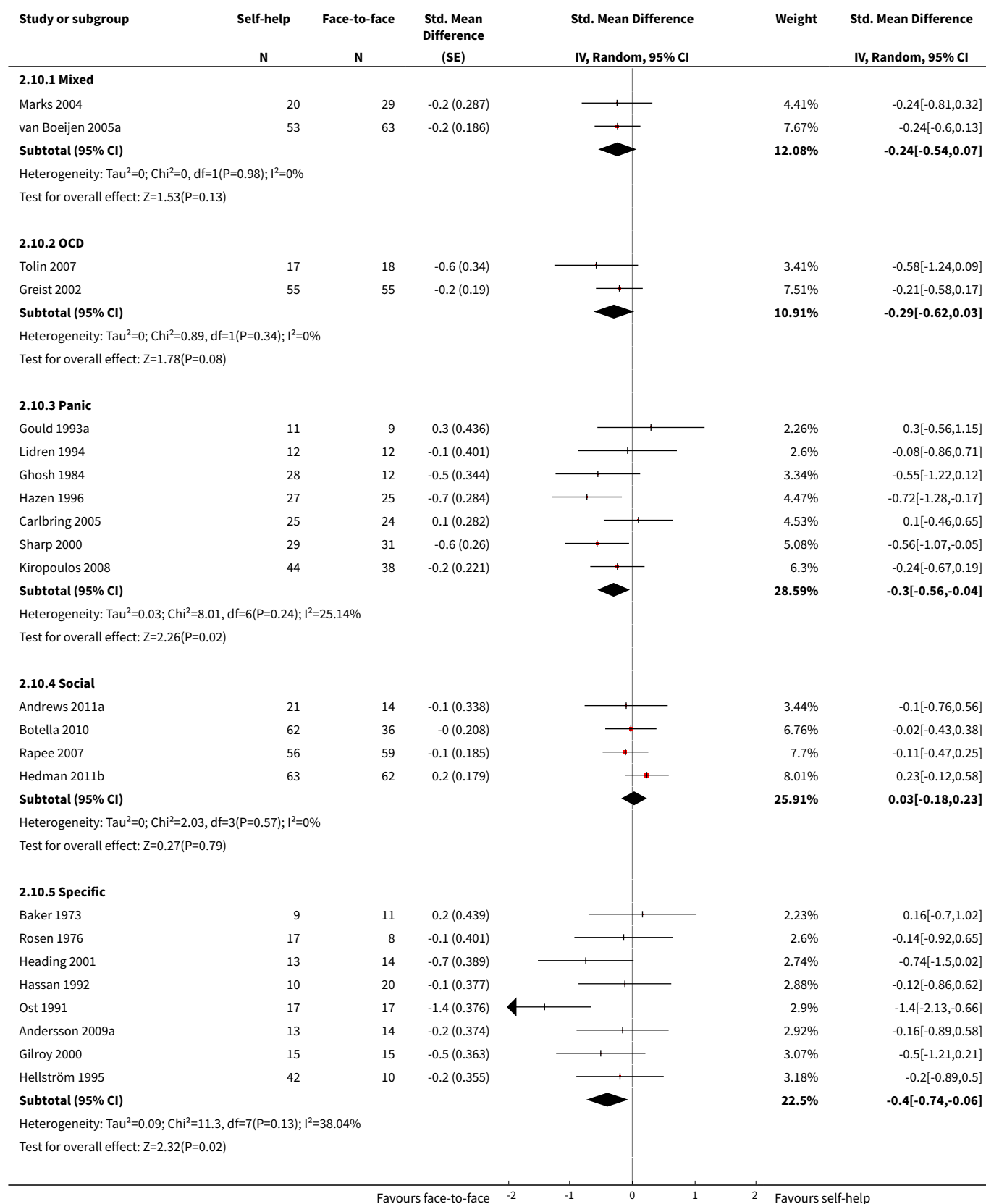
Analysis 2.8. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 8 Specific symptoms, self-rated at post-treatment.

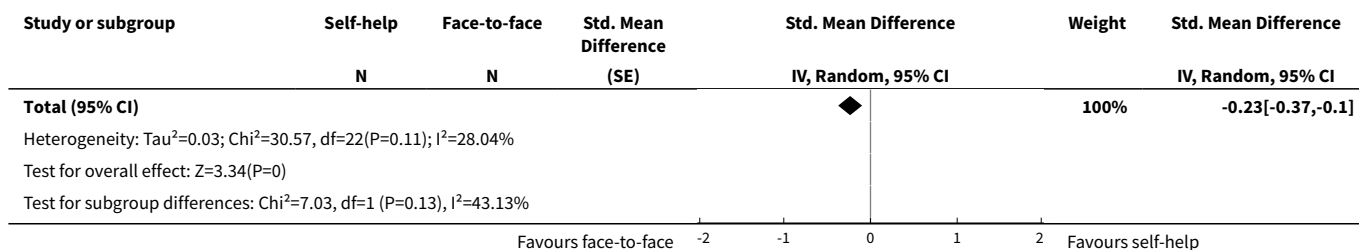


Analysis 2.9. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 9 Specific symptoms, self-rated at post-treatment (group versus individual).

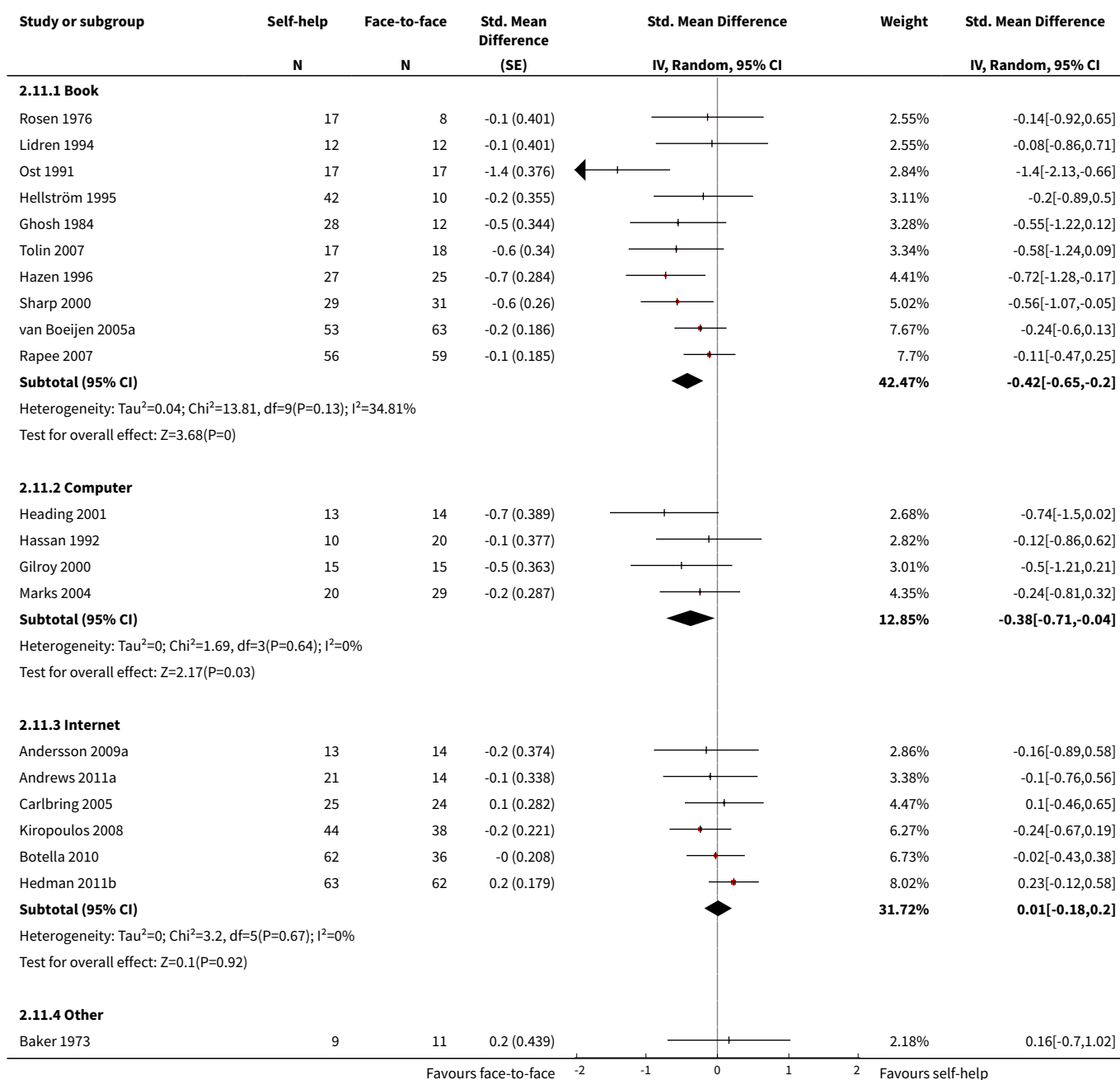


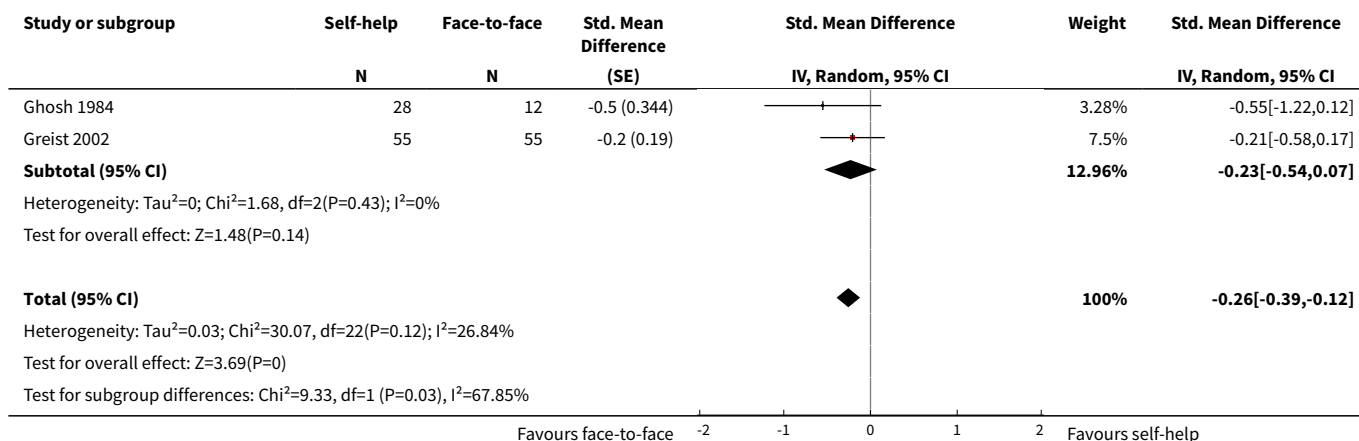
Analysis 2.10. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 10 Specific symptoms, self-rated at post-treatment (by disorder).



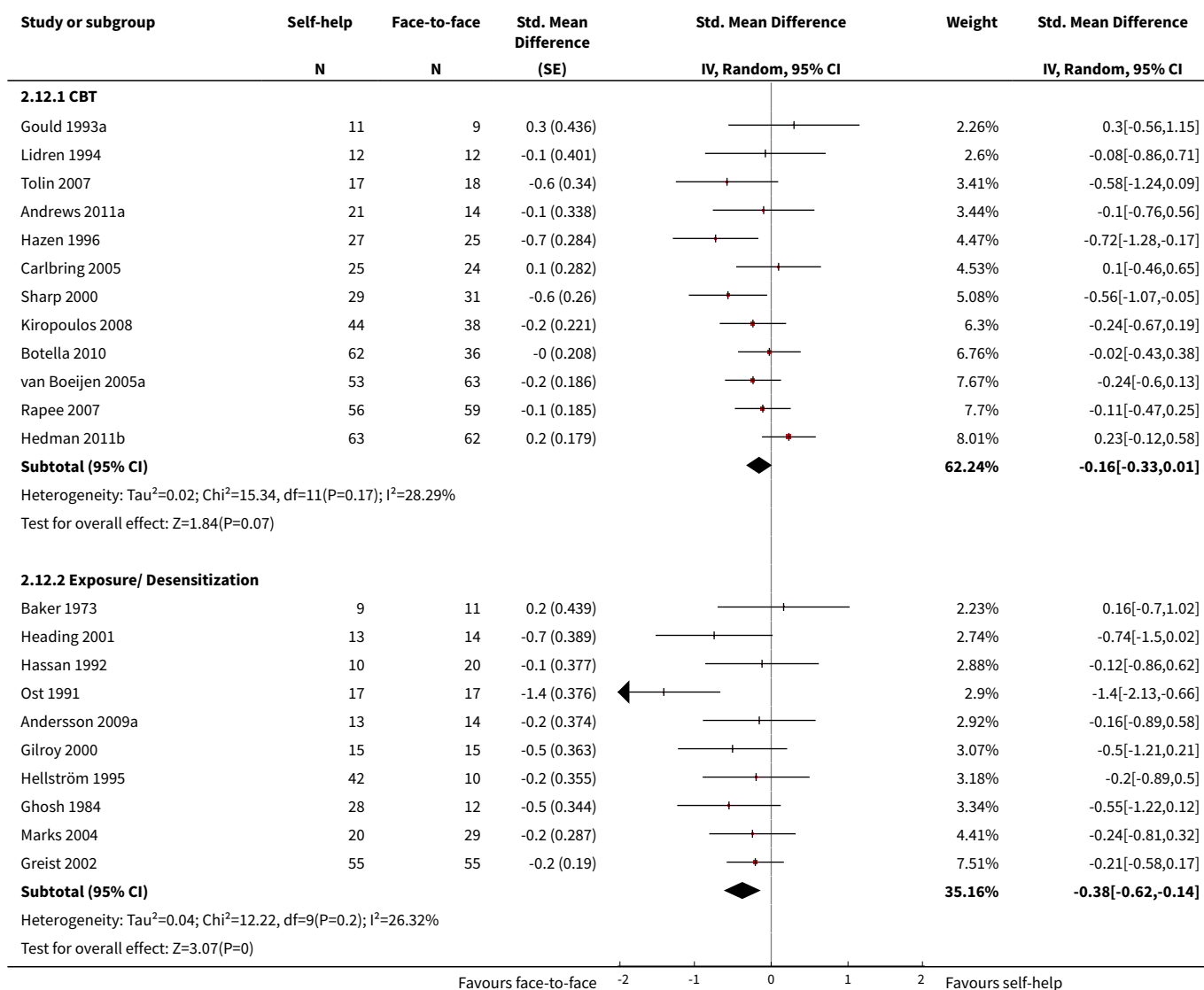


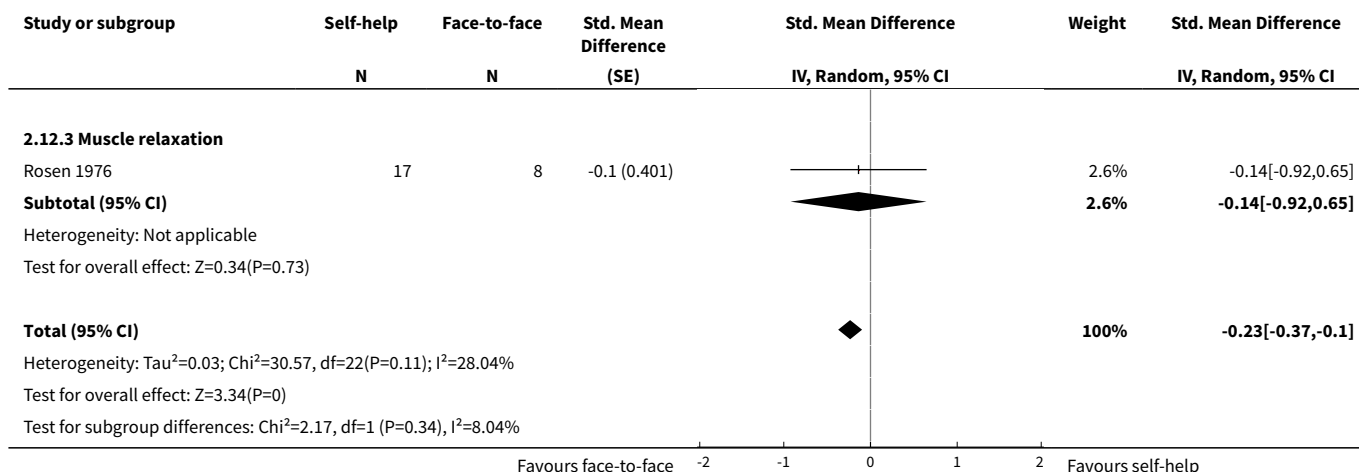
Analysis 2.11. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 11 Specific symptoms, self-rated at post-treatment (by media).



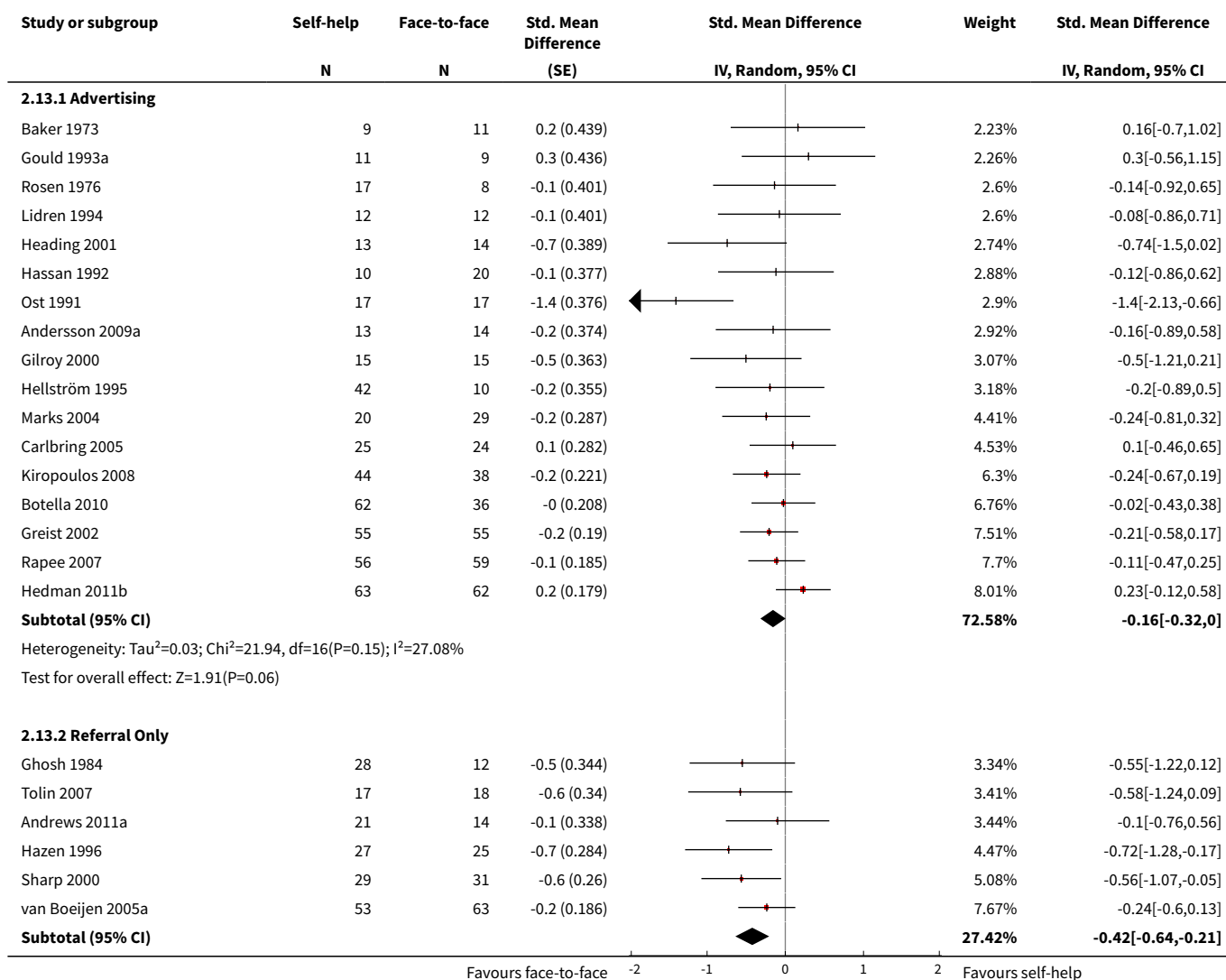


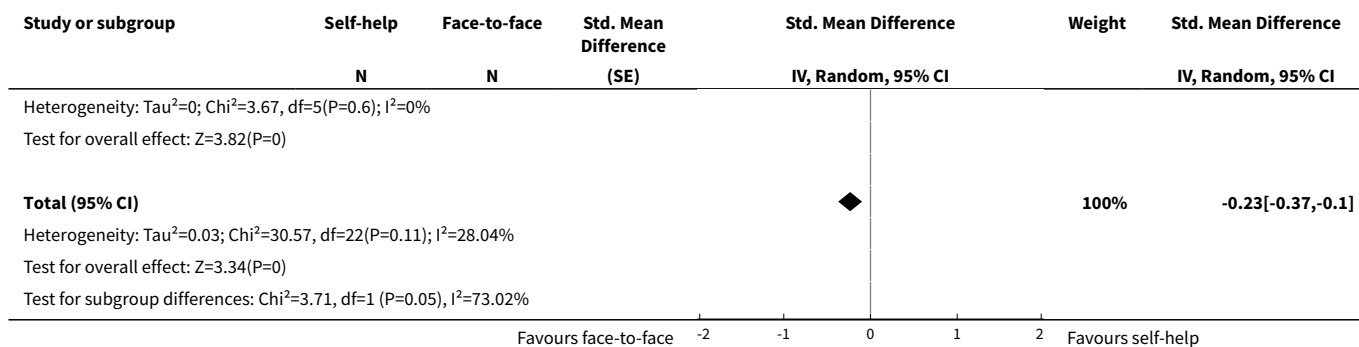
Analysis 2.12. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 12 Specific symptoms, self-rated at post-treatment (by therapy).



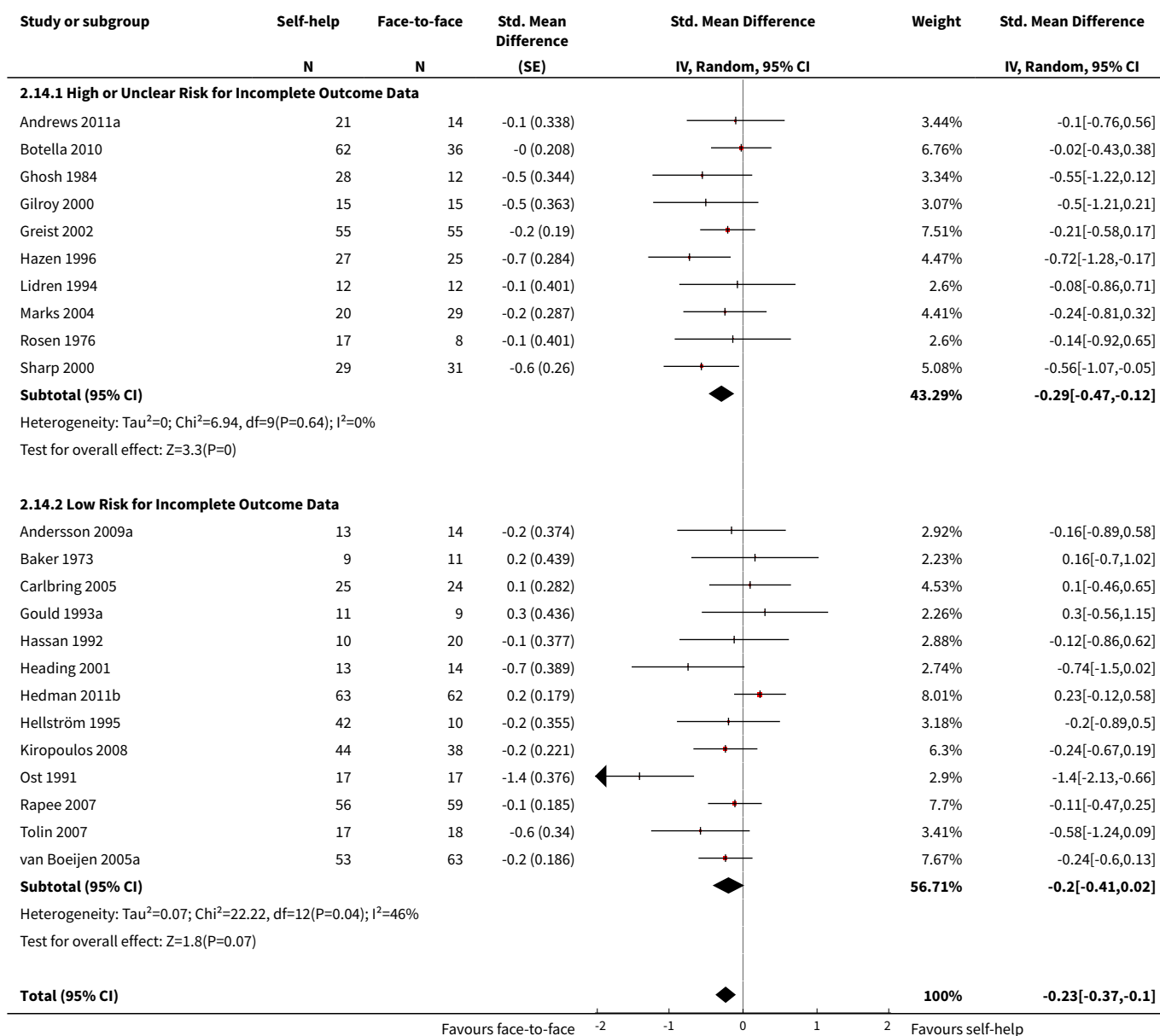


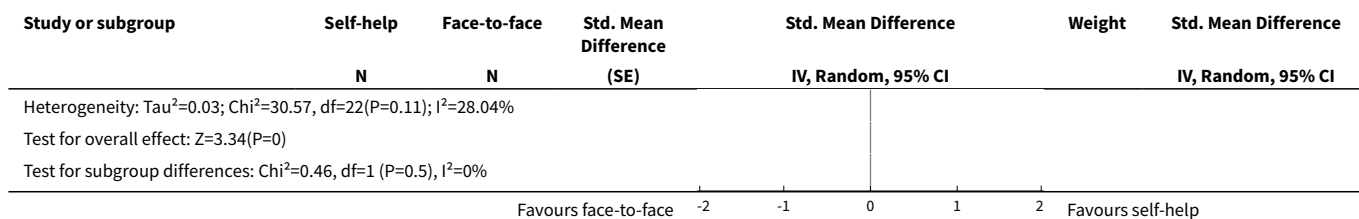
Analysis 2.13. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 13 Specific symptoms, self-rated at post-treatment (by setting).



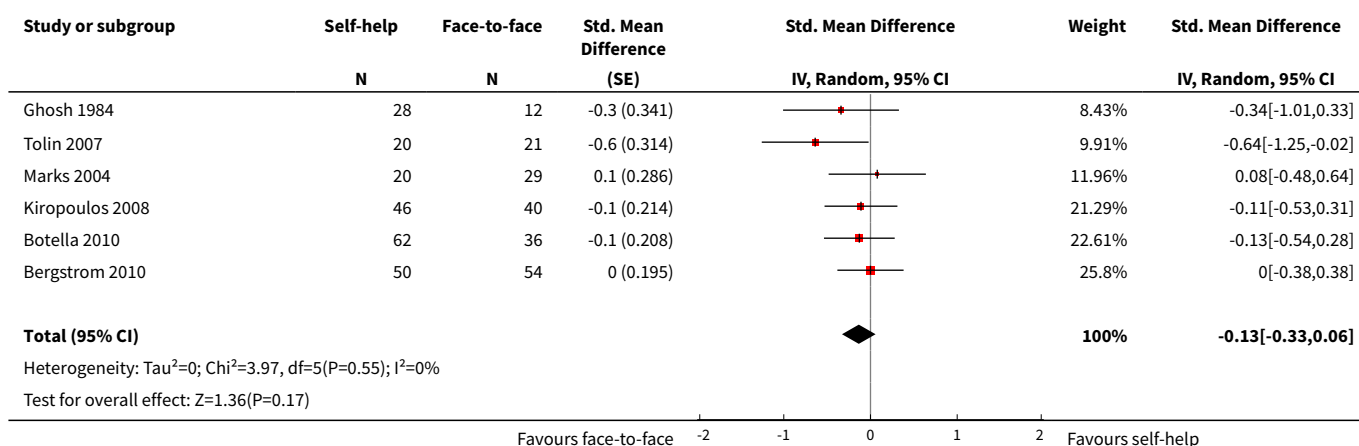


Analysis 2.14. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 14 Specific symptoms, self-rated at post-treatment (sensitivity analysis for bias).

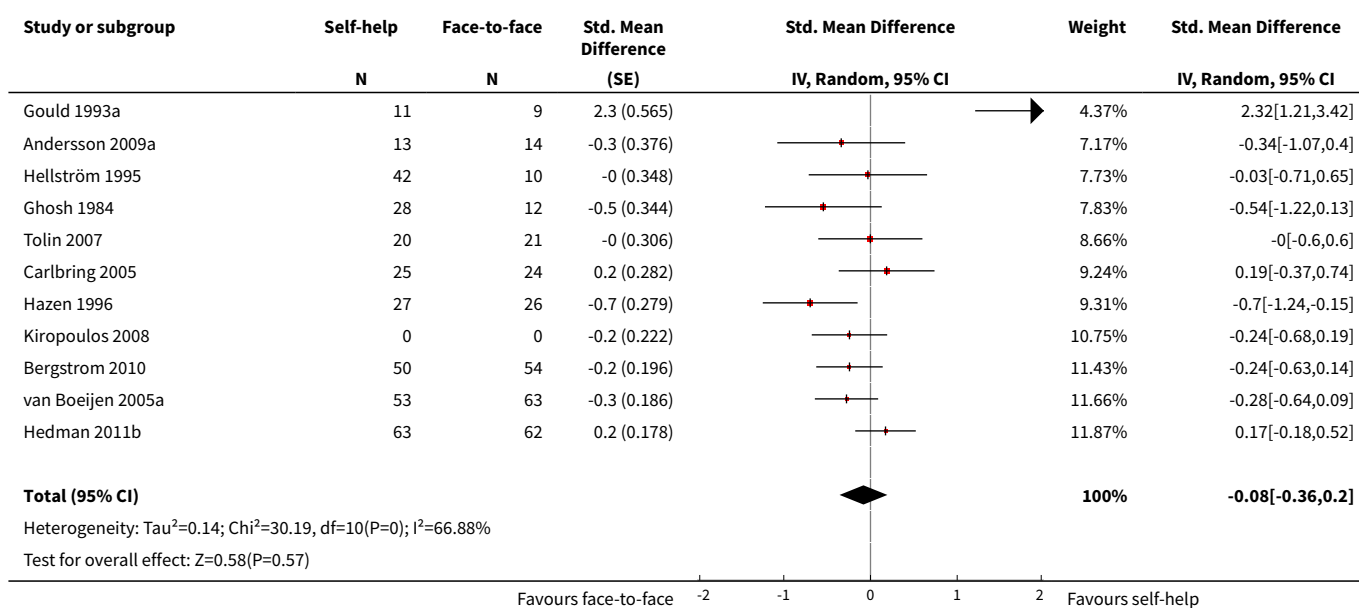




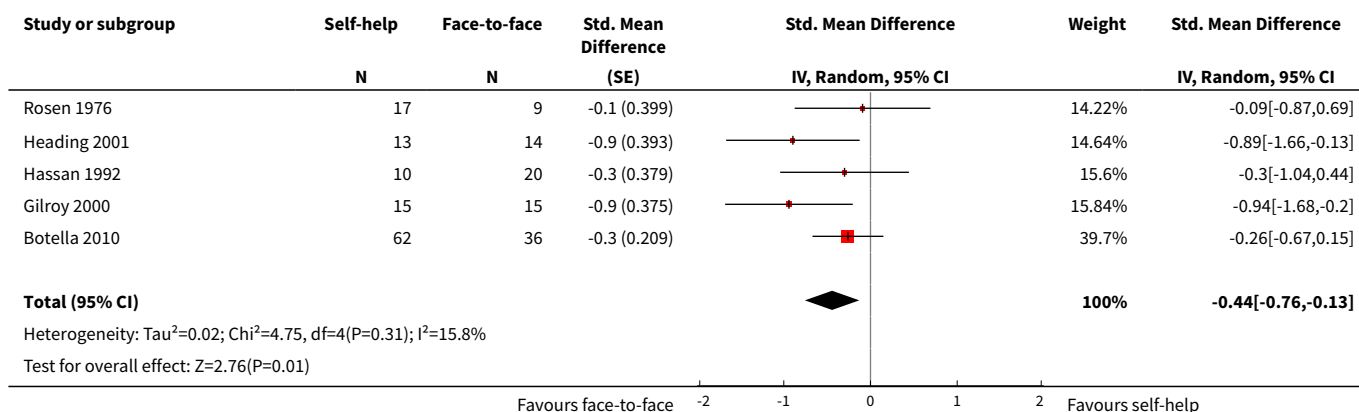
Analysis 2.15. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 15 Specific symptoms, clinician-rated at post-treatment.



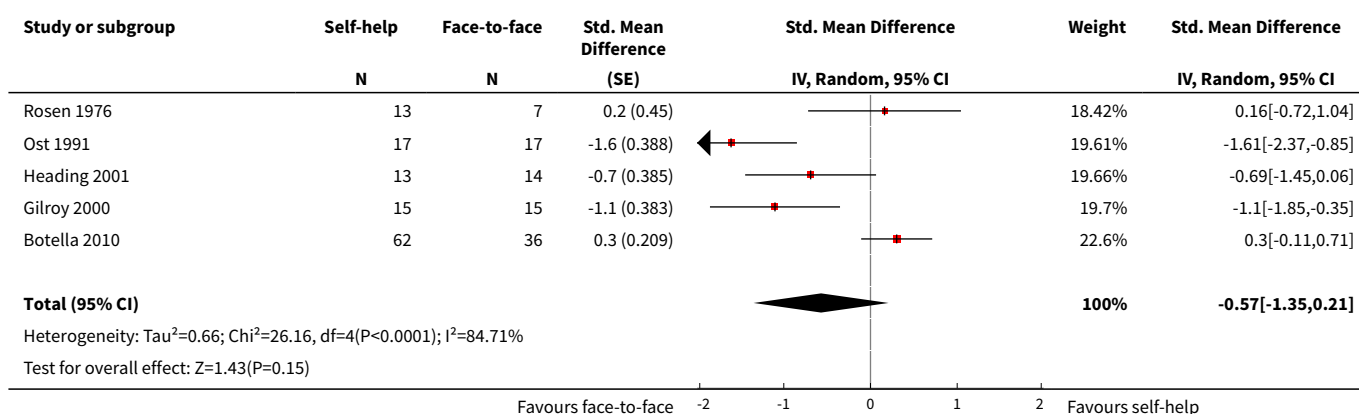
Analysis 2.16. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 16 General symptoms, self-rated at post-treatment.



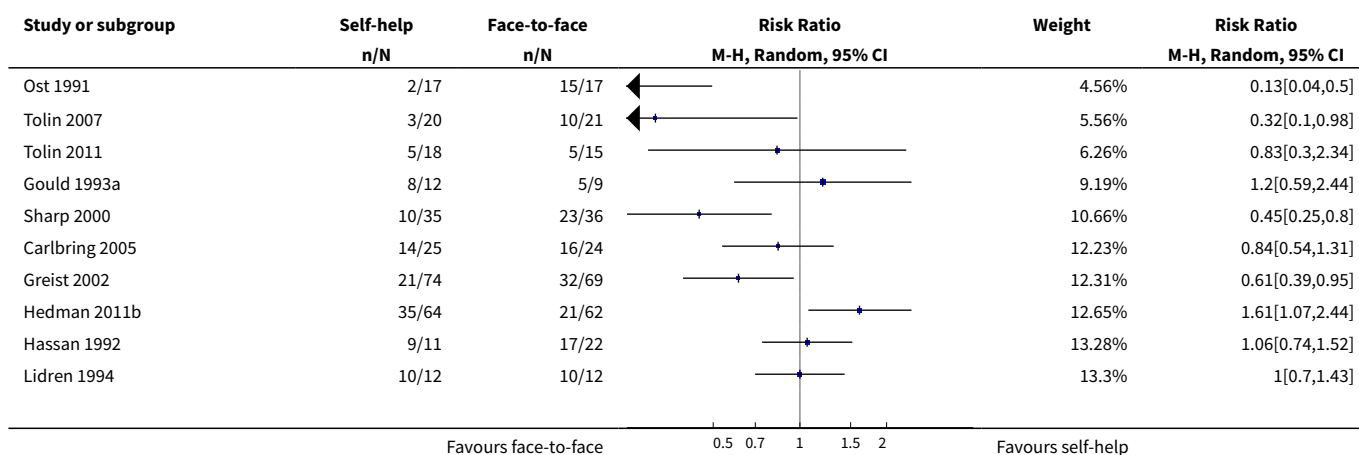
Analysis 2.17. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 17 Behavioural Test (level), clinician-rated at post-treatment.

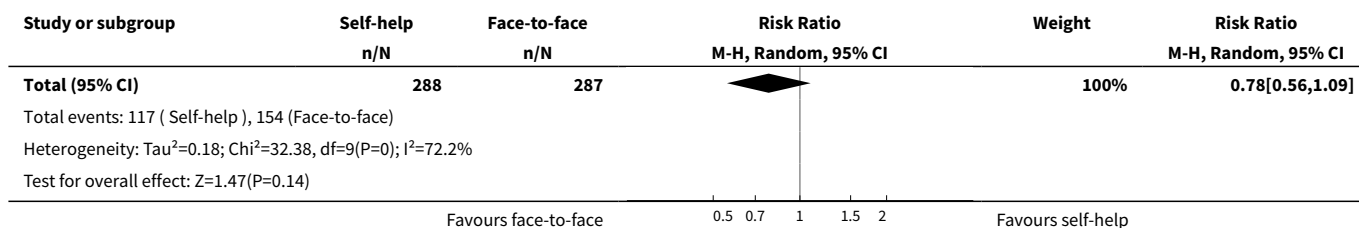


Analysis 2.18. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 18 Behavioural Test (symptoms), self-rated at post-treatment.

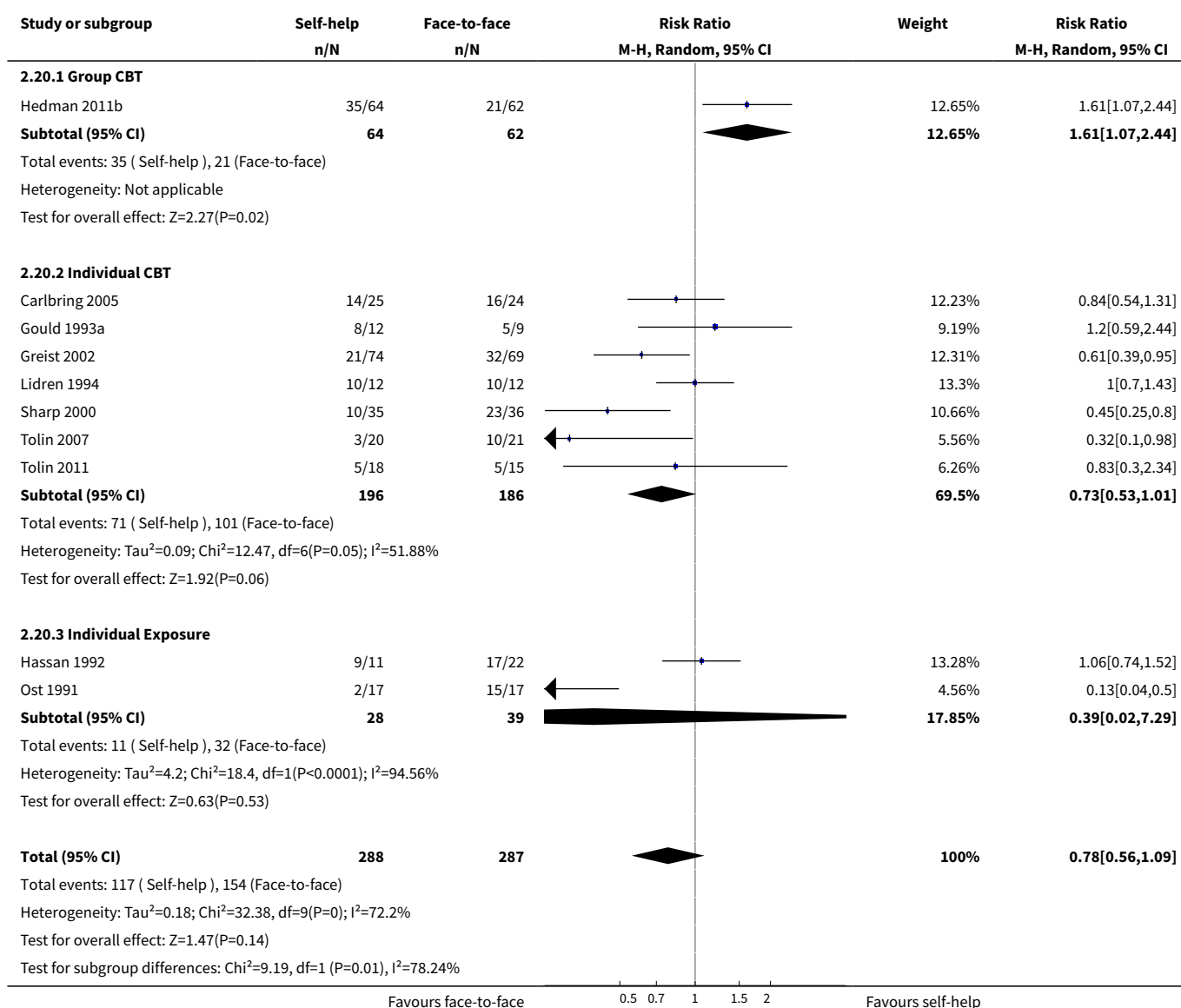


Analysis 2.19. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 19 Response (specific), self-rated at post-treatment.

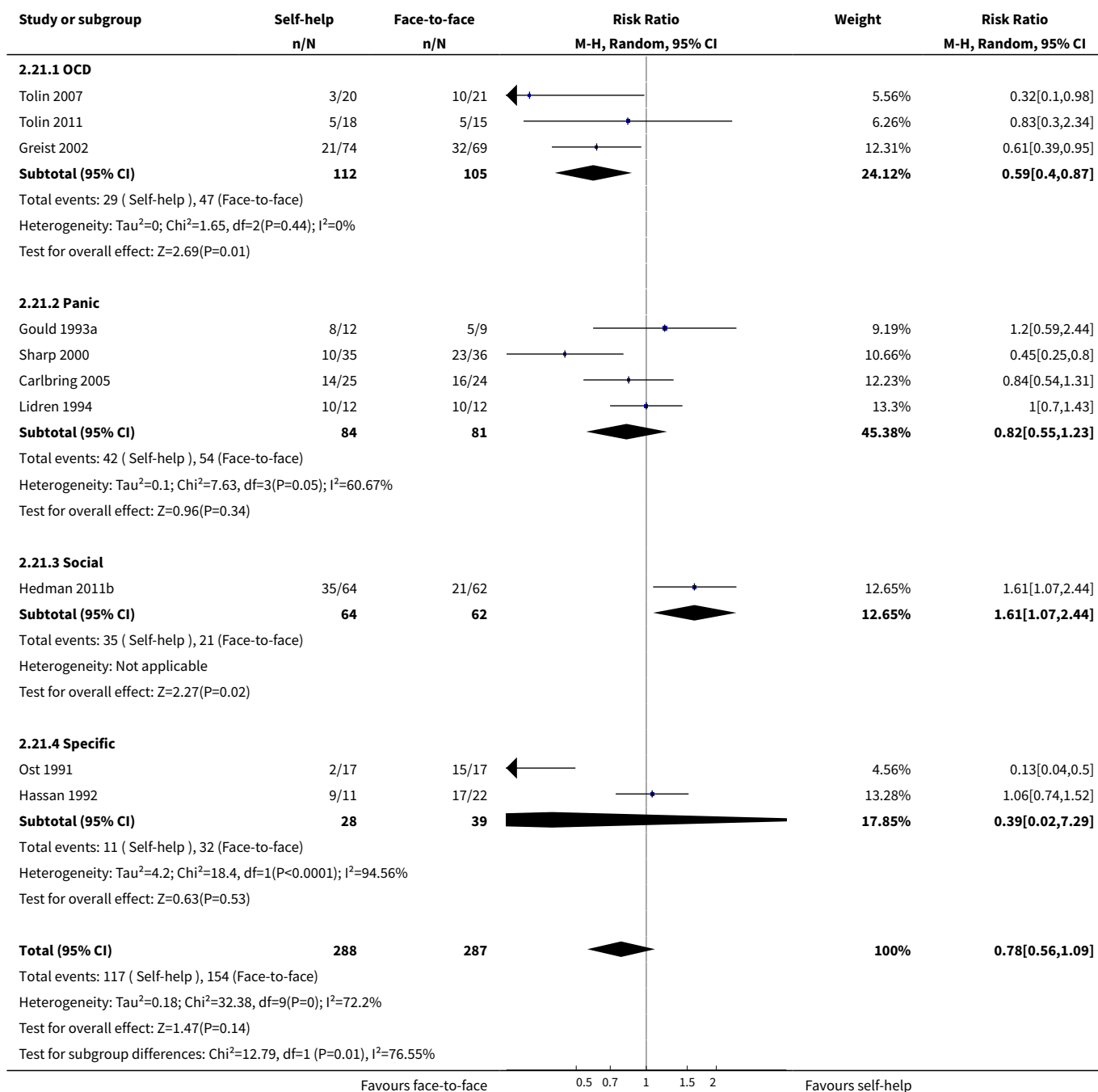




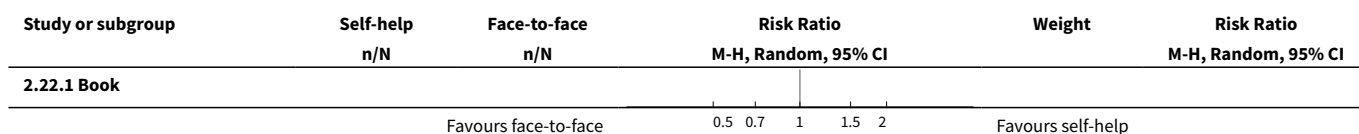
Analysis 2.20. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 20 Response (specific), self-rated at post-treatment (group versus individual).

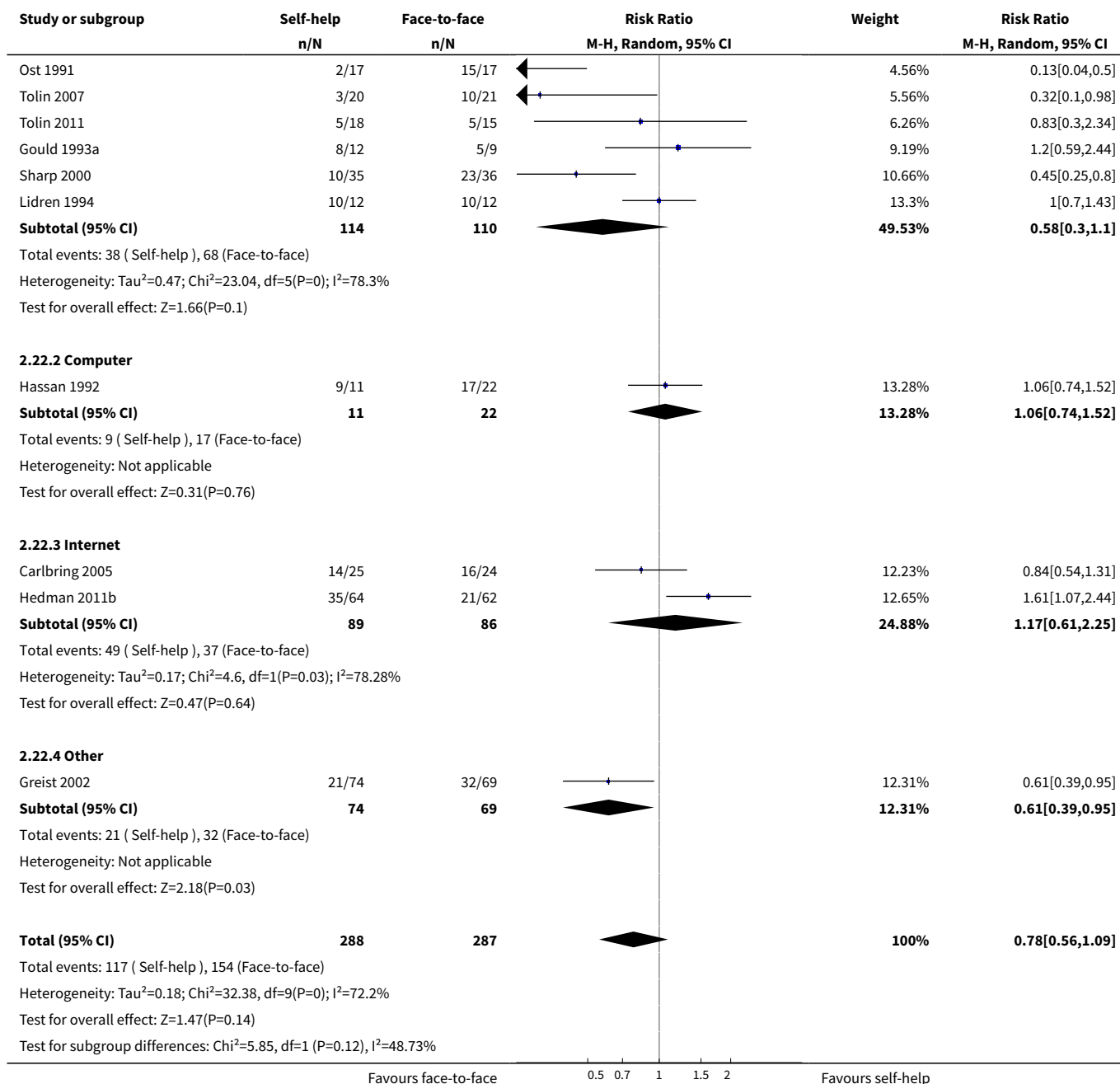


Analysis 2.21. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 21 Response (specific), self-rated at post-treatment (by disorder).

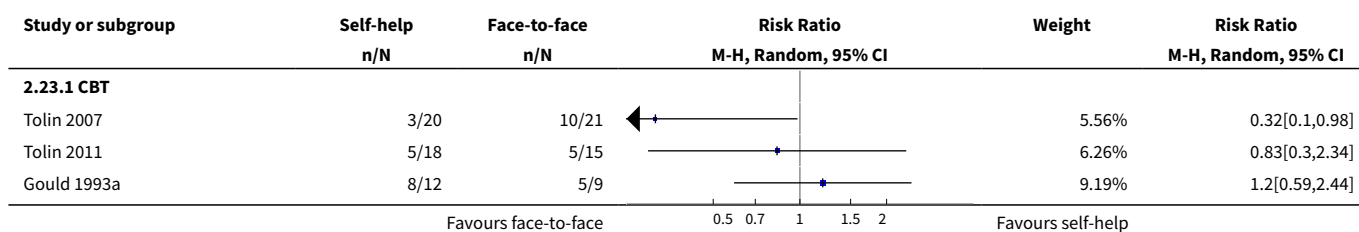


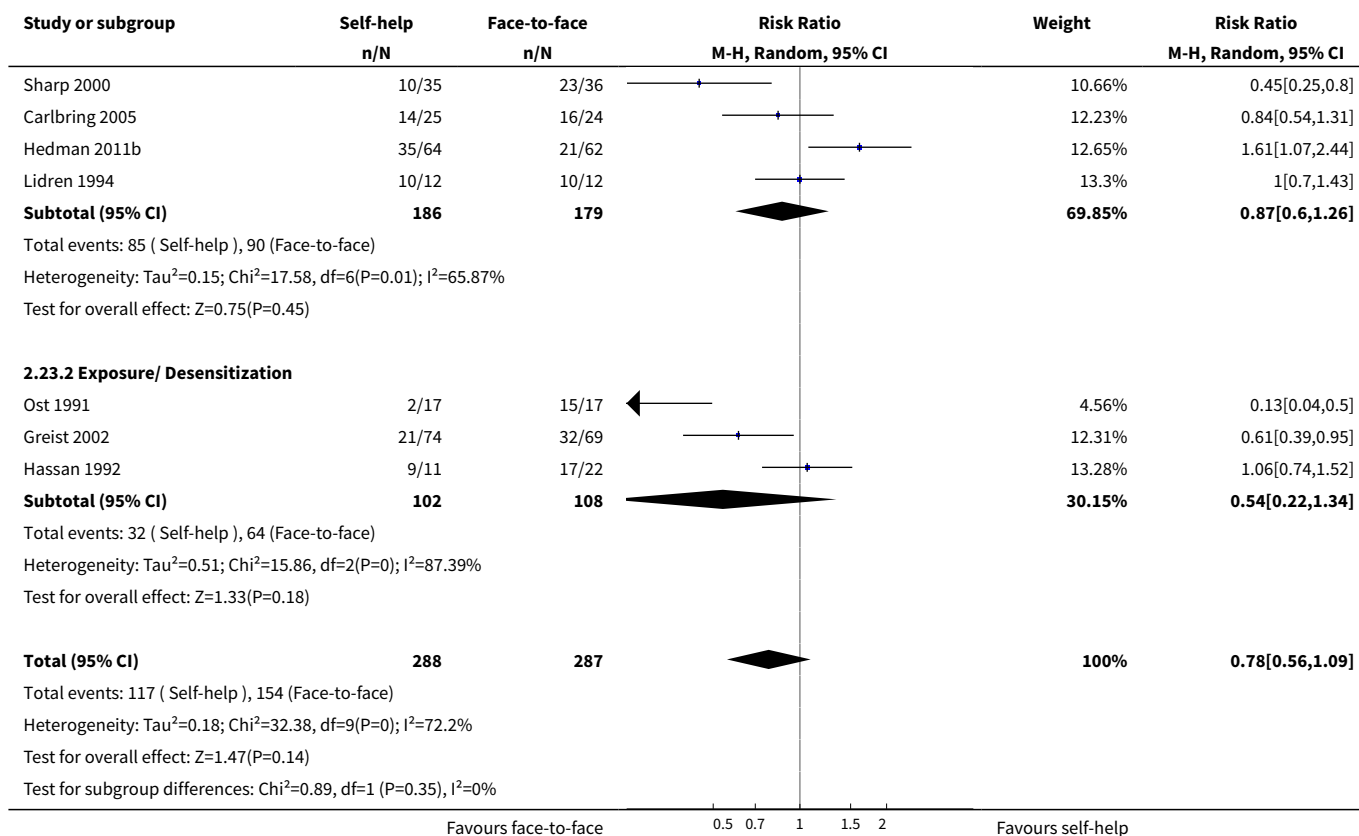
Analysis 2.22. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 22 Response (specific), self-rated at post-treatment (by media).



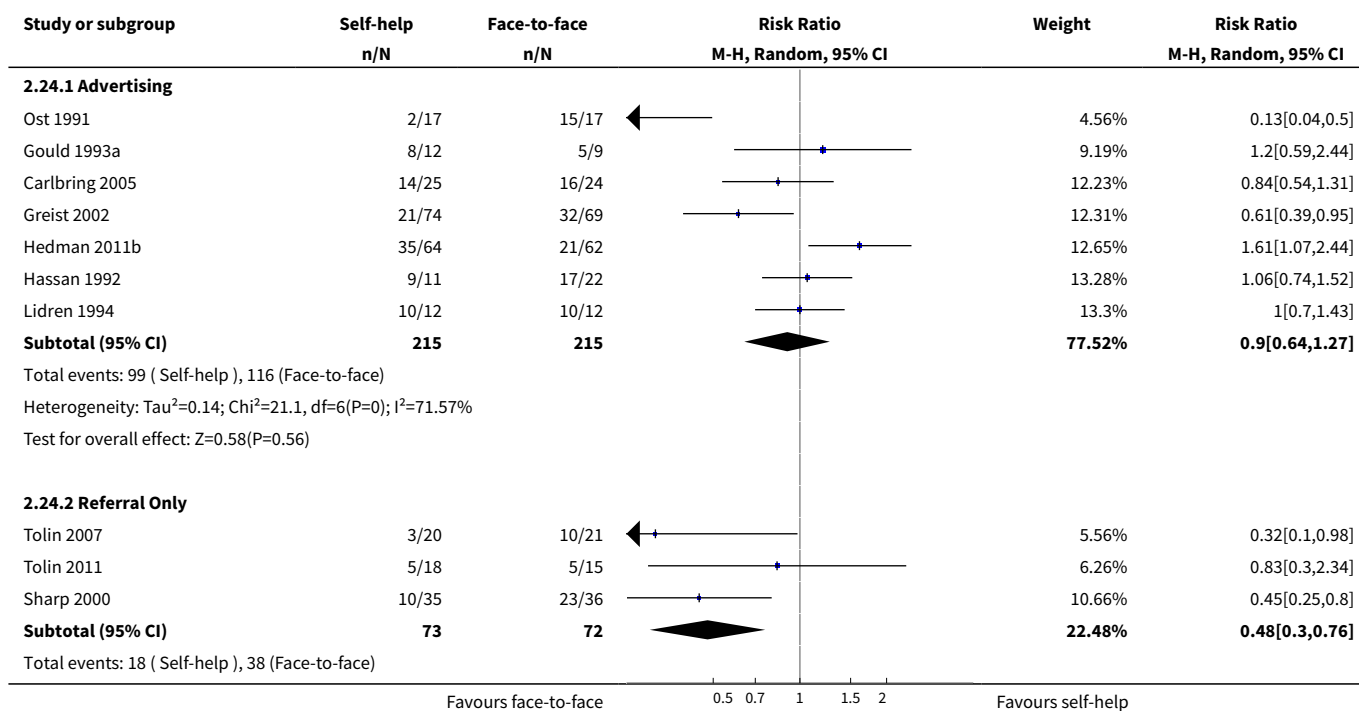


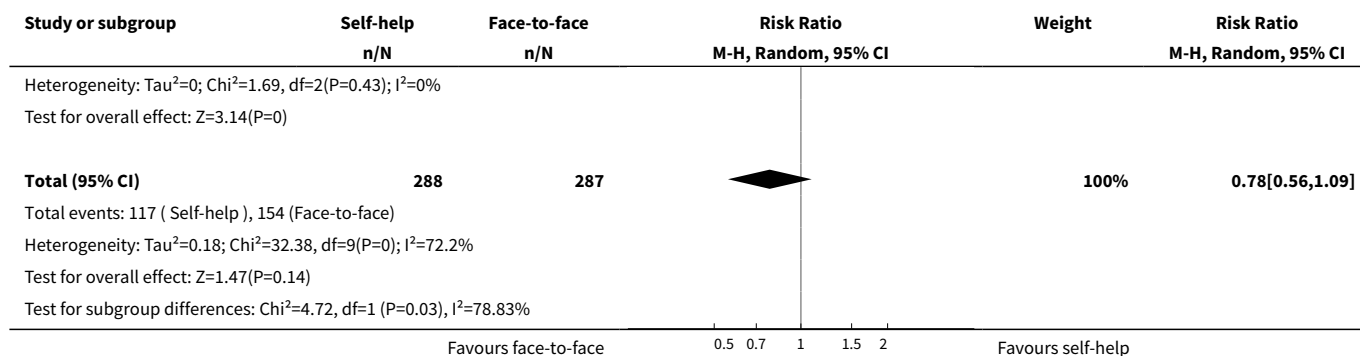
Analysis 2.23. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 23 Response (specific), self-rated at post-treatment (by therapy).



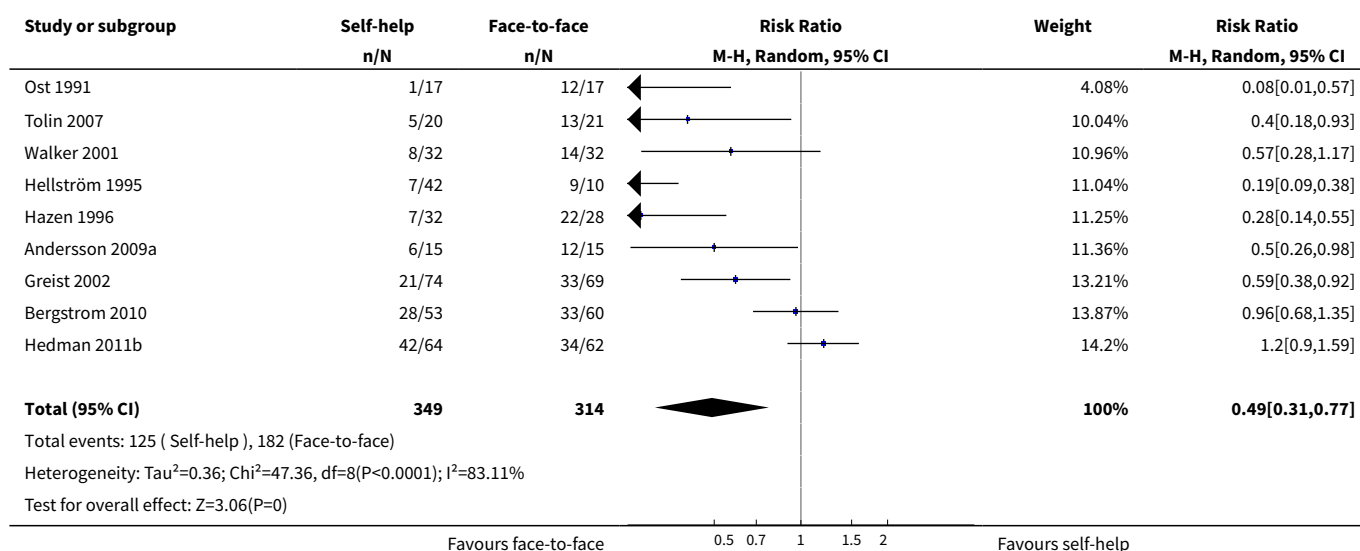


Analysis 2.24. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 24 Response (specific), self-rated at post-treatment (by setting).

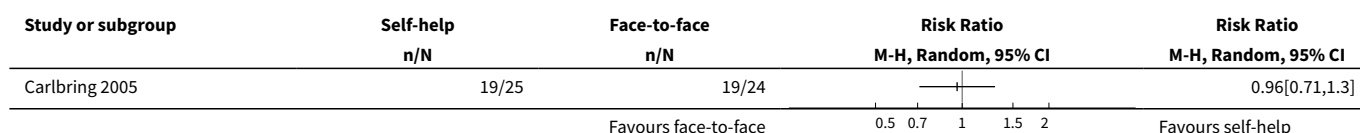




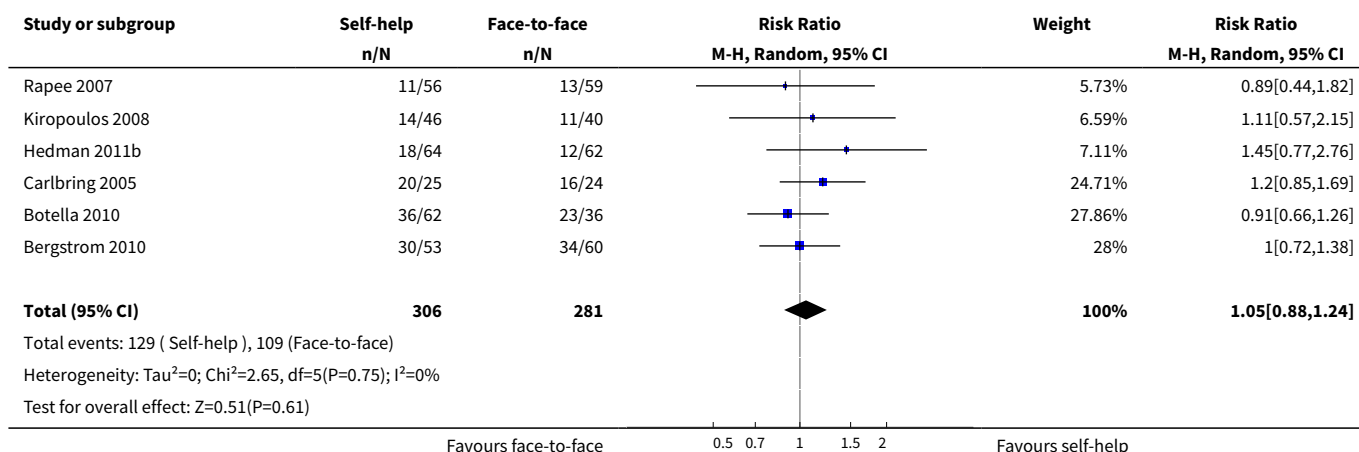
Analysis 2.25. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 25 Response (specific), clinician-rated at post-treatment.



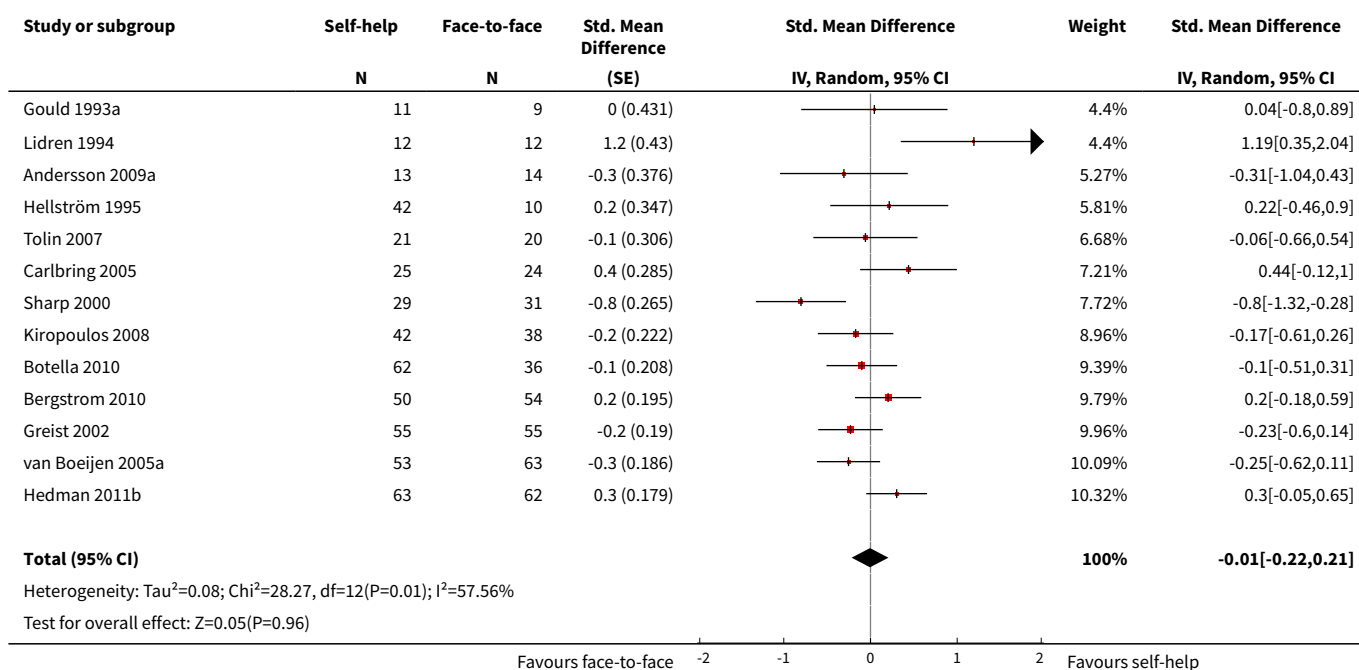
Analysis 2.26. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 26 Response (general), self-rated at post-treatment.



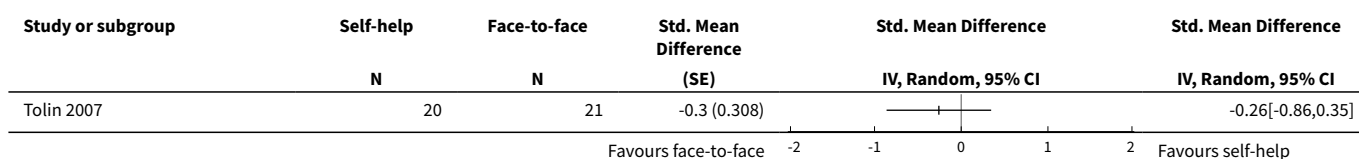
Analysis 2.27. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 27 Recovery, clinician-rated at post-treatment.



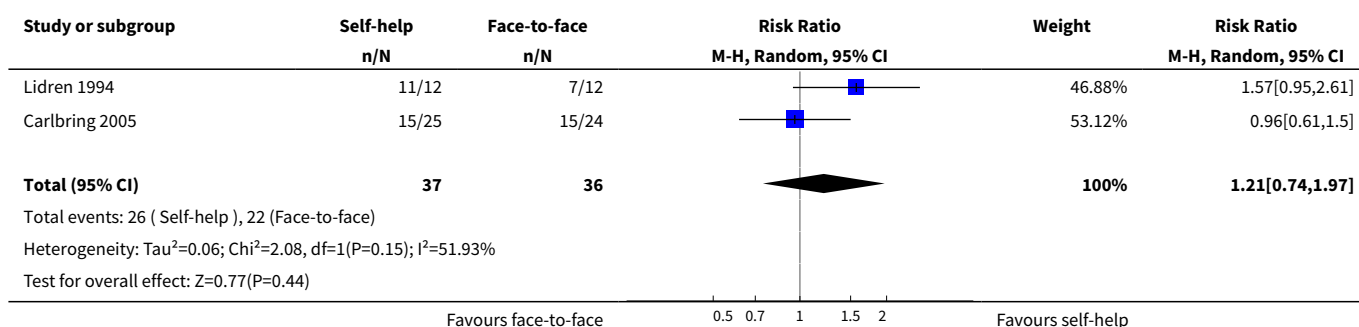
Analysis 2.28. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 28 Depression, self-rated at post-treatment.



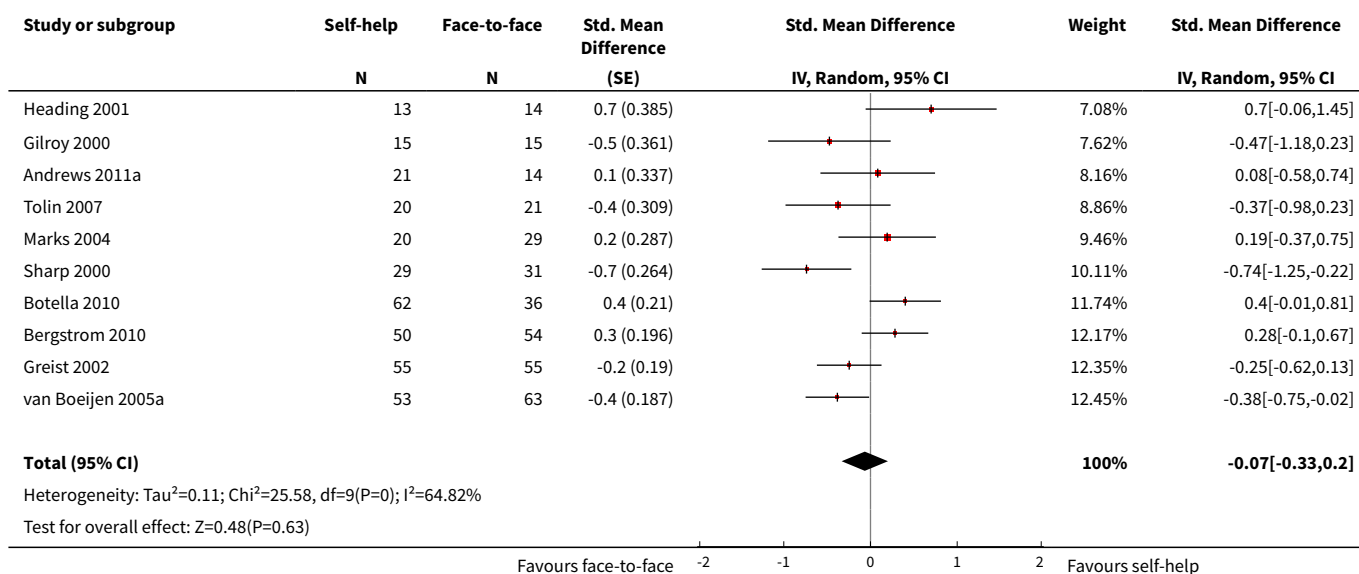
Analysis 2.29. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 29 Depression, clinician-rated at post-treatment.



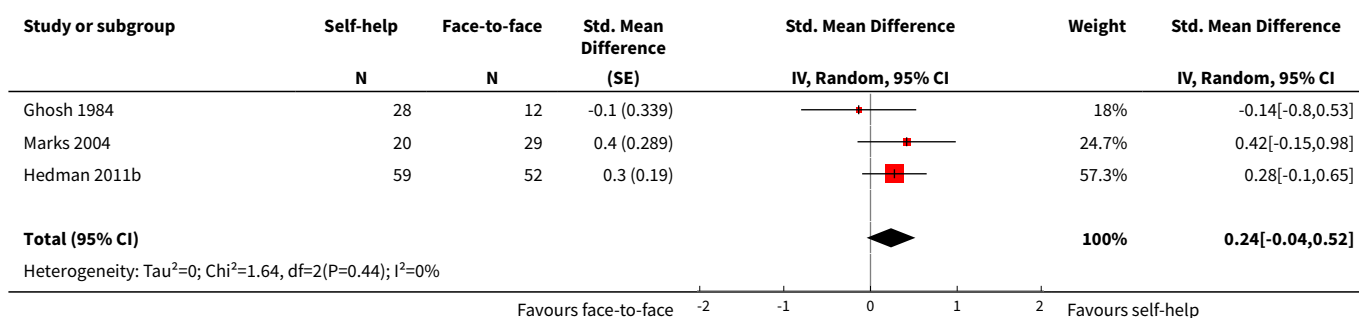
Analysis 2.30. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 30 Depression improvement, self-rated at post-treatment.



Analysis 2.31. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 31 Disability, self-rated at post-treatment.



Analysis 2.32. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 32 Disability, clinician-rated at post-treatment.



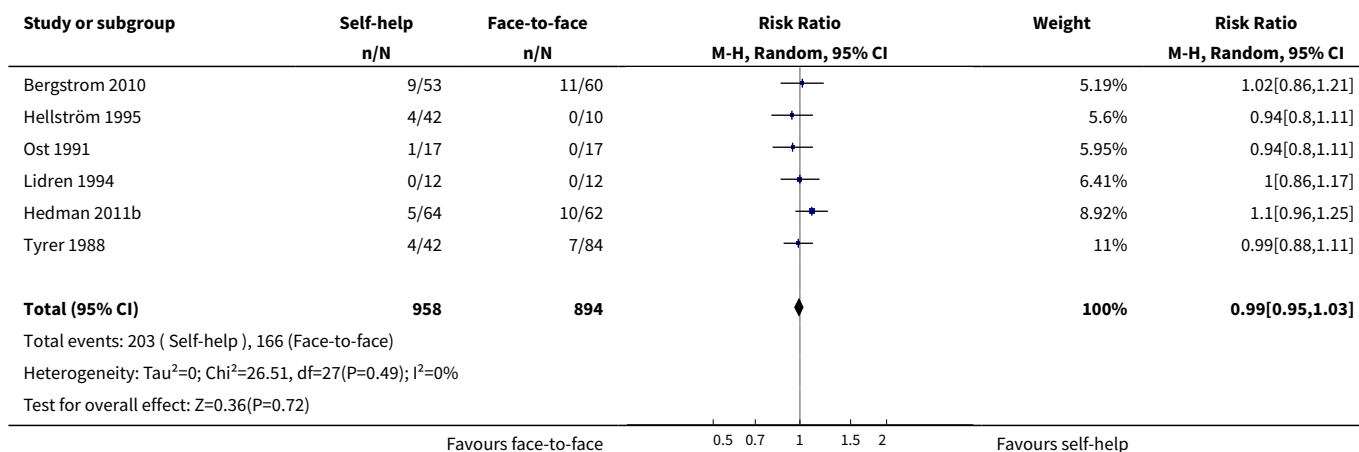
Study or subgroup	Self-help	Face-to-face	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
Test for overall effect: $Z=1.65(P=0.1)$						
Favours face-to-face -2 -1 0 1 2 Favours self-help						

Analysis 2.33. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 33 Quality of Life, self-rated at post-treatment.

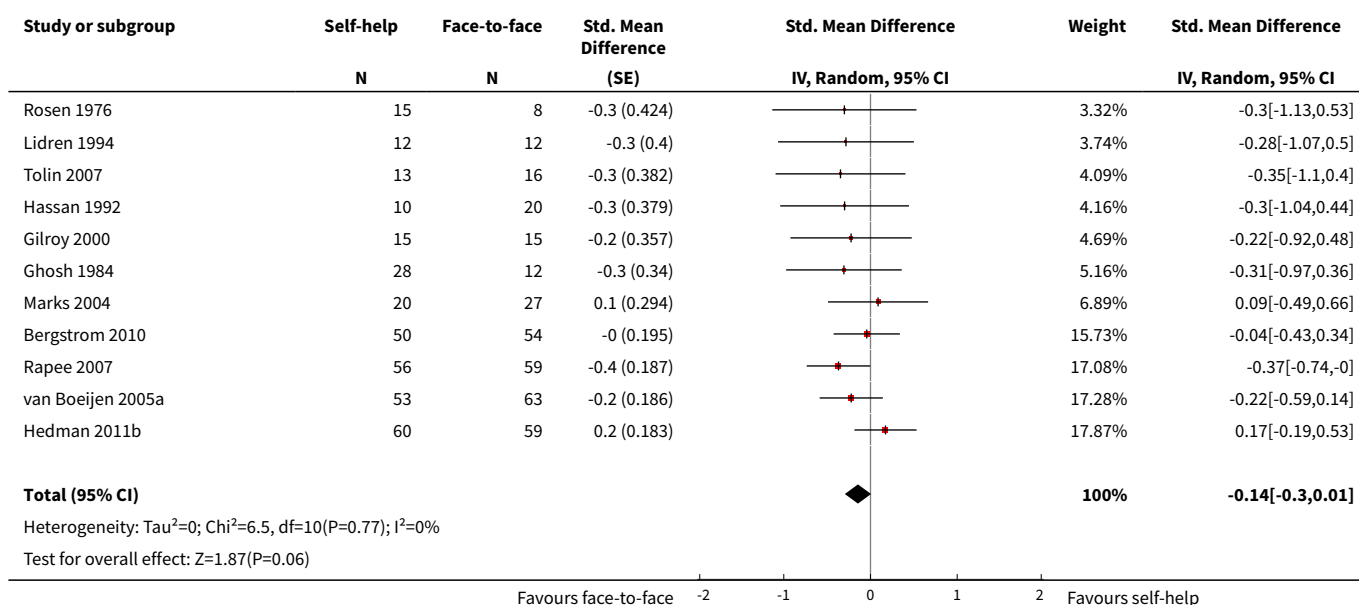
Study or subgroup	Self-help	Face-to-face	Std. Mean Difference	Std. Mean Difference	Weight	Std. Mean Difference
	N	N	(SE)	IV, Random, 95% CI		IV, Random, 95% CI
Tolin 2007	16	16	-0.5 (0.351)		15.36%	-0.54[-1.23,0.15]
Carlbring 2005	25	24	0.2 (0.282)		21.15%	0.2[-0.35,0.76]
Kiropoulos 2008	40	36	0 (0.227)		27.76%	0.03[-0.42,0.47]
Hedman 2011b	63	62	0.3 (0.179)		35.73%	0.3[-0.05,0.65]
Total (95% CI)					100%	0.08[-0.23,0.38]
Heterogeneity: $\tau^2=0.04$; $\chi^2=4.82$, $df=3(P=0.19)$; $I^2=37.72\%$						
Test for overall effect: $Z=0.48(P=0.63)$						
Favours face-to-face -2 -1 0 1 2 Favours self-help						

Analysis 2.34. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 34 Dropout at post-treatment.

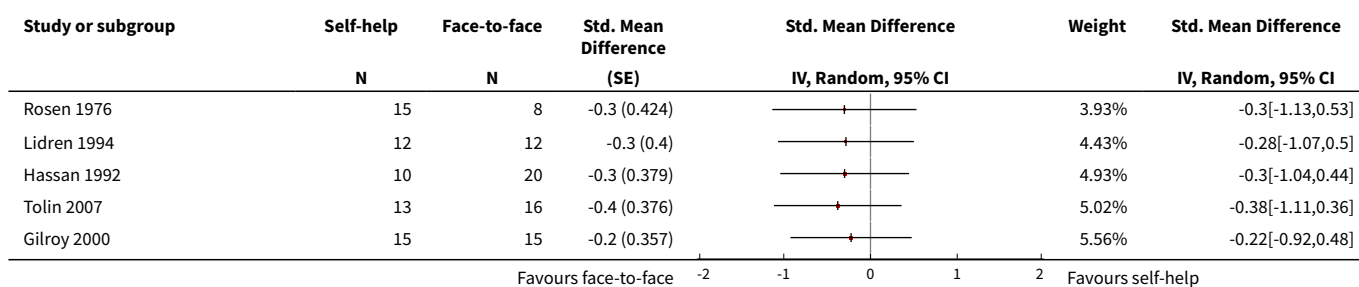
Study or subgroup	Self-help	Face-to-face	Risk Ratio	Weight	Risk Ratio
	n/N	n/N	M-H, Random, 95% CI		M-H, Random, 95% CI
Rosen 1976	7/22	5/11		0.41%	1.25[0.68,2.3]
Tolin 2011	7/19	5/15		0.62%	0.95[0.58,1.56]
Andrews 2011a	9/23	3/14		0.83%	0.77[0.51,1.19]
Baker 1973	0/9	2/9		1.02%	1.27[0.86,1.86]
Botella 2010	32/62	14/36		1.13%	0.79[0.55,1.14]
Marks 2004	17/37	9/39		1.29%	0.7[0.5,0.99]
Kiropoulos 2008	20/46	9/40		1.65%	0.73[0.54,0.99]
Ghosh 1984	7/59	10/29		1.93%	1.35[1.02,1.78]
Tolin 2007	3/20	4/21		1.97%	1.05[0.8,1.39]
Kenardy 2003	15/50	11/45		2.5%	0.93[0.72,1.18]
Heading 2001	2/15	1/14		2.51%	0.93[0.73,1.19]
Gould 1993a	1/12	0/9		2.58%	0.93[0.73,1.19]
Andersson 2009a	2/15	1/15		2.63%	0.93[0.73,1.18]
Gilroy 2000	1/15	2/15		2.63%	1.08[0.85,1.37]
Hassan 1992	1/11	2/22		2.89%	1[0.8,1.26]
Sharp 2000	6/35	6/36		3.44%	0.99[0.81,1.23]
Carlbring 2005	3/25	3/24		3.46%	1.01[0.82,1.24]
van Boeijen 2005a	12/59	17/67		4.2%	1.07[0.88,1.29]
Hazen 1996	5/32	2/28		4.62%	0.91[0.76,1.09]
Greist 2002	19/74	14/69		4.72%	0.93[0.78,1.12]
Rapee 2007	8/56	14/59		4.78%	1.12[0.94,1.34]
Walker 2001	3/32	4/32		5.13%	1.04[0.87,1.23]
Favours face-to-face 0.5 0.7 1 1.5 2 Favours self-help					

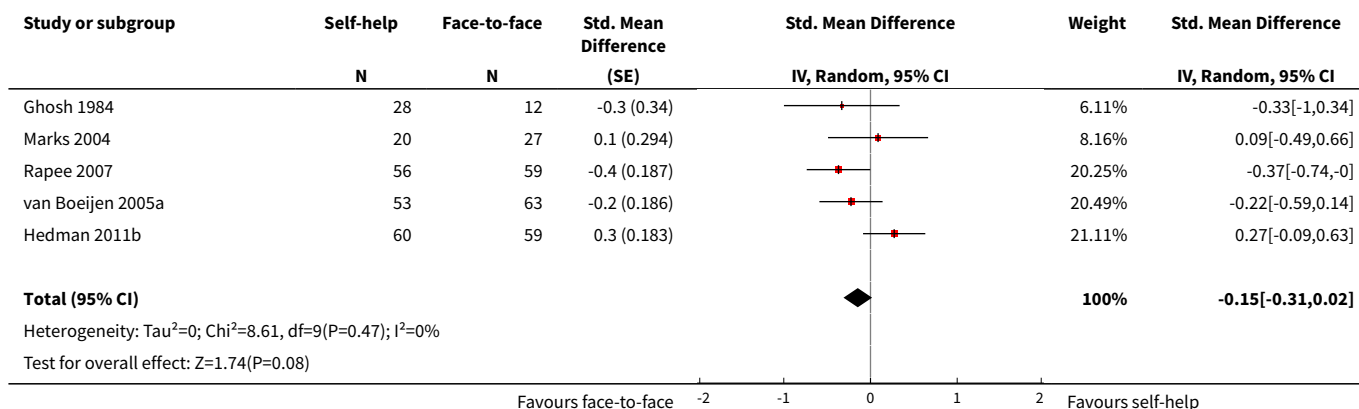


Analysis 2.35. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 35 Anxiety (any measure), self-rated at follow-up.

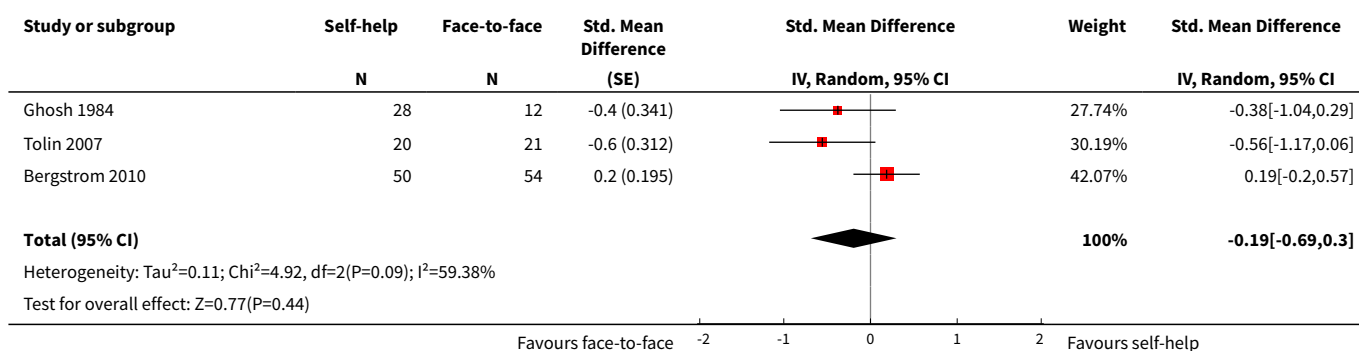


Analysis 2.36. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 36 Specific symptoms, self-rated at follow-up.

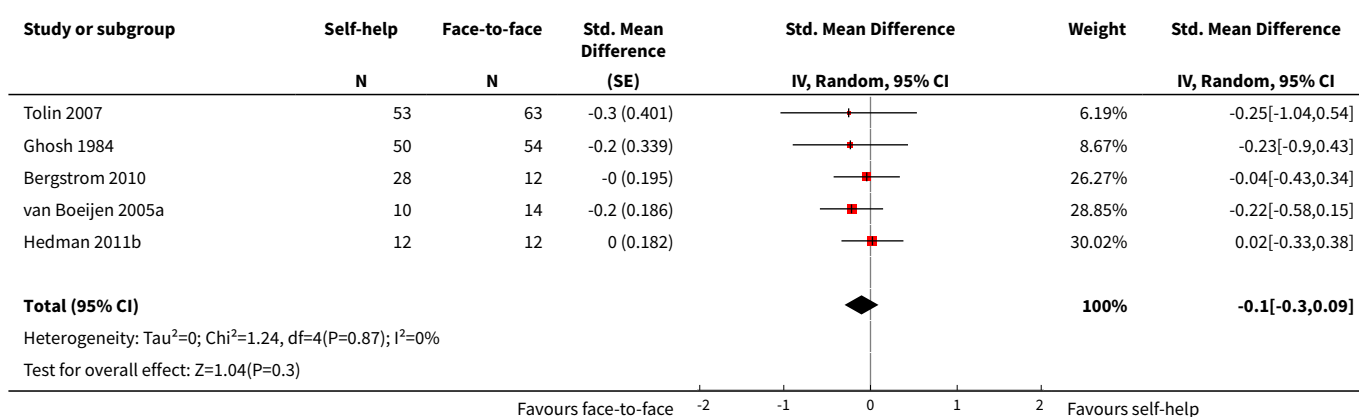




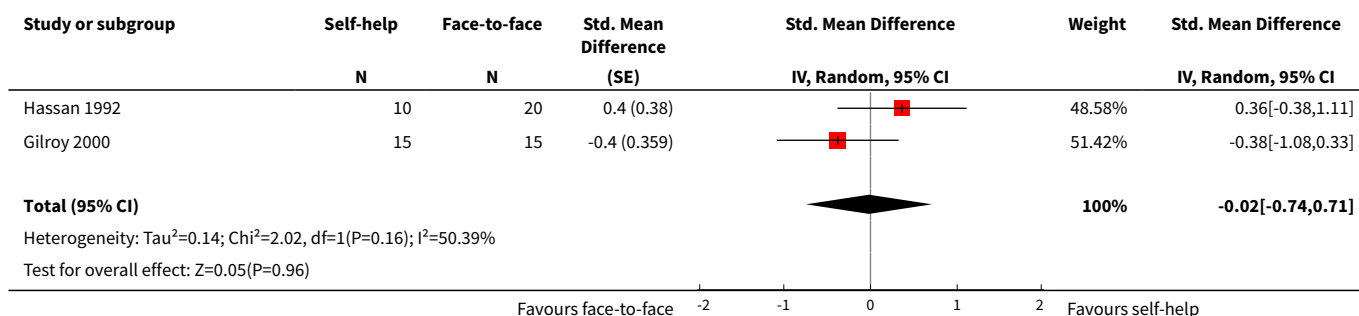
Analysis 2.37. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 37 Specific symptoms, clinician-rated at follow-up.



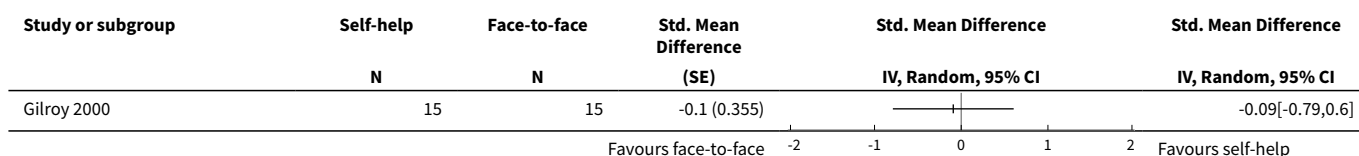
Analysis 2.38. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 38 General symptoms, self-rated at follow-up.



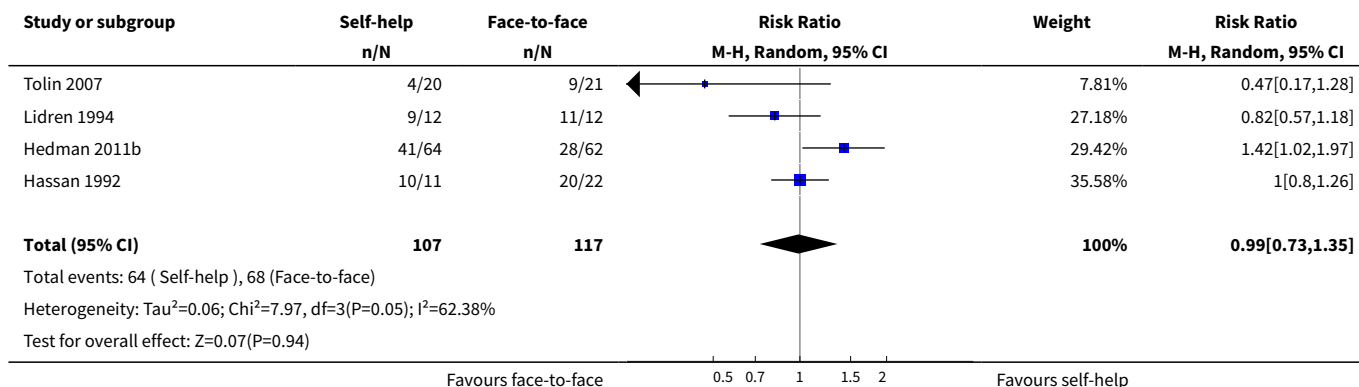
Analysis 2.39. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 39 Behavioural Test (level), clinician-rated at follow-up.



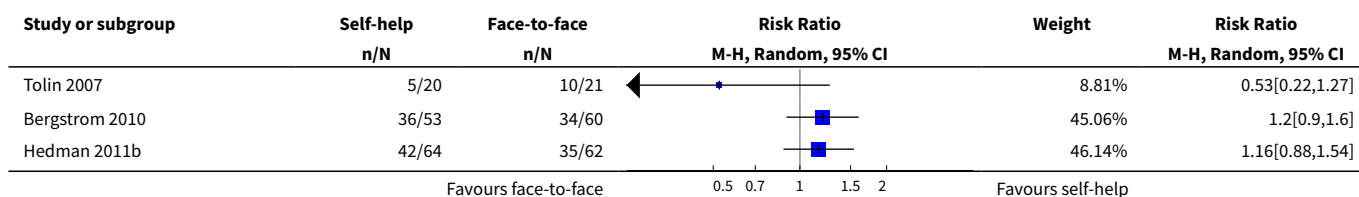
Analysis 2.40. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 40 Behavioural Test (symptoms), self-rated at follow-up.

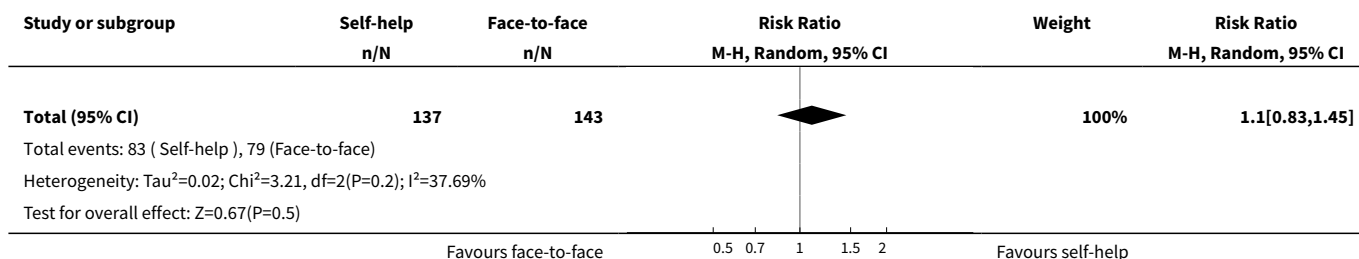


Analysis 2.41. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 41 Response (specific), self-rated at follow-up.

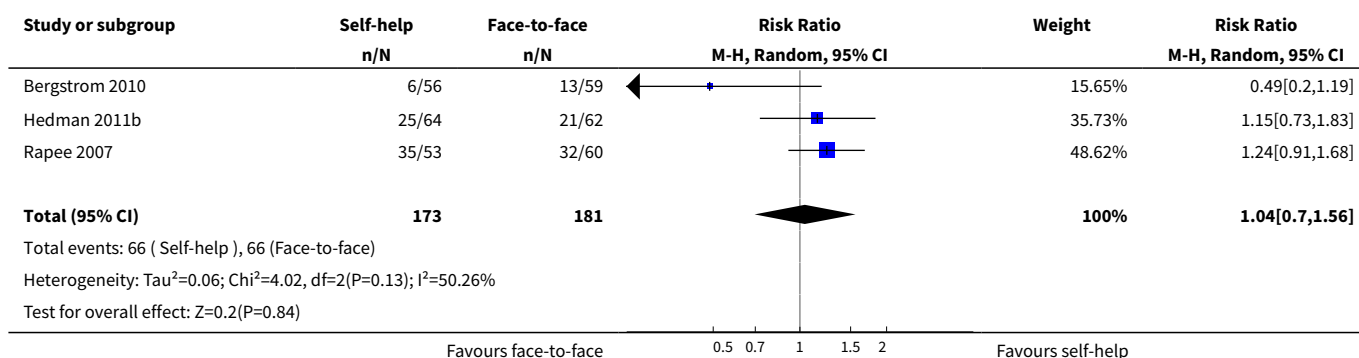


Analysis 2.42. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 42 Response (specific), clinician-rated at follow-up.

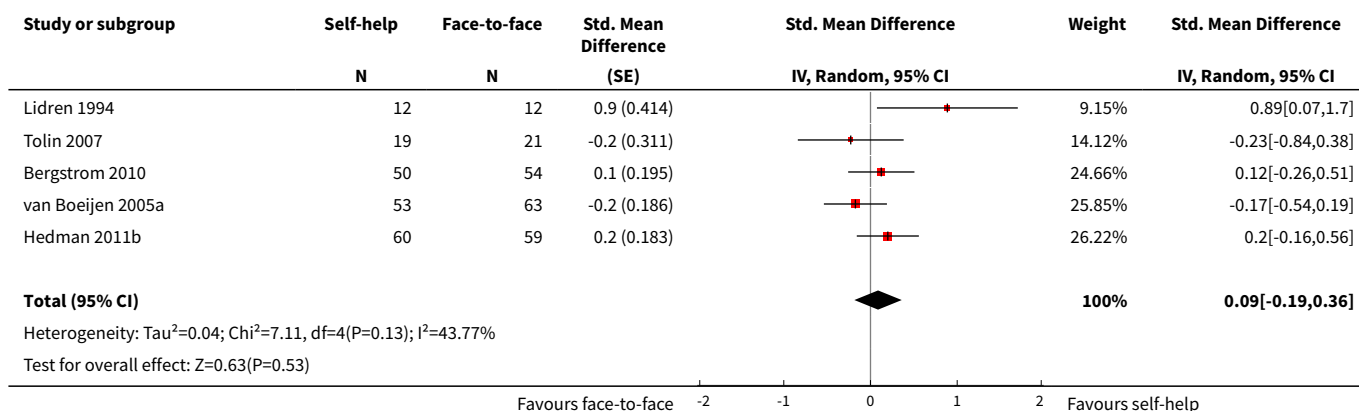




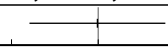
Analysis 2.43. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 43 Recovery, clinician-rated at follow-up.



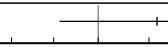
Analysis 2.44. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 44 Depression, self-rated at follow-up.



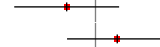
Analysis 2.45. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 45 Depression, clinician-rated at follow-up.

Study or subgroup	Self-help N	Face-to-face N	Std. Mean Difference (SE)	Std. Mean Difference IV, Random, 95% CI	Std. Mean Difference IV, Random, 95% CI
Tolin 2007	10	14	-0 (0.4)		-0.01[-0.79,0.77]
			Favours face-to-face		Favours self-help

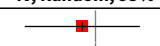
Analysis 2.46. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 46 Depression improvement, self-rated at follow-up.

Study or subgroup	Self-help n/N	Face-to-face n/N	Risk Ratio M-H, Random, 95% CI	Risk Ratio M-H, Random, 95% CI
Lidren 1994	8/12	5/12		1.6[0.73,3.49]
			Favours face-to-face	Favours self-help

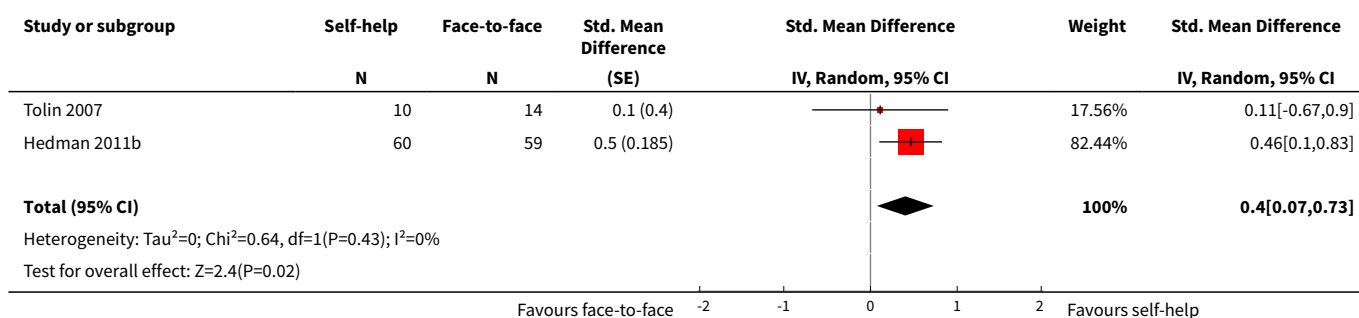
Analysis 2.47. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 47 Disability, self-rated at follow-up.

Study or subgroup	Self-help N	Face-to-face N	Std. Mean Difference (SE)	Std. Mean Difference IV, Random, 95% CI	Weight	Std. Mean Difference IV, Random, 95% CI
Gilroy 2000	15	15	-0.4 (0.359)		12.77%	-0.37[-1.07,0.33]
Tolin 2007	20	21	-0.3 (0.309)		15.81%	-0.33[-0.94,0.27]
Marks 2004	19	27	0.3 (0.295)		16.77%	0.25[-0.33,0.83]
Bergstrom 2010	50	54	0.2 (0.195)		26.76%	0.2[-0.18,0.58]
van Boeijen 2005a	53	63	-0.4 (0.187)		27.89%	-0.36[-0.72,0.01]
Total (95% CI)					100%	-0.1[-0.39,0.19]
Heterogeneity: Tau ² =0.04; Chi ² =6.8, df=4(P=0.15); I ² =41.16%						
Test for overall effect: Z=0.7(P=0.48)						
			Favours face-to-face			Favours self-help

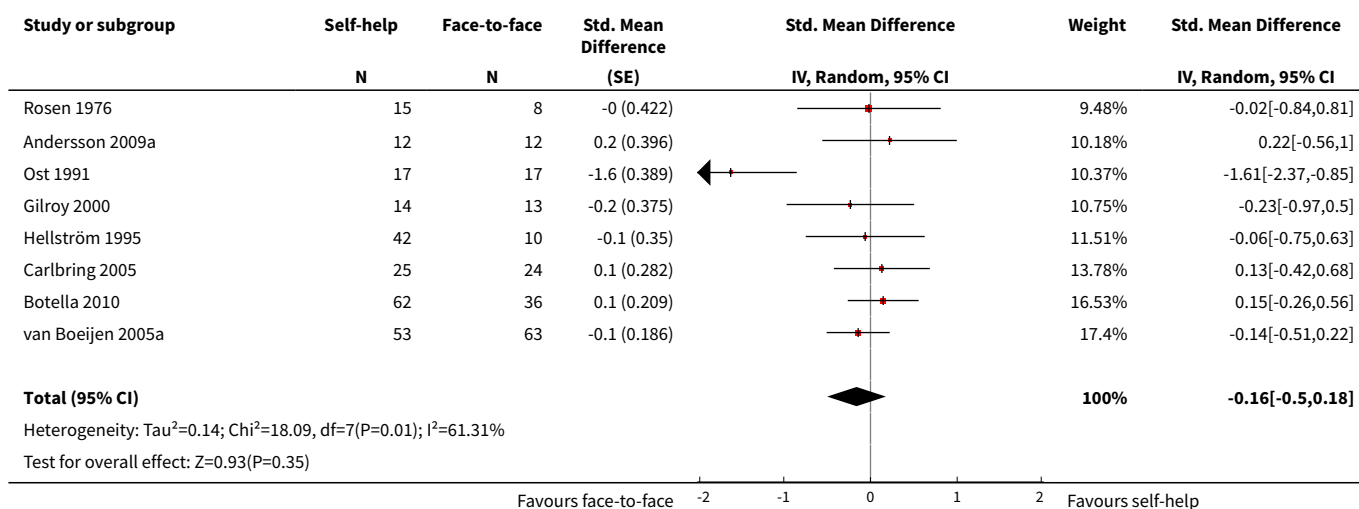
Analysis 2.48. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 48 Disability, clinician-rated at follow-up.

Study or subgroup	Self-help N	Face-to-face N	Std. Mean Difference (SE)	Std. Mean Difference IV, Random, 95% CI	Weight	Std. Mean Difference IV, Random, 95% CI
Ghosh 1984	28	12	-0.2 (0.339)		35.13%	-0.15[-0.82,0.51]
Hedman 2011b	54	51	0.4 (0.195)		64.87%	0.36[-0.03,0.74]
Total (95% CI)					100%	0.18[-0.3,0.65]
Heterogeneity: Tau ² =0.05; Chi ² =1.68, df=1(P=0.19); I ² =40.63%						
Test for overall effect: Z=0.73(P=0.47)						
			Favours face-to-face			Favours self-help

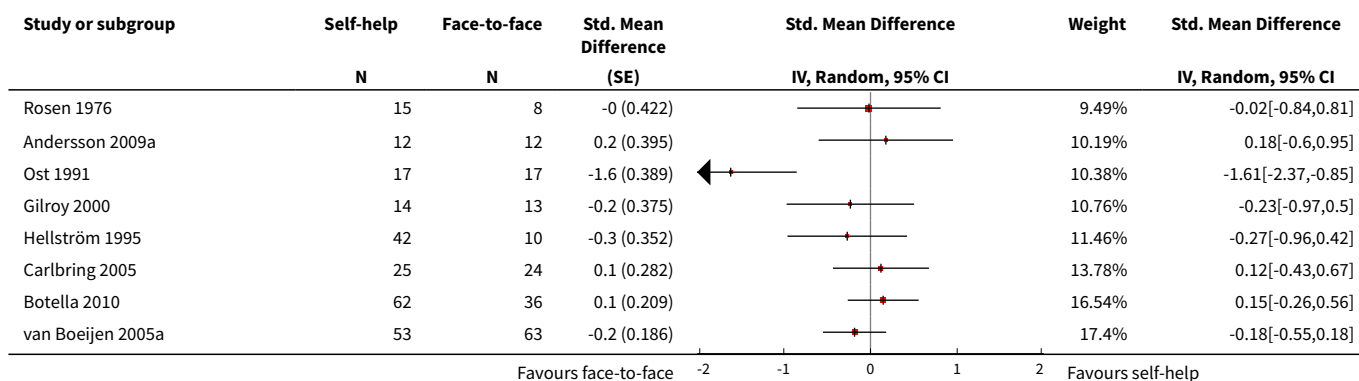
Analysis 2.49. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 49 Quality of Life, self-rated at follow-up.

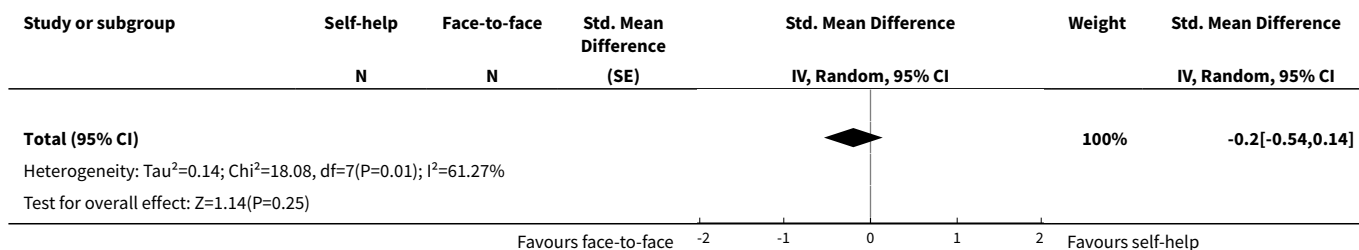


Analysis 2.50. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 50 Anxiety (any measure), self-rated at >6m.

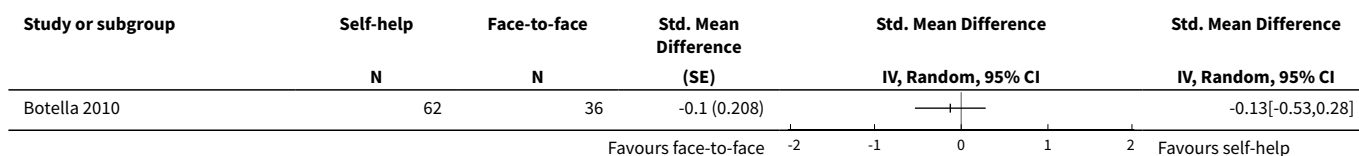


Analysis 2.51. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 51 Specific symptoms, self-rated at >6m.

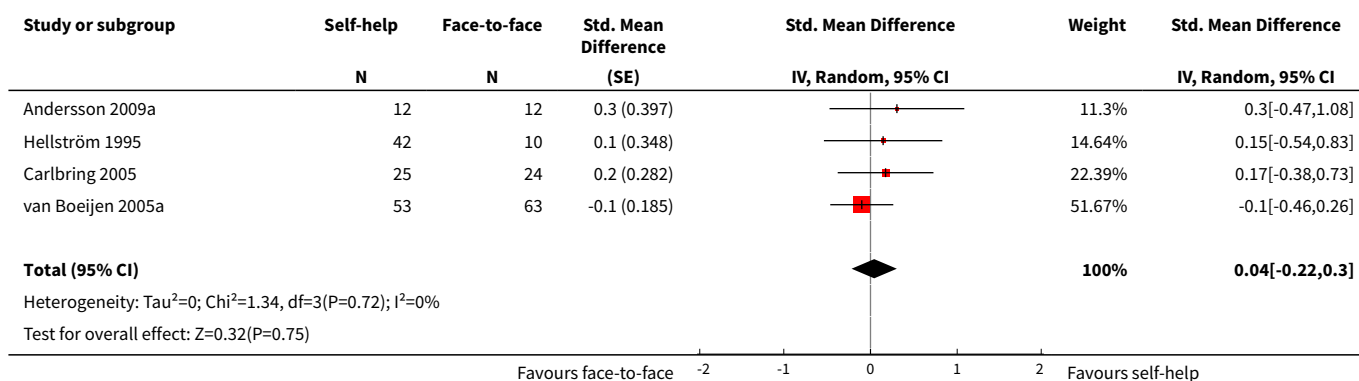




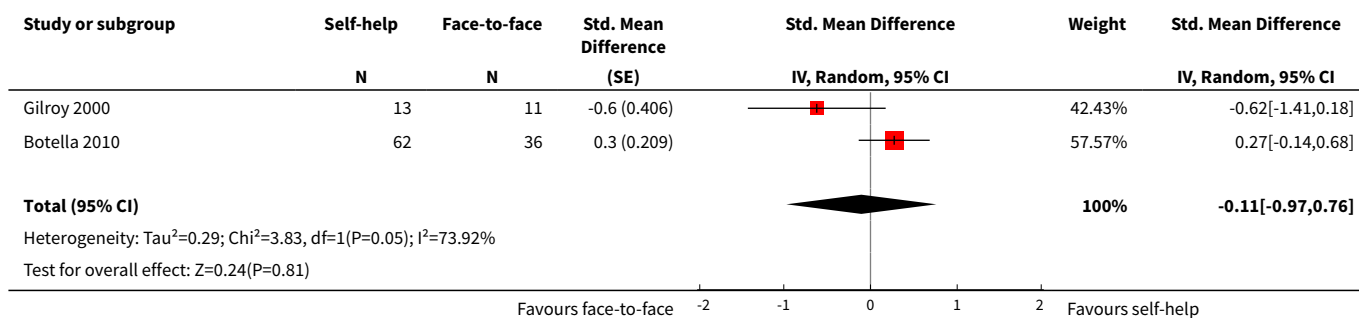
Analysis 2.52. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 52 Specific symptoms, clinician-rated at >6m.



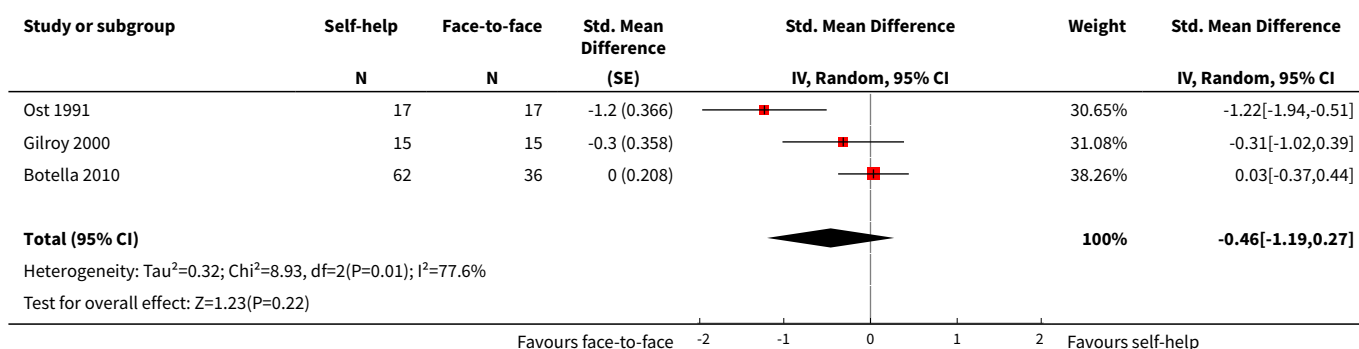
Analysis 2.53. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 53 General symptoms, self-rated at >6m.



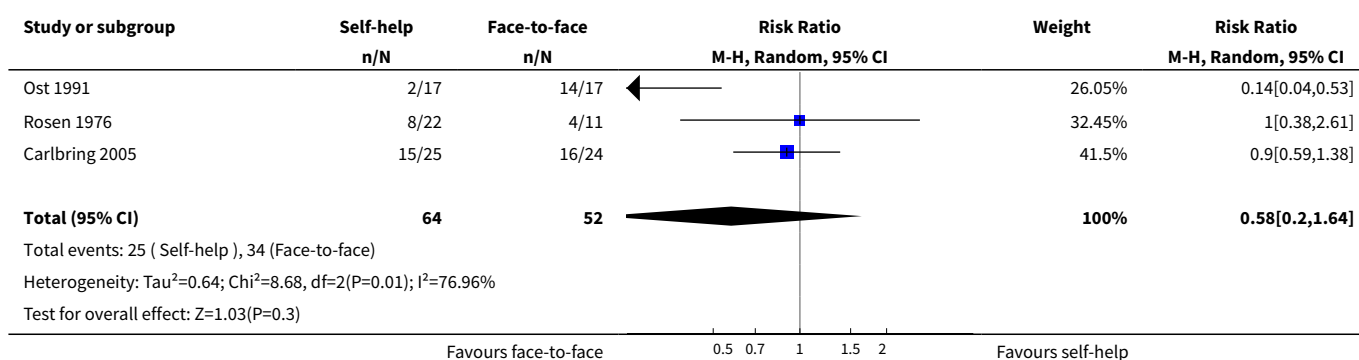
Analysis 2.54. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 54 Behavioural Test (level), clinician-rated at >6m.



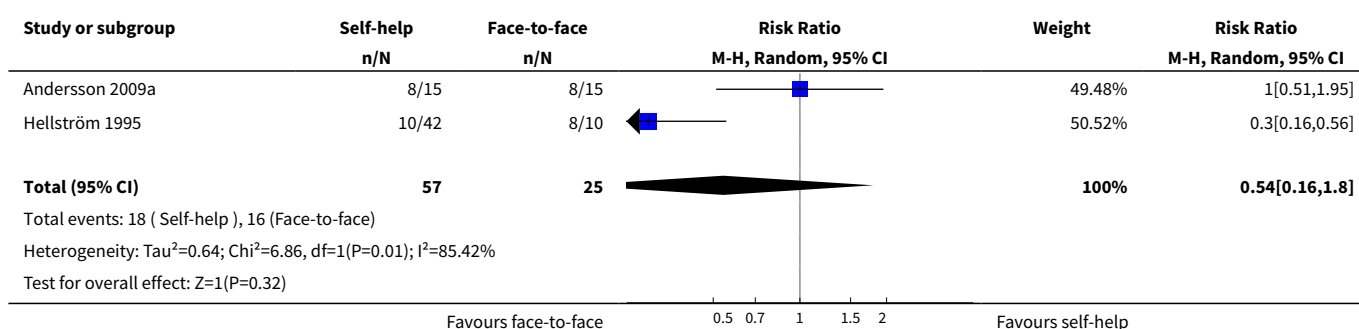
Analysis 2.55. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 55 Behavioural Test (symptoms), self-rated at >6m.



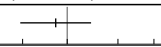
Analysis 2.56. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 56 Response (specific), self-rated at >6m.



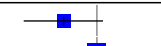
Analysis 2.57. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 57 Response (specific), clinician-rated at >6m.



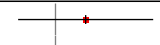
Analysis 2.58. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 58 Response (general), self-rated at >6m.

Study or subgroup	Self-help n/N	Face-to-face n/N	Risk Ratio M-H, Random, 95% CI	Risk Ratio M-H, Random, 95% CI
Carlbring 2005	19/25	20/24		0.91[0.69,1.21]
Favours face-to-face			0.5 0.7 1 1.5 2	Favours self-help

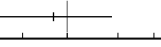
Analysis 2.59. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 59 Recovery, clinician-rated at >6m.

Study or subgroup	Self-help n/N	Face-to-face n/N	Risk Ratio M-H, Random, 95% CI	Weight	Risk Ratio M-H, Random, 95% CI
Botella 2010	33/62	25/36		42.1%	0.77[0.56,1.05]
Carlbring 2005	23/25	22/24		57.9%	1[0.85,1.19]
Total (95% CI)	87	60		100%	0.9[0.64,1.25]
Total events: 56 (Self-help), 47 (Face-to-face)					
Heterogeneity: Tau ² =0.04; Chi ² =3.6, df=1(P=0.06); I ² =72.21%					
Test for overall effect: Z=0.64(P=0.52)					
Favours face-to-face			0.5 0.7 1 1.5 2	Favours self-help	

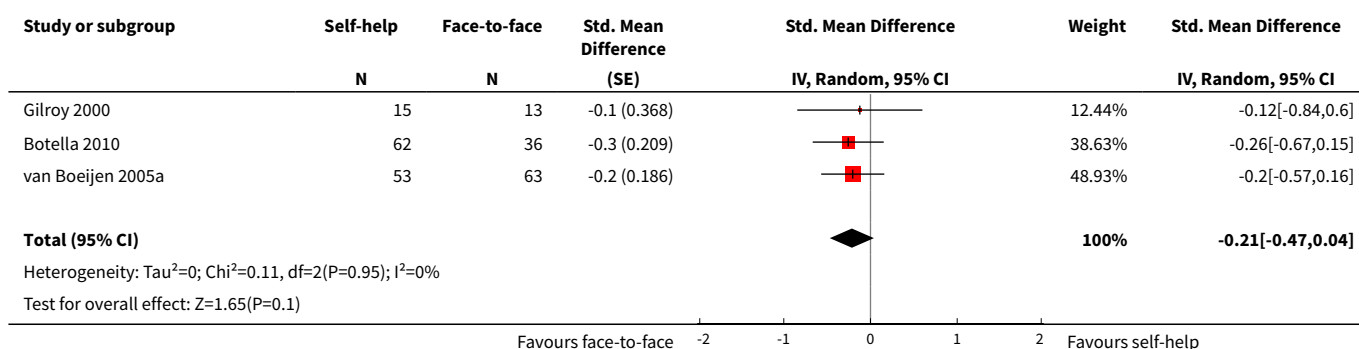
Analysis 2.60. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 60 Depression, self-rated at >6m.

Study or subgroup	Self-help N	Face-to-face N	Std. Mean Difference (SE)	Std. Mean Difference IV, Random, 95% CI	Weight	Std. Mean Difference IV, Random, 95% CI
Carlbring 2005	12	12	0.3 (0.397)		8.15%	0.35[-0.43,1.13]
Botella 2010	42	10	0.5 (0.349)		10.51%	0.45[-0.23,1.14]
Andersson 2009a	25	24	0.4 (0.284)		15.86%	0.37[-0.19,0.92]
Hellström 1995	62	36	-0 (0.208)		29.12%	-0.03[-0.43,0.38]
van Boeijen 2005a	53	63	-0.1 (0.185)		36.37%	-0.12[-0.49,0.24]
Total (95% CI)					100%	0.08[-0.14,0.3]
Heterogeneity: Tau ² =0; Chi ² =4.08, df=4(P=0.4); I ² =1.88%						
Test for overall effect: Z=0.72(P=0.47)						
Favours face-to-face			-2 -1 0 1 2	Favours self-help		

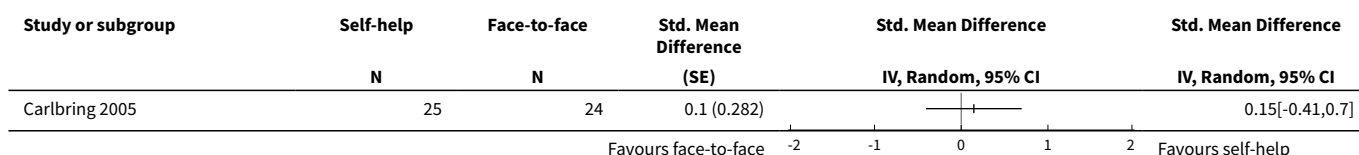
Analysis 2.61. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 61 Depression improvement, self-rated at >6m.

Study or subgroup	Self-help n/N	Face-to-face n/N	Risk Ratio M-H, Random, 95% CI	Risk Ratio M-H, Random, 95% CI
Carlbring 2005	14/25	15/24		0.9[0.56,1.43]
Favours face-to-face			0.5 0.7 1 1.5 2	Favours self-help

Analysis 2.62. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 62 Disability, self-rated at >6m.



Analysis 2.63. Comparison 2 Media-delivered interventions compared to face-to-face interventions, Outcome 63 Quality of Life, self-rated at >6m.



APPENDICES

Appendix 1. Search strategies

1. The Cochrane Depression, Anxiety and Neurosis Review Group's Specialised Register (CCDANCTR)

The Cochrane Depression, Anxiety and Neurosis Group (CCDAN) maintains two clinical trials registers at its editorial base in Bristol, UK: a references register and a studies-based register. The CCDANCTR References Register contains more than 31,900 reports of randomised controlled trials in depression, anxiety and neurosis. Approximately 65% of these references have been tagged to individual, coded trials. The coded trials are held in the CCDANCTR Studies Register, and records are linked between the two registers through the use of unique Study ID tags. Coding of trials is based on the EU-Psi coding manual. Please contact the CCDAN Trials Search Co-ordinator for further details.

Reports of trials for inclusion in the Group's registers are collated from routine (weekly), generic searches of MEDLINE (1950-), EMBASE (1974-) and PsycINFO (1967-) and from quarterly searches of the Cochrane Central Register of Controlled Trials (CENTRAL) and review-specific searches of additional databases. Reports of trials are also sourced from international trials registers c/o the World Health Organisation's trials portal (ICTRP), ClinicalTrials.gov, drug companies and handsearching of key journals, conference proceedings and other (non-Cochrane) systematic reviews and meta-analyses.

Details of CCDAN's [generic search strategies](#) can be found on the Group's Website.

Both Registers (CCDANCTR Studies and CCDANCTR References) were searched by CCDAN's Trials Search Co-ordinator (TSC) (all years to 1 January 2013) using the following free-text terms:

CONDITION = (anxiety or compulsi* or fear* or GAD or hypochondria* or obsess* or OCD or panic or *phobi* or "public speaking" or shy or shyness or "social avoidance")

+

INTERVENTION = (*psychotherap* or *cognitive* or *behavio* or bibliotherap* or *CBT* or acceptance* or assertive* or attribution or reattribution or re-attribution or (brief* and *therap*) or commitment* or desensiti* or expos* or *feedback* or "group therapy" or guided or imag* or implosive or "problem solving" or problem-solving or "solution focused" or solution-focused or schema or "contingency management" or "functional analys*" or mindfulness* or "mind training" or psychoeducat* or "rational emotive" or rational-emotive or REBT or relax* or "role play*" or (self* and (*administer* or *direct* or *examin* or *help)))

+

MODE OF DELIVERY = (audio* or bibliotherapy* or book* or "chat room*" or computer* or CD-ROM or distance* or DVD or homework or information or instruct* or "instant messaging" or iCBT or Internet* or web* or WWW or phone or mobile or e-mail* or email* or leaflet* or material* or manual or manuals or multi-media or multimedia or online* or on-line or pamphlet or pamphlets or program or programme or remote or tele* or tape or taped or video* or virtual* or workbook*)

An additional search of the CCDANCTR-Studies Register was conducted using a set of controlled vocabulary terms for Condition + Intervention (only) to ensure no studies had been missed:

CONDITION = (anxiety or compulsi* or fear* or GAD or hypochondria* or obsess* or OCD or panic or *phobi* or "psychological stress" or "public speaking" or shy or shyness or "social avoidance")

+ INTERVENTION = (*self* or internet or computer or format or setting)

The searches of the Studies Register were limited to adult studies.

2. The Cochrane Central Register of Controlled Trials (CENTRAL)—restricted to records submitted from the CCDANCTR References Register (SR-DEPRESSN)

Authors carried out their own, independent search of the CCDANCTR c/o CENTRAL (all years to 2 April 2012) using the following free-text terms:

#1. anxiety or anxious or agoraphobi* or phobi* or "panic disorder" or asthenia or obsess* or compulsi*

#2. (audiotherapy or bibliotherapy or book* or CD* or computer* or cyber* or DVD or internet* or "interactive voice response" or interapy or inter-therapy or intertherapy or multimedia or multi-media or online or on-line or pamphlet or pamphlets or pc* or pcs or self administered or self desensiti* or self directed or self exposure or self help or tape or taped or video* or web* or www) or ("beating the blues" or "blues begone" or btstep* or bt step* or caccbt or CALM or CAVE or ccbt or "coping with panic" or fearfighter or "fear fighter" or ffeducation or "ff education" or "glasgow steps" or internetff or internet ff or "living with fear" or "living life to the full" or "managing anxiety" or moodgym or "mood gym" or netcope or net cope or nettff or nett ff or ocfighter or oc fighter or "overcoming anxiety" or "overcoming obsessive" or "overcoming panic" or "overcoming shyness" or "panic control" or "panic online" or "panic surfing" or positivestep or "positive step" or standaloneff or standalone ff or "stop obsessing" or stresspac or "talk to me"):ti,ab,kw

#3. SR-DEPRESSN

3. Ovid MEDLINE and PsycINFO (cross-search)

Additional searches were conducted on Ovid MEDLINE (23 February 2013) and PsycINFO (February, Week 2, 2013) simultaneously, using the following terms:

1. exp Agoraphobia/
2. exp Hypochondriasis/
3. exp Obsessive-compulsive disorder/
4. exp Panic disorder/
5. exp Phobic disorders/
6. (fear\$ adj3 (air travel or animal\$ or blood\$ or buses or ((closed or public) adj2 space\$) or crowd\$ or dark\$ or dental\$ or dentist\$ or dog\$1 or dying or flight\$ or fly or flying or height\$ or hypochondriacal or injection\$ or injur\$ or lightening or movement\$ or needle\$ or negative evaluation or panic\$ or performance or plane\$ or snake\$ or social or space\$ or spider\$ or sweat\$ or thunder\$ or train\$ or travel \$ or tremble\$ or water)).ti,ab.
7. (acrophob\$ or agoraphob\$ or agrophobi\$ or ((anxiety\$ or phobi\$) adj2 neuros\$) or anxiety disorders or claustrophob\$ or Compul\$ or emetophob\$ or GAD or ((generalised or generalized or health) adj2 anxiety) or hypochondri\$ or ((interpersonal or inter personal or social or speak\$) adj3 (anxiety\$ or avoid\$ or fear\$)) or kinesiphob\$ or (specific adj2 (phobi\$ or fear\$)) or Obsess\$ or OCD or (panic adj3 (attack \$ or disorder\$ or fear or symptoms)) or SAD or shyness).ti,ab.
8. anxiety.ti.
9. stress.ti.
10. anxiety.ti,ab.
11. 9 and 10
12. 1 or 2 or 3 or 4 or 5 or 6 or 7 or 8 or 11
13. exp bibliotherapy/
14. (audiotherapy or bibliotherapy or biblio therap\$ or book\$ or CD\$ or computer\$ or cyber\$ or DVD or internet\$ or interactive voice response or interapy or inter-therapy or intertherapy or multimedia or multi media or online or on-line or pamphlet or pamphlets or pc\$ or pcs or (self adj (administered or desensiti\$ or directed or exposure or help)) or tape or taped or video\$ or web\$ or www).ti,ab.
15. 13 or 14
16. (CBT or (cognitive adj2 behav\$) or ((cognitive or behav\$) adj3 (therap\$ or intervention\$ or program\$ or restructur\$)) or CT or desensiti \$ or exposure or flooding or implosion or MCBT or mindfulness\$ or metacognitive or mymct or modelling or relax\$).ti,ab.
17. 15 and 16
18. (blues begone or caccbt or CALM or CAVE or ccbt or c cbt or (coping adj2 panic) or ffeducation or ff education or glasgow steps or internetff or internet ff or (living adj2 fear) or (living life adj3 full) or managing anxiety or moodgym or mood gym or netcope or net cope or nettff or nett ff or ocfighter or oc fighter or overcoming anxiety or overcoming obsessive or overcoming panic or overcoming shyness or (panic adj2 control) or (panic adj2 online) or (panic adj2 surfing) or positivestep or positive step or ((shyness or wellbeing) adj4 (program or programme)) or standaloneff or standalone ff or stop obsessing or stresspac or talk to me).ti,ab.

19. (bibliotherapy or self-help or interapy or (internet-based adj2 (intervention\$ or management or self-help or treatment\$ or therapy or therapies or program\$)))ti.
20. 17 or 18 or 19
21. 12 and 20
22. ((beating adj2 blues) or btstep\$ or bt step\$ or fearfighter or fear fighter).ti,ab.
23. 21 or 22
24. exp clinical trial/
25. random\$.af.
26. ((clinical\$ adj2 control\$) or cross over or crossover or dummy or experiment\$ or factorial or mask\$ or ((single\$ or doubl\$ or trebl\$ or tripl\$) adj2 blind) or (singleblind\$ or doubleblind\$ or trebleblind\$ or tripleblind\$) or placebo\$ or trial or ((wait list or waitlist or waiting list or no treatment) adj (condition or control or group)))ti,ab.
27. 24 or 25 or 26
28. limit 27 to animal
29. limit 27 to animal studies
30. 28 or 29
31. limit 30 to humans
32. 30 not 31
33. 27 not 32
34. 23 and 33
35. remove duplicates from 34

CONTRIBUTIONS OF AUTHORS

Both review authors contributed to designing the project and to writing the protocol. EM-W conducted the searches and screened citations for eligibility with PM or with G.J. Melendez-Torres. EM-W extracted data (with Robyn Cox (RC), Jean Junior (JJ), Andreas Hein Willius (AH) or Kayleigh Kew (KK)), conducted the analyses and wrote a first draft of the results. Both review authors contributed to the interpretation and discussion.

DECLARATIONS OF INTEREST

None.

SOURCES OF SUPPORT

Internal sources

- Centre for Evidence-Based Intervention (Department of Social Policy and Intervention), University of Oxford, UK.
- Centre for Outcomes Research and Effectiveness (Research Department of Clinical, Educational & Health Psychology), University College London, UK.

External sources

- No sources of support supplied

DIFFERENCES BETWEEN PROTOCOL AND REVIEW

In addition to the CCDAN Controlled Trials Register, we searched MEDLINE and PsycINFO. In both cases, we used terms to identify media-delivered interventions for anxiety. Methods for assessing risk of bias were updated to follow current guidelines. We included measures of disability that were not specified in the protocol. We were unable to assess the use of other services, satisfaction, adverse events or cost.

The prespecified primary outcome, "Specific symptoms of anxiety", had to be refined because studies used several relevant measures. First, we analysed the average effects of all measures for each study. For studies of a single disorder, we also analysed specific measures of that disorder (e.g. LSAS) apart from general measures of anxiety (e.g. BAI). For studies including behavioural tests, which are commonly used to evaluate phobic avoidance, we analysed the level in the hierarchy achieved and subjective distress.

We added a Summary of findings table.

INDEX TERMS

Medical Subject Headings (MeSH)

Anxiety Disorders [*therapy]; Cognitive Behavioral Therapy [*methods]; Depression [therapy]; Desensitization, Psychologic [methods]; Health Services Accessibility; Pamphlets; Randomized Controlled Trials as Topic; Relaxation Therapy [methods]; Self Care [*methods]; Therapy, Computer-Assisted [methods]; Treatment Outcome; Videotape Recording

MeSH check words

Adult; Humans